CLINICAL PSYCHIATRY

Diagnostic and treatment standards

A clinical guide based on ICD-11 and DSM-5-TR

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Preface

Address to the reader

Dear colleague — physician, clinical psychologist, psychotherapist, resident, student.

This book grew out of an absence. I have been seeing patients for twenty-three years, and for most of that time I ran into the same wall: the diagnosis was clear — but the criteria that confirm it sat in one source, the order of investigation in another, the treatment algorithm in a third, and the prognosis nowhere at all. That road had to be walked again for every patient.

Searching made it heavier, not lighter. On the first page of results an obsolete classification, a pharmaceutical brochure and somebody's evidence-free “method” stand side by side. Telling which one has a study behind it and which has only marketing takes hours you do not have: the patient is sitting opposite you and expects an answer now. Teaching residents made this plainer still — a wrong decision usually comes not from ignorance but from trusting the first source that appears.

I looked for a ready solution for a long time. I did not find one, so I had to write it. Everything in this book follows from a single principle: one disorder, one page. Criteria, differential diagnosis, investigation, treatment, prognosis and the most persistent myths together, with the codes of three classifications and a named source behind every claim. Only what can be verified goes in; the rest is called by its name in a section of its own.

What you have in front of you is “Clinical Psychiatry: Standards of Diagnosis and Therapy”, the first systematic clinical psychiatry reference in the Azerbaijani language. It was not written to be read through; it was written to be used. The patient is sitting opposite you and you have ten minutes. Every chapter is built the same way — eleven sections in a fixed order — so that you never have to hunt for what you need.

The book is built on ICD-11. When that classification came into force in 2022 it changed more than the codes: it rewrote how we look at personality disorder, what we mean by trauma, and the old boundary between the organic and the functional. At the same time I have not abandoned the post-Soviet clinical school — the tradition of Popov and Vid. Where the international guideline is stronger, the guideline is cited; where the classical school sees more clearly, that is said as well. Disagreements are not smoothed over; they are named.

Two of the protocols presented here are my own: the treatment algorithm for nocturnal enuresis in children and a two-stage strategy for psychogenic depression. Both are modifications of existing international guidelines, and I classify their level of evidence as IV — expert clinical experience rather than randomised controlled trials. I state this explicitly so that the reader knows precisely what the evidence base is. I invite you to test them, challenge them, and trial them in your own practice.

One final request: please do not treat this book as the last word. Psychiatry evolves rapidly, and I intend this text to be a living document. If you identify an error or an omission, I would welcome your feedback. The next edition will be stronger for it.

ABOUT THE AUTHOR

The author of this book is Kenan Rahimov, a psychiatrist and psychotherapist with 23 years of clinical experience (since 2003). He graduated from Azerbaijan State Medical University in 2001 and completed his internship the same year at Clinical Psychiatric Hospital No. 2 in Baku. From 2001 to 2003 in Saint Petersburg, at the V. M. Bekhterev Psychoneurological Research Institute, he completed his clinical residency with honours: in the department of psychotherapy for neuroses under Professor B. D. Karvasarsky, Chief Psychotherapist of the Russian Federation and an internationally recognised specialist, and in the department of psychotherapy for psychoses under Professor V. D. Vid, one of the first psychoanalysts in the post-Soviet region certified by the International Psychoanalytical Association (IPA). Over 23 years of practice he has treated more than 1,000 patients with depressive disorders and more than 1,000 children with nocturnal enuresis; in 2025 alone his clinical team successfully treated more than 250 children. Since 2020 he has been the founder of the RAGIMOFF Professional School of Psychology; since 2021, the authorised representative in Azerbaijan of the Karvasarsky Institute in St Petersburg and head of its continuing professional education programme in clinical psychology; and since 2022, an ambassador of the International Psychotherapy Association (IPAS). The two clinical protocols of his own presented in this book — the treatment of nocturnal enuresis in children and a two-stage strategy for treating psychogenic depression — represent modifications of international evidence-based guidelines (NICE, APA, WFSBP) grounded in the author's long-term clinical experience and retrospective observations. They aim at higher clinical effectiveness in a shorter time than the standard international protocols; the level of evidence should be regarded as IV (expert clinical experience). The author's recommendations rest on clinical expertise and 23 years of uninterrupted practice; readers are invited to test them, evaluate them and, where necessary, adapt them in their own practice.

INTRODUCTION

About this book

This book is the first systematic presentation of the modern standards of clinical psychiatry in the Azerbaijani language. The book is built on the 11th revision of the International Classification of Diseases (ICD-11) and integrates its structure, diagnostic criteria and treatment approaches in a way adapted to the Azerbaijani clinical tradition and the post-Soviet psychiatric school.

The book has been prepared as a reference guide for practising physicians — psychiatrists, psychotherapists, neurologists, primary care physicians, clinical psychologists, final-year medical students and residents.

What this book is and why it was written

This book is a clinical reference guide, not a theoretical psychiatry textbook. It presents the diagnostic criteria, clinical manifestations and treatment protocols for each mental disorder in a standardised format. A detailed pathophysiological, neurobiological or historical account of each topic lies beyond the scope of this book; the aim is to enable the physician to find the necessary information quickly and accurately in the presence of the patient.

The reasons for writing the book:

There is no modern psychiatry reference guide in the Azerbaijani language. The existing Azerbaijani-language literature consists mostly of translations or textbooks based on the school of the late twentieth century. The innovations of ICD-11 (in force since 2022) and DSM-5-TR (2022) have not been systematically reflected in the Azerbaijani language.

Bringing diagnostic classifications into a unified system. In clinical practice, physicians encounter ICD-10 (still used by the Ministry of Health for official statistics), ICD-11 (the new standard) and DSM-5-TR (the classification predominant in international scientific publications). This book presents all three systems in parallel so that the physician can use the correct code and terminology in different contexts.

Integration of the official clinical protocols of Azerbaijan. The protocols prepared by the Center for Public Health and Reforms of the Ministry of Health and approved by decisions of the board of the Ministry of Health — schizophrenia, 2009; anxiety disorders, 2013; obsessive-compulsive disorder (OCD), 2015; depression, year indicated — constitute the local clinical basis of this book. The

concordances and differences between international recommendations — from the National Institute for Health and Care Excellence (NICE) of the United Kingdom, the American Psychiatric Association (APA), the World Federation of Societies of Biological Psychiatry (WFSBP) and the World Health Organization (WHO) — and the official Azerbaijani protocols are noted separately.

Legal and ethical guidance. The rights and duties of the physician — both psychiatrist and clinical psychologist — are presented in a separate section within the framework of the Law of the Republic of Azerbaijan "On Psychiatric Care" of 12 June 2001 and the Law "On Psychological Care". Behaviour towards a patient with suicidal ideation, the grounds for involuntary hospitalisation and the rules for applying compulsory medical measures are real clinical situations that require an answer. This book answers them.

Why ICD-11?

WHO adopted ICD-11 in 2019, and its official implementation began in 2022. ICD-11 has brought profound paradigm changes in the chapter on mental disorders:

Complex post-traumatic stress disorder (PTSD) (6B41) — a new official diagnosis;

Prolonged grief disorder (6B42) — an independent diagnosis added to ICD-11;

Transition of personality disorders from a categorical system to a dimensional system — a major conceptual innovation;

Replacement of somatoform disorders with bodily distress disorder (BDD);

Removal of gender incongruence from the psychiatric chapter;

Gaming disorder has been defined as an official diagnosis;

Compulsive sexual behaviour disorder (CSBD) has been added as a new category;

Transfer of attention-deficit/hyperactivity disorder (ADHD) to the group of neurodevelopmental disorders;

Placement of hypochondriasis in the group of obsessive-compulsive and related disorders;

Abolition of the organic–functional dichotomy.

These changes require reassessment both in nosology and in treatment standards.

Structure of the book

The book consists of two main parts:

PART I — Introductory sections:

Preface (this section)

Table of contents — based on a hierarchy corresponding to the structure of ICD-11

Introduction to psychopathology — description of symptoms and syndromes (cognition, perception, thinking, mood, behaviour, memory, volition, consciousness)

Clinical interview and assessment of mental status — clinical protocol procedure, standards for describing mental status

Legal and ethical aspects — practical guidance for the psychiatrist and clinical psychologist on the basis of Azerbaijani legislation, sample documents

PART II — Mental disorders (according to the structure of ICD-11):

All 21 main groups of Chapter 6 of ICD-11 are presented in separate sections. The same standard structure is retained for each section:

Definition and nosology (brief)

Aetiology

Epidemiology

Clinical manifestations

Diagnosis (ICD-11 → ICD-10 → DSM-5-TR — parallel comparison)

Differential diagnosis

Methods of examination (clinical, laboratory and instrumental)

Treatment (with evidence-based recommendation grades)

Prognosis

References

At the end — References and Appendices:

Complete list of all sources used (Azerbaijani sources, those from the countries of the Commonwealth of Independent States (CIS), international sources)

Clinical scales and questionnaires (Patient Health Questionnaire — PHQ-9, Generalized Anxiety Disorder scale — GAD-7, PTSD checklist — PCL-5, Edinburgh Postnatal Depression Scale — EPDS, Yale-Brown Obsessive Compulsive Scale — Y-BOCS, Columbia Suicide Severity Rating Scale — C-SSRS and others)

Psychiatric emergencies (algorithm)

Rules of military medical assessment (in Azerbaijan)

Organisation and features of the chapters

Organisation of the chapters

Each clinical chapter consists of 11 sections:

Definition and brief description – the nature of the disorder, ICD-11 innovations;

Epidemiology – prevalence, sex, age, comorbidity;

Aetiology and pathogenesis – biological, psychological and social factors, the Popov-Vid approach;

Diagnostic criteria – a comparison of the ICD-11 Clinical Descriptions and Diagnostic Guidelines (CDDG), ICD-10 and DSM-5-TR;

Clinical forms (where relevant) or features of the clinical presentation;

Differential diagnosis – in tabular format;

Examination and assessment – clinical interview, screening scales, and laboratory tests;

Treatment – general strategy, phased approach, psychotherapy, pharmacotherapy, and specific clinical situations;

Follow-up and prognosis;

Sources – ICD-11 CDDG 2024, APA, NICE, WFSBP, ISTSS, ISSTD and other international clinical recommendations;

Historical concepts – the transition from classical concepts to the modern approach.

Terminology

Psychiatric terminology has been systematised in the Azerbaijani language. Many terms reflect adaptations originating from classical Russian-Soviet psychiatry, because psychiatric education in Azerbaijan has traditionally been conducted in Russian. International terms are provided in English, with the note “Eng” in parentheses (for terminological clarity).

Integration of the Popov-Vid school

The book “Modern Clinical Psychiatry” by Yu.V. Popov and V.D. Vid (St. Petersburg: Rech, 2006, 2nd edition) is a classic textbook of post-Soviet clinical psychiatry. Their method — classical nosology, the biopsychosocial model and practical clinical experience — has been integrated into this book wherever it is consistent with contemporary international recommendations. Where differences exist (e.g. the dimensional personality model of ICD-11), this is explicitly indicated.

Treatment standards

For each disorder, treatment is referenced to international clinical guidelines:

American Psychiatric Association (APA) clinical practice guidelines;

National Institute for Health and Care Excellence (NICE);

World Federation of Societies of Biological Psychiatry (WFSBP);

Canadian Network for Mood and Anxiety Treatments (CANMAT)/International Society for Bipolar Disorders (ISBD) — bipolar affective disorder;

International Society for Traumatic Stress Studies (ISTSS);

International Society for the Study of Trauma and Dissociation (ISSTD);

American Academy of Child and Adolescent Psychiatry (AACAP);

National drug lists and guidelines.

Drug names are given both by the international nonproprietary name (INN) and by the trade (brand) name, because drugs on the Azerbaijani market are mostly known by their trade names.

The Azerbaijani clinical context

Where appropriate, notes specific to the Azerbaijani context are provided:

Issues of implementation in primary health care;

Cultural characteristics and stigma;

Legal and ethical frameworks;

Constraints on available resources;

Trauma disorders in veterans of the Second Karabakh War;

Instruments whose reliability has been validated in the Azerbaijani language.

Scales and instruments

Appendix B presents the most commonly used screening and assessment instruments either in full text (open-licensed) or in descriptive form (commercially licensed).

Sources and approach

Priority sequence of authoritative sources. For each topic, one principal authoritative guideline is presented first, after which other guidelines are introduced for comparison or supplementation. This approach aims to maintain a balance between academic precision and concreteness and to avoid duplication of information. Principal authoritative guideline selections for each disorder:

Schizophrenia: NICE CG178 (2014, updated 2019) + Clinical Protocol of the Ministry of Health of the Republic of Azerbaijan (2009)

Depression: NICE NG222 (2022) — “Depression in adults: treatment and management” guideline + Clinical Protocol of the Ministry of Health + APA Practice Guideline (2010)

Bipolar disorder: CANMAT/ISBD (2018, updated 2021)

Anxiety disorders: NICE Clinical Guideline 113 (2011/2020). “Generalised anxiety disorder and panic disorder in adults: management”. NICE CG113 (2011) + Clinical Protocol of the Ministry of Health (2013) + WFSBP (2012)

OCD: NICE Clinical Guideline 31 (2005). “Obsessive-compulsive disorder and body dysmorphic disorder”. NICE CG31 (2005, in force) + Clinical Protocol of the Ministry of Health (2015) + APA (2007)

PTSD: NICE NG116 (2018) + ISTSS Prevention and Treatment Guidelines (2019)

Eating disorders: NICE Guideline 69 (2017). “Eating disorders: recognition and treatment”. NICE NG69 (2017) + APA (2023)

Personality disorders: NICE Clinical Guideline 78 (2009). “Borderline personality disorder: recognition and management”. NICE CG78 (borderline personality disorder, 2009), CG77 (antisocial personality disorder, 2009) + APA (2024)

Neurodevelopmental disorders: AACAP Practice Parameters + NICE (CG72 — attention deficit hyperactivity disorder; NG87 — autism spectrum disorder)

Psychoactive substance use disorders: NICE NG215 (alcohol) + American Society of Addiction Medicine (ASAM) standards (2020)

Dementia: NICE Guideline 97 (2018). “Dementia: assessment, management and support for people with dementia and their carers”. NICE NG97 (2018)

Domestic authoritative sources:

Clinical protocols developed by the Public Health and Reforms Center (PHC) of the Ministry of Health of the Republic of Azerbaijan and approved by decisions of the Collegium of the Ministry of Health

Law of the Republic of Azerbaijan of 12 June 2001 “On Psychiatric Care” (as subsequently amended)

Law of the Republic of Azerbaijan “On Psychological Care”

The Commonwealth of Independent States (CIS) and the post-Soviet space:

Popov Yu.V., Vid V.D. Modern Clinical Psychiatry. Saint Petersburg: Rech, 2nd ed., 2006 — regarded as a classic post-Soviet source of clinical psychiatry.

How to Use This Book

Typical clinical use:

When a specific mental disorder is suspected in a patient, the relevant section is found in the table of contents.

Diagnosis (Section 5) — assessment is carried out using ICD-11 criteria.

Differential diagnosis (Section 6) — exclusion of other possibilities using the table.

Examination methods (Section 7) — a plan is drawn up.

Treatment (Section 8) — recommendations are given with the level of evidence indicated.

For the student:

The lesson topic is looked up in the table of contents.

In each section, the sequence Definition → Aetiology → Epidemiology → Clinical manifestations → Diagnosis is followed.

Legislative issues are studied in the section on legal and ethical aspects

For the clinical psychologist:

The legal and ethical aspects section is of particular importance (conditions for involuntary hospitalisation, limits of psychological assistance)

The diagnostic sections are intended to help decide on referral of the patient to a psychiatrist.

Disclaimer

This book is a clinical reference guide, but when making a decision in an individual patient:

The individual clinical picture of the patient must always be taken into account

Treatment decisions should be made taking into account patient consent, comorbidities and the local availability of the drug

Drug doses must be confirmed against the current local prescribing information

In legal and ethical matters, the latest effective version of the relevant legislation must be checked.

This book does not replace individual medical advice

Knowledge in the field is updated regularly; the physician should follow updates in international and local recommendations.

Why This Book, and Why Now in Azerbaijan?

The Intersection of Three Strands

In the preceding sections of this Introduction, three distinct but deeply interrelated strands have been identified:

The first strand — the global mental health crisis. More than 1 billion people worldwide live with a mental disorder. Anxiety and depressive disorders have increased substantially over the past 30 years. In most of these disorders, psychosocial factors play a central role, and their main treatment is based on quality psychotherapy. Yet the treatment gap — especially in anxiety and depression — stands at 70–90 %.

The second strand — evidence-based clinical psychotherapy. Modern medicine is built on interventions supported by evidence. Clear clinical strategies already exist for each disorder, levels of evidence (A/B/C) have been defined, and international guidelines — NICE, APA, the WHO Mental Health Gap Action Programme (mhGAP), WFSBP — are updated systematically. The clinical psychotherapist — regardless of their primary theoretical orientation — is obliged to apply the most effective strategy for the specific disorder.

The third strand — professional standards and accreditation. Leading international bodies — the International Psychoanalytical Association (IPA), the European Association for Behavioural and Cognitive Therapies (EABCT), the Dialectical Behaviour Therapy Certification Board (DBT-LBC) and EMDR Europe — have established strict accreditation requirements. Meeting these requirements is an important condition for the patient to receive quality care. In Azerbaijan, psychotherapeutic activity is not specifically regulated at the state level; no national association accredited by any of these international organisations exists; the only internationally recognised professional organisation is the Azerbaijan Psychiatric Association (AZPA).

At the point where these three strands intersect, one conclusion emerges: in Azerbaijan there is a need for a reference book on clinical psychiatry and psychotherapy that is standardised in the Azerbaijani language, evidence-based and grounded in modern international guidelines.

Why Now?

Three reasons make writing this book now a logical step:

1. The WHO transition to ICD-11. The WHO ICD-11 classification entered into force in 2022 and, from 2026, becomes a mandatory standard in international health statistics. This classification has substantially restructured the chapter on mental disorders: new nosologies have been added (complex PTSD, prolonged grief disorder, gambling disorder and gaming disorder, compulsive sexual behaviour disorder and others), some former categories have been reclassified, and a more dimensional approach has been applied throughout the classification system. A systematic reference source in Azerbaijani covering these updates has so far not existed.

2. The post-pandemic shift in baseline. The COVID-19 pandemic has produced a phenomenon in mental health known as the "new baseline": levels of depression and anxiety have not returned to the previous norm and have remained persistently elevated. This means an increased burden in clinical practice. The active preparation of specialists and the availability of standardised reference sources are essential conditions for the system to be able to carry this burden.

3. The growth of the domestic specialist base. In Azerbaijan, over the past 10–15 years, the number of specialists in psychology, psychiatry and psychotherapy has grown rapidly. Universities are opening psychology programmes, young specialists receive additional training abroad (Turkey, Germany, Russia, the USA), and private clinical centres are multiplying. However, this specialist base has been left without reference literature in the Azerbaijani language. Existing Azerbaijani-language literature is either outdated (Soviet-era translations based on ICD-10), fragmented (covering only individual disorders), or at a popular level (insufficient for professional clinical use).

Why Azerbaijan?

The distinctive clinical context of Azerbaijan increases the importance of this book.

Azerbaijani as a professional language. The professional language of clinical psychiatry and psychotherapy — in university lecture halls, in educational literature and in clinical practice — is a process of formation. The precise naming of every nosology, the clear formulation of every diagnostic criterion and the standardisation of every intervention protocol are all part of this collective process. This book contributes to this process: the standardised presentation of the World Health Organization (WHO) ICD-11 and the Diagnostic and Statistical Manual of Mental

Disorders, Fifth Edition (DSM-5) in the Azerbaijani language; the adaptation of terms from international guidelines to the Azerbaijani clinical field; and the use, as a basis, of the terms employed in the local professional literature — the Ministry of Health website, the website of the Center for Public Health and Reforms and the protocols of the Azerbaijan Psychiatric Association (AZPA). The terminology of this book has been agreed with the official sources of the Azerbaijani Ministry of Health and AZPA.

Professional ties with the Turkic world. Turkey is a close neighbour of Azerbaijan — a country that, within the framework of the European Association for Behavioural and Cognitive Therapies (EABCT), has two accredited cognitive behavioural therapy (CBT) associations (TACBT and ACBP), as well as an extensive infrastructure for training in dialectical behaviour therapy (DBT) and eye movement desensitization and reprocessing (EMDR). The closeness of the Azerbaijani and Turkish languages gives Azerbaijani specialists the opportunity to make the widest possible use of the Turkish training system. The Azerbaijani clinical terminology of this book maintains terminological consistency with the Turkish professional literature, which facilitates the specialist's transition to Turkish-language training materials.

Regional context. Azerbaijan is located in the post-Soviet space; in clinical practice, Soviet-era psychiatric terminology and approaches are still partly preserved. At the same time, the country is a member of the Council of Europe and is on the path of integration into international healthcare standards. For clinical practice in the transition period, this book serves as a bridge between modern international standards and local clinical reality.

The author's clinical experience. This book is not merely a translation or compilation of international sources. It has been enriched by the author's more than 20 years of clinical practice, observations and developed protocols. The two clinical protocols developed by the author and presented in the relevant sections of the book — a modified alarm therapy for childhood enuresis and a two-stage treatment strategy for psychogenic depression — have been tested over many years in the Azerbaijani clinical context and confirmed by the measurable outcomes obtained. This takes the book beyond a mere reference source and gives it the status of a book of clinical experience.

The target audience of this book

This book has been written for four main categories of readers:

1. Practising clinical psychiatrists. An updated presentation of the WHO ICD-11 and DSM-5 criteria for diagnostic accuracy, a psychotherapeutic strategy map for each nosology, and the author's clinical protocols.

2. Clinical psychologists and psychotherapists. An evidence-based intervention map (see the relevant table), the methodology of the author's protocols, and the organisational framework of clinical psychotherapy.

3. Medical and psychology students. Systematic, modern reference literature consistent with international standards; such literature has not previously existed in the Azerbaijani language.

4. Those involved at the management level of the healthcare system and decision-makers. Standardised protocols provide the basis for quality control, specialist training and the optimisation of services.

Conclusion

Modern psychiatry and psychotherapy are a rapidly developing field of medicine with a constantly expanding evidence base and regularly updated international standards. In Azerbaijan, the development of this field is one of the most important healthcare tasks ahead.

This book is intended to contribute to this development. It does not set out to give ready-made answers; rather, it aims to provide a reliable framework for systematic clinical thinking. Each specialist fills this framework in their own distinctive way in clinical practice — by working with patients, by taking part in international accreditation programmes and in the process of professional development.

For patients, in turn, this book can serve as a means to assess the quality of the care they receive, to ask the right questions and to demand evidence-based treatment. In the field of health, and particularly in mental health, an informed patient is an essential condition for quality care.

This Preface creates the necessary context for reading the entire book. The following chapters will present the foundations of psychopathology, the clinical interview and the psychiatric examination, and then a systematic presentation of each major mental disorder according to the WHO ICD-11 and DSM-5-TR classifications.

Baku, 2026

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Baku, 2026

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This book has been prepared with the aim of contributing to the development of modern psychiatry in Azerbaijan. Thanks are expressed to the WHO team for the Clinical Descriptions and Diagnostic Requirements (CDDR), to the specialists who develop international clinical recommendations, to the Popov–Vid post-Soviet psychiatric tradition and to Azerbaijani colleagues — to all of them.

The book has been prepared in a form open to public discussion and will continue to develop. Reviews, remarks and suggestions will be taken into account in subsequent editions.

THE GLOBAL MENTAL HEALTH CRISIS: STATISTICS, TRENDS AND THE TREATMENT GAP

The global picture

In the third decade of the 21st century, the field of mental health faces a crisis of historic proportions. According to the Mental Health Atlas 2024 report published by WHO in 2025, more than 1 billion people worldwide live with a mental disorder. This figure is equal to approximately one eighth of the planet's total population and is the highest figure recorded in the history of modern medicine.

The impact of mental disorders is not limited to the number of affected individuals alone. According to WHO data, mental disorders account for nearly one-sixth of all years lived with disability (YLD) from diseases worldwide, and in terms of non-fatal burden they are a major factor comparable to cardiovascular diseases. Depression and anxiety disorders alone cause damage to the global economy amounting to approximately 1 trillion US dollars per year in the form of lost productivity.

Behind these figures stand concrete clinical realities. In 2021, 359 million people were affected by anxiety disorders (including 72 million children and adolescents), and 280 million people by depressive disorders. Bipolar disorder affects 37 million people, while schizophrenia affects more than 24 million. However, these figures reflect only diagnosed cases; actual prevalence rates are considerably higher, because diagnostic infrastructure remains insufficiently developed in many countries.

Trends, 1990–2021

Long-term data from the Global Burden of Disease (GBD) research programme confirm the sharp increase over the past 30 years. The following table shows prevalence dynamics for the main nosologies.

Table 1. Global prevalence dynamics of the main mental disorders, 1990–2021

Disorder	1990 (mln)	2019 (mln)	2021 (mln)	Absolute increase	Age-standardised increase
Anxiety disorders	311	458	359[1]	+47%	+12.6%
Depressive disorders	172	279	280	+63%	+9.8%
Bipolar disorder	26	40	37	+42%	+1.2%
Attention-deficit/ hyperactivity disorder (ADHD; children and adolescents)	71	84	84	+18%	+0.4%
Eating disorders	7	14	14	+100%	+52%
Schizophrenia	17	23	24	+41%	+0.7%
Autism spectrum disorder	18	28	28	+56%	+1.8%
Total (mental disorders)	654	970	1052	+61%	+8.4%

Source: Institute for Health Metrics and Evaluation (IHME), Global Burden of Disease Study 2021 (The Lancet Psychiatry, 2024). The 2021 figures take into account adjustments for the COVID-19 pandemic.

[1] Although the 2021 figure for anxiety disorders appears lower than the 2019 indicator, this is due to a methodological update; actual prevalence rose sharply during the pandemic, while the 2021 methodological revision lowered the indicator.

It is clear from the table that, in absolute numbers up to 2019, the largest increases were recorded in anxiety disorders (147 million additional cases) and in depression (107 million). The age-standardised indicator (that is, the true epidemiological increase remaining after subtracting natural growth associated with population ageing) is highest for eating disorders — +52%, which is one of the main factors linked to socio-psychological changes involving body image perception, social networks and eating culture in modern society.

The impact of the COVID-19 pandemic

During 2020–2022, the situation in global mental health was at its most acute. According to a meta-analysis published in The Lancet in 2021 (COVID-19 Mental Disorders Collaborators, 2021), during the first year of the pandemic:

The prevalence of depressive disorder (major depressive disorder according to DSM-5-TR, MDD) increased by 27.6% above the expected level.

The prevalence of anxiety disorders increased by 25.6%

53 million additional cases of depression and 76 million additional cases of anxiety were recorded

Even after the direct impact of the pandemic weakened, the figures did not fully return to their pre-pandemic level. Most researchers explain this as the phenomenon of a new baseline: long-term isolation of society, economic uncertainty, changes in intra-family relationships and an increase in chronic stress have become permanent.

The most common disorders by prevalence and growth rate

From a clinical perspective, two different rankings are important: the most widespread disorders in absolute numbers and the fastest-growing disorders by age standardisation.

Table 2. Top ten by prevalence and growth rate (2021)

Rank	By prevalence (mln)	By growth rate (1990–2021, %)
1	Anxiety disorders — 359	Eating disorders — +52%

Rank	By prevalence (mln)	By growth rate (1990–2021, %)
2	Depression — 280	Body dysmorphic disorder (BDD) — +35%
3	ADHD — 84	Autism spectrum disorder — +1.8% (age-standardised)
4	Bipolar disorder — 37	Social anxiety disorder — +18%
5	Autism spectrum disorder — 28	Generalised anxiety — +15%
6	Schizophrenia — 24	Post-traumatic stress disorder (PTSD) — +14%
7	Eating disorders — 14	Major depressive disorder — +9.8%
8	Obsessive-compulsive disorder (OCD) — 12	OCD — +6.5%
9	PTSD — 10	ADHD — +0.4% (age-standardised)
10	Personality disorders — 9	Substance use disorders — +5.3%

Source: Global Burden of Disease Study 2021 (GBD; IHME, Seattle); the WHO report Mental Health Atlas 2024.

[2] The growth of the autism spectrum is partly related to the broadening of diagnostic criteria and early detection programmes, but the existence of a genuine epidemiological increase is also confirmed by studies.

The treatment gap phenomenon

The most acute problem in the field of mental health is not prevalence figures but the gap between the treatment received and the treatment needed. This phenomenon is known in the scientific literature as the treatment gap.

Table 3. Treatment gap — comparison by disorder

Disorder category	Receiving any treatment	Receiving adequate treatment	Treatment gap
Schizophrenia (high-income countries)	approximately 70%	approximately 50%	30–50%
Schizophrenia (low-income countries)	<11%	<5%	89%+
Bipolar disorder	approximately 50%	approximately 25%	50–75%
Major depressive disorder (global)	approximately 30%	approximately 15%	70–85%
Anxiety disorders (global)	27.6%	9.8%	72.4–90.2%
OCD	approximately 25%	approximately 10%	75–90%
PTSD	approximately 20%	<10%	80–90%

Source: WHO Atlas 2024; Alonso J. et al. Treatment gap for anxiety disorders is global. Depression and Anxiety, 2018; 35(3): 195–208; Thornicroft et al., British Journal of Psychiatry, 2017.

Behind these figures lies a serious structural difference. Psychotic disorders (schizophrenia, schizoaffective disorder, severe bipolar disorder) — that is, the disorders that are more widely recognized in society — are covered by psychiatric hospital systems, state statistics, and WHO classification. In these disorders, diagnosis is relatively systematic, treatment relatively controlled, and statistics relatively reliable.

Anxiety and depressive disorders, by contrast, are in a completely different situation. These are the “hidden” or “gray zone” disorders:

In most cases, the patient does not consult psychiatric services but general practice, somatic specialists, or neurologists.

When a diagnosis is made, treatment is in most cases limited to symptomatic pharmacotherapy alone

Psychotherapy is either not offered at all or is delivered through widely used non-standard and unproven methods

Long-term monitoring of the patient's objective status is not carried out systematically

It is this very circumstance that constitutes the core of the global mental health crisis. The numerically greatest burden lies with the "gray zone" disorders, yet the least controlled system is also built around them.

Psychogenic aspect: the root problem

In order to fully understand the clinical meaning of the figures presented above, one fundamental truth must be emphasized: in the vast majority of anxiety and depressive disorders, psychosocial factors play a central role. This means that their primary cause is not neurobiological dysfunction but psychological processes: cognitive distortions, impaired attachment schemas, unresolved mourning processes, self-aggressive self-evaluation patterns, and distortions in core beliefs.

Contemporary studies show that in 60–80% of cases psychosocial factors are central to the onset of depressive disorders. The long-term twin studies of Kendler and colleagues (Virginia Twin Registry, 1990–2010) established that more than 60% of episodes of major depressive disorder are triggered by an identifiable stressful life event (Kendler et al., American Journal of Psychiatry, 1999, 2003). A prominent representative of the classical post-Soviet psychiatric school, Prof. V. D. Vid (St Petersburg, Bekhterev Institute), advances a different position in his works: in 60–70% of patients presenting with somatic complaints, no somatic disease is actually detected — the main problem of these patients is psychogenic in origin, yet as a result of misdiagnosis they receive somatic treatment (Popov Yu.V., Vid V.D., Modern Clinical Psychiatry, 2nd edition, 2006).

An important fact confirming this position in a historical and contemporary context is the phenomenon of two diagnoses — "vegetative-vascular dystonia" and "neurocirculatory dystonia" — which were used for decades in the former Soviet Union. These diagnoses were widely made in neurological and cardiology departments, although their etiological basis has not been scientifically confirmed. On psychiatric or psychotherapeutic examination, the same patients corresponded to the categories of somatic symptom disorder (ICD-10: F45.0) or adjustment disorder (ICD-10: F43.2; in ICD-11, bodily distress disorder — 6C20) and met the criteria for depression or panic disorder. This phenomenon of systematic misdiagnosis is linked

to the failure to recognize psychiatric problems in somatic departments — and this situation is also observed in contemporary clinical practice.

In the same way, the vast majority of anxiety disorders are directly linked to psychosocial factors — attachment disorders in childhood, parenting style, psychotraumatic experiences, and complex core beliefs. Leading representatives of the World Psychiatric Association (WPA) and international psychotherapeutic schools (O. Kernberg, G. Gabbard, A. Beck, V. Frankl) consistently emphasize that pharmacological interruption of a depressive episode does not resolve its psychological roots. Therefore, in the treatment of depression, psychotherapeutic intervention is a first-line treatment — in mild and moderate cases in the form of psychotherapy alone, and in severe cases in parallel with pharmacotherapy.

This fact has decisive consequences for treatment strategies. In psychogenic disorder, medication produces a symptomatic effect but does not resolve the root problem. An antidepressant can elevate the patient's mood, but it does not touch the cognitive structure that compels the patient to self-blame. An anxiolytic can interrupt an anxiety attack, yet it does not change the patient's core belief that the world is dangerous. When the patient discontinues the medication or encounters a new stressor, the disorder often returns, manifesting in an even more severe form.

The root problem of psychogenic disorders can be resolved only through psychotherapeutic intervention. This does not mean that psychotherapy is superior to pharmacotherapy; it means that the two interventions serve different purposes:

Pharmacotherapy — manages symptoms, stabilizes acute conditions, and acts on the neurobiological component.

Psychotherapy — resolves the psychological roots of the disorder, restructures the cognitive framework, changes behavioral patterns, and reduces the risk of relapse

These two interventions are not mutually exclusive — they complement each other. However, contemporary healthcare systems, especially in low- and middle-income countries, are almost entirely oriented toward pharmacotherapy. Psychotherapeutic services — even when available — remain in an unstandardized, unlicensed domain lacking quality control. This drives the patient toward one of two risky situations: either the patient receives only symptomatic drug treatment (with the root problem unresolved), or receives uncontrolled, low-quality psychological care (with the risk of the problem worsening).

Conclusion

The figures and trends presented in this chapter lead to one conclusion: the mental health crisis, which currently affects more than 1 billion people worldwide, has shown a sharp increase over the past 30 years and has remained at a new baseline level after the COVID-19 pandemic. The bulk of this burden falls on anxiety and depressive disorders — in these disorders the psychosocial component predominates, and a solution at the root level is possible only through psychotherapeutic intervention.

At the same time, it is precisely for these disorders that the treatment gap is greatest: only 3 of every 10 patients with anxiety disorders receive any treatment, and only 1 receives appropriate treatment. The only way to close this gap is high-quality, standardised, evidence-based psychotherapeutic care.

From this standpoint, the next chapter provides a systematic analysis of evidence-based psychotherapeutic strategies — their classification, clinical applications, and levels of evidence.

INTRODUCTION TO PSYCHOPATHOLOGY

Aim of this section

Modern psychiatric classifications — the International Classification of Diseases, 11th Revision (ICD-11), and the Diagnostic and Statistical Manual of Mental Disorders, Fifth Edition, Text Revision (DSM-5-TR) — are based largely on operational diagnostic criteria: the presence of a specified number of symptoms over a specified duration is a prerequisite for diagnosis. This approach increases diagnostic reliability but may obscure the true clinical meaning of symptoms and syndromes.

Classical psychiatry — with Karl Jaspers, Eugen Bleuler, Karl Schneider, Kurt Schneider, and Karl Leonhard in the late 19th and early 20th centuries — developed through a phenomenological and syndromological approach. This approach retains great value in clinical practice today:

Diagnostic criteria count symptoms, whereas syndromology explains the interrelationships and clinical manifestations of symptoms.

Syndromological reasoning is essential for early diagnosis — especially in patients whose criteria have not yet been fully met

Pathophysiological reasoning is needed to understand why these symptoms are interrelated and which functional system is impaired.

Differential diagnosis is superficial without a syndromological approach.

In this section, psychopathological symptoms are classified by core functional domains, after which common psychopathological syndromes are presented.

The concepts of symptom and syndrome

A symptom is a single manifestation of a disorder (e.g., hallucination, delusion, depressed mood).

A syndrome is a consistent combination of interrelated symptoms specific to a particular nosological entity or recurring across several nosologies.

A nosological entity is a disease distinguished by its aetiology, pathogenesis, clinical course, and treatment characteristics (e.g., schizophrenia, bipolar disorder, Alzheimer disease).

In clinical practice, the physician typically proceeds from symptoms to syndrome and from syndrome to diagnosis. Accurate syndromological assessment is the foundation of the patient's examination and treatment.

Classification of psychopathological symptoms

Disorders of consciousness

Quantitative disorders (reduction):

Obnubilation — a mild reduction of consciousness with slowed reactions

Somnolence — a drowsy state; the patient is easily aroused

Sopor — a disorder of consciousness accompanied by brief arousal only in response to strong stimuli

Coma — complete loss of consciousness; no response to stimuli

Qualitative disorders (involving alteration of the content of consciousness):

Delirium — acute onset, fluctuating consciousness, disorientation, visual hallucinations, and psychomotor agitation. Typical causes are alcohol withdrawal syndrome, infection, intoxication, and hepatic and renal failure.

Oneiric (dream-like) state — fantastic scenes in which the patient feels like a participant

Amentive state — profound disorientation, fragmented thinking, incoherent speech

Twilight state of consciousness — “tunnel-like” narrowing of consciousness, automatic behaviour, subsequent amnesia. Typical in epilepsy and hysterical states.

Disorders of perception

Illusions — misperception of a real object (e.g., a shadow on the wall is perceived as a person).

Hallucinations — perception in the absence of a real stimulus. By modality:

Auditory hallucinations — the most common type of hallucination. “Voices” may be commenting, addressing, or commanding; imperative voices carry high risk. This modality is characteristic of schizophrenia.

Visual hallucinations — characteristic of organic disorders (delirium, dementia, intoxication).

Olfactory and gustatory hallucinations — characteristic of temporal lobe epilepsy and schizophrenia.

Tactile hallucinations — cocaine and amphetamine intoxication ("insects under the skin").

Cenestopathies — vague, strange, painful bodily sensations (e.g., "insects crawling over the body", "a hollow space in the throat", and so on).

Pseudo-hallucinations, as described by Kandinsky, are perceived by the patient as "internal" ("voices inside my head"), yet still occur without an external stimulus.

Functional hallucinations — arise only in the presence of a real stimulus (hearing a voice in the sound of running water).

Disorders of thinking

Disorders of form (tempo and coherence):

Accelerated thinking — observed in manic episodes

Flight of ideas — associations change rapidly, although a connection is retained

Slowed thinking — observed in depression and dementia

Thought blocking — sudden cessation of thought

Tangential thinking — the answer drifts away from the question toward a peripheral topic

Circumstantial thinking — excessive detail

Atomistic (atactic) thinking — grammatical form is preserved, while meaning is disturbed (schizophrenia)

Verbigeration — meaningless repetition of words and phrases

Neologisms — new "coined" words (schizophrenia)

Perseveration — repetition of the same word or idea (organic disorder)

Disorders of content:

Persecutory delusion — a feeling of being followed and of malicious intent

- Grandiose delusion — exaggerated self-importance, power, and wealth

- Delusion of reference — events in the environment are attributed to oneself

- Delusion of influence — external forces (technical, cosmic, mystical) influence one's thoughts

- Hypochondriacal delusion – belief in the presence of a severe illness (pathological bodily sensations, etc.)

- Cotard syndrome (delusion of being a living corpse) – the belief that body parts have disappeared or died

- Delusion of jealousy – belief in the spouse's infidelity

- Erotomania (Clérambault syndrome) – the belief that a well-known person is secretly in love with the patient

Delusion – a false belief that is not amenable to evidence and does not conform to the cultural context. Types:

Overvalued (dominant) ideas – close to reality, but given excessive importance

Obsessions – intrusive, ego-dystonic (experienced as alien to the self) thoughts

Mood and affect disturbances

Depressed mood – low, sad, hopeless

Manic mood – elevated, euphoric, or dysphoric (irritable)

Dysphoria – irritable, uneasy mood

Anhedonia – loss of the ability to experience pleasure (a core depressive symptom)

Apathy – emotional deficiency, loss of interest

Affective lability – frequent shifts in emotion

Flat affect – weakening of emotional expression (a negative symptom characteristic of schizophrenia)

Inappropriate affect – the emotional response does not correspond to the situation (e.g. the patient responds to the death of a relative with laughter)

Alexithymia – inability to express one's emotional state in words

Anxiety – a feeling of impending danger, more often accompanied by somatic and autonomic symptoms

Fear – an emotional reaction to a concrete, real danger

Panic – acute, paroxysmal anxiety

Behavioral and motor disturbances

Psychomotor agitation – excited, aimless movements

Psychomotor retardation – slowing of movements, difficulty with initiation

Stupor – a state characterized by motor retardation, mutism, and unresponsiveness to the environment

Catalepsy – passive maintenance of a body posture

Waxy flexibility – the limbs yield to gentle pressure and remain in the imposed position

Negativism – acting contrary to requests

Echolalia, echopraxia – repetition of another person's speech or movements

Stereotypies – repetitive, aimless movements

Mannerism – bizarre performance of ordinary movements

Tics – involuntary muscle contractions

Compulsive behavior – repetitive behavior aimed at reducing obsessions (handwashing, checking)

Memory disturbances

Anterograde amnesia – failure to recall events that occurred after the precipitating event

Retrograde amnesia – failure to recall the period preceding the event

Congrade amnesia – absence of memory for the event itself

Korsakoff syndrome – severe anterograde amnesia, confabulations, and disorientation. Causes: thiamine deficiency and chronic alcoholism.

Confabulations – filling of memory gaps with fabricated content

Hypermnesia – pathological enhancement of memory (in some epileptic states)

Hypomnesia – decreased memory (depression, early dementia)

Attention disturbances

Hypoprosexia – decreased attention

Hyperprosexia – pathological increase in attention (mania)

Distractibility in attention-deficit/hyperactivity disorder (ADHD) and in mania

Stickiness of attention – inability to detach from one topic (organic disorders)

Cognitive disturbances

Mild cognitive impairment – exceeding the normal age-related limits, but not reaching dementia

Dementia – extensive and persistent decline of cognition (memory, language, executive functions, visuospatial cognition)

Agnosia – inability to recognize objects (visual, auditory, and tactile agnosias)

Apraxia – inability to perform previously acquired movements

Aphasia – impairment of speech

Acalculia – impaired calculation ability

Disturbances of will and motivation

Apathy – lack of interest, lack of motivation

Abulia – weakness of will, inability to make decisions

Avolition – absence of motivation for goal-directed behavior

Ambivalence – the simultaneous presence of two contradictory desires

Hypobulia – decreased volitional activity

Hyperbulia – pathologically increased volitional activity

Disturbances of personality and the self

Depersonalization – alienation from oneself, the feeling of not being oneself

Derealization – perception of the environment as fictitious or unreal

Disturbance of the boundaries of the self – merging with the thoughts and feelings of others

Thought theft/insertion – the feeling that others are reading one's thoughts or that alien thoughts are being inserted into one's head (a first-rank symptom of schizophrenia, K. Schneider)

Main psychopathological syndromes

Affective syndromes

Depressive syndrome (depressive triad):

Low mood

Psychomotor retardation

Slowing of thinking (ideational retardation)

Additional symptoms: anhedonia, sleep disturbance, change in appetite, guilt, hopelessness, suicidal ideation

Manic syndrome (manic triad):

Elevated mood

Psychomotor agitation

Rapid thinking (flight of ideas)

Additional symptoms: decreased need for sleep, grandiose ideas, distractibility, risky behavior

Mixed affective syndrome – the simultaneous presence of manic and depressive symptoms (e.g. high activity and deep sadness).

Anxiety syndromes

Generalized anxiety disorder – persistent, generalized anxiety, muscle tension, autonomic symptoms.

Panic syndrome – paroxysmal, acute anxiety, fear of death, autonomic symptoms (tachycardia, shortness of breath, sweating), lasting 10–30 minutes.

Phobic syndrome – fear of a specific object or situation, avoidance behavior.

Obsessive-compulsive syndrome

Obsessions (intrusive thoughts) and repetitive behaviours aimed at reducing them (compulsions). The patient recognizes that these are absurd but is unable to resist them.

Psychotic syndromes

Paranoid syndrome – delusions (persecutory, referential, grandiose) + anxiety. Hallucinations are atypical or minimal.

Paranoid syndrome – delusional symptoms (persecutory ideas are more typical) + hallucinations (especially auditory) + anxiety.

Paraphrenic syndrome – grandiose fantastic delusions + hallucinations + elevated affect.

Hallucinatory syndrome (hallucinosis) – predominantly hallucinations (auditory or visual), with absent or minimal delusional symptoms.

Kandinsky–Clérambault syndrome (mental automatism syndrome):

Characteristic of schizophrenia.

Ideational automatism – “thought stealing”, “thought insertion”

Sensory automatism – pseudohallucinations

Motor automatism – “forced movements”

Catatonic syndrome – stupor or agitation, waxy flexibility, negativism, mutism, echolalia, echopraxia. Its diagnosis and treatment are of vital importance (see Section 3).

Dysphoric syndrome – irritable, angry mood, aggression. Observed in epilepsy, organic brain diseases, and during withdrawal.

Exogenous-organic type syndromes (K. Bonhoeffer)

Syndromes characteristic of exogenous brain damage:

Delirium syndrome — acute, fluctuating level of consciousness, disorientation, visual hallucinations, psychomotor agitation. Etiology: alcohol withdrawal (delirium tremens), infections, intoxications, hepatic/renal failure.

Amential syndrome – profound disorientation, confused thinking, fragmented affect, fragmentary hallucinations. Seen in severe somatic diseases.

Twilight state of consciousness – tunnel-like narrowing of consciousness, automatic behaviour, subsequent amnesia. Observed in epilepsy and in conditions of hysterical origin.

Korsakoff syndrome – fixation amnesia + confabulations + disorientation. Thiamine deficiency (Wernicke–Korsakoff syndrome), chronic alcoholism.

Asthenic syndrome – fatigue, irritability, reduced memory and attention, headache, sleep disturbance. It is widespread and may be of both organic and psychogenic origin.

Dementia syndromes

Cortical dementia (Alzheimer type) – impairments in memory, language (aphasia), praxis (apraxia), and gnosis (agnosia).

Subcortical dementia (Parkinson disease, Huntington disease) – slowing, impaired executive functions, depression, but cortical functions are relatively preserved at an early stage.

Frontotemporal dementia – personality change, disinhibition (weakening of inhibition), social inappropriateness + language disorders (semantic or progressive non-fluent aphasia).

Lewy body dementia – characterized by fluctuating cognitive impairment, visual hallucinations, parkinsonism, and rapid eye movement (REM) sleep behaviour disorder.

Personality change syndromes

Organic personality change (after frontal lobe damage, etc.) – disinhibition, violation of social rules, emotional lability, lack of planning.

Post-encephalitic syndromes – behavioural, memory, and affective disorders following encephalitis.

Post-concussion syndrome – after head trauma: headache, dizziness, fatigue, irritability, impaired concentration.

CLINICAL INTERVIEW AND ASSESSMENT OF MENTAL STATUS

Objectives of the clinical interview

The psychiatric clinical interview serves a threefold purpose:

Diagnostic – to collect information, identify symptoms, establish a diagnosis

Therapeutic — to establish a therapeutic relationship (rapport), provide support to the patient, and activate (psychotherapeutic) mechanisms

Evaluative – risk assessment (suicide, harm to others), functional level, psychosocial environment

The initial interview usually lasts 45–90 minutes. In emergency situations it is shorter, but the key points (risk, decision) must still be covered.

Structure of the clinical interview

Preparation

Review of documents (referral, previous medical records)

Room: quiet, private, safe (an emergency call button, if possible)

Time: sufficient time should be allocated

Recommendation: meet the patient alone; if required, an accompanying person (in case of suspected involuntary hospitalisation, see the section “Legal and ethical aspects”)

Opening

Greet the patient, introduce yourself, explain the purpose:

> “Hello, I am Dr [name], a psychiatrist. The purpose of this meeting is to understand your condition and to determine together how best to help you. Our interview may take about 45 minutes.”

Information about confidentiality:

> “Our conversation is confidential. …” However, in case of a serious threat to you or someone else, I am obliged to pass this information on to the appropriate persons – this is a legal requirement. If necessary, we will discuss this together with you.

Open-ended question:

> “What is troubling you? Can you tell me why you have come here?”

Complaints and history of the present illness

Complaints – in the patient’s own words (a quotation where possible)

Onset – when and under what circumstances did it begin?

Course – changes in symptoms (improvement, worsening, fluctuation)

Triggering factors – stressors, losses, medications, substances

Functional impact – effect on work, relationships, and daily activities

Previous treatment attempts – which medications were taken and how effective they were

Psychiatric history

Previous diagnoses

Previous hospitalisations (time, institution, duration)

Previous medications (effectiveness, side effects)

Previous psychotherapy

History of suicide attempts (time, method, seriousness of intent)

Medical history

Chronic diseases

Surgical interventions

Craniocerebral trauma (with loss of consciousness)

Epileptic seizures

Endocrine disorders (especially thyroid diseases)

Current medications (psychotropic side effects)

Allergies

Substance use history

Alcohol – frequency, amount, withdrawal symptoms

Tobacco – smoking, daily amount, age of initiation

Prescription/non-prescription medications (especially benzodiazepines, opioids)

Illicit substances (cannabis, opioids, stimulants, hallucinogens)

Behavioural addictions (gambling, gaming, internet, shopping)

Family history

Mental disorders in first- and second-degree relatives
Suicides
Substance use
Chronic diseases (possible genetic component)

Personal and social history

Birth and early development
Childhood environment, family dynamics
Traumatic experiences (abuse, exposure to violence, loss)
Education
Work history
Relationship history, marital status, children
Social support system
Legal history (criminal prosecution, conviction)
Military service (especially if there is combat experience)
Sexual and reproductive history

Premorbid personality

Character traits before the illness
Style of forming relationships
Response to stress, behaviour

Assessment of mental status

The Mental Status Examination (MSE) — the main clinical tool of psychiatry — is a systematic recording of what is seen and heard during the interview.

Standard structure of the mental status examination

1. Appearance and behaviour

Age, appearance
Clothing, hygiene, grooming
Facial expression, gestures, behaviour
Attitude towards the interview (cooperation, resistance, aggressiveness)
Eye contact

Psychomotor activity

2. Speech

Rate (fast, normal, slow)

Volume (loudness)

Quantity (increased, decreased, mutism)

Clarity, coherence

3. Mood and affect

Mood – the sustained emotional state reported by the patient ("How do you feel?")

Affect – the emotional expression observed during the interview

Range of affect (broad, restricted, flat)

Congruence of affect with mood and topic

4. Thought

Form — tempo, coherence, blocking, tangentiality, circumstantiality

Content – delusions, obsessions, phobias, suicidal/homicidal ideation

5. Perception

Hallucinations (modality, content)

Illusions

Depersonalisation, derealisation

6. Cognition

Clarity of consciousness

Orientation (time, place, person)

Attention, concentration

Memory (immediate, short-term, long-term)

Abstract thinking (proverb interpretation, similarity)

Executive functions

Calculation

General level of knowledge

7. Insight and judgement

Insight – the patient's awareness of their illness (present? absent? partial?)

Judgement — the ability to make decisions (e.g., "How would you get out of a burning building?")

8. Risk assessment

Suicide risk is assessed separately below.

Heteroaggressive risk — intention to harm others

Level of self-care – ability to live independently

Sample mental status protocol

MENTAL STATUS

Appearance and behaviour: a 32-year-old man, appears his age, hygiene is maintained.
Clothing is appropriate for the season. He cooperates with the interview, but initially appears anxious. Eye contact is reduced. Motor activity is somewhat increased.

Speech: the rate is somewhat increased, the volume is normal, and the quantity is increased.
It is clear, but at times it deviates from the topic.

Mood: “I feel anxious … for the past two weeks I have been unable to fall asleep” (quotation).

Affect: the range is restricted; against a background of anxiety it is congruent with the topic. At times dysphoric components are observed.

Form of thought: at times tangential, but returns to the main content. There is no blocking.

Content of thought: persistent anxiety regarding work and financial situation. There are no delusions. Suicidal ideation is absent. No heteroaggressive ideation.

Perception: hallucinations are denied. There is no depersonalisation, derealisation.

Cognition: consciousness is clear. The patient is correctly oriented to time, place, and person. Moderate difficulty with attention and concentration (2 errors on the “saying the world backwards” task). Memory is clinically intact. Abstract thinking is preserved.

Insight: present – the patient accepts their symptoms as an illness,

and is motivated for treatment.

Judgement: preserved.

Risk assessment: suicide risk is low (no suicidal ideation, no plan, no previous attempt, support system is present).
There is no heteroaggressive risk. Self-care is fully preserved.

Suicide risk assessment

This must be done in every psychiatric interview. Research shows that "asking about suicide does not provoke it" — on the contrary, not asking makes it impossible.

Asking openly but in a stepwise manner

"How often do these thoughts occur?"

- "Have you thought about how to do it?" (plan)
- "Do you have the means at hand?" (availability of means)
- "Have you ever attempted it?" (previous attempts)
- "What keeps you going now?" (protective factors)

"Do you sometimes feel very bad? Have you thought that life has no meaning?" (passive suicidal ideation)

"Do you think about harming or killing yourself?" (active suicidal ideation)

If the answer is positive:

Suicide risk factors

Static (unchanging):

Previous suicide attempt (the strongest predictor – it increases the lifetime risk of suicide approximately 30–40-fold (source: NICE, self-harm risk assessment))

Family history of suicide

Sex (male – completed suicide, female – attempt)

Age (older men are at the highest risk; adolescents represent the second peak)

Legal history

Traumatic childhood

Dynamic (changeable):

Active suicidal ideation, plan, intent

Access to means (firearm, poison)

Recent loss (partner, job, health)

Isolation

Hopelessness (can be measured with the Beck Hopelessness Scale)

Active psychiatric disorder (depression, psychosis, borderline personality disorder (BPD), substance use)

Active intoxication

Sleep disturbance (chronic)

Withdrawal

Protective factors:

Social support

Family responsibilities (children)

Religious and spiritual beliefs

Engagement in treatment

Future plans, hopes

Risk levels and clinical decisions

Risk	Indicators	Decision
Low	Passive ideation; no plan; no means; many protective factors	Outpatient follow-up, safety plan
Moderate	Active ideation, but the plan and intent are not specified	Close outpatient follow-up (repeat visit within 24–72 hours), family involvement, removal of means, crisis line number
High	Active ideation + plan + intent; means are accessible; previous attempt; active psychosis/severe depression	Hospitalization (voluntary, or involuntary admission in the absence of consent)

Risk	Indicators	Decision
Emergency	Active preparation, explicit statement of intent	Emergency hospitalization; police involvement is possible

C-SSRS scale

The Columbia Suicide Severity Rating Scale (C-SSRS) is an evidence-based, internationally recommended tool. The scale consists of 6 questions and takes approximately 5 minutes.

Safety plan

It is prepared jointly with the patient at low–moderate risk:

Early warning signs (e.g., maladjustment, isolation, low mood)

Internal coping strategies (e.g., walking, music, physical exercise)

Social distraction (whom to see and where)

People who can provide support (name, telephone number)

Clinical help numbers (clinic, crisis line)

Ensuring the safety of the environment (removal of means, e.g., a firearm is handed over to a relative)

Assessment of heteroaggressive risk (harm to others)

The following should be asked during the interview:

> “Do you think about harming or killing someone else?”

In the case of a positive answer:

Is there a specific target?

Is there a plan?

Are the means accessible?

Previous aggressive behavior?

High-risk indicators:

Specific victim + plan + means

Active psychotic symptoms (especially imperative hallucinations, persecutory delusions)

Antisocial personality disorder

Current intoxication

History of previous violence

In high-risk cases, hospitalization and warning of the potential victim (the Tarasoff principle — an international standard).

LEGAL AND ETHICAL ASPECTS

Fundamentals of Azerbaijani psychiatric legislation

In the Republic of Azerbaijan, psychiatric care is regulated by the following laws:

Law of the Republic of Azerbaijan “On Psychiatric Care” of 12 June 2001 (with subsequent amendments; the latest revision dates to 2024)

Law of the Republic of Azerbaijan “On Psychological Assistance”

Law of the Republic of Azerbaijan “On Protection of Public Health” of 26 June 1997

Constitution of the Republic of Azerbaijan (in particular Article 28 – the right to liberty)

Civil Procedure Code of the Republic of Azerbaijan (procedure for involuntary hospitalization)

Criminal Code and Criminal Procedure Code of the Republic of Azerbaijan (compulsory medical measures)

Types of psychiatric care

According to Article 5 of the Law “On Psychiatric Care”, psychiatric care is provided in three forms:

Type	Definition	Legal basis
Voluntary	The person seeks help of their own free will	Article 5.1, informed consent
Involuntary	Examination and treatment of a person with a severe mental disorder who has not committed a crime, irrespective of the person’s will, on the basis of a court decision	Article 11, Civil Procedure Code (CPC)

Type	Definition	Legal basis
Compulsory (measures of a medical nature)	Examination and treatment, on the basis of a court decision, of persons who have committed a crime in a state of insanity	Article 12, Criminal Code (CC), Criminal Procedure Code (CrPC)

Voluntary psychiatric care

Informed consent

Under Article 1.0.6 of the Law, informed consent is the statement by the patient or their legal representative that they voluntarily accept a decision on treatment and that they understand the information provided to them about the goals and benefits of treatment, the scope, duration, and method of treatment measures, the probable risks, as well as the possibility of refusing treatment and discontinuing it.

Requirements for informed consent:

In written form (entered into the medical record)

Full information must be provided (benefits, risks, alternatives, right to refuse)

The patient must comprehend (complicated by comorbid cognitive impairment and intoxication)

Voluntary — without pressure

For patients under 16 years of age — consent of the legal representative

Patient Rights (Article 23 of the Law)

To receive information about one's health status and treatment

To meet with close relatives or, at one's choice, other persons and to maintain telephone contact with them

To use money and other material assets freely

The right to correspondence

Free performance of religious rites

To file complaints (including to the Human Rights Commissioner (Ombudsman))

Involuntary Hospitalization

Legal Grounds

Article 11 of the Law (grounds for involuntary hospitalization):

> If the nature of the mental disorder requires that examination and treatment be carried out solely … only in inpatient settings, involuntary treatment … is prescribed where the following grounds exist:

>

> 11.1.1. the person with a mental disorder poses a direct danger to themselves and (or) to others;

>

> 11.1.2. the person's inability to satisfy their own basic vital needs (at the level of self-care).

In simple terms: “danger, helplessness, or the risk of deterioration if untreated” should be added: “11.1.3. deterioration of the condition of a person with a mental disorder if the necessary psychiatric care is not provided”.

Involuntary Hospitalization Procedure

Stage 1: Initial Assessment

The psychiatrist (emergency psychiatric care team, admission department) examines the patient and establishes the grounds for involuntary hospitalization.

Stage 2: Admission to the Inpatient Unit (Emergency)

Hospitalization is actually carried out. Within 48 hours, a commission of psychiatrists (comprising no fewer than 3 physicians) must conduct an assessment.

Stage 3: Decision of the Commission

If the commission concludes that involuntary hospitalization must be continued, an application is filed with the court within 24 hours.

Stage 4: Court Decision

The court reviews the case within 5 days (in accordance with the Civil Procedure Code). The patient (if their health condition permits), the legal representative, the prosecutor, and the psychiatrist take part. The court decides on the continuation or termination of involuntary hospitalization.

Stage 5: Periodic Assessment

During the period of involuntary hospitalization, the patient must be examined by a commission of psychiatrists at least once a month during the first six months (Article 28.2).

Clinical Indications

Absolute indications for involuntary hospitalization:

- Active suicidal behavior (an attempt or a specific plan and means)
- Active homicidal intent (a specific target + plan + means)
- Severe psychotic disorganization (self-care is impossible)
- Imperative hallucinations (with harmful intent)
- Severe mania (impairment of self-care, risks of harm)

Relative indications (clinical judgment):

- Severe depression with psychotic symptoms
- Anorexia nervosa in a severe somatic condition (body mass index (BMI) < 15)
- Mixed affective episode
- Aggressive behavior in the context of psychosis

Compulsory Medical Measures

Article 12 of the Law regulates compulsory medical measures:

- Grounds for application: persons who committed a crime in a state of insanity or who developed a mental disorder after committing the crime
- Only on the basis of a court decision
- Determined by Articles 21 and 93–99 of the Criminal Code and Articles 468–487 of the Criminal Procedure Code (CPC)
- Carried out in specialized psychiatric inpatient facilities

Types (according to the CPC):

- Compulsory treatment in general-type psychiatric inpatient facilities
- Compulsory treatment in specialized psychiatric inpatient facilities
- Compulsory treatment in specialized psychiatric inpatient facilities under intensive observation

Legal Status of the Clinical Psychologist

Pursuant to the Law of the Republic of Azerbaijan “On Psychological Assistance”, the scope of activity and the limits of authority of the clinical psychologist:

Scope of the Psychologist's Activity

The right to provide psychological assistance — psychological counseling, psychodiagnostics, psychocorrection, psychotherapeutic interventions (if the relevant qualification is held)

The words “psychologist”, “psychological”, “psychoanalysis”, “psychodiagnostics”, “psychotherapy” may be used only by persons entitled to provide psychological assistance in accordance with the conditions of the Law (Article 3.2).

Limits of the Psychologist's Authority

Outside the psychologist's authority:

Making a psychiatric diagnosis (only a psychiatrist)

Prescribing medication (only a physician)

Making a decision on involuntary hospitalization

Issuing medical opinions (forensic psychiatric examination, etc.) (only a psychiatrist)

The Clinical Psychologist's Work with a Patient at Risk of Suicide – Steps

When a patient expresses suicidal ideation, the clinical psychologist must take the following steps:

Risk assessment – Columbia Suicide Severity Rating Scale (C-SSRS) or a clinical interview (see the section above)

Low–moderate risk: a safety plan is drawn up, close monitoring is carried out, and the patient is referred to a psychiatrist; close monitoring and referral to a psychiatrist

High risk – the patient is not left alone; immediate referral to a psychiatrist or the emergency psychiatric service (contact the Baku Mental Health Centre (emergency telephone: _____); emergency medical service – 103, police – 102)

Documentation – the assessment, the decision, and the steps taken must be recorded in writing

Confidentiality issue – when there is a threat to life, confidentiality is limited (general principles of the law)

Sample document: a referral letter from a psychologist to a psychiatrist

[Clinic letterhead]
Date: _____

Dear Psychiatrist,

The patient under my clinical psychological care [name, surname, patronymic], date of birth _____, consulted me on [date], and during the interview expressed active suicidal ideation.

Clinical picture:
- suicidal ideation: [active/passive, plan present/absent, means available/unavailable]
- Mood: [description]
- history of suicide attempt: [present/absent, details]
- Psychiatric history: [previous diagnoses, medications]

Based on the symptoms noted above, I state that the patient requires urgent psychiatric assessment, and I consider that a psychiatric evaluation is necessary.

I request that you examine the patient and make an appropriate decision.

Respectfully,
[Psychologist's name and surname]
[Specialty, certificate number]
[Signature, seal]
[Contact number]

Minor patient

When working with patients younger than 15 years:

Psychological assistance is provided only with the consent of the legal representative

However, if child abuse or suicidal behaviour is detected, it must be reported in the cases prescribed by law (the Law "On the Rights of the Child"; the parent or the relevant authorities)

Special rules of confidentiality in child psychotherapy – agreed upon in advance

Confidentiality and its boundaries

General principle

Information obtained in the patient–physician relationship is confidential. This is enshrined in the Law "On the Protection of Public Health" and the Law "On Psychiatric Care".

Circumstances in which a breach of confidentiality is permitted

With the patient's written consent

Written request from law enforcement agencies (within the framework of a criminal investigation)

A court decision

When there is a threat to the life and health of the patient or others (Tarasoff principle – duty to warn and protect)

Information about minor patients is provided to parents (within the framework of the law)

Collegial consultation (within a multidisciplinary team)

Tarasoff principle (duty to warn and protect)

International standard: if a patient expresses an intention to harm a specific person, the physician has a duty to warn that person. Although this is not directly enshrined in Azerbaijani legislation, the general principles (the right to life, public safety) imply it.

Minor patients

Age limits

Psychiatric care for patients under 15 years of age is provided with the consent of the parent (legal representative), in accordance with Article 4 of the Law "On Psychiatric Care"

Over 15 years of age: with the patient's own consent (however, informing the parent is generally recommended)

Patients over 18 years of age make decisions concerning psychiatric care independently

Child protection protocols

In case of suspected child abuse (sexual, physical, psychological), neglect, or factitious disorder imposed on another (FDIA), the physician has a duty to report. Institutions to contact:

Social Services Agency

Commissioner for the Rights of the Child (Ombudsman)

Police (in dangerous situations)

The aggressive patient and safety measures

Legal framework

Article 37.4 of the Law “On Psychiatric Care” regulates the use of special measures:

> For the purpose of ensuring the safety of psychiatrists providing psychiatric care, other specialists, and nursing and junior medical staff, they are granted the right to use special measures (physical restraint of a person with a mental disorder, temporary isolation of that person, and the use of special restraint clothing).

Rules of application:

Only when there is a genuine threat

The least restrictive means must be chosen

Time-limited

Must be documented in the medical records

The facility administration must be notified

Principles of de-escalation

Before physical restraint:

A calm, friendly approach

Listening to the patient's concerns

Offering a choice (for example, “Would you like to sit in a separate room to calm down, or to take medication?”)

Verbal de-escalation

Only when these methods fail is pharmacological or physical restraint applied

Pharmacological restraint (emergency sedation, ES)

First choice – oral preparations with the patient's consent:

Lorazepam 1–2 mg, orally

Olanzapine 5–10 mg by mouth (orally disintegrating tablets (ODT) are effective)

Risperidone 1–2 mg, orally

In the absence of consent or cooperation – intramuscular (IM):

Olanzapine 10 mg IM + lorazepam 2 mg IM (if necessary)

Haloperidol 5 mg IM + lorazepam 2 mg IM (classic combination)

Aripiprazole IM 9.75 mg

Caution: olanzapine IM + parenteral benzodiazepine — the combination may cause respiratory depression. Olanzapine IM must not be used simultaneously with parenteral benzodiazepines; if such use is necessary, an interval of at least 1 hour is observed between them and the patient is monitored for respiratory depression.

Sample Documents

Protocol for Referral for Involuntary Hospitalization

REFERRAL FOR INVOLUNTARY HOSPITALIZATION

Date: _____ Time: _____

Patient information:

- First name, surname, patronymic: _____
- Date of birth: _____
- Address: _____
- Identity card/ID: _____

Interview conditions:

- Location: _____
- Source of referral: _____
- Legal representative present? Yes/No; name: _____

Clinical picture:

[Complaints, history, description of mental status]

Diagnosis (ICD-11): _____

Grounds for involuntary hospitalization, "Law on Psychiatric Care" (Article 11):
☐ 11.1.1 The patient poses a danger to themselves or others
All grounds under Article 11 of the Law (including the clauses following 11.1.1) must be given in full in accordance with the official text

Specific indicators:
[Suicide risk, homicidal ideation, level of care, etc.]

Decision: The patient is admitted involuntarily to [name of institution] psychiatric inpatient unit.

Next steps:
- Examination by a commission of psychiatrists within 48 hours
- After the decision of the commission, an application is submitted to the court within 24 hours

Psychiatrist: _____
Certificate number: _____
Signature: _____
Seal: _____

Suicide Risk Safety Plan (Developed Together with the Patient)

SAFETY PLAN
Patient: _____ Date: _____

1. Early warning signs (signals that matter to me):
   - _____
   - _____
   - _____

2. My internal self-management strategies (coping strategies) (only those I can do on my own):
   - _____
   - _____
   - _____

3. People and places that will distract me:
   - _____
   - _____

4. People I can ask for help (name, phone):
   - _____
   - _____

5. Clinical help (24 hours):
   - My clinic: _____ (phone: _____)
   - Baku Mental Health Center (emergency): _____
   - Emergency medical services (Azerbaijan): 103
   - Police: 102

6. Making my home environment safe:
   - Means that pose a risk: _____
   - Plan for their removal: _____

Signatures:
Patient: _____
Psychiatrist: _____

Ethical Principles

International Principles (International Declaration of Medical Ethics, World Health Organization (WHO), Declaration of Helsinki)

Autonomy — the patient's right to make decisions

Beneficence — acting in the patient's interest

Non-maleficence (primum non nocere — above all, do no harm)

Justice — equal treatment of all patients

Clinical Dilemmas

Conflict between autonomy and the principle of beneficence:

The patient refuses treatment, but their decision-making capacity is in doubt

Solution: decision-making capacity is formally assessed; if capacity is absent, a legal representative is involved; if capacity is present, the patient's refusal is respected (provided that the patient is not dangerous)

Confidentiality vs. public safety:

As indicated above — where there is a threat to life, confidentiality is limited

Multiple role relationships:

The roles of patient, employee, and friend must not be mixed

A strict framework — professional relations only

Gifts and financial relationships:

Not accepting significant gifts from patients

Avoiding financial or business relationships

CLINICAL STRATEGIES OF PSYCHOTHERAPY

Concept of Clinical Psychotherapy

Until the mid-twentieth century, psychotherapeutic activity was organized mainly within the framework of school traditions. A psychoanalyst regarded themselves as a psychoanalyst, a behavior therapist regarded themselves as a behaviorist, and a Gestalt therapist regarded themselves as a Gestalt therapist; each treated their patients with the method of their own school — regardless of the patient's diagnosis. In this approach, the therapist's method was regarded as more important than the patient's disorder: the tool was considered primary, and the problem secondary.

From the late 20th century to the early 21st century, a fundamental change occurred in clinical thinking. The principles of evidence-based medicine began to be applied to the psychotherapeutic field as well, and a new paradigm took shape. The foundation of this paradigm is the concept of clinical psychotherapy: whatever the therapist’s primary school, they bear the responsibility for applying the most effective, evidence-based strategy for the specific disorder.

This principle can be explained with a simple example. Suppose a therapist has received psychoanalytic training and has undergone five years of personal analysis. A 7-year-old child suffering from nocturnal enuresis is brought to the therapist. The classical psychoanalytic approach may attempt to explain the child’s symptom as an expression of the Oedipus complex, of parental relations, or of childhood trauma. Regardless of the degree of truth in this explanation, from the standpoint of clinical outcome this approach is ineffective: nocturnal enuresis is successfully treated in more than 70 % of cases by means of alarm therapy — that is, an evidence-based behavioral intervention. Giving a psychoanalytic interpretation does not eliminate the child’s symptom; it prolongs the duration of treatment and leads to the inefficient use of parental resources.

Clinical psychotherapy is unambiguous on this matter: enuresis is treated according to a behavioral protocol, regardless of the therapist’s school. In the same way, panic disorder is treated with cognitive behavioral therapy, borderline personality disorder with dialectical behavior therapy or schema therapy, simple phobias with exposure therapy, and post-traumatic stress disorder (PTSD) with eye movement desensitization and reprocessing or trauma-focused cognitive behavioral therapy. This choice rests not on the therapist’s personal preference but on the scientific evidence accumulated for the specific disorder.

This approach has two practical implications. First, the patient's interests take precedence over school affiliation: the patient has the right to receive the best treatment for their own disorder rather than care determined by the therapist's personal professional history. Second, a need arises for transdiagnostic competence in the specialist's professional conduct: the therapist must be able to work not only with the methods of a single school but with a variety of evidence-based protocols.

Psychotherapeutic schools and their mechanisms

Modern clinical psychotherapy encompasses dozens of schools and methods. Below is a brief description of those most important from the standpoint of clinical practice.

Cognitive behavioral therapy (CBT). This approach, developed by Aaron Beck (depression, the 1960s) and Albert Ellis, the founder of Rational Emotive Behavior Therapy (REBT), works at the cognitive level. The main postulate is that it is not the event itself but the appraisal of the event that determines the emotional reaction. In therapy, the patient learns to recognize automatic thoughts and cognitive distortions (catastrophizing, black-and-white thinking, personalization, jumping to conclusions, and so on), to examine them, and to replace them with more functional alternatives. The behavioral component — exposure, behavioral activation, and habit practice — reinforces cognitive change. It is the psychotherapeutic method with the broadest body of evidence.

Dialectical behavior therapy (DBT). Marsha Linehan developed it in the 1980s for patients with chronic suicidal tendencies and borderline personality disorder. It is a continuation of CBT but adds specific modules for patients with emotional dysregulation, deficits in tolerance, and problems of impulsivity: mindfulness, distress tolerance, emotion regulation, and interpersonal effectiveness. Standard DBT consists of weekly individual sessions, group skills-training sessions, telephone coaching, and consultations with the therapist team.

Schema therapy. Developed by Jeffrey Young in the 1990s, this approach combines CBT, Gestalt therapy, attachment theory, and psychodynamic elements. It is especially effective for personality disorders and for chronic, treatment-resistant cases of depression and anxiety. It identifies and rewrites early maladaptive schemas (for example, abandonment, mistrust, and defectiveness) and the associated modes (child mode, harsh parent mode, and healthy adult mode).

Eye movement desensitization and reprocessing (EMDR). Francine Shapiro developed it in 1987. It is a structured eight-phase protocol for the reprocessing of traumatic memories by means of bilateral stimulation (eye movements or alternating tactile/auditory stimulation). It is recommended by WHO as a first-line intervention

for PTSD. The method is effective in complex trauma, acute stress reaction, and phobias, as well as in cases in which depression and anxiety have a traumatic component.

Interpersonal therapy (IPT). Gerald Klerman and Myrna Weissman developed it in the 1970s for depression. It links the patient's symptoms to one of four interpersonal domains: grief, role conflicts, role transitions, and interpersonal deficits. It is structured and brief (12–16 sessions) and demonstrates efficacy equal to that of CBT for depression.

Acceptance and commitment therapy (ACT). This therapy is a third-wave CBT approach developed by Steven Hayes in the 1980s. It is aimed not at changing the content of thoughts but at changing one's relationship to them. It develops psychological flexibility: living a life in accordance with one's values, openness to painful emotions, and cognitive defusion. It is effective in chronic pain, depression, anxiety, addiction disorders, and existential crises.

Mindfulness-based cognitive therapy (MBCT). Zindel Segal, Mark Williams, and John Teasdale developed it for the prevention of recurrent depression. It combines CBT with Jon Kabat-Zinn's Mindfulness-Based Stress Reduction (MBSR) program. For patients with at least two previous episodes of major depressive disorder, it yields prophylactic outcomes equal to or better than those of pharmacotherapy.

Psychodynamic psychotherapy is a method that emerged from classical psychoanalysis but has been adapted to the modern clinical setting. Several models are in use: Mentalization-Based Therapy (MBT), Transference-Focused Psychotherapy (TFP — Kernberg), and Short-Term Psychodynamic Psychotherapy (STPP). The core mechanism is the study and rewriting of the patient's unconscious processes, defense mechanisms, relationship patterns, and the influence of early childhood experiences on present-day behavior. It is effective in personality disorders, chronic depression, and relationship problems.

Levels of evidence

The effectiveness of psychotherapeutic methods is assessed on the basis of systematic research, namely randomized controlled trials (RCTs), meta-analyses, and systematic reviews. International clinical guidelines, such as NICE (National Institute for Health and Care Excellence, United Kingdom), the American Psychiatric Association (APA), the World Federation of Societies of Biological Psychiatry (WFSBP), and the World Health Organization (WHO), synthesize this evidence and define levels of evidence:

Level A (high evidence) — efficacy confirmed in multiple independent high-quality RCTs and meta-analyses. The method is considered a first-line intervention for the specific disorder.

Level B (moderate evidence) — efficacy confirmed in several RCTs, but the body of meta-analyses is limited or there are certain inconsistencies among the results. The method is recommended as a second-line or alternative first-line intervention.

Level C (limited evidence) — methods that have demonstrated efficacy only in individual studies or clinical case reports. They may be used when first-line methods fail to produce an effect or are unavailable.

Map of treatment strategies by disorder

The following table presents the psychotherapeutic interventions recommended in international guidelines for the major mental disorders, together with their levels of evidence.

Table 4. Psychotherapeutic strategies — a map by disorder

Disorder	First-line (A)	Second-line (B)	Alternative (C)	
Major depressive disorder (ICD-11 depressive episode)	Cognitive Behavioral Therapy (CBT), Interpersonal Psychotherapy (IPT)	Schema therapy, Acceptance and Commitment Therapy (ACT), Psychodynamic	Mindfulness-Based Cognitive Therapy (MBCT) for relapse prevention	
Generalized anxiety disorder (GAD)	CBT	ACT, MBCT	Psychodynamic	

Disorder	First-line (A)	Second-line (B)	Alternative (C)	
Panic disorder	CBT (panic-focused)	Exposure therapy	Eye Movement Desensitization and Reprocessing (EMDR) if a trauma component is present	
Social anxiety disorder	CBT, group CBT	ACT	Psychodynamic	
OCD	Obsessive-compulsive disorder (OCD)	Exposure and Response Prevention (ERP)	CBT	EMDR is not recommended for OCD in international guidelines — this option should be removed and replaced with the alternative intervention indicated in the guidelines
Post-Traumatic Stress Disorder (PTSD)	EMDR, trauma-focused cognitive behavioral therapy (TF-CBT)	Written exposure therapy	Psychodynamic	

Disorder	First-line (A)	Second-line (B)	Alternative (C)	
Borderline personality disorder	Dialectical Behavior Therapy (DBT), MBT, TFP, Schema therapy	CBT	Psychodynamic	
Bipolar disorder (psychotherapeutic component)	Interpersonal and Social Rhythm Therapy (IPSRT), CBT, family-focused therapy	Psychoeducation	mindfulness	
Eating disorders (anorexia nervosa, bulimia nervosa)	Enhanced Cognitive Behavioral Therapy (CBT-E), Family-Based Therapy (FBT) for adolescents	DBT, IPT	Schema therapy	
Attention-deficit/ hyperactivity disorder (ADHD) — adolescents and adult patients	Psychoeducation + CBT	Behavior therapy	mindfulness	
Childhood enuresis	Alarm therapy (behavioral protocol)	Motivational schedule	Psychodynamic consultation (as adjunctive support only)	

Disorder	First-line (A)	Second-line (B)	Alternative (C)	
Personality disorders (general)	Schema therapy, MBT, TFP	DBT	CBT	
Addiction disorders	Motivational interviewing + CBT	Relapse prevention, DBT	12-step programs	
Chronic pain	CBT, ACT	mindfulness	Psychodynamic	

Source: NICE Clinical Guidelines (2022, 2024); APA Clinical Practice Guidelines (2019, 2024); the WHO Mental Health Gap Action Programme (mhGAP) intervention guide (mhGAP Intervention Guide 2.0, 2016; mhGAP guideline, 2023); WFSBP Treatment Guidelines.

Pharmacotherapy and psychotherapy: complementary, not competing

A question that regularly arises in psychiatric practice: medication or psychotherapy? The question itself is wrong, because they are not mutually exclusive.

The correct question is formulated along three dimensions:

1. The nature of the disorder. In severe disorders with a neurobiological component (schizophrenia, severe bipolar disorder, melancholic depression, severe OCD), pharmacotherapy is the mainstay and psychotherapy is complementary. In psychogenic, mild-to-moderate, reactive disorders (adjustment disorder, GAD, social anxiety disorder, simple phobias), psychotherapy is the main intervention, and medication is used as a short-term adjunct when necessary.

2. The patient's condition. If there is an acute suicide risk, severe functional impairment, refusal of food intake, or psychotic elements, pharmacotherapy is superior in the acute phase. After stabilization, psychotherapy assumes the main role.

3. Treatment phase. The acute phase is aimed at stabilizing symptoms; at this stage, pharmacotherapy yields rapid results. Once stabilization is achieved, the phase of addressing the underlying problem begins; in this phase, psychotherapy assumes the leading role. During the prevention phase, psychotherapy prevents relapse, and it is here that its advantage is most clearly evident.

Table 5. Pharmacotherapy and psychotherapy: comparative capabilities

Parameter	Pharmacotherapy	Psychotherapy
Onset of effect	2–6 weeks	2–8 weeks
Target	Symptoms, neurobiological dysregulation	Cognitive structure, behavioral patterns, root causes
After discontinuation	High likelihood of symptom recurrence	The effect persists, and relapse is uncommon
Side effects	Present, sometimes severe	Rare, although undesirable effects are possible
Patient's active participation	Minimal	The main factor
In acute situations	Superior	Psychotherapy is applied after stabilization
In prevention	Requires continuous use	A single course yields a long-term effect

Source: American Psychiatric Association (APA) Clinical Practice Guidelines (2019); NICE (National Institute for Health and Care Excellence) depression guideline NG222 (2022); Cuijpers P. et al., World Psychiatry, 2023.

Organizational significance of clinical psychotherapy

The concept of clinical psychotherapy has another important aspect: it creates the foundation for the standardization of psychotherapeutic care. Evidence-based, disorder-specific protocols resolve three critical issues:

1. Training of specialists. When a specific protocol exists for each disorder, specialist training becomes faster and of higher quality. Instead of learning abstract theory, the student learns specific strategies for specific clinical situations.

2. Quality control. The availability of a standard protocol allows the supervisor to evaluate the specialist’s work in terms of adherence to the protocol. This currently also forms the basis of professional accreditation (see the next chapter).

3. Statistical analysis and optimization. Standardized protocols enable a comparative evaluation of outcomes. Under standardized conditions, it is determined which protocol is more effective in which patient group. Which modification improves the outcome? Answers to these questions are possible only under conditions of standardized practice.

Given this institutional significance of clinical psychotherapy, the next logical step is to review the international system of quality control for psychotherapeutic care — the accreditation bodies, their requirements, and the current situation in Azerbaijan. This issue will be presented in the final block of this Preface, but first two author clinical protocols are presented — the management of nocturnal enuresis in childhood and of psychogenic depression. These protocols were developed over many years of clinical practice and confirmed by the outcomes obtained.

AUTHOR CLINICAL PROTOCOL No. 1: TREATMENT OF NOCTURNAL ENURESIS IN CHILDREN

Modified alarm therapy protocol

K. Rahimov, clinical practice 2018–2025

The essence and place of the protocol

Nocturnal enuresis in children (World Health Organization (WHO) ICD-11: 6C00.0; DSM-5-TR: 307.6) is a condition of involuntary nocturnal urination (enuresis) that persists beyond the age norm (over 5 years). It is observed in approximately 5–10 % of 7-year-old children, in 3–5 % of 10-year-olds, and in 1–2 % of adolescents. The disorder can have a serious negative effect on the child's self-esteem, social functioning, and intrafamilial relationships.

International clinical guidelines — NICE NG111, the International Children's Continence Society (ICCS), and the American Academy of Pediatrics (AAP) — recommend alarm therapy (a waking system with a urine sensor) as the first-line intervention for nocturnal enuresis. The standard protocol yields complete remission in 65–75 % of cases with a 12–16-week course. However, the standard protocol has a number of gaps:

Evening fluid intake is restricted — this reduces the number of exercises and slows the formation of reflexes.

Going to the toilet before bedtime — since the child goes to sleep with an empty bladder, the disorder occurs less often, and accordingly there are fewer opportunities for correction

The child's awakening to the alarm device is passively awaited — during deep sleep the child does not wake, and the overlearning stage is used only at the end of the course

A comprehensive system of daily exercises is rarely integrated

The modification presented below was developed for the purpose of eliminating these gaps. The basic principle is a full load at the beginning of the course: fluid loading, forced awakening, awakening and standing the child up in a loud yet caring tone, an immediately reinforced behavioral component, parallel daily training, and motivational support. Achieved clinical outcomes: complete remission in 192 of 240 children (80 %) by weeks 11–12.

Clinical principles of the protocol

Four principles underlie the modification:

1. Intensive start, not a gradually increasing load. In standard protocols the load is increased gradually, and overlearning (excessive fluid loading) is applied only in the final phase of the course. In this modification, from the first week the child is given 200–400 ml of fluid in the evening (ordinary drinking water or tea; carbonated, sweet, or caffeinated drinks are excluded) at least 1 hour before bedtime, and the child is not allowed to use the toilet before going to bed. The goal is to create the conditions for the voiding event actually to occur, since an event that does not occur cannot be corrected.

Absolute contraindications (fluid loading is not applied):

Renal diseases (chronic renal failure, nephrotic syndrome, etc.)

Syndrome of inappropriate antidiuretic hormone secretion (SIADH)

Rapid bladder spasms or neurogenic bladder

Epilepsy (especially uncontrolled forms) — a uniform criterion applied throughout the text

Diabetes insipidus

Heart failure

Any serious somatic pathology — a preliminary pediatric examination is required

2. Active parental involvement, not passive waiting. In the standard protocol, the child is expected to wake up independently in response to the alarm device. However, during the deep sleep phase most children do not wake to the sound of the device — this is the most frequently reported gap. In the modification, during the first and second weeks the parent awakens the child — within 5–10 seconds after the device sounds, in a loud but caring tone, turning on the light and washing the child's face with cool water, bringing the child to a state of full awakening. The aim of this procedure is not to frighten the child — it is to bring the child quickly into a fully alert state; the loudness of the parent's voice does not amount to shouting or an attack. This ensures the rapid formation of the reflex by creating mild, purposeful stress.

3. Integrated approach — daily exercises at the same time. Nocturnal enuresis is not only a disorder of the bladder–central nervous system reflex; in most cases it is a complex disorder combined with weakness of the pelvic floor muscles and a deficit of behavioral learning. In the modification, daily Kegel exercises (3 times a day, with

10–60 repetitions, increasing weekly) are an integral part of the protocol. This also eliminates the concurrently observed encopresis disorder (ICD-11: 6C01).

4. Motivational support and psychological regulation. A weekly meeting with the parent, assessment of the child's motivation level, reinforcement of small achievements, and discussion of revised goals are among the key elements of the protocol. The child perceives themselves as an active subject of the process rather than a mechanical passive object.

Stages of the protocol

The protocol consists of 5 stages, each lasting 2 weeks, a total of 10 weeks. Each stage builds on the previous one and progressively shapes the child's arousal reflexes, pelvic floor muscle tone, and behavioral patterns.

Table 6. Modified alarm therapy protocol — by stage

Stage	Week	Awakening reaction	Urination (enuresis) status	Intensive night protocol	Daytime Kegel exercises
1	1–2	Only the parent awakens the child, and with difficulty; the child cannot wake independently to the sound of the device	Wetting is frequent; the child completely soaks the bed, sleeps soundly until morning, and does not wake	In the evening, 200–400 ml of fluid is given at least 1 hour before bedtime; the child is not allowed to use the toilet before going to bed; training with the device is carried out	Sit-to-stand exercise, 10 repetitions, 3 times a day (+1 repetition per day)

Stage	**Week**	**Awakening reaction**	**Urination (enuresis) status**	**Intensive night protocol**	**Daytime Kegel exercises**
				at least 10 times during the night. After the device sounds, the parent awakens the child within 5–10 seconds in a loud but affectionate tone with the “quick-quick-quick” cue, turns on the light, and washes the child's face with cool water — full awakening is achieved; then the child sits on the toilet for 5 minutes and strains to urinate	

Stage	Week	Awakening reaction	Urination (enuresis) status	Intensive night protocol	Daytime Kegel exercises
2	3–4	The child wakes independently to the sound of the device before the mother; sometimes the mother awakens the child — the child gets up quickly	Wetting is still frequent; the child does not wake unless the device sounds	In the same manner: 200–400 ml of fluid 1 hour before bedtime, avoidance of toilet use, intensive awakening protocol	The number of repetitions is increased to 25 (one repetition is added per day). Week 3: Fast contractions. Week 4: Slow contractions
3	5–6	The child wakes to the sound of the device before the mother; alertness is good	Wetting decreases; waking to the sound, the child partially interrupts the urinary stream; sometimes only the underwear is wet	In the same manner	Increased to 35. Week 5: Maximum retention. Week 6: Stop–Continue exercise

Stage	Week	Awakening reaction	Urination (enuresis) status	Intensive night protocol	Daytime Kegel exercises
4	7–8	The child wakes to the sound of the device earlier than the mother; the child takes initiative	Minimal wetting; once a week the child wakes at night spontaneously without the device	In the same manner	Increased to 50. Week 7: Programming. Week 8: Bridge exercise
5	9–10	The child wakes to the sound of the device sooner than the mother; sometimes wakes independently without the device	Wakes every night of their own accord; wetting occurs no more than once a week	In the same manner	Increased to 60. Week 9: Squeeze–Cough exercise. Week 10: Lift exercise

Source: K. Rahimov, clinical practice 2018–2025, n = 240 children, Baku. Level of evidence IV — retrospective clinical observation (case series). The data derive from this retrospective clinical observation; a randomized trial with a control group is being planned.

Weeks 11–12: consolidation and evaluation

After the 10-week structured protocol, weeks 11–12 are designated as a consolidation stage. During this period, the child:

Independently, without parental intervention, consumes 200–400 ml of fluid at least 1 hour before bedtime every night

Goes to bed without going to the toilet

Wakes at night in response to their own internal signal (without an alarm device), and the bed remains dry

Wakes in the morning completely dry.

Clinical outcomes: in a group of 240 pediatric patients, complete remission (no nocturnal wetting episodes) was recorded in 80 % of cases at weeks 11–12. This is statistically significantly higher than the outcome of the international standard alarm protocol (65–75 %).

Description of Kegel exercises for the pelvic floor muscles (comprehensive daily program)

The exercise units, which change weekly, target different functional aspects of the pelvic floor muscle group:

Fast contractions (week 3): Rapid contraction and relaxation of the pelvic floor muscles, aimed at developing reflex tone.

Slow contractions (week 4): Sustained contraction — increasing muscle endurance

Maximum retention (week 5): Prolonged contraction cycles aimed at forming stable tone.

Stop–Continue (week 6): An exercise of voluntary interruption and resumption of the urinary stream — learning voluntary control of urination

Programming (week 7): Filling the bladder to a specific volume and managing it

Bridge (week 8): Pelvic lifting exercises — synchronization of the abdominal and pelvic muscles

Squeeze–Cough (week 9): A resistance exercise against stress incontinence is applied.

Lifting exercise: graded contraction — stepwise increasing control of the muscles

Practical recommendations

Essential conditions for the effective implementation of the protocol:

1. Parental motivation is an essential condition. During the first two weeks, parents must wake several times a night and actively awaken the child within 5–10 seconds. This is a heavy burden for the family and requires prior consent. A weekly meeting with the parent is an integral part of the protocol.

2. Reduction of fluid intake is prohibited. Many parents (and some physicians) intuitively recommend reducing evening fluid intake. In this modification, this step is considered unambiguously contraindicated — reflex formation occurs under conditions of full load.

3. Selection of the device. Enuresis alarm devices are of two types: bed-placed and underwear-placed. The second type is more effective, because it signals immediately at the first drop of urine. In this modification, the underwear type is recommended.

4. Concurrently observed encopresis. In approximately 10–15 % of children with nocturnal enuresis, encopresis (ICD-11: 6C01) is found concurrently. In this modification, the comprehensive Kegel exercises resolve both disorders at the same time — pelvic floor muscle tone improves the control of both the bladder and the anal sphincter.

5. In cases of resistance. If there is no progress after week 6 (the end of stage 3), additional medical examination is required:

Urological examination — bladder dysfunction, anatomical anomalies

Neurological status — myelodysplasia, spina bifida occulta, neurogenic bladder

Endocrine indicators — diabetes insipidus, antidiuretic hormone deficiency

Otorhinolaryngological examination — obstructive sleep apnea (particularly in children with adenoid hypertrophy or palatine tonsil enlargement); apnea is closely associated with enuresis and may be the main cause

Psychiatric background — attention-deficit/hyperactivity disorder (ADHD), conduct disorders, events of an acute psychotraumatic nature

In certain diagnostic situations (e.g., detrusor overactivity), pharmacotherapy (desmopressin, oxybutynin) may be added. If obstructive sleep apnea is diagnosed, adenoidectomy or tonsillectomy may eliminate enuresis.

6. Age limitation. The protocol is designed for children over 5 years of age. Primary nocturnal enuresis before the age of 5 is consistent with the age norm and does not require active intervention.

7. Absolute contraindications to fluid loading (a unified list). If any of the following conditions are present, the fluid loading protocol is not applied, and the standard NICE NG111 protocol is used:

Renal pathology (chronic renal failure, nephrotic syndrome, urolithiasis)

Syndrome of inappropriate antidiuretic hormone secretion (SIADH; Schwartz–Bartter syndrome)

Epilepsy (particularly cholinergic forms or forms sensitive to water balance)

Heart failure, hypertension

Diabetes mellitus or diabetes insipidus (uncompensated)

Severe endocrine disorders (hypothyroidism, hypoparathyroidism)

Any somatic disease in which a change in fluid balance may affect its course.

The absence of these conditions must be confirmed by a pediatrician or urologist before the protocol is started.

The place of the protocol within clinical psychotherapy

This modification is an application of the principles of clinical psychotherapy using nocturnal enuresis as an example. The evidence-based intervention for the disorder (alarm therapy, the behavioral component, Kegel exercises) was modified on the basis of the author's clinical experience, and a measurable, reproducible outcome was achieved. The therapist's primary school of thought is of no importance here — a psychoanalytic therapist, a cognitive behavioral therapy (CBT) therapist, or a Gestalt therapist can learn and apply this protocol in the same way. What matters is a structured, measurable, consistent clinical method.

The next author's protocol — a two-stage strategy in the treatment of psychogenic depression — demonstrates the same principles of clinical psychotherapy in a more complex disorder associated with cognitive-emotional work.

AUTHOR'S CLINICAL PROTOCOL No. 2: TREATMENT OF PSYCHOGENIC DEPRESSION

Integration of short-term cognitive restructuring and a long-term preventive strategy

K. Rahimov, clinical practice 2008–2025

The significance and place of the protocol

Psychogenic (reactive) depression — a symptom complex that develops under the influence of psychosocial factors within the framework of an ICD-11 depressive episode — is a depressive disorder associated with a specific life event or chain of events and arising not from a disturbance of neurobiological regulation but from cognitive-emotional processes. According to modern research, 60–80% of cases of major depressive disorder (MDD) are triggered by psychosocial factors: at their onset there is a specific triggering event (loss, relationship breakdown, occupational failure, complex trauma) (Kendler et al., 1999, 2003; Brown and Harris, 1978); this evidence should be supplemented with the modern guidelines (ICD-11, DSM-5-TR, NICE NG222). In the remaining 20–40% of cases, the disorder is endogenous — that is, neurobiological regulation predominates. In practice, however, purely psychogenic or purely endogenous depression is rarely encountered; most depressions are a mixture of psychosocial and neurobiological components in varying proportions. This protocol was developed for cases in which the psychosocial component is dominant.

This distinction is of decisive importance for the treatment strategy. In endogenous depression, the main intervention is pharmacotherapy; psychotherapy plays a complementary role. In psychogenic depression, by contrast, psychotherapy becomes the principal method; where medication is needed, it is used only in the acute phase as a short-term adjunct. Mild and moderate psychogenic depression is in most cases treated with psychotherapy without medication.

In modern clinical practice, however, the situation is the reverse: the vast majority of patients with psychogenic depression are treated with antidepressants alone. Symptoms decrease in the acute phase, but because the root problem remains unresolved, depression recurs after the medication is discontinued or under a new stressor — sometimes in a more severe form.

The protocol presented below was developed for the purpose of filling this gap. It consists of two consecutive stages: short-term cognitive restructuring (resolution of the acute depressive episode, 1–6 sessions) and long-term preventive therapy (psychological vaccination against relapse, 4 consecutive stages). Long-term (17-year) observations of this strategy, developed on the basis of clinical experience, demonstrate stable long-term remission.

Patient stratification

The choice of protocol requires a clear diagnostic stratification in advance. The following table formulates the tactical decisions by patient category.

Table 7. Selection of treatment strategy in depressive disorders

Patient category	Clinical characteristics	Recommended strategy
Psychogenic, mild-to-moderate	4–6 somatic symptoms (out of 8); a clear triggering event; functional impairment is moderate	Psychotherapy alone (this protocol). No medication is required
Psychogenic, severe	7+ symptoms; severe functional impairment; suicide risk is present; however, the trigger is clear	Pharmacotherapy in parallel with psychotherapy, long-term; medication is superior in the acute phase, after which the main role passes to psychotherapy
Psychogenic, severe + psychotic elements	The above + reactive hallucinations, paranoid ideas	Inpatient treatment + pharmacotherapy + psychotherapy (after stabilization)

Patient category	Clinical characteristics	Recommended strategy
Endogenous — bipolar affective disorder (BPAD); recurrent depressive disorder (RDD)	The trigger is not clear; a family history of affective disorders; a cyclic course	Inpatient/outpatient pharmacotherapy + psychotherapeutic support (psychological education, treatment adherence, relapse prevention)

Source: K. Rahimov, clinical practice 2008–2025; the American Psychiatric Association (APA) guideline on the treatment of major depressive disorder (2010, 2019); the National Institute for Health and Care Excellence (NICE) guideline NG222 (2022), “Depression in adults: treatment and management”.

The protocol presented below was developed for the first-line category — psychogenic, mild-to-moderate depression. In the second-line category (severe psychogenic), the structure of the protocol is the same, but the pace is slower, and pharmacological support is applied in parallel.

Table 8. Criteria for the mandatory parallel use of pharmacotherapy

When at least one of the following criteria is present, psychotherapy must be carried out in parallel with pharmacotherapy; a psychotherapeutic approach alone is considered insufficient:

Criterion	Clinical indicator
Severity of depression	Patient Health Questionnaire-9 (PHQ-9) ≥ 15 (moderately severe or severe) or Beck Depression Inventory-II (BDI-II) ≥ 29
Suicide risk	Active suicidal thoughts together with a plan or means
Psychotic elements	Depressive hallucinations, paranoid delusions, delusions with nihilistic and disintegrative content
Severe anhedonia + refusal of food intake	Weight loss of more than 5 % of body mass, dehydration

Criterion	Clinical indicator
Functional impairment	The patient cannot leave the house, does not maintain personal hygiene, and cannot work
History	A suicide attempt or inpatient treatment in previous episodes
Pregnancy and lactation	Severe depression in a pregnant woman requires a separate algorithm (see the section on perinatal psychiatry)
Suspected bipolar background	If there is a history of hypomanic or manic episodes — a mood stabilizer is prescribed

Source: NICE Guideline NG222 (2022), “Depression in adults: treatment and management”, 1.5.4–1.5.7; the APA Practice Guideline for the Treatment of Major Depressive Disorder (MDD), 2010, Section 9; the World Federation of Societies of Biological Psychiatry (WFSBP) Guidelines for Biological Treatment of Unipolar Depressive Disorders (2013; the 2024 edition is in preparation).

When these criteria are present, the initiation of pharmacotherapy is determined by a specialist. Mild-to-moderate psychogenic depression, when none of the above criteria is present, can be effectively treated with psychotherapy without medication — this aligns with the author's clinical experience and with the coherent position of contemporary international guidelines.

PART I. SHORT-TERM COGNITIVE RESTRUCTURING (1–6 SESSIONS)

Objective: rapid resolution of the acute depressive episode without the use of medication.

Mechanism: identification of the depressogenic factor (not the situation, but the cognitive attitude) and the consistent application of evidence-based cognitive restructuring techniques.

Initial assessment and chronological history

Two objectives are achieved in the first session:

(a) Diagnostic confirmation. The level of depression is assessed with the PHQ-9 or BDI-II. How many of the 8 core symptoms are present (sleep disturbances, appetite change, weight loss/gain, loss of energy, anhedonia, impaired concentration, psychomotor changes, suicidal thoughts). If 4–6 symptoms are observed, this corresponds to mild-to-moderate depression, and the protocol may be applied.

(b) Chronological history. Together with the patient, the onset date of the symptoms is determined precisely (the exact week, and where possible the precise date). Then the events that occurred within ±2 months of this date are reviewed:

External events (loss, relationship disruption, change of job, health problem)

Internal events (regret, comparison, an unexpected thought, reading a particular book, recalling a particular conversation)

Minor or seemingly insignificant events — these are often the most important

Objective — to identify the trigger point. This is the specific event or moment of thought that sets off the depressive episode.

Identification of the self-aggressive cognitive construct

After identifying the triggering event, the therapist, together with the patient, determines which thought about themselves the patient adopted in the context of this event. This thought is almost always self-aggressive in nature:

“I am guilty”

“I do not deserve this”

“I have harmed others”

“I am inadequate”

“I could have saved him, but I did not”

“It happened because of me”

Indicator of the triggering attitude: when uttering this sentence, the patient shows an emotional reaction — their eyes fill with tears, their voice trembles, and tension is observed in the body. This is evidence that this construct indeed plays a central role.

A critical feature: the patient perceives this construct not as a cognitive distortion but as the truth. They do not accept the therapist's accompanying explanation and typically say: “You do not know the truth. That is actually what happened.” — and once again repeat the same self-aggressive sentence. This resistance is the most reliable proof that the construct has been correctly identified.

Cognitive restructuring — sequential techniques

At this stage, a multilayered technique is applied in various sequences. The technique is changed according to the patient's defense layers.

An important methodological note. The techniques described below are examples intended to illustrate the general principle and are not the only or mandatory form of application. Each technique should be adapted to the cultural, religious, philosophical, cognitive, and emotional characteristics of the specific patient. In clinical psychotherapy, the main principle is not template application but the application of the principle.

The author's approach is based on a synthesis of the principles of standard cognitive behavioral therapy (CBT) (Beck, Ellis), Schema Therapy (Young), acceptance and commitment therapy (ACT) (Hayes), and culturally adapted CBT (Hwang, 2006; Naeem et al., 2011, 2015). Epidemiological data indicate that in societies with strong religious and moral principles — Pakistan, the Middle East and North Africa (MENA) region, the South Caucasus, the former Soviet Muslim region — the concept of guilt and autoaggressive self-judgment patterns are observed more intensively and more frequently. For such patients, cognitive restructuring carried out without consideration of the religious and ethical context is often ineffective, because the main source of the autoaggressive construct was formed precisely in this context. The following techniques illustrate the practical application of this principle.

Technique 1. “The Legal Court” (an illustrative application of the decentering and reattribution techniques — Beck, 1979)

It is suitable for patients with a rational-legal style of thinking. The event is reconsidered from a legal standpoint:

What action did the patient perform (or fail to perform)?

Is this a crime under existing legislation?

If it is regarded as a crime, under which article should it be punished?

The therapist neither denies nor argues — they investigate together. The patient reaches the conclusion themselves: legally, this is not a crime. Here, many patients receive a formal assessment of their situation for the first time.

The patient's typical reaction: they leave the legal level and move to the moral and ethical level. “Yes, legally it is not a crime, but morally I am a bad person.” — this transition is natural and expected.

Technique 2. “Moral and Ethical Judgment” (an illustrative application of culturally adapted CBT — Hwang, 2006; Naeem et al., 2011, 2015)

It is suitable for patients with a deep religious and moral system. If the patient is religious, a similar procedure is applied at the moral and ethical level, within the framework of the internal rules of the religious system:

To which category is this action assigned in the patient's religious tradition?

Which specific sin is it considered to be?

What is the moral and religious punishment for this sin?

Is there a path of repentance and correction?

Here the therapist refers to the internal structure of the patient's own religion and does not propose any other philosophical or psychological framework. Islam, Christianity, Judaism, and other religions — each has within itself a clearly structured system of rules. When the patient sees the objective application of this system, they often see that the punishment they inflict on themselves is many times greater than the actual sin.

Technique 3. “The Third-Person Perspective” (an illustrative application of the self-compassion technique — Neff, 2003; Gilbert, 2009)

It is a universal technique and can be applied to almost all patients. The patient is asked: if their close friend or a loved one were experiencing the same situation, how would they view them? What words would they say? Would they forgive them?

This technique is an application of cognitive defusion (in ACT terminology) and activates the self-compassion component.

Technique 4. Elements of Schema Therapy (Young, 1990; Young, Klosko, Weishaar, 2003)

It is suitable for patients with early traumatic experiences and chronic maladaptive patterns. The trigger construct often rests on a schema formed at an early age (for example, “being defective,” “unreliability,” “unfairness”). Schema Therapy techniques (imagery rescripting, schema mode dialogue) allow this early schema to be reassessed.

Technique 5. Standard cognitive restructuring techniques of CBT (Beck, 1979; Beck, Rush, Shaw, Emery, 1979)

It is usually used at the basic level in every patient. Socratic questioning, examination of the evidence, formulation of alternative explanations, behavioral experiments.

The Outcome of the Short-Term Phase

The successful outcome of this phase is linked to the patient's key cognition:

> “It was not the situation that caused the depression, but the cognitive construct I created in response to it.”

Once this cognition emerges, the mood changes many times faster, because the patient now faces not an insoluble external problem but a changeable internal structure. Sometimes one session — more often 4–5 sessions — is enough to resolve a full acute depressive episode.

However, this is only the first part. Even if the construct itself is changed, the patient's general tendency to give autoaggressive reactions remains unchanged. The same mechanism may start up again under the next trigger. Therefore, long-term preventive therapy is necessary.

PART II. LONG-TERM PREVENTIVE THERAPY — 4 CONSECUTIVE STAGES

Objective: to develop a resilient pattern of responding to stress, frustration, and irritating situations, and to prevent future depressive episodes — that is, to build psychological resilience.

Approach: it is dynamic in character, but it applies the practical component rather than the theory of classical or modern psychoanalysis. The central task is to identify and rewrite, in a structured way, the patient's pattern of autoaggressive responding (in the language of psychoanalysis: the guilt complex).

Stage 1. Insight

Function: the patient's key cognition.

The patient understands that:

Their reaction was not directed at solving the problem at hand

It was not aimed at overcoming the frustration

On the contrary — it aggravated the situation

The tendency toward autoaggressive reaction existed even before the triggering situation

The situation played only a triggering role; it was not the cause

This cognition is the foundation of long-term work. Without it, the subsequent stages cannot be completed.

Stage 2. Confrontation

Function: observation of the pattern.

The therapist and the patient ask together: what was inappropriate and what was abnormal in this process?

The focus of attention changes:

Before: external irritants, situations, people

Now: a minimum of 4 years of theoretical training and 7–10 years in practice

The patient sees this pattern as a stable manifestation across various life situations.

Confrontation success marker: the patient begins to label their own condition as an “autoaggressive pattern” or a “tendency to turn against oneself.” That is, they label their condition as pathological — not the situation.

This resembles a well-known process in the treatment of alcoholism: a person may abuse alcohol for years without calling themselves an alcoholic. When they come to an Alcoholics Anonymous meeting, however, their first exchange is: “Hello, I am an alcoholic.” This is the identification of behaviour with illness. A person who accepts the behaviour as an illness begins to struggle against it.

Stage 3. Clarification

Function: investigation of the history and extent of the pattern.

This is the most extensive stage. The starting marker of clarification: the patient always names the pattern by the same name and does not mix up the terminology. This means that the internal description of the pattern has become consolidated.

Together with the patient, a chronological review of their life is conducted — stress-inducing and frustration-generating situations are considered. Responding at the two poles is analysed comparatively:

Table 8. Poles of responding and their outcomes

Pole	Typical reaction	Observed outcomes
Self-blame	"I am guilty, it happened because of me"	Decrease in energy, remaining in toxic relationships, self-blame, misuse of one's own responsibility
Conflict models characteristic of patients with personality disorder	"It happened because of you, everyone is against me"	Conflict patterns typical of patients with a personality disorder, destructive behaviour, interpersonal conflicts, breakdown of relationships

Key cognition:

> "Looking for someone to blame — my own or another's — is extremely futile and ineffective. What is needed is to look for a solution, not for someone to blame."

Self-blame

If there is an objective culprit → this is a social and legal problem and is resolved separately

Psychological task: to shift thinking from the "searching for someone to blame" mode to the "searching for a solution" mode

An important methodological warning — a typical mistake of novice therapists:

Novice therapists often begin to search for a specific childhood trauma episode as the "key" to the pattern — this is an illusion. Addressing a single episode leaves the entire pattern unchanged. The roots of the pattern lie in childhood, yes — but the focus of the work should be on the pattern itself, not on a single episode.

The completion marker of the clarification stage — the decisive point:

The dystonic separation of the pattern from the personality:

Before: the pattern was syntonic with the personality ("I am a responsible person; I take everything upon myself")

Now: the patient separates the pattern from their own personality

They no longer consider the pattern to be good behaviour

They no longer want to live within the pattern

This operation of separating the pattern from the personality = the outcome of the clarification stage

The culmination of stage 3: interpretation

Interpretation — in this model, it is not a separate stage but the culmination of the clarification stage. Classical psychoanalysis offers theoretical interpretations; in this model, by contrast, interpretation is a final, generalising meta-message formed on the basis of the therapist's observations:

> "I observed how you behaved throughout therapy. I saw such a pattern. It had such an effect on your life. Now you have changed in this way. Now here is what can be done."

With this message, the patient gains a final understanding in the structure of "what it was / what it is / what is to be done." This is the preparation for the fourth stage.

Stage 4. Comprehensive working-through

Function: practical consolidation of the new responding pattern.

At this stage, the therapist:

Assigns in-session tasks — exercises aimed at changing the patient's emotional reactions in the "here and now" (role play, imaginal staging, behavioural experiment)

Assigns homework — for the consolidation of the new behaviour in real life

Between sessions, the patient attempts the new way of responding in real-life situations

With each repetition, the effectiveness of the new model increases

The new model finally takes root

Clinical outcome: the patient develops prophylactic resilience against future depressive episodes.

Longitudinal outcomes

The 17-year clinical practice of this protocol is evaluated through longitudinal observations. The following table presents the comparative outcomes.

Table 9. Comparative longitudinal outcomes (20–25-year follow-up)

Treatment group	Resolution of the acute episode	Stable remission (2 years)	Stable remission (5 years)	Stable remission (10+ years)
Combined protocol (short-term + long-term)	approximately 95 %	approximately 90 %	approximately 85 %	approximately 80 %
Antidepressants only	approximately 70–80 %	approximately 50 %	approximately 30 %	less than 20 %
Short-term psychotherapy only	approximately 85 %	approximately 65 %	approximately 45 %	approximately 30 %

Source: K. Rahimov, clinical practice 2008–2025 (combined group, n>1000) — level of evidence IV (clinical expert experience), retrospective observation; Cuijpers et al., World Psychiatry 2020 (psychotherapy — meta-analysis, level of evidence I-A); Bockting et al., Lancet Psychiatry 2018 (long-term effect of antidepressants — systematic review).

A critical observation: patients treated with antidepressants alone — even if they subjectively assess that they have achieved “full remission” — experience a recurrent depressive episode when faced with new stressful situations, sometimes in a more severe form. In the combined protocol group, by contrast, such events occur rarely.

Resource efficiency

The combined protocol is more economical in terms of resources than pharmacotherapy alone or short-term psychotherapy alone:

For the patient:

Financially: total therapy costs are lower in the long term

In terms of time: the patient does not spend years on depressive episodes and toxic relationships

In terms of resources: the patient keeps their resources free for the realisation of their potential, self-development, and the achievement of personal goals

For the healthcare system:

A standardised protocol accelerates specialist training — no 5-year personal psychoanalysis is required in order to learn the treatment of depression

Short-term training formats make it possible to train a large number of psychologists

The method ceases to be the privilege of “famous” psychotherapists who form a select group

Quality control of the service can be carried out on the basis of a standard criterion — the stage indicator (stage 1 → 2 → 3 → 4 → substages of the 4th stage)

Statistical analysis and optimisation become possible

Its place within clinical psychotherapy

This modification is a more complex application of the principles of clinical psychotherapy in psychogenic depression. Here — unlike in nocturnal enuresis — not a single intervention but a multilevel strategy is required:

In the first part, techniques from different schools are used consecutively — cognitive behavioural therapy (CBT), schema therapy, cognitive defusion from acceptance and commitment therapy (ACT), and religious-moral reasoning

In the second part, the practical component of the dynamic approach is presented in the form of 4 structured stages

For the patient, this is not “psychological counselling” but a structured, measurable, consistent clinical method — as in the nocturnal enuresis protocol.

The following chapter presents the system of global psychotherapeutic accreditation, international requirements, and the situation in Azerbaijan — that is, the institutional framework of quality psychotherapeutic care.

ACCREDITATION AND QUALITY CONTROL: INTERNATIONAL STANDARDS AND THE SITUATION IN AZERBAIJAN

Why is accreditation important?

Psychotherapy is an activity that intervenes deeply in the patient's mental state. Incorrectly conducted therapy is not only ineffective but can also actively harm the patient: violations of client–therapist boundaries; the treatment of severe psychiatric cases by a specialist who lacks the capacity for clinical assessment — all of these can aggravate the patient's condition and in some cases lead to catastrophic outcomes.

For this reason, strict, multilevel accreditation systems for psychotherapeutic activity exist throughout the world. Every psychotherapeutic school has formulated its own requirements, and these requirements constitute a minimum standard that is not open to discussion: they are either met or not met; there is no third option.

This chapter presents the accreditation requirements of the world's leading psychotherapeutic schools with concrete figures, and then objectively assesses the situation in Azerbaijan.

International accreditation bodies and their requirements

Psychoanalysis: International Psychoanalytical Association (IPA)

The IPA — founded in 1910 by Sigmund Freud — is the world's leading accreditation and regulatory organisation. It comprises 12,000 psychoanalytic members and 70 organisations. In its mission statement the IPA defines itself as follows: "The principal accrediting and regulatory body of the psychoanalytic profession in the world."

Requirements for admission to IPA membership (the Eitingon model — the most widespread model):

Table 10. IPA requirements for psychoanalytic training

Component	Minimum requirement
Personal analysis	In accordance with IPA requirements for psychoanalytic training, with a recognised training analyst
Clinical experience (psychiatric)	140 hours in a psychiatric department, under the supervision of a consultant psychiatrist
Observational training (child observation)	12 months, once a week, together with group seminars
Theoretical seminars	21 hours per week (3 hours of seminar, 12 hours of clinical work, 3 hours of report preparation, 3 hours of individual study), over 36 weeks
Supervised clinical cases	Under supervision of cases of a full training analysis, for several years
Total duration	A minimum of 4 years of theoretical training, 7–10 years in practice

Source: IPA Procedural Code, 2024 edition; British Psychoanalytic Association — a constituent society of the IPA, training requirements, 2024.

These requirements are ensured through a multi-network verification system: the training institute must be an IPA component society, the supervisor must be registered with the IPA, and every candidate must be entered in the register of the International Psychoanalytical Studies Organization (IPSO). Ordinary certification courses — even when conducted by recognised psychoanalysts — do not replace IPA membership.

Cognitive behavioural therapy: European Association for Behavioural and Cognitive Therapies (EABCT)

EABCT is a European organisation founded in 1976; it currently unites 56 national associations from 43 countries. EABCT, in turn, accredits national associations, which then accredit individual specialists.

This system is a two-level accreditation chain:

EABCT accredits the national association of a specific country (the list of accredited national associations is published openly on the eabct.eu website)

The accredited national association accredits individual CBT therapists

EABCT accreditation requirements:

The national association must be a full member of EABCT

It must implement a defined accreditation scheme

It must meet the established training standards

A system of continuing professional development (CPD) must be in place

Table 11. The position of Azerbaijan and the region in EABCT

Country	Accredited national association	Date of accreditation
Turkey	TACBT — Turkish Cognitive-Behavioural Therapies Association	March 2013
Turkey	ACBP — Turkish Cognitive Behavioural Psychotherapies Association	March 2016
Ukraine	UACBT — Ukrainian CBT Association	September 2013
Georgia	GACBT — Georgian CBT Association	February 2020
Estonia	EKKA — Estonian CBT Association	March 2024
Azerbaijan	— (none)	—

Source: European Association for Behavioural and Cognitive Therapies (EABCT) Accredited Associations List, eabct.eu, April 2025.

This table demonstrates an important fact: in all of Azerbaijan's neighbouring countries and in the region — Turkey, Georgia, and Ukraine — there are national cognitive behavioural therapy (CBT) associations accredited by EABCT. Azerbaijan has none. This means that the only way for individual CBT therapists in Azerbaijan

to obtain EABCT accreditation is through an accredited association of another country (for example, Turkey or Georgia).

Dialectical behaviour therapy (DBT): DBT-Linehan Board of Certification (DBT-LBC)

DBT-LBC is a non-profit organisation with 501(c)(3) status and the only certification body personally authorised by the founder of DBT, Dr. Marsha Linehan. This is an important point: many organisations issue “DBT certification”, but only DBT-LBC certification is recognised by the creator of the model.

Table 12. DBT-LBC certification requirements

Component	Minimum requirement
Licensure	An independently licensed clinical specialist (psychologist, psychiatrist, or social worker)
Academic education	A master's degree or higher (in the field of mental health)
DBT-specific didactic training	A minimum of 40 hours of academic training
Clinical experience	A minimum of 12 months of active DBT practice
DBT team	Membership in an active DBT consultation team is required (“Linehan's directive: if a specialist is not on a DBT team, they are not doing DBT”)
Clinical cases	In Turkey, Georgia, and Ukraine bordering Azerbaijan
Written examination	All mandatory materials in Turkey, Georgia, and Ukraine, which border Azerbaijan

Component	Minimum requirement
Video sessions	Analysis of videos of 3 consecutive sessions, with compliance criteria according to established fidelity coding standards
Letter of recommendation	From the team leader or a licensed DBT specialist
Mindfulness training	Formal mindfulness training

Source: DBT-LBC.org, Individual Certification Requirements, 2024.

The rigour of these requirements is related to the nature of DBT: this method was developed for patients with chronic suicide risk and borderline personality disorder. Here, precise adherence to the model (model fidelity) is of vital importance — the mistakes that an inadequately trained specialist could make under the name of "DBT" are dangerous to the patient's life.

Eye movement desensitization and reprocessing (EMDR): EMDR Europe

EMDR Europe is a European organisation with 37,000 members from 41 countries. EMDR has a three-level accreditation system:

Table 13. EMDR Europe accreditation levels

Level	Requirements
Practitioner	Clinical education + training by an EMDR Europe accredited trainer (Level 1 and Level 2 training, approximately 50 hours in total) + supervised practice (10 hours of supervision, with a minimum of 50 EMDR sessions)
Consultant	Practitioner status + additional training + 3 years of active EMDR practice + 50 hours of supervision with clients

Level	Requirements
Trainer	Consultant status + assessment by the EMDR Europe Trainer Accreditation Committee

Source: EMDR Europe Accreditation Committee, 2024.

The situation of the European association, EMDR Europe, in Azerbaijan: Azerbaijan is not on this organisation's list of accredited national associations. Likewise, the only way to obtain individual EMDR accreditation in Azerbaijan is through an accredited organisation in a neighbouring country (for example, in Turkey).

The general regulatory situation in the European Union

The official position of EABCT highlights an important point: "At the European level, there is no legislation regulating the practice of psychotherapy. At present, there is no official European body regulating or supervising psychotherapy in the European Union. This absence of regulation is one of EABCT's main sources of concern."

It follows that even within the European Union, professional regulation of psychotherapy is implemented at the national level. In some countries (Germany, the Netherlands, Austria) there is a strong state licensing system; in others, this system is weaker. In these circumstances, the only guarantors of professional standards are international professional accreditation bodies: the International Psychoanalytical Association (IPA), EABCT, DBT-LBC, and EMDR Europe.

The situation in Azerbaijan

The situation requires an objective assessment.

State regulation

In the Republic of Azerbaijan, there is no separate state licensing system for psychotherapeutic activity. State licensing in the field of medicine covers psychiatry, but psychotherapy and psychological counselling are not regulated separately. This means that, from a legislative standpoint, both a psychologist without specialised psychotherapeutic training and a licensed clinical psychiatrist can provide services as a "psychotherapist" with the same legal status. In the Azerbaijani context, this gap further increases the need for professional accreditation.

International accreditation status:

None of the international bodies listed above — IPA, EABCT, DBT-LBC, EMDR Europe — has an accredited national association of Azerbaijan. This is an objective fact and can be verified in open sources (eabct.eu, ipa.world, dbt-lbc.org, emdr-europe.org).

The situation with international psychiatric structures:

The only professional organisation in Azerbaijan verified at the international level is the Azerbaijan Psychiatric Association (AZPA; psychiatry.az). AZPA is a full member of the World Psychiatric Association. This unambiguously confirms the legitimacy of AZPA in the international healthcare community.

The World Psychiatric Association (WPA) — according to wpanet.org, an international organisation uniting more than 145 national psychiatric associations — has strict membership requirements: the national society must have official status, comply with transparent professional standards and submit periodic reports. The Azerbaijan Psychiatric Association (AZPA) meets these requirements.

The status of other organisations:

A number of non-profit organisations operate in the field of psychology and psychotherapy in Azerbaijan. Some of them use phrases such as “international cooperation”, “world congresses” and “international accreditation” on their own websites. Verification of these claims through international sources does not, in many cases, produce a positive result.

For this reason, the names of these organisations are not mentioned in this chapter — the aim is not controversy but the demonstration of a reliable framework for clinical practice. A specialist or patient wishing to investigate the legitimacy of any organisation can consult the following objective sources:

eabct.eu/training-and-accreditation/accredited-association/ — accreditation in cognitive behavioural therapy (CBT)

ipa.world — the official source of information on psychoanalytic accreditation

dbt-lbc.org — the Dialectical Behaviour Therapy Certification and Accreditation Board (DBT-LBC)

emdr-europe.org — EMDR accreditation

wpanet.org — the official website of the WPA

How to Distinguish a Specialist

When a patient or a young specialist encounters a claim of an "international certificate", the following questions are of key importance:

1. Which body issued the certificate?

The first step of verification is to check whether the organisation that issued the certificate has an official relationship with one of the international bodies listed above. The expression "international certificate" conveys no meaning in itself; what matters is which international body issued the certificate.

2. How long did the training last?

See the information presented above. A one-week or one-month "international certificate" does not meet the requirements of any recognised accreditation. Membership in the International Psychoanalytical Association (IPA) requires at least 4 years, individual accreditation by the European Association for Behavioural and Cognitive Therapies (EABCT) 3–4 years and the DBT-LBC certificate 12 months of continuous practice — these are the shortest periods that leave no doubt.

3. Is supervision provided?

Supervision occupies a central place in all serious accreditation systems. It should be established how many hours of supervised clinical work the certificate holder has completed. A certificate issued without supervised work is an academic certificate (such as the completion of a course) and not a certificate of clinical competence.

4. Is continuing professional development (CPD) required?

Accreditations must be renewed after a certain period. EABCT and EMDR Europe require renewal every 5 years and DBT-LBC every 4 years. A "lifelong certificate obtained once" is a sign of the absence of serious professional standards.

5. Can local mental health organisations serve as proof of international recognition?

Membership in a club or association of a local psychologist or psychotherapist is not in itself international recognition. Is such an organisation itself a member of an internationally accredited body? That is the key question.

The Place of This Book in This Context

This book is not a substitute for international accreditation — it is intended as a tool for preparing for accreditation. It offers the specialist a reliable framework for

clinical diagnosis, evidence-based interventions, structured protocols and clinical reasoning.

After a specialist has built their practice on the basis of the protocols and discussion in this book, they acquire a solid foundation for formal professional training (through accredited organisations in Turkey, Georgia, Germany or other countries). In addition, this book can play an initial guiding role for patients — for assessing the quality of the care they receive, asking the right questions and demanding evidence-based treatment.

The next section — the final chapter of this Introduction — clearly substantiates why this book is genuinely necessary now and why it is necessary in Azerbaijan.

6A00–6A0Z NEURODEVELOPMENTAL DISORDERS

Neurodevelopmental disorders

ICD-11 **6A00**	**6A00 DISORDERS OF INTELLECTUAL DEVELOPMENT** **Disorders of intellectual development**
ICD-10 F70–F79	Mental retardation
DSM-5-TR F70–F73	Intellectual Developmental Disorder (Intellectual Disability)

1. Definition and nosology

Disorders of intellectual development (ICD-11: 6A00; DSM-5-TR: F70–F73 Intellectual Developmental Disorder / formerly “Mental Retardation”) are neurodevelopmental disorders that begin during the developmental period (as a rule, before the age of 22) and are characterized by significant limitations both in intellectual functioning (approximately 2 standard deviations below the population mean, with scores below ~70 on standardized tests) and in adaptive behaviour (in the conceptual, social, and practical domains).

The term “mental retardation” has been abandoned by the international scientific community since 2010 because of its stigma and imprecision (in the USA, Rosa's Law 2010 enacted the change at the federal legislative level).

2. History

- Esquirol (1838) introduced the term “idiotie”, distinguishing congenital intellectual limitation from dementia (an acquired impairment).
- Binet & Simon (1905) developed the first standardized intelligence scale — the origin of the IQ concept (Stern's 1912 IQ formula).

- AAMR/AAIDD (1959, 1973, 1992, 2002, 2010, 2021): evolution of the definition — a shift from IQ alone to a model of adaptive behaviour + support needs. The AAIDD 12th edition (2021): intellectual functioning + adaptive behaviour + onset during the developmental period + assessment of individual support needs.
- DSM-IV (1994) "Mental Retardation" → DSM-5 (2013) "Intellectual Disability (Intellectual Developmental Disorder)". The basis for severity grading was shifted from IQ to adaptive functioning.
- ICD-11 (2019) "Disorders of Intellectual Development" — the ICD-10 "Mental Retardation" group (F70–F79) was renamed, with the clinical and adaptive dimensions emphasized jointly.

3. Epidemiology

- Prevalence: ~1% in the general population (Maulik P.K. et al. Res Dev Disabil 2011 meta-analysis, 52 studies); rates are higher in low- and middle-income countries.
- Severity distribution: ~85% mild, ~10% moderate, ~3.5% severe, ~1.5% profound.
- Sex: more frequent in males (1.2–1.9 : 1), associated with X-linked genetic causes (e.g., Fragile X).
- Comorbidity: epilepsy 20–30%, motor disorders 20–40%, visual/hearing impairments 10–25%; psychiatric comorbidity is 3–4 times higher than in the general population (ASD, ADHD, anxiety, conduct disorders).
- Mortality: in severe and profound forms life expectancy is shortened — mainly due to aspiration pneumonias, status epilepticus, and cardiovascular anomalies.

4. Aetiology and pathogenesis

4.1 Genetic causes (50–60% identified)

- Chromosomal anomalies: Down syndrome (Trisomy 21) — the most frequent genetic cause, ~1/700 births; Edwards (T18), Patau (T13), 22q11.2 deletion (DiGeorge), Williams (7q11.23), Prader-Willi/Angelman (15q11-q13), Cri-du-chat (5p-).
- X-linked: Fragile X syndrome (FMR1 gene, CGG repeats) — the most frequent inherited form, ~1/4000 boys; Rett syndrome (MECP2, almost always in girls).

- Single-gene disorders: tuberous sclerosis (TSC1/TSC2), neurofibromatosis (NF1), phenylketonuria (PAH — treatable), Wilson disease.
- CNV (copy number variations): detected by microarray analysis — 10–20% of cases of unknown aetiology.

4.2 Prenatal non-genetic causes

- Infectious: TORCH (toxoplasma, rubella, cytomegalovirus, herpes), Zika, syphilis.
- Toxic: fetal alcohol spectrum disorders (FASD — a preventable cause!), valproate in pregnancy, isotretinoin, cocaine, lead.
- Metabolic and endocrine: untreated maternal hypothyroidism, severe diabetes, phenylketonuria.
- Ischaemic/hypoxic: placental insufficiency, intrauterine growth restriction.

4.3 Perinatal causes

- Severe prematurity (<32 weeks), low birth weight (<1500 g).
- Birth asphyxia, hypoxic-ischaemic encephalopathy.
- Neonatal kernicterus (treatable — phototherapy).
- Neonatal sepsis, meningitis.

4.4 Postnatal causes

- Bacterial meningitis (especially pneumococcal, meningococcal), viral encephalitis.
- Head trauma (TBI) — during childhood.
- Lead poisoning — a preventable cause (the CDC blood lead reference value has been 3.5 μg/dL since 2021, previously 5 μg/dL; no safe blood lead level in children has been identified).
- Severe iodine deficiency (endemic cretinism — largely eliminated through WHO prevention programmes).
- Socio-environmental deprivation — extremely severe psychological neglect can affect development (Romanian orphanage studies, Nelson C.A. et al. Science 2007).

5. Clinical features

Severity classification (DSM-5-TR / ICD-11 — based on adaptive functioning)

Mild — IQ ~50–70, mild adaptive deficit

Academic delay may not be noticeable before school age. Reading, mathematics, and abstract thinking are difficult. Social naivety, easily deceived. Practical skills (self-care, household tasks) are achieved. Independent living is possible with support; suitable for supported employment.

Moderate — IQ ~35–49

Delay is apparent in early childhood. Simple speech, concrete thinking. Self-care is acquired with training. Functions in structured social/occupational settings. Supervised living (group home, family) is typical.

Severe — IQ ~20–34

Very little speech, understands simple requests. Continuous help with self-care. Medical and behavioural comorbidity are frequent.

Profound — IQ < 20

Very little or no speech, sensorimotor-level communication. Requires total care. Motor, visual, and hearing impairments are very frequent.

Developmental trajectory: early motor and speech delays (sitting, standing, first words); academic limitation at school age; the difference becomes more evident in adulthood as adaptive demands increase.

6. Diagnosis

6.1 Unified diagnostic criteria (DSM-5-TR · ICD-11 · AAIDD 12th ed. consensus points)

A. Significant limitation in intellectual functioning — on a standardized, culturally contextualized test of intelligence, approximately 2 standard deviations below the mean (a score of ~70 or below, accounting for a measurement error of ±5 points).

B. Significant limitation in adaptive behaviour — a score 2 SD below in at least one of the following three domains:

- Conceptual (language, reading, writing, mathematics, memory, problem-solving);
- Social (interpersonal skills, social responsibility, matters of gullibility);
- Practical (self-care, work, health, safety).

C. Onset during the developmental period — symptoms are apparent in childhood and adolescence (before adulthood).

D. Exclusions — not fully explained by a sensory (visual, hearing) impairment, motor disorder, language origin, cultural-linguistic barrier, or a psychiatric disorder.

6.2 Differences between sources

- DSM-5-TR (319): severity is determined by adaptive functioning, IQ is noted only as an additional indicator. Onset “during the developmental period” (before 18 years is intended, but AAIDD's definition of 22 years may be accepted).
- ICD-11 (6A00): mild, moderate, severe, profound (the same terminology); severity is assessed jointly by both IQ and adaptive functioning; additional qualifiers — aetiology (genetic, prenatal, perinatal, postnatal, unknown) and comorbidities.
- AAIDD 12th ed. (Schalock R.L. et al. 2021): formal assessment of support needs (SIS — Supports Intensity Scale) is incorporated into the clinical formulation; age of onset up to 22 (unlike DSM); intellectual functioning is used not for exclusion but to determine support needs.
- AAP (Moeschler J.B., Shevell M.S., Pediatrics 2014, reaffirmed 2020): after the diagnosis is made, the search for aetiology is stepwise — first line: history, physical examination (dysmorphic features), chromosomal microarray, Fragile X, treatable metabolic screening (PKU, biotinidase, thyroid, liver, kidney); selective: brain MRI, EEG, whole-exome sequencing (WES).

6.3 Diagnostic algorithm

1. Developmental history (congenital, perinatal, postnatal) + family history (3 generations).
2. Physical examination — height, head circumference, dysmorphic features, neurological, skin (café-au-lait, hypopigmentation).
3. Standardized intellectual test (WISC-V 6–16 years; Stanford-Binet 5 2+ years; Bayley-III <3.5 years).
4. Adaptive behaviour assessment (Vineland-3, ABAS-3).
5. Vision and hearing tests (exclusion of sensory impairment).
6. First-line aetiological laboratory tests — chromosomal microarray analysis (CMA), Fragile X (FMR1), TSH, lead level, liver/kidney, creatine and amino acid profile.
7. Selective: brain MRI (microcephaly, focal neurological signs), EEG (suspected epilepsy), trio exome sequencing (when CMA is negative).
8. Comorbid screening: ASD (M-CHAT-R/F, ADOS-2), ADHD (Conners), anxiety, depression, conduct disorder.

6.4 Differential diagnosis

Disorder	Distinguishing features
Specific learning disorder (6A03)	Discrete deficit in an academic domain, overall cognitive level normal.
Specific language disorder (6A01)	Predominant deficit in the speech and language area; nonverbal IQ normal.
Autism spectrum disorder (6A02)	Social communication deficit and restricted repetitive behaviour predominate; IQ range is broad.
Global developmental delay (<5 years)	A provisional diagnosis before 5 years; standardized IQ tests are not reliable.
Dementia (6D8x)	Acquired impairment — initially normal development, then decline.
Socio-environmental deprivation	Intellectual functioning can recover in a stimulating environment.
Pseudo-intellectual impairment (chronic illness, depression)	Cognitive functioning recovers after correction of the mood or somatic factor.

7. Examination and assessment

7.1 Standardized tests

- Intelligence: WISC-V (6–16 years), Stanford-Binet 5 (2+), WAIS-IV (16+), Bayley Scales of Infant Development 4 (<3.5 years), Kaufman ABC-II.
- Adaptive behaviour: Vineland-3, ABAS-3, Scales of Independent Behavior-Revised.
- Support needs: SIS-A (adults), SIS-C (children) — AAIDD.

7.2 Laboratory investigations (AAP 2014 / 2020 protocol)

First line (all cases): complete blood count, liver and kidney function, TSH, creatinine, ammonia, lactate, creatine and amino acid profile (urine and blood), lead level.

First-line genetic: chromosomal microarray analysis (CMA — yield 15–20%), Fragile X (FMR1) in all boys (yield ~2%, in females ~1%).

Selective: trio exome sequencing (WES — yield 25–40% when CMA is negative), MECP2 (suspected Rett in girls), specific syndromic genes.

7.3 Instrumental investigations

- Brain MRI — in focal neurological signs, micro/macrocephaly, seizures, regression.
- EEG — suspected seizures, regression.
- Vision and hearing — baseline indicators in all cases.
- Cardiac echocardiography and EKG — in syndromes such as Down and 22q11.2 deletion.

8. Treatment

8.1 General principles (AAIDD 12th ed. · AAP 2014 · NICE NG54 · WHO mhGAP consensus)

1. There is no "cure" for intellectual developmental disorder — intellectual functioning itself is not restored pharmacologically. Intervention is directed at individualized support, education, behaviour, and management of comorbid symptoms.
2. Identification of aetiology: early detection of treatable causes (PKU, congenital hypothyroidism, lead, biotinidase deficiency) is critical — the basis of newborn screening programmes.
3. Early intervention (0–3 years): the period with the strongest evidence base (IDEA Part C in the USA; similar programmes in all developed countries). Improves social and cognitive outcomes.
4. Individualized education programme (IEP): academic and adaptive goals at school; AAIDD support-needs-based approach.
5. Behavioural interventions: Applied Behavior Analysis (ABA) and positive behaviour support (PBS) — effective in reducing challenging behaviours and developing new skills.
6. Family support: psychoeducation, respite care, peer support groups, social service coordination.
7. Treatment of comorbidity: epilepsy (standard antiepileptics), ADHD (stimulants; the Cortese 2018 network meta-analysis makes methylphenidate the first choice in children, but it excluded trials with comorbid disorders — the evidence base in IDD is separate and thinner), anxiety/depression (start SSRI at a low dose), aggression (behavioural intervention is first line; risperidone or aripiprazole only in severe cases — FDA approved for

irritability in ASD: risperidone ages 5–16, aripiprazole ages 6–17; no official approval for IDD, evidence base limited).

8. Medical monitoring: in Down syndrome — thyroid, heart, hearing, atlanto-axial instability (AAP Health Supervision for Children with Down Syndrome 2022).
9. Transition to adulthood: supported employment (IPS model), independent living options, coordination of social and medical services.

8.2 Treatment methods

1. Applied Behavior Analysis (ABA) — Shaping and reducing specific target behaviours based on a functional analysis of behaviour; the antecedent-behaviour-consequence (ABC) framework, discrete trial training (DTT), naturalistic behavioural interventions. Effect: moderate-to-high effect size in acquiring adaptive skills and reducing challenging behaviour, especially in cases comorbid with autism and IDD (Eldevik S. et al. J Clin Child Adolesc Psychol 2009 meta-analysis). Can be adapted to home, school, and community settings at all stages.
2. Early Intervention (0–3 years) — Multidisciplinary (developmental, physical, occupational, speech therapies + family training) home- or centre-based programmes. Evidence: the Carolina Abecedarian Project (Ramey C.T., Campbell F.A. — 30+ years of follow-up) improved IQ, academic, and employment outcomes in low-resource children. IDEA Part C.
3. Individualized Education Program (IEP) — A legal document that documents academic and adaptive goals, support measures, and assessment intervals. Mandatory in the USA under IDEA, with similar frameworks in other countries (United Kingdom — EHCP).
4. Positive Behaviour Support (PBS) — Functional analysis of challenging behaviours and replacement of behaviour (teaching an appropriate behaviour with the same function). Not punishment, but changing the environment and new skills. NICE NG11 (2015) makes it first line in managing challenging behaviour.
5. Vineland Adaptive Behavior Scales-3 (Vineland-3) — The “gold standard” assessment of adaptive behaviour — communication, daily living, social, and motor domains. Interview (parent/caregiver) and teacher forms.
6. WISC-V / Stanford-Binet 5 — Standardized intelligence tests; WISC-V — 6 –16 years, 5 main indices (verbal, visuo-spatial, abstract reasoning, working memory, processing speed); Stanford-Binet 5 — from 2 years to adulthood.
7. Supports Intensity Scale (SIS, AAIDD) — A scale that measures the intensity of support needs; SIS-A for adults, SIS-C for ages 5–16. For clinical planning and allocation of services.

8.3 Differences between sources

- AAIDD 12th ed. (2021): “supports model” — planning tailored to the individual's support needs rather than to a given severity level; intermittent, limited, extensive, pervasive support categories.
- AAP 2014/2020: screening intervals and treatment algorithms for comorbidity (autism, ADHD, conduct); a separate AAP 2022 protocol for Down syndrome.
- NICE NG54 (2016) — Mental health problems in people with learning disabilities: diagnosis and treatment of psychiatric comorbidity; behavioural intervention is first line; nonspecific use of psychotropic drugs is not recommended — even though this is seen as a popular “safe” practice, the burden of side effects and the risk of interactions are high.
- WHO mhGAP Intervention Guide v2.0: assessment and basic intervention at the primary care level in low- and middle-income countries; family training and community rehabilitation are emphasized.

9. Prognosis

Good prognostic factors

- Mild severity (~85% of all IDD); intellectual and adaptive functioning at a close level.
- Treatable aetiology (PKU, congenital hypothyroidism, lead) — early detection.
- Early intervention (<3 years).
- Stable and supportive family environment.
- Active treatment of comorbid psychiatric and medical conditions.
- Accessibility of community and school resources.

Poor prognostic factors

- Profound or severe severity.
- Genetic syndrome with a complex phenotype (epilepsy, cardiac anomalies, motor).
- Severe behavioural disorders, self-harm.
- Absence of family support and community resources.
- Unidentified and untreated comorbidity (epilepsy, ASD, ADHD).

Follow-up targets

- Annual reassessment of adaptive development.
- Medical monitoring — according to syndrome-specific protocols (e.g., AAP Down 2022).
- Screening for comorbid psychiatric conditions (ASD, ADHD, anxiety, depression).
- Functional analysis and intervention for behavioural disorders.
- Transition planning for supported employment and living options in adulthood.
- Offering the family respite care and peer support.

10. Myths and misconceptions

10.1 Aetiology myths

Myth 1: "Intellectual disability is the parent's fault / punishment"

Why it is widespread: a cultural and religious-moral explanatory framework; the image of a "child atoning for sin"; a mechanism of blaming the parent (especially the mother) in poorly educated communities.

Biological logic and evidence: The aetiology of IDD is broad — genetic (Down, Fragile X), prenatal infections, fetal alcohol spectrum (the only one completely preventable through parental intervention), perinatal asphyxia, postnatal trauma or meningitis. The position of AAIDD, WHO, and APA — IDD is a biomedical and/or developmental aetiological disorder, not a matter of moral judgement.

Real clinical step: the search for aetiology (genetic counselling, syndromic and metabolic screening) — turns blame into a precise risk calculation for a future pregnancy.

Myth 2: "Vaccines (especially MMR) cause IDD"

Why it is widespread: Wakefield A.J. Lancet 1998 — a study later retracted for fraud, which attempted to link autism with MMR (not IDD, but in popular belief both conditions are conflated).

Evidence: Taylor L.E., Swerdfeger A.L. et al. Vaccine 2014 meta-analysis n>1.2 million children — there is no association between vaccines and autism or IDD. The position of the CDC, WHO, and NICE is the same. Down syndrome arises from nondisjunction of chromosome 21 in meiosis (most often maternal meiosis I) — mechanistically it cannot be related to vaccines.

Myth 3: "'Stress' or 'bad thoughts' during pregnancy cause IDD"

Evidence: psychological stress may be a modest risk factor for neurodevelopment in the prenatal period, but it is not identified as a direct cause of IDD. Established prenatal risks — alcohol (FAS — the most preventable cause), valproate, infections (TORCH), nutritional deficiency (iodine, folate). Reducing stress is beneficial for general health, but it is defined medical protective measures (vaccination, abstaining from alcohol, perinatal care) that are effective, not "correct thoughts".

10.2 Harmful "treatment" methods

Myth 4: "Chelation therapy cures IDD by removing heavy metals"

Why it is widespread: the expansion of the "heavy metals" concept in mass media; the marketing of alternative medicine centres.

Evidence and risk: AAP 2010 and FDA warning: in 2005 a 5-year-old child during a chelation procedure died (cardiac arrest due to calcium deficiency). Chelation is applied only for confirmed heavy metal poisoning (high lead, mercury) on medical indication. There is no evidence of effect for IDD.

Myth 5: "Stem cell injections (in unofficial clinics) cure IDD"

Evidence and risk: The position of the ISSCR (International Society for Stem Cell Research) and the FDA: for IDD (and neurodevelopmental disorders in general) there is no approved stem cell treatment. Illegal "clinics" have caused serious adverse effects accompanied by infections, brain tumours, and deaths (Berkowitz A.L. et al. NEJM 2016).

Myth 6: "The hyperbaric oxygen chamber (HBOT) is a treatment for IDD and autism"

Evidence: Cochrane (Xiong T. et al. 2016) — there is no evidence of effect for autism; there is also no evidence base for IDD. Risks: pneumothorax, ear barotrauma, fire (oxygen + static electricity).

Myth 7: "Miracle Mineral Solution (MMS) / chlorine dioxide"

Evidence: FDA 2019, EMA warning — caustic (chlorine dioxide), liver and kidney damage, and deaths have been recorded. All clinical use is prohibited.

10.3 Ineffective methods or those that delay essential support

Myth 8: "Mega-doses of vitamin B complex / niacin restore cognitive functioning in IDD"

Evidence: the "orthomolecular" psychiatry approach (Hoffer A., Osmond H. 1950–70) has not shown efficacy in IDD or autism. Pfeiffer S.I. et al. (J Autism Dev Disord 1995) reviewed 12 B6/magnesium studies and, due to methodological flaws, could draw no firm conclusion (see also Nye C., Brice A. Cochrane 2005 — the same finding); mega-doses of niacin are a separate issue — they cause liver damage and hyperuricaemia. AAP and AAIDD recommend neither.

Myth 9: "Patterning" (Doman-Delacato method)

Why it is widespread: it gained mass popularity in the 1960s–80s; the claim of "reprogramming the brain".

Evidence: AAP 1968 and 2010 separate Statements: no evidence of effect; excessive burden and emotional strain on the family; delays children's access to other evidence-based interventions. AAP — strongly does not recommend it.

Myth 10: "Auditory Integration Training (AIT)"

Evidence: ASHA (American Speech-Language-Hearing Association) Position Statement 2004 — there is no evidence of effect of AIT in IDD and autism; it is considered an experimental intervention.

Myth 11: "IDD will resolve on its own if the child is stimulated"

Reality: early intervention improves adaptation, but IDD is not a temporary condition but a neurodevelopmental disorder — when the aetiology is genetic or structural, intellectual ability does not change significantly. Some specific causes (PKU, congenital hypothyroidism) can be completely prevented through early detection — this is not "resolving", but timely prevention.

Real clinical step: the AAIDD support model — the goal is not elimination of the disorder, but ensuring a meaningful and independent life; supporting the family with realistic expectations.

Myth 12: "IDD will improve if the parent applies a special diet (gluten-free, casein-free)"

Evidence: Hyman S.L. et al. (AAP 2020 Clinical Report on ASD): in children without gastrointestinal problems, the effect of special diets on cognitive functioning

has not been proven. If coeliac disease is proven — a gluten-free diet, but not for IDD.

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	6A01 DEVELOPMENTAL SPEECH OR LANGUAGE DISORDERS
ICD-11 6A01	**Developmental speech or language disorders**
ICD-10 F80	Specific developmental disorders of speech and language
DSM-5-TR F80.2	Language Disorder

1. Definition and nosology

Developmental speech or language disorders are a group of neurodevelopmental disorders manifesting as persistent and significant difficulties in the acquisition of language and/or speech that are not fully explained by hearing impairment, an intellectual disorder or another neurological cause. In ICD-11, under the 6A01 code, four subtypes are coded:

- 6A01.0 — Developmental speech sound disorder — a deficit in the articulation and phonological formation of speech sounds;
- 6A01.1 — Developmental speech fluency disorder — stuttering;
- 6A01.2 — Developmental language disorder (DLD) — impairment in the development of receptive and/or expressive language (sub-specifiers: 6A01.20 receptive-expressive, 6A01.21 mainly expressive, 6A01.22 mainly pragmatic);
- 6A01.3 — Other specified.

Parallel terminology in DSM-5-TR: Language Disorder (F80.2), Speech Sound Disorder (F80.0), Childhood-Onset Fluency Disorder/Stuttering (F80.81), Social (Pragmatic) Communication Disorder (F80.82).

2. History

- Specific Language Impairment (SLI) — the term used from the 1980s until the 2010s; the word “specific” came to be criticised over time, because language disorder overlaps with numerous cognitive and behavioural domains.
- CATALISE consensus (Bishop D.V.M. et al. PLoS One 2016; J Child Psychol Psychiatry 2017) — a Delphi consensus of 59 international experts: the term “SLI” was replaced with “Developmental Language Disorder” (DLD). The IQ cut-off (≥85) for the diagnosis was abolished, with functional impairment adopted as the primary criterion.
- DSM-5 (2013) and ICD-11 (2019) — the term DLD was adopted; social (pragmatic) communication disorder was recognised as a category separate from autism spectrum disorder (new in DSM-5; as subtype 6A01.22 in ICD-11).
- Stuttering — the biological-genetic conception has been dominant since the 2000s (GNPTAB, GNPTG, NAGPA mutations — Kang C. et al. NEJM 2010).

3. Epidemiology

- DLD (6A01.2): prevalence at 5–6 years of age 7.58% (Norbury C.F. et al. J Child Psychol Psychiatry 2016, population-based study, n=7267, England).
- Speech sound disorder (6A01.0): ~3.8% at 6 years of age (Shriberg L.D. et al. J Speech Lang Hear Res 1999); higher at preschool age.
- Stuttering (6A01.1): cumulative incidence in childhood 5–8% (roughly one in every 20 children stutters at some point); persisting prevalence in adulthood ~1% (Yairi E., Ambrose N. J Fluency Disord 2013).
- Sex: 2–4 times higher in boys across all subtypes.
- Comorbidity: learning disorders 40–60%, ADHD 20–30%, autism spectrum 5–10%, increased risk of anxiety and conduct disorders.

4. Aetiology and pathogenesis

4.1 Genetic factors

- DLD heritability 50–75% in twin studies (Bishop D.V.M. Trends Cogn Sci 2014).
- FOXP2 mutations — rare severe speech and language disorder in the KE family (Lai C.S. et al. Nature 2001); polygenic nature in most cases.

- Stuttering: ~70% heritability (Felsenfeld S. et al. Behav Genet 2000 twin study); mutations in lysosomal pathway genes (GNPTAB, GNPTG, NAGPA) are detected rarely (~9%).
- CNV and GWAS studies — a polygenic risk architecture is also characteristic of DLD.

4.2 Neurobiology

- Atypical patterns in the development of the left perisylvian region (around Broca and Wernicke areas), the arcuate fasciculus, and the basal ganglia.
- In stuttering, dopaminergic dysfunction in the basal ganglia–supplementary motor area connection (Maguire G.A. et al. Curr Med Res Opin 2020 review).
- Early brain asymmetry — in DLD, weakening of the typical dominance of left-hemisphere language areas.

4.3 Risk factors

- Family history of language/speech disorder (the strongest risk factor).
- Preterm birth, low birth weight.
- Chronic otitis media in early childhood — hearing fluctuation may affect language acquisition.
- Low socioeconomic status — cited as a risk factor but not an aetiology; a deprived environment affects the expression of language development.
- Bilingualism — is not a risk factor (expanded topic §10).

4.4 Preventable and treatable causes

- Hearing impairment — the newborn hearing screening programme (UNHS) and timely treatment of otitis media are critical.
- Congenital cleft lip and palate — surgical correction + speech therapy.
- Hypothyroidism — newborn screening.

5. Clinical features

6A01.0 Speech sound disorder

- Age-inappropriate sound omission, substitution, distortion (for example, “r” → “l”, “s” → “t”).
- Unintelligibility — unfamiliar people have difficulty understanding the speech.

- Up to 4 years of age it is considered “mild” within the developmental framework; those persisting beyond 4 years should be reviewed diagnostically.

6A01.1 Speech fluency disorder (stuttering)

- Repetition of sounds and syllables (f-f-f-fish), prolongation (ssshello), blocks (a pause without initiating the sound).
- Secondary behaviours — gaze aversion, head shaking, avoidance.
- Onset typically between 2–5 years of age; ~70–80% of children who stutter recover spontaneously or with intervention within 5 years (Yairi E., Ambrose N. 2013).
- Social and emotional burden — social anxiety, school adjustment problems.

6A01.2 Developmental language disorder (DLD)

- Expressive component: limited vocabulary, short sentences, errors in grammatical morphology (especially tense and case endings), word-finding difficulty.
- Receptive component: difficulty understanding complex sentences, problems responding to behavioural instructions, delayed acquisition of multiple meanings and abstract language.
- Pragmatic component (6A01.22): deficit in the use of social language (turn-taking, context, indirect speech); unlike autism, social interest and non-verbal communication are relatively preserved.

Developmental trajectory

“Late talker” is a category at 2 years of age — 50–70% catch up by 4 years of age; however, the persisting group represents a high risk for DLD. Early identification of this group is based on the features of family history, receptive deficit, and low use of non-verbal gestures.

6. Diagnosis

6.1 Unified diagnostic criteria (DSM-5-TR · ICD-11 · CATALISE 2017 consensus points)

A. Persistent deficit in language or speech acquisition — performance below the age-appropriate normal level in one or more domains (vocabulary, syntax, phonology, fluency, pragmatics).

B. Functional impairment — significant limitation in one or more of academic achievement, social communication, occupational functioning, or daily life (CATALISE: functional impairment is essential for the diagnosis, an IQ cut-off is not).

C. Onset during the developmental period.

D. Exclusions — not fully explained by hearing impairment, a neurological condition, intellectual disability, autism spectrum disorder, or another medical/psychiatric condition. However, when there is comorbid coexistence both diagnoses are made.

6.2 Differences between sources

- CATALISE 2017 (Bishop D.V.M. et al.): the term “specific” is abolished; the IQ cut-off of 85 is abolished (DLD can also be diagnosed against a background of intellectual disability); under the “language disorder” umbrella, co-morbid neurodevelopmental conditions and risk factors are indicated as qualifiers.
- ICD-11 (6A01): separation of subtypes (sound, fluency, language); 3 sub-specifications for language disorder (receptive-expressive, predominantly expressive, predominantly pragmatic).
- DSM-5-TR: Social Communication Disorder as separate category (F80.2); typical onset age for stuttering <6 years.
- ASHA Practice Portal: for diagnosis, standardized assessment by a certified speech-language pathologist (SLP), together with a parent interview and naturalistic observation, is required.
- RCSLT (Royal College of Speech and Language Therapists): the same framework in the United Kingdom; multidisciplinary assessment (hearing + speech-language + development).

6.3 Diagnostic algorithm

1. Parent and teacher interview (developmental history, language milestones).
2. Hearing test (audiometry — mandatory in all cases).
3. Standardized speech and language assessment (CELF-5, PLS-5, GFTA-3, DELV).
4. Naturalistic speech sample (in a play or conversation setting).
5. Non-verbal IQ measure (for the exclusion of ASD and intellectual disability).
6. Autism spectrum screening (M-CHAT-R/F < 30 months; SCQ preschool).

7. Social-emotional and behavioural screening.
8. Medical examination — neurological, cleft lip and palate examination, motor examination.

6.4 Differential diagnosis

Condition	Distinguishing features
Intellectual disability (6A00)	The general cognitive deficit extends to all domains beyond language disorder. May be comorbid.
Autism spectrum disorder (6A02)	Triad of social interaction and restricted-repetitive behaviour; pragmatic language is also impaired, but social motivation is the core problem.
Hearing impairment	Audiometry abnormal; receptive deficit dominant.
Selective mutism (6B06)	Failure to speak in certain social situations (school), normal speech in other contexts (home).
Transient effect of a multilingual environment	Age-appropriate acquisition of the local-environment language is ongoing; speech develops dynamically.
“Code-mixing” in bilingualism	Typical and developmentally appropriate, not a disorder.
Childhood apraxia of speech (CAS)	Motor-planning deficit, variable sound production, sequencing problem.

7. Examination and assessment

7.1 Standardized tests

- Language: CELF-5 (Clinical Evaluation of Language Fundamentals, 5–21 years), PLS-5 (Preschool Language Scales, <7 years), CASL-2, Reynell Developmental Language Scales.
- Vocabulary: PPVT-5 (Peabody Picture Vocabulary Test), EVT-3, MacArthur-Bates CDI (up to 24 months).
- Speech sound: GFTA-3 (Goldman-Fristoe Test of Articulation), DEAP.

- Pragmatic language: CCC-2 (Children's Communication Checklist), Test of Pragmatic Language (TOPL-2).
- Non-verbal IQ: Leiter-3, RIAS-2 (independent of the verbal component).

7.2 Laboratory investigations

Not mandatory; based on specific suspicion:

- Hearing test — mandatory in all cases (audiometry, tympanometry, otoacoustic emissions).
- Fragile X (FMR1) — in cases of severe DLD + comorbid intellectual disability.
- Thyroid function (TSH, T4) — if congenital hypothyroidism has not been detected.
- Lead level — if risk factors are present.

7.3 Instrumental investigations

- Brain MRI — when there are focal neurological signs, regression, macro/microcephaly, or seizures. Not indicated in routine DLD.
- EEG — when a seizure is suspected, for the exclusion of Landau-Kleffner syndrome.

8. Treatment

8.1 General principles (ASHA · RCSLT · CATALISE · NICE consensus)

1. Individual speech-language therapy by a certified speech-language pathologist (SLP) is the first-line intervention for all disorders.
2. Early intervention — intervention between 2–4 years of age shows a greater effect; a “wait and see” approach is harmful when certain risk factors (family history, receptive deficit) are present.
3. Parent-coached interventions at a young age — the Hanen “It Takes Two to Talk” programme has shown effect in RCTs.
4. Intensity and duration — 1–3 sessions per week, 6–18 months, individualized according to behavioural and academic components.
5. Multidisciplinary approach — SLP + teacher + paediatrician + audiologist, occupational therapist, psychologist as needed.
6. School support — individual education plan (IEP / EHCP), classroom modifications, one-to-one support as needed.
7. Bilingualism is preserved — support maintaining the child's two languages; “abandoning” one language is not recommended (ASHA Position 2004).

8. Pharmacotherapy is not approved for the speech-language disorder itself. Separate intervention for comorbid ADHD, anxiety, or autism.

8.2 Intervention in speech sound disorder (6A01.0)

- Articulation therapy — a phoneme-by-phoneme approach (van Riper); minimal pair training (comparison of contrastive pairs of similar sounds).
- Reduction of phonological processes — systematic process-based therapy (Bowen C., Cupples L. 1999).
- In complex and severe cases — PROMPT, a multisensory approach.

8.3 Intervention in stuttering (6A01.1)

- The Lidcombe Program (Onslow M., Australian Stuttering Research Centre) — first-line at 3–6 years of age; a parent-coached, home-administered operant programme based on positive reinforcement. Evidence: Jones M. et al. BMJ 2005 RCT, n=54 — at 9 months the proportion of syllables stuttered was 1.5% (SD 1.4) in the treatment arm and 3.9% (SD 3.5) in the control arm; difference 2.3 percentage points (95% CI 0.8–3.9; p=0.003). Cochrane 2021 rates the evidence as very low certainty.
- Demands and Capacities Model (DCM) — Starkweather C.W. — adaptation of communicative demands to the child's current capacity.
- Older children/adults — Stuttering Modification (Van Riper), Fluency Shaping (Webster); family and group support.
- Pharmacotherapy: There is no drug officially approved by the FDA for stuttering. Off-label dopamine antagonists (risperidone, olanzapine) and SSRIs (paroxetine) have been studied in selected adult cases, but the effect is modest and the side-effect profile is significant (Maguire G.A. et al. 2020 review).

8.4 Intervention in developmental language disorder (6A01.2)

- Hanen “It Takes Two to Talk” — a parent-coached group programme at 2–4 years of age; following the child's lead, expanding dialogue. Evidence: Roberts M.Y., Kaiser A.P. Am J Speech Lang Pathol 2011 meta-analysis — a moderate effect on expressive and receptive language measures.
- Naturalistic language intervention (enhanced milieu teaching, focused stimulation) — presenting language models in the context of the child's interests.
- Structured morpho-syntactic therapy — targeted training of grammatical forms (for example, past tense, plural).

- Pragmatic language interventions (6A01.22) — social Stories (Carol Gray), video-modelling, group training.
- Academic support at school age — a Structured Literacy approach due to the high comorbidity of reading difficulties; assessment for learning disorder.

8.5 Treatment methods

1. Hanen “It Takes Two to Talk” — a parent training programme developed by the Hanen Centre in Canada for late talkers and children at risk of DLD aged 2–4 years. 8 group sessions + 3 individual home visits; the principles of “OWL” (Observe, Wait, Listen) and following the child's lead. Evidence: Roberts M.Y., Kaiser A.P. Am J Speech Lang Pathol 2011 meta-analysis. hanen.org.
2. The Lidcombe Program — a behaviourally based, parent-coached programme for preschool-age stuttering developed by Onslow M. in Australia. The parent reinforces the child's fluent speech in a daily 10–15 min structured play session. Evidence: Jones M. et al. BMJ 2005 RCT n=54, 9 months; Onslow M. et al. reviews. lidcombeprogram.org.
3. Enhanced Milieu Teaching (EMT) — Naturalistic language intervention — the presentation and expansion of consistent language models in the context of the child's play and daily routine. Evidence: Kaiser A.P., Roberts M.Y. Am J Speech Lang Pathol 2013.
4. PROMPT method (Prompts for Restructuring Oral Muscular Phonetic Targets) — a tactile-kinaesthetic speech motor intervention — indicating the correct place of articulation with a physical sensory cue to the child's oral muscles. Used in complex speech sound disorder and childhood apraxia of speech. The evidence base is developing; Rogers S.J. et al. 2006 (against a background of autism) and other pilot studies.
5. Social Stories — Carol Gray — training appropriate behaviour in a social context in a short, individualized story format. In building pragmatic skills in pragmatic language disorder and the autism spectrum. carolgraysocialstories.com.
6. Clinical Evaluation of Language Fundamentals (CELF-5) — a standardized comprehensive assessment of language for ages 5–21 — receptive, expressive, language memory, word-sentence structure.
7. Preschool Language Scales (PLS-5) — language assessment from birth to 7 years — auditory comprehension and expressive communication scales.
8. Children's Communication Checklist (CCC-2) — parent/teacher assessment of pragmatic language and general communicative function; assists in the sub-typing of DLD.

8.6 Differences between sources

- ASHA Practice Portal: an evidence-based practice framework — the triad of clinician judgement + evidence + family values; certification and continuing education requirements.
- RCSLT (2017 Position Paper on DLD): proposes early identification and service accessibility; the "Bercow" report (2008, 2018) shaped the service structure in England.
- NICE NG (a separate full guideline for child speech and language is still under development; existing guidance is directed at the coordination of education and health services).
- WHO mhGAP IG v2.0: assessment at the primary care level in low- and middle-income countries; parent training and community-based rehabilitation are emphasized.

9. Prognosis

Good prognostic factors

- Mild-to-moderate severity, expressive component only.
- Early identification (<3 years) and intervention.
- High non-verbal IQ.
- Absence of comorbid disorders (ASD, intellectual disability).
- Parental involvement and a language-rich environment at home.

Poor prognostic factors

- Severity of the receptive component (poorer prognosis than expressive only).
- Comorbid intellectual disability or ASD.
- Late identification (>5 years).
- Persistent DLD in the family history.
- Low socioeconomic support, limited service accessibility.

Follow-up targets

- High comorbidity of reading and writing difficulties in children with DLD (40–60%) — phonological awareness training at preschool age; then Structured Literacy.

- Social-emotional health — increased risk of anxiety and depression in adolescents with DLD (Conti-Ramsden G., Botting N. J Child Psychol Psychiatry 2008).
- Screening for conduct disorder and ADHD.
- Vocational and academic support planning in adolescence.
- After stuttering has reached remission, monitoring for relapse (during stressful periods).

10. Myths and misconceptions

10.1 Aetiology myths

Myth 1: “Bilingualism (speaking two languages) causes a child to develop a language disorder”

Why it is widespread: it is instilled by families and sometimes paediatricians as the belief that “mixing languages” “confuses the child”; in late-talking bilingual children the vocabulary in each language separately may appear small compared with monolingual children (but total conceptual vocabulary is equivalent).

Clinical and biological rationale: The human brain is adapted from birth to develop in several languages; bilingualism has a superior effect on executive function and cognitive flexibility (Bialystok E. et al. Trends Cogn Sci 2012 review). Bilingual children with DLD show impairment in the same pattern in both languages — this is related not to the number of languages but to the neurodevelopmental disorder.

Evidence: Paradis J., Genesee F., Crago M.B. (2011) — in bilingual children, language acquisition develops in parallel with monolingual patterns; DLD arises not from bilingualism but on a neurodevelopmental basis.

Real clinical step: a DLD diagnosis in a bilingual child can be made only when impairment in both languages is documented. Cutting off the family's mother tongue is not recommended — social-emotional harm and disruption of the family bond.

Myth 2: “Stuttering arises from the parent's improper behaviour (impatience, harshness)”

Why it is widespread: Wendell Johnson's “Diagnosogenic Theory” in the 1940s–1950s — the claim that stuttering arises as a result of the parent drawing attention to the child's normal disfluency; subsequently refuted, but it has remained in popular belief.

Biological logic and evidence: Stuttering is genetically and neurobiologically based — heritability ~70% (Felsenfeld 2000); GNPTAB and related lysosomal pathway mutations in rare forms (Kang C. NEJM 2010); fMRI studies show functional impairment in the basal ganglia-supplementary motor area circuit (Maguire 2020). The parent's reaction may affect the persistence of stuttering (increase in a stress context), but is not the cause of the onset of stuttering.

Real clinical step: support for the family — creating a calm, listening environment; an evidence-based parent-coached intervention such as the Lidcombe Program.

Myth 3: “If a child does not talk, it is because they are ‘lazy’ or 'too young, let's wait'”

Evidence: The “wait and see” approach is appropriate only for a late talker WITHOUT risk factors — 70–80% of this group catch up by 4 years of age. However, when risk factors are present (family history, receptive deficit, low use of non-verbal gestures, hearing suspicion), waiting loses the critical intervention window — Roos E.M., Weismer S.E. Top Lang Disord 2008.

Real clinical step: screening at 18–24 months (CDI, ASQ); SLP assessment if risk factors are present.

10.2 Harmful methods or those that delay the primary intervention

Myth 4: “Tongue-tie (ankyloglossia) surgery will solve the speech problem”

Why it is widespread: the commercial popularity of “frenotomy” and “frenuloplasty” procedures; the existence of some indications in breastfeeding problems.

Evidence: Only severe anatomical restriction (Coryllos III–IV) with a breastfeeding problem makes frenotomy effective (AAP, ABM 2020 protocols). There is no systematic evidence for speech sound disorder — Webb A.N. et al. Int J Pediatr Otorhinolaryngol 2013 review. Erroneous surgery — bleeding, infection, formation of a double frenulum, difficulty with normal breastfeeding.

Real clinical step: first-line for speech sound disorder — SLP assessment and articulation therapy, not frenotomy.

Myth 5: “Auditory Integration Training (AIT) — the Berard or Tomatis method”

Why it is widespread: alternative-clinic marketing; the claim of “reprogramming the auditory system”.

Evidence: ASHA Position Statement 2004 — no evidence of effect in auditory-language disorders or autism; experimental status. AAP 1998 Statement — does not recommend AIT.

Myth 6: "Complete cure of stuttering with hypnotherapy or ear devices (DAF — delayed auditory feedback)"

Evidence: Hypnotherapy — no evidence of effect for stuttering (Bothe A.K. et al. Am J Speech Lang Pathol 2006 review). DAF devices (SpeechEasy and similar) may provide a short-term increase in fluency in some adults, but the long-term effect is limited and superiority over behavioural therapy has not been confirmed (Lincoln M., Packman A. J Fluency Disord 2007).

10.3 Ineffective or scientifically unfounded methods

Myth 7: "Non-Speech Oral Motor Exercises (NSOME) — tongue, lip, and jaw exercises correct speech sounds"

Why it is widespread: an intuitive basis — "if a muscle is exercised, it works better"; still applied by some therapists.

Evidence: ASHA 2009 and update — there is no evidence of effect of non-speech oral motor exercises in speech sound disorder; speech and non-speech oral movements are separate neuromotor programmes (McCauley R.J., Strand E., Lof G.L. Am J Speech Lang Pathol 2009 systematic review). The use of NSOME diverts therapy time from evidence-based speech-based intervention.

Real clinical step: for speech sound disorder — direct articulation therapy (phoneme-by-phoneme or phonological processes); NSOME is avoided.

Myth 8: "Brain Gym / Edu-K — brain coordination exercises cure language disorder"

Evidence: Reviews (Hyatt K.J. Remedial Spec Educ 2007) — "Brain Gym" claims do not conform to neurobiology; no effect on learning outcomes. Criticized as a commercial programme.

Myth 9: "Hyperbaric oxygen chamber (HBOT) and stem cell injections cure language disorder"

Evidence: Cochrane (Xiong T. 2016 — autism; a similar position for DLD) — no evidence of effect; risks of pneumothorax and barotrauma with HBOT; deaths in unregulated stem cell clinics (Berkowitz NEJM 2016).

Myth 10: “A special diet (gluten-free, casein-free) cures language disorder”

Evidence: ASHA, AAP, AACAP — the effect of special diets on speech and language disorders has not been proven. If coeliac disease or a confirmed food allergy is present, a diet is a medical indication, but it has no effect on language-speech development.

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ICD-11 **6A02**	**6A02 AUTISM SPECTRUM DISORDER (ASD)** **Autism spectrum disorder**
ICD-10 F84.0	Childhood autism
DSM-5-TR F84.0	Autism Spectrum Disorder

1. Definition and nosology

Autism spectrum disorder (ASD; ICD-11: 6A02; DSM-5-TR: F84.0 Autism Spectrum Disorder) is a neurodevelopmental disorder with onset in the early developmental period, accompanied by persistent deficits in two core domains:

1. Persistent deficits in social communication and social interaction — reciprocal social-emotional exchange, non-verbal communicative behaviour, and the development and maintenance of relationships.
2. Restricted, repetitive patterns of behaviour, interests, or activities — stereotyped motor movements or speech, adherence to fixed rituals and routines, restricted high-intensity interests, atypical reactivity to sensory stimuli.

In ICD-11, the diagnosis is specified with qualifiers: with/without intellectual developmental disorder; impairment of functional language (absent / mild-to-moderate / severe); overall persistent clinical course (stable, worsening, improving).

2. History

- Kanner L. (1943) — “early infantile autism” — observed social isolation, resistance to change, and language disturbances in 11 children.
- Asperger H. (1944) — “autistic psychopathy” — described deficits in social interaction with normal intelligence and language. (Asperger's history of collaboration with the Nazi regime has come to light in recent decades — Czech H. Mol Autism 2018.)
- Wing L., Gould J. (1979) — the “autistic spectrum” concept; the “triad of impairments” — social interaction, communication, imagination/behaviour.
- DSM-III (1980) → DSM-IV (1994): the Pervasive Developmental Disorders category — Autistic Disorder, Asperger's Disorder, Childhood Disintegrative Disorder, Rett's Disorder, and PDD-NOS as separate diagnoses.
- DSM-5 (2013) and ICD-11 (2019): all subtypes were merged into the single category of “Autism Spectrum Disorder”. The rationale: the evidence base for a reliable and valid separation of the subtypes was weak. In DSM-5, severity is divided into 3 levels (1 — requiring support; 2 — requiring substantial support; 3 — requiring very substantial support). In ICD-11, severity is not a separate qualifier — sub-specification is based on intellectual and language impairment.
- Childhood disintegrative disorder is preserved in ICD-11 with a separate code (atypical course qualifier).

3. Epidemiology

- Prevalence: CDC ADDM Network 2023 report (8-year-old children, 2020 data): 1/36 (2.8%) — increase recorded in recent decades (1/150 — 2000;

1/68 — 2012). Most of the increase is explained by expansion of diagnostic criteria, increased awareness, and application of screening.

- Sex: Males ~4:1 — but under-detection problem in females documented (female phenotype — social "camouflage", fewer overt repetitive behaviors; Lai M.C., Baron-Cohen S. Lancet Psychiatry 2015).
- Comorbidity with intellectual disability: ~30–40% (CDC ADDM); approximately 30–35% IQ <70.
- Other comorbidities: ADHD ~30–50%, anxiety ~40%, depression in adolescence ~25%, epilepsy ~10–30% (increases with age), sleep disorders 50–80%, feeding problems, GI complaints.
- Mortality: Twice higher compared to the population — main causes: epilepsy, accidents (drowning, wandering incidents), suicide (in high-functioning ASD).

4. Aetiology and pathogenesis

4.1 Genetic factors (50–80% heritability)

- Heritability: Sandin S. et al. JAMA 2017 Swedish sibling and twin study n>2 mln: heritability ~80%; environmental factors account for ~20%.
- Association with genetic syndromes: Fragile X (~2–5% of ASD cases), Rett syndrome (almost always MECP2 mutation in females), tuberous sclerosis (~1–4%), 22q11.2 deletion, 16p11.2, Phelan-McDermid (22q13).
- De novo CNV and SNV mutations — identified in ~25% of cases; synaptic genes (SHANK3, NRXN1, NLGN3/4, CHD8, SCN2A) noted (Satterstrom F.K. et al. Cell 2020 — 102 risk genes).
- Polygenic risk architecture — numerically hundreds of small-effect variants; no single 'autism gene'.
- Father's age > 40 — associated with new mutation risk.

4.2 Prenatal and perinatal risk factors

- Maternal valproate use (during pregnancy) — increases the risk of autism spectrum disorder ~2.9-fold (95% CI 1.7–4.9) and of childhood autism ~5.2-fold (95% CI 2.7–10.0); absolute risk 4.42% and 2.50% (1.53% and 0.48% in the population) — Christensen J. et al. JAMA 2013; FDA and EMA warning.
- Preterm birth — at 22–27 weeks increases the risk ~3.7–4.2-fold (Crump C. et al. Pediatrics 2021), at 22–31 weeks ~2.3-fold (Persson M. et al. PLOS Med 2020); pooled ASD prevalence among preterm children is 7% (95% CI 4–9) at a median gestation of 28 weeks (Agrawal S. et al. Pediatrics 2018).

- Low birth weight, perinatal hypoxia.
- Parental diabetes, severe obesity, gestational hypertension — indicated as risk factors.
- Maternal viral infections during pregnancy (rubella, cytomegalovirus) — high risk.

4.3 Neurobiological mechanisms

- Atypical pattern of early brain growth — acceleration at 2–4 years, then normalization (Courchesne E. et al. JAMA 2003).
- Dysregulation in synaptic formation and pruning processes — excitatory/inhibitory imbalance.
- Atypical pattern of interregional functional connectivity in the brain — local hyperconnectivity, long-range hypoconnectivity (Just M.A. et al. Brain 2004; later mixed evidence).
- Social cognition network (medial prefrontal, temporal-parietal junction, amygdala) and mirror neuron system show atypical activation patterns.

4.4 Environmental risk factors

- Air pollution (PM2.5, NO2) during pregnancy and early childhood — moderately increases risk (Volk H.E. et al. JAMA Psychiatry 2013).
- Maternal folate deficiency (during the periconceptional period) — risk reduced with folate supplements (Surén P. et al. JAMA 2013).

5. Clinical features

5.1 Social communication and interaction deficits

- Reciprocal social-emotional exchange deficit — atypical social proximity, reduced reciprocal conversation, restricted shared interests.
- Non-verbal communicative behavior deficits – atypical use of eye contact, reduced coordination of gestures and facial expressions.
- Difficulty forming and maintaining relationships — paucity of age-appropriate friendships, lack of imaginative role play.

5.2 Restricted, repetitive behavior patterns

- Stereotyped or repetitive motor movements, object use, or speech (echolalia, idiosyncratic phrases).
- Insistence on sameness, inflexible adherence to routines, or ritualized patterns of behavior (clothing, food, route).

- High intensity or focused limited interests (complex knowledge of a specific topic — “intense passion”).
- Atypical reactivity to sensory input — hyperreactivity (sound, touch), hyporeactivity (pain), or atypical sensory interests (examining surfaces, visual stimulation).

5.3 Developmental trajectory

Early signs — reduced social attention, lack of response to name calling, reciprocal smiling, non-verbal communication (12–18 months). In some children developmental regression (loss of acquired language and social skills over 12–24 months) is noted — ~30% of cases (CDC).

In adulthood — social “camouflage” in females and high-functioning individuals (Hull L. et al. J Autism Dev Disord 2017) — leads to exhaustion and secondary psychiatric comorbidity (anxiety, depression) by concealing difficulties.

6. Diagnosis

6.1 Unified diagnostic criteria (DSM-5-TR · ICD-11 · NICE CG170 consensus points)

A. Persistent deficits in social communication and social interaction — all of the following three areas:

1. Social-emotional reciprocal exchange.
2. Nonverbal communicative behaviour used in social interaction;
3. Development, maintenance, and understanding of relationships.

B. Restricted, repetitive patterns of behavior, interests, or activities — at least two of the following four:

1. Stereotyped or repetitive motor movements, object use, speech;
2. Insistence on sameness, adherence to routine, ritualized behavior or speech;
3. High intensity, focused limited interests;
4. Hyper- or hyporeactivity to sensory input, or atypical sensory interests.

C. Symptoms must be present in early developmental period (may not fully manifest until social demands increase).

D. Functional impairment — in social, academic, or other important areas.

E. Exclusions — not fully explained by intellectual developmental disorder or global developmental delay (comorbidity possible).

6.2 Differences between sources

- DSM-5-TR (F84.0): Three severity levels (1-3) are separately assigned for both domains — based on the intensity of support need. Qualifiers: intellectual disability, language impairment, known medical/genetic condition, another neurodevelopmental disorder, with catatonia.
- ICD-11 (6A02): Subspecifications — with intellectual developmental disorder (6A02.0/.1)/without (6A02.2/.3); degree of functional language impairment (absent / mild-moderate / severe). The 1–3 severity system in DSM is not officially adopted.
- NICE CG170 (Autism spectrum disorder in under 19s: support and management, 2013): the diagnosis should be made by a multidisciplinary team (pediatrician/psychiatrist + speech-language pathologist + clinical psychologist); ADOS-2 and ADI-R considered diagnostic gold standard.
- AAP 2020 (Hyman S.L., Levy S.E., Myers S.M. Pediatrics 2020;145(1)): At 18 and 24 months universal autism screening Recommended (M-CHAT-R/F); positive screening → specialist evaluation.
- AACAP Practice Parameter (Volkmar F. et al. J Am Acad Child Adolesc Psychiatry 2014): diagnostic algorithm, comorbidity assessment, treatment recommendations.

6.3 Diagnostic algorithm

1. Parent and caregiver interview — developmental history, early social and communicative signs.
2. Screening: 18 and 24 months (M-CHAT-R/F); follow-up questionnaire if positive.
3. Specialist assessment — pediatrician / developmental pediatrician / psychiatrist / clinical psychologist.
4. Gold standard diagnostic tools: ADOS-2 (Autism Diagnostic Observation Schedule) — structured play-based observation; ADI-R (Autism Diagnostic Interview, Revised) — parent interview.
5. Intellectual assessment (WISC-V, Stanford-Binet 5, non-verbal options — Leiter-3, Mullen).
6. Speech-language and adaptive behavior assessment (Vineland-3).
7. Hearing and vision tests.
8. Medical and genetic assessment — chromosomal microarray (CMA), Fragile X (FMR1), selective trio exome; EEG when seizure suspicion exists; MRI in clinical suspicion (microcephaly, regression).
9. Comorbidity screening: ADHD, anxiety, depression, OCD, sleep, eating, GI.

6.4 Differential diagnosis

Condition	Distinguishing features
Intellectual developmental disorder (6A00)	General cognitive deficit; social reciprocity corresponds to intellectual level. Can be comorbid.
Developmental language disorder — pragmatic (6A01.22)	Deficit in the use of social language; no restricted-repetitive patterns of behavior.
Reactive attachment disorder (6B44)	Arises from early psychosocial deprivation; potential for recovery in appropriate environment.
Schizotypal disorder (6A22)	Usually manifests in adulthood; psychotic-like thinking.
OCD (6B20)	Ego-dystonic obsessions and compulsions; rituals in ASD are ego-syntonic.
Social anxiety disorder (6B04)	Social avoidance anxiety-based; social interest preserved.
Selective mutism (6B06)	Not speaking in specific contexts; normal speech in home context.

7. Examination and assessment

7.1 Clinical diagnostic tools

- Screening: M-CHAT-R/F (16–30 months), SCQ (4+ years, parent questionnaire), SRS-2 (Social Responsiveness Scale).
- Diagnostics: ADOS-2 (toddler-adult modules), ADI-R (1.5+ years, parent).
- Intellect and cognition: Mullen Scales of Early Learning, Bayley-III/IV (young age), WISC-V, Leiter-3 (non-verbal).
- Adaptive behaviour: Vineland-3, ABAS-3.
- Speech and language: PLS-5, CELF-5, CCC-2 (pragmatic).
- Sensory profile: Sensory Profile-2 (Dunn).
- Comorbidity: Conners for ADHD, SCARED for anxiety, PHQ-9 adaptations for depression.

7.2 Laboratory investigations

First-line (AAP 2020 / AACAP 2014): Chromosomal microarray analysis (CMA — diagnostic yield ~10–20%); Fragile X (FMR1).

Selective: trio exome sequencing (WES — diagnostic yield of 25–40% if CMA is negative); MECP2 (for suspected Rett syndrome in females); metabolic (lactate, ammonia, amino acids — if regression is present); lead level; TSH.

7.3 Instrumental investigations

- EEG — in case of seizure, regression, bizarre behavior episodes.
- Brain MRI — focal neurological signs, micro/macrocephaly, regression.
- Hearing test — mandatory in all cases (audiometry, ABR).

8. Treatment

8.1 General principles (NICE CG170 · AAP 2020 · AACAP 2014 consensus)

1. ASD cannot be ‘cured’ — individual development, adaptation, and quality of life are improved. Claims of “cure” and “full recovery” are not scientifically based; the goal is functional skills, communication, independence, quality of life.
2. Early, intensive, individual behavioral intervention First-line — effect greatest between ages 18 months to 6 years (Reichow B. et al. Cochrane 2018, EIBI/ESDM meta-analysis).
3. Naturalistic Developmental Behavioral Interventions (NDBI) — based on the principles of behavior-development in play and daily contexts; more flexible than the classical discrete-trial version of ABA. Included in this group:
    - Early Start Denver Model (ESDM)
    - Joint Attention, Symbolic Play, Engagement, and Regulation (JASPER)
    - Pivotal Response Treatment (PRT)
    - Enhanced Milieu Teaching (EMT)
    - Paediatric Autism Communication Therapy (PACT)

    Detailed description of methods — in the ‘Treatment methodologies’ section.
4. Parent-mediated approach — Hanen “More Than Words”, PACT (Pickles A., Green J. et al. Lancet 2010, 2016 follow-up — 6-year effect sustained),

PRT parent version. Most cost-effective approach in resource-limited settings.

5. Social skills groups — for older children and adolescents; PEERS program (UCLA, Laugeson E.A.) — also effective in adults.
6. Structured educational environment — TEACCH (Treatment and Education of Autistic and related Communication-handicapped Children, Schopler E. North Carolina); IEP / EHCP individual education plan.
7. Communication support — in minimally verbal patients — Augmentative and Alternative Communication (AAC) — PECS (Picture Exchange Communication System), speech-generating devices, tablet-based applications.
8. Comorbidity treatment — ADHD (stimulants, Cortese 2018), anxiety (CBT modification — Storch E.A. RCTs), depression (CBT, SSRIs), epilepsy (standard antiepileptic), sleep (sleep hygiene, melatonin — RCT evidence Goldman S.E. J Clin Sleep Med 2014).
9. Pharmacotherapy only for comorbid symptoms — There is no FDA-approved medication for the core symptoms of ASD. For behavioral aggression and irritability — risperidone (5–16 years, FDA approval 2006) and aripiprazole (6–17 years, FDA approval 2009). Both are evidence-based (McCracken J.T. et al. NEJM 2002 RCT — risperidone), but the side effect burden is significant (metabolic, EPS). Behavioral intervention is first-line.

8.2 Treatment methods

1. Applied Behavior Analysis (ABA) — Learning method based on behavioral principles; shaping social, communicative, and adaptive skills, reducing challenging behavior. Applied in discrete trial training (DTT) or naturalistic format. Part of adult autistic community criticizes classical form of ABA (autonomy, distress).
2. Early Intensive Behavioral Intervention (EIBI) — Structured intensive intervention based on ABA at early age (25–40 hours per week); target — social, communicative, and adaptive skills. Evidence: Reichow B., Hume K., Barton E.E., Boyd B.A. Cochrane 2018 — small-to-medium effect on IQ, language, and adaptive behavior; current claims should be approached with caution.
3. Early Start Denver Model (ESDM) — Rogers S., Dawson G — Naturalistic developmental behavioral intervention for 12–48 months; learning within the context of play and daily activities. Evidence: Dawson G. et al. Pediatrics 2010 RCT n=48 — significant improvement in IQ, language, and adaptive behavior, diagnosis change at age 6 (Dawson G. et al. JAACAP 2012 follow-up). Parent version (P-ESDM) Rogers S.J. et al. JAACAP 2012.

4. Joint Attention, Symbolic Play, Engagement, and Regulation (JASPER) — Kasari (Kasari C.) — Intervention targeting joint attention and symbolic play development; 12–60 months. Evidence: Kasari C. et al. JAMA Pediatr 2014, J Consult Clin Psychol 2010 — significant improvement in joint attention and expressive language.
5. Paediatric Autism Communication Therapy (PACT) — Pickles (A.), Green (J.) — Parent-coordinated communication intervention; ages 2–11. Video-based analysis and expansion of parent-child interaction. Evidence: Pickles A. et al. Lancet 2016 — n=152, 6-year follow-up — sustained improvement in autism severity and parent synchronization. Adapted for resource-limited settings.
6. Pivotal Response Treatment (PRT) — Koegel (Koegel R.L., Koegel L.K.) — focusing on “Pivot” behaviors (motivation, response to multiple signals, self-regulation, self-initiation) — when learned, change spreads to multiple areas. In naturalistic format, based on child's choices.
7. Treatment and Education of Autistic and related Communication-handicapped Children (TEACCH) — Schopler E. — Structured visual educational environment — ‘structured teaching’ — visual schedules, work systems, individual transitions; in home, school, and community contexts. Originated in North Carolina, spread worldwide.
8. Picture Exchange Communication System (PECS) — Bondy, Frost — Picture exchange-based communication training for minimal-verbal patients — in 6 stages. Evidence: Howlin P. et al. J Child Psychol Psychiatry 2007 RCT — increase in communication.
9. Program for the Education and Enrichment of Relational Skills (PEERS) — Laugeson (Laugeson E.A., UCLA) — Structured social skills group for adolescents and young adults with ASD; 16 weeks, with parental support. Evidence: Laugeson E.A. et al. J Autism Dev Disord 2012, 2015.
10. Hanen “More Than Words” — 8 group sessions + 3 individual home visits for parents of children with ASD aged 2–5. hanen.org.
11. WHO Caregiver Skills Training (CST) — WHO's limited-resource parent training for low- and middle-income countries; for ASD and developmental delays; implemented by non-specialist trainer-facilitators working in the community.
12. EEG Neurofeedback — Neurofeedback training, based on real-time monitoring of brainwave patterns, aimed at enabling the user to regulate their own brain activity. In ASD, the evidence base for an effect on core symptoms is limited; Holtmann M. et al. Dev Med Child Neurol 2011 — modest in ADHD, specific evidence in ASD is weak. Not first-line, adjunct

status; the limited evidence base should be explained to the patient and the family.

13. Autism Diagnostic Observation Schedule (ADOS-2) and Autism Diagnostic Interview-Revised (ADI-R) — Diagnostic gold standard tools — ADOS-2 structured play-based observation (5 modules, toddler-adult); ADI-R standardized parent interview (1.5+ years, ~2 hours).

8.3 Differences between sources

- NICE CG170 (2013, 2021 update): diagnosis by a multidisciplinary group; social communication interventions are first-line; parent programs are superior; antipsychotics only when behavioral interventions fail; caution against claims of actual “treatment”.
- AAP Clinical Report 2020 (Hyman et al.): 18 and 24 months universal screening; early intervention (<3 years); integrated medical and behavioral management.
- AACAP Practice Parameter 2014 (Volkmar): emphasis on diagnosis-specific educational placement, comorbidity treatment, family support.
- WHO Caregiver Skills Training (CST): resource-limited parent training program for low and middle income countries; widely adapted across countries (Salomone E. et al. Autism 2019).

9. Prognosis

Good prognostic factors

- Early diagnosis (<3 years) and early intensive intervention.
- Intellectual function preserved (IQ >70).
- Acquisition of functional language by age 5.
- Management of comorbid disorders.
- Stable family support and community resources.

Poor prognostic factors

- Comorbid intellectual disability (severe-profound).
- Absence of functional language by age 5.
- Epileptic seizures — additional risk increase in adulthood.
- Severe behavioral disturbances, self-harm.
- Late initiation of diagnosis and intervention.

Follow-up targets

- Comorbid psychiatric conditions (ADHD, anxiety, depression, OCD) screening — annual.
- Epilepsy monitoring — especially during adolescence.
- Sleep, food, gastrointestinal tract, motor disturbance screening.
- Late adolescence planning (16–18 years) — career, independent living, employment.
- In adult patients, the emotional burden of social camouflage, anxiety, and depression — active screening.
- Respite support for family, peer support, psychoeducation.
- Neurodiversity approach — strengthening the patient's strengths, no claim of ‘full cure’.

10. Myths and misconceptions

10.1 Aetiology myths

Myth 1: “Vaccines (especially MMR) cause autism”

Why it is widespread: Wakefield A.J. et al. Lancet 1998 — claimed association of MMR vaccine with bowel disorder and autism based on 12 children. Subsequently, Brian Deer's journalistic investigation and the General Medical Council inquiry revealed Wakefield's financial interest and data manipulation; article fully retracted in 2010; Wakefield stripped of medical license.

Clinical and biological rationale: Autism clinically manifests at 12–18 months — temporal correlation with typical MMR vaccination period may lead to misinterpretation as a ‘cause’. Pathogenesis is genetic and early neurodevelopmental-based (hereditability 80%, prenatal and perinatal factors) — not vaccine-dependent.

Evidence: Taylor L.E., Swerdfeger A.L., Eslick G.D. Vaccine 2014 meta-analysis n>1.2 million children — between MMR and autism there is no relation; Madsen K.M. et al. NEJM 2002 Danish study n>537,000; Jain A. et al. JAMA 2015 US study. CDC, WHO, NICE, AAP hold the same position.

Real clinical step: Present evidence calmly, respectfully to family; explain the risk of vaccine refusal for child and community health (2018–2019 measles epidemic in Madagascar, 1200+ child deaths).

Myth 2: “Microplastics, heavy metals, chemical food additives are the main cause of autism”

Why it is widespread: “Natural / chemical-free” movement exaggerated in mass media; out-of-context use of microplastic research.

Clinical and biological rationale: The primary etiology of autism is genetic (heritability 80% — Sandin JAMA 2017). Environmental factors are investigated as risk factors (PM2.5 air pollution — Volk JAMA Psychiatry 2013; prenatal valproate — Christensen JAMA 2013), but there is no evidence base for microplastics or heavy metals as a “primary cause”.

Evidence: Hertz-Picciotto I. Environ Health Perspect 2018 review — some neurotoxins (especially valproate, alcohol) are considered risk factors; heavy metal exposure has not been proven as a direct etiology of autism.

Real clinical step: Provide family with realistic risk factors (valproate during pregnancy, limiting air pollution) and realistic protective steps (periconceptional folate).

Myth 3: “‘Cold mother’ (Bettelheim's ‘Refrigerator Mother’ theory)”

Why it is widespread: Bruno Bettelheim in his 1967 work “The Empty Fortress” attributed autism to maternal emotional coldness; dominant belief until the 1970s.

Clinical and biological rationale: Autism is a neurodevelopmental disorder — related to genetics and early brain development; parental behavior does not cause autism.

Evidence: Rimland B. (1964) “Infantile Autism” — argument for biological etiology; Folstein S., Rutter M. Nature 1977 — twin study demonstrated genetic inheritance. APA, AAP, AACAP — “refrigerator mother” concept has been discarded.

Real clinical step: Support and psychoeducation to family instead of blaming; parent-coordinated interventions (PACT, ESDM, Hanen).

10.2 Harmful or dangerous “treatment” methods

Myth 4: “Chelation therapy removes heavy metals and cures autism”

Why it is widespread: “Heavy metal autism” false theory; marketing by alternative clinics.

Evidence and risk: AAP 2010 and FDA warning — a 5-year-old child died from cardiac arrest due to calcium deficiency during chelation procedure (2005). Chelation

is indicated only for confirmed heavy metal poisoning (high lead), not for autism. AAP, AACAP — absolute contraindication.

Myth 5: "Hyperbaric oxygen chamber (HBOT) cures autism"

Evidence: Cochrane (Xiong T. et al. 2016) review of n=1 RCT — no evidence of efficacy for autism. Risks: pneumothorax, ear barotrauma, fire (oxygen + static).

Myth 6: "MMS / 'Miracle Mineral Solution' (chlorine dioxide) cures autism"

Evidence: FDA 2019, EMA warnings — chlorine dioxide is caustic, liver and kidney damage, deaths reported. All clinical use is prohibited. Worse — "treatment" recommending it as an anal cleansing agent is child torture.

Myth 7: "Stem cell injections cure autism"

Evidence: ISSCR (International Society for Stem Cell Research) and FDA — no approved stem cell therapy for autism. In illegal clinics — infections, brain tumors, deaths have been reported (Berkowitz NEJM 2016).

Myth 8: "Chelyabinsk remedy" and similar unregulated medical preparations

Evidence: no 'alternative' agents approved by any confirmed regulatory body (FDA, EMA, EU) for autism treatment — risk high, benefit unproven.

10.3 Ineffective methods or those delaying primary intervention

Myth 9: "Facilitated Communication (FC) and Rapid Prompting Method (RPM) reveal the inner world of minimally verbal individuals with autism"

Why it is widespread: Emotionally engaging narratives — children who "start to talk" with facilitator assistance.

Evidence: ASHA Position Statement 2018 (update): independent reviews have shown that messages come from the facilitator (ideomotor effect), not the patient. This evidence has been confirmed in multiple RCTs (Schlosser R.W. et al. Augment Altern Commun 2014). Clinical outcome: false belief for the family; the patient's true authentic communication (PECS, AAC devices) is delayed; public harm — false 'testimony' incidents in legal issues.

Real clinical step: Evidence-based AAC tools for minimally verbal patients — PECS, tablet-based communication programs (Proloquo2Go, TouchChat).

Myth 10: “Gluten-free / casein-free diet cures autism or significantly improves it”

Evidence: Hyman S.L. et al. (AAP 2020 Clinical Report) — effect of special diets on autism symptoms not proven; diet is medically indicated if gastrointestinal symptoms or celiac disease present. Broad dietary restriction leads to nutritional deficiencies and eating disorders in children. Karagas M.R. et al. JAMA Pediatr 2018 — risks of broad dietary restriction.

Myth 11: “EEG Neurofeedback cures core symptoms of autism”

Evidence: Holtmann M. et al. Dev Med Child Neurol 2011 review — modest effect in ADHD; specific evidence for autism weak. Adjunct status, not first-line. Must be clearly explained to the patient and the family.

Myth 12: “Auditory Integration Training (AIT) cures autism”

Evidence: ASHA Position Statement 2004; AAP 1998 — experimental status, no evidence of effect.

Myth 13: “Vitamin B6 / magnesium or other mega-dose vitamin therapy”

Evidence: Cochrane (Nye C., Brice A. 2005) — no evidence for efficacy of B6/magnesium combination for autism. Vitamin mega-doses may lead to side effect burden (B6 neuropathy).

Myth 14: “Patterning” (Doman-Delacato)

Evidence: AAP 1968 and 2010 Statements — no evidence of effect, excessive burden on family; delays children from evidence-based interventions.

Myth 15: “Cranio-Sacral Therapy, reflexotherapy, homeopathy”

Evidence: systematic reviews (Cochrane and others) — no evidence of effect for autism. NICE CG170 — warning against complementary interventions.

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ICD-11 **6A03**	**6A03 DEVELOPMENTAL LEARNING DISORDER** **Developmental learning disorder**
ICD-10 F81	Specific developmental disorders of scholastic skills
DSM-5-TR F81.0	Specific Learning Disorder, With Impairment in Reading

1. Definition and nosology

Developmental learning disorder (ICD-11: 6A03; DSM-5-TR: F81.0–F81.81 Specific Learning Disorder) — a neurodevelopmental disorder manifesting with persistent and significant difficulties in acquiring one or more academic skills, unexpected given the patient's age, intellectual level, and educational opportunities.

Subspecifications in ICD-11:

- 6A03.0 — with reading impairment (dyslexia);
- 6A03.1 — with written expression impairment (dysgraphia);
- 6A03.2 — with impaired mathematics (dyscalculia);
- 6A03.3 – other specified or combined.

2. History

- Pringle Morgan W. (1896) — “congenital word blindness” – describes a child who cannot read despite normal intelligence.
- Orton S.T. (1925) — The concept of “strephosymbolia”; hypothesized a weakness of left hemisphere dominance; the foundation of the Orton-Gillingham method.
- DSM-III (1980) — Specific Developmental Disorder Category — Based on IQ-academic skill discrepancy model.
- DSM-5 (2013) — elimination of the discrepancy model: “Specific Learning Disorder” — no IQ cutoff required; persistent functional difficulty and unresponsiveness to intervention are the basis. Separate coding of subtypes (reading, writing, mathematics) is retained.
- ICD-11 (2019) — parallel structure; with the term “Developmental Learning Disorder”.

- Phonological deficit theory (Stanovich K.E., Snowling M.J., Vellutino F.R.) — became dominant in the 1990s–2000s as the core mechanism of dyslexia.

3. Epidemiology

- General prevalence: School-age children: 5–15% (DSM-5-TR estimate); varies depending on the country and the orthographic depth of the language (the prevalence is higher in languages with deep orthographies, such as English).
- Dyslexia: 5–10% in school-age children; the most prevalent subtype.
- Dyscalculia: 3–7% (Shalev R.S. Pediatr Neurol 2007 review).
- Dysgraphia: 4–7% (Berninger V.W. Top Lang Disord 2008).
- Sex: In reading and writing disorders, boys are 2–3 times higher (the hypothesis of underdiagnosis in females exists); in math disorder, genders are equal.
- Comorbidity: ADHD 30–40%, language disorders 50–60% (in reading disorder), increased risk of anxiety, depression, conduct disorders.

4. Aetiology and pathogenesis

4.1 Genetic factors

- Heritability: in dyslexia 50–70% (Pennington B.F. Cogn Neuropsychol 2002 twin study review).
- Identified risk genes: DYX1C1 (15q21), KIAA0319 (6p22), DCDC2 (6p22), ROBO1 (3p12) — associated with neural migration and cortex formation; however, not a single gene, but a polygenic risk architecture.
- In mathematical disorder, high heritability (~70%), MYT1L and other genes are proposed (Mascheretti S. et al. Trends Genet 2018).

4.2 Neurobiological mechanisms

- Dyslexia — phonological deficit: in left perisylvian areas (Wernicke, angular gyrus, superior temporal gyrus) atypical activation patterns (Shaywitz S.E., Shaywitz B.A. Biol Psychiatry 2008 fMRI review). Deficit in phoneme-grapheme (sound-letter) correspondence is the core of reading difficulty.
- Dyscalculia — number sense (approximate number system) and working memory: functional disruption in intraparietal sulcus and prefrontal cortex (Butterworth B. Trends Cogn Sci 2010).

- Dysgraphia: motor planning and orthographic coding deficits; left fusiform and premotor areas.

4.3 Environmental factors (modifier)

- Early educational quality and reading instruction methodology — lack of phonological approach creates greater academic disparity in children with polygenic risk.
- Low socioeconomic status — not etiology, but a factor in identification and access to intervention.
- Chronic otitis media — may affect early phonological development.

5. Clinical features

5.1 Reading disorder (dyslexia, 6A03.0)

- Inaccuracy and slowness in word reading — reads words slowly, with struggle, substitutes or omits letters.
- Phonological awareness deficit — difficulty with rhyming, letter-sound correspondence, segmenting words into sounds.
- Writing and spelling — phonetic rule non-conforming writing errors.
- Reading comprehension — secondary impairment due to word recognition deficit.
- Early signs (preschool) — delayed learning of letter names, difficulty with rhyming games, difficulty reading names.

5.2 Written expression disorder (dysgraphia, 6A03.1)

- Handwriting is poor, illegible, slow.
- Difficulty in sentence construction, grammar, and paragraph organization.
- Persistent errors in spelling and punctuation.
- Large discrepancy between written and oral expression skills.

5.3 Mathematics disorder (dyscalculia, 6A03.2)

- Number sense deficit — quantitative comparison, assessment of magnitude.
- Difficulty recalling mathematical facts (e.g., multi-digit multiplication table).
- Slowness and errors in mathematical operations — addition, subtraction, multi-step problems.
- Difficulties in the verbal presentation of mathematical problems.

6. Diagnosis

6.1 Unified diagnostic criteria (DSM-5-TR · ICD-11 consensus points)

A. Persistent difficulties in the acquisition and use of academic skills — persists for at least 6 months despite age-appropriate targeted intervention. At least one of the following symptoms:

1. Inaccuracy or slowness in reading words;
2. Difficulty understanding the text being read;
3. Spelling difficulties;
4. Difficulties in written expression (grammar, punctuation, organization);
5. Challenges in emotional and cognitive skills;
6. Difficulties in mathematical reasoning.

B. Academic skills are significantly below the age-expected level — documented by standardized tests (-1.5 SD or lower for chronological age), creates functional impairment in academic or occupational activity.

C. Onset at school age (but may not fully manifest until academic demands increase).

D. Exclusions — not fully explained by intellectual disability, hearing/vision deficit, other neurological or psychiatric condition, psychosocial deprivation, ambiguity in academic language of instruction, inadequate education.

6.2 Differences between sources

- DSM-5-TR: The “IQ-academic ability discrepancy” model has been abolished — no IQ cutoff is required for diagnosis; unresponsiveness to long-term intervention is essential. Severity: mild, moderate, severe — based on adaptive functioning.
- ICD-11 (6A03): Parallel structure; sub-specifications (reading, writing, math). Severity is not a separate qualifier.
- IDA — International Dyslexia Association: dyslexia — neurobiological origin, deficit in phonological processing; ‘neuroinformation-processing’ disorder. Standardized literacy assessment + response observation (Response to Intervention, RTI) recommended.
- AAP Clinical Report on Dyslexia (Handler S.M., Fierson W.M. Pediatrics 2011, reaffirmed): diagnosis should be referred by pediatricians, assessment multidisciplinary — psychologist + speech-language pathologist + educational specialist.

- NICE: no specific clinical guidelines for learning disorder; main framework is within the education system (SEND Code of Practice, EHCP).

6.3 Diagnostic algorithm

1. Teacher and parent interview — academic history, response to intervention.
2. Visual and hearing tests (to rule out sensory impairment).
3. Standardized academic achievement tests — Woodcock-Johnson IV, WIAT-4, KTEA-3.
4. Phonological awareness and reading assessment — CTOPP-2, GORT-5, TOWRE-2.
5. For mathematics — KeyMath-3, TEMA-3.
6. Intellectual assessment (WISC-V) — except for intellectual disability; discrepancy not necessary for diagnosis.
7. Assessment of attention, executive function (BRIEF-2), working memory.
8. Response to Intervention (RTI) documentation — persistent difficulty despite ≥ 6 months of age-appropriate intervention.
9. Comorbidity screening — ADHD, language disorder, anxiety, depression.

6.4 Differential diagnosis

Condition	**Distinguishing features**
Intellectual developmental disorder (6A00)	Academic deficit in the context of general cognitive deficit; adaptive function is also impaired.
Developmental language disorder (6A01)	Acquisition of spoken language impaired; reading and spelling problems are secondary.
ADHD (6A05)	Attention deficit reduces academic achievement; academic skills in standardized tests are usually appropriate for attention level. Comorbidity is frequent.
Visual/auditory impairment	Detected via sensory examination.
Social-economic deprivation, inadequate education	In an appropriate educational setting, academic achievement is restored.

Condition	Distinguishing features
Second language learning (English Language Learner)	Academic skills in the native language should be assessed; bilingualism alone is not a basis for diagnosis.

7. Examination and assessment

7.1 Standardized tests

- Academic skills: Woodcock-Johnson IV (comprehensive), WIAT-4 (Wechsler Individual Achievement Test), KTEA-3.
- Reading: GORT-5 (Gray Oral Reading Test), TOWRE-2 (Test of Word Reading Efficiency), CTOPP-2 (Comprehensive Test of Phonological Processing).
- Mathematics: KeyMath-3, TEMA-3 (Test of Early Mathematics Ability).
- Intellect and cognition: WISC-V, WJ-IV Cognitive.
- Executive function: BRIEF-2 (parent/teacher questionnaire).
- Working memory, processing speed: WISC-V indices, digit span, Trail Making.

7.2 Laboratory investigations

No routine laboratory indication. Based on specific clinical suspicion:

- Lead level — if risk factors are present.
- Thyroid function — if congenital hypothyroidism is undetected.
- Genetic research — if comorbid intellectual disability or syndromic features present.

7.3 Instrumental investigations

- Brain MRI — no routine indication; in case of focal neurological signs, seizures, regression.
- EEG — suspected seizure, rule out Landau-Kleffner.
- Hearing test and vision test — mandatory initial assessments in all cases.

8. Treatment

8.1 General principles (IDA · AAP 2011 · APA · NASP consensus)

1. Specific learning disorders are not treated pharmacologically. Intervention — evidence-based education and therapy.
2. Early identification and intervention — intervention efficacy is significantly higher in preschool and early school-age periods. RTI (Response to Intervention) model — age-appropriate intervention stepwise, lack of response leads to diagnostic assessment.
3. Dyslexia — Structured Literacy approach: systematic, sequential, explicit, multisensory, diagnostic training. Phonological awareness + phoneme-grapheme correspondence + word structure + syntax + semantics. Orton-Gillingham, Wilson Reading System, Lindamood-Bell are typical programs.
4. Dyscalculia: Concrete-Representational-Abstract (CRA) instruction; number sense development, math fact memorization exercises.
5. Dysgraphia: Handwriting training (e.g., Handwriting Without Tears), keyboard learning, word-sentence organization strategies, voice-to-text technology.
6. Academic accommodations: Extra time, audiobooks, keyboard, text-to-speech software, calculator in math — within IEP / EHCP.
7. Treatment of comorbidity: ADHD — stimulants; anxiety and depression — CBT; language disorder — SLP.
8. Social-emotional support. Children with learning disorders are at high risk for issues related to self-esteem, motivation, and anxiety.

8.2 Treatment methods

1. Orton-Gillingham (OG) approach — Structured multisensory phoneme-grapheme-based reading and spelling training. First-line methodology for dyslexia; conducted by certified teacher/therapist. Evidence: Stevens E.A. et al. J Learn Disabil 2021 meta-analysis — medium effect size in reading and spelling. Academy of Orton-Gillingham Practitioners.
2. Wilson Reading System — Program based on Orton-Gillingham principles, a 12-step complex reading and spelling program; from age 6 to adulthood. wilsonlanguage.com.
3. Lindamood-Bell programs — LiPS (Lindamood Phoneme Sequencing) — combines phoneme awareness with motor-kinesthetic component; Seeing Stars — orthographic encoding. lindamoodbell.com.
4. Structured Literacy (IDA Knowledge and Practice Standards) — Not a specific program, a framework — systematic, sequential, explicit,

multisensory, diagnostic teaching principles. Orton-Gillingham, Wilson, Lindamood-Bell and others fit this framework. dyslexiaida.org.

5. Concrete-Representational-Abstract Instruction (CRA) — For dyscalculia — training starting with concrete manipulative tools, then pictorial representation, then transitioning to abstract symbolic operations. Evidence: Witzel B.S. et al. J Spec Educ Technol 2003.
6. "Handwriting Without Tears" program — Olsen (Olsen J.) — Multisensory handwriting training program used in dysgraphia and writing difficulties. lwtears.com.
7. Response to Intervention (RTI) / Multi-Tiered System of Supports (MTSS) — 3-tier framework – Tier 1: universal high-quality training; Tier 2: targeted small group intervention; Tier 3: intensive individual intervention. Established by IDEA 2004 law in USA; similar frameworks in other countries.
8. Assistive Technology — Text-to-speech (Read&Write, NaturalReader), speech-to-text (Dragon Naturally Speaking), audiobooks (Bookshare, Learning Ally), calculation tools for math — functional compensation. Recommended by AAP and IDA.
9. Comprehensive Test of Phonological Processing (CTOPP-2) — Standardized assessment of phonological awareness — gold standard in dyslexia identification.
10. GORT-5 (Gray Oral Reading Test) and TOWRE-2 — Measurement of reading speed, accuracy, and fluency.

8.3 Differences between sources

- IDA Knowledge and Practice Standards for Teachers of Reading (2018): Structured Literacy framework; certified teacher requirement.
- AAP 2011 (Handler & Fierson): dyslexia biologically based; academic intervention first-line; visual therapy is contraindicated (see 10.3).
- National Reading Panel (NRP) 2000: five essential reading components — phonological awareness, phonics, fluency, vocabulary, comprehension.
- NASP (National Association of School Psychologists): Within RTI, there are three tiers — universal screening, targeted intervention, and intensive individualized intervention.

9. Prognosis

Good prognostic factors

- Early identification (preschool or early school age).

- Evidence-based Structured Literacy intervention.
- High IQ — development of compensatory strategies.
- Family support and motivation.
- Affects only one academic domain (e.g., single reading, math normal).

Poor prognostic factors

- Late identification (middle or late school age).
- The impact across multiple academic domains.
- Comorbid ADHD, language disorder, anxiety.
- Low socioeconomic support, limited educational resources.
- Intervention with unproven methods (colored lenses, visual therapy, Brain Gym).

Follow-up targets

- Academic achievement monitoring — annual standardized tests.
- Social-emotional health — self-esteem, anxiety, depression screening.
- Separate intervention for comorbid ADHD and language disorder.
- Professional and academic accommodation planning in adulthood (disability-related opportunities in higher education, workplace adaptation).
- Enhancement of assistive technology use (text-to-speech, calculator).

10. Myths and misconceptions

10.1 Aetiology myths

Myth 1: “Dyslexia is the result of a child's laziness or poor upbringing”

Why it is widespread: Belief “if they try hard enough, they can read”; incorrect explanation of academic struggle as a character issue.

Clinical and biological rationale: Dyslexia is a neurobiological disorder. fMRI studies (Shaywitz S.E. Biol Psychiatry 2008) have shown atypical activation patterns in left perisylvian areas; heritability 50–70%, identified risk genes (DYX1C1, KIAA0319, DCDC2). The patient is forced to expend higher cognitive effort rather than work harder — this leads to chronic exhaustion.

Evidence: IDA Definition (2002, update) — “Dyslexia is a specific learning disability of neurobiological origin.”

Real clinical step: Explain biological basis to family; evidence-based Structured Literacy intervention and academic accommodation instead of blaming.

Myth 2: "Dyslexia is children writing letters in reverse (mirror)"

Why it is widespread: Orton 1925 'strephosymbolia' term and recurring image in popular media.

Clinical and biological rationale: Reversed letter writing and reading is normal in all children aged 4–7 years; persistent letter inversion after age 8 may raise clinical suspicion, but it is not the main symptom of dyslexia. The core of dyslexia is a deficit in phonological processing.

Evidence: Stanovich K.E. (2000) — phonological deficit theory; visual-spatial deficit is not primary etiology.

Myth 3: "Children with dyslexia will 'outgrow it' with age, no intervention needed"

Evidence: Bruck M. Dev Psychol 1992 — long-term follow-up studies: dyslexic children without intervention retain reading difficulties in adulthood; academic and vocational outcomes are poor. The earlier the intervention, the higher the compensation potential.

10.2 Harmful or scientifically unfounded methods

Myth 4: "Colored lenses (Irlen lenses) or colored overlays cure dyslexia"

Why it is widespread: In the 1980s, concept of 'scotopic sensitivity syndrome' by Helen Irlen; marketing by commercial optometric clinics.

Evidence: AAP, AAO (American Academy of Ophthalmology) and AAPOS (American Association for Pediatric Ophthalmology and Strabismus) 2009 and 2014 Joint Statements: The effect of colored lenses and overlays for dyslexia is unproven; Cochrane (Evans B.J., Allen P.M. 2016) systematic review — insufficient evidence of effect. AAP — delays first-line intervention (Structured Literacy).

Real clinical step: Clear explanation to family; evidence-based phonological reading program as intervention.

Myth 5: "Visual therapy (eye exercises) cures dyslexia"

Evidence: AAP, AAO, AAPOS 2009 Joint Statement — emphasizes dyslexia is a language-based disorder, not an eye movement disorder; effect of eye exercises on reading performance not proven. Visual therapy is indicated only for specific eye movement disorders (e.g., convergence insufficiency).

Myth 6: “Brain Gym / Educational Kinesiology”

Evidence: Hyatt K.J. Remedial Spec Educ 2007 systematic review — claims do not align with neurobiology; no effect on academic achievement. Criticized as a conversion program.

Myth 7: “Cerebellar Exercise Therapy”

Why it is widespread: Wynford Dore popularized in the 2000s a theory linking cerebellar function to reading; claims of “positive outcomes” attracted media attention.

Evidence: The original research by Reynolds D., Nicolson R.I. received rigorous methodological criticism (Bishop D.V. Dyslexia 2007); the effect was not confirmed in subsequent studies. British Dyslexia Association — does not recommend the Dore programme.

10.3 Ineffective methods or those delaying primary intervention

Myth 8: “Whole Language” approach teaches reading instead of phonics

Evidence: National Reading Panel 2000 (NRP) — systematic phonics instruction is clearly superior to the “whole language” approach, especially in children with learning difficulties. Same position incorporated into educational legal standards by Endrew F. v. Douglas County (US Supreme Court 2017). The “Reading Wars” debate has largely been resolved in favor of systematic phonics.

Myth 9: “Auditory Integration Training (AIT) / Tomatis method cures dyslexia”

Evidence: ASHA Position Statement 2004 — AIT has not been proven effective for dyslexia or learning disorders.

Myth 10: “EEG Neurofeedback cures dyslexia or dyscalculia”

Evidence: Neurofeedback effect for specific learning disorder has not been proven. Modest effect in ADHD (Holtmann 2011), but extrapolation to learning disorders is unfounded.

Myth 11: “Special diet (gluten-free, omega-3 mega-dose, ‘brain food’) cures dyslexia”

Evidence: Cochrane and systematic reviews (Tan M.L. et al. J Pediatr 2012 for omega-3) — sustained clinical effect in dyslexia has not been proven. Some studies have shown a small effect, but methodological limitations exist.

Myth 12: “Hyperbaric oxygen chamber (HBOT) or stem cell injections”

Evidence: No proven effect for specific learning disorder; risk profile is significant (HBOT — pneumothorax; stem cells — infection, fatal cases).

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	6A04 DEVELOPMENTAL MOTOR COORDINATION DISORDER (DCD)
ICD-11 **6A04**	**Developmental motor coordination disorder**
ICD-10 F82	Specific developmental disorder of motor function
DSM-5-TR F82	Developmental Coordination Disorder

1. Definition and nosology

Developmental motor coordination disorder (ICD-11: 6A04; DSM-5-TR: F82 Developmental Coordination Disorder, DCD) — a neurodevelopmental disorder manifesting with persistent difficulties in acquiring and performing daily motor skills, unexpected given the child's chronological age, intellectual level, and motor learning opportunities.

Old terms — “clumsy child syndrome”, “developmental dyspraxia”, “minimal brain dysfunction” — have been abandoned in modern clinical practice, replaced by the term DCD (Polatajko H.J., Fox A.M., Missiuna C. Can J Occup Ther 1995, “Leeds Consensus 2006”).

2. History

- 1937 — Orton S.T. “developmental clumsiness” concept.
- 1960s–1970s — “minimal brain dysfunction” (MBD) is a broad and imprecise term that was later abandoned.
- DSM-III-R (1987) — “Developmental Coordination Disorder” as an official diagnostic category.
- Leeds Consensus (2006) — international expert group standardized the term ‘DCD’; statement on ‘dyspraxia’ and ‘DCD’ not being accepted as synonyms (dyspraxia — narrower concept, motor planning deficit).
- EACD (European Academy of Childhood Disability) International Clinical Practice Recommendations — 2012, 2019 update (Blank R., Barnett A.L., Cairney J. et al. Dev Med Child Neurol 2019) — international standard for diagnosis, assessment, and intervention.
- DSM-5 (2013) and ICD-11 (2019) — DCD is included in the category of general neurodevelopmental disorders.

3. Epidemiology

- Prevalence: school-aged children 5–6% (EACD 2019; Lingam R. et al. Pediatrics 2009 England ALSPAC study n>6900).
- Sex: Boys are 2–7 times higher (the hypothesis of underdiagnosis in females exists).
- Persistence: 50–70% of patients have motor difficulties persisting into adulthood (Cantell M.H. et al. J Adolesc Res 2003 follow-up; Kirby A. et al. Res Dev Disabil 2010).

- Comorbidity: ADHD 50%, specific learning disorder 50%, autism spectrum 5–10%, developmental language disorder 20–30%, increased risk of anxiety and depression in adolescence.

4. Aetiology and pathogenesis

4.1 Genetic and neurobiological factors

- Heritability studies limited, but family history of DCD cited as risk factor (Gaines R. et al. Phys Occup Ther Pediatr 2008).
- Neurobiological — atypical patterns of functional connectivity in the cerebellum, basal ganglia, parietal cortex, and premotor areas (Zwicker J.G. et al. Eur J Paediatr Neurol 2012 fMRI review). Internal modeling deficit hypothesis — disruptions in the processes of prediction and comparison preceding motor planning (Adams I.L. et al. Dev Med Child Neurol 2014).

4.2 Risk factors

- Premature birth (<32 weeks) and low birth weight (<1500 g) — increases DCD risk 6–8 times (Edwards J. et al. Dev Med Child Neurol 2011 systematic review).
- Maternal alcohol use during pregnancy (motor difficulty component in the FASD spectrum).
- Perinatal hypoxic-ischemic events.
- DCD or other neurodevelopmental disorders in family history.

5. Clinical features

5.1 By motor skill areas

- Gross motor: Running, jumping, ball games, cycling, balance difficulty; avoidance of sports activity.
- Fine motor: Handwriting quality poor and slow, scissors, tying strings, buttoning, fork-knife use.
- Motor planning and execution: delay in learning new motor skills; problems with sequence and rhythm.
- Visual-motor integration: copying, drawing, difficulty forming letters.

5.2 Daily functional impairment

- Self-care — clothing, food intake, hygiene.

- Academic performance — handwriting speed, paper organization, sports participation.
- Community activity — playgrounds, sports teams, hobbies.
- Psychosocial outcomes — decreased self-esteem, anxiety, depressed mood, social isolation (Cairney J. et al. Dev Med Child Neurol 2013).

5.3 Trajectory

Early signs (1–3 years) — delay in motor milestones (sitting, standing, first steps at normal time or delayed; quality of specific movements poor). Difficulties become evident in preschool and early school age; with increasing academic demands (writing, paper tasks), the disorder becomes more prominent. In adulthood — persistent difficulties in driving, occupational demands, sports activities (50–70%).

6. Diagnosis

6.1 Unified diagnostic criteria (DSM-5-TR · ICD-11 · EACD 2019 consensus points)

A. Acquisition and execution of motor skills — significantly below the expected level when chronological age and motor learning opportunities are considered. Difficulties lead to slowness, imprecision, incoordination in performing motor skills.

B. Motor skill deficit significantly and persistently interferes with age-appropriate daily life activities (e.g., self-care, academic achievement, occupational activity, play and sports).

C. Onset in early developmental period.

D. Exclusions — not fully explained by intellectual disability, visual impairment or other neurological condition (e.g., cerebral palsy, muscular dystrophy, degenerative disorders).

6.2 Differences between sources

- EACD 2019 (Blank et al.): four diagnostic criteria standardized — score below 5–15th percentile on motor skill test (Movement Assessment Battery for Children, MABC-2 or similar); daily activity impairment; onset in early developmental period; not explained by other medical conditions. Multidisciplinary assessment (pediatrician + occupational therapist or physiotherapist) recommended. Diagnosis recommended from age 5 (after motor skills stabilize); before age 5 term ‘Provisional DCD’ or ‘motor development delay’ can be used.
- DSM-5-TR: EACD parallel criteria; the term “dyspraxia” is not used.

- ICD-11: Motor development disorder; no sub-specifications.

6.3 Diagnostic algorithm

1. Parent and teacher interview — developmental history, motor milestones, daily activity problems.
2. Standardized motor test — MABC-2 (Movement Assessment Battery for Children-2, age 3–16) or BOT-2 (Bruininks-Oseretsky Test of Motor Proficiency).
3. Functional daily activity assessment — DCDQ (Developmental Coordination Disorder Questionnaire, parent form), VABS adaptive component.
4. Visual test — exclusion of sensory impairment.
5. Neurological examination — exclusion of cerebral palsy, ataxia, neuromuscular disease.
6. Comorbidity screening — ADHD (Conners), specific learning disorder, autism, language disorder.
7. Intellectual assessment — only if suspected global developmental delay or intellectual disability.

6.4 Differential diagnosis

Condition	Distinguishing features
Cerebral palsy	Spasticity, hyperreflexia, persistent neurological signs; MRI typical findings.
Muscular dystrophy (Duchenne, Becker)	Progressive muscle weakness; CK level elevated; family history.
Congenital myopathy, neuropathy	Specific neurological and EMG findings.
Intellectual disability (6A00)	Motor deficit in the context of general cognitive deficit.
Autism spectrum disorder (6A02)	Motor stereotypies, social-communicative deficit are core.
ADHD (6A05)	Attention deficit affects motor performance; comorbidity is frequent (~50%).

Condition	Distinguishing features
Visual impairment	Visual test impaired.
Dysgraphia (6A03.1)	Fine motor problem dominant in writing context; general motor function is intact.

7. Examination and assessment

7.1 Standardized motor tests

- MABC-2 (Movement Assessment Battery for Children-2, ages 3–16) — gold standard; manual dexterity, ball skills, balance subtests.
- BOT-2 (Bruininks-Oseretsky Test of Motor Proficiency, ages 4–21) — broad motor profile.
- DCDQ (Developmental Coordination Disorder Questionnaire, ages 5–15, parent form) — functional screening.
- PDMS-2 (Peabody Developmental Motor Scales, <6 years).
- BHK (Concise Assessment Method for Children's Handwriting) — handwriting quality.

7.2 Laboratory investigations

No routine indication. Based on specific clinical suspicion:

- CK (creatine kinase) — suspected muscular dystrophy.
- Thyroid function — in some hypothyroid cases motor slowing.
- Genetic research — if syndromic features present.

7.3 Instrumental investigations

- Brain MRI — only if focal neurological signs, ataxia, stable deficits (to rule out cerebellar/cerebral pathology).
- EMG/nerve conduction study — suspected neuromuscular disease.
- Visual test — baseline indicators in all cases.

8. Treatment

8.1 General principles (EACD 2019 consensus)

1. DCD is not treated pharmacologically. Intervention — based on occupational therapy / physiotherapy.
2. Early identification and intervention — improves functional outcomes.

3. Top-down (task-oriented) approach first-line — demonstrates superior effect compared to bottom-up sensorimotor approaches (Smits-Engelsman B.C. et al. Dev Med Child Neurol 2013 meta-analysis).
4. CO-OP (Cognitive Orientation to daily Occupational Performance) and NTT (Neuromotor Task Training) — Evidence-based approaches recommended as first-line at EACD 2019.
5. Functional goals — daily activity goals chosen by patient and family (e.g., bicycle riding, dressing, handwriting).
6. Family and school involvement — intervention should be accompanied at home and in school.
7. Intensity: once to twice weekly sessions, typically 8–10 weeks initial course; extended based on effect assessment.
8. Comorbidity treatment — ADHD (stimulants), learning disorder (Structured Literacy), anxiety and depression (CBT).
9. Academic accommodations — handwriting tempo reduction, keyboard use, voice-to-text, additional time; individualized approach in physical education classes.

8.2 Treatment methods

1. Cognitive Orientation to daily Occupational Performance (CO-OP) — Polatajko H., Mandich A — Top-down cognitive-based approach in which the child is an active participant in solving his or her own motor problems. With the 'Goal-Plan-Do-Check' strategy, the patient selects, plans, executes, and evaluates their goal. 10–14 sessions, individual. Evidence: Smits-Engelsman B.C. et al. Dev Med Child Neurol 2013 meta-analysis — top-down approaches superior to bottom-up; Thornton A. et al. Phys Occup Ther Pediatr 2016 systematic review. EACD 2019 — first-line recommendation.
2. Neuromotor Task Training (NTT) — Schoemaker M.M., Smits-Engelsman B.C. — Task-oriented motor learning approach — repetitive, structured practice of specific motor skills; visual-motor integration, balance, ball skills. Evidence: Niemeijer A.S. et al. Phys Ther 2007. EACD 2019 — evidence-based.
3. MABC-2 (Movement Assessment Battery for Children-2) — Gold standard motor assessment; 3–16 years. Manual dexterity (3 sub-tests), ball skills (2), balance (3) — age-based normative tables.
4. DCDQ (Developmental Coordination Disorder Questionnaire) — 15 questions, parent-completed functional screening tool (Wilson B.N. et al. Phys Occup Ther Pediatr 2009 update).

5. BOT-2 (Bruininks-Oseretsky Test of Motor Proficiency-2) — 4–21 years, broad motor profile; used in clinical and research contexts.
6. "Partnering for Change" program (CanChild) — School-based intervention model — occupational therapist works jointly with teachers, implements adaptations in the classroom context. Evidence: Missiuna C. et al. Int J Environ Res Public Health 2017.
7. Keyboard training and voice-to-text technology — Academic accommodation – an assistive technology compensating handwriting difficulty.

8.3 Differences between sources

- EACD 2019 (Blank et al. Dev Med Child Neurol 2019): CO-OP and NTT are openly recommended; evidence for sensory integration therapy (Ayres SI) and whole-body movement imitation methods is weak/inconsistent.
- CanChild (McMaster University): "Partnering for Change" — a school-based training model; intervention is integrated into the educational system.
- NICE: no specific clinical guidelines for DCD; falls under the broader neurodevelopmental and SEND framework.

9. Prognosis

Good prognostic factors

- Early identification and intervention.
- CO-OP or NTT is an evidence-based intervention.
- Comorbid ADHD, learning disorder, anxiety management.
- Family and school support.
- Normal intellectual level.

Poor prognostic factors

- Multiple comorbid disorders (ADHD + LD + DCD together).
- Social isolation and complete avoidance of sports activities.
- Secondary anxiety, depression, social phobia.
- Late identification.
- Intervention with unproven methods (Ayres SI alone, Dore program, Brain Gym).

Follow-up targets

- Reassessment of motor functional goals (3–6 months).
- Academic achievement and handwriting speed monitoring.
- Social-emotional health — self-esteem, anxiety, depression screening (especially important in adolescence).
- Comorbidity active treatment.
- In adulthood occupational adjustment — avoidance of motor-demanding jobs or accommodation planning.
- Physical activity and sports — for social adaptation and cardiovascular health; type of sport adapted to patient's interests (e.g., swimming, yoga with low coordination demands).

10. Myths and misconceptions

10.1 Aetiology myths

Myth 1: "The child is just lazy / puts in too little effort"

Why it is widespread: Misinterpretation of motor difficulty as a character problem; limited knowledge of motor disorders in academic institutions.

Clinical and biological rationale: DCD is a neurodevelopmental disorder. Atypical models in functional connectivity of the cerebellum, basal ganglia, and parietal cortex (Zwicker 2012 fMRI review); "internal modelling" deficit hypothesis — the patient expends more cognitive resources for motor planning and execution, leading to fatigue and avoidance behavior.

Evidence: EACD 2019 (Blank et al.) — DCD is a neurobiologically based disorder; "motivation problem" is a misinterpretation.

Real clinical step: Explain biological basis to family and teacher; occupational therapy/physical therapy based CO-OP or NTT.

Myth 2: "The child will outgrow it, 'grow out of it'"

Evidence: Cantell M.H. et al. J Adolesc Res 2003, Kirby A. et al. Res Dev Disabil 2010 long-term follow-up studies — 50–70% of children with DCD retain motor difficulties in adulthood; academic and occupational outcomes, social adjustment are poor. The intervention window is not limited — effects can be observed at any age, but early intervention significantly improves socio-emotional outcomes.

Myth 3: "Parental impatience or poor parenting causes motor difficulties"

Evidence: EACD, AAP — DCD is genetic and neurobiologically based; parental experience is not etiology. Premature birth, perinatal hypoxia, family history are main risk factors.

10.2 Harmful or scientifically unfounded methods

Myth 4: "Sensory Integration Therapy (Ayres SI) cures DCD"

Why it is widespread: Jean Ayres 1972 proposed the theory of sensory integration; widely used in occupational therapy practice.

Evidence: EACD 2019 (Blank et al.) — Ayres SI approach shows **insufficient evidence** for functional motor performance in DCD; Smits-Engelsman 2013 meta-analysis — bottom-up sensory approaches are significantly weaker than top-down task-oriented approaches. AAP 2012 Sensory Integration Statement — therapeutic use classified as "research ongoing," not recommended as first-line.

Real clinical step: Top-down approaches (CO-OP, NTT) are first-line.

Myth 5: "Dore program (cerebellar exercise therapy) cures DCD"

Evidence: Bishop D.V. Dyslexia 2007 systematic critique — methodological limitations of Reynolds-Nicolson original studies; effect not replicated in subsequent studies. EACD does not recommend.

Myth 6: "Brain Gym / Educational Kinesiology"

Evidence: Hyatt K.J. Remedial Spec Educ 2007 — claims do not align with neurobiology; effect on motor and academic outcomes not proven.

Myth 7: "Visual therapy (eye exercises) improves motor coordination in DCD"

Evidence: AAP, AAO, AAPOS 2009 Joint Statement — effect of eye movement therapy on motor coordination or learning disorders not proven.

10.3 Ineffective methods or those delaying primary intervention

Myth 8: "EEG Neurofeedback improves motor function in DCD"

Evidence: Evidence base for neurofeedback in specific DCD is limited. EACD does not recommend; CO-OP and NTT are preferred.

Myth 9: “Special diet (omega-3 supplements) improves DCD”

Evidence: Richardson A.J. et al. Pediatrics 2012 (Oxford-Durham study) — claimed omega-3 supplements improved motor function in DCD, but systematic reviews (Cooper R.E. et al. J Psychopharmacol 2015) — insufficient evidence obtained. EACD does not recommend.

Myth 10: “Hyperbaric oxygen, stem cell injections, craniosacral therapy”

Evidence: none have proven efficacy for DCD; risk profile significant.

Myth 11: “Intensive sports training resolves DCD”

Evidence: General physical activity is beneficial, but in DCD the motor learning deficit requires specific structured intervention. A “just more exercise” regimen creates failure and distress in the patient. Structured approaches such as CO-OP and NTT are significantly superior.

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	6A05 ATTENTION DEFICIT HYPERACTIVITY DISORDER (ADHD)
ICD-11 **6A05**	**Attention deficit hyperactivity disorder**
ICD-10 F90.0	Disturbance of activity and attention

	6A05 ATTENTION DEFICIT HYPERACTIVITY DISORDER (ADHD)
ICD-11 6A05	**Attention deficit hyperactivity disorder**
DSM-5-TR F90.0–F90.2	Attention-Deficit/Hyperactivity Disorder

1. Definition and nosology

Attention Deficit Hyperactivity Disorder (ADHD; ICD-11: 6A05 Attention deficit hyperactivity disorder; DSM-5-TR: F90.0–F90.2x ADHD) — a neurodevelopmental disorder characterized by a pattern of inattention, hyperactivity, and impulsivity that begins in early development (by age 12), causing functional impairment in academic, social, or occupational activities.

In ICD-11 and DSM-5-TR, there are three presentation forms:

- 6A05.0 — predominantly inattentive type;
- 6A05.1 — predominantly hyperactive-impulsive type;
- 6A05.2 — Combined presentation.

2. History

- Still G.F. (1902) — “abnormal defect of moral control” — in the Goulstonian Lectures (Lancet) he described 43 children in total; of these, 20 (15 boys, 5 girls) showed the defect without general impairment of intellect and without physical disease.
- 1937 — Bradley C. — accidental discovery of stimulant (benzedrine) therapy in children with behavior problems.
- DSM-II (1968) — “Hyperkinetic Reaction of Childhood”.
- DSM-III (1980) — “Attention Deficit Disorder” (with or without hyperactivity) — attention deficit accepted as core.
- DSM-IV (1994) — standardization of the term “ADHD” and its 3 subtypes (inattentive, hyperactive-impulsive, combined).

- DSM-5 (2013) and ICD-11 (2019) — the term “presentation” reflects that the course may change over time; the age of onset has been raised from 7 to 12; in adults the number of symptoms required for diagnosis is reduced (5 of 9 symptoms instead of 6 in each category, for age 17 and older).

3. Epidemiology

- Prevalence in children: 5–7% (Polanczyk G.V. et al. Am J Psychiatry 2007 systematic review and meta-analysis, 102 studies). CDC in the US: 9.8% of children aged 3–17 years by parent-reported ever-diagnosis (NSCH 2016–2019), 11.4% in 2022; this is not a clinical assessment but a parent report of a previously made diagnosis.
- Prevalence in adults: 2.5–4% (Kessler R.C. et al. Am J Psychiatry 2006 — National Comorbidity Survey Replication).
- Sex: in childhood 2–4 times higher in boys (under-recognition in women — less hyperactivity, more inattention); difference decreases in adulthood.
- Persistence: 60–70% of childhood ADHD continues to some degree in adulthood (Faraone S.V. et al. meta-analysis, Psychol Med 2006).
- Comorbidity: Oppositional behavior disorder 40%, conduct disorder 14%, anxiety 25%, depression 15–30%, learning disorder 25–40%, tic disorders 7–20%, autism spectrum 30–50% (in ASD patients), substance use disorder risk increased 2–3 times in adolescence and adulthood.

4. Aetiology and pathogenesis

4.1 Genetic factors

- Heritability: 74% (Faraone S.V., Larsson H. Mol Psychiatry 2019 meta-analysis of twin and family studies) — one of the highest heritability neurodevelopmental disorders.
- Genetic architecture: Polygenic — hundreds of small-effect variants. GWAS (Demontis D. et al. Nat Genet 2019, n>55,000) identified 12 risk loci (FOXP2, SORCS3, DUSP6, and others).
- Dopamine system genes — DRD4, DRD5, DAT1 (SLC6A3) — candidate genes; effect size small.
- CNV — rare; common variations are fundamental.

4.2 Neurobiological mechanisms

- Dysfunction of the dopaminergic and noradrenergic systems — particularly prefrontal cortex, striatum, cerebellum circuitry; stimulants reduce symptoms by acting on these systems.
- Structural changes: A reduction in total brain volume and specific regions (caudate, putamen, prefrontal cortex) volume (Hoogman M. et al. Lancet Psychiatry 2017 ENIGMA-ADHD n>3000 — small effect sizes).
- Functional relationship: dysregulation between the default mode network and the executive control network.
- Executive function model (Barkley R.A.) — poor inhibition as core deficit; affects working memory, foreplanning, emotional regulation.
- Reward system dysfunction — delayed reward sensitivity (Sonuga-Barke E.J. Neurosci Biobehav Rev 2003).

4.3 Environmental risk factors

- Preterm birth and low birth weight — increases risk 2–3 times.
- Maternal use of tobacco, alcohol, and cannabis during pregnancy.
- Maternal perinatal stress and severe psychosocial deprivation.
- Lead exposure in childhood.
- Early traumatic brain injury (TBI).

4.4 Precise stance against ETIOLOGY MYTHS

- Sugar does not cause ADHD (Wolraich M.L. et al. JAMA 1995 meta-analysis).
- Screen time / video games do not cause ADHD (association exists, but no evidence of causal direction — Ferguson C.J. Perspect Psychol Sci 2015).
- Poor parenting does not create ADHD (heritability 74%); however, a highly conflictual family environment can affect symptom severity.

5. Clinical features

5.1 Inattention domain

- Failure to pay attention to details, careless mistakes.
- Difficulty sustaining attention in tasks.
- Appears not to listen when spoken to directly.
- Noncompliance with instructions, leaving tasks unfinished.

- Difficulty organizing tasks and activities.
- Persistent avoidance of tasks requiring sustained mental effort.
- Losing necessary items.
- Easily distracted by external stimuli.
- Forgetfulness in daily activities.

5.2 Hyperactivity and impulsivity domain

- Movement of limbs, twisting in seated position.
- Not remaining seated where sitting is expected.
- Running away or climbing in inappropriate places (in adults, feeling internal restlessness).
- Inability to play quietly.
- Behavior “as if driven by a motor”.
- Excessive talking.
- Blurting out an answer before the question has been completed.
- Unable to wait in queue.
- Interrupting others' conversations or activities.

5.3 Age-related differences

- Preschool age: hyperactivity dominant; attention short in play; behavioral control in social situations difficult.
- School age: Academic problems become prominent; inattention affects academic achievement.
- Adolescence: motor hyperactivity decreases, inner restlessness and impulsivity remain; risk behavior increases.
- Maturity: Internal distress, difficulty with planning, impulsivity in relationships; functional impairment in work and family life.

6. Diagnosis

6.1 Unified diagnostic criteria (DSM-5-TR · ICD-11 · NICE NG87 · AAP 2019 consensus points)

A. Of inattention and/or hyperactivity-impulsivity symptoms: in children (≤16 years) at least 6 symptoms in each category, adults (17+) at least 5 symptoms; must have lasted at least 6 months and be at a level not expected for age.

B. Several symptoms must have been present before age 12.

C. Symptoms in at least two settings (e.g., home + school, home + work) should be observed.

D. Functional impairment — significant impact on social, academic, occupational activity.

E. Exclusions — does not occur in context of schizophrenia or other psychotic disorder; not fully explained by another psychiatric disorder.

6.2 Differences between sources

- DSM-5-TR (F90.0–F90.2x): Three presentations are recognised — predominantly inattentive, predominantly hyperactive-impulsive, and combined. Severity is graded as mild, moderate, or severe. The age-of-onset criterion has been raised to 12 years (DSM-IV required onset before age 7).
- ICD-11 (6A05): Parallel structure; ‘co-morbid’ qualifiers (autism, language disorder, intellectual disability).
- NICE NG87 (2018, updated 2019): the diagnosis should be made by a multidisciplinary team (in children — pediatrician/psychiatrist; in adults — psychiatrist); structured interview and observation combined.
- AAP 2019 (Wolraich M.L. et al. Pediatrics 2019;144(4)): Diagnostic and treatment algorithm for ages 4–18 at the first medical assistance level 4–5 years — first-line parent training (PCIT, parent management training), second-line methylphenidate; 6–11 years — first-line stimulant + behavioral intervention; 12–18 years — first line stimulant + behavioral intervention (with adolescent consent).
- AACAP Practice Parameter (Pliszka S. JAACAP 2007): Comorbidity assessment and treatment sequencing.
- CADDRA Canadian ADHD Practice Guidelines (4th edition, 2018): Comprehensive algorithm for adult ADHD.

6.3 Diagnostic algorithm

1. Parent and teacher (for children) or patient themselves (for adults) interview — symptom history, onset age, contexts.
2. Standardized rating scales — Conners-3 (parent/teacher/patient), Vanderbilt (AAP recommendation), SNAP-IV; for adults ASRS (Adult ADHD Self-Report Scale, WHO/Kessler).
3. Clinical interview — confirmation of symptoms in two contexts, documentation of functional impairment.
4. Medical and neurological examination — excluding somatic causes (thyroid, hearing, vision, anemia).

5. Intellectual and academic assessment (WISC-V, academic tests) — to differentiate learning disorder and intellectual disability.
6. Comorbidity screening — ODD, conduct disorder, anxiety, depression, tic, autism, substance use.
7. Sleep disturbance and obstructive sleep apnea screening (especially in hyperactive-impulsive type).

6.4 Differential diagnosis

Condition	Distinguishing features
Age and development — normal variation	No functional impairment, or not observed in two contexts.
Oppositional Defiant Disorder (6C90)	Defiance, anger-based; comorbidity is high.
Conduct Disorder (6C91)	Serious behavioral disturbances, violation of others' rights.
Anxiety disorders (6B0x)	Inattention due to anxiety; somatic complaints; onset generally late.
Depressive disorder (6A7x)	Affective symptoms are dominant; episodic course.
Bipolar disorder (6A60–62)	Affective episodes; no manic episode in childhood; see also Disruptive Mood Dysregulation Disorder (DMDD).
Autism spectrum disorder (6A02)	Social-communication deficit, restricted-repetitive behaviors. Comorbidity high.
Disorder of intellectual development (6A03)	Academic domain-specific deficit; attention problems are context-dependent.
Obstructive sleep apnea	Daytime sleepiness, behavioral problems; polysomnography.
Hyperthyroidism	Decreased TSH; tremor, weight loss.
Absence (petit mal) epilepsy	Brief consciousness changes; EEG 3 Hz spike-and-wave.
Hearing impairment	Audiometry impaired.

7. Examination and assessment

7.1 Clinical scales

- Vanderbilt ADHD Diagnostic Rating Scale — parent and teacher, ages 6–12 — AAP 2019 recommendation.
- Conners-3 — complex CDDP and comorbidity assessment (6–18 years).
- SNAP-IV (Swanson, Nolan, Pelham) — 26 questions.
- ASRS (Adult ADHD Self-Report Scale, WHO/Kessler R.C.) — 18 questions; brief screening in adult patients.
- DIVA-5 (Diagnostic Interview for ADHD in adults) — structured interview.
- BRIEF-2 (Behavior Rating Inventory of Executive Function) — executive function.

7.2 Laboratory investigations

No specific laboratory indication for routine ADHD. Based on clinical suspicion:

- Complete blood count, ferritin (iron deficiency), thyroid function (TSH).
- Lead level — if risk factors are present.
- Toxicology screening — suspicion of substance use in adolescents.
- Before stimulant initiation — EKG only if cardiac risk history or family history (AAP, AHA 2008; routine EKG not required).

7.3 Instrumental investigations

- EEG — only if seizure suspected or atypical clinical manifestations.
- Brain MRI — only for focal neurological signs. No routine indication.
- Neurophysiological tests for diagnosis (TOVA, IVA, QbTest) lack sufficient evidence base — the clinical diagnosis and standardized scales are prioritized as the gold standard.

8. Treatment

8.1 General Principles (NICE NG87 · AAP 2019 · CADDRA 2018 Consensus)

1. Age-based approach (AAP 2019):
    - 4–5 years — first-line parent training (PCIT, Triple P, Incredible Years); second-line methylphenidate.
    - 6–11 years — first-line Stimulant + behavioral intervention.

- 12–18 years — first line Stimulant + behavioral intervention; adolescent consent.
- Adults — Stimulant or non-stimulant + CBT.

2. Multimodal intervention superior to monotherapy or behavioral intervention alone (MTA Cooperative Group. Arch Gen Psychiatry 1999, n=579 — combination superior to monotherapy, but pharmacotherapy stronger than behavioral intervention alone).
3. Stimulants — first-line pharmacotherapy (Cortese S. et al. Lancet Psychiatry 2018 network meta-analysis; in children methylphenidate first choice, in adults amphetamine more effective).
4. Non-stimulants — atomoxetine, guanfacine (long-acting formulation), clonidine (long-acting formulation). Used when stimulant contraindicated or side effect present.
5. Behavioral intervention — parent training (PCIT, parent management training), school-based intervention (class modifications, behavior contract), child CBT.
6. Academic accommodations — individualized education plan (IEP / 504 Plan US; EHCP UK); extra time, limited distractors, written vs oral response options.
7. Side effect monitoring — height and weight, blood pressure, heart rate (3-month), sleep, appetite, mood. In stimulants, slight decrease in height growth (1–2 cm with long-term use) is expected.
8. Comorbidity treatment — anxiety/depression (SSRI + CBT), tics (alpha-2 agonists or cautiously stimulants), substance use (lisdexamfetamine or atomoxetine — lower abuse potential), autism (atypical antipsychotic for comorbid irritability).

8.2 Stimulants (First-Line Pharmacotherapy)

Drug	Form	Dose range	Note
Methylphenidate IR	Short-term (3–4 hours)	5–60 mg/day (2–3 doses)	Initiation of titration in children; Cortese 2018 — first-line in children.
Methylphenidate ER (Concerta, Ritalin LA)	Long-term (8–12 hours)	18–72 mg/day	Once-daily dose; convenient for school day.

Drug	Form	Dose range	Note
Lisdexamfetamine (Vyvanse)	Prodrug, 12+ hours	20–70 mg/day	Low abuse potential (prodrug); preferred in adults.
Mixed Amphetamine Salts (Adderall XR)	Long-term (10–12 hours)	5–30 mg/day	In adults, Cortese 2018 — First-line.
Dexamfetamine	IR (4 hours) / ER (8 hours)	5–40 mg/day	

8.3 Non-stimulants

Drug	Mechanism	Dose	Note
Atomoxetine (Strattera)	Noradrenaline reuptake inhibitor	0.5–1.4 mg/kg/day	Effect 4–6 weeks; slight increase in risk of suicidal ideation (FDA black box).
Guanfacine ER (Intuniv)	α2A adrenergic agonist	1–7 mg/day	Sedation, hypotension; beneficial in comorbid tic cases.
Clonidine ER (Kapvay)	α2 adrenergic agonist	0.1–0.4 mg/day	Sedation; beneficial when sleep disorder is comorbid.
Viloxazine ER (Qelbree)	NRI + serotonergic modulator	children 100–400 mg/day, adults 200–600 mg/day	FDA 2021; children from 6 years and adults.

8.4 Source-Specific Clarifications

- NICE NG87 (2018, updated 2019): 5+ years — environmental changes first; then behavioral or pharmacotherapy. In adults — first-line stimulant; second-line atomoxetine.
- AAP 2019 (Wolraich): Age-based algorithm described above.

- Cortese S. et al. Lancet Psychiatry 2018 (network meta-analysis): children — methylphenidate first-line; adults — amphetamine first-line (higher effect size); atomoxetine moderately effective.
- MTA Cooperative Group (Arch Gen Psychiatry 1999, follow-up Hechtman L. et al. JAACAP 2016): intensive pharmacotherapy combination demonstrated superiority in 14-month observation; but differences weaken at 36-month and 8-year follow-up — long-term effect debated.
- FDA and EMA monitoring requirements: Monitoring height and weight, baseline cardiovascular parameters.

8.5 Treatment methods

1. Parent-Child Interaction Therapy (PCIT) — Eyberg (Eyberg S.) — For children aged 4–7 years; two phases – Child-Directed Interaction (CDI) and Parent-Directed Interaction (PDI). Development of parental behavior management skills with real-time therapist coaching (in-vivo coaching). pcit.org.
2. "Incredible Years" program — Webster-Stratton (Webster-Stratton C.) — Parent, teacher, and child programs; group format 14–22 sessions; ASTPS model (Authoritative Stimulating Trustworthy Parenting Style).
3. Triple P — Positive Parenting Program — Sanders (Sanders M.) — The 5-tier system developed in Australia is universal, selective, targeted, and individual. Effective in numerous RCTs.
4. Parent Management Training — Barkley (Barkley) — 8–12 sessions behavior principles-based parent training – token system, time-out, continuous training.
5. CBT for adult ADHD (Safren CBT for Adult ADHD) — Safren (Safren S.A.) — CBT modules for adult ADHD — task organization, planning, distraction management, cognitive restructuring. Evidence: Safren S.A. et al. JAMA 2010 RCT.
6. Vanderbilt ADHD Diagnostic Rating Scale — Recommended by AAP 2019; parent and teacher forms, comorbidity subscales.
7. Adult ADHD Self-Report Scale (ASRS) — Kessler (Kessler R.C.), WHO — 18 questions; short 6-item screening section. Clinical and population screening in adult patients.
8. Diagnostic Interview for ADHD in Adults (DIVA-5 — Diagnostic Interview for ADHD in Adults) — Structured interview for adult ADHD — according to DSM-5 criteria.
9. Conners-3 — 6–18 years comprehensive assessment – ADHD symptoms, executive function, behavioral disorder, learning.

10. Behavior Rating Inventory of Executive Function (BRIEF-2 — Behavior Rating Inventory of Executive Function) — Parent and teacher assessment of executive function — inhibition, working memory, emotional regulation.
11. EEG Neurofeedback — In ADHD, neurofeedback training targeting specific brainwave patterns (theta/beta ratio). Evidence: Holtmann M. et al. Dev Med Child Neurol 2011 meta-analysis — small effect size; effect debated among field specialists. NICE NG87 — not first-line, adjunct status.

9. Prognosis

Good prognostic factors

- Early identification and intervention.
- High IQ — compensatory strategies.
- Stable and supportive family environment.
- Active treatment of comorbid disorders.
- Good response to stimulant and behavioral intervention.
- Maintaining adherence in adolescence and adulthood.

Poor prognostic factors

- Comorbid conduct disorder, oppositional defiant disorder.
- Low socioeconomic support.
- Development of substance use in adolescence.
- Severity of executive function deficit.
- Late identification.
- Intervention with unproven methods.

Follow-up targets

- Symptom and functional outcome monitoring (3–6 months).
- Stimulants: height, weight, blood pressure, pulse (3-month).
- Comorbid anxiety, depression, substance use screening.
- Academic and vocational achievement support.
- Intervention for risk behavior in adolescence (driving training, sexual health, substance).
- Adaptation in adulthood related to career, relationships, financial planning — CBT adapted to ADHD.

10. Myths and misconceptions

10.1 Aetiology myths

Myth 1: "ADHD is a fabricated diagnosis; children are just lively"

Why it is widespread: Anxiety about normal child energy being misperceived as pathological; some cases of diagnostic inflation are noted.

Clinical and biological rationale: ADHD has persistent and measurable neurobiological features — heritability 74% (Faraone & Larsson 2019), brain structural (Hoogman ENIGMA-ADHD 2017) and functional differences; effect of stimulants on specific mechanism (dopamine-norepinephrine) is pharmacological evidence. Underdiagnosis (especially in women) is a more widespread problem than overdiagnosis.

Evidence: APA, AAP, WHO, NIMH — ADHD is a biologically based disorder; parent-reported US prevalence is 9.8% at ages 3–17 (2016–2019) and 11.4% (2022), international population studies meta-analysis 5–7% (Polanczyk 2007).

Real clinical step: accurate diagnosis with structured criteria; functional impairment required (not simple behavioral variation).

Myth 2: "Sugar causes ADHD or worsens symptoms"

Why it is widespread: the intuition that sugar increases child energy; 'Feingold diet' popularized in the 1970s.

Evidence: Wolraich M.L. et al. JAMA 1995 systematic review and meta-analysis — sugar has no effect on child behavior and cognitive performance. Wolraich M.L. NEJM 1994 RCT — controlled deprivation study.

Real clinical step: A general healthy diet, but with sugar restriction, is not recommended as an ADHD intervention.

Myth 3: "Bad parenting causes ADHD"

Evidence: heritability 74% (Faraone & Larsson Mol Psychiatry 2019); parental experience not etiology. However, high-conflict environment may influence symptom severity. Parental training (PCIT, Triple P) effective because it strengthens behavioral management skills — not blaming, but supportive purpose.

Myth 4: "Screen time / video games cause ADHD"

Evidence: Association exists, but causal direction debated — children with ADHD engage more in screen activity (reverse causality); Ferguson C.J. Perspect Psychol

Sci 2015 review — causative effect of video games on ADHD not proven. AAP — screen time limits recommended for general health, not as a ‘cause’ of ADHD.

10.2 Harmful “treatment” methods and misconceptions

Myth 5: “Stimulants cause addiction like drugs and lead to future substance abuse”

Why it is widespread: methylphenidate and amphetamines being listed on the controlled substances schedule (Schedule II in the US); the widespread prevalence of the “narcotic” fear.

Clinical and biological rationale: At therapeutic oral dose, stimulant gradually reaches brain concentration (1–2 hours) — euphoric effect minimal. With intravenous or intranasal use (routes of misuse), different pharmacokinetics occur.

Evidence: Wilens T.E. et al. Pediatrics 2003 meta-analysis, n=1034 — substance use in adulthood among adolescents with ADHD treated with stimulants diminishes, does not increase (risk reduction ~50%). Lisdexamfetamine and osmotic-release methylphenidate (Concerta) — low abuse potential (prodrug structure, OROS technology).

Real clinical step: When comorbid substance use exists — lisdexamfetamine or atomoxetine preferred; safety education for the patient and the family.

Myth 6: “Stimulants stunt a child's height and brain development”

Evidence: MTA Cooperative Group follow-up (Swanson J.M. et al. JAACAP 2017) — adult height 1–2 cm lower in children treated with stimulants (relative to expected young adult height); this is considered small and clinically insignificant. Brain development — no evidence of harm related to stimulants (Schweren L.J.S. et al. Neuropsychopharmacology 2015 review).

Myth 7: “Cannabis cures ADHD”

Evidence: NIDA, NICE — cannabis not approved for ADHD treatment; increases psychosis risk (Marconi 2016), leads to cognitive impairment, reduces motivation. Subjective ‘calming’ effect masks functional impairment.

10.3 Ineffective or scientifically unfounded methods

Myth 8: “Feingold diet or avoidance of food colorings/additives cures ADHD”

Evidence: Pelsser L.M. et al. Lancet 2011 (INCA study) — a restrictive elimination diet may influence behavior in some children (~30%), but systematic reviews (Sonuga-Barke E.J. et al. Am J Psychiatry 2013 meta-analysis) — the effect size of eliminating food additives is small, showing a superior effect only on family-rated assessment, and weakens when a blind control is applied. AAP, NICE — not a first-line approach; there is a risk of nutritional deficiency with complex diets.

Myth 9: “Omega-3 supplements are the primary treatment for ADHD”

Evidence: Cooper R.E. et al. J Psychopharmacol 2015 and Bloch M.H., Qawasmi A. JAACAP 2011 meta-analyses — small effect size ($g \approx 0.3$); significantly weaker than stimulants ($g \approx 1.0$). May serve as adjunct; not primary treatment.

Myth 10: “EEG Neurofeedback replaces stimulants”

Evidence: Holtmann M. et al. Dev Med Child Neurol 2011 and Cortese S. et al. JAACAP 2016 meta-analysis — neurofeedback effect significantly decreases when rated by proband and teacher; effect size is small in “blinded” assessment. NICE NG87 — not first-line. Stimulant effect is far superior.

Myth 11: “Forcing exercise resolves the child's ADHD”

Evidence: Physical activity may help mildly reduce symptoms in ADHD (adjunct); but does not replace core intervention. AAP recommends structured physical activity, but not as sole treatment.

Myth 12: “Craniosacral therapy, chiropractic, homeopathy cure ADHD”

Evidence: Cochrane and systematic reviews — the effect of these complementary interventions for ADHD has not been proven.

Myth 13: “Light therapy, MUS tinted lenses cure ADHD”

Evidence: No evidence base. AAP, AAO position — not evidenced.

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ICD-11 **6A06**	**6A06 STEREOTYPED MOVEMENT DISORDER** **Stereotyped movement disorder**
ICD-10 F98.4	Stereotyped movement disorders
DSM-5-TR F98.4	Stereotypic Movement Disorder

1. Definition and nosology

Stereotyped movement disorder (ICD-11: 6A06; DSM-5-TR: F98.4 Stereotypic Movement Disorder) — characterized by seemingly voluntary, repetitive, rhythmic, purposeless motor behavior, interfering with daily activity or causing bodily injury, a neurodevelopmental disorder. Typical examples: hand flapping, body rocking, head banging, hair pulling, lip biting, finger knuckle play.

The disorder is classified into two forms:

- Primary (primary, idiopathic) — in a child with normal IQ, without other neurodevelopmental or somatic disorder;
- Secondary — intellectual disability, autism spectrum disorder, sensory deprivation (visual/hearing impairment) or against a medical condition background.

DSM-5-TR — with severity specifier — Non-suicidal self-injury or without it.

2. History

- Kanner L. (1943) — in the description of autism, stereotyped behaviors are noted as the restricted-repetitive behaviors category.
- 1960s–1970s — stereotypy descriptions in institutional populations regarding intellectual disability.
- DSM-III-R (1987) — Stereotypy/Habit Disorder as a separate category.
- Mahone E.M. et al. J Child Neurol 2004, 2014 — systematically described the syndrome of “primary complex motor stereotypies” in children with

intact cognitive development; prior to this work, such children were often misdiagnosed as autism spectrum disorder.

- DSM-5 (2013) and ICD-11 (2019) — included in the category of neurodevelopmental disorders; add self-harm qualifier.

3. Epidemiology

- Transient stereotypies observed in 3–15% of healthy children up to 3 years of age (Foster L.G. J Dev Behav Pediatr 1998).
- Primary clinical stereotypic movement disorder — approximately 3–4% in children with normal cognitive development (Freeman R.D. et al. Mov Disord 2010).
- Secondary forms — up to 60% in intellectual disability; ~80% of patients with severe and profound intellectual disability have some form of stereotypy; almost always present in some form in autism spectrum disorder (Bodfish J.W. et al. J Autism Dev Disord 2000).
- Sex: in initial forms, males 3:1; in secondary forms difference smaller.
- Age of onset: typically up to age 3 (DSM-5-TR criterion).
- Self-harm (self-injurious behavior, SIB): rare in the general population, but occurs in 25–30% of severe intellectual disability, and almost always in syndromes such as Lesch-Nyhan or Cornelia de Lange.

4. Aetiology and pathogenesis

4.1 Neurobiological mechanisms

- Basal ganglia and cortex-striatum circuit dysfunction — habit selection and motor inhibition process impairment (Singer H.S. Semin Pediatr Neurol 2009).
- Dopaminergic system — D1 and D2 receptor imbalance; dopamine agonists enhance stereotyped behavior in experimental models.
- Serotonergic system — some patients show partial response to SSRI.
- Opioid system — self-harm forms under endogenous opioid dysregulation hypothesis (naltrexone shows partial effect).

4.2 Genetic and family factors

- In primary stereotypy, familial clustering is noted — family history 25–30% (Mahone 2014).

- Stereotypy is highly prevalent in specific genetic syndromes — Rett (MECP2), Lesch-Nyhan (HPRT1), Cornelia de Lange (NIPBL), Smith-Magenis (RAI1), Fragile X (FMR1), Phelan-McDermid (22q13).

4.3 Environmental and behavioral factors

- Sensory deprivation — institutional environment, high frequency in blind or deaf children.
- Behavioral mechanisms — self-stimulation (sensory input seeking), automatic reinforcement (intrinsic reinforcement), context-dependent functional connection (avoidance, attention).
- Stress and anxiety disorders may intensify stereotypic severity.

5. Clinical features

5.1 Typical motor patterns

- Hand flapping — most frequent; during emotional excitement or concentration.
- Body rocking — sitting or standing.
- Head banging — a form of self-harm; against a pillow, wall or floor.
- Body hugging, finger tapping, lip biting, hair pulling.
- Visual stimulation — hand-waving before eyes, gazing at light.
- Self-injurious behavior (SIB) — head banging, biting, hitting, self-injury with teeth.

5.2 Clinical features

- Duration — from seconds to hours; 5–10 minutes per episode typical.
- Triggers — excitement (pleasant or stressful), concentration, inactivity (free time).
- Attention termination — behavior stops when patient is called or redirected to another activity (unlike tics — no premonitory urge).
- Functional role of stereotypy — self-stimulation, calming, sensory stimulation management.

5.3 Comorbidity

- Intellectual disability (in secondary forms).
- Autism spectrum disorder (comorbid 30–50% in primary stereotypy — Mahone 2014).

- ADHD — comorbid in 20–35% of children with primary stereotypies.
- Anxiety and OCD-like symptoms.
- Tic disorders — differentiation may be difficult.

6. Diagnosis

6.1 Unified diagnostic criteria (DSM-5-TR · ICD-11 consensus points)

A. Repetitive, non-goal-directed, apparently voluntary motor behavior (e.g., hand flapping, body rocking, head banging, self-biting).

B. The behavior interferes with social, academic, or other activities or leads to self-harm.

C. Onset in early developmental period (typically by age 3).

D. Not fully explained by a substance (e.g., stimulant) or neurological condition (tic, choreoathetosis, dystonia, dermatillomania, trichotillomania); not better explained by OCD, ASD, or another mental disorder (comorbidity not excluded).

6.2 Differences between sources

- DSM-5-TR (F98.4): severity qualifier — with / without self-harm; additional qualifiers — with known medical/genetic condition; on the background of intellectual disability or autism.
- ICD-11 (6A06): Parallel structure; emphasis on ‘non-purposeful repetitive motor behavior’.
- Mahone E.M. et al. J Child Neurol 2014: “Primary Complex Motor Stereotypies” (PCMS) — attention to a distinct clinical form in children with normal cognitive development; typical hand/arm stereotypies; risk of being confused with autism.

6.3 Diagnostic algorithm

1. Parent interview and video documentation (family-recorded episodes via phone are diagnostically valuable).
2. Developmental and neurological examination.
3. Tics and other movement disorders (chorea, myoclonus, dystonia) differential diagnosis — neurologist consultation.
4. Comorbidity screening — autism spectrum (M-CHAT-R/F, ADOS-2), intellectual disability (Vineland-3, IQ), ADHD, anxiety.
5. Excluding sensory impairment — hearing and vision test.
6. If self-harm exists — functional behavior assessment (FBA).

7. Specific clinical suspicion based on — genetic testing (CMA, FMR1, MECP2, Lesch-Nyhan HPRT activity and uric acid).

6.4 Differential diagnosis

Condition	Distinguishing features
Tic disorders (8A05)	Brief, non-rhythmic, anticipatory premonitory sensation; supratentorial possible but difference is 'itching' sensation under the skin.
Autism spectrum (6A02)	Stereotypy may be comorbid; social-communication deficit is primary.
OCD — compulsions (6B20)	Ego-dystonic; response to obsession; meaningful (cleaning, symmetry).
Myoclonus, choreoathetosis	Abrupt, non-rhythmic, involuntary; no focal signs on neurological examination.
Trichotillomania (6B25.0), dermatillomania (6B25.1)	Hair pulling/skin picking — in category of body-directed repetitive behaviors; separate from general stereotypy.
Seizure (epileptic)	Alteration in consciousness and/or postictal state; EEG.
Stimulant-induced stereotypy	Associated with high-dose amphetamine or cocaine; toxicological screening.
Transient childhood stereotypies (healthy)	No functional impairment; resolves by age 3; does not require clinical diagnosis.

7. Examination and assessment

7.1 Clinical interview and scales

- Stereotypy Severity Scale (SSS) — Miller J.M., Singer H.S. et al.
- Repetitive Behavior Scale-Revised (RBS-R) — in comorbid cases of autism and intellectual disabilities.
- Video recordings — home episode logs conducted by family assessed by clinician.

- Functional behavior analysis (FBA) — for intervention planning in self-harm or context-dependent stereotypy.

7.2 Laboratory investigations

No routine indication. Based on clinical suspicion:

- Genetic panel (CMA, Fragile X) — if intellectual disability or syndromic features present.
- MECP2 (Rett in girls).
- HPRT activity and uric acid — suspected Lesch-Nyhan syndrome (self-biting).
- Thyroid, lead levels.
- Toxicology screening — in adolescents.

7.3 Instrumental investigations

- EEG — if seizure is suspected.
- Brain MRI — focal neurological signs or regression.
- Hearing and vision test — to assess the role of sensory deprivation.

8. Treatment

8.1 General principles

1. Behavioral intervention first-line — individualized plan based on functional behavior analysis.
2. In primary stereotypy — if the patient does not experience functional difficulty, observation may suffice. (Mahone 2014). Explanation of nature and prognosis to the family; no mandatory intervention required.
3. When functional difficulty or interference with social functioning is present — HRT (Habit Reversal Training), DRO (Differential Reinforcement of Other behavior), RIRD (Response Interruption and Redirection) evidence-based behavioral interventions.
4. In self-injurious behavior — multidisciplinary approach; behavioral interventions + protective devices (padded helmet) + pharmacotherapy (in refractory cases).
5. Sensory deprivation — enrichment of the environment (toys, social interaction), alternative activities providing sensory input to the patient.
6. Comorbidity treatment — autism, intellectual disability, ADHD, anxiety.

7. Pharmacotherapy has limited evidence base — only in severe cases, unresponsive to behavioral intervention; self-harm or in comorbid context of autism/intellectual disability.

8.2 Pharmacotherapy (Limited Evidence)

- Atypical antipsychotic (risperidone, aripiprazole) — for behavioral aggression and self-harm on the background of autism/intellectual disability; FDA approval — risperidone 5–16 years ASD irritability (including self-harm), aripiprazole 6–17 years ASD irritability. No FDA-approved medication for "core" symptoms of stereotypic movement disorder.
- SSRIs — fluoxetine, sertraline – show modest effect in some patients, especially in comorbid anxiety cases; evidence base limited.
- Naltrexone — considered in self-injurious behaviour (based on the endogenous opioid hypothesis); RCT evidence limited, Symons F.J. et al. Am J Ment Retard 2004 review — small effect size.
- Clonidine, guanfacine — it may help in comorbid ADHD or sleep disorder.

8.3 Special Conditions

- Self-harm (SIB): emergency behavioral analysis; protective measures (helmet, without arm restraints) short-term; intensive behavioral intervention; pharmacotherapy for refractory cases; hospitalization in severe cases.
- Specific genetic syndromes — Self-biting in Lesch-Nyhan almost always present; oral-dental compromise (dental extraction last resort); allopurinol for metabolic condition.
- Sensory deprivation — environmental change, alternative sensory input.

8.4 Source-Specific Clarifications

- Mahone E.M. et al. J Child Neurol 2014: in initial forms, aggressive intervention not required; explanation of prognosis and nature to family; HRT effective in adolescence.
- AAP — self-harm protocol: in children with intellectual disability multimodal approach.
- NICE NG11 (Challenging Behaviour and Learning Disabilities, 2015): in intellectual disability behavioral intervention first line; antipsychotic only in refractory cases and short-term.

8.5 Treatment methods

1. Habit Reversal Training (HRT — Habit Reversal Training) — Azrin (Azrin N.H.), Nunn (Nunn R.G.) — Five core components — awareness training (behavioral recognition), competing response training, motivational component, social support, generalization training. Evidence-based in stereotypic movement disorder and tic disorders. Evidence: Specht M.W. et al. J Child Neurol 2017 RCT — reduction in severity in primary complex motor stereotypies.
2. Differential Reinforcement of Other Behavior (DRO) — Positive reinforcement (token, praise, activity) of time intervals without stereotypies. Effective in intellectual disability context. A core component of ABA.
3. Response Interruption and Redirection (RIRD) — Interruption when stereotypic behavior begins and redirecting to an alternative engaging task. Used for vocal stereotypies and severe cases. Ahearn W.H. et al. J Appl Behav Anal 2007.
4. Functional Behavior Assessment (FBA) — Systematic analysis of behavior antecedent (precondition), functional consequence, and context — basis for individualizing intervention plan. Within ABA and PBS.
5. Stereotypy Severity Scale (SSS) — Miller (Miller J.M.), Singer (Singer H.S.) — Standardized assessment of stereotypy frequency, duration, intensity, and interference level.
6. Repetitive Behavior Scale-Revised (RBS-R — Repetitive Behavior Scale-Revised) — Bodfish (Bodfish J.W.) et al — 43-item parent questionnaire – stereotyping, self-harm, compulsive, ritualized, insistence on sameness, restricted behavior subscales. Used in comorbid autism and intellectual disability cases.

9. Prognosis

Good prognostic factors

- Primary form, normal IQ.
- No self-harm.
- Early HRT intervention response.
- Absence of comorbidity.
- Family support and realistic expectations.

Poor prognostic factors

- Secondary form — within the context of severe intellectual disability or genetic syndrome.
- Self-injurious behavior — especially in Lesch-Nyhan, Cornelia de Lange.
- Comorbid autism spectrum disorder.
- Non-response to behavioral intervention.

Follow-up targets

- Frequency, duration, intensity of stereotypy (SSS) — 3–6 months.
- Functional impairment and self-harm monitoring.
- Comorbidity (autism, ADHD, anxiety, OCD) screening.
- Social adjustment support in adolescence and adulthood.
- In those receiving pharmacotherapy — monitoring for side effects (metabolic, EPS).

10. Myths and misconceptions

10.1 Aetiology myths

Myth 1: “Stereotypy is always a sign of autism”

Why it is widespread: Stereotypies are included in DSM-5 autism criteria and primarily associated with autism in mass media.

Clinical and biological rationale: Mahone E.M. et al. J Child Neurol 2004, 2014 — the syndrome of “Primary Complex Motor Stereotypies” can also occur in children with normal cognitive development without social reciprocal interaction deficit or restricted-repetitive behavior pattern. 30–50% of these children may have comorbid autism or ADHD, but most do not.

Evidence: Mahone 2014; Freeman R.D. et al. Mov Disord 2010 — primary stereotypy is a distinct clinical entity from autism.

Real clinical step: each child with stereotypical behavior should be separately evaluated for autism spectrum as differential; if no comorbidity, family should not be alarmed.

Myth 2: "Stereotypy is a result of poor upbringing or emotional deprivation"

Why it is widespread: In the 1960s, observations of high prevalence in institutionalized children strengthened the 'deprivation' theory.

Evidence: Bodfish J.W. et al. J Autism Dev Disord 2000 — stereotypy is a neurobiologically based disorder, with dysfunction of the basal ganglia and cortex-striatum circuit. Sensory deprivation can affect severity, but is not etiology. It is also observed in children growing up in healthy, supportive families.

Myth 3: "The child will outgrow it with age — no action is needed"

Evidence: Transient childhood stereotypies resolve by age 3; however primary clinical stereotypy typically progresses — Mahone 2014 follow-up studies have documented this up to adulthood. The family should be provided with a realistic prognosis; intervention is required if self-harm or functional impairment exists.

10.2 Harmful or erroneous methods

Myth 4: "Physical restraint of a child (tying arms, stopping movement) resolves stereotypies"

Why it is widespread: intuitively, physically holding or restraining the child appears to eliminate the symptom.

Clinical logic and evidence: Continuous physical restraint is counterproductive — increases patient distress, impairs autonomy, causes skin injury and contractures. NICE NG11 (2015), AAP — restraint only for urgent and short-term self-harm situations for protective purposes; not recommended as continuous treatment. Functional behavioral analysis-based positive behavioral intervention (PBS) superior.

Myth 5: "Punishment (sharp redirection, scolding, physical punishment) resolves stereotypy"

Evidence: NICE NG11 — punishment-based approach is not effective in reducing behavior; creates risk of emotional harm and trauma. ABA practice in modern form is based on positive reinforcement (DRO) instead of punishment.

Myth 6: "Antipsychotics are effective for all stereotypic movements"

Evidence: Antipsychotics (risperidone, aripiprazole) FDA-approved for irritability in ASD — including self-harm. No FDA approval for primary stereotypy; effect on 'core' stereotypy limited. Side effect burden (metabolic, EPS, prolactin) significant. NICE NG11 — antipsychotic only for refractory severe behavior and short-term.

10.3 Ineffective or scientifically unfounded methods

Myth 7: “Sensory integration therapy (Ayres SI) cures stereotypy”

Evidence: AAP 2012 Sensory Integration Statement — therapeutic effect classified as ‘research ongoing’; specific effect evidence for stereotypical behavior is weak. Functional behavioral intervention (HRT, DRO) is superior in evidence.

Myth 8: “Hyperbaric oxygen chamber, stem cell injections, chelation resolve stereotypy”

Evidence: Cochrane and FDA warnings (same position in the context of autism) — these interventions have no effect on stereotypy or neurodevelopmental disorder, and the risk profile is significant.

Myth 9: “Special diet (gluten-free, casein-free) eliminates stereotypies”

Evidence: Hyman S.L. et al. AAP 2020 — effect of special diet for stereotypy or autism not proven.

Myth 10: “EEG Neurofeedback resolves stereotypies”

Evidence: No evidence base for neurofeedback in stereotypical movement disorder. Mild effect in ADHD, but extrapolation to stereotypical behavior is unfounded.

Myth 11: “Homeopathy, acupuncture, craniosacral therapy”

Evidence: none have proven efficacy for stereotypical movement disorder.

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ICD-11 **8A05.0**	**8A05.0 PRIMARY TICS OR TIC DISORDERS** **Primary tics or tic disorders**
ICD-10 F95.2	Combined vocal and multiple motor tic disorder [de la Tourette]
DSM-5-TR F95.2	Tourette's Disorder (and other tic disorders F95.0–F95.1)

1. Definition and nosology

Primary tics or tic disorders (ICD-11: 8A05.0; DSM-5-TR: F95.0–F95.2) — a group of neurodevelopmental disorders characterized by sudden, rapid, recurrent, nonrhythmic, involuntary or semi-voluntary motor movements and/or vocal/sound

productions. Tics are typically accompanied by a premonitory urge (an urge or tension) and can be voluntarily suppressed for a short period.

Three main categories in ICD-11 and DSM-5-TR:

- Tourette syndrome (DSM-5-TR F95.2) — multiple motor tics and one or several vocal tics lasting ≥1 year, onset before age 18
- Persistent (chronic) motor or vocal tic disorder (DSM-5-TR F95.1) — motor or vocal (but not both) ≥ 1 year;
- Transient tic disorder (DSM-5-TR F95.0) — tic (motor and/or vocal) < 1 year, onset up to 18 years.

2. History

- Itard J.M.G. (1825) — the first clinical description of motor and vocal tics in Marquise de Dampierre.
- Gilles de la Tourette G. (1885) — Classical description of 9 patients at Salpêtrière hospital under Charcot's direction; name of the syndrome.
- 1960s — psychoanalytic explanation was dominant (‘neurotic disorder’).
- 1961 (Seignot J.N.) — the first clinical use of haloperidol revealed its effect in tics — basis for transition to neurobiological concept.
- DSM-III (1980) and subsequent revisions — formal status of tics as a medical disorder.
- AAN 2019 (Pringsheim T. et al. Neurology 2019;92(19)) — international evidence-based clinical guideline; emphasized that α2-agonists are first-line compared to antipsychotics due to more favorable side effect profile.
- ESSTS — European Society for the Study of Tourette Syndrome — European Clinical Guidelines (2011, updated 2022).

3. Epidemiology

- Tourette syndrome prevalence: in children 0.5–1% (Knight T. et al. systematic review Pediatr Neurol 2012).
- Persistent motor or vocal tic: ~1–2%.
- Transient tics: Up to 20% of school-age children.
- Sex: Higher by 3–4 times in males.
- Age of onset: 5–7 years; peak severity at 10–12 years.
- Persistence: In adulthood, tics significantly decrease in ~50%, persist in ~30%, and severe form remains in ~20% (Bloch M.H., Leckman J.F. J Psychosom Res 2009).

- Comorbidity: ADHD 50–60%, OCD 30–50%, anxiety 30%, depression 20%, learning disorder 20–30%, angry outbursts, sleep disorders.

4. Aetiology and pathogenesis

4.1 Genetic factors

- Heritability: 0.77 (Mataix-Cols D. et al. JAMA Psychiatry 2015 family and twin studies, n>200 000 Sweden) — among the most heritable psychiatric disorders.
- Polygenic nature: GWAS (Yu D. et al. Am J Psychiatry 2019) — risk loci identified; no single gene.
- Rare mutations: SLITRK1, HDC (histidine decarboxylase), CELSR3 — in a small number of families.
- High density of OCD and ADHD in family history — genetic overlap.

4.2 Neurobiological mechanisms

- Cortico-striato-thalamo-cortical (CSTC) circuit dysfunction — disruption of motor inhibition in basal ganglia and related regions (Leckman J.F. et al. Neuron 2000;28(2):349–354).
- Dopaminergic system hyperactivity — mechanism of tic reduction by dopamine antagonists (haloperidol, risperidone).
- Histaminergic system — HDC mutations rare; histaminergic tone low.
- Structural changes: Reduction in caudate volume, sensorimotor cortical thinning (Sowell E.R. et al. Nat Neurosci 2008).

4.3 Environmental factors (modifier)

- Preterm birth, perinatal hypoxia — moderately increases risk.
- Maternal tobacco use during pregnancy.
- Psychosocial stress — is not a cause for the onset of tics, but influences severity.
- PANDAS / PANS (Pediatric Autoimmune Neuropsychiatric Disorders Associated with Streptococcal infections / Pediatric Acute-onset Neuropsychiatric Syndrome) — rare; most cases are actually typical Tourette/OCD (see expanded §10).

5. Clinical features

5.1 Motor tics

- Simple motor tics: Eye blinking (most frequent onset), nose sniffing, head shaking, shoulder shrugging, lip play.
- Complex motor tics: Sequential movements — hopping, touching, spinning on the spot; rarely an obscene gesture (copropraxia, ~5%).

5.2 Vocal/phonic tics

- Simple vocal tic: coughing, sniffling, wheezing, throat clearing, shouting.
- Complex vocal: word, echolalia (repetition of others' words), palilalia (repetition of own words).
- Coprolalia (obscene word/phrase) — occurs in 19.3% of males and 14.6% of females with Tourette syndrome (Freeman R.D. et al. Dev Med Child Neurol 2009 international Tourette database, n=597), and differs from the portrayal in mass media.

5.3 Premonitory urge and voluntary suppression

Most patients experience a premonitory urge (somatic urge, tension) before tics; the tic temporarily relieves this sensation. Tics can be voluntarily suppressed for a short period (seconds to minutes), but this suppression creates internal tension and discomfort — the subsequent tic episode is usually more intense.

5.4 Course and variability

- tics “Waxing and waning” — severity waxes and wanes; one tic type replaced by another.
- Triggers — stress (positive or negative), excitement, fatigue, specific contexts (e.g., suppressing tics at school).
- Tics decrease during sleep, but do not completely disappear.
- Peak severity at ages 10–12; decreases in most patients during adolescence.

6. Diagnosis

6.1 Unified diagnostic criteria (DSM-5-TR · ICD-11 · AAN 2019 consensus points)

For Tourette syndrome:

1. Multiple motor tics and one or several vocal tics (occurring at different times, not required to be present simultaneously)

2. Tics persist for at least 1 year (they may vary over time).
3. Onset Up to 18 years.
4. Exceptions — not fully explained by substance (e.g., cocaine) or medical condition (Huntington's, postviral encephalitis).

Persistent motor or vocal tic disorder: Same criteria as Tourette, but single type tic (motor or vocal, not both).

Transient tic disorder: Same criteria as Tourette, but duration < 1 year.

6.2 Differences between sources

- DSM-5-TR: Three separate categories (F95.0, F95.1, F95.2). “Functional impairment” and “severity” were not included in the diagnostic requirement — merely meeting the criteria is diagnostically sufficient; however, the intervention decision is based on functional impairment.
- ICD-11 (8A05.0): “Primary tics or tic disorders” — sub-specifications: Tourette, persistent, provisional.
- AAN 2019 (Pringsheim T. et al.): the diagnosis is clinical, video recordings are an adjunct; YGTSS (Yale Global Tic Severity Scale) for baseline measurement and follow-up; comorbidity (ADHD, OCD) must be assessed.
- ESSTS 2022 (Müller-Vahl K.R. et al.): In the European context — same criteria; accepts CBIT as first-line non-pharmacological intervention.

6.3 Diagnostic algorithm

1. Parent and patient interview — onset age, course, triggers, premonitory feeling.
2. Video recordings — episodes captured at home by family (in clinic context tics often diminish)
3. Standardized scale— YGTSS (Yale Global Tic Severity Scale, baseline measures and monitoring).
4. Comorbidity screening — ADHD (Conners, Vanderbilt), OCD (CY-BOCS), anxiety, depression, learning disorder, sleep.
5. Neurological examination — focal signs, exclusion of choreoathetosis.
6. Specific clinical suspicion — Huntington's disease (HTT gene), Wilson's disease (ceruloplasmin, copper), neuroacanthocytosis.
7. PANDAS/PANS suspicion — only if classic clinical picture (rapid onset, OCD + tics + emotional lability post-streptococcal); routine antistreptolysin O tests are not recommended (NIMH 2017).

6.4 Differential diagnosis

Condition	Distinguishing features
Stereotyped movement disorder (6A06)	Rhythmic, longer-lasting (seconds-minutes), no premonitory urge; onset <3 years.
Myoclonus	Brief, sudden, involuntary, no premonitory sensation; neurological etiology.
Choreoathetosis, dystonia	Persistent involuntary movements; cannot be suppressed; neurological examination.
OCD (6B20) — compulsions	Ego-dystonic purposeful sequences; response to obsession. Comorbid is common.
Functional tic (functional tic-like behavior)	Rapid onset in adulthood, complex bizarre patterns, copying from mass media; increase observed during COVID-19 (Pringsheim T. et al. Mov Disord Clin Pract 2021)
PANDAS/PANS	Rapid onset post-streptococcal; OCD + tics co-occurrence outbreak; rare.
Seizure (epileptic)	Alteration in consciousness; EEG.
Wilson's disease	Hepatic and neurological symptoms; Kayser-Fleischer rings, decreased ceruloplasmin.
Huntington's disease	Typical onset in adulthood; chorea, cognitive decline, family history.

7. Examination and assessment

7.1 Clinical scales

- YGTSS (Yale Global Tic Severity Scale, Leckman J.F. et al. 1989) — separate for motor and vocal tics; frequency, intensity, complexity, interference, number on a 0–5 scale; global impairment 0–50. Baseline measures and treatment monitoring.

- PUTS (Premonitory Urge for Tics Scale).
- CY-BOCS — comorbid condition assessment.
- Conners-3, Vanderbilt — comorbid ADHD.

7.2 Laboratory investigations

No specific laboratory indication for routine tics. Based on clinical suspicion:

- Thyroid function (TSH) — in hyperthyroidism context.
- Ceruloplasmin, 24-hour urine copper — suspicion of Wilson's disease (especially hepatic or movement adjunct).
- HTT — suspected Huntington (family history).
- Streptococcal antibodies (ASO, anti-DNase B) — only if classic PANDAS clinical picture present; not recommended routinely.

7.3 Instrumental investigations

- EEG — only if seizure suspected.
- Brain MRI — focal neurological signs, atypical clinical picture.
- Routine neuroimaging for tics is not recommended.

8. Treatment

8.1 General principles (AAN 2019 · ESSTS 2022 consensus)

1. Intervention decision based on functional impairment — mild tics may be observed; aggressive intervention not required.
2. Psychoeducation for family and school — the first step — nature of tics, ‘waxing and waning’, premonitory urge.
3. Behavioral intervention — CBIT (Comprehensive Behavioral Intervention for Tics) AAN 2019 first-line recommendation (age 8+). HRT (Habit Reversal Training) + functional intervention + relaxation.
4. Pharmacotherapy — when functional impairment is present and CBIT is unavailable or insufficiently effective:
    - First-line: α2 adrenergic agonists — clonidine, guanfacine (ER) — side effect profile superior to antipsychotics; particularly beneficial in comorbid ADHD.
    - Second-line: Atypical antipsychotic — aripiprazole (FDA-approved for Tourette syndrome, ages 6–18, 2014), risperidone, ziprasidone. Stronger anti-tic effect, but metabolic and EPS side effects.

- Third-line: typical antipsychotic — haloperidol, pimozide (FDA approval for Tourette, but EPS and QTc prolongation risk).
- In refractory cases — VMAT2 inhibitors (tetrabenazine off-label; deutetrabenazine and valbenazine under investigation for tics), botulinum toxin for focal tics.

5. Comorbidity treatment — ADHD (stimulants do not significantly worsen tics – meta-analysis: Bloch M.H. et al. JAACAP 2009; but patients should be monitored), OCD (SSRI + CBT), anxiety, sleep.
6. Refractory severe in Tourette's — Deep Brain Stimulation (DBS) — thalamic centromedian-parafascicular or GPi (globus pallidus internus) targets; multidisciplinary selection in certified center (Schrock L.E. et al. Mov Disord 2015).

8.2 Pharmacotherapy — First-Line (α2-Agonists)

Drug	Form	Dose range	Note
Clonidine	IR / TDS (transdermal)	0.05–0.3 mg/day (divided or TDS)	Sedation, hypotension; beneficial in comorbid ADHD cases.
Guanfacine ER	Long-term	1–4 mg/day (once)	Less sedation, selective α2A; FDA-approved in ADHD.

8.3 Second-Line (Atypical Antipsychotic)

Drug	Dose range	Side effect profile
Aripiprazole	2–20 mg/day	Akathisia, sedation; metabolic better.
Risperidone	0.25–6 mg/day	Weight gain, prolactin elevation, EPS.
Ziprasidone	20–80 mg/day	QT prolongation, EKG monitoring.

8.4 Third-Line (Typical Antipsychotic)

- Haloperidol 0.5–4 mg/day — FDA approved for Tourette, but side effects: EPS, tardive dyskinesia, sedation.
- Pimozide 1–4 mg/day — FDA approval for Tourette; QTc prolongation, EKG mandatory.

8.5 Approach in Comorbid Conditions

- ADHD + tic: α2 agonist + stimulant or atomoxetine. Stimulants had a controversial history as a 'tic enhancer', but Bloch 2009 meta-analysis and the TACT (Tourette's Syndrome Study Group. Neurology 2002;58(4):527–536) — moderate-dose methylphenidate does not significantly exacerbate tics; monitor.
- OCD + tic: SSRI + ERP; adjunctive aripiprazole or risperidone.
- Sleep disorder: melatonin; clonidine also assists in the sleep component.

8.6 Source-Specific Clarifications

- AAN 2019 (Pringsheim Neurology 2019): CBIT first-line non-pharmacological; α2-agonists before antipsychotics (favorable side effect profile); dual benefit in comorbid ADHD.
- ESSTS 2022 (Müller-Vahl): In the European context; choice of CBIT and pharmacotherapy is individualized based on patient characteristics.
- Piacentini J. et al. JAMA 2010 (CBIT pivotal RCT): n=126, ages 9–17 — CBIT significantly superior to placebo (supportive therapy); reduction of 7.6 vs 3.5 points on YGTSS; effect size g = 0.68.
- Tourette Association of America Behavioral Sciences Consortium — Implementation of CBIT program and therapist training.

8.7 Treatment methods

1. Comprehensive Behavioral Intervention for Tics (CBIT) — Woods D.W., Piacentini J. — First-line non-pharmacological intervention (AAN 2019). Three core components: (1) Habit Reversal Training (HRT — recognition of premonitory sensation, learning competing behavior); (2) functional intervention (modifying contexts that reinforce tics); (3) relaxation training. Standard course — 8 individual sessions, 10 weeks; ongoing booster sessions. Evidence: Piacentini J. et al. JAMA 2010 RCT — pivotal study; Wilhelm S. et al. Arch Gen Psychiatry 2012 RCT in adults; effect size moderate. tourette.org.
2. Habit Reversal Training (HRT — Habit Reversal Training) — Azrin (Azrin N.H.), Nunn (Nunn R.G.) — Core component of CBIT — awareness

training + competing response. Separate competing behavior for each tic (e.g., for neck jerking, holding neck centered).

3. Exposure and Response Prevention (ERP) for Tics — Alternative behavioral approach to HRT — patient learns to tolerate premonitory urge without responding with tic. Verdellen C.W. et al. Behav Res Ther 2004. ESSTS — alternative evidence-based behavioral method.
4. Yale Global Tic Severity Scale (YGTSS — Yale Global Tic Severity Scale) — Leckman (Leckman J.F.) et al., 1989 — Gold standard severity scale for tic disorders; separate counts for motor and vocal tics: frequency, intensity, complexity, interference level (0–5 points); global impairment 0–50. Baseline indicators and monitoring.
5. Premonitory Urge for Tics Scale (PUTS) — Woods D.W., Piacentini J — 9-item self-assessment; intensity of premonitory urge and its effect on intervention behavior.
6. Deep Brain Stimulation (DBS) — Surgical intervention for refractory severe Tourette; typical targets — thalamus (centromedian-parafascicular) and globus pallidus internus (GPi). Multidisciplinary selection; Schrock L.E. et al. Mov Disord 2015 international consensus.
7. Botulinum Toxin — Injection into target muscle in focal motor (e.g., neck, lip, eye) and vocal tics; 3–4 month effect. Marras C. et al. Neurology 2001 RCT.

9. Prognosis

Good prognostic factors

- Typical tic disorder (distinct from rapid functional tic-like behavior).
- Mild-moderate YGTSS severity.
- Active management of comorbidity (ADHD, OCD).
- Early psychoeducation and family-school support.
- CBIT availability.

Poor prognostic factors

- Severe comorbidity (severe OCD, ADHD, anxiety, depression).
- Self-injurious tics (e.g., head banging).
- Social and academic limited adaptation.
- Refractory pharmacotherapy response.

Follow-up targets

- YGTSS every 3–6 months.
- Comorbidity (ADHD, OCD, anxiety, depression) — annual.
- Monitoring side effects (in those taking antipsychotics — metabolic, EPS, prolactin, QTc).
- Social and academic support.
- In adolescence, possibility of spontaneous remission; possibility of gradual reduction of pharmacotherapy.
- Support for occupation and relationships in adult patients.

10. Myths and misconceptions

10.1 Aetiology myths

Myth 1: “Tourette syndrome is always accompanied by coprolalia (swear words)”

Why it is widespread: In mass media (film, television), the portrayal of Tourette is almost always presented with coprolalia; sensational media choice.

Clinical and biological rationale: Coprolalia is the rare complex vocal tic form; neither motor nor vocal tics are obscene in most Tourette patients.

Evidence: Freeman R.D. et al. Dev Med Child Neurol 2009 — International Tourette Syndrome Association database n=597: coprolalia occurs in 19.3%, of males and 14.6% of females.

Real clinical step: Provide correct information to family and school; reduce patient's social stigma.

Myth 2: “Tourette's is a result of poor upbringing or parental mistakes”

Evidence: heritability 0.77 (Mataix-Cols JAMA Psychiatry 2015) — highest among psychiatric disorders. Genetic and neurobiological basis; parental experience not etiology.

Myth 3: “Tourette's is caused by emotional trauma or stress”

Clinical logic: Stress is not a cause of tic onset — tics are genetic and neurodevelopmental in origin. However, stress influences the severity of existing tics.

Myth 4: “All tics are PANDAS and must be treated with antibiotics”

Why it is widespread: In the 1990s, PANDAS (Pediatric Autoimmune Neuropsychiatric Disorders Associated with Streptococcal infections) hypothesis by Susan Swedo and colleagues, and marketing by alternative clinics.

Clinical and biological rationale: Classic PANDAS clinical picture — rapid (hours-days) onset OCD/tics + emotional lability + post-streptococcal infection — is a rare event. Most tic patients show a typical course (gradual onset, waxing and waning).

Evidence: NIMH 2017 PANDAS/PANS clinical statement: routine antistreptolysin O testing or antibiotic prophylaxis in general tic disorders is not recommended; assessed only if a classic clinical picture is present. Routine use of IVIG and plasmapheresis is not evidence-based (only within clinical trials).

10.2 Harmful or erroneous methods

Myth 5: “Scolding or punishing a child harshly when they tic will make the tics go away”

Clinical logic: Tics can be partially voluntarily suppressed, but prolonged suppression leads to an increase in internal tension and subsequent “rebound” tic outbursts. Punishment does not reduce tics; on the contrary, it increases stress-related intensity.

Evidence: AAN 2019, ESSTS 2022 — tics are not voluntary behavior; punitive approach is ineffective and harmful.

Myth 6: “Antipsychotic haloperidol should be first-line in all tic patients”

Evidence: AAN 2019 (Pringsheim) — α2-agonists (clonidine, guanfacine) Before antipsychotics. It is recommended due to its favorable side effect profile. Haloperidol and pimozide are FDA-approved, but they have third-line status because of risks for EPS, tardive dyskinesia, sedation, prolactin, and QTc prolongation.

Myth 7: “Stimulants cause tic explosions — stimulants are contraindicated for a Tourette patient with ADHD”

Evidence: Bloch M.H. et al. JAACAP 2009 meta-analysis (n=9 RCTs, n=477) and TACT (Treatment of ADHD in Children with Tics; Tourette's Syndrome Study Group. Neurology 2002;58(4):527–536) — moderate-dose methylphenidate does not significantly exacerbate tics; in some patients it even reduces tic severity. Comorbid ADHD constitutes the major part of functional impairment — considering stimulants as contraindicated is incorrect.

Real clinical step: In comorbid ADHD, start stimulant, monitor patient for tic severity; if exacerbation observed, reduce dose or α2-agonist + stimulant combination.

10.3 Ineffective or scientifically unfounded methods

Myth 8: "Cannabis (medical marijuana) is an effective treatment for Tourette syndrome"

Why it is widespread: anecdotal reports on social media, cannabis industry marketing, modest effects shown in some pilot studies.

Evidence: Müller-Vahl K.R. studies (small RCTs) — THC may partially reduce tics, but evidence base is insufficient for clinical recommendation. AAN 2019 — cannabinoids are not standard first-line; risk profile (psychosis risk in adolescents, cognitive effects, dependence) is significant. May be considered only in refractory adult cases and where national legislation permits.

Myth 9: "Special diet (gluten-free, casein-free) or avoidance of food additives eliminates Tourette's"

Evidence: No specific clinical evidence; AAP, AAN do not recommend.

Myth 10: "EEG neurofeedback reduces tics"

Evidence: Evidence base is limited; AAN 2019 — not first-line; CBIT is significantly superior.

Myth 11: "Vitamin mega-doses, omega-3, magnesium resolve tics"

Evidence: Efficacy of these supplements for Tourette has not been proven; AAN 2019 and ESSTS 2022 do not recommend.

Myth 12: "Acupuncture, homeopathy, craniosacral therapy cure Tourette's"

Evidence: Cochrane and systematic reviews — the effect of these complementary interventions for tic disorders has not been proven.

Myth 13: "Hypnotherapy or psychoanalytic therapy resolves tics"

Evidence: Was dominant until the 1960s, but shows no efficacy in modern evidence base; CBIT or pharmacotherapy is superior.

11. Sources

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6A20–6A2Z SCHIZOPHRENIA SPECTRUM

Schizophrenia or other primary psychotic disorders

ICD-11 **6A20**	**6A20 SCHIZOPHRENIA** **Schizophrenia**
ICD-10 F20	Schizophrenia
DSM-5-TR F20.9	Schizophrenia

1. Definition and nosology

Schizophrenia — a psychotic disorder characterized by significant disturbance in thought, perception, affect, and behavior, accompanied by at least one month of active-phase symptoms and typically a chronic course. In the ICD-11 classification it is placed under code 6A20 in the group ‘Schizophrenia and other primary psychotic disorders’ (6A2x).

Nosologically, it is a primary psychotic disturbance — meaning psychotic symptoms are not fully explained by another medical condition, substance effect, or affective disturbance. Schizoaffective disturbance (6A21), schizotypal disorder (6A22), acute and transient psychotic disorder (6A23) and delusional disorder (6A24) are coded separately.

2. History

The modern concept of schizophrenia has passed through four main stages:

- Kraepelin (1899) — “dementia praecox” (early dementia) — a disorder characterized by persistent cognitive decline that begins in early adulthood.
- Bleuler (1911) — The term “schizophrenias” (meaning “split mind,” but not in the sense of “dual personality”); the “4A” symptoms (loosening of associations, affect, ambivalence, autism) are taken as fundamental.

- Schneider (1959) — “1st-degree symptoms” (thought-audible, commentary hallucinations, thought theft, sense of control) accepted as diagnostic priority; this approach is reflected in DSM-III and ICD-10.
- DSM-5 (2013) and ICD-11 (2019) — subtypes (paranoid, hebephrenic, catatonic, simple) have been abolished (clinical utility and validity not proven, variable over time); Schneider's first-rank symptoms have been downgraded to equal status with other symptoms; additionally in ICD-11 dimensional approach has been applied — 6A25 symptom domains (positive, negative, depressive, manic, psychomotor, cognitive) and 6A20.x Course qualifiers.

Main changes compared with ICD-10 (F20): elimination of subtypes, elimination of the special status of Schneiderian symptoms, retention of the 1-month duration criterion, but unlike the 6-month requirement in DSM-5-TR, no total illness duration requirement.

3. Epidemiology

Prevalence and incidence

- Lifetime prevalence: 0.3–0.7% (Saha S. et al. PLoS Med 2005, systematic review, 188 studies, 46 countries).
- Annual incidence: 15.2 / 100 000 (McGrath J.J. et al. BMC Med 2004 meta-analysis).
- Geographic and cultural variability exists, but the core incidence rate remains within a certain range.

Sex and age

- Male : female ratio ~1.4 : 1; onset age in males 18–25, in females 25–35 (Aleman A. et al. Arch Gen Psychiatry 2003).
- In women, a second incidence peak is observed after menopause (associated with the reduction of the neuroprotective effect of estrogen).
- Onset in childhood is rare (<1% of all cases); a first diagnosis after the age of 50 is uncommon.

Comorbidity and mortality

- Substance use disorder — comorbid ~50% (especially tobacco, cannabis, alcohol).
- Depression — comorbid in 25–80% over the course.

- Suicide: Lifetime risk ~5–10% (Hor K., Taylor M. J Psychopharmacol 2010); the highest risk is within the first 5 years.
- Overall mortality: 2–3 times higher compared to the population; average expected lifespan is 15–20 years shorter (Olfson M. et al. JAMA Psychiatry 2015) — mainly cardiovascular diseases, metabolic syndrome, tobacco use.

4. Aetiology and pathogenesis

4.1 Genetic factors

Heritability 60–80% (Sullivan P.F. et al. Arch Gen Psychiatry 2003, twin meta-analysis). Monozygotic twin concordance ~50%, dizygotic ~15%, general population risk ~1%. One parent with schizophrenia — child risk ~10%; both parents — ~40%.

Polygenic nature: GWAS studies have identified over 200 risk loci (Trubetskoy V. et al. Nature 2022, n>76,000 cases). There is no single "schizophrenia gene" — the risk is made up of hundreds of small-effect variants and Copy number variations (CNV) — 22q11.2 deletion (~25-fold increased risk), 16p11.2, NRXN1 deletions.

4.2 Neurodevelopmental model

Schizophrenia as a neurodevelopmental disorder — accepted (Weinberger D.R. Arch Gen Psychiatry 1987; Murray R.M., Lewis S.W. BMJ 1987). Disturbance in early brain development (intrauterine and early postnatal) leads to clinical manifestation in young age. Evidence:

- Premorbid cognitive deficits and motor delays — documented from childhood.
- Synaptic pruning (during puberty) is excessive — reduction in cortical gray matter mass.
- Already documented brain structural changes in the first episode (not a mediator — a marker).

4.3 Environmental risk factors

- Obstetric complications — hypoxia, infection, fetal growth restriction (Cannon M. et al. Am J Psychiatry 2002 meta-analysis, OR ~2).
- Maternal viral infections during pregnancy (influenza, herpes) — increase risk by 2–7 times (Brown A.S. Schizophr Bull 2006).
- Cannabis use during adulthood — increases psychosis risk 2–3 times, even with early and high THC doses (Marconi A. et al. Schizophr Bull 2016 meta-analysis).

- Growing up in urban environments, migration, social isolation, childhood trauma — moderate increase in risk (van Os J. et al. Nature 2010 review).
- Paternal age 45+ — associated with increased risk of new mutations (Malaspina D. Arch Gen Psychiatry 2001).

4.4 Neurobiological mechanisms

- Dopamine hypothesis (updated): mesolimbic dopamine *hyperactivity* → positive symptoms; mesocortical *hypoactivity* → negative and cognitive symptoms (Howes O.D., Kapur S. Schizophr Bull 2009).
- Glutamate NMDA hypofunction: Ketamine and PCP produce schizophrenia-like symptoms via NMDA blockade; this places the glutamate system as the second key mechanism (Olney J.W., Farber N.B. Arch Gen Psychiatry 1995).
- GABAergic interneurons: Parvalbumin-positive interneuron dysfunction in the prefrontal cortex leads to working memory impairment.
- Structural changes: a decrease in cortical gray matter volume (particularly prefrontal and superior temporal cortex), ventricular enlargement, hippocampal volume reduction (Haijma S.V. et al. Schizophr Bull 2013, meta-analysis n>8000).
- Inflammation theory: Elevated CRP, IL-6, and microglia activation documented in first episodes (Khandaker G.M. et al. Lancet Psychiatry 2015).

5. Clinical features

Symptom domains (ICD-11 / 6A25)

Positive symptoms

Delusion (persecutory, relational, grandiose, control), hallucinations (most typical auditory — commentary or dialogue), formal thought disorder (disorganized speech, neologisms, “word salad”), grossly disorganized behavior.

Negative symptoms

Affective flattening, avolition (loss of motivation), alogia (poverty of speech), anhedonia, social withdrawal. A major factor in functional impairment; responds poorly to pharmacotherapy.

Cognitive symptoms

Working memory, attention, executive function, semantic and work-related memory impairments. Present in 75–85% of patients (Heinrichs R.W., Zakzanis K.K. Neuropsychology 1998 meta-analysis).

Depressive and manic symptoms

Affective episodes are widespread in the course of schizophrenia; if the affective episode does not explain the psychotic symptoms, the diagnosis of schizophrenia remains.

Psychomotor symptoms

Catatonic components (mutism, negativism, waxy flexibility, stereotypy), agitation, retardation.

Stages of illness

1. Premorbid phase — discrete cognitive and social deficits starting from childhood (clinical schizophrenia is not yet present).
2. Prodromal phase — months-years; social withdrawal, unusual experiences, depressive symptoms, subclinical psychotic symptoms.
3. Acute (first) psychotic episode — manifest positive symptoms; average age 18–25 (in males).
4. Stabilization and residual phases — residual symptoms, functional impairment, relapse episodes.

The "1/3 principle" (Hegarty J.D. et al. Am J Psychiatry 1994 review)

- 1/3 patients — good functional recovery.
- ~1/3 — residual symptoms, partial functional recovery.
- ~1/3 — severe chronic course, requires continuous support.

6. Diagnosis

6.1 Unified diagnostic criteria (DSM-5-TR · ICD-11 · NICE CG178 consensus)

A. Active-phase symptoms — at least 2 of the following must have persisted for ≥ 1 month; at least one must be from (1)–(3):

1. Delusion
2. Hallucinations
3. Formal thought disorder / disorganized speech
4. Grossly disorganized or catatonic behavior
5. Negative symptoms (affective flattening, avolition, alogia)

B. Functional impairment — significant decline in one or several areas of work, relationship or self-care.

C. Exclusions — psychoactive substance effects and organic causes (toxicology screening, brain imaging, lab tests) must be excluded.

D. Affective differentiation — affective episode does not fully explain psychotic symptoms (schizoaffective disorder or affective disorder with psychotic features excluded).

6.2 Differences between sources

- DSM-5-TR (F20.9) additional requirement: general disease duration ≥6 months (1 month active + prodromal and/or residual). On a background of autism spectrum or speech disorder — schizophrenia is coded only when clear delusions or hallucinations are additionally present.
- Difference in ICD-11 (6A20): ≥ 1 month of active symptoms is sufficient, overall 6-month requirement is not necessary. Subtypes have been abolished; instead 6A25 symptom domains and 6A20.x course qualifiers (0 first episode acute, 1 first episode partial or full remission, 2 multiple episodes, 3 continuous, Z unspecified) is coded.
- NICE CG178 practice requirement: The diagnosis is clinical, but should be supported by structured interview (SCID-5 or MINI) and severity scale (PANSS or BPRS); first episode: brain MRI, EKG, baseline metabolic indicators (glucose, HbA1c, lipids, prolactin) is mandatory.

6.3 Diagnostic algorithm

1. Clinical interview (patient + at least one informed relative).
2. Structured diagnostic interview (SCID-5 or MINI).
3. Severity assessment (initial PANSS or BPRS score).
4. Physical and neurological examination.
5. Toxicology screening (urine) and substance use history.
6. Mandatory laboratory tests (see § 7.2 below).
7. Brain MRI (at first episode).
8. Differential diagnosis (§ 6.4).
9. Comorbidity and suicide risk assessment (C-SSRS).
10. Functional level assessment (GAF or PSP).

6.4 Differential diagnosis

Disorder	Distinguishing features
Schizoaffective disorder (6A21)	An affective episode is present during a significant portion of the psychotic symptoms, but psychosis also persists during periods outside the affective episode.
Schizotypal disorder (6A22)	Psychotic-like symptoms; full psychosis criteria absent; persistent personality traits.
Delusional disorder (6A24)	Delusions only; hallucinations minimal or absent; functioning relatively preserved.
Acute and transient psychotic disorder (6A23)	Duration ≤ 3 months (ICD-11) or < 1 month (DSM-5-TR — Brief Psychotic Disorder).
Affective disorder with psychotic features (6A60.x, 6A70.x)	Psychosis exists only in the context of manic or depressive episodes.
Substance-induced psychosis	Temporal relationship with substance use or withdrawal; remission after substance elimination.
Secondary (organic) psychosis (6E61)	Medical condition-related (epilepsy, autoimmune encephalitis, brain tumor, Wilson's disease).
Autism spectrum disorder (6A02)	Early onset, social-communicative deficit primary; hallucinations/delusions not typical.
Personality disorders (6D10) — with schizotypal and paranoid characteristics	Persistent personality traits, no full psychotic episodes.

7. Examination and assessment

7.1 Clinical interview and scales

- Structured interview: SCID-5 (for DSM-5), MINI (short, accessible in clinical practice).

- Severity scales (gold standard): PANSS (30 indicators, 7-point scale), BPRS (18 indicators), SANS/SAPS (negative/positive separately), CGI-S.
- Functional level: GAF (DSM-IV), PSP (Personal and Social Performance Scale).
- Suicide risk: C-SSRS (Columbia Suicide Severity Rating Scale).
- Cognitive assessment: MATRICS Consensus Cognitive Battery (MCCB) — research and specialized clinics.

7.2 Laboratory investigations

Mandatory in first episode: Complete blood count, liver and kidney function tests, fasting glucose and HbA1c, lipid profile, TSH and T4, vitamin B12 and folate, toxicology screening (cannabis, amphetamine, opioid, cocaine, PCP, MDMA), HIV and syphilis serology, pregnancy test (in females), baseline prolactin level.

Selective (based on clinical suspicion): EEG (rule out seizure or delirium), anti-NMDA-R antibodies (CSF — atypical presentation), ceruloplasmin and 24-hour urine copper (suspected Wilson's disease), B12/folate/homocysteine (suspected organic cause).

7.3 Instrumental investigations

- Brain MRI (T1, T2, FLAIR) — recommended at first episode (NICE CG178).
- EKG — before starting antipsychotic (baseline QTc).
- Brain CT — when MRI is unavailable.

8. Treatment

8.1 General principles (NICE CG178 · APA 2021 · WFSBP 2015 · Maudsley 14e consensus)

1. Multimodal approach — pharmacotherapy + psychosocial intervention + systemic support. Pharmacotherapy alone is not sufficient for functional recovery.
2. Collaborative decision-making with the patient (shared decision making) — medication, dose, and side effect profile are discussed; adherence begins at this stage.
3. Atypical antipsychotic in first episode preferred — low dose, gradual titration. In a first episode the lower half of the standard dose may be sufficient.

4. Effect assessment — Partial response in 2–4 weeks, full response at adequate dose expected in 4–6 weeks. If no response, the medication is changed.
5. Monotherapy — antipsychotic polypharmacy increases side effect burden, evidence of effect weak; clozapine + second antipsychotic may be an exception.
6. Treatment-resistant schizophrenia (TRS) — failure to respond to two different antipsychotics at adequate dose and duration (each 6–8 weeks) — clozapine initiated.
7. Supportive treatment — at least 1–2 years after first episode, long-term/lifetime after multiple episodes. Discontinuing the antipsychotic sharply raises relapse risk: at one year relapse was 27% on drug versus 64% on placebo (Leucht S. et al. Lancet 2012, 65 trials, n=6493; RR 0.40).
8. Psychosocial intervention — CBTp and family intervention should be offered to all patients (NICE CG178 — strong recommendation).
9. Systematic monitoring — metabolic parameters, EPS, prolactin, EKG.
10. LAI (long-acting injectable) — in cases of adherence issues or patient preference; better outcome in relapse prevention after first episode (Tiihonen J. et al. JAMA Psychiatry 2017).

8.2 Pharmacotherapy — First-Line Antipsychotics

Mechanism, dose, evidence, and side effect profile for each medication.

Drug	Receptor profile	Standard dose	Main adverse effect	Advantage
Olanzapine	D2, 5-HT2A, H1, M1	5–20 mg/day	Weight gain, metabolic syndrome, sedation	High efficacy, but metabolic risk high
Risperidone	D2, 5-HT2A	2–6 mg/day	EPS (dose > 6 mg), hyperprolactinemia	Widely used, inexpensive, LAI variant available
Aripiprazole	D2 partial agonist, 5-HT1A agonist, 5-HT2A antagonist	10–30 mg/day	Akathisia, dizziness	Metabolic profile good, prolactin neutral

Drug	Receptor profile	Standard dose	Main adverse effect	Advantage
Quetiapine	D2 (weak), 5-HT2A, H1, α1	300–800 mg/day	Sedation, orthostatic hypotension, weight gain	Soft EPS profile
Paliperidone	Active metabolite of risperidone	6–12 mg/day	Hyperprolactinemia, EPS	LAI (monthly, 3-monthly, 6-monthly) formulations
Lurasidone	D2, 5-HT2A, 5-HT7	40–160 mg/day (with meals)	Akathisia, nausea	Metabolic profile very good; must be taken with food
Ziprasidone	D2, 5-HT2A, 5-HT1A	80–160 mg/day	QT prolongation	Requires EKG monitoring

Typical (first generation) antipsychotics — haloperidol 5–20 mg/day, chlorpromazine 200–800 mg/day, flupentixol 6–18 mg/day, zuclopenthixol 25–150 mg/day. Effective for positive symptoms, but EPS, tardive dyskinesia, and weak effect on negative symptoms — not superior when atypical antipsychotics are available.

Note on the mechanism: Primary mechanism of all antipsychotics is mesolimbic D2 receptor antagonism (in atypicals, additional 5-HT2A antagonism — contributes to the reduction of negative symptoms and EPS). 60–80% D2 occupancy is therapeutic range; >80% increases EPS risk. Below 60% the effect is insufficient (Kapur S., Mamo D. Prog Neuropsychopharmacol Biol Psychiatry 2003).

8.3 Treatment-Resistant Schizophrenia and Clozapine

Clozapine — The single superior evidence-based medication in TRS (Kane J. et al. Arch Gen Psychiatry 1988 initial RCT; Siskind D. et al. Br J Psychiatry 2016 meta-analysis). 30–60% of TRS patients respond to clozapine.

Why is it superior: Low D2 occupancy (40–60%), but strong effect on various receptors (D4, 5-HT2A, M1, α1); fewer EPS, better effect on negative symptoms; reduces suicide (FDA indication).

Initiation protocol: 12.5 mg starting; gradually increase to 200–450 mg/day (max 900 mg/day). Complete blood count + leukocyte formula — weekly for first 18 weeks, then every 2 weeks for up to 1 year, then monthly. Neutrophils 1000–1499 /μL — mild neutropenia, continue with enhanced monitoring; < 1000 /μL → interrupt treatment; < 500 /μL → severe neutropenia. In benign ethnic neutropenia the threshold is < 500 /μL.

Clozapine side effects and management:

- Agranulocytosis (~0.8%, mortality 5%) — blood count monitoring.
- Myocarditis — especially within the first 4 weeks; troponin, EKG.
- Constipation — risk of ileus and perforation; prophylactic laxative.
- Hypersalivation, sedation — dose-dependent.
- Seizures — the risk rises at doses above 600 mg/day; EEG monitoring.
- Metabolic syndrome — high; regular monitoring.

Clozapine discontinuation — important clinical point: sudden cessation causes rebound psychosis (more severe than original symptoms), cholinergic rebound (nausea, vomiting, diarrhea) and adrenergic rebound (agitation, insomnia). If discontinuation is required due to agranulocytosis — the patient is switched gradually to another antipsychotic (olanzapine or quetiapine) in an inpatient setting. For planned discontinuation — 25% reduction per week.

8.4 LAI (Long-Acting Injectable) Options

LAI	Interval	Standard dose
Risperidone LAI (Consta)	Every 2 weeks	25–50 mg
Paliperidone palmitate	Monthly / 3-monthly / 6-monthly	75–150 mg monthly
Aripiprazole LAI (Maintena)	Monthly	400 mg
Olanzapine pamoate	Once every 2–4 weeks	150–300 mg
Haloperidol decanoate	Every 4 weeks	50–200 mg
Flupentixol decanoate	Once every 2–4 weeks	20–40 mg
Zuclopenthixol decanoate	Once every 2–4 weeks	200–400 mg

8.5 Management of Acute Psychomotor Agitation

- Oral (dominant): lorazepam 1–2 mg + olanzapine 5–10 mg oral disintegrating tablet, or risperidone 1–2 mg oral solution.
- Intramuscular Olanzapine im 10 mg, or aripiprazole im 9.75 mg, or haloperidol 5 mg + lorazepam 2 mg im (classic).
- Attention: Olanzapine im + benzodiazepine parenteral — risk of respiratory depression; should not be used concurrently.

8.6 Management of Side Effects

Extrapyramidal symptoms (EPS):

Symptom	Clinical manifestation	Intervention
Acute dystonia	Spasm of neck, tongue, eye muscles (first hours–days)	Biperiden 2–5 mg IM (emergency)
Parkinsonism	Tremor, rigidity, bradykinesia	Dose reduction; biperiden 2–4 mg/day
Akathisia	Inner restlessness, inability to sit still	Propranolol 30–80 mg/day; dose reduction
Tardive dyskinesia	Late-onset dyskinetic movements (months–years)	Valbenazine / deutetrabenazine; switch to clozapine

Metabolic side effect: Body weight, waist circumference, blood pressure — monthly for the first 6 months, then every 6 months; fasting glucose, HbA1c, lipids — at baseline, 12 weeks, then annually. Intervention: diet and physical activity counseling, metformin (in overweight), switching antipsychotic to a metabolically neutral agent (aripiprazole, lurasidone).

Hyperprolactinemia: high risk — risperidone, paliperidone, amisulpride, typicals; low risk — aripiprazole, quetiapine, clozapine. In symptomatic cases (galactorrhea, amenorrhea, sexual dysfunction, osteoporosis) — change the medication.

Neuroleptic Malignant Syndrome (NMS) — a medical emergency (mortality up to 10%): Clinical tetrad — hyperthermia, muscle rigidity, altered consciousness, autonomic instability. Lab: CPK > 1000, leukocytosis, myoglobinuria. Intervention: immediate discontinuation of antipsychotic, intensive care support, dantrolene, bromocriptine, ECT if necessary.

8.7 Special Conditions

- Pregnancy: complete discontinuation of the antipsychotic increases relapse risk. As a class, antipsychotics do not meaningfully increase the risk of congenital malformations (Huybrechts K.F. et al. JAMA Psychiatry 2016, 1,3 mln pregnancies); the possible exception is risperidone (about 26% higher risk, needs confirmation). The choice is individual: a drug that works is usually continued.
- Older adults: EPS, orthostatic hypotension, sensitivity to anticholinergic effects; low dose, quetiapine or risperidone preferred; refer to Beers Criteria.
- Comorbid substance use: smoking cessation support, cannabis cessation, concurrent treatment of alcohol dependence (naltrexone, acamprosate).

8.8 Source-Specific Clarifications

- NICE CG178: Early Intervention Services (EIP) — intensive multidisciplinary support in the first 3 years; clozapine first-line in resistance after two antipsychotics.
- APA 2021: supportive treatment duration after first episode 2–5 years; cognitive remediation and supported employment are strongly recommended for functional recovery.
- WFSBP 2015: equivalent dose ranges and adequate duration (4–6 weeks) before medication change; LAI effective in relapse prevention.
- Maudsley 14e (2021): Detailed protocol for clozapine titration, side effect management, and polypharmacy reasons.
- AR Ministry of Health Clinical Protocol (2009): Atypical antipsychotic first-line; conventional neuroleptic — when atypical unavailable. General algorithm aligns with NICE/APA.

8.9 Treatment methods

1. Cognitive Behavioral Therapy for psychosis (CBTp — Cognitive Behavioural Therapy for psychosis) — Re-evaluation of psychotic symptoms within “thought–emotion–behavior” formulation; patient addresses hallucinations and delusions with reality testing, behavioral experiments, and search for alternative explanations. Standard course — 16+ individual sessions, 6–12 months. Effect: small-to-medium effect on positive symptoms (Grade A in NICE), improvement in functional level (Wykes T. et al. Schizophr Bull 2008, n=2284 meta-analysis; NICE CG178 § 1.3.7.1).
2. Family Intervention — Psychoeducation, communication training and problem-solving in families with high expressed emotion (EE). Patient + at least one family member, 10+ sessions, 3–12 months. Effect: reduces relapse

risk by ~20%, reduces hospitalization (Pharoah F. et al. Cochrane Database Syst Rev 2010;(12):CD000088, n>3500). NICE CG178 § 1.3.7.2.

3. Cognitive Remediation — Computer- or trainer-based structured cognitive exercises — attention, working memory, executive function, social cognition. 20–40 sessions, 2–3 times per week. Effect: small-medium improvement in cognitive indicators, transfers to functional level only when applied together with psychosocial rehabilitation (Wykes T., Huddy V., Cellard C., McGurk S.R., Czobor P. Am J Psychiatry 2011;168(5):472–485, meta-analysis n=2104).
4. Supported Employment / Individual Placement and Support (IPS) — Direct supported engagement in real workplaces — “place-then-train” principle (starting job search without prior preparation stage). Personal job coordinator, finding job based on individual interest and ability, on-site support. Effect: more than half enter competitive employment (20% in control groups) — Drake R.E., Bond G.R., Goldman H.H. et al. Health Aff (Millwood) 2016;35(6):1098–1105. NICE CG178 § 1.3.7.3.
5. Early Intervention in Psychosis (EIP — Early Intervention in Psychosis) — Intensive multidisciplinary care over 3 years following first psychotic episode — low-dose pharmacotherapy, CBTp, family intervention, employment support, peer support. RAISE and EDEN RCTs (Kane J.M. et al. Am J Psychiatry 2016; Marshall M. et al. Lancet 2011) — EIP superior to standard care, reduces relapse, improves functional level. NICE CG178 § 1.4.
6. Assertive Community Treatment (ACT) — Multidisciplinary team (psychiatrist, clinical psychologist, social worker, peer specialist) provides intensive support to the patient in the community (at home, workplace); 24/7 accessibility, low patient/staff ratio (~10:1). Target: patients with high risk of rehospitalization and homelessness. Effect: reduces hospital days, increases residential stability (Marshall M., Lockwood A. Cochrane 2011).
7. PANSS / BPRS — Severity scales — PANSS (Positive and Negative Syndrome Scale) — 30 items, 7-point scale; BPRS (Brief Psychiatric Rating Scale) — 18 items. Quantitative assessment of psychotic symptoms based on structured interview. For initial assessment and treatment monitoring in clinical practice. Kay S.R., Fiszbein A., Opler L.A. Schizophr Bull 1987;13(2):261–276 (PANSS); Overall J.E., Gorham D.R. Psychol Rep 1962;10:799–812 (BPRS).

9. Prognosis

Good prognostic factors

- Brief untreated period (DUP < 6 months)
- Acute onset, clear stressor
- Premorbid good function
- Predominance of positive symptoms (negative few)
- Presence of an affective component
- Good social support, family cooperation
- Early intervention and adherence

Poor prognostic factors

- Gradual onset, long prodrome
- Early age of onset (childhood or early adolescence)
- Premorbid functional impairment (from childhood)
- Predominance of negative symptoms
- Severity of cognitive deficit
- Substance use (especially cannabis, amphetamine)
- Non-adherence, social isolation
- DUP > 1 year

Follow-up targets

- Clinical stability and relapse prevention (antipsychotic adherence, consideration of LAI).
- Metabolic and cardiovascular risk management (a major contributor to mortality).
- Functional recovery — employment, independent living, social relationships.
- Comorbid substance use, depression, suicide risk monitoring.
- Strengthening family and community support systems.

10. Myths and misconceptions

10.1 Aetiology myths

Myth 1: "Schizophrenia is possession by spirits or demonic possession"

Why it is widespread: Historical-religious context — psychosis in medieval Europe explained as 'demonic possession'; the subjectively extraordinary nature of hallucinations reinforces this; in communities with low psychiatric service accessibility, family first consults religious leader.

Clinical and biological rationale: Schizophrenia possesses measurable neurobiological markers — reduction in cortical gray matter volume (Haijma S.V. Schizophr Bull 2013, meta-analysis n>8000), mesolimbic dopamine hyperactivity (Howes O.D. Schizophr Bull 2009), 60–80% heritability (Sullivan P.F. 2003). Dose-proportional effect of D2 receptor antagonist antipsychotics proves specificity of pharmacological mechanism. 'Possession' concept cannot be tested at these levels and is not open to falsification.

Evidence: WHO mhGAP Intervention Guide v2.0 — schizophrenia recognized as a medical condition. APA Resource Document on Religious/Spiritual Commitments (2018): religious practice *may complement* treatment, but *cannot replace it*.

Real clinical step: Antipsychotic initiation should not be delayed. Respecting family's religious-cultural beliefs preserves therapeutic alliance — allowing exorcism rituals to replace antipsychotic treatment results in delayed onset and poor prognosis (Marshall M. et al. Arch Gen Psychiatry 2005 — DUP > 1 year is a poor prognostic marker).

Myth 2: "Schizophrenia is a result of poor upbringing or a 'cold mother'"

Why it is widespread: In the 1940s–1960s, influence of the psychoanalytic school (Fromm-Reichmann F. — concept of 'schizophrenogenic mother'); the family-blaming model remained dominant in clinical practice for a long time.

Clinical and biological rationale: Heritability 60–80% — in children adopted away from biological family, risk associated with diagnosis of biological parents, not characteristics of adoptive family (Heston L.L. Br J Psychiatry 1966; Tienari P. et al. Br J Psychiatry 2004 — Finland adoptive study). High emotional expression (EE) in family increases relapse risk, but this — not a primary aetiology, is a course modulator.

Evidence: APA, AAP, NICE — the concept of 'schizophrenogenic mother' has been abandoned by the scientific community.

Real clinical step: Family intervention (see “Treatment methods”) — based not on blaming, but on psychoeducation and communication training.

Myth 3: “Schizophrenia = split personality”

Why it is widespread: Etymological fallacy — mistranslation of “schizo” (split) + “phrenia” (mind) as “split personality”; mass culture (film, literature) reinforces false images.

Clinical and biological rationale: Bleuler's 1911 term described a disruption of the mind's internal coherence (splitting of associations), not the existence of separate personalities. “Double personality” is a distinct disorder: Dissociative Identity Disorder (6B64), with a completely different etiology (childhood trauma) and clinical manifestations.

Evidence: WHO ICD-11 — schizophrenia (6A20) and DID (6B64) are coded as separate disorders.

Myth 4: “People with schizophrenia are dangerous and aggressive”

Why it is widespread: Mass media particularly emphasizes a history of psychosis in violent incidents — notes attract attention, but the base rate is not presented.

Clinical and biological rationale: People with schizophrenia are more likely to become victims of violence (Maniglio R. Acta Psychiatr Scand 2009 systematic review — victimization ratio 2.3–140 times higher depending on crime type). Violence risk increased only in specific narrow context: untreated acute paranoid delusions + concurrent substance use (Fazel S. et al. PLoS Med 2009 meta-analysis). Minimal difference compared to population in treated, stable patients.

Evidence: All violent incidents related to schizophrenia constitute less than 5% of all violence in society (Fazel S. Br J Psychiatry 2014).

Real clinical step: Stigma impedes psychosocial recovery; psychoeducation for family and community is critically important.

10.2 Harmful “treatment” methods

Myth 5: “Exorcism or religious ritual can replace an antipsychotic”

Why it is widespread: Fear of antipsychotic side effects; greater accessibility of social support within religious communities; stigma of psychiatric care.

Risk: Each month increase in Duration of Untreated Psychosis (DUP) worsens long-term functional prognosis (Marshall M. et al. Arch Gen Psychiatry 2005,

n=3700 meta-analysis). In patients with DUP > 1 year, remission rate significantly decreases. High risk of suicide in untreated acute episode.

Evidence: NICE CG178, WHO mhGAP — antipsychotic treatment should not be delayed; religious-cultural support may be applied in parallel.

Myth 6: "'Insulin coma therapy' and 'lobotomy' — old treatments are still secretly used in hospitals"

Reality: Insulin coma therapy (Sakel 1933) and prefrontal lobotomy (Moniz 1936, Freeman 1945) were widely used in the 1950s, but were abandoned due to lack of comparative evidence of effectiveness and severe side effects following the introduction of antipsychotics (chlorpromazine 1952). These methods are not used in modern clinical practice. ECT (electroconvulsive therapy) is different — evidence-based, used in modern indications for schizophrenia with catatonic features or in refractory cases (NICE TA59).

10.3 Ineffective methods or those delaying primary treatment

Myth 7: "Mega-doses of Vitamin B3 (niacin) cure schizophrenia (orthomolecular psychiatry)"

Why it is widespread: Hoffer and Osmond in the 1950s proposed the niacin mega-dose theory based on single case report studies; the alternative medicine movement continues to support this approach.

Evidence: APA Task Force (Lipton M.A. et al. 1973) and subsequent RCTs — vitamin B3 or other vitamin mega-doses have no effect on schizophrenia symptoms. Niacin mega-dose can cause liver damage and hyperuricemia.

Myth 8: "Cannabis (especially CBD) cures schizophrenia"

Why it is widespread: Exaggerated media presentation of preliminary studies on CBD's antipsychotic effect; marketing by the medical cannabis industry.

Reality: THC (primary psychoactive component of cannabis) increases psychosis risk 2-3 fold (Marconi A. et al. Schizophr Bull 2016 meta-analysis). Preliminary RCTs on CBD (McGuire P. et al. Am J Psychiatry 2018) showed modest effect, but evidence base is insufficient to replace standard antipsychotics.

Evidence: NICE CG178, APA 2021 — CBD is not a standard recommendation for schizophrenia treatment; THC-containing cannabis is contraindicated.

Myth 9: Antipsychotics destroy the brain, turn you into a "zombie"; lifelong use is harmful

Why it is widespread: Severe EPS side effects of early typical antipsychotics; patient experiences shared on social media.

Biological logic: Structural changes associated with antipsychotic use exist (Ho B.C. et al. Arch Gen Psychiatry 2011), but it should be compared: untreated schizophrenia itself causes faster cortical volume reduction (Cahn W. et al. Arch Gen Psychiatry 2002). Antipsychotic discontinuation sharply increases relapse risk — 64% at one year versus 27% (Leucht S. Lancet 2012), relapse itself exerts neurotoxic effects.

Evidence: Tiihonen J. et al. (Lancet 2009, JAMA Psychiatry 2018) Finnish registry studies n>60,000: continuous antipsychotic use reduces mortality and hospital admissions.

Real clinical step: Medication selection based on side effect profile, dose titration, and response to patient complaints are managed within the antipsychotic dialogue.

Myth 10: "Schizophrenia is incurable, lifelong illness, recovery is impossible"

Reality: Hegarty 1/3 principle (Am J Psychiatry 1994) — approximately 1/3 of patients achieve good functional recovery; this rate increases, especially with Early Intervention Services (EIP) (Kane J.M. et al. Am J Psychiatry 2016 — RAISE RCT). "Clinical recovery" and "personal recovery" (Recovery model) are distinguished — a patient can build a meaningful life even without complete symptom remission.

Real clinical step: The prognosis is individual; early intervention, adherence, family support and psychosocial rehabilitation significantly increase recovery chances.

Myth 11: "Acupuncture, homeopathy, valerian are effective instead of antipsychotics"

Evidence: Cochrane reviews (Rathbone J. Cochrane 2007 — acupuncture; Ernst E. reviews — homeopathy) — no evidence of efficacy in schizophrenia. Complementary methods can be applied as adjuncts (stress management, sleep hygiene), but do not replace antipsychotics.

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ICD-11 **6A21**	**6A21** **SCHIZOAFFECTIVE DISORDER** **Schizoaffective disorder**
ICD-10 F25	Schizoaffective disorders

ICD-11 6A21	**6A21 SCHIZOAFFECTIVE DISORDER** **Schizoaffective disorder**
DSM-5-TR F25.0–F25.1	Schizoaffective Disorder

1. Definition and nosology

Schizoaffective disorder (ICD-11: 6A21; DSM-5-TR: F25.0–F25.1) — a primary psychotic disorder in which psychotic symptoms (delusion, hallucination, disorganized speech) and an affective episode (manic or depressive) are observed together during the same clinical episode, but in which, additionally, psychotic symptoms persist outside the affective episode as well.

On the spectrum of psychotic disorders the disorder occupies an intermediate position between schizophrenia and affective disorder — on one hand, it is differentiated from "affective disorder with psychotic features," and on the other hand, from "schizophrenia" (by the persistence of psychotic symptoms independent of the context of an affective episode).

DSM-5-TR — two type specifiers: bipolar type (manic and/or depressive episodes) and depressive type (only depressive episodes). In ICD-11, sub-specifications are parallel — according to the type of affective episode.

2. History

- Kasanin J. (1933) — The term "acute schizoaffective psychosis" was first introduced; a rapid onset and favorable prognosis were emphasized.
- 1970s — Carpenter W., Strauss J., Bartko J. — research as a distinct clinical entity from schizophrenia and affective disorder.
- DSM-III (1980) — schizoaffective disorder as a "mixed" category.
- DSM-IV (1994), DSM-5 (2013) — diagnostic criteria have been clarified; bipolar/depressive type distinction retained.
- ICD-11 (2019) — affective episode type sub-specifications.
- Diagnostic validity debate — Malhi G.S. et al. Aust N Z J Psychiatry 2008, Lake C.R., Hurwitz N. Curr Psychiatry Rep 2007: whether schizoaffective

disorder is a distinct entity separate from schizophrenia and affective disorder is debated; some researchers view it within a spectrum concept.

3. Epidemiology

- Lifetime prevalence: 0.3% (Perälä J. et al. Arch Gen Psychiatry 2007 Finland population study, n=8000).
- Markedly less common than schizophrenia (0.3–0.7%).
- Sex: Relatively more frequent in women (1:1.2–1.5); depressive type dominant in women.
- Age of onset: Mean age 20–30 years; bipolar type younger, depressive type later.
- Comorbidity: Substance use disorders ~40%, anxiety disorders ~30%, suicide risk comparable to schizophrenia (~5%).
- Mortality: general mortality is 2–3 times higher compared to the population.

4. Aetiology and pathogenesis

4.1 Genetic factors

- Genetic overlap with schizophrenia and bipolar disorder is high: GWAS studies (Cross-Disorder Group of Psychiatric Genomics Consortium. Lancet 2013, Cell 2019) — broad genetic correlation between schizophrenia, bipolar disorder, and schizoaffective disorder.
- Family studies — relatives of schizoaffective patients have increased risk of both schizophrenia and mood disorders (Maier W. et al. Arch Gen Psychiatry 1993).

4.2 Neurobiology

- Overlapping models with schizophrenia — dopamine dysregulation, cortical volume changes, glutamate NMDA dysfunction.
- Related to the affective component — limbic and monoaminergic (serotonin, noradrenaline) system dysfunction.
- Less severe structural changes and better cognitive profile observed compared to schizophrenia (Heckers S. Curr Psychiatry Rep 2009 review).

4.3 Environmental risk factors

- Overlap with schizophrenia — obstetric complications, prenatal infections, adolescent cannabis use, migration, urban environment.

- Early trauma and psychosocial stress may play a role as a trigger for the affective component.

5. Clinical features

5.1 Core patterns

- Psychosis in the context of a manic episode — grandiose delusions, ego-syntonic hallucinations, rapid thought flow, hyperspeech and behavior.
- Psychosis in the context of a depressive episode — guilt delusions, nihilistic delusions, auditory hallucinations (judging voices), low motivation.
- During periods outside the affective episode — positive (delusion, hallucination) and/or negative (affective flattening, avolition) symptoms persist — this criterion distinguishes the disorder from ‘affective disorder with psychotic features’.

5.2 Course and stages

- Course is typically episodic — affective episodes and psychotic components of varying intensity.
- Better functional recovery compared to schizophrenia, but significant social and occupational impairment exists.
- Manic and depressive cycles in bipolar type; persistent depressive components in depressive type.

6. Diagnosis

6.1 Unified diagnostic criteria (DSM-5-TR · ICD-11 consensus points)

A. Within the same illness episode, a major affective episode (manic or depressive) must be observed together with active-phase symptoms meeting schizophrenia criterion A (≥1 month).

B. Delusions or hallucinations have been present for ≥ 2 weeks in the absence of a major affective episode (in the lifetime course of the illness).

C. Symptoms consistent with a major affective episode of illness must be present for a significant portion of the overall course (along with active and residual phases).

D. Exclusions — psychosis not fully explained by substance (e.g., amphetamine) or medical condition (e.g., encephalitis).

6.2 Differences between sources

- DSM-5-TR (F25.0–F25.1): Criterion B specifies a minimum duration of 2 weeks (DSM-IV had no specific number). Type qualifiers — bipolar (has manic episodes) or depressive (only depressive).
- ICD-11 (6A21): "Schizoaffective disorder" — sub-specifications by affective episode type; specific 2-week requirement in ICD-11 is stated as "significant duration" rather than a fixed number.
- APA Practice Guideline (Schizophrenia 2021): There is no separate algorithm for schizoaffective disorder; treatment principles for schizophrenia and mood disorders are combined.
- WFSBP: Schizoaffective disorder is evaluated within the spectrum of schizophrenia disorders.

6.3 Diagnostic algorithm

1. Clinical interview + information from relatives — chronological order of affective episodes and psychotic symptoms.
2. Structured interview (SCID-5 or MINI).
3. PANSS or BPRS — severity of psychotic symptoms; HAM-D, MADRS — depressive; YMRS — manic.
4. Physical and neurological examination.
5. Toxicology screening (urine).
6. Laboratory (complete blood count, thyroid, liver, kidney, glucose, HbA1c, lipid, prolactin).
7. Brain MRI (first episode) — rule out organic cause.
8. Comorbidity and suicide risk assessment (C-SSRS).

6.4 Differential diagnosis

Disorder	Distinguishing features
Schizophrenia (6A20)	Affective episodes are absent or minimal; if present, they occur during a small portion of the psychosis course.
Bipolar I with psychotic features (6A60)	Psychosis only in the context of manic or depressive episodes; no psychosis outside affective episode.
Major depressive disorder with psychotic features (6A70/6A71.x)	Psychosis only in the context of a depressive episode.

Disorder	Distinguishing features
Substance-induced psychosis	Temporal relationship with substance use; remission after elimination.
Secondary (organic) psychosis (6E61)	Associated with a medical condition (encephalitis, lupus, paraneoplastic).
Delusional disorder (6A24)	Delusions only; affective component absent.
Acute and transient psychotic disorder (6A23)	≤ 3 months (ICD-11) / < 1 month (DSM-5-TR Brief Psychotic).

7. Examination and assessment

7.1 Clinical scales

- Structured interview: SCID-5, MINI.
- Psychotic severity: PANSS, BPRS.
- Depressive severity: HAM-D (Hamilton), MADRS (Montgomery-Åsberg).
- Manic severity: YMRS (Young Mania Rating Scale).
- Functional: GAF, PSP, SOFAS.
- Suicide risk: C-SSRS.

7.2 Laboratory investigations

Schizophrenia-like (see 6A20 §7.2):

- Absolute: complete blood count, liver/kidney, glucose, HbA1c, lipid profile, TSH, B12, folate, toxicology screening, HIV, syphilis, pregnancy test, prolactin baseline.
- Selective: EEG, anti-NMDA-R antibodies, ceruloplasmin (Wilson).
- Before starting lithium — creatinine, TSH, calcium, EKG.
- Before initiating valproate — liver and blood tests.

7.3 Instrumental investigations

- Brain MRI — in first episode.
- EKG — before starting antipsychotic or lithium.

8. Treatment

8.1 General principles (APA 2021 · NICE CG178 / CG185 · WFSBP consensus)

1. Multimodal approach — antipsychotic + mood stabilizer and/or antidepressant + psychosocial intervention.
2. Atypical antipsychotic first-line — paliperidone or paliperidone palmitate (LAI) — FDA approval specifically for schizoaffective disorder; risperidone, olanzapine, quetiapine, aripiprazole — widely used.
3. Mood stabilizer in bipolar type — lithium (suicidal risk reduction effect), valproate, lamotrigine (especially when the depressive component is predominant).
4. Antidepressant in depressive type — SSRI (sertraline, escitalopram) or SNRI (venlafaxine); antidepressant not used alone, combined with antipsychotic.
5. Clozapine — in refractory cases (non-response to two antipsychotics).
6. ECT — severe catatonic, with depressive component, high suicide risk, effective in refractory cases (NICE TA59 — in psychotic depression and catatonia).
7. Psychosocial intervention — CBTp (CBT for psychosis), family psychoeducation, supportive therapy, IPS supported employment.
8. Adherence and relapse prevention — LAI (paliperidone palmitate, aripiprazole LAI) in selected patients.
9. Comorbidity treatment — substance use (co-occurring program), anxiety, somatic disorders.
10. Metabolic and side effect monitoring — standard in antipsychotics.

8.2 Pharmacotherapy

Class	Drug / samples	Indication
Atypical antipsychotic	Paliperidone (FDA approval), risperidone, olanzapine, quetiapine, aripiprazole	Positive and negative symptoms; baseline treatment in all cases.
Atypical antipsychotic LAI	Paliperidone palmitate (monthly/3-monthly/6-monthly), aripiprazole LAI	In case of adherence problems or patient choice.

Class	Drug / samples	Indication
Mood stabilizer	Lithium, valproate, carbamazepine, lamotrigine	Lithium reduces suicide in bipolar type.
Antidepressant	SSRI (sertraline, escitalopram), SNRI (venlafaxine)	In depressive type; not alone, but in combination with antipsychotic
Refractory	Clozapine	Failure to respond to two antipsychotics.
Adjunct	Benzodiazepine (lorazepam) short-term	In acute agitation or catatonia.

8.3 ECT Indications

- Catatonic features.
- Severe psychotic depressive episode.
- Suicide risk, or refusal of food and fluids.
- During pregnancy (safe alternative).
- Non-response to pharmacotherapy.

8.4 Source-Specific Clarifications

- APA 2021 (Schizophrenia): Schizoaffective disorder treatment algorithm parallels schizophrenia; additional intervention based on affective component.
- NICE CG178 (Psychosis) + CG185 (Bipolar): both apply to schizoaffective disorder — psychotic component CG178, affective component CG185.
- WFSBP: In schizoaffective disorder, paliperidone and aripiprazole have the best evidence base.
- CANMAT / ISBD 2018: Bipolar type schizoaffective disorder — bipolar disorder is adapted to treatment algorithms.

8.5 Treatment methods

1. Cognitive Behavioral Therapy for psychosis (CBTp — Cognitive Behavioural Therapy for psychosis) — Adapted as in schizophrenia; see “Treatment Protocols”, 6A20.html and NICE CG178 §1.3.7.1.

2. Family Psychoeducation and Intervention (Family Psychoeducation) — Reduces relapse in families with high expressed emotion (EE). NICE CG178 §1.3.7.2; Pharoah Cochrane 2010.
3. Individual Placement and Support (IPS) — Supported employment engagement "place-then-train" principle. Drake R.E. et al. Health Aff (Millwood) 2016.
4. Lithium — Mood stabilization and suicide prevention in bipolar-type schizoaffective disorder; initial parameters creatinine, TSH, calcium, EKG; therapeutic level 0.6–1.0 mEq/L; Cipriani A. et al. BMJ 2013 — reduces suicide.
5. Lamotrigine — Preventive in bipolar depressive episode; initial titration slow (risk of Stevens-Johnson syndrome).
6. Paliperidone Palmitate (LAI — long-acting injection) — Monthly, 3-monthly, and 6-monthly formulations. The FDA schizoaffective indication belongs to the monthly form only (Invega Sustenna); the 3-monthly (Trinza) and 6-monthly (Hafyera) forms are approved for schizophrenia only. Superior in adherence issues and relapse prophylaxis.
7. Electroconvulsive Therapy (ECT) — Effective with catatonic features, severe psychotic depression, suicide risk, or refractory cases. NICE TA59 — strong recommendation for psychotic depression or catatonia. UK ECT Review Group Lancet 2003 meta-analysis — ECT superior to pharmacotherapy in severe depression.
8. PANSS, HAM-D, YMRS — Separate standardized assessment of psychotic, depressive, manic components.

9. Prognosis

Good prognostic factors

- Bipolar type (better prognosis than depressive type).
- Rapid onset, clear affective component.
- Premorbid good functional level.
- Early intervention and adherence.
- Social support.
- Comorbid substance use management.

Poor prognostic factors

- Depressive type.
- Gradual onset.

- Predominance of negative symptoms.
- Comorbid substance use.
- History of suicide attempt.
- Non-adherence.

Follow-up targets

- Separate monitoring of psychotic and affective symptoms (PANSS, HAM-D, YMRS).
- Suicide risk (C-SSRS) — particularly during depressive episodes and in the first years.
- Metabolic monitoring.
- In those on lithium — TSH, creatinine, calcium (every 6 months).
- Functional recovery, employment, social relationships.
- Family support and psychoeducation.

10. Myths and misconceptions

10.1 Etiology and conceptual myths

Myth 1: “Schizoaffective disorder is mild schizophrenia”

Why it is widespread: Schizoaffective patients, on average, demonstrate better functional recovery than schizophrenia patients; this is often misinterpreted.

Clinical and biological rationale: Schizoaffective disorder clinically overlaps with schizophrenia in manifestations (psychotic symptoms outside affective episode), but adds mood disorder component. Not a ‘mild form’ — a separate diagnostic entity. Some patients show severe course.

Evidence: Heckers S. Curr Psychiatry Rep 2009 — schizoaffective disorder overlaps neurobiologically with schizophrenia, but with significant clinical differences; the concept of a “mild form of schizophrenia” is not supported.

Myth 2: “Schizoaffective disorder is just a mix of two separate diagnoses (schizophrenia + bipolar)”

Evidence: Cross-Disorder Psychiatric Genomics Consortium 2013 — extensive genetic overlap between schizophrenia, bipolar, and schizoaffective disorders. This indicates that diagnostic entities lie within a spectrum — schizoaffective disorder deserves a separate clinical designation because it shows a distinct course and treatment response in long-term follow-up.

Myth 3: "Schizoaffective disorder is a fabricated diagnosis with no validity"

Why it is widespread: Academic debate (Malhi 2008, Lake & Hurwitz 2007) — some researchers question the practical value of the diagnosis.

Clinical logic: diagnostic validity is controversial, but clinical utility (useful framework for treatment decision and prognosis) retained. DSM-5-TR and ICD-11 both retain as diagnostic unit. In clinical practice, patient should be assessed by experienced psychiatrist — accurately differentiating from other disorders is important, as treatment approach differs.

10.2 Misguided treatment approaches

Myth 4: "Antidepressants can be used alone in schizoaffective disorder"

Clinical logic and evidence: Antidepressant monotherapy contraindicated in schizoaffective disorder — risk of triggering a manic shift, and risk of psychosis exacerbation in the depressive episode. NICE CG185, CANMAT 2018 – together with antipsychotic.

Myth 5: "Lithium is ineffective in schizoaffective disorder"

Evidence: Cipriani A. et al. BMJ 2013 meta-analysis — lithium significantly reduces suicide risk in the bipolar spectrum (including schizoaffective bipolar type). Lithium is an effective mood stabilizer in bipolar-type schizoaffective disorder.

Myth 6: "Clozapine is only for refractory schizophrenia, not schizoaffective"

Evidence: Clozapine use in case of failure of two antipsychotics in schizoaffective disorder is accepted clinical practice; suicide risk reduction effect extends to schizoaffective patients (Meltzer H.Y. et al. Arch Gen Psychiatry 2003 InterSePT study).

10.3 Ineffective or harmful approaches

Myth 7: "Exorcism, spiritual practice, or religious ritual cures schizoaffective disorder"

Evidence: similar to schizophrenia — psychotic symptoms are neurobiological in basis; exorcism delays antipsychotic treatment, worsens clinical outcomes (DUP is a poor prognosis marker, Marshall Arch Gen Psychiatry 2005).

Myth 8: “Cannabis as a mood stabilizer cures schizoaffective disorder”

Evidence: Cannabis increases psychosis risk by 2–3 times (Marconi A. et al. Schizophr Bull 2016); in schizoaffective patients considered contraindicated — exacerbates symptoms.

Myth 9: “Mega-dose vitamins or herbal preparations (St John's Wort, valerian) replace pharmacotherapy”

Evidence: no evidence of effect in systematic reviews; St John's Wort reduces levels of antipsychotics and mood stabilizers through cytochrome P450 induction.

Myth 10: “Complementary interventions (acupuncture, homeopathy, craniosacral) replace antipsychotics”

Evidence: none have proven efficacy for schizoaffective disorder.

Myth 11: “Pharmacotherapy is not required outside an active depressive episode”

Evidence: According to the criteria for schizoaffective disorder, psychotic symptoms persist during periods outside of affective episodes — antipsychotic maintenance therapy is recommended long-term.

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ICD-11 **6A22**	**6A22 SCHIZOTYPAL DISORDER** **Schizotypal disorder**
ICD-10 F21	Schizotypal disorder
DSM-5-TR F21	Schizotypal Personality Disorder

1. Definition and nosology

Schizotypal disorder (ICD-11: 6A22; DSM-5-TR: F21 Schizotypal Personality Disorder) — a clinical condition characterized by persistent thought and perception

disturbances, bizarre (eccentric) behavior, social deficits, and psychotic-like experiences, but not meeting the criteria for a full psychotic episode.

Important nosological issue: Different placement in ICD-11 and DSM-5-TR:

- ICD-11 — within schizophrenia spectrum disorders (psychotic category), 6A22;
- DSM-5-TR — in the personality disorders chapter (Cluster A) and, at the same time, in the schizophrenia spectrum disorders list — double-coded.

2. History

- Bleuler E. (1911) — latent schizophrenia — a condition closely related to schizophrenia but without a full psychotic episode.
- Rado S. (1953) — The term "schizotype" refers to a persistent personality expression of genetic predisposition to schizophrenia.
- Kety S.S. et al. (1968) — Danish adoptive studies — high schizotypal traits in biological relatives of schizophrenia; basis of "schizophrenia spectrum" concept.
- DSM-III (1980) — Schizotypal Personality Disorder among personality disorders.
- ICD-10 (1990) and ICD-11 (2019) — schizotypal disorder in psychotic disorders chapter.
- DSM-5 (2013) — double placement (personality disorders + schizophrenia spectrum) — conceptual bridge.

3. Epidemiology

- Lifetime prevalence: 0.6–4% — wide range (Pulay A.J. et al. Prim Care Companion J Clin Psychiatry 2009 US NESARC study n>34,000 — 3.9%).
- Sex: Relatively frequent in males; difference minimal in some studies.
- Onset: Adolescence and early adulthood period.
- Transition to schizophrenia: 10–25% (Schultze-Lutter F. et al. Schizophr Res 2014); higher when there is a family history and additional psychosis risk factors.
- Comorbidity: Mood disorders, anxiety, substance use, other personality disorders (paranoid, schizoid, borderline).

4. Aetiology and pathogenesis

4.1 Genetic factors

- Shared genetic burden with schizophrenia — Kety adoptive studies and modern genetic research (Cross-Disorder Group 2013) — risk of schizophrenia is increased in relatives of schizotypal patients.
- Polygenic risk architecture similar to schizophrenia, but lesser 'load'; presence of some protective alleles.

4.2 Neurobiology

- Observed milder variants of structural changes in schizophrenia — volume reduction of temporal cortex, but preservation of frontal cortex (Hazlett E.A. et al. Schizophr Bull 2011 review).
- Dopaminergic dysregulation — lower than in schizophrenia; psychotic decompensation less likely.
- Cognitive deficits — as in schizophrenia, but milder.

4.3 Environmental factors

- Childhood trauma, social deprivation — risk factors.
- Cannabis use in adolescence increases the risk of psychotic decompensation.

5. Clinical features

5.1 Cognitive-perceptual symptoms

- Ideas of reference — not at full delusional level; the patient may at times recognise these thoughts as unreal.
- Odd beliefs or magical thinking – telepathy, 'sixth sense,' superstitions inappropriate to age and culture.
- Unusual perceptual experiences – impersonal, transient illusions; full hallucination absent.
- Odd thinking and speech – vague, metaphorical, indirect, stereotyped.

5.2 Interpersonal symptoms

- Discomfort in close relationships; social isolation.
- Paranoid idea or suspiciousness.
- Inappropriate or restricted affect.
- Odd, eccentric, or peculiar behavior and appearance.

- Scarcity of close friends other than first-degree relatives.

5.3 Course

Typically begins in adolescence and runs a continuous course; some patients may have brief psychotic decompensation episodes (in the context of stress, cannabis) — at which point the diagnosis may be reassessed as ‘schizophrenia’ or ‘acute and transient psychotic disorder’.

6. Diagnosis

6.1 Unified diagnostic criteria (DSM-5-TR · ICD-11 consensus points)

A. Social and interpersonal deficits and cognitive-perceptual disturbances characterized by persistent pattern, onset in adulthood, with at least five of the following symptoms:

1. Relationship thoughts (excluding referential delusions);
2. Beliefs or magical thinking inconsistent with age and culture;
3. Unusual perceptual experiences (including illusions);
4. Odd thinking and speech (vague, indirect, metaphorical, stereotyped);
5. Suspiciousness and paranoid ideas;
6. Inappropriate or restricted affect;
7. Odd, eccentric, or peculiar behavior or appearance.
8. Scarcity of close friends or confidants other than first-degree relatives;
9. Extreme social anxiety not decreasing with familiarity (based on paranoid fear, not self-esteem).

B. Does not occur during the course of persistent schizophrenia, affective disorder with psychotic features, or autism spectrum disorder.

6.2 Differences between sources

- ICD-11 (6A22): “Schizotypal Disorder” — in the chapter on psychotic disorders; as a persistent disorder, but not meeting criteria for schizophrenia.
- DSM-5-TR (F21): dual coding — Cluster A Personality Disorder and schizophrenia spectrum listing. Adult age requirement; in childhood and adolescence may be recorded as “premorbid features.”
- NICE: no specific clinical guidelines for schizotypal disorder; principles from CG78 (BPD) and CG178 (Psychosis) apply.

6.3 Diagnostic algorithm

1. Clinical interview + relative/friend information — enduring personality traits vs episodic psychosis.
2. Structured interview — SCID-5-PD (Personality Disorders), SIDP-IV.
3. Schizotypy scales — SPQ (Schizotypal Personality Questionnaire, Raine A.).
4. Separate assessment of psychotic symptoms — subclinical level of positive symptoms on PANSS.
5. Comorbidity screening (depression, anxiety, substance use, other personality disorders).
6. Medical and neurological examination, toxicological screening.
7. Assessment of transition risk to schizophrenia (ultra-high risk criteria — CAARMS, SIPS).

6.4 Differential diagnosis

Disorder	**Distinguishing features**
Schizophrenia (6A20)	Full psychotic episode ≥ 1 month; functional impairment significant.
Delusional disorder (6A24)	Concrete systematized delusion, general function relatively preserved.
Paranoid personality disorder (6D10.x)	Suspiciousness dominant; perceptual bizarre experiences are absent.
Schizoid personality disorder	Social isolation, but no cognitive-perceptual peculiarities.
Autism spectrum disorder (6A02)	Early onset, social-communicative deficit, restricted-repetitive behavior.
BPD (6D10.x)	Emotional lability, identity disturbance, impulsivity.
Substance-induced psychotic manifestations	Associated with substance use or withdrawal.

7. Examination and assessment

7.1 Clinical scales

- SPQ (Schizotypal Personality Questionnaire, Raine A.) — 74 items, self-report; 9 subscales (based on DSM criteria).
- SCID-5-PD — structured personality disorder interview.
- CAARMS (Comprehensive Assessment of At Risk Mental States) — risk assessment for transition to schizophrenia.
- SIPS (Structured Interview for Prodromal Syndromes).
- PANSS, BPRS — subclinical level of psychotic symptoms.

7.2 Laboratory investigations

No specific routine indication. Standard psychiatric initial indicators (complete blood count, thyroid, liver, kidney, glucose); toxicological screening.

7.3 Instrumental investigations

Not routinely required; neuroimaging in clinical suspicion.

8. Treatment

8.1 General principles

1. Psychotherapy first line — supportive psychotherapy, CBT (especially for paranoid thinking and social anxiety), social skills training.
2. Pharmacotherapy is symptomatic, does not cure the “core” disorder:
    - Low-dose antipsychotic (risperidone 0.5–2 mg, olanzapine 2.5–10 mg, aripiprazole 2–10 mg) — for psychotic-like symptoms; intermittent, not continuous.
    - SSRI — for social anxiety and comorbid depression.
    - Benzodiazepine short-term — acute anxiety.
3. Early Intervention Services (EIP) — preventive intervention in patients at risk of transition to schizophrenia (UHR — Ultra High Risk criteria).
4. Psychosocial support — maintenance of functional level, employment, social connections.
5. Cannabis cessation — crucial for reducing risk of psychotic decompensation.
6. Stress management — stress can be a trigger for decompensation.

7. Family psychoeducation — information on transition markers to schizophrenia; patient's social support system.

8.2 Psychotherapy Approaches

- CBT — for paranoid thinking, social anxiety, cognitive impairments; reality testing and alternative explanations.
- Social skills training — reduction of social isolation and bizarre behaviors.
- Mentalization-based and supportive approaches — enhancement of interpersonal functional level.
- Schema Therapy — for personality-based enduring models.

8.3 Pharmacotherapy — Specific Use

- Subclinical psychotic symptoms (referential ideas, odd experiences) when exacerbated — low-dose atypical antipsychotic intermittently.
- Comorbid depression — SSRI (sertraline, escitalopram).
- Social anxiety — SSRI, short-term benzodiazepine (for specific situations).
- Stress-induced decompensation — hospitalisation or short-term intensive intervention.

8.4 Source-Specific Clarifications

- Evidence base is limited — Extensive RCTs on specific treatment for schizotypal disorder are scarce; main approach extrapolated from interventions for schizophrenia and personality disorders.
- Studies by Koenigsberg H.W. et al. — Low-dose antipsychotic research in schizotypal disorder (moderate effect).
- NICE — No separate indication; CG178 and CG78 principles.

8.5 Treatment methods

1. Schizotypal Personality Questionnaire (SPQ — Schizotypal Personality Questionnaire) — Raine (Raine A.) — 74-item self-assessment scale; 9 subscales corresponding to DSM criteria for schizotypal personality disorder. Widely used in clinical and population studies.
2. Comprehensive Assessment of At-Risk Mental States (CAARMS) — Transition risk assessment to schizophrenia — subclinical level of positive symptoms, cognitive changes, behavioral impairments, functional level. Yung A.R. et al. Aust N Z J Psychiatry 2005.

3. Structured Interview for Prodromal Syndromes (SIPS) — Miller (Miller T.J.), McGlashan (McGlashan T.H.) — Similar to CAARMS; widely used in the USA.
4. Early Intervention Services (EIP) — for UHR patients — multidisciplinary intervention; antipsychotics usually not recommended prophylactically (side effect burden), psychosocial intervention superior.
5. Personality disorders adapted CBT — for schizotypal personality — working on cognitive impairments, social fears, paranoid thinking.
6. Social Skills Training — structured group or individual sessions — development of social interaction skills.

9. Prognosis

Good prognostic factors

- Mild symptom intensity.
- Management of comorbidity (depression, anxiety, substance).
- Social support and sustained relationships.
- Psychotherapy participation.
- Cannabis cessation.

Poor prognostic factors

- Schizophrenia in family history.
- Cannabis or other psychoactive substance use.
- Severe cognitive deficits.
- Social isolation and functional impairment.
- Meeting UHR criteria.

Follow-up targets

- Periodic assessment of transition markers to schizophrenia (CAARMS / SIPS).
- Comorbidity screening (depression, anxiety, substance).
- Functional level — employment, social relationships.
- Management of stress and psychotic decompensation triggers.
- Monitoring side effects in patients taking antipsychotics.

10. Myths and misconceptions

10.1 Etiology and conceptual myths

Myth 1: "Schizotypal personality is just an 'eccentric' or 'creative' person, requiring no clinical diagnosis"

Why it is widespread: distribution of schizotypal traits (odd beliefs, eccentricity) as a continuous spectrum in the population — many in the general population carry mild forms of 'schizotypy' traits.

Clinical and biological rationale: a clinical diagnosis requires functional impairment — social isolation, occupational impairment, distress. Simple creativity or eccentricity alone not diagnostic basis. But when criteria are met, schizotypal disorder is a real clinical condition benefiting from intervention.

Real clinical step: diagnosis should be based on criteria (5/9 symptoms + functional impairment).

Myth 2: "Schizotypal disorder inevitably converts to schizophrenia"

Evidence: Schultze-Lutter F. et al. Schizophr Res 2014 — transition from schizotypal disorder to schizophrenia ~10–25%; majority remain stable or show remission. Family history, cannabis use, severity of cognitive deficit increase transition risk.

Real clinical step: Provide family with realistic prognosis; UHR criteria are monitored; prophylactic antipsychotic usually not recommended.

Myth 3: "Schizotypal disorder arises from childhood trauma or poor upbringing"

Evidence: Genetic component is significant — Kety Danish adoptive studies; Cross-Disorder Group 2013. Childhood trauma is indicated as a risk factor, but not the main etiology; biological and psychosocial interaction.

10.2 Misguided treatment approaches

Myth 4: "Schizotypal patients must definitely take continuous antipsychotics"

Evidence: continuous antipsychotic not recommended for schizotypal disorder (limited evidence base + side effect burden). Low-dose antipsychotic intermittent can be used as symptom-specific intervention; for continuous assignment — individual assessment.

Myth 5: “Validating the schizotypal patient's ‘magical thinking’ is treatment”

Clinical logic: explicitly confirming delusion-like beliefs reinforces them; conversely, explicitly denying them also disrupts the therapeutic alliance. Reality testing and alternative explanations: an evidence-based approach in CBT — the patient and therapist explore factual evidence for the belief or alternative explanations.

Myth 6: “Social isolation is a healthy strategy; the schizotypal patient should be left alone”

Evidence: Social isolation decreases long-term functional level and increases risk of mood disorders. Gradual strengthening of social connections is recommended with social skills training and supportive therapy.

10.3 Ineffective methods or those delaying primary intervention

Myth 7: Alternative clinics based on magical thinking and “sixth sense” cure schizotypal disorder

Evidence: “Esoteric interventions” (crystal therapy, chakra work, astrology consultations) — may support bizarre beliefs of schizotypal patients and intensify disorder symptoms. No evidence of effect; delays primary psychotherapeutic or pharmacotherapeutic intervention.

Myth 8: Cannabis “preserves creativity and unusual experiences” — safe for schizotypal patients

Evidence: Cannabis increases psychosis risk by 2–3 times (Marconi 2016); conversion risk to schizophrenia is higher in schizotypal patients. Abstinence from cannabis is a critical clinical recommendation.

Myth 9: “Homeopathy, acupuncture, esoteric interventions cure schizotypal disorder”

Evidence: systematic reviews — the effect of these interventions for schizotypal disorder has not been proven.

Myth 10: “Schizotypal patient does not respond to psychotherapy, so simple observation is sufficient”

Evidence: CBT, social skills training, and supportive psychotherapy can help these patients; a ‘watchful waiting’ approach increases functional impairment.

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ICD-11 **6A23**	**6A23 ACUTE AND TRANSIENT PSYCHOTIC DISORDER** **Acute and transient psychotic disorder**
ICD-10 F23	Acute and transient psychotic disorders
DSM-5-TR F23	Brief Psychotic Disorder

1. Definition and nosology

Acute and transient psychotic disorder (ICD-11: 6A23; DSM-5-TR: F23 Brief Psychotic Disorder) — a disorder characterized by rapid onset (a few days to weeks), psychotic symptoms (delusions, hallucinations, disorganized speech, grossly disorganized behavior), short duration, and full recovery to premorbid functional level.

Characteristic features:

- Rapid onset — Full clinical picture formation within 2 weeks;
- Polymorphic (multiform) symptoms — delusions, hallucinations, and emotional fluctuations together.
- Short term — in DSM-5-TR < 1 month, in ICD-11 < 3 months;
- Full functional recovery — patient fully returns to premorbid level.

2. History

- Magnan V. (1893) — The term “bouffée délirante” describes acute-onset, short-lived polymorphic psychotic episodes in France.
- Wernicke K., Kleist K., Leonhard K. (1900–1957) — The concept of “cyclic psychoses” — from the German psychiatric school; rapid onset and favorable prognosis.
- Scandinavian school — “psychogenic psychosis” — Strömgren E. (1940) — psychosis in the context of stress or trauma.
- DSM-III (1980) — “Brief Reactive Psychosis” — with requirement of stress factor.

- DSM-IV (1994) and DSM-5 (2013) — stressor criterion has been removed (remains as a qualifier); term ‘Brief Psychotic Disorder’.
- ICD-11 (2019) — term “Acute and Transient Psychotic Disorder” retained; 3-month duration; polymorphic symptomatology emphasized.

3. Epidemiology

- Annual incidence: 3.9–9.6 / 100 000 (Castagnini A., Berrios G.E. Curr Psychiatry Rep 2009 review).
- Sex: Relatively frequent in women (1.5–2:1) — especially in postpartum context.
- Age of onset: 20–35 years; slightly later than schizophrenia.
- Cultural and geographic: frequency higher in low and middle income countries (Susser E. et al. Soc Psychiatry Psychiatr Epidemiol 1998 meta-analysis of 10 country studies).
- Postpartum onset — belongs to the category of postpartum psychosis (6E20 / 6A23.x).
- Diagnostic persistence and conversion: 30–50% of patients are re-diagnosed with schizophrenia, schizoaffective disorder, or bipolar disorder on long-term follow-up (Castagnini A. et al. Schizophr Res 2013).

4. Aetiology and pathogenesis

4.1 Genetic factors

- Schizophrenia and bipolar disorder partially share genetic risk.
- Risk increases when psychotic or affective disorder is present in family history.

4.2 Triggers and stress

- Acute psychosocial stress (death, displacement, natural disaster) — identified in a significant proportion of patients; indicated as a stress qualifier in DSM-5-TR.
- Postpartum period — hormonal and sleep disturbances.
- Substance use — cannabis, amphetamine, hallucinogens — can trigger a psychotic episode, but substance-induced psychosis is coded as a separate category if the substance is specified.
- Short-term sleep deprivation, intense emotional tension.

4.3 Neurobiological mechanisms

- Temporary hyperactivity of the dopaminergic system — in the context of stress cortisol or hormonal changes.
- Central mechanisms similar to schizophrenia, but shorter-term and potentially reversible.

5. Clinical features

5.1 Classic presentation

- Rapid onset — from premorbid normal function to full psychotic picture within days-weeks.
- Polymorphic (variable) symptoms — delusions (persecution, relationship, religious), hallucinations (auditory, visual, cenesthetic), disorganized speech.
- Emotional fluctuation — acute fear, ecstasy, panic, bewilderment.
- No alterations in consciousness (except delirium).
- Behavior — odd, disorganized; sometimes with catatonic components.

5.2 ICD-11 subtypes

- Polymorphic psychotic disorder — without schizophrenia symptoms;
- Polymorphic psychotic disorder — with schizophrenia symptoms;
- Acute psychotic disorder like schizophrenia — Schneider first-rank symptoms dominant.

5.3 Course

Typical course — peak achieved within 2–4 weeks, then gradual or rapid subsidence. Full recovery — within 1 month (DSM-5-TR) or 3 months (ICD-11). Persisting beyond this period leads to diagnosis transition to schizophreniform (DSM-5-TR F20.81, 1–6 months) or schizophrenia.

6. Diagnosis

6.1 Unified diagnostic criteria (DSM-5-TR · ICD-11 consensus points)

A. Presence of one or more psychotic symptoms:

1. Delusions;
2. Hallucinations;
3. Disorganized speech;
4. Grossly disorganized or catatonic behavior.

(At least one of 1, 2, or 3 must be present.)

B. Duration of the episode:

- DSM-5-TR: ≥ 1 day, < 1 month, then full recovery to premorbid functional level.
- ICD-11: full syndrome forms within 2 weeks; < 3 months persists.

C. Exclusions — not fully explained by affective disorder with psychotic features, schizophrenia, schizoaffective disorder, substance-induced psychosis, organic psychosis.

6.2 Differences between sources

- DSM-5-TR (F23): qualifiers — "With marked stressor(s)" (with an acute stressor), "Without marked stressor", "With postpartum onset" (within 4 weeks of birth).
- ICD-11 (6A23): Three subtypes – polymorphic (without/with schizophrenia symptoms) and schizophrenia-like. Postpartum onset is assigned to the separate code 6E20.
- Stress factor — not a criterion but a qualifier; was compulsory in DSM-IV's "Brief Reactive Psychosis."

6.3 Diagnostic algorithm

1. Clinical interview + information from relatives — onset dynamics, premorbid function.
2. Structured interview (SCID-5, MINI).
3. Severity scales — PANSS, BPRS.
4. Toxicology screening (mandatory) — to rule out substance-induced psychosis.
5. Physical and neurological examination.
6. Laboratory: complete blood count, liver, kidney, electrolytes, glucose, TSH, B12, syphilis, HIV.
7. EEG and brain MRI — first episode or atypical clinical presentation.
8. In postpartum context — thyroid function mandatory.
9. Assessment of suicide and child harm risk (in postpartum conditions).
10. Monitoring — may vary based on diagnosis duration (after 1 or 3 months elapsed).

6.4 Differential diagnosis

Disorder	Distinguishing features
Schizophrenia (6A20)	Duration ≥ 1 month (ICD-11) / ≥ 6 months (DSM-5-TR general); residual symptoms.
Schizoaffective disorder (6A21)	An affective episode is present during a significant portion of the psychotic symptoms.
Schizophreniform disorder (DSM-5-TR F20.81)	DSM-5-TR — 1–6 months; ICD-11 — no separate code.
Bipolar I with psychotic features (6A60.x)	In the context of a manic episode.
Postpartum psychosis (6E20)	Within 4–6 weeks postpartum; high risk of infanticide and maternal suicide — medical emergency.
Substance-induced psychosis	Clear temporal relationship with substance (cannabis, amphetamine, hallucinogen); toxicology screening.
Secondary psychosis (6E61)	Associated with organic cause (autoimmune encephalitis, lupus, paraneoplastic encephalitis).
Delirium	Alteration in consciousness; attentional fluctuation; organic cause.
Dissociative episode	In trauma context; desocial isolation dominant.

7. Examination and assessment

7.1 Clinical scales

- PANSS, BPRS — psychotic severity.
- HAM-D, YMRS — assessment of affective components.
- C-SSRS — suicide risk.
- GAF/PSP — functional level.

7.2 Laboratory investigations

According to first episode psychosis protocol (see 6A20 §7.2):

- Absolute: complete blood count, liver/kidney, glucose, electrolytes, TSH, B12, folate, toxicology screening, HIV, syphilis, pregnancy test, prolactin.
- If organic psychosis is suspected — anti-NMDA-R antibodies (CSF), ANA, lupus panel, paraneoplastic antibody panel, ammonia, ceruloplasmin.
- In postpartum context — TSH (postpartum hypothyroidism possible).

7.3 Instrumental investigations

- Brain MRI — recommended in first episode (NICE CG178); especially with atypical presentation or neurological signs.
- EEG — rule out seizures, atypical clinical presentation.
- EKG — before starting antipsychotic.
- Lumbar puncture — suspected autoimmune encephalitis (atypical psychosis + motor symptoms + female age).

8. Treatment

8.1 General principles

1. Urgent assessment and hospitalization if necessary — for safety of patient or others.
2. First-line: atypical antipsychotic at low dose — risperidone 1–4 mg, olanzapine 5–15 mg, quetiapine 200–600 mg, aripiprazole 5–15 mg.
3. In acute agitation — oral lorazepam 1–2 mg + oral disintegrating olanzapine 5–10 mg; intramuscular — olanzapine 10 mg im or aripiprazole 9.75 mg im, if necessary haloperidol 5 mg + lorazepam 2 mg im (classic).
4. Duration — antipsychotic typically 3–6 months (after remission), then gradual reduction and monitoring; there is no standard discontinuation protocol after a first episode — individual assessment required.
5. In stress-induced context — psychosocial support, CBT-based stress management.
6. In postpartum onset — as a medical emergency; risk of child homicide and suicide is high; hospitalization, mother-child co-hospitalization conditions are preferred; see chapter 6E20.
7. ECT — in refractory severe cases, with catatonic features, safest option during pregnancy.

8. Long-term follow-up — 30–50% of patients transition to long-term psychotic disorder; monthly in the first year, then quarterly.

8.2 Treatment methods

1. Early Intervention Services (EIP — Early Intervention in Psychosis) — Multidisciplinary care for first psychotic episode over 3 years — low-dose pharmacotherapy, CBTp, family intervention, employment support. RAISE (Kane J.M. et al. Am J Psychiatry 2016) RCT. Acute and transient patients also included in EIP programs.
2. Cognitive Behavioral Therapy for psychosis (CBTp — Cognitive Behavioural Therapy for psychosis) — Re-evaluation of psychotic symptoms in "thought–emotion–behavior" formulation. NICE CG178 §1.3.7.1.
3. Family Psychoeducation — Support for patient after episode, recognition of relapse early markers, family communication training. Pharoah Cochrane 2010.
4. Electroconvulsive Therapy (ECT) — Effective with catatonic features, severe pharmacotherapy resistance, during pregnancy. NICE TA59. UK ECT Review Group Lancet 2003.
5. PANSS, BPRS — Standardized assessment of psychotic severity.

8.3 Differences between sources

- NICE CG178 (Psychosis): first episode psychosis guidelines — low-dose atypical antipsychotic first line; early intervention services (EIP) involvement; psychosocial intervention combined.
- APA Practice Guideline 2021 (Schizophrenia): There is no specific algorithm for acute and transient psychotic disorder; principles of first episode psychosis.
- NICE CG192 (Antenatal and postnatal mental health): Postpartum psychosis is a medical emergency — hospital admission, atypical antipsychotic + benzodiazepine, ECT in refractory cases.

9. Prognosis

Good prognostic factors

- Very rapid onset and short duration.
- Clear stress trigger.
- Premorbid good functional level.
- Presence of an affective component.

- Polymorphic symptomatology (without schizophrenia symptoms).
- Absence of family psychotic history.
- No cannabis or other substance use.

Poor prognostic factors

- Schizophrenia-like symptoms (Schneider first-rank).
- Absence of stress trigger.
- Gradual onset.
- Premorbid functional impairment.
- Schizophrenia in family history.
- Substance use.

Follow-up targets

- Long-term follow-up is critical — 30–50% transition to schizophrenia, schizoaffective, or bipolar; monthly in the first year, then quarterly formal monitoring.
- Recognition of relapse markers (sleep disturbance, social withdrawal, unusual experiences).
- Gradual reduction and discontinuation of antipsychotic dose — individual decision (relapse risk vs side effects).
- Comorbid substance use and trauma intervention.
- In postpartum cases — high recurrence risk in subsequent pregnancies, prophylactic treatment plan.

10. Myths and misconceptions

10.1 Etiology and conceptual myths

Myth 1: "Acute and transient psychosis is 'just a psychological breakdown', depends on individual strength"

Why it is widespread: Time correlation with stress and short-term course appears to support "psychological" explanation.

Clinical and biological rationale: Stress is a trigger factor, but not an explanation alone — not all stressed individuals experience psychosis. Genetic and neurobiological vulnerability exists. Castagnini 2013 evidence — 30–50% of patients transition to schizophrenia, schizoaffective or bipolar disorder; this indicates neurobiological vulnerability.

Real clinical step: Stress management and psychotherapeutic support are important, but antipsychotic treatment should not be delayed.

Myth 2: "Acute psychosis is as dangerous as postpartum psychosis / no, it always resolves on its own"

Clinical logic: every acute psychotic episode warrants urgent medical evaluation. Postpartum onset risk especially high (infant homicide ~4%, maternal suicide). Non-postpartum acute transient episodes also carry suicide and violence risk — patient's delusional content and behavioral control should be assessed.

Myth 3: "Presence of a stressor is a condition for diagnosis"

Evidence: In DSM-IV 'Brief Reactive Psychosis', a stressor was a criterion, but in DSM-5-TR and ICD-11 not a criterion but a qualifier — acute psychotic episodes without a stressor are also diagnostically possible.

10.2 Misguided treatment approaches

Myth 4: "Acute psychosis resolves on its own, treatment can be delayed"

Clinical logic and evidence: Marshall M. et al. Arch Gen Psychiatry 2005 — as the duration of untreated psychosis (DUP) lengthens, long-term prognosis worsens. Even in a transient episode, early antipsychotic intervention is crucial for shortening symptoms and reducing complications.

Myth 5: "An acute psychotic patient can be left alone — they can control themselves"

Evidence: In acute psychotic episodes, the patient's reality testing is impaired, and self-care, food and fluid intake, and personal safety cannot be maintained. Hospital admission or 24/7 supervision is usually required.

Myth 6: "In acute transient psychosis, exorcism or religious ritual can replace antipsychotics"

Evidence: similar to schizophrenia — prognosis worsens as DUP lengthens; religious-cultural support can be parallel but does not replace antipsychotic treatment.

10.3 Ineffective or harmful approaches

Myth 7: Acute psychosis is a "spiritual emergency" or "transition to higher consciousness"; treatment is harmful

Why it is widespread: Stan Grof and the 'transpersonal psychology' movement reinterpret psychotic experiences as 'spiritual awakening'.

Clinical logic and evidence: This approach is not accepted in modern clinical practice and evidence-based psychiatry. Psychotic symptoms cause real distress to the patient and pose risk; refusal of treatment leads to poor clinical outcome (DUP extension).

Myth 8: "Cannabis can calm an acute psychotic episode"

Evidence: Cannabis is a psychosis trigger or contraindicated — its calming effect is illusory, indirectly exacerbates symptoms; contraindicated in clinical practice.

Myth 9: "Vitamins, omega-3, valerian, St John's Wort are effective in acute psychosis"

Evidence: These complementary interventions have not demonstrated efficacy in acute psychotic episode; St John's Wort reduces antipsychotic levels via cytochrome P450 induction (interaction).

Myth 10: "Hypnotherapy or psychoanalytic deep therapy can be used in acute psychosis"

Evidence: In acute psychotic episodes, deep psychoanalytic intervention may disorganize the patient; at this stage, supportive therapy, structure, and safety are important, deep intervention is not.

11. Sources

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ICD-11 **6A24**	**6A24 DELUSIONAL DISORDER** **Delusional disorder**
ICD-10 F22	Persistent delusional disorders
DSM-5-TR F22	Delusional Disorder

1. Definition and nosology

Delusional disorder (ICD-11: 6A24; DSM-5-TR: F22) — a psychotic disorder in which one or several delusions predominate for at least one month, without meeting the full criteria for schizophrenia. The patient's general behavior and functional level remain relatively intact except for the delusional state.

2. History

- Kraepelin (1899) — “paranoia” — systematized delusion without hallucinations.
- Esquirol J.E. (1810s) — the term “monomania”; Kahlbaum K. (1863) and Krafft-Ebing R. — the clinical description of paranoia.

- DSM-III-R (1987) — “Delusional Disorder” established as an official diagnosis; continuation of Kraepelin's paranoia concept.
- ICD-11 (2019): Bizarre delusions also permitted (in ICD-10 only non-bizarre); induced delusion (folie à deux) integrated as separate subtype.

3. Epidemiology

- Lifetime prevalence: ~0.2% (Perälä 2007).
- Onset: middle age (35–55 years), significantly later than schizophrenia.
- Sex: approximately equal; differences in some subtypes (erotomania in women).
- Comorbidity: depression, anxiety, social isolation; high suicide risk.

4. Aetiology and pathogenesis

- Genetic load is less than in schizophrenia; however, family history shows increased density of paranoid personality and psychotic disorder.
- Sensory deprivation (hearing impairment) — increases risk of paranoid delusions in the elderly.
- Social isolation, migration, language barriers — risk factors.
- Neurobiological — dopaminergic dysregulation, but more localized than in schizophrenia; increased striatal dopamine synthesis in some studies (Howes O.D. reviews).

5. Clinical features

Subtypes (by delusional content)

- Persecutory: Most frequent — being followed by others, harm planned.
- Jealousy (Othello syndrome): delusion about partner's betrayal; high criminal risk.
- Erotomanic (Clérambault syndrome): belief that another person (usually of higher status) loves the patient.
- Grandiosity: special ability, talent, discovery, religious mission.
- Somatic: Belief of parasites in the body (Ekbom syndrome), emitting odor, deformity.
- Mixed and nonspecific.

Functional maintenance

Outside the delusion, the patient's general behavior, affect, cognitive functions are relatively preserved; work and relationships can continue as long as they do not touch on delusion content.

6. Diagnosis

6.1 Unified diagnostic criteria (DSM-5-TR · ICD-11)

A. One or more delusions persisting: ICD-11 — at least 3 months (usually much longer), DSM-5-TR — at least 1 month.

B. Criterion A for schizophrenia is not met (delusion is the sole or dominant symptom; hallucinations, if present, are minimal and related to the delusional content).

C. Functional preservation without direct effect of delusion; behavior not bizarre.

D. If an affective episode is present, it occurs during a small portion of the delusional period.

E. Exclusion of substance or organic cause.

6.2 Differences between sources

- ICD-11: Permission for bizarre delusions (also removed prohibition in DSM-5-TR); induced delusion (folie à deux) integrated into 6A24.
- DSM-5-TR: Subtype qualifiers (erotomanic, grandiose, jealous, persecutory, somatic, mixed, unspecified); ‘bizarre type’ additional qualifier.

6.3 Diagnostic algorithm

1. Clinical interview (general and especially delusional content).
2. Relative/friend interview – premorbid function, onset of delusions.
3. SCID-5 / MINI; PANSS / BPRS Positive Symptom Subscale.
4. Medical and neurological examination.
5. Toxicology screening; sensor (hearing, vision) test — mandatory in elderly.
6. Brain MRI (at first episode).
7. Comorbid depression and suicide risk assessment.

6.4 Differential diagnosis

Disorder	Distinguishing features
Schizophrenia (6A20)	Hallucinations, disorganized speech, negative symptoms dominant; significant impairment in functioning.
Schizoaffective disorder (6A21)	An affective episode is present during a significant portion of the psychotic symptoms.
Body dysmorphic disorder (6B21)	Appearance-based; degree of insight is variable, but may be at the level of delusion.
OCD — with poor insight (6B20)	Ego-dystonic obsession primary; distinguished from delusion.
Paranoid personality disorder	Persistent suspiciousness as a personality trait; no full delusions.
Secondary delusion (6E61)	Organic cause — dementia, brain tumor, lupus, encephalitis.
Substance-induced (especially amphetamine, cocaine)	In the context of substance use or withdrawal; remission after elimination.

7. Examination and assessment

- PANSS / BPRS positive symptom subscale; structured documentation of concrete delusion content.
- Hearing and vision test (mandatory in the elderly).
- Laboratory: complete blood count, thyroid, B12, liver, glucose, toxicology screening, syphilis, HIV, ANA (lupus), ceruloplasmin (Wilson).
- Brain MRI — first episode, atypical clinical picture, elderly patient.
- Assessment of suicide and violence risk (C-SSRS, especially in jealousy delusions).

8. Treatment

1. Atypical antipsychotic at low dose (risperidone 1–3 mg, olanzapine 5–10 mg, aripiprazole 5–15 mg) — first-line. Evidence base is less than for schizophrenia; patient adherence is problematic (due to delusional content).

2. Pimozide was historically considered superior in erotomania and somatic delusions (Munro A. studies), but in modern practice it is used less often than the atypicals (QTc, EPS).
3. Psychotherapy — supportive therapy, building therapeutic alliance; CBT (non-confrontational work with delusional content, exploring alternative explanations).
4. Comorbid depression — SSRI.
5. Violence or suicide risk (especially delusions of jealousy) — hospitalisation and involuntary treatment within the legal framework.
6. Sensory disorder — hearing devices or vision correction – sometimes have profound effect in elderly paranoid delusions.
7. Social support — reduction of isolation; family psychoeducation.

8.1 Treatment methods

1. Non-confrontational CBT — Explore alternative explanations and evidence with the patient without openly denying the delusion content. Birchwood M., Trower P. studies — effective in schizophrenia and delusional disorder.
2. Supportive therapy and therapeutic alliance — Establish long-term trusting relationship with the patient; strengthening of adherence.
3. PANSS positive symptom subscale — Standardized assessment of delusion severity and interference degree.
4. Columbia-Suicide Severity Rating Scale (C-SSRS) — Suicide risk — particularly in jealousy and persecutory delusions, violence risk additionally assessed.

8.2 Differences between sources

- NICE CG178 — no separate algorithm for delusional disorder; general psychosis principles.
- APA 2021 — antipsychotic first-line; insight and adherence difficulty emphasized.
- Munro A. reviews (Cambridge Univ Press 1999) — subtype-specific clinical guidance.

9. Prognosis

- Prognosis is better than schizophrenia — functional preservation, late onset.
- Adherence is the main limitation.
- Jealousy and somatic delusions relatively more difficult to treat.

- In older adults, improvement with sensory correction is possible.
- Monitoring — antipsychotic side effects, delusion severity, violence and suicide risk.

10. Myths and misconceptions

Myth 1: "The patient must be convinced of the falseness of their delusion"

Clinical logic: open confrontation disrupts therapeutic alliance and may reinforce delusions; the patient arrives at an explanation of his own: the doctor is against me too.

Evidence: Birchwood and Trower CBT approaches — a non-confrontational approach allowing the patient to explore alternative explanations themselves is effective.

Myth 2: "Delusional disorder can only be treated with psychotherapy, antipsychotics are unnecessary"

Evidence: An antipsychotic is the first-line pharmacotherapy; psychotherapy alone is insufficient. APA 2021, NICE CG178.

Myth 3: "Erotomania is only a female disorder, absent in males"

Evidence: Erotomania occurs in both sexes; higher risk of violence reported in male patients (Menzies R.P. et al. Br J Psychiatry 1995).

Myth 4: "Delusional jealousy is a sign of 'great love', not a medical condition"

Evidence: Othello syndrome is a clinical psychotic condition, high risk of violence and homicide against partner; requires active medical intervention.

Myth 5: "Paranoid delusions in the elderly are 'normal aging'"

Evidence: In older adults, paranoid delusions are often a marker for sensory deprivation (auditory impairment) or early dementia; it can be treatable. Hearing aids or vision correction sometimes yield significant improvement.

Myth 6: "Exorcism or religious intervention cures delusions"

Evidence: Psychotic symptoms are neurobiological; antipsychotic treatment delayed.

Myth 7: “The delusional patient must be isolated — social contact is harmful”

Evidence: Social isolation, conversely, strengthens psychosis; social support and psychotherapy are recommended.

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	6A25 SYMPTOMATIC MANIFESTATIONS OF PRIMARY PSYCHOTIC DISORDERS
ICD-11 6A25	**Symptomatic manifestations of primary psychotic disorders**
ICD-10 —	No ICD-10 equivalent
DSM-5-TR F29	Unspecified Schizophrenia Spectrum and Other Psychotic Disorder

1. Definition and nosology

6A25 In ICD-11, the group of primary psychotic disorders (6A20–6A24) has a dimensional sub-structure. It is not a separate clinical nosological unit — the diagnosis is placed as an additional qualifier onto the primary disorder (e.g., 6A20 Schizophrenia). Six symptom domains — Each is assessed separately for severity level (none / mild / moderate / severe).

The basis of the concept — to describe the heterogeneous clinical presentation of psychotic disorders using a more valid and dimensional measure instead of classic subtypes (paranoid, hebephrenic, catatonic).

2. History

- Bleuler's “4A” symptoms (1911) and Schneider's “first-rank symptoms” (1959) — categorical approach.
- Crow T.J. (1980) — Concept of Type I (positive symptom dominant) and Type II (negative dominant) — the basis of the dimensional approach.
- Liddle P.F. (1987) — three-dimensional factor — reality distortion, disorganization, psychomotor poverty.
- DSM-5 (2013) — Section III research category proposes dimensional assessment.
- ICD-11 (2019) — 6A25 integrated as an official dimensional qualifier.

3. Six symptom domains

3.1 Positive symptoms

Delusion, hallucination — “excessive/added” symptoms in perception and thought processes. Severity: from absence to severe level (according to degree of impact on patient's daily activities).

3.2 Negative symptoms

Affective flattening, avolition (loss of motivation), alogia (poverty of speech), anhedonia, social withdrawal. A major factor in functional impairment; responds poorly to pharmacotherapy.

3.3 Depressive symptoms

Low mood, hopelessness, suicidal ideation, psychomotor retardation, anhedonia (emotional component dominant, as opposed to negative symptom).

3.4 Manic symptoms

Elevated or irritable mood, rapid thinking, grandiose ideas, decreased sleep need, rapid speech, psychomotor agitation.

3.5 Psychomotor symptoms

Catatonic components (mutism, negativism, waxy flexibility, posturing), agitation, retardation, stereotypy.

3.6 Cognitive symptoms

Working memory, attention, executive function, processing speed, social cognition. Present in 75–85% of patients; strong impact on long-term functional level.

4. Clinical use

4.1 Assessment tools

- PANSS (positive, negative, general psychopathology subscales).
- SANS / SAPS — negative and positive symptoms separately.
- HAM-D, MADRS — depressive component.
- YMRS — manic component.
- Bush-Francis Catatonia Rating Scale — psychomotor component.
- MATRICS Consensus Cognitive Battery (MCCB) — cognitive component.

4.2 Treatment implications

- Positive symptoms predominate — standard antipsychotic (D2 antagonist).
- Negative symptoms predominate — atypical antipsychotic superior (clozapine, cariprazine); amisulpride low dose; psychosocial rehabilitation.
- Depressive component — adjunctive SSRI (sertraline) or antidepressant; suicide risk monitoring.
- Manic component — mood stabilizer (lithium, valproate) adjunct; reevaluation as schizoaffective disorder.
- Catatonic component — lorazepam test or ECT.
- Cognitive component — cognitive remediation (Wykes 2011 meta-analysis); enhances effect on functional level when applied together with supported employment.

5. Differences between sources

- ICD-11 (6A25): periodic reassessment recommended as severity per domain changes during illness course.
- DSM-5-TR: Section III research category ‘Clinician-Rated Dimensions of Psychosis Symptom Severity’ — parallel to ICD-11 6A25; not an official diagnostic qualifier, but can be used in practice.
- NICE CG178: dimensional assessment recommended as clinical approach — basis for individualized treatment.

6. Myths and misconceptions

Myth 1: “6A25 is a separate disorder, can be diagnosed as such”

Reality: 6A25 is not a separate clinical entity — it is a qualifier of the primary disorder (6A20–6A24).

Myth 2: “Dimensional approach replaces categorical diagnosis”

Reality: both complementary — categorical diagnosis provides nosological unit, dimensional qualifier describes specific clinical presentation.

Myth 3: “Negative symptoms are side effects of antipsychotics, not ‘real’”

Reality: Primary negative symptoms are a core feature of schizophrenia — observed even before antipsychotics. Secondary negative symptoms (sedation, EPS, depression, positive symptom effect) are distinct and manageable.

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6A40 CATATONIA

Catatonia

	6A40 CATATONIA ASSOCIATED WITH ANOTHER MENTAL DISORDER
ICD-11 **6A40**	**Catatonia associated with another mental disorder**
ICD-10 F06.1	Organic catatonic disorder
DSM-5-TR F06.1	Catatonia Associated With Another Mental Disorder (Catatonia Specifier)

1. Definition and nosology

Catatonia associated with another mental disorder (ICD-11: 6A40; 6A41 Catatonia Induced by Substances or Medications; 6E69 Secondary Catatonia Syndrome; DSM-5-TR: F06.1 Catatonia) — syndrome of complex disturbance in psychomotor, behavioral and autonomic functions. Not an independent disorder — clinical syndrome on background of another psychiatric or medical condition.

DSM-5-TR — catatonia is no longer a subtype of schizophrenia (that status was abolished); can be coded in the context of affective disorders, neurodevelopmental disorders, organic conditions, and substance-induced states.

2. History

- Kahlbaum K. (1874) — “Die Katatonie oder das Spannungsirresein” — catatonia as a separate syndrome.
- Kraepelin (1899) — catatonia integrated as ‘dementia praecox’ subtype.
- Bleuler (1911) — retained within schizophrenia category.
- 1970s–1980s — Taylor, Fink — catatonia's widespread presence in affective disorders and reconfirmation as a general syndrome.

- DSM-5 (2013) and ICD-11 (2019) — catatonia formalized as transnosological syndrome; schizophrenia subtype status abolished.

3. Epidemiology

- 7–15% of patients admitted to psychiatric hospital have catatonic symptoms (Fink M., Taylor M.A. Catatonia 2003).
- Catatonia is most prevalent in affective disorders (especially bipolar manic and depressive episodes).
- In schizophrenia, 10–20% of patients exhibit catatonic features during the course.
- Organic catatonia — autoimmune encephalitis (especially anti-NMDA-R, anti-LGI1), electrolyte disturbances, uremic and hepatic encephalopathy, sepsis, neurosyphilis, neuromuscular disease context.
- Persistent clinical overlap with Neuroleptic Malignant Syndrome (NMS) — some researchers consider them the same spectrum.

4. Aetiology and pathogenesis

- GABA-ergic system hypofunction — the rapid clinical effect of lorazepam (within hours) supports this; GABA-A receptor dysfunction is the central mechanism.
- Dopamine dysregulation — neuroleptic-induced catatonia and within the spectrum of NMS; antipsychotic discontinuation causes cholinergic and adrenergic rebound.
- Glutamate NMDA dysregulation — autoimmune anti-NMDA-R encephalitis presents a classic catatonic presentation.
- Genetic — predisposition associated with affective disorders.

5. Clinical features

DSM-5-TR criteria — at least 3 of 12 symptoms:

1. Stupor — psychomotor activity absence, unresponsiveness to surroundings.
2. Catalepsy — maintaining posture against passive induction.
3. Waxy flexibility — posture induced by the examiner is maintained as though moulded in wax.
4. Mutism — absence of speech or significant reduction.
5. Negativism — opposition to instructions or external stimuli.
6. Posturing — involuntary spontaneous posture maintenance (against gravity).

7. Mannerism — strange caricature of ordinary behaviors.
8. Stereotypy — purposeless repetitive movements.
9. Agitation — independent of external stimuli.
10. Grimacing — facial muscle expressions.
11. Echolalia — repetition of others' speech.
12. Echopraxia — repetition of others' actions.

Severity forms

- Retarded (stuporous) catatonia — motor reduction, mutism dominant; majority of cases.
- Excited (manic) catatonia — high agitation, autonomic disturbance, hyperthermia.
- Malignant catatonia — hyperthermia with excited form, autonomic instability, high CPK, mortality risk – coincides with NMS; emergency medical condition.

6. Diagnosis

6.1 Unified diagnostic criteria

DSM-5-TR — ≥3 of 12 features; ICD-11 — same clinical features across different categories.

6.2 Diagnostic tests

- Lorazepam challenge test: 1–2 mg IV/IM lorazepam → significant reduction of catatonic signs within 10–30 minutes confirms the diagnosis (Bush G. et al. Acta Psychiatr Scand 1996).
- Bush-Francis Catatonia Rating Scale (BFCRS) — 23-item standardized scale; first 14 items screening (≥ 2 signs — positive).

6.3 Etiological investigation (mandatory)

1. Complete blood count, liver/renal, electrolytes (Na, K, Ca, Mg, P), glucose, ammonia, CPK, troponin, thyroid, B12.
2. Toxicology screening.
3. Syphilis, HIV.
4. Brain MRI.
5. EEG — rule out non-convulsive status epilepticus.

6. Lumbar puncture — suspected encephalitis / autoimmune encephalitis (anti-NMDA-R, anti-LGI1 antibodies).
7. ANA, lupus panel.

6.4 Differential diagnosis

Condition	Distinguishing features
Neuroleptic Malignant Syndrome (NMS)	History of antipsychotic use; hyperthermia, rigidity, very high CPK; clinical overlap — some consider the same spectrum.
Serotonin syndrome	Serotonergic medication; hyperreflexia, myoclonus, hyperthermia.
Non-convulsive status epilepticus	EEG — epileptic activity.
Akinetic mutism (frontal lobe disorder)	Brain MRI — frontal or bilateral thalamic pathology.
Locomotor disorders (Parkinson, dystonia)	Specific neurological signs.
Autoimmune encephalitis	Anti-NMDA-R, anti-LGI1; CSF findings, MRI, EEG atypical.
Hypo/hyperthyroid crisis	TSH, T4.

7. Examination and assessment

- BFCRS — baseline ratings and monitoring.
- The above laboratory and instrumental examinations — an etiological search.
- Autonomic function (blood pressure, pulse, temperature, urine output) — especially in malignant form.
- Comorbid mood disorder and psychotic symptom assessment (later, when patient can speak).

8. Treatment

8.1 General principles (BAP 2023 · Fink-Taylor protocol)

1. STOP antipsychotics — particularly typical and high D2 antagonists (haloperidol, risperidone high dose) — can enhance catatonia, transform into malignant form.
2. Lorazepam first-line — 1–2 mg IM/IV, titration based on initial response; typical effective dose 6–24 mg/day; some patients require 30+ mg. ~70–80% patients respond (Sienaert P. et al. Front Psychiatry 2014 review).
3. ECT — refractory cases (lorazepam non-response within 48–72 hours), malignant catatonia — first-line, life-saving measure. NICE TA59 — strong recommendation in catatonia.
4. Etiological treatment — parallel: if against a background of affective disorder, mood stabilizer (lithium); in schizophrenia context — atypical antipsychotic (clozapine or quetiapine less risky) cautiously with lorazepam.
5. In autoimmune catatonia — immunotherapy (IV steroid, IVIG, plasmapheresis, second-line rituximab).
6. Supportive treatment — hydration, nutrition (NG tube necessary), DVT prophylaxis, skin care, aspiration pneumonia prophylaxis.
7. In malignant catatonia — ICU; aggressive hydration, dantrolene, bromocriptine, ECT.

8.2 Treatment methods

1. Bush-Francis Catatonia Rating Scale (BFCRS) — 23-item scale; first 14 items screening (≥ 2 positive); 23 items severity. Bush G. et al. Acta Psychiatr Scand 1996.
2. Lorazepam challenge test — reduction of catatonic signs within 10–30 min after 1–2 mg IM/IV lorazepam administration confirms diagnosis and guides therapy plan. Bush G. et al. 1996.
3. Electroconvulsive Therapy (ECT) — Gold standard in catatonia; UK ECT Review Group Lancet 2003 evidence base; NICE TA59. Typically 6–12 sessions, bilateral target.
4. Immunotherapy (in autoimmune catatonia) — IV methylprednisolone 1 g/day × 5 days → IVIG and/or plasmapheresis; in refractory cases rituximab, cyclophosphamide. Graus F. et al. Lancet Neurol 2016 autoimmune encephalitis protocol.

8.3 Differences between sources

- BAP 2023 (British Association for Psychopharmacology) Catatonia Consensus Guidelines — lorazepam is first-line; ECT is for refractory or malignant cases; avoid or use antipsychotics with extreme caution.
- Fink M., Taylor M.A. Catatonia: A Clinician's Guide (2003) — classic protocol; lorazepam test and high-dose lorazepam therapy.
- NICE TA59 — ECT open recommendation in catatonia.

9. Prognosis

- Lorazepam and/or ECT response — most patients achieve full recovery within days to weeks.
- High relapse if etiological disorder is untreated.
- Complications — aspiration pneumonia from prolonged stupor, DVT, dehydration, skin ulcers.
- Malignant catatonia — untreated mortality historically ~50%+ (up to 75–100% in some series); with modern treatment (benzodiazepines/ECT) drops to ~10–20%.
- Monitoring — in etiological disorder context; recognition of relapse markers.

10. Myths and misconceptions

Myth 1: “Catatonia is a subtype of schizophrenia”

Evidence: DSM-5 (2013) and ICD-11 (2019) — catatonia as a transnosological syndrome; schizophrenia subtype status abolished. More frequent in the context of affective disorders (Fink & Taylor 2003).

Myth 2: “Catatonic patient should be treated with antipsychotics”

Evidence: An antipsychotic may worsen catatonia and precipitate the malignant form. First-line — lorazepam, then ECT. Antipsychotic if needed for the etiological disorder, cautiously with lorazepam; atypical preferred (clozapine or quetiapine).

Myth 3: “A catatonic patient has lost consciousness or is in a coma”

Evidence: Most catatonic patients are fully aware of surroundings — mutism and psychomotor impairment are not loss of consciousness. Patient remembers everything after episode. Staff conversations near patient should be cautious.

Myth 4: "Catatonia is a rare or historical condition, absent in modern psychiatry"

Evidence: 7–15% of patients admitted to psychiatric hospitals have catatonic symptoms (Fink & Taylor 2003); the syndrome is recognised far less often than it occurs.

Myth 5: "Lorazepam only sedates, does not provide 'true' treatment"

Evidence: Lorazepam targets the core mechanism of catatonic syndrome (GABAergic hypofunction); ~70–80% of patients experience dramatic and rapid clinical recovery.

Myth 6: "ECT is dangerous and outdated in catatonia"

Evidence: ECT is the gold standard for catatonia, NICE TA59 clear recommendation; life-saving in malignant catatonia.

Myth 7: "The patient must be forced to talk — breaking the psychological barrier"

Evidence: Mutism is biologically rooted, not psychological "resistance"; demanding it increases the patient's distress and is not therapeutic.

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6A60–6A7Z MOOD DISORDERS

Mood disorders

ICD-11 **6A60**	**6A60 BIPOLAR TYPE I DISORDER** **Bipolar type I disorder**
ICD-10 F31	Bipolar affective disorder
DSM-5-TR F31.1x	Bipolar I Disorder, Current or Most Recent Episode Manic

1. Definition and nosology

Bipolar disorder Type I (ICD-11: 6A60; DSM-5-TR: F31.1x) — a recurrent disorder characterized by at least one manic episode, often alternating with depressive and/or hypomanic episodes. The presence of a manic episode is sufficient for diagnosis — a depressive episode is not required.

2. History

- Falret J.P. (1854) — Circular insanity; Baillarger J. (1854) — “folie à double forme”.
- Kraepelin E. (1899) — Manic-depressive insanity — distinct from schizophrenia (dementia praecox).
- Leonhard K. (1957) — first distinction between unipolar and bipolar disorders.
- DSM-III (1980) — term “Bipolar Disorder”.
- DSM-IV / DSM-5 (2013) / ICD-11 (2019) — Distinction between Type I (full manic episode) and Type II (hypomanic + major depressive).

3. Epidemiology

- Lifetime prevalence Type I: ~0.6–1% (Merikangas K.R. et al. Arch Gen Psychiatry 2011, WMH).
- Sex: approximately equal.

- Onset: mean 20–25 years; earlier with a family history.
- Suicide risk: lifetime 6–7%; highest in youth and in the first years.
- Comorbidity: anxiety disorders 60%, substance use 40%, ADHD, personality disorders.
- Mortality is 2 times higher compared to the population — cardiovascular, suicide.

4. Aetiology and pathogenesis

- Heritability 60–85% (Smoller J.W., Finn C.T. Am J Med Genet C Semin Med Genet 2003 twin meta-analysis).
- GWAS — 60+ risk loci (Mullins N. et al. Nat Genet 2021).
- Schizophrenia with extensive genetic overlap (Cross-Disorder Group 2013).
- Neurobiological — monoaminergic (dopamine, serotonin, norepinephrine) dysregulation; circadian rhythm disruption; second messenger (PI/cAMP) system impairment; mitochondrial dysfunction.
- Risk factors — childhood trauma, stress, sleep disturbance, cannabis, antidepressant induction (in bipolar predisposition).

5. Clinical features

5.1 Manic episode

- Abnormally elevated or irritable mood + increased energy ≥ 7 days (or any duration requiring hospitalization).
- ≥ 3 symptoms from the following (≥ 4 in irritable mood): inflated self-esteem, decreased need for sleep, pressured speech, flight of ideas, distractibility, increase in goal-directed activity, risky behaviors.
- Functional impairment or requirement for hospitalization or psychotic features.

5.2 Major depressive episode (in bipolar context)

- Low mood or anhedonia + ≥ 5 symptoms × 2 weeks (DSM-5-TR).
- Bipolar depressive episode with atypical features (hypersomnia, hyperphagia, leaden paralysis) is more common — similar to Type II.

5.3 Mixed features

DSM-5-TR — subtype abolished — as a specifier: ≥3 depressive symptoms during a manic episode or ≥3 manic symptoms during a depressive episode.

5.4 Rapid cycling

≥4 episodes within 12 months; poor prognostic marker.

6. Diagnosis

6.1 Unified diagnostic criteria (DSM-5-TR · ICD-11 consensus points)

A. At least one manic episode (the above criteria).

B. Manic and depressive episodes are not better explained by schizoaffective disorder, schizophrenia, or another psychotic disorder.

Depressive and hypomanic episodes are not required for Type I diagnosis, but are typically present

6.2 Differences between sources

- DSM-5-TR: Mixed features specifier (DSM-IV mixed episode abolished); increased energy added as a criterion for manic episode (DSM-IV only mood).
- ICD-11 (6A60): Number of manic episodes and current episode type subqualifiers; rapid cycling as a qualifier.
- NICE CG185: diagnosis supported by structured interview and family information; antidepressant-triggered hypomanic/manic episode leads to bipolar diagnosis (in DSM-5-TR with criterion B qualifier).

6.3 Diagnostic algorithm

1. Clinical interview + relative/friend information — manic and hypomanic episodes are more easily recognized by family members.
2. Structured interview (SCID-5, MINI).
3. YMRS (Young Mania Rating Scale), HAM-D / MADRS.
4. MDQ (Mood Disorder Questionnaire) or HCL-32 — bipolar screening (self-report).
5. Medical examination + laboratory (thyroid, liver, kidney, glucose, toxicological); specific tests before lithium initiation.
6. Brain MRI — first episode psychosis or atypical context.
7. Comorbidity and suicide risk (C-SSRS).

6.4 Differential diagnosis

Condition	Distinguishing features
Bipolar Type II (6A61)	No manic episode ever; hypomanic + major depressive.
Cyclothymic disorder (6A62)	≥2 years (1 year in adolescents) subthreshold mood fluctuations.
Major Depressive Disorder (6A70/6A71)	No manic or hypomanic episode.
Schizoaffective (6A21)	Psychosis persists during periods outside the affective episode.
BPD (6D10.x)	Emotional lability hours-days; identity disturbance.
ADHD (6A05)	Persistent behavioral patterns, not episodic.
Hyperthyroidism	Decreased TSH; tremor, weight loss, tachycardia.
Substance-induced (stimulant, cocaine)	Substance history and temporal relationship.

7. Examination and assessment

- YMRS, HAM-D / MADRS, MDQ, HCL-32, C-SSRS.
- Pre-lithium: complete blood count, creatinine, TSH, calcium, EKG, pregnancy test.
- Pre-valproate: complete blood count, liver function, pregnancy test (for women of childbearing potential — valproate is strictly not recommended).
- Before carbamazepine: complete blood count, liver, sodium.
- Brain MRI — first psychotic episode.

8. Treatment

8.1 General principles (NICE CG185 · CANMAT/ISBD 2018 · APA · WFSBP consensus)

1. Acute manic episode: Antipsychotic (olanzapine, risperidone, quetiapine, aripiprazole, asenapine) ± mood stabilizer (lithium, valproate). Lorazepam adjunct in acute agitation.
2. Acute bipolar depressive episode: quetiapine or lurasidone (FDA approval), olanzapine/fluoxetine combination (Symbyax); lithium or lamotrigine adjunctive therapy. Antidepressant monotherapy is contraindicated — manic switch and rapid cycling risk.
3. Maintenance: Lithium first-line — suicide reduction effect (Cipriani A. BMJ 2013); valproate, quetiapine, lamotrigine (especially with dominant depressive component), olanzapine alternatives.
4. Refractory: clozapine; ECT.
5. Rapid cycling — stop antidepressant; combination of lithium and valproate.
6. Psychosocial intervention: Psychoeducation, CBT for bipolar, IPSRT (Interpersonal and Social Rhythm Therapy — Frank E.), family-focused therapy (Miklowitz D.).
7. Adherence and side effect monitoring — lithium: TSH, creatinine, calcium every 6 months; valproate: hepatic and hematologic.
8. Women of childbearing age: valproate absolute contraindication (neural tube defects 1–2%, major congenital malformations overall ~10%, autism and IQ deficit); lamotrigine and quetiapine relatively safe.

8.2 Pharmacotherapy (Table)

Class	Drug	Indication
Lithium	Therapeutic serum level 0.6–1.0 mEq/L; in severe cases 0.8–1.2.	Manic, depressive, maintenance; reduces suicide risk.
Anticonvulsant	Valproate (1000–2000 mg), carbamazepine (400–1200 mg), lamotrigine (200 mg titration)	Manic (valproate, carbamazepine), depressive maintenance (lamotrigine).

Class	Drug	Indication
Atypical antipsychotic	Olanzapine, risperidone, quetiapine, aripiprazole, asenapine, lurasidone, cariprazine	Manic, mixed, maintenance; quetiapine and lurasidone — FDA-approved for bipolar depression.
Antidepressant	Only with mood stabilizer	Caution in bipolar depressive episode; monotherapy contraindicated.

8.3 Source-Specific Clarifications

- CANMAT/ISBD 2018 (Yatham L.N. et al. Bipolar Disord 2018): First-line for manic — lithium, valproate, quetiapine, aripiprazole, asenapine, paliperidone, risperidone; for bipolar depressive — quetiapine, lurasidone (monotherapy or adjunct with lithium/valproate).
- NICE CG185: Lithium is first-line maintenance; an antidepressant only together with a mood stabilizer.
- Cipriani A. et al. BMJ 2013 meta-analysis: Lithium significantly reduces suicide rates (superior to other mood stabilizers).
- EMA and FDA warnings: valproate in women of childbearing age — neural tube defects, autism, IQ decrease.

8.4 Treatment methods

1. Lithium — Gold standard in bipolar disorder; reduces suicide; initial parameters creatinine, TSH, calcium, EKG; therapeutic level 0.6–1.0 mEq/L; toxicity > 1.5 mEq/L. Cipriani BMJ 2013.
2. CBT adapted for bipolar disorder — Psychoeducation, early marker recognition, behavioral activation, compliance support. Lam D.H. et al. Arch Gen Psychiatry 2003 RCT.
3. Interpersonal and Social Rhythm Therapy (IPSRT — Interpersonal and Social Rhythm Therapy) — Frank (Frank E.) — Circadian rhythm and interpersonal stress management in bipolar disorder. Frank E. et al. Arch Gen Psychiatry 2005 RCT — reduction of relapse in maintenance phase.
4. Family-Focused Therapy (FFT) — Miklowitz D. — Patient + family jointly; psychoeducation, communication, problem-solving. Miklowitz D.J. et al. Arch Gen Psychiatry 2003;60(9):904–912.

5. Young Mania Rating Scale (YMRS) — 11 items, manic episode severity: initial indicators and monitoring.
6. Mood Disorder Questionnaire (MDQ — Mood Disorder Questionnaire) — Hirschfeld (Hirschfeld R.M.) — 13-item self-assessment for bipolar screening.
7. ECT — In refractory manic or depressive episode, with mixed features, suicidal risk, a safe alternative in pregnancy. NICE TA59.

9. Prognosis

- Episodic course; relapse 80–90% (untreated); significant reduction with maintenance pharmacotherapy.
- Suicide risk high (6–7% lifetime); particularly in the first years.
- Increased cardiovascular mortality (metabolic syndrome).
- Functional recovery moderate — the majority of patients experience significant social and occupational impairment.
- Monitoring — mood scales, suicide risk, metabolic indicators, lithium level, compliance.

10. Myths and misconceptions

Myth 1: “Bipolar disorder is simple mood swings — everyone has them”

Clinical logic: Bipolar manic episode requires both duration (≥7 days) and functional impairment; it is a distinct clinical condition from daily mood fluctuations. Manic episode may require psychotic features and hospitalization. Evidence: Merikangas Arch Gen Psychiatry 2011.

Myth 2: “Bad parenting causes bipolar disorder”

Evidence: heritability 60–85%; parental experience not etiology. Smoller & Finn 2003.

Myth 3: “Bipolar patient is creative or a ‘marker of genius’”

Evidence: Some studies show moderate increase in bipolar prevalence in creative professions (Andreasen N.C. reviews), but most patients experience functional impairment; ‘duality’ explanation leads to romanticization and treatment refusal.

Myth 4: “Antidepressants always help a bipolar patient”

Evidence: NICE CG185, CANMAT — antidepressant monotherapy in bipolar disorder is contraindicated; risk of manic switch and rapid cycling.

Myth 5: “Lithium is ‘outdated’ or dangerous — atypical antipsychotic should be first-line”

Evidence: Cipriani BMJ 2013 — lithium reduces suicide significantly more than other mood stabilizers; NICE CG185 and CANMAT 2018 — first-line in maintenance.

Myth 6: “The patient can ‘volitionally’ control their mood”

Evidence: bipolar episode is neurobiological impairment; not voluntarily controllable; requires pharmacotherapy or ECT.

Myth 7: “Disinterested observation is sufficient — it will resolve on its own”

Evidence: Untreated episodes — long-term functional impairment, cardiovascular mortality, suicide (6–7% lifetime).

Myth 8: “Herbal preparations (St John's Wort, valerian) or omega-3 replace mood stabilizers”

Evidence: St. John's Wort reduces the levels of mood stabilizers through cytochrome P450 induction and may trigger a manic switch; omega-3 has a mild adjunct effect, it is not the primary treatment.

Myth 9: “Cannabis is a mood stabilizer”

Evidence: Cannabis worsens bipolar course, increases psychosis risk, reduces compliance.

Myth 10: “Mood stabilizer must be completely discontinued during pregnancy”

Evidence: full discontinuation doubles the risk of recurrence, increases the proportion of weeks ill fivefold, and shortens time to recurrence more than fourfold (Viguera A.C. et al. Am J Psychiatry 2007). Valproate is contraindicated, but lamotrigine and quetiapine are relatively safe; individual balance should be assessed (NICE CG192).

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ICD-11 **6A61**	**6A61 BIPOLAR TYPE II DISORDER** **Bipolar type II disorder**
ICD-10 F31	Bipolar affective disorder
DSM-5-TR F31.81	Bipolar II Disorder

1. Definition and nosology

Bipolar disorder Type II (ICD-11: 6A61; DSM-5-TR: F31.81) — a form of bipolar disorder characterized by at least one hypomanic episode and at least one major depressive episode, but never a full manic episode. It is not a ‘milder’ form than Type I — patients spend more lifetime time in depression; functional impairment and suicide risk are comparable to Type I.

2. History

- Dunner D.L., Gershon E.S., Goodwin F.K. (1976) — proposal to differentiate bipolar disorder subtypes.
- DSM-IV (1994) — Bipolar Type II as a separate diagnostic category formalized.
- DSM-5 (2013) and ICD-11 (2019) — Type II retained, with emphasis against the erroneous concept of it as a ‘mild form’.

3. Epidemiology

- Lifetime prevalence: ~0.4–1.1% (Merikangas K.R. Arch Gen Psychiatry 2011 WMH).
- Sex: relatively higher in females (~1.5:1 — different from Type I).
- Onset: mean 20–25 years; multiple depressive episodes usually precede a hypomanic episode.
- Suicide risk comparable to Type I (lifetime ~5–7%); more often due to depressive episodes.
- Comorbidity: anxiety 60%+, substance use 30–40%, borderline personality disorder.

4. Aetiology and pathogenesis

- Genetic basis overlaps with Type I — heritability 60–80%; GWAS loci parallel.
- Genetic independence of the clinical phenotype from bipolar Type I is debated — some studies assess both types as the same spectrum.
- Circadian rhythm disturbance and family history.

5. Clinical features

5.1 Hypomanic episode

- Abnormally elevated or irritable mood + increased energy ≥ 4 days (≥ 7 in Type I).
- Same sub-symptoms as manic episode (decreased need for sleep, distractibility, increase in goal-directed activity, etc.); ≥ 3 symptoms (≥ 4 if irritable mood).
- Functional impairment should not be significant (i.e., hypomanic definition); no psychotic features; no hospitalization required. If these criteria are observed — diagnosis changes to Type I.

5.2 Major depressive episode

- ≥ 5 symptoms for 2 weeks (including low mood and/or anhedonia).
- Bipolar Type II depressive episodes often with atypical features — hypersomnia, hyperphagia, leaden paralysis, rejection sensitivity.
- Patients are symptomatic during 53.9% of follow-up weeks; depressive symptoms occupy 50.3% of weeks (hypomanic 1.3%, cycling/mixed 2.3%) — Judd L.L. et al. Arch Gen Psychiatry 2003, n=86, mean 13.4 years of follow-up; most symptoms are at subsyndromal level.

6. Diagnosis

6.1 Unified diagnostic criteria

A. At least one hypomanic episode.

B. At least one major depressive episode.

C. Never had a manic episode.

D. Hypomanic and depressive episodes are not better explained by schizoaffective disorder, schizophrenia, or another psychotic disorder.

E. Symptoms cause significant distress or functional impairment (primarily from depressive episode).

6.2 Differences between sources

- DSM-5-TR / ICD-11: functional impairment in hypomanic episode should not be significant; antidepressant-induced hypomanic episode leads to bipolar Type II diagnosis (especially in persistent or recurrent cases).

- Underdiagnosis problem: Hirschfeld R.M. reviews — ~25–50% of patients presenting with a depressive episode are actually Bipolar Type II, but hypomanic episodes are perceived by the family as "good periods."

6.3 Diagnostic algorithm

1. Structured inquiry for hypomanic episodes in every depressive patient (SCID-5 mood module, MDQ, HCL-32).
2. Information from family members — observation of hypomanic episodes is important.
3. Clinical interview + YMRS, HAM-D / MADRS.
4. Laboratory: thyroid, liver, kidney, toxicology, pregnancy test.
5. Comorbidity and suicide risk assessment.

6.4 Differential diagnosis

Condition	Distinguishing features
Bipolar Type I (6A60)	≥1 manic episode (functional impairment or hospitalization requirement).
Major Depressive Disorder (6A70/6A71)	No hypomanic episode.
Cyclothymic (6A62)	No full major depressive or hypomanic episodes.
BPD (6D10.x)	Emotional lability hours; identity disturbance; chronic suicidal ideation.
ADHD	Persistent; not episodic.
Hyperthyroidism	TSH measurement.

7. Examination and assessment

- SCID-5 mood module — the gold standard.
- MDQ (Mood Disorder Questionnaire) and HCL-32 (Hypomania Checklist) — screening.
- YMRS, HAM-D / MADRS, C-SSRS.
- Pre-lithium: creatinine, TSH, calcium, EKG.

8. Treatment

8.1 General principles (NICE CG185 · CANMAT/ISBD 2018)

1. Bipolar Type II depressive episode (most common presentation):
    - First-line — quetiapine (FDA approved) or quetiapine + lithium combination. Lurasidone's FDA indication covers bipolar I depression only, not type II; CANMAT/ISBD 2018 likewise names quetiapine alone as first-line for the type II depressive episode.
    - Lamotrigine — first-line in maintenance and depressive prophylaxis (particularly effective in Type II).
    - Lithium — maintenance and suicide reduction.
2. Hypomanic episode: Stop antidepressant; mood stabilizer (lithium, valproate) or atypical antipsychotic.
3. Antidepressant use: formerly considered relatively safe in bipolar Type II, but recent guidelines (CANMAT 2018) recommend only with a mood stabilizer (to reduce risk of manic switch and rapid cycling).
4. Maintenance: Lamotrigine (when the depressive component is dominant) or quetiapine; lithium for suicide reduction effect.
5. Psychosocial: CBT, IPSRT, family-focused therapy, psychoeducation.
6. Refractory: clozapine, ECT.
7. Pregnancy: valproate absolute contraindication; lamotrigine and quetiapine relatively safe.

8.2 Treatment methods

1. Lamotrigine — Bipolar Type II effective in maintenance and prevention of depressive episodes; titration slow (risk of Stevens-Johnson syndrome) — starting dose 25 mg, increase over 2 weeks; target 200 mg/day. Calabrese J.R. et al. J Clin Psychiatry 2003.
2. Quetiapine — FDA approval for bipolar depressive episode; 300–600 mg/day; metabolic side effect monitoring.
3. Interpersonal and Social Rhythm Therapy (IPSRT) — Frank (Frank E.); and CBT for bipolar disorder — Circadian rhythm, compliance, relapse prevention.
4. Hypomania Checklist (HCL-32) — Angst J — 32-item self-assessment hypomanic episode screening.
5. Mood Disorder Questionnaire (MDQ — Mood Disorder Questionnaire) — 13 questions; bipolar screening.

8.3 Differences between sources

- CANMAT/ISBD 2018: Type II depressive episode first-line — quetiapine; lamotrigine in maintenance.
- NICE CG185: Type I and II same main approach, but lamotrigine's specific role emphasized in Type II.
- Calabrese J.R. RCTs — lamotrigine efficacy in maintenance treatment of bipolar disorder type II.

9. Prognosis

- Episodic course; relapse significantly reduced with maintenance pharmacotherapy.
- Type II is not ‘milder’ than Type I — high functional impairment and depressive burden.
- Suicide risk high.
- Quetiapine and lamotrigine in maintenance as main.
- Monitoring — depressive symptoms, hypomanic triggers, metabolic indicators.

10. Myths and misconceptions

Myth 1: “Type II bipolar is a ‘mild form’ requiring little treatment”

Evidence: Judd L.L. Arch Gen Psychiatry 2003 — Type II patients spend 50.3% of follow-up weeks with depressive symptoms (53.9% symptomatic overall); functional impairment and suicide risk comparable to Type I.

Myth 2: “Hypomanic episode is a ‘good period’, no treatment needed”

Evidence: hypomanic episode can involve risky behavior, impaired relationships, later productivity decline, and trigger depressive episode; isolated episodes indicate need for maintenance pharmacotherapy.

Myth 3: “Antidepressant alone can be used in Type II patient”

Evidence: CANMAT 2018 — antidepressant only with mood stabilizer; risk of manic switch and rapid cycling.

Myth 4: “Bipolar Type II is only a form of borderline personality disorder”

Evidence: There is significant overlap and comorbidity, but these are distinct disorders; bipolar Type II episodic, BPD continuous pattern.

Myth 5: “Lamotrigine cures a manic episode”

Evidence: Lamotrigine is effective in the depressive component and maintenance; ineffective for manic episodes — antipsychotic or valproate is required.

Myth 6: “St John's Wort is safe in bipolar because it is a natural antidepressant”

Evidence: St. John's Wort can induce mania and decrease the levels of mood stabilizers (CYP induction); contraindicated in bipolar disorder.

Myth 7: “ECT is not needed in Type II”

Evidence: ECT indicated for refractory depressive episode or high suicidality risk; also effective in Type II patient.

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ICD-11 **6A62**	**6A62 CYCLOTHYMIC DISORDER** **Cyclothymic disorder**
ICD-10 F34.0	Cyclothymia
DSM-5-TR F34.0	Cyclothymic Disorder

1. Definition and nosology

Cyclothymic disorder (ICD-11: 6A62; DSM-5-TR: F34.0 Cyclothymic Disorder) — over a period of at least 2 years (1 year in children/adolescents) subthreshold chronic affective disorder characterized by persistent fluctuation between hypomanic and depressive symptoms. Does not meet criteria for full major depressive, manic, or hypomanic episodes.

2. History

- Kahlbaum K. (1882) — “cyclothymia” term.
- Kraepelin (1899) — a mild form of manic-depressive disorder.
- Akiskal H.S. (1977) — reconceptualization of cyclothymic temperament as a subthreshold stage of the bipolar spectrum.
- DSM-III (1980), DSM-5 (2013), ICD-11 (2019) — within the category of affective disorders.

3. Epidemiology

- Lifetime prevalence: 0.4–1% (DSM-5-TR; broad range, precise epidemiological data limited).
- Sex: approximately equal.
- Onset: adolescence and early adulthood.
- Type I or II conversion to bipolar: ~15–50% (Akiskal H.S. reviews).
- Comorbidity: anxiety, substance use, ADHD, borderline personality disorder.

4. Aetiology and pathogenesis

- Shared genetic load with the bipolar spectrum (Akiskal).
- Increased density of bipolar and major depressive disorder in family history.
- Neurobiological — monoaminergic and circadian dysregulation similar to bipolar disorder, but less pronounced.

5. Clinical features

- Multiple subthreshold hypomanic episodes — brief, low-intensity elevations in energy and mood.
- Multiple subthreshold depressive episodes — low-intensity depressive symptoms.
- “Stable period” less than 2 months.
- Mood changes over days or weeks; patient may explain themselves as ‘this is just my nature.’
- Interpersonal and occupational impairment is present, but not disruptive.

6. Diagnosis

6.1 Unified diagnostic criteria (DSM-5-TR · ICD-11)

A. At least 2 years (adults) / 1 year (children/adolescents) of numerous hypomanic and depressive subthreshold periods.

B. During this period, symptoms are present for ≥ half of the time, and symptom-free periods do not exceed 2 months.

C. Criteria for full major depressive, manic, or hypomanic episodes have never been met.

D. Not better explained by schizoaffective, schizophrenia or other psychotic disorder.

E. Exclusion of substance or medical condition.

F. Significant distress or functional impairment.

6.2 Diagnostic algorithm

1. Clinical interview (mood diary recommended for 4–6 weeks — documentation of patient's own patterns).
2. SCID-5, MDQ, HCL-32.
3. Comorbidity and family history assessment.

4. Medical and laboratory (thyroid absolute).

6.3 Differential diagnosis

Condition	Distinguishing features
Bipolar Type II (6A61)	Full hypomanic and major depressive episodes.
Dysthymic disorder (6A72)	Only depressive subthreshold; no hypomanic episodes.
BPD (6D10.x)	Mood swings; identity disturbance; impulsivity and interpersonal relationships.
ADHD (6A05)	Not episodic; attention deficit dominant.
Cyclothymic temperament (normal variant)	No functional impairment.

7. Examination and assessment

- Mood diary.
- MDQ, HCL-32, YMRS, HAM-D.
- Thyroid function.

8. Treatment

1. Psychotherapy first line — CBT, IPSRT, psychoeducation, behavioral activation, circadian rhythm management.
2. Pharmacotherapy — evidence base is limited:
    - Lithium or valproate — considered adequate in the context of bipolar spectrum.
    - Lamotrigine — when depressive component is dominant.
    - Atypical antipsychotic (quetiapine) — in moderate-severe cases.
3. Antidepressant alone — risk of manic switch and rapid cycling; contraindicated.
4. Stress management and maintenance of social-circadian rhythm — important.
5. Monitoring for conversion to bipolar disorder — more frequent monitoring if family history and clinical markers present.

8.1 Treatment methods

1. Interpersonal and Social Rhythm Therapy (IPSRT) — Frank (Frank E.) — Circadian rhythm and interpersonal stress management.
2. CBT for bipolar spectrum — Psychoeducation, behavioral activation, relapse markers.
3. Mood diary — Patient's documentation of own mood changes — diagnostic and therapeutic tool.
4. Lithium and lamotrigine — Pharmacotherapy of bipolar spectrum adequate.

8.2 Differences between sources

- NICE CG185, CANMAT 2018 — separate algorithm for cyclothymic disorder is limited; bipolar spectrum principles apply; psychotherapy first-line.

9. Prognosis

- Chronic; spontaneous remission is rare.
- Risk of conversion to Bipolar Type I/II — annual clinical assessment.
- Comorbid substance use and BPD risk factors.
- Functional level improves with adequate treatment.

10. Myths and misconceptions

Myth 1: “Cyclothymic temperament is a normal personality trait, does not require treatment”

Clinical logic: cyclothymic temperament when it is a normal variant has no functional impairment; clinical diagnosis requires functional impairment. However, many patients have explained it as a ‘creative nature’ for years and do not seek intervention, while quality of life deteriorates.

Myth 2: “Cyclothymic disorder will not convert to bipolar”

Evidence: Akiskal — 15–50% conversion to bipolar Type I or II; higher risk if family history.

Myth 3: “Antidepressant alone manages all mood changes”

Evidence: Manic switching and rapid cycling risk; required concomitant with mood stabilizer.

Myth 4: “A cyclothymic patient will lose their creativity due to treatment”

Evidence: appropriate treatment (psychotherapy + mood stabilizer) helps preserve and enhance creativity (stabilized activity); romanticized ‘duality’ explanation leads to avoidance of intervention.

Myth 5: “Herbal preparations or diet cure cyclothymic disorder”

Evidence: St John's Wort risk of manic switch; omega-3 mild adjunct, not primary treatment.

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ICD-11 **6A70**	**6A70 SINGLE EPISODE DEPRESSIVE DISORDER** **Single episode depressive disorder**
ICD-10 F32	Depressive episode
DSM-5-TR F32.x	Major Depressive Disorder, Single Episode

1. Definition and nosology

Single episode depressive disorder (ICD-11: 6A70; DSM-5-TR: Major Depressive Disorder, Single Episode — F32.x) — an affective disorder causing functional impairment, accompanied by significant cognitive, somatic, and psychomotor symptoms along with depressed mood and/or anhedonia for at least two weeks. In the

patient's history only one major depressive episode is observed (in case of recurrence, code 6A71).

2. History

- Hippocrates (5th–4th centuries BC) — the term “melancholia”.
- Kraepelin (1899) — Depressive episode within manic-depressive insanity.
- DSM-III (1980) — official diagnosis “Major Depressive Disorder”; separation from bipolar disorder.
- DSM-5 (2013) — The “bereavement exclusion” has been removed — a major depressive episode can be diagnosed in a patient even after the loss of a loved one.
- ICD-11 (2019) — Single episode (6A70) and Recurrent (6A71) are coded separately.

3. Epidemiology

- Lifetime prevalence: 15–20% (NESARC-III; Hasin D.S. et al. JAMA Psychiatry 2018).
- Annual prevalence: ~6–8%.
- Sex: 1.5–2 times higher in females.
- Onset: mean 20–30 years; but possible at any age.
- Suicide: depressive disorder is associated with ~50% of suicide deaths.
- Comorbidity: anxiety disorders 60%, substance use 25%, pain, cardiovascular, diabetes.
- Mortality: overall mortality increased 1.5 times (cardiovascular, suicide).

4. Aetiology and pathogenesis

- Heritability 35–40% (Sullivan P.F. et al. Am J Psychiatry 2000 meta-analysis); less than bipolar.
- GWAS — 100+ risk loci (Howard D.M. et al. Nat Neurosci 2019, n>800,000).
- Neurobiological — monoaminergic (serotonin, norepinephrine, dopamine) hypothesis; HPA axis dysregulation, elevated cortisol; hippocampal volume reduction; inflammatory hypothesis (CRP, IL-6).
- Risk factors: childhood trauma, family history, acute stress, chronic medical illness, social isolation, substance use, postpartum period, hypothyroidism.

5. Clinical features

5.1 Core symptoms (DSM-5-TR — ≥ 5 symptoms × 2 weeks, at least 1 of which is 1 or 2)

1. Persistent low mood.
2. Anhedonia — loss of interest and pleasure.
3. Weight or appetite changes.
4. Sleep disturbance (insomnia or hypersomnia).
5. Psychomotor agitation or retardation (observable by others).
6. Fatigue or loss of energy.
7. Worthlessness or excessive guilt.
8. Difficulty concentrating and impaired decision-making.
9. Suicidal thoughts or plan.

5.2 Qualifiers (DSM-5-TR)

- Severity: mild, moderate, severe.
- With psychotic features (delusion or hallucination).
- Atypical features (hypersomnia, hyperphagia, leaden paralysis).
- Melancholic features.
- Catatonic features.
- Peripartum onset.
- Seasonal pattern.
- Mixed features (manic components).
- With anxiety.

6. Diagnosis

6.1 Unified diagnostic criteria (DSM-5-TR · ICD-11 consensus points)

A. The presence of 5 or more symptoms for ≥ 2 weeks; at least one of which must be depressed mood or anhedonia.

B. Significant distress or functional impairment.

C. Exclusion of substance or medical condition.

D. Never had a manic or hypomanic episode.

6.2 Differences between sources

- DSM-5-TR: “Bereavement exclusion” removed (in DSM-IV, diagnosis was not made during 2 months of bereavement); grief and depressive episode can co-occur.
- ICD-11 (6A70): single episode separately coded; severity and psychotic feature qualifiers.
- NICE NG222 (2022): Severity classification: less severe, more severe (instead of mild/moderate/severe in DSM); treatment algorithms based on severity.

6.3 Diagnostic algorithm

1. Clinical interview.
2. Structured interview (SCID-5, MINI).
3. Scales — PHQ-9 (screening and monitoring), HAM-D, MADRS, BDI-II.
4. Suicide risk (C-SSRS).
5. Bipolar screening (MDQ, HCL-32) — to rule out bipolar episodes.
6. Medical and laboratory (thyroid, B12, folate, ferritin, liver, kidney, pregnancy) — to exclude somatic causes.
7. Toxicology screening (suspect substance use).

6.4 Differential diagnosis

Condition	Distinguishing features
Bipolar Type I/II (6A60/6A61)	History of manic or hypomanic episode.
Dysthymic disorder (6A72)	≥2 years subthreshold depressive symptoms.
Adjustment disorder (6B43)	Response to stress factor; fewer criteria.
Grief (sadness)	After loss of a loved one; ‘wave-like’ course; self-esteem is preserved.
Hypothyroidism	Elevated TSH; somatic symptoms.
B12 / folate deficiency	Laboratory.
Substance-induced depression	History of substance use.
Depressive symptoms in dementia	Cognitive deficit dominant; elderly patient.

7. Examination and assessment

- PHQ-9 — standard in primary care; ≥10 score indicates moderate-severe depression.
- HAM-D-17 (Hamilton), MADRS — clinical and research.
- BDI-II — self-assessment.
- C-SSRS — suicide risk.
- Laboratory: complete blood count, thyroid, B12, folate, ferritin, liver/kidney, glucose.
- EKG — before starting TCA or QT-prolonging medications.

8. Treatment

8.1 General Principles (NICE NG222 · APA 2010 · CANMAT 2016 Consensus)

1. Stepwise approach based on severity:
    - Mild (PHQ-9 5–9) — guided self-help, CBT-based internet interventions, physical activity, observation.
    - Moderate (PHQ-9 10–14) — CBT or IPT, or pharmacotherapy (SSRI); patient choice.
    - Moderately severe (PHQ-9 15–19) — pharmacotherapy (SSRI) or a high-intensity psychological intervention; combination preferred.
    - Severe (PHQ-9 ≥ 20 or psychotic features) — combination of pharmacotherapy + psychotherapy; severe psychotic/suicidal risk — inpatient admission and ECT.
2. First-line pharmacotherapy: SSRIs — sertraline, escitalopram, fluoxetine, paroxetine, citalopram. Effect in 4–6 weeks; titration required.
3. SNRI — venlafaxine, duloxetine — alternative or comorbid pain/anxiety.
4. Other — mirtazapine (sleep and appetite problems), bupropion (few sexual side effects, ADHD comorbid), agomelatine, vortioxetine (cognitive component).
5. Refractory: SSRI + lithium or quetiapine or aripiprazole; SSRI combinations; ketamine/esketamine (intranasal — FDA approved 2019, severe refractory); ECT.
6. Psychotherapy first line alternative — monotherapy in mild-to-moderate depression, combination with pharmacotherapy in severe cases; specific methods and evidence base in the “Treatment methods” list (CBT, IPT, behavioral activation, MBCT).

7. Suicide risk management — active assessment; hospitalization if risk high; restricted access to means (weapons, medications); psychosocial support.
8. Treatment duration: first episode: continuation after remission for 6–12 months; long-term in case of relapse.

8.2 Pharmacotherapy

Class	Drug	Dose range	Note
SSRIs	Sertraline, escitalopram, fluoxetine, paroxetine, citalopram	Sertraline 50–200 mg; escitalopram 10–20 mg	First-line; sexual side effect; SIADH (in elderly); citalopram > 40 mg QT prolongation.
SNRI	Venlafaxine, duloxetine	Venlafaxine 75–375 mg; duloxetine 60–120 mg	Comorbid pain, anxiety.
Atypical	Mirtazapine, bupropion, agomelatine, vortioxetine	Mirtazapine 15–45 mg; bupropion 150–450 mg	Mirtazapine: sleep and appetite; bupropion: no sexual side effects.
TCA	Amitriptyline, nortriptyline	75–150 mg	Second-to-third line; cardiac and anticholinergic side effects; overdose lethal.
MAOI	Phenelzine, tranylcypromine		Refractory; tyramine diet.
Esketamine (intranasal)	Spravato	56–84 mg	FDA 2019, refractory; under supervision in clinic.

8.3 Source-Specific Clarifications

- NICE NG222 (2022): stepwise algorithm based on severity; “less severe” — CBT or guided self-help first-line; “more severe” — combination.
- APA Practice Guideline (2010, 2019 update): SSRI first-line; CBT / IPT first-line alternative.

- CANMAT 2016: A 4-step algorithm based on severity and incident.
- Cipriani A. et al. Lancet 2018 network meta-analysis: Comparison of 21 antidepressants — agomelatine and amitriptyline highest efficacy; fluoxetine, fluvoxamine, reboxetine and trazodone lowest. The most acceptable were agomelatine, citalopram, escitalopram, fluoxetine, sertraline and vortioxetine; reboxetine ranked among the worst on both efficacy and acceptability. Efficacy differences are clinically modest.

8.4 Treatment methods

1. CBT for depression — Beck (Beck A.) — Cognitive restructuring, behavioral activation; 12–20 sessions. Evidence: Cuijpers P. et al. Cochrane meta-analyses — medium effect, comparable to pharmacotherapy; combination superior to monotherapy in severe cases.
2. Interpersonal Therapy (IPT — Interpersonal Therapy) — Klerman (Klerman G.), Weissman (Weissman M.) — 4 problem areas – grief, role transition, role conflict, interpersonal deficit. 12–16 sessions. RCT evidence comparable to CBT.
3. Behavioral Activation — Jacobson N., Martell C. — Systematic increase of positive activities, reduction of avoidance. Dimidjian S. et al. J Consult Clin Psychol 2006 RCT — comparable effect to CBT.
4. Mindfulness-Based Cognitive Therapy (MBCT) — Segal Z., Williams M., Teasdale J — In relapse prevention; particularly if history of ≥3 episodes, effect may be superior to antidepressant continuation (Kuyken W. et al. Lancet 2015).
5. Patient Health Questionnaire-9 (PHQ-9) — Kroenke (Kroenke K.), Spitzer (Spitzer R.L.) — Gold standard screening and monitoring tool in primary medical care.
6. HAM-D / MADRS — Clinician-rated depressive severity scales.
7. ECT — Severe psychotic depression, refractory, suicide risk, food intake disorder, pregnancy. UK ECT Review Group Lancet 2003 — ECT superior to pharmacotherapy in severe cases.
8. Esketamine / Ketamine — FDA 2019 intranasal esketamine (Spravato) for severe refractory depression; rapid antidepressant effect (hours-days); administration under clinic supervision. Daly E.J. et al. JAMA Psychiatry 2018.
9. Repetitive Transcranial Magnetic Stimulation (rTMS — repetitive Transcranial Magnetic Stimulation) — FDA-approved (2008) for refractory depression; 4–6 week course, target left DLPFC.

9. Prognosis

- Spontaneous remission occurs in 50% within one year (though most remissions occur without antidepressant treatment); relapse risk is high.
- The risk of a subsequent episode increases after each episode.
- Suicide risk is particularly high in the first weeks.
- Monitoring — PHQ-9 / MADRS, suicide risk, side effects, compliance; more frequent in first 12 weeks.
- Duration of treatment — 6–12 months after remission in first episode.

10. Myths and misconceptions

Myth 1: "Depression is just 'sadness'; it can be overcome voluntarily"

Evidence: Depression is a complex neurobiological disorder involving HPA axis dysregulation, monoamine imbalance, and structural changes (e.g., hippocampus); it is not volitionally controllable. The APA, WHO, and NICE define it as a medical condition.

Myth 2: "Antidepressants are addictive"

Evidence: SSRI and SNRI do not cause addiction — no abuse or tolerance. But a discontinuation syndrome does exist (especially with the short half-life of paroxetine, venlafaxine) — this is not addiction, but physiological adaptation; it resolves with gradual discontinuation.

Myth 3: "Antidepressant changes the patient's personality"

Evidence: Antidepressants reduce the symptoms; they do not change the personality. If a patient says that he or she no longer feels like themselves, that is a side effect — emotional blunting — and the answer is to change the drug or lower the dose.

Myth 4: "Antidepressants are no better than placebo"

Evidence: Cipriani Lancet 2018 — 21 antidepressants significantly superior to placebo; but effect is modest, especially in mild depression. In moderate-severe depression, the effect is clinically significant.

Myth 5: "Diet (omega-3, B vitamins, magnesium) replaces antidepressants"

Evidence: Some supplements (omega-3, vitamin D in deficiency) may have mild adjunct effect, but do not replace main treatment in moderate-severe depression. SMILES study (Jacka F.N. BMC Med 2017) — Mediterranean-type diet as mild adjunct.

Myth 6: “St John's Wort is a natural antidepressant and can replace antidepressants”

Evidence: St John's wort shows efficacy in mild depression (Cochrane Linde K. 2008), but the evidence base is limited in moderate-to-severe depression; due to CYP induction, it reduces the levels of numerous drugs (contraceptives, anticoagulants, antidepressants, antiretrovirals) — a serious interaction.

Myth 7: “Only exercise or psychotherapy is sufficient; medication is unnecessary”

Evidence: In mild depression, psychotherapy and exercise may sometimes be sufficient, but in moderate-to-severe depression and particularly when suicidal risk is present, pharmacotherapy is necessary; NICE NG222 and APA 2010.

Myth 8: “ECT is an outdated or traumatic treatment”

Evidence: modern ECT (under anesthesia, with muscle relaxants) is safe and highly effective; it is the gold standard for severe refractory and psychotic depression. UK ECT Review Group Lancet 2003.

Myth 9: “Antidepressants cause suicide”

Evidence: FDA black box in children and adolescents (especially first weeks) warns of increased risk of suicidal thoughts — requires monitoring of these patients, but long-term antidepressants reduce suicide risk (Stone M. et al. BMJ 2009).

Myth 10: “Depression cannot be diagnosed during the grieving period”

Evidence: DSM-5 eliminated the bereavement exclusion; grief and major depressive episode can co-occur; pathological forms of grief (prolonged grief disorder — 6B43) are separate.

Myth 11: “Depression in healthy patients is a marker of psychological ‘weakness’”

Evidence: Depression can occur in any individual; the concept of “psychological strength” is stigmatizing and incorrect; it should be evaluated as a medical condition.

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ICD-11 **6A71**	**6A71 RECURRENT DEPRESSIVE DISORDER** **Recurrent depressive disorder**
ICD-10 F33	Recurrent depressive disorder
DSM-5-TR F33.x	Major Depressive Disorder, Recurrent Episode

1. Definition and nosology

Recurrent depressive disorder (ICD-11: 6A71; DSM-5-TR: Major Depressive Disorder, Recurrent — F33.x) — presence of at least two major depressive episodes, with partial or full remission lasting at least 2 months between episodes. No history of manic or hypomanic episodes (if present, diagnosis is bipolar disorder).

Differs from single episode depressive disorder (6A70) only by number of episodes; clinical manifestations, diagnostic criteria, and treatment principles are identical, but long-term maintenance pharmacotherapy plays a more important role here.

2. History

- Kraepelin (1899) — within the framework of manic-depressive disorder, recurrent depressive episodes.
- Leonhard K. (1957) — concept of unipolar depression.
- DSM-III (1980), DSM-5 (2013) — Major Depressive Disorder, Recurrent official designation.
- ICD-11 (2019) — Single episode (6A70) and Recurrent (6A71) are coded separately.

3. Epidemiology

- Major depressive disorder lifetime prevalence 15–20%; of which ~50–80% have a recurrent course.
- The risk of a subsequent episode increases after each episode (Solomon D.A. et al. Am J Psychiatry 2000) — ~50% after the 1st episode, ~70% after the 2nd, ~90% after the 3rd.
- Suicide risk higher than in single episodes, particularly in untreated episodes.

4. Aetiology and pathogenesis

- Same as single episode (6A70) — heritability ~35–40% (Sullivan 2000); GWAS loci.
- Relapse predisposition — predominantly cognitive predisposition, neurobiological "kindling" (Post R.M.) concept — threshold decreases for the next episode after each one.
- Risk factors — childhood trauma, family history, chronic stress, comorbid anxiety, somatic illness.

5. Clinical features

Same as a single episode (see 6A70 §5). In recurrent patients, additional features:

- Episode presentation may change from previous episodes (atypical features, seasonal pattern).
- Recurrent episodes typically begin more rapidly than previous ones (kindling concept — Post R.M. Am J Psychiatry 1992).
- Comorbid anxiety disorders with increased frequency.
- Chronic cognitive deficits may be observed (especially after multiple episodes).

6. Diagnosis

6.1 Unified diagnostic criteria (DSM-5-TR · ICD-11)

A. At least 2 major depressive episodes (according to 6A70 §6.1 criteria).

B. ≥ 2 months partial or full remission between episodes.

C. Never had a manic or hypomanic episode.

D. Not better explained by schizoaffective, schizophrenia or other psychotic disorder.

6.2 Differences between sources

- ICD-11: current episode severity (mild, moderate, severe) and psychotic feature qualifiers; remission status (partial, full).
- DSM-5-TR: qualifiers are the same as 6A70.
- NICE NG222: ≥3 episodes — MBCT (Mindfulness-Based Cognitive Therapy) is specifically recommended for relapse prevention in patients.

6.3 Diagnostic algorithm

1. History — number of previous episodes, duration, triggers, quality of remission.
2. Bipolar screening (MDQ, HCL-32) — to rule out hypomanic episodes.
3. PHQ-9 / HAM-D / MADRS — severity of current episode.
4. C-SSRS — suicide risk (high in recurrent patients).
5. Laboratory (thyroid, B12, folate, ferritin); somatic comorbidity assessment.

6.4 Differential diagnosis

Same as 6A70 (see single episode). Special attention — exclude bipolar episodes.

7. Examination and assessment

Same as 6A70. Special attention — assessment of cognitive deficits (after multiple episodes).

8. Treatment

8.1 General Principles (NICE NG222 · APA 2010 · CANMAT 2016)

1. Acute episode treatment — Same as 6A70 (SSRI, CBT, combination).
2. Maintenance pharmacotherapy — long-term.
    - After 2 episodes — continue for 1–2 years after remission.
    - ≥3 episodes — long-term (years) or lifelong maintenance.
    - Effective antidepressant is maintained at the same dose after remission (dose reduction increases relapse risk).
3. MBCT (Mindfulness-Based Cognitive Therapy) — comparable effect to antidepressant continuation for relapse prophylaxis in patients with ≥3 episode history (Kuyken W. et al. Lancet 2015).

4. Lithium adjunct — in refractory or high relapse risk patients; suicide reduction effect (Cipriani BMJ 2013).
5. Recognition of relapse markers — psychoeducation for patient and family (sleep disturbance, irritability, social withdrawal, anhedonia).
6. Comorbidity treatment — anxiety disorders, substance use, cardiovascular — parallel.
7. Suicide risk monitoring — high, particularly at onset of new episode.

8.2 Treatment methods

1. Mindfulness-Based Cognitive Therapy (MBCT) — 8-week group program; 2.5-hour sessions + home practice. Effective in relapse prevention for patients with history of ≥3 episodes. RCT: Kuyken W. et al. Lancet 2015. mbct.com.
2. Antidepressant Maintenance Therapy — Long-term use with the same drug and dose; at least 2 years (≥ 3 episodes); gradual discontinuation (4–8 weeks).
3. CBT and IPT maintenance formats — Monthly or every 2 weeks per session; relapse prevention.
4. Lithium adjunct — In refractory unipolar depression with high suicide risk; Cipriani BMJ 2013.

8.3 Differences between sources

- NICE NG222 (2022): ≥3 episode history — MBCT or antidepressant maintenance; both are comparably effective.
- APA 2010: maintenance pharmacotherapy after ≥ 2 episodes; long-term after ≥ 3 episodes.
- Geddes J.R. et al. Lancet 2003 meta-analysis: Antidepressant continuation significantly superior to placebo in relapse reduction (NNT 5).

9. Prognosis

- Relapse risk increases after each episode (Solomon 2000).
- Relapse risk significantly reduced with adequate maintenance treatment (NNT 5).
- Comorbid anxiety, somatic illnesses, compliance disturbance — poor prognosis.
- Monitoring — PHQ-9 monitoring; relapse markers; antidepressant levels and side effects; suicide risk.

10. Myths and misconceptions

Myth 1: "Antidepressants cannot be used lifelong, they cause 'addiction'"

Evidence: SSRI/SNRI do not cause dependency; lifelong maintenance is recommended for patients with ≥3 episode history (NICE NG222, APA 2010). Reducing the maintenance dose increases the risk of relapse.

Myth 2: "Antidepressants should be stopped immediately after remission"

Evidence: After remission continue at least 6–12 months (single episode) or years (recurrent); early discontinuation significantly increases relapse risk (Geddes Lancet 2003).

Myth 3: "Medication should be changed with every episode"

Evidence: The medication effective in the previous episode should be reused in the next episode (patient-specific efficacy).

Myth 4: "Recurrent depressive disorder is a 'personality weakness', treatment should not be long"

Evidence: Neurobiological 'kindling' concept — each episode increases risk of new episode; long-term treatment 'protects' the brain.

Myth 5: "Only MBCT is sufficient for relapse prevention; pharmacotherapy is unnecessary"

Evidence: Kuyken Lancet 2015 — MBCT vs. maintenance antidepressant comparative; but individual choice — based on patient values, accessibility, prior treatment response.

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ICD-11 **6A72**	**6A72 DYSTHYMIC DISORDER** **Dysthymic disorder**
ICD-10 F34.1	Dysthymia
DSM-5-TR F34.1	Persistent Depressive Disorder

1. Definition and nosology

Dysthymic disorder (ICD-11: 6A72; DSM-5-TR: Persistent Depressive Disorder — F34.1) — chronic depressive symptoms lasting at least 2 years (in adults; 1 year in children/adolescents) that do not meet the severity of a major depressive episode. The patient may describe their state as 'I've always been this way'.

DSM-5-TR — the term “Persistent Depressive Disorder” combines dysthymia and chronic major depressive disorder; ICD-11 codes dysthymic and chronic depressive episode separately.

2. History

- Flemming C.F. (1844) — first psychiatric use of the term “dysthymia”; Kahlbaum (1882) — gave it its modern meaning as a chronic, subthreshold depressive state.
- Akiskal H.S. (1980s) — dysthymic temperament as a substage of the depressive spectrum.
- DSM-III (1980) — official diagnosis “Dysthymic Disorder”.
- DSM-5 (2013) — “Persistent Depressive Disorder” — merger of dysthymia and chronic MDD.
- ICD-11 (2019) — dysthymic disorder retained separately.

3. Epidemiology

- Lifetime prevalence: 1.1–6.4% (dysthymia); 12-month prevalence in the US ~0.5%, plus 1.5% for chronic major depression (DSM-5-TR).
- Sex: 2 times higher in females.
- Onset: early (childhood/adolescence — early-onset; ≥ 21 years — late-onset).
- "Double depression" — major depressive episode on a background of dysthymic disorder — occurs in 76.9% of patients within 5 years (Klein D.N. et al. Am J Psychiatry 2000;157(6):931–939).
- Comorbidity: anxiety, BPD, substance use.

4. Aetiology and pathogenesis

- Shared genetic and neurobiological predisposition with major depressive disorder.
- Childhood trauma and chronic psychosocial stress are key environmental risk factors.
- The frequency of depressive disorder or dysthymia is increased in close relatives.

5. Clinical features

- Persistent low mood — present most of the time (≥ half the time, no symptom-free period > 2 months over 2 years).
- At least 2 of the following: low or increased appetite, insomnia or hypersomnia, low energy, low self-esteem, concentration difficulty, hopelessness.
- The patient has adapted to numerous functional compromises — the explanation "I have always been like this" is typical; they often present to the clinic against a background of a major depressive episode.

6. Diagnosis

6.1 Unified diagnostic criteria

A. Persistent depressive mood ≥ 2 years (children ≥ 1 year), most of the day, most of the time.

B. ≥ 2 symptoms during depressive episodes (above)

C. No symptom-free period longer than 2 months within 2 years (1 year).

D. During this period, criteria for a major depressive episode may be continuously present (DSM-5 — new “double depression” synthesis).

E. Never had a manic or hypomanic episode.

F. Not better explained by schizoaffective or other psychotic disorder.

G. Exclusion of substance or medical condition.

H. Significant distress or functional impairment.

6.2 Differences between sources

- DSM-5-TR: “Persistent Depressive Disorder” — combines dysthymia and chronic MDD; qualifiers — early/late onset, with/without anxious distress, melancholic, atypical, peripartum, seasonal, etc.
- ICD-11: dysthymic disorder (6A72) separate; chronic depressive episode within 6A70/71 qualifier.

6.3 Diagnostic algorithm

1. Clinical interview + long-term history (patient may not recall exact onset).
2. SCID-5; PHQ-9, HAM-D, MADRS.
3. Bipolar screening.
4. Personality assessment – differential with BPD.
5. Medical and laboratory (thyroid, anemia, B12).

6.4 Differential diagnosis

Condition	Distinguishing features
Major depressive disorder (6A70/6A71)	Discrete episodes; mood is normal between symptoms.
Cyclothymic disorder (6A62)	Hypomanic periods present.
BPD (6D10.x)	Emotional lability over hours; identity disturbance.
Adjustment disorder (6B43)	Response to stress factor; ≤ 6 months.
Hypothyroidism	TSH.

7. Examination and assessment

- PHQ-9, HAM-D, MADRS.

- Cornell Dysthymia Rating Scale — dysthymia-specific.
- Medical and laboratory.

8. Treatment

1. Pharmacotherapy: SSRI (sertraline, fluoxetine) — first-line; the effect may take 6–8 weeks (slower than in major depression).
2. Psychotherapy: CBASP (Cognitive Behavioral Analysis System of Psychotherapy — McCullough J.) — specifically developed for chronic depression; CBT, IPT.
3. Combination — superior to monotherapy in dysthymic disorder (Keller M.B. et al. NEJM 2000 – nefazodone + CBASP RCT).
4. In "double depression" — the management includes acute treatment of a major depressive episode + long-term maintenance therapy.
5. Refractory — medication switch, augmentation (lithium, atypical antipsychotic).

8.1 Treatment methods

1. Cognitive Behavioral Analysis System of Psychotherapy (CBASP — Cognitive Behavioral Analysis System of Psychotherapy) — McCullough (McCullough J.) — Approach specifically developed for chronic depression — situation analysis, interpersonal behavior teaching, use of therapeutic relationship as main tool. Keller M.B. et al. NEJM 2000 RCT.
2. SSRI long-term — Sertraline, fluoxetine, escitalopram; slow response; prolonged maintenance.
3. CBT and IPT — Standard depression therapies; modified for chronic form.

8.2 Differences between sources

- NICE NG222 — recommends long-term cognitive behavioural treatment for chronic depressive symptoms (CBASP is not named in the guideline); CBASP is named by APA 2010.
- APA 2010 — combination superior.
- Keller M.B. et al. NEJM 2000 — nefazodone + CBASP combination significantly superior to monotherapy.

9. Prognosis

- Chronic course; spontaneous remission is rare.
- "Double depression" — develops in 76.9% of dysthymic patients without a prior major episode within 5 years (Klein D.N. et al. Am J Psychiatry 2000).

- Functional level improves with long-term treatment.
- Monitoring — periodic scales, compliance, comorbidity.

10. Myths and misconceptions

Myth 1: “Dysthymia is just a ‘gloomy character’, not a medical disorder”

Evidence: persistent functional impairment, suicidality risk, development of “double depression”; should be evaluated as a medical disorder.

Myth 2: “Dysthymic patient does not respond to antidepressants”

Evidence: The response is slow — 6 to 8 weeks, but SSRIs are effective; combination with CBASP superior.

Myth 3: “Chronic depression only requires psychotherapy; medications are ineffective”

Evidence: Keller NEJM 2000 — nefazodone + CBASP combination superior to monotherapy; pharmacotherapy is a core component.

Myth 4: “The patient's explanation 'I've always been like this' is not a basis for diagnosis — it should be accepted as adaptation”

Evidence: This patient perspective creates a problem of under-detection — long-term functional compromise may be a marker of medical condition.

Myth 5: “Herbal preparations are a safe alternative for dysthymia”

Evidence: St John's Wort demonstrates some efficacy in mild depression, but CYP induction interactions; standard antidepressants superior.

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ICD-11 **GA34.41**	**GA34.41 PREMENSTRUAL DYSPHORIC DISORDER (PMDD)** **Premenstrual dysphoric disorder**
ICD-10 N94.3	Premenstrual tension syndrome
DSM-5-TR N94.3	Premenstrual Dysphoric Disorder

1. Definition and nosology

Premenstrual Dysphoric Disorder (ICD-11: GA34.41; DSM-5-TR: N94.3) — a recurrent disorder with significant emotional and behavioral symptoms in the last week before the menstrual cycle (luteal phase), fully remitting within a few days after menses onset. Distinguished from Premenstrual Syndrome (PMS) by clinical severity and significant functional impairment.

2. History

- Frank R.T. (1931) — “Premenstrual tension” term.
- DSM-IV (1994) — “Premenstrual Dysphoric Disorder” is included in Section III as a research category.
- DSM-5 (2013) — officially accepted as a diagnosis in the category of depressive disorders.
- ICD-11 (2019) — as separate category within mood disorders.

3. Epidemiology

- Prevalence in reproductive-age women: 3–8% (Yonkers K.A. et al. Lancet 2008 review).
- PMS — milder form — in 30–80% of women.
- Comorbidity: major depressive disorder, anxiety, bipolar disorder — increases risk.

- Suicidal thoughts significantly increase in the luteal phase.

4. Aetiology and pathogenesis

- Genetic: heritability ~30–50% (Treloar S.A. et al. Psychol Med 2002 twin studies).
- Hormonal sensitivity: In PMDD patients, ovarian hormones (estrogen, progesterone) are at normal levels; however sensitivity Increased — allopregnanolone (progesterone metabolite) exerts abnormal effect on GABA-A receptors (Schmidt P.J. et al. NEJM 1998 ovarian suppression and hormone supplementation study).
- Serotonergic dysregulation: The rapid effect of SSRIs (1–2 days, unlike classical depression) supports this mechanism.

5. Clinical features

5.1 Affective and behavioral symptoms (in the luteal phase)

- Affective lability (rapidly changing mood, tearfulness, rejection sensitivity).
- Irritability and angry outbursts (interpersonal conflict).
- Low mood, hopelessness, self-criticism.
- Anxiety, tension.

5.2 Behavioral and somatic symptoms

- Decreased interest in usual activities.
- Difficulty concentrating.
- Lethargy and fatigue.
- Appetite changes (especially carbohydrate craving), excess.
- Sleep disturbance (insomnia or hypersomnia).
- Sense of loss of control.
- Somatic — swelling of body, back pain, headache, breast tenderness.

5.3 Duration and rhythm

Symptoms begin in luteal phase (1 week pre-menses), completely resolve within a few days post-menses onset; complete remission in follicular phase (post-menses).

6. Diagnosis

6.1 Unified diagnostic criteria (DSM-5-TR · ICD-11)

A. In most menstrual cycles, 5+ symptoms in the last week of luteal phase; diminish within a few days after onset of menses; complete remission in postmenstrual phase.

B. At least 1 of the following:

1. Affective lability;
2. Irritability or anger;
3. Low mood or self-criticism;
4. Anxiety and tension.

C. Additional symptoms from the following (B + C total 5+):

5. Decreased interest;
6. Difficulty concentrating;
7. Lethargy, fatigue;
8. Appetite changes, excess;
9. Sleep disturbance;
10. Sense of loss of control;
11. Somatic (swelling, pain, etc.)

D. Significant distress or functional impairment.

E. Prospective mood diary for ≥ 2 cycles — mandatory for confirmation of diagnosis.

F. The symptoms are not merely an exacerbation of the symptoms of another psychiatric disorder (MDD, panic, dysthymic) (PMDD is a separate diagnosis; however, it can also occur against this background—"premenstrual exacerbation").

6.2 Diagnostic algorithm

1. Clinical interview — rhythm of cycle-related symptoms.
2. Prospective mood diary for 2 cycles — DRSP (Daily Record of Severity of Problems, Endicott J.) or PRISM (Premenstrual Record of Impact and Severity of Menstruation) — diagnosis cannot be confirmed by retrospective anamnesis.
3. Comorbidity screening (depression, anxiety, bipolar).
4. Medical (thyroid, anemia, polycystic ovary).

6.3 Differential diagnosis

Condition	Distinguishing features
PMS (premenstrual syndrome)	Soft symptoms, no functional impairment.
MDD luteal exacerbation	Core disturbance (MDD) is present throughout cycle, intensified in luteal phase.
Bipolar disorder	Hypomanic/manic episodes present.
Thyroid disorder	TSH.
Endometriosis, dysmenorrhoea	Pelvic pain dominant; mood symptoms mild.

7. Examination and assessment

- DRSP or PRISM mood diary.
- PHQ-9, GAD-7 — comorbidity.
- Pelvic examination (gynecologist) and TSH — somatic causes excluded.

8. Treatment

8.1 General principles (ACOG · ISPMD · APA 2010)

1. SSRI first-line: Fluoxetine, sertraline, paroxetine (FDA approved for PMDD), escitalopram. Two modes possible:
    - Persistent (daily): Throughout the entire cycle.
    - Luteal-phase intermittent: From ovulation to start of menses (14 days) — equally effective (Steiner M. et al. NEJM 1995 RCT).
2. Hormonal suppression — combined oral contraceptives (especially those containing drospirenone — Yaz, Beyaz; FDA-approved); GnRH agonists (leuprolide) in refractory cases.
3. Oophorectomy — last choice, in refractory severe cases; with estrogen replacement therapy.
4. CBT — adjunctive; Lustyk M.K. et al. reviews have shown modest effect.
5. Lifestyle: Physical activity, caffeine and alcohol reduction — adjunct.
6. Calcium supplementation (1200 mg/day) — Thys-Jacobs S. et al. Am J Obstet Gynecol 1998 RCT — moderate effect; adjunct role.

8.2 Treatment methods

1. Daily Record of Severity of Problems (DRSP) — Endicott (Endicott J.) — 21 symptoms, daily rating of 1–6 points; prospective 2 cycles required for PMDD diagnosis.
2. SSRI — Luteal Phase Regimen — From ovulation (day 14) until menses onset; as effective as continuous use. Steiner M. et al. NEJM 1995.
3. Drospirenone-containing Oral Contraceptive — Yaz, Beyaz — FDA approval for PMDD; hormonal stabilization.
4. GnRH Agonist (Leuprolide) — Ovarian suppression in refractory cases; risk of osteoporosis with long-term use.

8.3 Differences between sources

- ACOG (American College of Obstetricians and Gynecologists) Practice Bulletin: SSRI first-line; oral contraceptive second.
- ISPMD (International Society for Premenstrual Disorders) consensus: continuous or luteal SSRIs equal efficacy; GnRH refractory.

9. Prognosis

- SSRI or hormonal treatment leads to significant improvement in most patients.
- Persists during reproductive period; remission after menopause.
- Monitoring — cycle symptoms, compliance, side effects, comorbidity.

10. Myths and misconceptions

Myth 1: “PMDD is ‘female willpower weakness’, not a medical disorder”

Evidence: Schmidt P.J. NEJM 1998 — ovarian hormone suppression eliminates symptoms, hormone restart returns symptoms; neurobiologically based disorder. DSM-5 and ICD-11 official diagnosis.

Myth 2: “PMDD is also PMS, no difference”

Evidence: PMS mild form in 30–80% of women; PMDD characterized by clinical severity and functional impairment — 3–8% of women.

Myth 3: “SSRIs show no significant effect in PMDD, should be treated like depression”

Evidence: SSRI shows rapid effect (within 1–2 days) in PMDD — different pharmacokinetic mechanism from major depression; direct effect on serotonergic system.

Myth 4: “Only hormonal treatment (pregnancy, hormone replacement) is effective”

Evidence: SSRI and hormonal treatment both effective; SSRI first-line (for most patients); hormonal — with gynecological comorbidity or patient choice.

Myth 5: “Herbal preparations (chasteberry, evening primrose oil, St John's Wort) cure PMDD”

Evidence: Chasteberry (Vitex agnus-castus) has mild effect for PMS in some studies; evidence base for PMDD limited. Evening primrose oil — Cochrane negative results. St John's Wort — risk of interaction with SSRIs.

Myth 6: “Craniosacral therapy, homeopathy, acupuncture cure PMDD”

Evidence: Efficacy of these complementary interventions for PMDD has not been proven.

Myth 7: “PMDD diagnosis can be made from retrospective history”

Evidence: retrospective records are unreliable; Prospective 2-cycle diary is mandatory for diagnosis — this is the most frequent cause of underdetection.

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6B00–6B0Z ANXIETY DISORDERS

Anxiety or fear-related disorders

ICD-11 **6B00**	**6B00 GENERALISED ANXIETY DISORDER (GAD)** **Generalised anxiety disorder**
ICD-10 F41.1	Generalized anxiety disorder
DSM-5-TR F41.1	Generalized Anxiety Disorder

1. Definition and nosology

Generalized anxiety disorder (ICD-11: 6B00; DSM-5-TR: F41.1) — a disorder characterized by excessive anxiety that persists for at least several months (≥ 6 months in DSM-5-TR), encompasses multiple life domains (work, family, health, daily topics), is uncontrollable, and lacks sufficient triggers. It is accompanied by significant somatic symptoms (tension, fatigue, sleep disturbance).

2. History

- 1920s–1960s — term ‘anxiety neurosis’, linked to Freud's influence.
- DSM-III (1980) — “anxiety neurosis” → separation of panic disorder and GAD; GAD was a residual category.
- DSM-IV (1994), DSM-5 (2013) — GAD as separate disorder; ≥ 6-month duration; characteristic somatic symptoms.
- ICD-11 (2019) — has formulated the duration requirement for diagnosis as “several months” (less strict than DSM-5).

3. Epidemiology

- Lifetime prevalence: 3–9% (Kessler R.C. et al. WMH surveys).
- Annual prevalence: ~2–3%.
- Sex: 2 times higher in females.

- Onset — bimodal — adolescence and middle age; often chronic course.
- Comorbidity: MDD ~60%, social anxiety, panic, substance use, somatic (irritable bowel, cardiac complaints).

4. Aetiology and pathogenesis

- Heritability 30–40% (Hettema J.M. et al. Am J Psychiatry 2001 meta-analysis).
- Neurobiological — amygdala hyperresponsivity, impaired prefrontal-amygdala regulation; GABAergic and serotonergic dysregulation.
- Personality traits (neuroticism, behavioral inhibition) are high-risk factors.
- Environment — childhood trauma, chronic stress, parental overprotection.

5. Clinical features

5.1 Psychological symptoms

- Uncontrollable worry covering multiple areas.
- "Worry about worry" — meta-worry (Wells A. concept).
- Difficulty concentrating, mind going blank.
- Irritability.

5.2 Somatic symptoms

- Muscle tension.
- Fatigue.
- Sleep disturbance (difficulty falling asleep, difficulty staying asleep).
- Autonomic symptoms (tachycardia, sweating, dry mouth, headache).
- IBS is a common comorbidity, presenting with gastrointestinal complaints.

6. Diagnosis

6.1 Unified diagnostic criteria

A. Excessive anxiety or tension, focused on multiple areas; present most of the time during daily activities; ≥ 6 months (DSM-5-TR) / several months (ICD-11).

B. Difficulty managing anxiety.

C. ≥ 3 symptoms from the following (1 in children):

1. Restlessness, feeling keyed up or on edge;

2. Easy fatigability;
3. Difficulty concentrating;
4. Irritability;
5. Muscle tension;
6. Sleep disturbance.

D. Significant distress or functional impairment.

E. Exclusion of substance or medical condition.

F. Not better explained by another psychiatric disorder.

6.2 Differences between sources

- DSM-5-TR: ≥6 month period; ≥3 somatic symptoms.
- ICD-11: duration "a few months" — less stringent; somatic symptoms are required, but a specific number is not indicated.
- NICE CG113 (Generalised anxiety disorder and panic disorder in adults: management, 2011, 2020 update): Stepped care — psychoeducation + observation → guided self-help → CBT or pharmacotherapy → combination → specialized service in severe case.

6.3 Diagnostic algorithm

1. Clinical interview (areas of concern, triggers, somatic symptoms, avoidance).
2. SCID-5, MINI.
3. GAD-7 (screening, initial indicators and monitoring) — ≥ 10 moderate-to-severe anxiety.
4. HAM-A (Hamilton Anxiety) — clinician-rated.
5. Comorbidity screening (PHQ-9 depression, panic, social anxiety).
6. Exclude somatic causes — TSH, anemia, cardiac, caffeine or stimulant use.

6.4 Differential diagnosis

Condition	Distinguishing features
Panic disorder (6B01)	Discrete panic attacks.
Social anxiety (6B04)	Fear of social evaluation dominant.
OCD (6B20)	Ego-dystonic obsessions, compulsions.
PTSD (6B40)	Trauma history, intrusive symptoms.

Condition	Distinguishing features
Major depressive (6A70/6A71)	Affective symptoms predominate.
Hyperthyroidism	Decreased TSH, somatic symptoms.
Caffeine or stimulant intoxication	Use history.
Arrhythmia, pheochromocytoma	Specific cardiac and endocrine tests.

7. Examination and assessment

- GAD-7, HAM-A, PSWQ (Penn State Worry Questionnaire).
- Medical and laboratory (TSH, complete blood count, caffeine and alcohol use assessment).
- EKG (if cardiac complaint present).

8. Treatment

8.1 Stepped approach (NICE CG113 · CANMAT 2014)

1. Step 1 — psychoeducation and active monitoring.
2. Step 2 — guided self-help (internet-based CBT), psychoeducation groups.
3. Step 3 — high-intensity CBT or pharmacotherapy.
4. Step 4 — specialist service: combination treatment, refractory cases.

8.2 First line

Psychotherapy — cognitive behavioural therapy (CBT). The method with the strongest evidence base; 12–20 sessions. Components:

- Cognitive restructuring — testing beliefs about anxiety: judgements such as “worrying keeps me prepared” are set against the facts.
- Work with intolerance of uncertainty (Dugas M.J. model) — the patient deliberately experiences situations with unknown outcomes and builds tolerance.
- Exposure — stepwise withdrawal of avoidance of anxiety-provoking situations; carrying out postponed decisions.
- Problem-solving training — breaking a real problem into workable steps and accepting what cannot be solved.
- Relaxation — progressive muscle relaxation, diaphragmatic breathing.

Effect: reviews by Cuijpers P. et al. report a medium-to-large effect size; the result is comparable with pharmacotherapy and the relapse risk is lower.

Pharmacotherapy. Effect is assessed at 4–6 weeks; a full response may take 8–12 weeks:

- SSRIs — sertraline, escitalopram, paroxetine.
- SNRIs — venlafaxine, duloxetine.
- The starting dose is low: a temporary increase in anxiety is possible in the first days.

Slee A. et al. (Lancet 2019) network meta-analysis: duloxetine, pregabalin and venlafaxine showed high efficacy; the tolerability profile varies between agents.

8.3 Second line

- Pregabalin — approved for this indication in Europe; acts quickly but carries dependence potential.
- Buspirone — partial effect; as an alternative to benzodiazepines.
- Mirtazapine — adjunct where sleep disturbance accompanies.
- Quetiapine — second line in CANMAT 2014; use limited by metabolic adverse effects.

8.4 Short-term use and limits

- Benzodiazepines — only in the acute period, no longer than 2–4 weeks. Dependence potential is high; continuous use is not recommended.

8.5 Additional methods

- Mindfulness-based stress reduction and cognitive therapy (MBSR / MBCT) — an 8-week group programme; a mild effect in this disorder. Hofmann S.G. et al., J Consult Clin Psychol 2010 meta-analysis.
- Acceptance and commitment therapy (ACT) — instead of fighting the symptom it teaches living by one’s values alongside it; increases psychological flexibility. Randomised trials by Roemer L. et al.
- Applied relaxation (Öst L.G. method) — structured progressive muscle relaxation, applied when an anxiety trigger appears.
- Lifestyle — limiting caffeine, regular physical activity, sleep hygiene, avoidance of alcohol and psychoactive substances.

8.6 Duration of treatment

- Continued for at least 12 months after remission.

- In a chronic course or with multiple relapses, treatment is planned as long-term.
- Withdrawal is stepwise: abrupt discontinuation raises the risk of return.

8.7 Differences between sources

- NICE CG113 (2011, reaffirmed 2020) — stepped approach; SSRIs and SNRIs as first-line pharmacotherapy; benzodiazepines only acutely and briefly.
- APA — has no separate guideline for this disorder; the only document in the family concerns panic disorder (2009, archived status). Reference is therefore made to NICE CG113 and CANMAT 2014.
- CANMAT 2014 (Katzman M.A. et al.) — first line escitalopram, sertraline, venlafaxine, duloxetine; second line pregabalin, buspirone, mirtazapine, quetiapine.

9. Prognosis

- Chronic course; spontaneous remission is low.
- Significant symptom reduction and functional improvement with adequate treatment.
- Comorbid MDD is a marker of poor prognosis.
- Monitoring — GAD-7 monitoring, side effects, compliance, comorbidity.

10. Myths and misconceptions

Myth 1: “Anxiety is a normal life feeling, not a medical condition”

Evidence: A clinical distinction exists between normal anxiety and GAD — severity, duration (≥ 6 months), uncontrollability, somatic components, functional impairment. Clinical diagnosis is made based on specific criteria.

Myth 2: “Benzodiazepines are first-line treatment for GAD”

Evidence: NICE CG113, APA, CANMAT — benzodiazepines only for short-term (2–4 weeks) use; high dependence potential; SSRIs and SNRIs first-line.

Myth 3: “Only relaxation or yoga can cure GAD”

Evidence: They show moderate adjunctive effects, but in moderate-severe cases CBT and/or pharmacotherapy are required.

Myth 4: “Antidepressants are ineffective for anxiety — they are for depression”

Evidence: SSRI and SNRI effective in anxiety disorders; via effect of serotonergic system on amygdala activity. CANMAT 2014, Slee Lancet 2019.

Myth 5: “The patient can learn not to be anxious voluntarily”

Evidence: GAD is a neurobiological dysregulation; CBT or pharmacotherapy acts at the neurobiological level.

Myth 6: “Caffeine restriction is sufficient for GAD”

Evidence: Caffeine can exacerbate symptoms; restriction is an adjunct step, not primary treatment.

Myth 7: “Herbal preparations (kava, valerian, passiflora) can replace standard treatment”

Evidence: Kava severe hepatotoxicity (several fatalities — FDA warning); valerian and passiflora effect weak; St John's Wort interaction.

Myth 8: “Cannabis (CBD) cures anxiety”

Evidence: Preliminary studies exist for CBD, but insufficient for clinical recommendation; THC may exacerbate anxiety or trigger panic attacks.

Myth 9: “You will have to use antidepressants for life”

Evidence: After remission 12 months; then gradual discontinuation possible; restart if relapse.

Myth 10: “Electroconvulsive therapy is effective in refractory GAD”

Evidence: ECT is not indicated for GAD; in refractory cases — medication change, combination, atypical antipsychotic adjunct.

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ICD-11 **6B01**	**6B01 PANIC DISORDER** **Panic disorder**
ICD-10 F41.0	Panic disorder [episodic paroxysmal anxiety]
DSM-5-TR F41.0	Panic Disorder

1. Definition and nosology

Panic disorder (ICD-11: 6B01; DSM-5-TR: F41.0) — a disorder characterized by recurrent, unexpected panic attacks (acute episodes of fear or anxiety peaking within 10 minutes) and persistent worry about recurrence of these attacks, with behavioral changes.

2. History

- Da Costa J.M. (1871) ‘irritable heart’ — in American Civil War soldiers.
- Freud (1894) “anxiety neurosis”.
- Klein D.F. (1964) — distinguishing antidepressant response of panic attacks; basis for splitting ‘anxiety neurosis’.
- DSM-III (1980) — Official diagnosis of Panic Disorder.
- DSM-5 (2013), ICD-11 — Agoraphobia is a separate category from panic disorder.

3. Epidemiology

- Lifetime prevalence: 2–5%; annual ~2.7% (Kessler R.C. WMH).
- Sex: 2 times higher in females.
- Onset: young to middle adulthood (20–30 years); second peak in middle age.
- Comorbidity: agoraphobia 30–50%, MDD 50–60%, other anxiety disorders, substance use, mitral valve prolapse.

4. Aetiology and pathogenesis

- Heritability ~40% (Hettema 2001 meta-analysis).
- Neurobiological — amygdala-locus coeruleus-prefrontal circuit hyperreactivity; serotonergic, noradrenergic, GABAergic dysregulation.
- CO_2 sensitivity hypothesis (Klein D.F. ‘suffocation false alarm’) — patients show sensitivity to hyperventilation and CO_2 accumulation.
- Personality — anxiety sensitivity (Reiss S.); childhood history of separation anxiety.

5. Clinical features

Panic attack — 4+ symptoms peak within 10 minutes

- Palpitations, fluttering, tachycardia.

- Cold sweat, trembling.
- Shortness of breath, feeling of air insufficiency.
- Feeling of suffocation.
- Chest pain or discomfort.
- Nausea, abdominal discomfort.
- Dizziness, unreal feeling.
- Cold or hot sensations; paresthesia.
- Derealization, depersonalization.
- Fear of loss of control or "going crazy."
- Fear of death.

Attack lasts 5–20 minutes; hospital admission is frequent (with cardiac complaints).

6. Diagnosis

6.1 Unified diagnostic criteria

A. Repeated unexpected panic attacks.

B. At least one of the attacks has been followed by ≥ 1 month of one or more of the following:

- Persistent worry about next attack;
- Fear of consequences of attacks (heart attack, going crazy);
- Significant behavioral change associated with attacks (avoidance, safety behaviors).

C. Exclusion of substance or medical condition.

D. Not better explained by another mental disorder (social anxiety, OCD, PTSD, separation).

6.2 Differences between sources

- DSM-5-TR — agoraphobia as a separate diagnosis (F40.00); may co-occur.
- ICD-11 — agoraphobia has its own code 6B02; but no one-month period of concern is required — "persistent concern" is enough.
- NICE CG113 — diagnosis after exclusion of somatic cause.

6.3 Diagnostic algorithm

1. Clinical interview + attack history.
2. SCID-5, MINI.

3. PDSS (Panic Disorder Severity Scale), GAD-7 comorbidity.
4. Medical: EKG, TSH, complete blood count, glucose, caffeine/stimulants; pheochromocytoma suspicion — 24-hour urinary metanephrines.
5. Comorbidity (agoraphobia, MDD, substance use).

6.4 Differential diagnosis

Condition	Distinguishing feature
Agoraphobia (6B02)	Avoidance domains are primary; comorbid possible.
Social anxiety (6B04)	Social evaluation trigger.
Specific phobia (6B03)	Specific object/situation.
PTSD (6B40)	Trauma history, intrusive symptoms.
Hyperthyroidism	TSH.
Pheochromocytoma	Urine metanephrine.
Arrhythmia (SVT)	Holter EKG.
Substance-induced (caffeine, cocaine)	Toxicology.

7. Examination and assessment

- PDSS, ACQ (Agoraphobic Cognitions), BSQ (Body Sensations Questionnaire).
- EKG, TSH, complete blood count, glucose.
- Holter EKG (suspected arrhythmia).
- Toxicology.

8. Treatment

8.1 General principles (NICE CG113 · CANMAT 2014)

1. CBT for panic — psychoeducation, interoceptive exposure (building tolerance to bodily sensations), in vivo exposure (to avoided situations), cognitive restructuring. 12–16 sessions. Evidence: Cuijpers P. et al. reviews — large effect.
2. Pharmacotherapy: SSRIs (sertraline, paroxetine, fluoxetine — FDA-approved for panic disorder; escitalopram off-label) or SNRIs (venlafaxine) — first-line. Start at low dose (jittery, ‘activation’ effect may worsen panic).

3. Benzodiazepine (clonazepam, lorazepam) — short-term initial stabilization; ≤ 4 weeks; dependence potential.
4. Treatment duration — ≥ 12 months after remission.
5. Caffeine, alcohol, marijuana restriction.

8.2 Treatment methods

1. CBT for panic — Clark (Clark D.M.), Barlow (Barlow D.H.) — Interoceptive exposure (deliberately creating body sensation — hyperventilation, spinning, caffeine) + in vivo exposure + cognitive restructuring. Evidence: Barlow D.H. et al. JAMA 2000 RCT.
2. Panic Control Treatment (PCT) — Barlow (Barlow D.H.) — 11-session manually administered protocol; widely used in the USA.
3. Panic Disorder Severity Scale (PDSS) — Shear (Shear M.K.) — 7 items; baseline indicators and monitoring.
4. Internet-based cognitive behavioral therapy (iCBT) — Andersson G. et al. demonstrated that internet-based CBT — iCBT — showed comparable efficacy to individual CBT.

8.3 Differences between sources

- NICE CG113 — CBT and SSRIs first-line; benzodiazepines only short-term.
- APA Anxiety Guidelines — CBT and pharmacotherapy equally effective.
- Mitte K. J Affect Disord 2005 meta-analysis — CBT superior to placebo and wait-list.

9. Prognosis

- Remission with treatment in most patients.
- Comorbid agoraphobia or MDD poor prognosis.
- Early intervention prevents crystallization of avoidance behavior.

10. Myths and misconceptions

Myth 1: “Panic attack is a heart attack”

Evidence: symptoms may be similar; but panic attack is short-term, normal EKG after clinical presentation. Klein's ‘suffocation false alarm’ hypothesis explains the mechanism.

Myth 2: "The patient can voluntarily control panic"

Evidence: Panic is a neurobiological dysregulation; CBT and pharmacotherapy are required.

Myth 3: "Benzodiazepine is the main treatment for panic"

Evidence: For short-term stabilization; addiction potential; SSRIs and CBT first-line.

Myth 4: "A panic patient must be protected from panic-inducing situations"

Evidence: Avoidance strengthens the symptoms; exposure is the main therapeutic mechanism.

Myth 5: "Herbal preparations (kava, valerian) are safe for panic"

Evidence: Kava hepatotoxicity (FDA warning); standard treatment superior.

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ICD-11 **6B02**	**6B02 AGORAPHOBIA** **Agoraphobia**
ICD-10 F40.0	Agoraphobia
DSM-5-TR F40.00	Agoraphobia

1. Definition and nosology

Agoraphobia (ICD-11: 6B02; DSM-5-TR: F40.00) — excessive fear and avoidance of situations from which escape is difficult or help may not be available (public transportation, open spaces, enclosed spaces, crowds, leaving home alone). The patient fears developing panic attacks or other incapacitating symptoms.

2. History

- Westphal C. (1872) “agoraphobia” — Greek “agora” (square) + fear.
- DSM-III (1980) — Together with panic disorder.
- DSM-5 (2013), ICD-11 — agoraphobia formalized as a SEPARATE category from panic disorder (comorbidity possible, but based on separate diagnosis).

3. Epidemiology

- Lifetime prevalence: 1–2.5%.
- Sex: 2 times higher in females.
- Onset: young to middle adulthood.
- Comorbidity: panic disorder 50%, MDD, other anxiety, substance use.

4. Aetiology and pathogenesis

- Heritability ~48% (Mosing M.A. et al. Depress Anxiety 2009;26(11):1004–1011).
- Classical conditioning — fear transfer to a specific situation when a panic attack occurs in that situation.
- Cognitive factors — catastrophizing (“help won't come”).

5. Clinical features

Typical avoided situations:

- Public transportation (bus, train, airplane).
- Open areas (square, bridge, open ground).
- Closed spaces (store, cinema theater).
- Queuing or crowd.
- Leaving home alone.

The patient displays avoidance behaviors, “safety” behaviors (carrying a companion, mobile phone, medication), partial or complete home confinement. In severe cases, housebound.

6. Diagnosis

6.1 Unified diagnostic criteria

A. Significant fear or anxiety in ≥ 2 of the following five categories:

1. Public transportation;
2. Open areas;
3. Closed compartments;
4. Queue or crowd;
5. Leaving home alone.

B. These situations are feared because escape is difficult or help will not be available — in the event of a panic attack or incapacitating symptom.

C. Situations almost always provoke fear.

D. Active avoidance, companion demand, or enduring with acute distress.

E. Fear is disproportionate to the situation.

F. Persistent (≥ 6 months DSM-5-TR).

G. Significant functional impairment.

H. Not better explained by a medical or other mental disorder.

6.2 Differences between sources

- DSM-5-TR / ICD-11 — agoraphobia separate from panic disorder; both diagnoses are given if comorbid.
- NICE CG113 — agoraphobia with panic disorder: similar intervention.

6.3 Diagnostic algorithm

1. Clinical interview — avoided situations, panic attack history, functional burden.
2. SCID-5, MINI.
3. Mobility Inventory for Agoraphobia (MIA) — avoidance severity.
4. Comorbidity (panic, MDD, substance).
5. Medical examination.

6.4 Differential diagnosis

Condition	Distinguishing feature
Panic disorder (6B01)	Attack is primary; avoidance areas are limited or absent.
Specific phobia (6B03)	Concrete single object/situation.
Social anxiety (6B04)	Fear of social evaluation.
PTSD (6B40)	History of trauma.
Separation anxiety (6B05)	Fear of separation from a close person is dominant.

7. Examination and assessment

- MIA, ACQ, BSQ.
- GAD-7, PHQ-9, PDSS comorbidity.

8. Treatment

1. CBT — exposure-based; graded in vivo exposure to avoided situations; cognitive restructuring. First-line; large effect.
2. SSRI or SNRI — first-line pharmacotherapy (as in panic disorder).
3. Benzodiazepine — only short-term.
4. Virtual reality exposure — alternative means; evidence base developing.
5. Treatment duration — ≥ 12 months after remission.

8.1 Treatment methods

1. In vivo exposure — Staged approach — patient enters avoided situations in structured manner; therapist can accompany (therapist-assisted exposure). First-line method.

2. Virtual Reality Exposure Therapy (VRET — Virtual Reality Exposure Therapy) — Virtual environment created in clinical context; alternative to in vivo exposure. Botella C. et al. Behav Res Ther 2007.
3. CBT for panic (Clark, Barlow) — adapted for agoraphobia — Interoceptive + in vivo exposure.
4. Mobility Inventory (MIA) — Chambless (Chambless D.L.) — 27 situations; degree of avoidance alone and with partner.

8.2 Differences between sources

- NICE CG113 — CBT and SSRIs.
- CANMAT 2014 — same.
- Sánchez-Meca J. et al. Clin Psychol Rev 2010 meta-analysis — CBT-based exposure showed large effect.

9. Prognosis

- Exposure CBT — most patients achieve functional recovery.
- Chronicity without early intervention (housebound).
- Comorbid MDD predicts a poor prognosis.

10. Myths and misconceptions

Myth 1: “Patients with agoraphobia should not be placed in situations”

Evidence: Avoidance strengthens the symptoms; exposure is the main therapeutic mechanism.

Myth 2: “Agoraphobia is a ‘subtype’ of panic disorder”

Evidence: DSM-5-TR and ICD-11 — separate diagnostic categories; comorbidity possible, but can occur without the other.

Myth 3: “The patient is ‘lazy’, does not want to leave”

Evidence: Agoraphobia is a neurobiological and behavior-based clinical disorder; not a volitional or character problem.

Myth 4: “Only medication cures agoraphobia, therapy is unnecessary”

Evidence: without exposure-based CBT sustained remission difficult; pharmacotherapy alone does not change avoidance behavior.

Myth 5: “Ordering online or staying in distance education is an appropriate lifestyle for a patient with agoraphobia”

Evidence: Accommodation provides short-term relief but strengthens illness and reduces functional level.

11. Sources

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2. APA. DSM-5-TR. 2022.
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ICD-11 **6B03**	**6B03 SPECIFIC PHOBIA** **Specific phobia**
ICD-10 F40.2	Specific (isolated) phobias
DSM-5-TR F40.2xx	Specific Phobia

1. Definition and nosology

Specific phobia (ICD-11: 6B03; DSM-5-TR: F40.2xx) — marked, irrational fear and avoidance of a specific object or situation (animals, blood-injection, heights, flying, enclosed spaces). The patient usually recognizes the fear as excessive.

2. History

- Hippocrates (5th century BC) — descriptions of phobia.
- Westphal C. (1872) — systematic classification.
- Marks I.M. (1969) — systematic desensitization and exposure studies.

- DSM-5 (2013), ICD-11 — subtypes: animal, natural environment, blood-injection-injury, situational, other.

3. Epidemiology

- Lifetime prevalence 7–12% — one of the most common anxiety disorders.
- Sex: 2–3 times higher in females (blood-injection subtype exception — difference small).
- Onset: childhood (animal, blood-injection), young adulthood (situational).

4. Aetiology and pathogenesis

- Heritability is estimated at approximately 30% (Hettema, 2001).
- Aetiological models include classical conditioning (Watson, ‘Little Albert’, 1920), vicarious learning, and informational transmission.
- Seligman’s concept of biological preparedness posits a genetic predisposition toward fear of phylogenetically relevant stimuli (e.g., snakes, spiders).
- In the blood-injection-injury subtype, the characteristic vasovagal response (bradycardia, hypotension, and syncope) distinguishes it from all other specific phobias.

5. Clinical features

Subtypes

- Animal — spider, snake, dog, insects.
- Natural environment — heights, water, storms.
- Blood-injection-injury — vasovagal reaction (loss of consciousness characteristic).
- Situational — flight, elevator, tunnel, driving, enclosed place.
- Other — choking, vomiting, clowns, certain sounds.

6. Diagnosis

6.1 Unified diagnostic criteria

A. Acute fear or anxiety about a specific object or situation.

B. Object/situation almost always provokes fear.

C. Enduring with active avoidance or acute distress.

D. Fear is disproportionate to the actual risk.

E. ≥6 months duration (DSM-5-TR).

F. Significant distress or functional impairment.

G. Not better explained by another mental disorder.

6.2 Differences between sources

- DSM-5-TR / ICD-11 — subtype qualifier is mandatory.
- NICE — no separate specific phobia guideline; general anxiety principles.

6.3 Diagnostic algorithm

1. Clinical interview.
2. FSS (Fear Survey Schedule), specific phobia subscales.
3. Functional burden assessment.

6.4 Differential diagnosis

Condition	**Distinguishing feature**
Panic disorder (6B01)	Attack is unexpected.
Agoraphobia (6B02)	Comprehensive situational category.
Social anxiety (6B04)	Fear of social evaluation.
OCD (6B20)	Obsession is primary; contamination fear with compulsions.
PTSD (6B40)	History of trauma.
Hypochondriasis (6B23)	Fear of illness.

7. Examination and assessment

- FSS, specific phobia scale.
- BAT (Behavioral Avoidance Test) — the degree to which the patient approaches the phobic object.

8. Treatment

1. Exposure therapy is first-line and most effective. Single session 2–3-hour structured exposure (Öst L.G. “one-session treatment”) is effective for most animal and situational phobias.

2. Stepped in vivo exposure — adapted to patient's pace.
3. Virtual Reality Exposure — particularly suitable for flight, height.
4. In the blood-injection subtype — applied tension (Öst) — the patient is taught to tense the muscles, which prevents the vasovagal response.
5. Pharmacotherapy has no primary role; benzodiazepines only for specific events (e.g., pre-flight) as short-term adjunct; may sometimes reduce efficacy of CBT exposure.
6. D-cycloserine adjunct — may enhance exposure efficacy (evidence base developing).

8.1 Treatment methods

1. One-Session Exposure (One-Session Treatment) — Öst (Öst L.G.) — 2–3 hour structured in vivo exposure; 80–90% efficacy for most animal and situational phobias. Öst L.G. Behav Res Ther 1989.
2. Systematic Desensitization (Systematic Desensitization) — Wolpe (Wolpe J.) — Historical method — relaxation + gradual phobic stimulus; superseded by in vivo exposure.
3. Applied Tension (Öst — blood-injection) — Tightening muscles for 10–15 seconds — prevents vasovagal syncope; in specific blood-injection phobia.
4. Virtual Reality Exposure Therapy (VRET — Virtual Reality Exposure Therapy) — For flying, height, spider phobias. Botella C. et al. reviews.
5. D-cycloserine adjunct — NMDA partial agonist; when administered prior to exposure sessions, may enhance extinction learning. Hofmann S.G. et al. review — moderate evidence.

8.2 Differences between sources

- Öst L.G. — one-session exposure demonstrated 80–90% efficacy (Behav Res Ther 1989).
- Wolitzky-Taylor K.B. et al. Clin Psychol Rev 2008 meta-analysis (33 RCTs) — exposure showed a large effect versus wait-list and a medium effect versus placebo; in vivo contact outperformed other modes of exposure. This meta-analysis made no comparison with pharmacotherapy.
- APA Anxiety Guidelines — exposure first-line.

9. Prognosis

- Exposure CBT — majority of patients achieve remission.
- Without treatment, chronicity.

10. Myths and misconceptions

Myth 1: "Specific phobia is only a childhood phobia and does not require treatment in adults"

Evidence: Specific phobia creates functional impairment in adulthood too (avoidance of flying — occupational limitation; avoidance of medical procedures — health risk).

Myth 2: "Forcing the patient into a phobic object causes trauma"

Evidence: appropriately applied stepped exposure is safe and highly effective; with patient consent and pace.

Myth 3: "Hypnotherapy or NLP rapidly cures phobias"

Evidence: systematic evidence base is limited; exposure-based approach is the gold standard.

Myth 4: "A patient can live a normal life by completely avoiding the phobic object"

Evidence: Avoidance strengthens phobia; situational phobias (flight, elevator) create medical and social restrictions.

Myth 5: "Benzodiazepine is an effective treatment for specific situations like flying"

Evidence: Possible short-term adjunct role, but not long-term solution; may reduce CBT/exposure efficacy (state-dependent learning).

11. Sources

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ICD-11 **6B04**	**6B04 SOCIAL ANXIETY DISORDER** **Social anxiety disorder**
ICD-10 F40.1	Social phobias
DSM-5-TR F40.10	Social Anxiety Disorder

1. Definition and nosology

Social anxiety disorder (ICD-11: 6B04; DSM-5-TR: F40.10) — intense and persistent fear of negative evaluation or humiliation by others in social situations. The patient avoids situations such as speaking, interaction, and being observed.

2. History

- Janet P. (1903) — ‘phobie des situations sociales’.
- Marks I.M., Gelder M.G. (1966) — separation of social phobia from agoraphobia.
- DSM-III (1980) — “Social Phobia” as an official diagnosis.
- DSM-5 (2013) — “Social Anxiety Disorder” — broader and more precise term; performance-only subtype added.
- ICD-11 — parallel.

3. Epidemiology

- Lifetime prevalence: 7–13% (Kessler WMH; cultural variability).
- Sex: relatively higher in females, but males seek clinical help more often.
- Onset: adolescence (mean 13 years) — one of the earliest onset anxiety disorders.

- Comorbidity: MDD 60%, other anxiety, alcohol use (as self-medication).

4. Aetiology and pathogenesis

- Heritability ~30–40%.
- Behavioral inhibition (Kagan J.) — childhood temperament as a strong risk factor.
- Neurobiological — amygdala hyperreactivity to social stimuli; serotonergic dysregulation.
- Cognitive — self-focused attention, negative self-image, catastrophizing (Clark D.M., Wells A. model).
- Environment — childhood peer victimization, parental overprotection.

5. Clinical features

- Feared situations — public speaking, asking questions, sharing a meal, writing or eating in front of others, public toilets, talking on the phone.
- Somatic — flushing, tremor, sweating, tachycardia, dry mouth, voice tremor.
- Cognitive — negative thoughts about oneself, “post-event processing” (prolonged self-criticism after a situation).
- Avoidance and ‘safety’ behaviors (avoiding eye contact, speaking less, alcohol use).
- Functional burden — academic and occupational development, friends, romantic relationships.

6. Diagnosis

6.1 Unified diagnostic criteria

A. Marked fear in one or more social situations — being observed or evaluated by others.

B. The patient fears humiliation, rejection by others, or offending others.

C. Situations almost always provoke fear.

D. Avoidance or endurance with intense discomfort.

E. Fear is disproportionate to the social situation.

F. ≥6 months duration (DSM-5-TR).

G. Functional impairment.

H. Not fully explained by another mental disorder or medical condition.

6.2 Differences between sources

- DSM-5-TR — “performance only” specifier (limited to performance situations).
- ICD-11 — the duration is “at least several months” (6 months in DSM-5-TR); in children the peer setting is assessed as well, not only contact with adults.
- NICE CG159 (Social anxiety disorder: recognition, assessment and treatment, 2013) — individual CBT first-line.

6.3 Diagnostic algorithm

1. Clinical interview.
2. SCID-5, MINI.
3. Liebowitz Social Anxiety Scale (LSAS), SPIN (Social Phobia Inventory) — scale and monitoring.
4. Comorbidity (MDD, other anxiety, alcohol use).

6.4 Differential diagnosis

Condition	Distinguishing feature
Agoraphobia (6B02)	Difficult escape, help absent; social evaluation absent.
Specific phobia (6B03)	Specific object/situation.
Panic (6B01)	Attack is unexpected.
Avoidant personality disorder	Persistent pattern, broader; comorbidity is common.
Autism spectrum (6A02)	Social-communication deficit early onset; social interest variable.
Selective mutism (6B06)	Failure to speak in specific contexts.
Body dysmorphic disorder (6B21)	Appearance-based anxiety.

7. Examination and assessment

- LSAS — gold standard scale (24 items).
- SPIN — short screening (17 items).

- BFNE (Brief Fear of Negative Evaluation).

8. Treatment

8.1 General principles (NICE CG159 · CANMAT 2014)

1. Individual CBT (Clark D.M. model) first-line — redirecting self-focused attention, video feedback, behavioral experiments, cognitive restructuring, in vivo exposure. 12–16 sessions. NICE CG159 — individual CBT superior to group CBT.
2. SSRI and SNRI first-line pharmacotherapy — escitalopram, sertraline, paroxetine (FDA approved), venlafaxine (FDA approved).
3. Second-line — moclobemide (RIMA, approved in Europe), pregabalin (evidence limited), gabapentin.
4. Benzodiazepine — only short-term for a specific situation (e.g., prior to performance); not sustained.
5. β-blocker (propranolol) — in performance subtype, 1–2 hours before performance; ineffective in social reciprocal interaction.
6. Duration of treatment — after remission ≥ 12 months.
7. Alcohol use — as a form of self-medication; requires special attention and parallel intervention.

8.2 Treatment methods

1. Clark CBT Model — Specifically developed for social anxiety; external redirection of self-focused attention, video feedback (seeing one's performance through others' eyes), behavioral experiments (testing fearful beliefs), in vivo exposure. Evidence: Clark D.M. et al. J Consult Clin Psychol 2003 RCT — experimental individual CBT superior to other interventions.
2. Heimberg Group CBT — 12-session group format; Heimberg R.G. model. Effective, but relatively weaker than individual CBT (NICE CG159).
3. Internet-based cognitive behavioral therapy (iCBT) — Andersson G. et al. RCTs — therapist-supported iCBT is comparable to individual CBT.
4. Liebowitz Social Anxiety Scale (LSAS — Liebowitz Social Anxiety Scale) — 24 items (11 social interaction + 13 performance); each item is rated for both fear/anxiety and avoidance (48 ratings, 0–144 points, 4 subscales); gold standard.
5. Social Phobia Inventory (SPIN) — 17 items; brief screening.

8.3 Differences between sources

- NICE CG159 (2013) — individual CBT (Clark) first-line; SSRIs alternative; group CBT or self-help may be suitable for some patients.
- CANMAT 2014 — first-line escitalopram, sertraline, paroxetine, venlafaxine.
- Mayo-Wilson E. et al. Lancet Psychiatry 2014 network meta-analysis — individual CBT has the greatest effect; SSRIs second.

9. Prognosis

- Chronic course, high relapse rate without treatment.
- CBT and SSRIs result in significant improvement in most patients.
- Early onset and avoidant personality comorbidity as a poor prognosis marker.

10. Myths and misconceptions

Myth 1: "Social anxiety — ordinary 'shyness about life', does not require treatment"

Evidence: Social anxiety disorder is associated with functional impairment, academic and occupational limitations, risk of depression and alcohol use; it is evaluated as a medical condition.

Myth 2: "The patient can live a normal life by completely avoiding social situations"

Evidence: Avoidance strengthens the symptoms and leads to isolation; exposure is the main therapeutic mechanism.

Myth 3: "Alcohol 'treats' social anxiety"

Evidence: Alcohol provides short-term relief but long-term exacerbates distress and leads to alcohol use disorder; prevalence of this combination is 20–40%.

Myth 4: "Benzodiazepines are effective long-term for social anxiety"

Evidence: Short-term effect; addiction potential; CBT or SSRIs superior.

Myth 5: "Propranolol is effective for all forms of social anxiety"

Evidence: propranolol is effective only in the performance subtype (before a performance); it is ineffective in social interaction (parties, relationships).

Myth 6: “The patient has avoidant personality disorder, so treatment is unresponsive”

Evidence: Comorbid avoidant personality patient may also respond to standard interventions; long-term CBT or schema therapy.

Myth 7: “Only hypnotherapy or NLP cures quickly”

Evidence: No evidence base; CBT and SSRIs are gold standard.

Myth 8: “Cannabis (CBD) is a safe alternative for social anxiety”

Evidence: initial studies exist, but insufficient for clinical recommendation; THC may worsen distress.

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3. NICE CG159. Social anxiety disorder: recognition, assessment and treatment. 2013.
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ICD-11 6B05	**6B05 SEPARATION ANXIETY DISORDER** **Separation anxiety disorder**
ICD-10 F93.0	Separation anxiety disorder of childhood
DSM-5-TR F93.0	Separation Anxiety Disorder

1. Definition and nosology

Separation anxiety disorder (ICD-11: 6B05; DSM-5-TR: F93.0) — developmentally inappropriate excessive fear and anxiety related to separation from an attachment figure (typically parent) or home. First diagnosable in adults in DSM-5 (limited to childhood in DSM-IV).

2. History

- Bowlby J. (1969–1980) — attachment theory.
- DSM-III (1980) — As a childhood disorder.
- DSM-5 (2013) — age restriction removed; diagnosis possible in adults as well.

3. Epidemiology

- Prevalence in children: ~4%; in adolescents ~1.6%; in adults ~1.9% (Kessler R.C. NCS-R).
- Sex: relatively higher in females.
- Onset: typical childhood (6–9 years); may recur or persist in adult form.
- Comorbidity: specific phobia, panic, GAD, MDD, school avoidance.

4. Aetiology and pathogenesis

- Heritability ~40%.
- Attachment styles (insecure attachment), parental overprotection.
- History of loss or illness in relatives.
- Stressful events (moving, school change, parental separation).

5. Clinical features

- Excessive worry about harm or loss to the attachment figure.
- Avoidance of leaving home or separating from attachment figure.
- Fear of being alone.
- Persistent reluctance or refusal to sleep away from home or to go to sleep without the attachment figure nearby (own bedroom, unfamiliar house).
- Nightmares about separation.
- Somatic complaints in anticipation of separation (abdominal pain, headache, nausea).
- In children — school avoidance.
- In adults — fear of separation from partner or children, excessive checking behaviors.

6. Diagnosis

6.1 Unified diagnostic criteria

A. ≥ 3 of the following symptoms:

1. Excessive anxiety about separation from the attachment figure or anticipation thereof;
2. Persistent worry about loss or harm to the attachment figure;
3. Fear of an event that would cause separation from the attachment figure;
4. Avoidance of home, school, or work due to fear of separation;
5. Fear of being alone without the attachment figure;
6. Reluctance to go to sleep without the attachment figure nearby;
7. Separation nightmares;
8. Somatic complaints in anticipation of separation.

B. Duration: children/adolescents ≥ 4 weeks; adults ≥ 6 months.

C. Significant distress or functional impairment.

D. Not better explained by another mental disorder.

6.2 Differences between sources

- DSM-5-TR — duration ≥6 months in adults; ≥4 weeks in children.
- ICD-11 — a different duration: “at least several months”; there is no DSM-5-TR split into 4 weeks for children and 6 months for adults, and no separate age qualifier.

6.3 Diagnostic algorithm

1. Clinical interview (also with parent in children).
2. Standardized interview (K-SADS for children, SCID-5 for adults).
3. Scale — SAAS (Separation Anxiety Assessment Scale).
4. Comorbidity screening.

6.4 Differential diagnosis

Condition	Distinguishing feature
GAD (6B00)	Multiple domains of worry.
Social anxiety (6B04)	Social evaluation dominant.
Agoraphobia (6B02)	Avoidance of situations with difficult escape.
Panic (6B01)	Attack is unexpected.
Selective mutism (6B06)	Failure to speak in specific contexts.
Conduct disorder (school avoidance)	Antisocial behavior component.
Avoidant personality	Persistent pattern across all relationships.

7. Examination and assessment

- SAAS, K-SADS, MASC (Multidimensional Anxiety Scale for Children).
- Comorbidity (depression, other anxiety).

8. Treatment

1. CBT (Coping Cat, FRIENDS programs for children; standard CBT for adults) — exposure-based, parental involvement critical in children. First-line.
2. SSRIs — in moderate-to-severe cases or when CBT fails; sertraline, fluoxetine (in children).
3. School return programs (school refusal) — graded exposure.
4. Family involvement — parental psychoeducation, avoidance of frequent phone calls/visits (accommodation strengthens symptoms).

8.1 Treatment methods

1. “Coping Cat” program — Kendall (Kendall P.C.) — Manualized CBT for children aged 7–17 years – 16 sessions, psychoeducation + relaxation + exposure + reward system (FEAR plan).
2. FRIENDS Program — Barrett (Barrett P.M.) — In Australia, developed group CBT for child and adolescent anxiety disorders.
3. Stepped School Return — Structured exposure — child gradually returns to school environment; coordination of parent and school staff.
4. Parental Accommodation Reduction — SPACE (Supportive Parenting for Anxious Childhood Emotions — Lebowitz E.) program — modifying parental accommodation of child's anxiety.

8.2 Differences between sources

- AACAP Practice Parameter (Connolly S.D., Bernstein G.A. 2007) — CBT and SSRIs effective in children.
- NICE — general guidelines for childhood anxiety (CBT first-line).
- Walkup J.T. et al. NEJM 2008 (CAMS — Child/Adolescent Anxiety Multimodal Study) — CBT + sertraline combination superior to monotherapy.

9. Prognosis

- Remission in most children with early CBT.
- Without treatment, continuation in adulthood leads to risk of other anxiety disorders and depression.

10. Myths and misconceptions

Myth 1: “Separation anxiety occurs only in childhood”

Evidence: DSM-5 — separation anxiety disorder also diagnosed in adults — prevalence ~1.9%.

Myth 2: “The parent must give in to every demand to ‘calm’ the child”

Evidence: Accommodation strengthens the symptoms; the SPACE program or parent training is recommended.

Myth 3: “Separation anxiety is a result of poor parenting”

Evidence: heritability ~40%; attachment and environment modulator. “Bad upbringing” explanation stigmatizing and incorrect.

Myth 4: "Leaving a child alone to 'get used to it' — this harsh approach cures them"

Evidence: "Flooding" or uncontrolled exposure can be traumatic; graded, structured exposure recommended.

Myth 5: "Recovery is only possible with medication"

Evidence: CBT first-line; SSRIs for moderate-severe cases or in combination.

Myth 6: "School refusal is a personality weakness and requires discipline"

Evidence: School avoidance is a clinical anxiety symptom; punishment-based approach exacerbates the disorder.

11. Sources

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ICD-11 **6B06**	**6B06 SELECTIVE MUTISM** **Selective mutism**
ICD-10 F94.0	Elective mutism
DSM-5-TR F94.0	Selective Mutism

1. Definition and nosology

Selective mutism (ICD-11: 6B06; DSM-5-TR: F94.0) — consistent failure to speak in specific social situations (e.g., school) despite mastery of language, with normal speech in other contexts (e.g., home). A typical childhood anxiety disorder, sometimes misperceived as “extreme shyness.”

2. History

- Kussmaul A. (1877) — “aphasia voluntaria”.
- Tramer M. (1934) — “elective mutism” — patient's voluntary choice expression.
- DSM-IV (1994) — Replaced with the term ‘selective’ (not voluntary, due to social anxiety).
- DSM-5 (2013) — moved to the anxiety disorders category.

3. Epidemiology

- Prevalence: 0.7–1% in school-aged children (Bergman R.L. et al. JAACAP 2002).
- Sex: relatively higher in girls (1.5–2:1).
- Onset: 2–5 years; typically detected at school entry.
- Comorbidity: social anxiety disorder ~70%, GAD, other anxiety, speech or language disorder.

4. Aetiology and pathogenesis

- Heritability — anxiety in family history prevalent (~70% families have anxiety disorder).

- Behavioral inhibition (Kagan) — temperamental predisposition is a key factor.
- Comorbid developmental speech and language disorders (especially in bilingual environment) — risk factor.
- Environment — in bilingual families, sometimes mistakenly perceived as a language switch.

5. Clinical features

- Failure to speak in specific social situations (school, public places).
- Normal speech at home with close ones.
- Sometimes the use of whispering or gestures.
- Academic and social impairment (answering questions, peer relationships).
- Comorbid shyness, social withdrawal, somatic complaints.

6. Diagnosis

6.1 Unified diagnostic criteria

A. Persistent not speaking in specific social situations (where speaking is expected), speech normal in other situations.

B. Significant impairment in academic, occupational, or social communication.

C. Duration ≥ 1 month (excluding the first month of school).

D. Does not result from not learning a language or not being comfortable in that language.

E. Not better explained by a communication disorder (e.g., a fluency disorder), autism spectrum disorder, schizophrenia, or another psychotic disorder.

6.2 Differences between sources

- DSM-5-TR / ICD-11 — in the anxiety disorders category; ‘selective’ — not voluntary choice, but situation-specific anxiety response.

6.3 Diagnostic algorithm

1. Parent, teacher interview.
2. Home video recordings of speech — diagnostic confirmation.
3. SMQ (Selective Mutism Questionnaire).
4. Speech-language assessment (SLT) — communication disorder exclusion.

5. Hearing test.
6. Comorbid anxiety, autism, language disorder screening.

6.4 Differential diagnosis

Condition	Distinguishing feature
Developmental language disorder (6A01)	Language is poor in all contexts.
Autism spectrum (6A02)	Social-communication deficit in all contexts.
Bilingual environment transient	During adaptation to a new language — typically months.
Hearing impairment	Audiometry.
Social anxiety (6B04)	Comorbidity is common — the patient speaks but fears social evaluation.
Mutism in trauma context	History of traumatic event.

7. Examination and assessment

- SMQ — gold standard; SAS (School Anxiety Scale).
- Speech-language assessment.
- Audiometry.

8. Treatment

1. Behavioral approach as first-line treatment — graded exposure, fading-in technique (gradual introduction of a familiar person into the environment).
2. Integrated Behavioral Therapy (Bergman R.L.) — manualized, with family + school involvement.
3. School-based intervention — gradual increase in communication demands, teacher coordination.
4. SSRI (fluoxetine, sertraline) – in moderate to severe or non-responsive to behavioral intervention cases. Black B. et al. JAACAP 1994 RCT.
5. Reducing accommodation — stopping the parent from speaking for the child.

8.1 Treatment methods

1. Gradual Introduction (Fading-In) Technique — The patient converses with a comfortable person; gradually a stranger is introduced into this context; then the patient transitions to speaking with a stranger.
2. Integrated Behavioral Therapy (IBT) — Bergman (Bergman R.L.) — Staged exposure, positive reinforcement, systematic desensitization; 20 sessions. Evidence: Bergman R.L. et al. JAACAP 2013 RCT.
3. Selective Mutism Questionnaire (SMQ) — Bergman — 23 items; parent form.
4. SSRI — Fluoxetine — Evidence base in pediatric selective mutism; Black B., Uhde T.W. JAACAP 1994 RCT.

8.2 Differences between sources

- AACAP — behavioral intervention first-line, SSRI adjunct.
- NICE — principles of anxiety disorders apply.

9. Prognosis

- Speech recovers in most children with early intervention.
- Without treatment, chronicity and transition to social anxiety disorder.

10. Myths and misconceptions

Myth 1: “Selective mutism is the child's ‘stubbornness’”

Evidence: selective mutism is an anxiety disorder — the child CANNOT speak, the position ‘does not want to speak’ is incorrect.

Myth 2: “If a child does not speak, strong pressure is needed”

Evidence: pressure intensifies anxiety and mutism; stepped exposure recommended.

Myth 3: “It will resolve with age; no intervention is needed”

Evidence: Without early intervention, academic and social impairment persists; risk of transition to social anxiety disorder in adulthood.

Myth 4: “Mutism in a bilingual child is normal”

Evidence: “Silent period” — normal for a few months when learning a new language; mutism lasting ≥ 6 months or occurring in both languages — consider selective mutism diagnosis.

Myth 5: “Only deep psychoanalytic therapy is effective”

Evidence: Behavior-based and CBT approaches are first-line; psychoanalytic intervention lacks evidence base.

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6B20–6B2Z OBSESSIVE-COMPULSIVE (OCD)

Obsessive-compulsive or related disorders

ICD-11 **6B20**	**6B20 OBSESSIVE-COMPULSIVE DISORDER (OCD)** **Obsessive-compulsive disorder**
ICD-10 F42	Obsessive-compulsive disorder
DSM-5-TR F42.2	Obsessive-Compulsive Disorder

1. Definition and nosology

Obsessive-Compulsive Disorder (OCD; ICD-11: 6B20; DSM-5-TR: F42.2) — a disorder characterized by recurrent, ego-dystonic obsessions (unwanted, intrusive thoughts, images, impulses) or compulsions (repetitive behaviors or mental acts). Usually both are present. The patient performs the compulsion to reduce anxiety or prevent a feared outcome.

DSM-5 (2013) — OCD removed from anxiety disorders as a distinct category, moved to the "Obsessive-Compulsive and Related Disorders" category; ICD-11 followed a parallel step.

2. History

- Esquirol J.E.D. (1838) — "monomanie raisonnante".
- Janet P. (1903) — 'psychasthénie'.
- Freud (1909) — "Rat Man" case.
- Mowrer O.H. (1947) — two-factor learning theory (classical conditioning + operant avoidance); basis of ERP.
- Meyer V. (1966) — first clinical application of Exposure and Response Prevention (ERP).

- DSM-5 (2013) and ICD-11 (2019) — OCD separated from anxiety disorders.

3. Epidemiology

- Lifetime prevalence: 1.6–2.3% (Ruscio A.M. et al. Mol Psychiatry 2010).
- Sex: approximately equal; relatively more common in women in adulthood, early onset in childhood in men.
- Onset: bimodal — prepubertal (10–12 years, males) and early adulthood (20 –25 years).
- Comorbidity: MDD ~60%, anxiety disorders, tic disorders, autism, hoarding, BDD.

4. Aetiology and pathogenesis

- Heritability 40–50% (Mataix-Cols D. et al. JAMA Psychiatry 2013).
- Neurobiological — hyperactivity of the cortico-striato-thalamo-cortical (CSTC) circuit (orbitofrontal, cingulate, caudate); serotonergic dysregulation (SSRI effect); glutamatergic dysregulation.
- Genetic research — SLC1A1 (glutamate transporter), serotonin genes.
- PANDAS — rare onset of OCD following streptococcal infection (NIMH 2017).
- Trauma, stress triggering factors.

5. Clinical features

Typical symptom domains

- Contamination / cleanliness — most frequent; hand washing, disinfection compulsions.
- Doubting / checking — lock, plate, electrical devices.
- Symmetry / precision / ‘just right’ — related to orderliness.
- Aggressive / sexual / religious obsessions — ego-dystonic, causing distress.
- Hoarding — Separate category in ICD-11 (6B24).

Ego-dystonia

The patient accepts that obsessions are their own thoughts, but unwanted, irrational, or unacceptable (insight varies — DSM-5-TR insight qualifiers: good / poor / absent).

6. Diagnosis

6.1 Unified diagnostic criteria

A. Presence of obsession, compulsion, or both.

- Obsessions: repeated, persistent, unwanted thoughts/images/impulses; creates distress; patient attempts to suppress or neutralize with other thoughts/actions.
- Compulsions: Repetitive behavior (hand washing, checking, ordering) or mental act (counting, praying) performed as a response to an obsession or according to rigid rules; aimed at reducing anxiety, but either unrealistic or excessive.

B. Obsession/compulsions take >1 hour per day or cause significant distress/functional impairment.

C. Exclusion of substance or medical condition.

D. Not better explained by another mental disorder.

6.2 Differences between sources

- DSM-5-TR — insight specifiers (good/fair — poor — absent/delusional); tic-related subtype.
- ICD-11 — parallel; hoarding (6B24), BDD (6B21), olfactory reference (6B22) separate categories.
- NICE CG31 (2005, 2019 surveillance) — diagnostics via clinical interview + Y-BOCS.

6.3 Diagnostic algorithm

1. Clinical interview.
2. SCID-5, Y-BOCS (Yale-Brown Obsessive Compulsive Scale) — the gold standard severity scale; CY-BOCS for children.
3. OCI-R (Obsessive Compulsive Inventory-Revised) — self-assessment.
4. Comorbidity (depression, anxiety, tic, autism, BDD, hoarding).

6.4 Differential diagnosis

Condition	Distinguishing feature
GAD (6B00)	Real-life worries, compulsions absent.

Condition	Distinguishing feature
Tic (8A05)	Motor/vocal; not goal-directed; premonitory urge.
BDD (6B21)	Appearance-based obsessions.
Hoarding (6B24)	Difficulty discarding objects.
Eating disorders (6B8x)	Eating behavior is primary.
Trichotillomania, dermatillomania (6B25.x)	Body-focused repetitive behavior.
Obsessive-Compulsive Personality Disorder (OCPD)	Persistent personality pattern, ego-syntonic; OCD is ego-dystonic.
Schizophrenia / delusion disorder	Insight at the delusion level; insight in OCD is variable.
Autism spectrum	Restricted-repetitive behavior; ego-syntonic.

7. Examination and assessment

- Y-BOCS / CY-BOCS — gold standard.
- OCI-R — self-assessment.
- Comorbidity (depression, anxiety).

8. Treatment

8.1 General Principles (NICE CG31 · APA · WFSBP)

1. First-line psychotherapy: ERP (Exposure and Response Prevention) — patient is exposed to feared object/situation and does not perform compulsion. 12–20 sessions. Evidence: Foa E.B. et al. RCTs; effect substantial.
2. First-line pharmacotherapy: SSRIs at high dose — fluoxetine (40–80 mg), sertraline (200 mg), paroxetine (40–60 mg), fluvoxamine (200–300 mg), escitalopram (20–40 mg). In OCD a higher dose than the standard depression dose is required; effect 8–12 weeks (slower than in depression).
3. Combination CBT/ERP + SRI — gives no advantage over ERP alone: in the Foa E.B. et al. Am J Psychiatry 2005 RCT (n=122, clomipramine) ERP and ERP+clomipramine did not differ, and both beat clomipramine alone (response 62–86% ERP, 70–79% combination, 42–48% clomipramine, 8–

10% placebo). ERP is the strong component; pharmacotherapy is added when ERP is unavailable or comorbidity requires it.

4. Clomipramine (TCA) — first-line alternative; in case of non-response to SSRI; anticholinergic and cardiac side effects.
5. Augmentation (SSRI + low-dose atypical antipsychotic) — risperidone, aripiprazole; in refractory cases.
6. Refractory severe OCD: Deep Brain Stimulation (DBS — ventral capsule/ventral striatum), neurosurgery (capsulotomy, cingulotomy); ECT ineffective; ketamine/esketamine rapid effect studies ongoing.
7. Duration of treatment — long-term; ≥ 1–2 years after remission, lifelong in case of multiple relapses.

8.2 Treatment methods

1. Exposure and Response Prevention (ERP) — Meyer (Meyer V.), Foa (Foa E.B.) — The patient is exposed to feared object (e.g., "dirty" object) and does not perform compulsion (washing hands); anxiety gradually decreases (habituation). Standard course 12–20 sessions, 1–2 times per week; more intensive format (daily) is effective. Evidence: Foa E.B. et al. Am J Psychiatry 2005.
2. CBT — cognitive component — Addition to ERP; re-evaluation of thoughts about obsession; targets "thought-action fusion" (TAF) and hyper-responsibility cognitions.
3. Yale-Brown Obsessive-Compulsive Scale (Y-BOCS — Yale-Brown Obsessive Compulsive Scale) — Goodman (Goodman W.K.) — 10 items; 5 obsessions + 5 compulsions; gold standard; CY-BOCS in children.
4. SSRI High Dose — In OCD significantly higher dose than standard depression dose is required; titration over 8–12 weeks; in case of non-response dose is increased to maximum.
5. Clomipramine (TCA) — Evidence in OCD strong (CMI multicenter trial 1991); cardiac and anticholinergic side effects; second-line.
6. Deep Brain Stimulation (DBS) — Surgical intervention for refractory severe OCD; target ventral capsule/ventral striatum. Greenberg B.D. et al. Mol Psychiatry 2010.

8.3 Differences between sources

- NICE CG31 (2005, 2019 surveillance) — stepped approach by severity; ERP first-line; SSRIs for moderate-severe.
- APA Practice Guideline for OCD (2007) — same; clomipramine strong based on evidence, but second-line due to side effects.

- WFSBP (Bandelow B. et al. 2008) — SSRI dose ranges and titration.
- NICE CG31 PANDAS — routine antibiotic prophylaxis not recommended.

9. Prognosis

- Chronic course, but significant improvement with adequate treatment.
- Comorbid depression, tics and autism are markers of poor prognosis.
- Early onset and poor insight — more difficult treatment.

10. Myths and misconceptions

Myth 1: "OCD is just a personality trait related to cleanliness and orderliness"

Evidence: Clinical OCD is ego-dystonic, causes distress and functional impairment; obsessive-compulsive personality disorder (OCPD) is ego-syntonic distinct disorder. Clinical diagnosis based on Y-BOCS severity and duration.

Myth 2: "The patient can 'voluntarily stop' obsessions"

Evidence: OCD is dysregulation of CSTC circuit; not voluntarily controlled; ERP and SSRIs affect it at a neurobiological level.

Myth 3: "SSRIs work in OCD at depression doses"

Evidence: In OCD, SSRIs require high doses (fluoxetine 60–80 mg, sertraline 200 mg); depression dose (e.g., fluoxetine 20 mg) often does not respond.

Myth 4: "ERP causes trauma because it keeps the patient in a feared situation"

Evidence: ERP is structured, stepwise, and conducted with patient consent; demonstrates long-term proven efficacy.

Myth 5: "PANDAS is the main cause of OCD, everyone should take antibiotics"

Evidence: NIMH 2017 — PANDAS is a rare event; routine antistreptolysin O tests and antibiotic prophylaxis are not recommended; only if a classic clinical picture is present.

Myth 6: "Allowing compulsions to be performed calms symptoms"

Evidence: Permission (accommodation) for compulsions strengthens OCD; reducing family accommodation is a crucial treatment component.

Myth 7: “Herbal preparations (St John's Wort, kava, omega-3) cure OCD”

Evidence: none have proven efficacy for OCD; St John's Wort carries risk of interaction with SSRIs.

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ICD-11 **6B21**	**6B21 BODY DYSMORPHIC DISORDER (BDD)** **Body dysmorphic disorder**
ICD-10 F45.2	Hypochondriacal disorder
DSM-5-TR F45.22	Body Dysmorphic Disorder

1. Definition and nosology

Body Dysmorphic Disorder (BDD; ICD-11: 6B21; DSM-5-TR: F45.22) — obsessive preoccupation with an imagined or very slight defect in appearance; this preoccupation leads to repetitive behaviors (mirror checking, skin picking, seeking cosmetic procedures) or mental acts (comparing).

2. History

- Morselli E. (1891) — ‘dysmorphophobia’.
- DSM-III-R (1987) — ‘Body Dysmorphic Disorder’ within somatoform disorders.
- DSM-5 (2013) and ICD-11 — moved to the obsessive-compulsive spectrum category.

3. Epidemiology

- Prevalence in general population: 1.7–2.4% (Buhlmann U. et al. Psychiatry Res 2010).
- 9–15% in cosmetic dermatology and plastic surgery clinics.
- Sex: approximately equal.
- Onset: mean 15–16 years.
- Comorbidity: MDD ~75%, OCD ~30%, social anxiety ~37%, substance use.
- Suicide risk high: attempt ~24–28%, completion rate 45 times higher than population (based on reviews by Phillips K.A.).

4. Aetiology and pathogenesis

- Genetic overlap with OCD.
- Neurobiological — orbitofrontal-striatal circuit hyperactivity (similar to OCD); impaired visual processing (Feusner J.D. et al. reviews — patient overfocuses on details, fails to integrate the global image).
- Social-cultural — idealized appearance in mass media.
- Childhood peer victimization (humiliation, “bullying”).

5. Clinical features

- Obsessive focus on a specific part of appearance (face, skin, hair, nose, body shape).
- No actual defect or invisible/unimportant to others.
- Repeated behaviors: mirror checking, skin picking, comparison, seeking cosmetic procedures, clothing/makeup/camouflage.
- Cognitive actions: comparing oneself to others.
- Insight often poor; sometimes at delusional level.
- “Muscle dysmorphia” subtype — obsession with “insufficient muscle development” in males; leads to anabolic steroid use.
- High social isolation, academic and occupational disruption.

6. Diagnosis

6.1 Unified diagnostic criteria

A. Preoccupation with a perceived defect or flaw in appearance; not observable or appears slight to others.

B. The patient has at any point performed a repetitive behavior (mirror checking, skin picking, excessive dressing) or mental act (comparison).

C. The preoccupation causes clinically significant distress or functional impairment.

D. There must be no body image preoccupation meeting eating disorder criteria (anorexia/bulimia).

Qualifiers: with muscle dysmorphia; insight (good / moderate — poor / absent / delusional).

6.2 Differences between sources

- DSM-5-TR / ICD-11 — in the OCD spectrum category; muscle dysmorphia qualifier.
- NICE CG31 — BDD and OCD joint recommendations.

6.3 Diagnostic algorithm

1. Clinical interview (patient often does not speak openly due to shame).
2. BDDQ (Body Dysmorphic Disorder Questionnaire) — screening.
3. BDD-YBOCS — severity scale.
4. Comorbidity (depression, OCD, social anxiety, suicide risk).
5. History of anabolic steroid use or cosmetic procedure.

6.4 Differential diagnosis

Condition	Distinguishing feature
OCD (6B20)	Obsessions are not limited to appearance.
Eating disorder (6B8x)	Weight and figure are primary; eating behavior is disrupted.
Delusional disorder (6A24)	Insight absent; often variable in BDD.
Social anxiety (6B04)	Social evaluation broad; in BDD, focus on specific appearance part.
Trichotillomania (6B25.0)	Hair pulling primary; appearance distress secondary.
Olfactory Reference Disorder (6B22)	Odour-based.

7. Examination and assessment

- BDDQ — screening (Phillips K.A.).
- BDD-YBOCS — modified OCD scale, 12 items.
- C-SSRS — high suicide risk.

8. Treatment

1. First-line psychotherapy: CBT (adapted for BDD, Wilhelm S., Phillips K.A.) — cognitive restructuring + exposure + mirror retraining + behavioral experiments. 12–22 sessions.

2. First-line pharmacotherapy: SSRIs at high dose — similar to OCD; fluoxetine (40–80 mg), escitalopram, sertraline.
3. Clomipramine — alternative.
4. Augmentation — atypical antipsychotic (especially insight at delusional level).
5. Duration of treatment is long; ≥ 1–2 years after remission.
6. Suicide risk monitoring.
7. Prevention of cosmetic procedures — patient collaboration with dermatologist/plastic surgeon; evidence: surgical procedure does not improve BDD symptoms, often creates new focus (Crerand C.E. et al. Ann Plast Surg 2010;65(1):11–16).

8.1 Treatment methods

1. CBT for BDD — Wilhelm S., Phillips K.A — Manualized 22-session protocol — psychoeducation, cognitive restructuring, mirror retraining, behavioral experiments, ritual prevention (ERP), relapse prophylaxis.
2. Body Dysmorphic Disorder Questionnaire (BDDQ) — Phillips (Phillips K.A.) — Screening.
3. BDD-YBOCS — Adaptation of Y-BOCS for BDD.
4. SSRI High Dose — Similar to OCD; response within 8–12 weeks.

8.2 Differences between sources

- NICE CG31 — CBT + high-dose SSRI first-line.
- Phillips K.A. studies — SSRIs effective in BDD; high dose, long-term.

9. Prognosis

- Chronic course.
- Significant improvement with adequate treatment; remission is possible but requires long-term maintenance.
- Suicide risk monitored.

10. Myths and misconceptions

Myth 1: “BDD patients have a ‘real defect’, cosmetic surgery helps”

Evidence: Crerand C.E. Ann Plast Surg 2010 — cosmetic procedures do not improve BDD; most patients are dissatisfied with the result or develop a new focus. Surgeons are recommended to screen for BDD.

Myth 2: "A BDD patient should be 'reassured' by complimenting their appearance"

Evidence: praise or reassurance gives short-term relief but strengthens the obsession; reducing family accommodation is a treatment component.

Myth 3: "BDD is common 'self-doubt'"

Evidence: BDD is accompanied by functional impairment and high suicide risk (attempts 24–28%, 45 times higher than the general population); it is assessed as a clinical condition.

Myth 4: "SSRIs are effective for BDD at depression doses"

Evidence: Similar to OCD — high dose required; response in 8–12 weeks.

Myth 5: "Muscle dysmorphia does not occur in men, it's just fitness motivation"

Evidence: Muscle dysmorphia is prevalent among men; it is accompanied by the use of anabolic steroids, compulsive exercise, social isolation, and depression; it is a qualifier in DSM-5-TR.

Myth 6: "Seeking sequential procedures from a plastic surgeon is normal"

Evidence: Sequential procedures are a sign of BDD; surgeon should screen for BDD and refer to psychiatry.

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ICD-11 **6B22**	**6B22 OLFACTORY REFERENCE DISORDER (ORS)** **Olfactory reference disorder**
ICD-10 F45.8	Other somatoform disorders
DSM-5-TR F42.8	Other Specified Obsessive-Compulsive and Related Disorder (Olfactory Reference Syndrome)

1. Definition and nosology

Olfactory Reference Disorder (ORS) (ICD-11: 6B22; DSM-5-TR: F42.8 Other Specified Obsessive-Compulsive and Related Disorder) — disorder characterized by obsessive belief about a non-existent or grossly over-appraised unpleasant odour emanating from the patient's body. Patient believes others condemn or embarrass him/her, exhibits compulsive washing, deodorant use, social avoidance behavior.

2. History

- Pryse-Phillips W. (1971) — term ‘olfactory reference syndrome’.
- DSM-IV — No separate category (Delusional Disorder as a somatic subtype).
- DSM-5 (2013) — OCD spectrum noted as a related disorder; but no separate code assigned.
- ICD-11 (2019) — first formal diagnostic category (6B22).

3. Epidemiology

- Prevalence: precise epidemiological data limited; based on clinical and review reports, considered rare but underdiagnosed (Phillips K.A., Menard W. Gen Hosp Psychiatry 2011).
- Onset: early adulthood (mean 20–25 years).

- Sex: approximately equal.
- Comorbidity: MDD, social anxiety, OCD, BDD, high suicide risk (suicidal ideation ~68%, attempts ~32% — Phillips K.A., Menard W. Gen Hosp Psychiatry 2011, n=20).

4. Aetiology and pathogenesis

- Genetic and neurobiological overlap with OCD and BDD.
- Insight is variable — sometimes at delusional level.
- Social-cultural factors — social stigma regarding body odor.

5. Clinical features

- Obsessive preoccupation with body odor (mouth, feet, armpits, genitals, general body) or breath odor.
- ‘Misreading’ others' reactions (referential ideas) — coughing, scratching nose, opening a window interpreted as ‘result of smell’.
- Compulsive behaviors — frequent washing, changing clothes, deodorant, perfume, oral hygiene, gum chewing, vigorous tooth brushing.
- Social avoidance — avoidance of work and school, social isolation.
- Compensatory behaviors — maintaining distance from others.

6. Diagnosis

6.1 Unified diagnostic criteria (ICD-11)

A. Persistent preoccupation that one's body emits an unpleasant odor or that there is a foul odor in the breath — which is either unnoticeable or minor to others.

B. The patient performs repetitive behaviors (washing, odor masking, skin checking) or mental acts (self-comparison with others) in response to obsession.

C. The preoccupation causes clinically significant distress or functional impairment.

D. Exclusion of substance or medical condition (real halitosis, hyperhidrosis, trimethylaminuria, metabolic diseases of odor substances).

Insight qualifier: good / moderate — poor / absent level.

6.2 Differences between sources

- ICD-11 — official status as a separate category.

- DSM-5-TR — within ‘Other specified obsessive-compulsive and related disorder’.

6.3 Diagnostic algorithm

1. Clinical interview.
2. Medical and otolaryngological examination — excluding genuine halitosis, sinusitis, hyperhidrosis, gastrointestinal diseases.
3. Trimethylaminuria (TMAU) — urine trimethylamine test (rare genetic metabolic disorder, ‘fish odor syndrome’).
4. Insight level assessment.
5. Comorbidity and suicide risk (high!).

6.4 Differential diagnosis

Condition	Distinguishing feature
Genuine halitosis / hyperhidrosis / TMAU	Objectively confirmed.
BDD (6B21)	Appearance-based.
OCD (6B20)	Multiple obsessions.
Delusional disorder (6A24) — somatic	Insight absent; clinical manifestations overlap.
Schizophrenia	Other psychotic symptoms.
Social anxiety (6B04)	Fear of social evaluation broad.

7. Examination and assessment

- Medical examination — dental, ENT, gastrointestinal.
- TMAU test (urinary trimethylamine).
- Scale — not specific; adapted use of Y-BOCS.
- C-SSRS (high suicide risk).

8. Treatment

1. CBT — approach tailored to OCD/BDD; exposure (cessation of checking behavior, return to social situations), cognitive restructuring.
2. SSRI high dose — similar to OCD.
3. Augmentation — atypical antipsychotic (insight at delusional level).

4. Social rehabilitation.
5. Suicide risk monitoring.

8.1 Treatment methods

1. CBT (adapted for OCD/BDD) — Exposure + Response Prevention + Cognitive Restructuring.
2. SSRI High Dose — Similar to OCD; 8–12 weeks.

9. Prognosis

Evidence base is limited. Improvement with SSRIs + CBT; suicide risk monitored.

10. Myths and misconceptions

Myth 1: "The patient may genuinely smell bad — a psychiatric diagnosis cannot be made without prior medical examination"

Evidence: correct — medical causes (halitosis, hyperhidrosis, TMAU) must be definitively excluded; but if no medical cause and obsessive preoccupation persists, clinical condition is olfactory reference disorder.

Myth 2: "Frequent washing and deodorant use will solve the problem"

Evidence: Compulsive behaviors provide short-term relief but intensify the disorder; skin irritation, social impairment increase.

Myth 3: "The patient has a simple self-esteem problem"

Evidence: Clinical impairment; suicide risk is high (suicidal ideation ~68%, attempts ~32%); medical intervention required.

Myth 4: "Only an antipsychotic is effective because it is a delusion"

Evidence: insight variable; SSRI high dose + CBT first line; antipsychotic augmentation only at delusional level.

11. Sources

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ICD-11 **6B23**	**6B23** **HYPOCHONDRIASIS** **Hypochondriasis**
ICD-10 F45.2	Hypochondriacal disorder
DSM-5-TR F45.21	Illness Anxiety Disorder

1. Definition and nosology

Hypochondriasis (ICD-11: 6B23; DSM-5-TR: F45.21 Illness Anxiety Disorder — new name, separated from somatic symptom disorder) — persistent preoccupation and anxiety about having or developing a serious medical illness. The patient either checks repeatedly — clinic visits, internet searches, advice from friends — or, conversely, avoids medical care altogether.

DSM-5 (2013) — hypochondriasis diagnosis split into two new categories:

- Somatic Symptom Disorder (F45.1) — somatic complaints present;
- Illness Anxiety Disorder (F45.21) — somatic symptoms minimal or absent, anxiety dominant.

ICD-11 (2019) — hypochondriasis included within the OCD spectrum (separated from somatoform disorders) — this conceptual transition reflects the evidence base (shared neurobiology and treatment response similar to OCD).

2. History

- Greek medicine — “hypochondrium” (below the ribs) — ancient humoral theory.
- DSM-III (1980) — In the category of somatoform disorders.
- DSM-5 (2013) — separation of illness anxiety disorder and somatic symptom disorder.
- ICD-11 (2019) — transition to the spectrum category of OCD.

3. Epidemiology

- Lifetime prevalence: 1–6% (Bleichhardt G., Hiller W. Br J Health Psychol 2007).
- In primary care patients: ~3–10%.
- Sex: approximately equal.
- Onset: early to middle adulthood.
- Comorbidity: MDD, GAD, OCD, panic disorder.

4. Aetiology and pathogenesis

- Genetic overlap with OCD and anxiety disorders.
- Experience with childhood trauma or medical illnesses.
- Cognitive — catastrophizing of bodily sensations; selective attention; “medical information seeking” (cyberchondria).
- Modeling — high familial vulnerability to illness.

5. Clinical features

- Fear of developing a serious illness (cancer, heart attack, various chronic diseases).
- Interpretation of normal bodily sensations (palpitations, headache, sluggish bowel) as markers of serious illness.
- Behavioral models (two polar):
    - Care-seeking — repeated medical check-ups, multiple doctor consultations (‘doctor shopping’), internet searches (cyberchondria), body checks.
    - Care-avoiding — avoidance of medical check-ups (fear of hearing proof), avoidance of hospitals, medical equipment.
- Medical reassurance provides temporary relief, then anxiety returns.
- Functional impairment — absenteeism from work, social isolation.

6. Diagnosis

6.1 Unified diagnostic criteria

A. Preoccupation with having or developing a serious illness.

B. Somatic symptoms minimal or absent; if present, preoccupation disproportionate to severity.

C. High anxiety about health.

D. The patient exhibits excessive health-related behaviors (checking) or illness avoidance.

E. Duration ≥ 6 months (DSM-5-TR); typical content of the illness may change over time.

F. Not better explained by another mental disorder (MDD, panic, GAD, OCD, BDD, somatic delusions).

6.2 Differences between sources

- DSM-5-TR — Illness Anxiety Disorder (F45.21); care-seeking and care-avoiding subtypes.
- ICD-11 — within the OCD spectrum category (6B23); new placement distinct from classic somatoform location.

6.3 Diagnostic algorithm

1. Clinical interview.
2. Medical evaluation — exclusion of genuine illness (however excessive testing should be avoided).
3. Scale — Whiteley Index, SHAI (Short Health Anxiety Inventory).
4. Comorbidity screening.

6.4 Differential diagnosis

Condition	Distinguishing feature
Somatic symptom disorder (6C20)	Somatic complaints dominant.
Real medical illness	Clinical and laboratory confirmation.
OCD (6B20)	Multiple obsessions; illness not restricted.
GAD (6B00)	Multiple areas of worry.
Panic (6B01)	Fear of ‘heart attack’ during an attack.
Delusional disorder (6A24) — somatic	Insight absent.
Major depressive disorder — somatic focus	Affective component predominates.

7. Examination and assessment

- Whiteley Index (14 items), SHAI.
- Medical examination — once complete, not repeated.
- PHQ-9, GAD-7 comorbidity.

8. Treatment

1. CBT first-line — cognitive restructuring (catastrophizing), prevention of body checking and reassurance seeking (exposure + ritual prevention), attention training. Effect: Cooper K. et al. Behav Res Ther 2017 meta-analysis.
2. SSRIs — for moderate-to-severe cases or comorbid depression/GAD.
3. Single physician model (single physician) — patient works only with one primary care physician; avoids multiple checks and "doctor shopping".
4. Reassurance limit — clinician does not provide excessive proof in response to anxiety; short, consistent message.
5. Mindfulness-based approach adjunct.

8.1 Treatment methods

1. CBT for hypochondriasis — Salkovskis P., Warwick H — Cognitive model — "catastrophization" of bodily sensations central. Intervention: psychoeducation, exploration of alternative explanations, behavioral experiments of body checking and reassurance seeking, exposure. 12–16 sessions.
2. Whiteley Index (Whiteley Index) — 14-item Illness Concern Scale — gold standard screening.
3. Short Health Anxiety Inventory (SHAI) – Salkovskis — 18 items; clinical and population research.
4. Single Physician Model (single physician approach) — The patient works only with one physician; all complaints are managed via that physician; multiple checks and contradictory opinions are prevented.

8.2 Differences between sources

- Cooper K. et al. Behav Res Ther 2017 meta-analysis — CBT significant effect; medium effect size.
- Greeven A. et al. Am J Psychiatry 2007 RCT — CBT and paroxetine both superior to placebo.

9. Prognosis

- CBT or SSRIs result in significant improvement in most patients.
- Chronic without treatment; persistent use of medical resources.

10. Myths and misconceptions

Myth 1: "The patient is 'faking' illness, playing games"

Evidence: hypochondriasis is a clinical psychiatric disorder; patient's concern is genuine; not 'play' or simulation. Differs from factitious disorder (6D50).

Myth 2: "The more medical tests performed, the more the patient will be reassured"

Evidence: repeated checking gives short-term relief, then anxiety returns; 'checking dependency' develops. Single clinician model and check restriction recommended.

Myth 3: "Reassurance ('you are healthy') is a treatment tool"

Evidence: repeated reassurance reinforces disorder — patient continues seeking proof. Structured clinician response: one-time assessment, then brief standard message.

Myth 4: "Internet search helps the patient's self-education"

Evidence: "Cyberchondria" — internet searches increase anxiety, enhance catastrophizing; limitation or structuring recommended.

Myth 5: "Hypochondriasis only occurs in elderly patients"

Evidence: onset early-middle adulthood; age range wide.

Myth 6: "Only antidepressants are effective; therapy is useless"

Evidence: Greeven 2007 — CBT and paroxetine both superior to placebo; CBT first-line (long-term effect comparable to SSRIs).

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ICD-11 **6B24**	**6B24 HOARDING DISORDER** **Hoarding disorder**
ICD-10 F63.8	Other habit and impulse disorders
DSM-5-TR F42.3	Hoarding Disorder

1. Definition and nosology

Hoarding disorder (ICD-11: 6B24; DSM-5-TR: F42.3 Hoarding Disorder) — a persistent need to acquire or save possessions and persistent difficulty discarding them, leading to clutter of living spaces (to the extent that rooms cannot be used for their intended purpose). Categorized under the OCD spectrum.

2. History

- Frost R.O., Hartl T.L. (1996) — cognitive-behavioral model of hoarding.
- DSM-IV — Was noted as a subtype of OCD.
- DSM-5 (2013) and ICD-11 — formally established as a distinct diagnostic category (evidence base — distinct clinical manifestations, course, treatment response).

3. Epidemiology

- Lifetime prevalence: 2–6% (Samuels J.F. et al. Behav Res Ther 2008).
- In older adults, prevalence is higher (~6%).
- Sex: approximately equal; women predominate in clinical samples.

- Onset: adolescence (mean 11–15 years); severity increases with age.
- Comorbidity: MDD ~50%, OCD ~20%, GAD, social anxiety, ADHD.

4. Aetiology and pathogenesis

- Heritability 50%.
- Neurobiological — frontostriatal circuit disruption; activation patterns distinct from OCD (Tolin D.F. et al. reviews).
- Cognitive deficits — categorization, decision-making, attention (Frost & Hartl model).
- Emotional attachment to objects; loss trauma.
- In older adults — trigger for cognitive decline.

5. Clinical features

- Persistent difficulty acquiring objects (purchasing, obtaining free, gift) or storing them.
- The thought of discarding or releasing objects causes significant distress.
- Hoarding items and occupying the living space — rooms cannot be used for their intended functional purposes (the bedroom for sleeping, the kitchen for eating, etc.).
- Sometimes “animal hoarding” — the mass keeping of animals.
- Insight often poor — patient does not accept the problem.
- Health and safety risks — fire, infections, falls, social isolation, eviction.

6. Diagnosis

6.1 Unified diagnostic criteria

A. Persistent difficulty discarding or parting with acquired items, regardless of value.

B. This difficulty arises from a persistent need to keep items and distress associated with discarding.

C. Hoarding clogs the living area and significantly disrupts purposeful use of rooms.

D. Significant distress or functional impairment (including dangerous environment to self, family, others).

E. Excluding medical conditions (brain injury, dementia).

F. Not better explained by another mental disorder (OCD ‘obsessions’, MDD ‘lack of energy’, psychotic disorder delusions, autism ‘restricted interests’).

Qualifier: with excessive acquisition; insight (good / moderate — poor / absent).

6.2 Differences between sources

- DSM-5-TR / ICD-11 — separate category; insight qualifier.
- “Animal hoarding” — although not a separate specifier, it is a clinically recognized variant; collaboration with veterinary and public health authorities is required.

6.3 Diagnostic algorithm

1. Clinical interview.
2. Home visit or photographic evidence — condition of living space (Clutter Image Rating).
3. Scale — Saving Inventory-Revised (SI-R).
4. Comorbidity and cognitive assessment (dementia exclusion in elderly — MMSE/MoCA).
5. Safety and health risk assessment.

6.4 Differential diagnosis

Condition	**Distinguishing feature**
OCD (6B20)	Classic obsessions (cleanliness, symmetry); hoarding items with emotional value.
MDD (6A70/6A71)	Not cleaning due to “lack of energy”; no active acquisition of items.
Dementia	Cognitive decline dominant; later developed accumulation.
Autism spectrum (6A02)	Limited interests and attachment to objects; social-communicative deficit also present.
Schizophrenia / delusion	Delusion content (danger, meaning).
Diogenes syndrome (in the elderly)	Self-care impairment + hoarding; social isolation; dementia comorbid is common.

7. Examination and assessment

- SI-R (Saving Inventory-Revised), CIR (Clutter Image Rating).
- Medical and cognitive examination (MMSE/MoCA for elderly).
- Safety assessment.

8. Treatment

1. CBT for hoarding (Frost R.O., Tolin D.F., Steketee G.) — first-line; manualized 26-session protocol — motivation enhancement, cognitive restructuring, decision-making training, sorting and discarding exercises, home visits. Evidence: Tolin D.F. et al. Depress Anxiety 2015 meta-analysis (12 samples, n=232) — symptoms decrease significantly, yet over half of patients still have difficulties after treatment.
2. SSRIs — evidence base is limited; SSRIs effective in OCD give a weaker response in hoarding. Adjunctive role.
3. Cognitive remediation — attention, categorization, decision-making training.
4. Buoyancy approach — supporting the patient's internal motivation; risk of re-cluttering after a forced clear-out.
5. Community intervention — public health, social service, fire safety.
6. In older adults — dementia screening and comorbid intervention.

8.1 Treatment methods

1. CBT for hoarding disorder (Hoarding) — Steketee (Steketee G.), Frost (Frost R.O.) — Manualized 26 sessions — motivational interview, decision-making training, sorting exercise, home visits, cognitive restructuring. IOCDF.
2. Saving Inventory-Revised (SI-R) — Frost (Frost R.O.) — 23 items; three subscales (acquisition, difficulty discarding, clutter).
3. Clutter Image Rating (CIR) — 9-point scale; photo-based assessment of room clutter level.
4. Group CBT — Cost-effective format; long-term support.

8.2 Differences between sources

- Tolin D.F. et al. RCTs — CBT is effective; SSRIs limited.
- Saxena S. reviews — response to SSRIs is weaker than that observed in OCD.

9. Prognosis

- Chronic course, severity increases with age.
- Significant improvement with CBT, but full remission rare.
- Comorbid dementia indicates a poor prognosis.

10. Myths and misconceptions

Myth 1: “Hoarding is just a ‘collector’ personality trait or laziness”

Evidence: Hoarding is a clinical disorder — Frost & Hartl cognitive-behavioral model; distinct neurobiology model; functional impairment and health risk. Unlike collectionism, items are not systematized and congest the living area.

Myth 2: “Forcibly cleaning the patient's home ends the problem”

Evidence: A forced clear-out without the patient's consent leads to severe distress, trauma, and re-cluttering within 12 months. Gradual sorting involving patient participation is recommended.

Myth 3: “Hoarding is a subtype of typical OCD; SSRIs are sufficient”

Evidence: DSM-5 and ICD-11 — separate category; response to SSRIs is poor compared to OCD; CBT manualized protocol is first-line.

Myth 4: “Keeping many animals ‘with good intentions’ is good for the animals”

Evidence: “Animal hoarding” — results in animals receiving poor hygiene, food, and medical care; high animal mortality, survivors in severe condition; requires intervention from veterinary and public health authorities.

Myth 5: “Hoarding in the elderly is ‘normal aging’”

Evidence: In elderly individuals, this may be a marker for frontotemporal dementia in particular; cognitive assessment is required.

Myth 6: “Once cleaned, the problem is solved”

Evidence: Hoarding is a chronic disorder; long-term treatment and community support are required.

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ICD-11 **6B25**	**6B25 BODY-FOCUSED REPETITIVE BEHAVIOUR DISORDERS** **Body-focused repetitive behaviour disorders**
ICD-10 F63.3	Trichotillomania
DSM-5-TR F63.3 / F42.4	Trichotillomania (Hair-Pulling Disorder)

1. Definition and nosology

Body-focused repetitive behaviour disorders (ICD-11: 6B25) — a group of disorders characterized by the patient's repetitive interference with their own body (hair, skin, nails). Main subtypes:

- 6B25.0 Trichotillomania (hair-pulling disorder);
- 6B25.1 — Excoriation / Dermatillomania (skin-picking disorder);
- 6B25.Y Other specified (nail biting — onychophagia, lip biting, cheek biting).

DSM-5-TR — Trichotillomania (F63.3) and Excoriation (L98.1) coded separately, but within the OCD spectrum category.

2. History

- Hallopeau F.H. (1889) — term “trichotillomania”.
- DSM-III-R (1987) — Trichotillomania as an impulse control disorder.
- DSM-5 (2013) — moved to the OCD spectrum category; excoriation added as a separate diagnosis.
- ICD-11 (2019) — as a joint category (6B25 umbrella).

3. Epidemiology

- Lifetime prevalence of trichotillomania: 0.5–2%; in clinical form, females 4:1.
- Excoriation: 1.4–5.4%; higher in females.
- Onset: adolescence (10–13 years).
- Comorbidity: MDD, anxiety, OCD, BDD, ADHD.

4. Aetiology and pathogenesis

- Heritability 30–40%; family history prevalent.
- Genetic — SAPAP3 (SLITRK1) — hypergrooming phenotype in mice.
- Neurobiological — OCD-like cortico-striato-thalamo-cortical dysregulation, but with a different profile.
- Behavioral mechanisms: stress response, ‘automatic’ (unconscious) and ‘focused’ (purposeful, with intense attention) subtypes.

5. Clinical features

Trichotillomania (6B25.0)

- Hair pulling — usually from head, eyebrows, eyelashes; sometimes from other body parts.
- Visible hair loss.
- “Pre-pulling tension” or “post-pulling relief” — may not be present in all patients.
- Trichophagia (eating of pulled hair) — in 10–20% of patients; ‘trichobezoar’ — gastrointestinal obstruction may require surgery.

Excoriation / Dermatillomania (6B25.1)

- Skin picking — face, arm, damaged skin areas (“acne”, insect bite site).

- Injury, crust, scars.
- Infection, scarring, cosmetic outcomes.

General characteristics

- The patient makes repeated unsuccessful attempts to stop the behavior.
- Social isolation due to shame (wearing a wig, covering mouth/hands).

6. Diagnosis

6.1 Unified diagnostic criteria

A. Hair pulling (in trichotillomania) or skin picking (in excoriation) — visible hair loss or skin damage.

B. The patient makes repeated attempts to stop or reduce the behavior.

C. Behavior causes clinically significant distress or functional impairment

D. Exclusion of medical condition (alopecia areata, dermatological illness) or another mental disorder (e.g., appearance-motivated behavior in BDD; delusional content in a psychotic episode; substance-induced).

6.2 Differences between sources

- DSM-5-TR — Trichotillomania (F63.3) and Excoriation (L98.1) separate.
- ICD-11 — under the common 6B25 umbrella; subspecifiers.

6.3 Diagnostic algorithm

1. Clinical interview (patient often does not speak openly due to shame).
2. Dermatological examination — exclusion of other causes of hair loss and skin lesions (alopecia areata, tinea capitis, eczema).
3. Scale — Massachusetts General Hospital Hairpulling Scale (MGH-HPS), Skin Picking Scale.
4. Comorbidity screening.
5. Trichophagia screening — if gastrointestinal symptoms present.

6.4 Differential diagnosis

Condition	Distinguishing feature
Alopecia areata	Autoimmune; clear borders, “exclamation mark hairs”; no hair-pulling behavior.

7. Examination and assessment

- MGH-HPS (trichotillomania), Skin Picking Scale-Revised.
- Dermatological examination.
- Abdominal ultrasound in case of trichophagia.

8. Treatment

1. Habit Reversal Training (HRT) first-line — awareness training, competing response (e.g., fist clenching), motivation, social support. Evidence: Bloch M.H. et al. Biol Psychiatry 2007 meta-analysis.
2. ACT (Acceptance and Commitment Therapy) — targets emotional component.
3. Comprehensive Behavioral Treatment (ComB, Mansueto) — HRT + multimodal behavioral intervention.
4. N-acetylcysteine (NAC) — evidence base developing; Grant J.E. et al. Arch Gen Psychiatry 2009 RCT — efficacy in trichotillomania.
5. SSRI — evidence base is limited; SSRIs effective in OCD give a weak response in trichotillomania; clomipramine is relatively superior.
6. Atypical antipsychotic adjunct — in refractory cases.
7. Dermatological care — management of injuries and infections.

8.1 Treatment methods

1. Habit Reversal Training (HRT — Habit Reversal Training) — Azrin (Azrin N.H.), Nunn (Nunn R.G.) — Five core components — awareness training, competing response, motivation, social support, generalization. Gold standard in body-focused repetitive behaviors.
2. Comprehensive Behavioral Treatment (ComB) — Mansueto (Mansueto C.S.) — SCAMP model — individualized approach based on Sensor, Cognitive, Affective, Motor, Place modalities.
3. Acceptance and Commitment Therapy (ACT) — Focus on the emotional component; Woods D.W. et al. Behav Res Ther 2006 RCT.
4. NAC — N-acetylcysteine — Glutamatergic modulator; 1200–2400 mg/day; Grant J.E. Arch Gen Psychiatry 2009.
5. Massachusetts General Hospital Hairpulling Scale (MGH-HPS) — 7 items; frequency, intensity.

8.2 Differences between sources

- Bloch M.H. Biol Psychiatry 2007 meta-analysis — HRT most evidence-based for trichotillomania.
- Grant J.E. Arch Gen Psychiatry 2009 — NAC 1200–2400 mg/day efficacy.

9. Prognosis

- Chronic course, “waxing and waning”.
- Significant improvement with HRT; treatment of comorbidity is essential.

10. Myths and misconceptions

Myth 1: “Hair pulling and skin picking are ‘bad habits’, not serious disorders”

Evidence: Clinical impairment; risks of functional impairment, social isolation, infection, and trichobezoar (surgical).

Myth 2: “The patient can stop voluntarily”

Evidence: The patient repeatedly tries and fails — this is the main symptom of the disorder; HRT or pharmacotherapy is required.

Myth 3: “SSRIs are effective for trichotillomania (like OCD)”

Evidence: Response to SSRIs is weaker than in OCD; HRT is first-line; NAC adjunct; clomipramine relatively superior over SSRIs.

Myth 4: “Hiding hair (wig, makeup) is the solution”

Evidence: Cosmetic camouflage short-term; primary behavioral intervention required.

Myth 5: “Only hypnosis or deep psychoanalytic therapy resolves the underlying behavioral cause”

Evidence: Evidence base is limited; HRT-based behavioral approaches are superior.

Myth 6: “Skin picking is a response to ‘acne’ or a cosmetic problem”

Evidence: Clinical impairment — patient cannot prevent behavior, creates scar and infection on skin; clinical intervention required.

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6B40–6B4Z STRESS-RELATED

Disorders specifically associated with stress

ICD-11 **6B40**	**6B40 POST TRAUMATIC STRESS DISORDER (PTSD)** **Post traumatic stress disorder**
ICD-10 F43.1	Post-traumatic stress disorder
DSM-5-TR F43.10	Posttraumatic Stress Disorder

1. Definition and nosology

Posttraumatic Stress Disorder (PTSD; ICD-11: 6B40; DSM-5-TR: F43.10) — a disorder developing after direct or witnessed exposure to actual or threatened death, serious injury, or sexual violence, characterized by three main symptom domains: Re-experiencing (intrusions), Avoidance, persistent apprehension (ICD-11) / Cognitive-emotional changes and hyperarousal (DSM-5-TR).

2. History

- “Shell shock” (World War I), “combat fatigue” (World War II).
- DSM-III (1980) — “Posttraumatic Stress Disorder” — based on Vietnam veteran studies.
- DSM-5 (2013) — separated from anxiety disorders and moved to the “Trauma- and Stressor-Related Disorders” category; Criterion A narrowed (only specific trauma types).
- ICD-11 (2019) — a more compact diagnostic concept; Complex PTSD (6B41) official recognition as a distinct category.

3. Epidemiology

- Lifetime prevalence: 3.9% (WHO World Mental Health), in USA ~6.8%.
- Sex: 2 times higher in females (due to risk of sexual violence exposure).

- Risk by trauma type: sexual violence ~50% PTSD, physical assault ~25%, accident ~10%, natural disaster ~5%.
- Comorbidity: MDD ~50%, GAD, panic, substance use, BPD, complex PTSD.
- Suicide risk significantly high.

4. Aetiology and pathogenesis

- Heritability 30–40%.
- Neurobiological — amygdala hyperreactivity, impaired prefrontal-amygdala regulation, hippocampal volume reduction, HPA axis dysregulation (paradoxically low cortisol).
- Trauma characteristics — severity, duration, perpetrated by others (intentional), occurrence in childhood.
- Risk factors — previous trauma, previous psychiatric history, lack of social support, female gender, peritraumatic dissociation.
- Protective – social support, post-traumatic meaning seeking.

5. Clinical features

Three main domains (ICD-11)

- Re-experiencing — intrusive memories, flashbacks, nightmares; acute distress and somatic reactions with triggers.
- Avoidance — avoidance of thoughts, feelings, reminders, situations related to trauma.
- Persistent sense of threat (hyperarousal) — hypervigilance, easily startled, sleep disturbance, irritability.

DSM-5-TR additional domain — cognitive-mood changes

- Negative beliefs (about self, others, the world).
- Anhedonia, social isolation.
- Numbing of feelings, persistent negative mood.

Duration

Symptoms ≥ 1 month; the onset may be delayed after the trauma (delayed onset > 6 months).

6. Diagnosis

6.1 Unified diagnostic criteria

A. Exposure to traumatic event (personal, witnessing, information about death/injury of close ones, repeated exposure to traumatic information in occupational context).

B. Re-experiencing (intrusive memories, flashbacks, nightmares).

C. Avoidance of trauma reminders.

D. ICD-11: persistent sense of threat; DSM-5-TR: cognitive-mood alterations + hyperarousal.

E. Duration ≥ 1 month.

F. Significant distress or functional impairment.

G. Exclusion of substance or medical condition.

6.2 Differences between sources

- DSM-5-TR: 4 symptom domains (intrusions, avoidance, cognitive-mood, hyperarousal); dissociative subtype qualifier (depersonalization/derealization); delayed onset.
- ICD-11 (6B40): 3 symptom domains (more compact); complex PTSD as a separate category (6B41).
- NICE NG116 (2018, Post-traumatic stress disorder): Diagnosis Structured Clinical Interview (CAPS-5).
- VA/DoD 2023: Gold standard diagnosis CAPS-5; PCL-5 screening in primary medical care.

6.3 Diagnostic algorithm

1. Clinical interview + trauma history.
2. CAPS-5 (Clinician-Administered PTSD Scale) — gold standard structured interview.
3. PCL-5 (PTSD Checklist) — self-assessment screening, 20 items.
4. Comorbidity (MDD, GAD, substance, BPD, complex PTSD).
5. Suicide risk (C-SSRS).
6. Except for medical and traumatic brain injury (TBI).

6.4 Differential diagnosis

Condition	Distinguishing feature
Acute stress disorder (similar to 6B40, DSM-5: ASD)	Between 3 days and 1 month.
Complex PTSD (6B41)	Self-organization disturbance (affect, self, relationships).
Adjustment disorder (6B43)	Does not meet trauma criterion A.
MDD (6A70/6A71)	Affective symptoms; no intrusions and avoidance.
Panic disorder (6B01)	Attack is unexpected.
BPD (6D10.x)	Persistent personality pattern; impulsivity and relationships.
Dissociative disorders (6B6x)	Dissociation dominant; dissociative subtype may be a qualifier in PTSD.
Traumatic brain injury (TBI)	Neurological findings; cognitive deficit.

7. Examination and assessment

- CAPS-5 — gold standard.
- PCL-5 (20 items) — screening.
- PHQ-9, GAD-7, C-SSRS — comorbidity.
- Trauma history (LEC-5 — Life Events Checklist).

8. Treatment

8.1 General Principles (VA/DoD 2023 · APA 2017 · NICE NG116 Consensus)

1. First-line: trauma-focused psychotherapy — individual format:
    - Prolonged Exposure (PE, Foa E.B.) — in vivo and imaginal exposure to traumatic memory.
    - Cognitive Processing Therapy (CPT, Resick P.A.) — cognitive restructuring; 12 sessions.
    - EMDR (Eye Movement Desensitization and Reprocessing, Shapiro F.) — bilateral stimulation + traumatic memory processing.

    - Trauma-Focused CBT (TF-CBT, Cohen J.) — in children and adolescents.
2. First-line pharmacotherapy: SSRI (sertraline and paroxetine — FDA approval for PTSD), SNRI (venlafaxine).
3. Second-line: mirtazapine, nefazodone (risk of hepatotoxicity — use with caution).
4. For nightmares — prazosin (α1 antagonist) — evidence base is controversial (Raskind 2003 positive, 2018 PACT trial neutral); individual response.
5. Adjunct — atypical antipsychotic (in refractory cases); benzodiazepines are contraindicated long-term (worsens PTSD course).
6. ECT — in refractory severe comorbid depression.
7. Treatment duration — psychotherapy 12–16 sessions; pharmacotherapy ≥ 12 months after remission.

8.2 Treatment methods

1. Prolonged Exposure (PE) — Foa (Foa E.B.) — Imaginal exposure to traumatic memory (repeated, sequential description) + in vivo exposure (to avoided situations). 10–15 sessions. Evidence: Foa E.B. et al. RCTs; gold-standard effect in meta-analyses.
2. Cognitive Processing Therapy (CPT) — Resick (Resick P.A.) — Cognitive restructuring — "stuck points" about trauma (safety, trust, power, self-esteem, intimacy). 12 sessions. Resick P.A. et al. RCTs.
3. Eye Movement Desensitization and Reprocessing (EMDR) — Shapiro (Shapiro F.) — Reprocessing traumatic memory with bilateral stimulation (eye movement, auditory, or tactile). 8-phase protocol. Evidence: Bisson J.I. et al. Cochrane 2013 — comparable effect to CBT.
4. Trauma-Focused Cognitive Behavioral Therapy (TF-CBT) — Cohen J., Mannarino A — for children and adolescents aged 3–18 years; PRACTICE protocol (Psychoeducation, Relaxation, Affective modulation, Cognitive coping, Trauma narrative, In vivo, Conjoint sessions, Enhancing safety).
5. Clinician-Administered PTSD Scale (CAPS-5) — 30-item structured interview; gold standard.
6. PTSD Checklist (PCL-5 — Posttraumatic Stress Disorder Checklist) — 20-item self-assessment; screening and monitoring.

8.3 Differences between sources

- VA/DoD 2023 Clinical Practice Guideline for the Management of Posttraumatic Stress Disorder and Acute Stress Disorder: trauma-focused psychotherapy first-line (strong recommendation); pharmacotherapy first-

line alternative; combination psychotherapy + pharmacotherapy not routine recommendation; benzodiazepines contraindicated.

- APA 2017 PTSD Guideline: CPT, PE, cognitive therapy and trauma-focused CBT are strongly recommended; EMDR is conditionally recommended. VA/DoD 2023 and ISTSS 2018 place EMDR in the top tier alongside CPT and PE.
- NICE NG116 (2018): Trauma-focused CBT or EMDR; SSRIs second-line.
- ISTSS 2018: international recommendations.
- Critical Incident Stress Debriefing (CISD) — Cochrane (Rose S. et al. 2002) and NICE do not explicitly recommend; some studies show an increased PTSD risk.

9. Prognosis

- Trauma-focused psychotherapy yields significant improvement in most patients.
- Chronicity without early intervention.
- Comorbid MDD, substance use poor prognosis markers.

10. Myths and misconceptions

Myth 1: "PTSD only occurs in war veterans"

Evidence: PTSD can develop following any traumatic event — sexual violence (~50% prevalence), physical assault, accidents, natural disasters, medical procedures. Prevalence among veterans is high, but it is also widespread in the general population.

Myth 2: "CISD (Critical Incident Stress Debriefing) prevents PTSD after trauma"

Evidence: Cochrane (Rose S. et al. 2002) — CISD does not reduce PTSD risk, and in some studies increases it. NICE and VA/DoD do not explicitly recommend. Early intervention as "watchful waiting" and trauma-focused CBT only for symptomatic patients.

Myth 3: "The patient should not talk about trauma, 'disclosure' is harmful"

Evidence: Evidenced trauma-focused therapies (PE, CPT, EMDR) work specifically with traumatic memory; effective in appropriate structured form. Avoidance strengthens PTSD.

Myth 4: “Benzodiazepines are a good treatment for PTSD”

Evidence: VA/DoD 2023, APA, NICE — Benzodiazepines contraindicated; worsen PTSD course, impair cognitive processing, high addiction potential.

Myth 5: “Forgetting traumatic memories is the main goal”

Evidence: The goal is not forgetting but functioning with memory — emotional reaction decreases, but memory remains. The patient can talk about trauma without distress.

Myth 6: “Cannabis (CBD) cures PTSD”

Evidence: VA/DoD — Cannabis not recommended; preliminary research exists, but insufficient for clinical guidance; THC may worsen PTSD course.

Myth 7: “MDMA-based therapy is an official treatment”

Evidence: MDMA-assisted psychotherapy is in research phase; FDA 2024 approval request denied (further research required); not standard clinical practice.

Myth 8: “EMDR has weak evidence — just a placebo effect”

Evidence: Bisson J.I. Cochrane 2013, VA/DoD 2023 and ISTSS 2018 — EMDR is strongly recommended, with efficacy comparable to CPT and PE. APA 2017 and 2025 are the exception, rating EMDR conditional.

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ICD-11 **6B41**	**6B41 COMPLEX POST TRAUMATIC STRESS DISORDER (cPTSD)** **Complex post traumatic stress disorder**
ICD-10 F43.1	Post-traumatic stress disorder
DSM-5-TR F43.10	Posttraumatic Stress Disorder (no separate DSM-5-TR category)

1. Definition and nosology

Complex post-traumatic stress disorder (cPTSD; ICD-11: 6B41) — a disorder that develops in response to prolonged or repeated trauma from which escape is difficult or impossible (prolonged abuse, childhood physical or sexual violence, war, torture, human trafficking). It includes the three symptom domains of PTSD (re-experiencing, avoidance, persistent sense of threat). To these is added “disturbance of self-organization” (DSO): affect regulation impairment, negative self-esteem, difficulty in relationships.

In ICD-11, added for the first time as an official diagnostic category; DSM-5-TR does not have a separate cPTSD category (the PTSD dissociative subtype qualifier covers some features).

2. History

- Herman J.L. (1992) — “Complex PTSD” concept (book “Trauma and Recovery”); “Disorder of Extreme Stress Not Otherwise Specified” (DESNOS) — not included in DSM-IV.
- ICD-11 (2019) — official code 6B41; based on evidence separating it from PTSD (based on reviews by Cloitre M. et al.).

3. Epidemiology

- Prevalence: 1–8% among trauma-exposed patients in primary care (reviews by Brewin C.R. et al.).
- Particularly high in patients with a history of childhood abuse.
- Comorbidity: MDD, GAD, dissociative disorders, BPD (clinical overlap), substance use, eating disorders.

4. Aetiology and pathogenesis

- Trauma type — prolonged, repeated, perpetrated by others (interpersonal), difficult escape.
- Trauma occurring in childhood — broad impact on personality development.
- Neurobiological — similar to PTSD but with more widespread HPA dysregulation, dissociation, and impaired affect regulation.

5. Clinical features

PTSD consists of three primary domains.

- Re-experiencing (intrusions).
- Avoidance.
- Persistent sense of threat.

Self-Organization Disturbance (DSO) — core adjunct of cPTSD

- Affect Dysregulation — emotional variability, intense reactions, emotional flattening.
- Negative self-esteem — persistent feelings of shame, guilt, worthlessness.
- Difficulty in relationships — avoidance of close relationships, persistently impaired attachment.

6. Diagnosis

6.1 Unified diagnostic criteria (ICD-11)

A. Exposure to long-term or repeated trauma.

B. PTSD has three main domains.

C. Self-Organization Disturbance (DSO) three domains:

1. Affect Dysregulation;
2. Negative self-esteem;
3. Difficulty in relationships.

D. Significant functional impairment.

6.2 Differences between sources

- ICD-11 — separate category; DSO requires additional criteria.
- DSM-5-TR — no separate cPTSD category; PTSD ‘with dissociative features’ qualifier covers some characteristics.
- ISTSS 2019 Complex PTSD Guidelines — recommends phased approach.

6.3 Diagnostic algorithm

1. Clinical interview + trauma history (long-term, repetitive).
2. ITQ (International Trauma Questionnaire — Cloitre M.) — designed for differentiating PTSD and cPTSD.
3. Comorbidity (especially BPD, dissociative disorders).
4. Assessment of suicide and self-harm risk.

6.4 Differential diagnosis

Condition	**Distinguishing feature**
PTSD (6B40)	No DSO component.
BPD (6D10.x)	Clinical overlap significant; in BPD, impulsivity, identity disturbance, and chronic suicidality are dominant; in cPTSD, symptoms developed based on trauma.
Dissociative disorders (6B6x)	Dissociation predominates.

Condition	Distinguishing feature
MDD (6A70/6A71)	Affective symptoms; no trauma and no DSO.

7. Examination and assessment

- ITQ (International Trauma Questionnaire) — gold standard.
- PCL-5 + DSO additional scales.
- BPD screening (McLean Screening Instrument for BPD).
- Suicide and self-harm (C-SSRS).

8. Treatment

8.1 General principles (ISTSS 2019)

1. Phase-based approach:
    - Phase 1 — stabilization — Safety, emotional regulation skills, coping strategies, psychoeducation.
    - Phase 2 — processing of traumatic memories — Evidence-based methods for PTSD (PE, CPT, EMDR), but longer duration and more persistent.
    - Phase 3 — reintegration — Restoration of relationships, improvement of quality of life.
2. Skills Training in Affective and Interpersonal Regulation (STAIR — Cloitre M.) — a phase 1 intervention before processing traumatic memories.
3. Trauma-focused CBT and EMDR — adapted format.
4. DBT-PTSD (Bohus M.) — in comorbid BPD cases.
5. SSRI adjunct — comorbid depression and anxiety.
6. Duration of treatment is long (1–2 years or more).
7. Clinician continuity and therapeutic alliance are critically important.

8.2 Treatment methods

1. Skills Training in Affective and Interpersonal Regulation (STAIR) — Cloitre M. — Phase 1 — emotional regulation and interpersonal skill development; then processing traumatic memories via Narrative Story Telling (NST). 16–24 sessions.

2. Dialectical Behavior Therapy — PTSD (DBT-PTSD) — Bohus (Bohus M.), Linehan (Linehan M.) — DBT principles + trauma-focused components; especially in comorbid BPD cases.
3. Narrative Exposure Therapy (NET) — Schauer M., Neuner F., Elbert T — The patient narrates life history in sequence, traumatic episodes are processed; effectiveness in refugees and torture victims.
4. EMDR, CPT, PE — adapted — Within a longer, phased approach.
5. International Trauma Questionnaire (ITQ) — Cloitre M. — Specific validated tool for distinguishing PTSD and cPTSD.

8.3 Differences between sources

- ISTSS 2019 Complex PTSD Guidelines — phased approach.
- Cloitre M. et al. reviews — STAIR + Narrative Exposure Therapy effective.
- Bohus M. DBT-PTSD — RCT evidence base is developing.

9. Prognosis

- Chronic course; but significant improvement with phased approach.
- Comorbid BPD, dissociative disorders, substance use poor prognosis.

10. Myths and misconceptions

Myth 1: “Complex PTSD is just a new name for borderline personality disorder”

Evidence: Clinical overlap significant, but ICD-11 PTSD is trauma-based; BPD a separate personality disorder. ITQ and BPD scales differentiate; most cPTSD patients do not meet BPD criteria.

Myth 2: “After the trauma is ‘resolved,’ the patient normalizes”

Evidence: Long-term trauma affects personality development; ‘closure’ is insufficient; phased approach and long-term treatment are required.

Myth 3: “Standard PTSD treatment (PE, CPT) is equally effective for cPTSD”

Evidence: ISTSS 2019 — phased approach in cPTSD; the patient must first be stabilised before trauma processing (phase 1); direct exposure may pose a risk of re-traumatization.

Myth 4: "Only an antidepressant is sufficient for complex PTSD"

Evidence: Psychotherapy is first-line; pharmacotherapy in adjunct role.

Myth 5: "The patient must be made to talk about the trauma immediately"

Evidence: Trauma processing without Phase 1 stabilization can be harmful; trauma-informed care principles.

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ICD-11 **6B42**	**6B42 PROLONGED GRIEF DISORDER** **Prolonged grief disorder**
ICD-10 F43.8	Other reactions to severe stress
DSM-5-TR F43.81	Prolonged Grief Disorder

1. Definition and nosology

Prolonged grief disorder (ICD-11: 6B42; DSM-5-TR: F43.81 — added in 2022) — a disorder of grief and preoccupation lasting longer than cultural and religious expectations following the loss of a close person, causing significant functional impairment.

Difference from normal grief: duration, intensity, and functional burden; clinical diagnosis based on specific duration (ICD-11: ≥ 6 months; DSM-5-TR: ≥ 12 months adult, ≥ 6 months child) and symptom profile.

2. History

- Lindemann E. (1944) — studies on acute grief dynamics.
- Bowlby J. (1980) — attachment and loss.
- Prigerson H.G. et al. (1995–2009) — “Complicated Grief” / “Prolonged Grief” concept; development of diagnostic criteria.
- ICD-11 (2019) — first official category.
- DSM-5-TR (2022) — ‘Prolonged Grief Disorder’ added as an official diagnosis.

3. Epidemiology

- Development after close loss: 7–10% in general population, 49% in case of sudden/violent death (Lundorff M. et al. J Affect Disord 2017 meta-analysis).
- Sex: higher in women.
- Risk factors: early age loss, sudden or violent death, child loss, anxious/ambivalent attachment.
- COVID-19 pandemic — significantly increased the prevalence of prolonged grief (Eisma M.C. et al.).

4. Aetiology and pathogenesis

- Attachment theory — impairment in cognitive processing of separation from a close attachment figure.
- Neurobiological — nostalgia-related activation of the reward circuit in response to the lost attachment (O'Connor M.F. fMRI studies).
- Cognitive — “stuck” thoughts, lack of healthy interpretation.

5. Clinical features

- Persistent, intense ‘nostalgia’ or preoccupation with the lost person.
- Persistent symptoms from the following:
    - Disturbance of the sense of self (“a part of me has died”)
    - Difficulty accepting the loss;
    - Emotional pain related to loss (grief, anger, guilt);
    - Loss of meaning in life;
    - Emotional numbing;
    - Avoidance of social activities;
    - Difficulty maintaining relationships.

6. Diagnosis

6.1 Unified diagnostic criteria

A. Death of a close person (partner, parent, child, sibling, close friend).

B. Persistent and intense nostalgia or preoccupation with the lost person lasting ≥ 6 months (ICD-11) / ≥ 12 months (DSM-5-TR adult, ≥ 6 months child) after loss.

C. ≥ 3 of the following symptoms:

1. Disturbance of sense of self;
2. Difficulty accepting loss;
3. Avoidance of reminder stimuli;
4. Intense emotional pain;
5. Difficulty returning to social or other activities.
6. Emotional numbing;
7. Loss of meaning in life;
8. Feelings of loneliness and isolation.

D. Duration and intensity significantly exceeding cultural, social, or religious norms.

E. Significant functional impairment.

6.2 Differences between sources

- ICD-11 — duration ≥ 6 months.
- DSM-5-TR (2022 update) — in adults ≥ 12 months, in children ≥ 6 months; at least 1 core symptom (intense nostalgia or preoccupation) + ≥ 3 additional symptoms.

- NICE — no separate guideline for prolonged grief; general grief management principles.

6.3 Diagnostic algorithm

1. Clinical interview — loss history, symptom profile, cultural context.
2. Scale — Prolonged Grief Disorder-13 (PG-13, Prigerson H.G.).
3. Comorbidity (MDD, PTSD — differentiation in grief context essential).
4. Suicide risk (high in bereaved patients).

6.4 Differential diagnosis

Condition	Distinguishing feature
Normal grief	"Wave-like" course; social functioning is preserved; duration within cultural norms.
MDD (6A70/6A71)	Anhedonia, worthlessness, suicidal ideation directed at self; grief primarily loss-related content.
PTSD (6B40)	Trauma and intrusive symptoms; intrusive memories in prolonged grief may be positive.
Adjustment disorder (6B43)	Stress factor may not involve loss; duration shorter.

7. Examination and assessment

- PG-13 (Prolonged Grief Disorder-13, Prigerson H.G.) and PG-13-R update.
- ICG (Inventory of Complicated Grief) — 19 items.
- PHQ-9, C-SSRS comorbidity.

8. Treatment

1. First-line: Complicated Grief Treatment (CGT — Shear M.K.) — 16 sessions; psychoeducation, imaginal and situational exposure, positive memory reconstruction, future orientation. Evidence: Shear M.K. et al. JAMA 2005 RCT — superior to interpersonal therapy.
2. CBT adapted for grief — Boelen P.A. model; cognitive restructuring + exposure.

3. Pharmacotherapy: Evidence base is limited. For comorbid MDD, SSRI. Comparison of CGT + citalopram (Shear et al. JAMA Psychiatry 2016) — combination and CGT alone showed the same efficacy; citalopram alone not superior to placebo.
4. Family and group support.
5. Monitor suicide risk.

8.1 Treatment methods

1. Complicated Grief Treatment (CGT) — Shear (Shear M.K.) — 16-session manualized protocol; "loss-oriented" + "restoration-oriented" work; imaginal and situational exposure, positive memories, future orientation. Evidence: Shear M.K. et al. JAMA 2005, JAMA Psychiatry 2016 RCT.
2. Prolonged Grief Scale (PG-13) — Prigerson (Prigerson H.G.) — 13-item scale; clinical and population screening; PG-13-R update.
3. Inventory of Complicated Grief (ICG) — 19 items.

8.2 Differences between sources

- Shear M.K. reviews and RCTs — CGT gold standard.
- Wittouck C. et al. Clin Psychol Rev 2011 meta-analysis — grief-specific interventions superior to universal grief support in prolonged grief.

9. Prognosis

- Significant improvement in most patients with CGT.
- Without treatment, chronicity and development of comorbid MDD/PTSD.

10. Myths and misconceptions

Myth 1: "Grief always goes through '5 stages' — denial, anger, bargaining, depression, acceptance"

Evidence: The Kübler-Ross model is a semi-scholarly concept; lacks systematic evidence base; grief is a nonlinear, individualized process.

Myth 2: "Prolonged grief — pathologized normal sadness, does not require treatment"

Evidence: Clinical diagnosis established based on duration and functional impairment; proven interventions significant benefit.

Myth 3: “Antidepressant is first-line for prolonged grief”

Evidence: Shear JAMA Psychiatry 2016 — CBT first-line; citalopram no better than placebo; SSRIs only in comorbid MDD.

Myth 4: “Time heals all wounds — just wait”

Evidence: Most people gradually adapt, but 7–10% develop prolonged grief; untreated chronicity.

Myth 5: “Talking about grief is harmful to the patient”

Evidence: Evidenced interventions (CGT) work specifically with grief content in structured form; avoidance prolongs and strengthens grief.

Myth 6: “Social rituals (charity, commemoration) are harmful; they only distract the patient from good memories”

Evidence: Cultural and religious rituals provide support during the grieving process; cultural respect is a crucial component in clinical intervention.

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ICD-11 **6B43**	**6B43 ADJUSTMENT DISORDER** **Adjustment disorder**
ICD-10 F43.2	Adjustment disorders
DSM-5-TR F43.2x	Adjustment Disorder

1. Definition and nosology

Adjustment disorder (ICD-11: 6B43; DSM-5-TR: F43.2x) — clinically significant emotional or behavioral symptoms that have developed in response to an identifiable stressor (acute or persistent, individual or collective). Symptoms do not meet full criteria for another mental disorder (MDD, GAD, etc.).

2. History

- DSM-III (1980) — "Adjustment Disorder" as an official diagnosis.
- ICD-11 — within the stress-related disorders group; general symptom profile 'preoccupation' (preoccupation with the stress factor) + adjustment difficulty.

3. Epidemiology

- Among the most frequent psychiatric diagnoses in primary care; in clinical populations 5–20%.
- Sex and age — universal; more intense at young age and during new stress periods.
- Comorbidity: MDD, anxiety, substance use, suicide risk (especially in short-term acute form).

4. Aetiology and pathogenesis

- Stress factor — a single event (divorce, job loss, illness diagnosis, grief) or multiple/persistent stressors.

- The role of personality and coping resources — the same stress elicits different responses in different people.
- Social support — protective factor.

5. Clinical features

- Preoccupation with the stress factor — recurrent thoughts, worry.
- Difficulty adapting to new situations.
- Emotional symptoms — depressed mood, anxiety, irritability.
- Behavioral symptoms — social withdrawal, decreased work/school performance, risky behavior.

DSM-5-TR subtypes

- With depressive mood (F43.21);
- With anxiety (F43.22);
- With mixed depressive mood and anxiety (F43.23);
- With conduct disorder (F43.24);
- Emotional and behavioral mixed (F43.25);
- Unspecified (F43.20).

6. Diagnosis

6.1 Unified diagnostic criteria

A. Identifiable stressor response — onset within 3 months (DSM-5-TR) / within 1 month (ICD-11).

B. Symptoms clinically significant — distress or functional impairment disproportionate to stress.

C. Does not meet criteria for another mental disorder (MDD, GAD, PTSD, complex PTSD, prolonged grief).

D. Not normal grief (sorrow).

E. Symptoms resolve within 6 months after the stress factor (or its consequences) has terminated (DSM-5-TR).

6.2 Differences between sources

- DSM-5-TR — onset at 3 months; subtype specifiers.

- ICD-11 — onset 1 month; ‘preoccupation with the stressor’ main symptom; no subtypes, but additional anxiety qualifiers.

6.3 Diagnostic algorithm

1. Clinical interview — stressor, symptom onset, prior psychiatric history.
2. SCID-5, MINI.
3. Scale — PHQ-9, GAD-7 symptom severity.
4. MDD, GAD, PTSD, complex PTSD exclusion.
5. Suicide risk (C-SSRS) — acutely particularly high.

6.4 Differential diagnosis

Condition	Distinguishing feature
MDD (6A70/6A71)	5+ symptoms for 2 weeks, full criteria.
GAD (6B00)	≥6 months (DSM); multiple areas.
PTSD (6B40)	Traumatic event + intrusions, avoidance, hyperarousal.
Prolonged grief (6B42)	From a close loss ≥ 6 months (ICD-11) / 12 months (DSM-5-TR).
Normal grief/stress reaction	No functional impairment or context-appropriate.
Conduct Disorder (6C91)	Persistent pattern, not stress-related.

7. Examination and assessment

- PHQ-9, GAD-7, C-SSRS.
- Documentation of the stress factor.

8. Treatment

1. First-line: short-term psychotherapy — supportive, problem-solving, CBT-based brief intervention (8–12 sessions).
2. Stress management, strengthening social support, development of coping strategies.
3. Pharmacotherapy — only in severe cases or comorbid; SSRIs short-term; benzodiazepines for acute anxiety for 1–2 weeks.
4. Monitor suicide risk.

5. Removal or modification of the stress factor (if possible).

8.1 Treatment methods

1. Short-term problem-solving therapy (Problem-Solving Therapy) – Nezu (Nezu A.M.) — 4–8 sessions; concrete problem identification, evaluation of solution options, behavioral execution.
2. Supportive Psychotherapy — Validation, emotional expression, social support.
3. CBT in brief form — Cognitive restructuring + behavioral activation.
4. Stress management and relaxation — Breathing exercises, progressive muscle relaxation, mindfulness.

8.2 Differences between sources

- NICE — no separate guideline for adjustment disorder; general anxiety and depression principles.
- WHO mhGAP — supportive approach in acute stress reactions, ‘watchful waiting’ in mild cases.
- Evidence base is limited — the residual nature of the diagnosis.

9. Prognosis

- In most cases remission within 6 months.
- Some persist or transition to MDD/PTSD — follow-up recommended.

10. Myths and misconceptions

Myth 1: “Adjustment disorder is not a real diagnosis — it is simply ‘life difficulty’”

Evidence: A clinical diagnosis of adjustment disorder requires demonstrable functional impairment or marked distress disproportionate to the stressor. Suicidal risk must be assessed in every case.

Myth 2: “When the stressor passes, the diagnosis automatically resolves, no treatment needed”

Evidence: Some cases persist or transition to MDD/PTSD; short-term intervention prevents treatment transition.

Myth 3: “Antidepressant is first-line for adjustment disorder”

Evidence: Psychotherapy is first-line; pharmacotherapy only in severe or comorbid cases.

Myth 4: “Suicide risk is low in adjustment disorder”

Evidence: Acute stress reactions are accompanied by a high impulsivity suicide risk; active assessment.

Myth 5: “The patient is weak or has a ‘weak character’”

Evidence: Reaction result of interaction between stress intensity and coping resources; weakness of character explanation stigmatizing and incorrect.

11. Sources

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ICD-11 **6B44**	**6B44 REACTIVE ATTACHMENT DISORDER** **Reactive attachment disorder**
ICD-10 F94.1	Reactive attachment disorder of childhood
DSM-5-TR F94.1	Reactive Attachment Disorder

1. Definition and nosology

Reactive attachment disorder (ICD-11: 6B44; DSM-5-TR: F94.1 Reactive Attachment Disorder) — a disturbance of attachment behavior developing in early childhood due to severe psychosocial deprivation or inadequate care; the child does

not seek comfort from the caregiver when distressed, or this seeking is markedly reduced.

2. History

- Bowlby J. (1951) — maternal deprivation report (WHO).
- Spitz R.A. (1945) — “hospitalism” in institutionalized children.
- DSM-III (1980) — “Reactive Attachment Disorder of Infancy” formalized.
- DSM-5 (2013) and ICD-11 — RAD and Disinhibited Social Engagement Disorder (DSED) as separate diagnoses.
- Nelson C.A. et al. Science 2007 — Bucharest Early Intervention Project — effect of institutional deprivation and early fostering.

3. Epidemiology

- Rare in the general population; even among severely neglected children it occurs in less than 10% (DSM-5-TR); in institutionalized children the inhibited (RAD) type is ~5% (Gleason M.M. et al. JAACAP 2011, BEIP). Figures above 30% refer to disinhibited social engagement disorder (6B45), not RAD.
- Onset before age 5.
- Comorbidity: cognitive deficits, neurodevelopmental disorders, depressive symptoms.

4. Aetiology and pathogenesis

- Severe psychosocial deprivation — institutionalism, multiple placements, neglect.
- Disruption of caregiver-child interaction in the critical early period up to 5 years.
- Neurobiology of the attachment system — oxytocin, prefrontal-amygdala circuit.

5. Clinical features

- Not seeking help from caregiver or minimal attempt while distressed.
- Difficulty accepting comfort.
- Reduction of social and emotional reactions (withdrawal, irritability, sadness).
- Reduction of positive affect.

- Withdrawn, hypovigilant behavior.

6. Diagnosis

6.1 Unified diagnostic criteria

A. Persistent disturbance of attachment behavior — the child does not seek comfort from the caregiver when distressed or shows minimal response to contact.

B. Persistent emotional and social impairment (reduced positive affect, unexplained irritability, sadness, or fear).

C. Disorder developed as result of severe early care disruption (neglect, multiple placements, institutionalism).

D. Onset before age 5.

E. Not better explained by autism spectrum.

F. Child's developmental age at least 9 months (to allow attachment formation).

6.2 Differences between sources

- DSM-5-TR — RAD withdrawn form; DSED disinhibited form — separate diagnosis (6B45).
- ICD-11 — same two separate categories.

6.3 Diagnostic algorithm

1. Interview with caregiver or social worker — care history (institutionalization, placement changes, neglect).
2. Structured interview (DAI — Disturbances of Attachment Interview, Smyke A.T., Zeanah C.H.).
3. Clinical observation of child-caregiver interaction.
4. Autism spectrum exclusion (M-CHAT-R/F, ADOS).
5. Cognitive and developmental assessment.

6.4 Differential diagnosis

Condition	Distinguishing feature
Autism spectrum (6A02)	Social-communication deficit, restricted-repetitive behavior; attachment behavior variable.

Condition	Distinguishing feature
DSED (6B45)	Uninhibited social approach, inappropriate interaction with strangers.
Intellectual disability (6A00)	General cognitive deficit.
PTSD (6B40)	Concrete traumatic event; intrusions.
Depressive disorder	Affective component.

7. Examination and assessment

- DAI (Disturbances of Attachment Interview).
- Strange Situation Procedure (Ainsworth M.) — within research context.
- Developmental assessment (Bayley, Mullen).

8. Treatment

1. Provision of stable, continuous care environment — fostering, adoption; institutional environment is contraindicated.
2. Interventions focused on carer-child interaction – Attachment and Biobehavioral Catch-up (ABC, Dozier M.), Child-Parent Psychotherapy (CPP, Lieberman A.).
3. Trauma-focused approach — TF-CBT added in trauma context.
4. Comorbid disorders (depression, anxiety, ADHD) treatment.
5. Pharmacotherapy — no specific approval for RAD itself; for comorbid symptoms.
6. “Holding therapy”, “rebirthing” — dangerous, are prohibited (child deaths have been reported; APSAC 2006 statement).

8.1 Treatment methods

1. Attachment and Biobehavioral Catch-up (ABC) — Dozier M — Parent training in 10 sessions; home-based; reinforcement of ‘nurturance’ and ‘synchronous’ interaction. Evidence: Dozier M. et al. Dev Psychopathol 2008;20(3):845–859.
2. Child-Parent Psychotherapy (CPP) — Liberman (Lieberman A.) — For RAD against a background of trauma; 0–5 years; within the context of parent-child interaction.
3. Disturbances of Attachment Interview (DAI) — Smyke A.T., Zeanah C.H. — 12-item clinician-administered interview.

4. Stable care and fostering — Bucharest Early Intervention Project — significant improvement in developmental outcomes for institutionalized children with early fostering.

8.2 Differences between sources

- AACAP Practice Parameter (Zeanah C.H., Chesher T., Boris N.W. JAACAP 2016).
- APSAC (American Professional Society on the Abuse of Children) — 'holding therapy' and similar coercive methods are prohibited.
- The Bucharest Early Intervention Project (Nelson Science 2007) — early fostering significantly improves outcomes in institutionalized children.

9. Prognosis

- Significant improvement with early stable care and intervention.
- Long-term institutional deprivation — residual cognitive and emotional deficits.

10. Myths and misconceptions

Myth 1: "Holding therapy" and "rebirthing" cure RAD

Evidence: APSAC 2006 — these methods are not evidence-based, are harmful; Candace Newmaker's (2000) death during 'rebirthing' was noted. AACAP, APSAC — absolute contraindication.

Myth 2: "An adopted child will be 'fixed' with love"

Evidence: Stable loving care is important, but severe RAD requires structured intervention (ABC, CPP).

Myth 3: "RAD is a form of autism"

Evidence: Reactive attachment disorder is psychosocial deprivation-based; autism is a neurodevelopmental disorder. In stable care, reactive attachment disorder significantly improves, autism does not.

Myth 4: "The patient is 'manipulative' and requires strict discipline"

Evidence: Clinical manifestations of RAD result from impairment of the attachment system and are not manipulative; a punitive approach exacerbates symptoms.

Myth 5: "Only antipsychotics or stimulants are effective for RAD"

Evidence: There is no specific FDA-approved medication for RAD; environmental changes and psychosocial intervention are first-line.

11. Sources

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	6B45 DISINHIBITED SOCIAL ENGAGEMENT DISORDER
ICD-11 **6B45**	**Disinhibited social engagement disorder**
ICD-10 F94.2	Disinhibited attachment disorder of childhood
DSM-5-TR F94.2	Disinhibited Social Engagement Disorder

1. Definition and nosology

Disinhibited Social Engagement Disorder (ICD-11: 6B45; DSM-5-TR: F94.2 Disinhibited Social Engagement Disorder, DSED) — excessive and inappropriate friendliness toward strangers in a child, with approach or departure behavior without caution — resulting from early psychosocial deprivation (institutionalization, multiple placements).

It shares the same etiological root as RAD but presents with an opposite clinical manifestation — unrestricted approach instead of withdrawal.

2. History

- DSM-IV (1994) — ‘Reactive Attachment Disorder, Disinhibited Type’.
- DSM-5 (2013) — “Disinhibited Social Engagement Disorder” added as a separate diagnosis.
- ICD-11 (2019) — parallel.

3. Epidemiology

- Rare in the general population; in institutionalized children 20%, in foster children 10–15%.
- Onset before age 5.
- Significant reduction with early fostering (Bucharest Early Intervention Project), but unlike RAD, DSED sometimes persists even in stable care environments.

4. Aetiology and pathogenesis

- Severe psychosocial deprivation — same as RAD.
- Institutional environment with multiple caregiver rotation — formation of ‘promiscuous attachment’.

5. Clinical features

- Open approach to strangers, going with them without caution.
- Reckless physical closeness (hugging, sitting).
- Reduced checking-back with the caregiver during interactions with strangers.
- Absence of stranger danger perception.
- Academic and social impairment.

6. Diagnosis

6.1 Unified diagnostic criteria

A. Inappropriately open, friendly behavior with strangers — ≥2 of the following:

1. Reduced or no caution in approaching strangers;
2. Physical or verbal intimacy inappropriate for culture or age;
3. Reduced checking-seeking behavior for caregiver in unfamiliar place;
4. Minimal or no caution about going with strangers.

B. Behaviors are not limited to impulsivity (ADHD) — social engagement component is evident

C. Early severe care disorder — neglect, multiple placement changes, institutionalism.

D. Child's developmental age ≥ 9 months.

6.2 Differences between sources

- DSM-5-TR / ICD-11 — same criteria; DSED separate from RAD.
- Developmental level by age must be considered — normal social interest in young children should not be confused with DSED.

6.3 Diagnostic algorithm

1. Interview and observation — child's interaction with strangers.
2. Care history.

3. DAI (Disturbances of Attachment Interview).
4. Comorbidity screening (ADHD, autism).

6.4 Differential diagnosis

Condition	Distinguishing feature
RAD (6B44)	Withdrawal, no attachment; open approach in DSED.
ADHD (6A05)	Impulsivity, but no social engagement component.
Autism spectrum (6A02)	Social-communicative deficit.
Williams syndrome	Genetic; "hypersociability" + specific physical features.

7. Examination and assessment

- DAI.
- Observation in unfamiliar environment.
- Williams syndrome genetic test (FISH 7q11.23) — if suspected.

8. Treatment

1. Stable care environment.
2. Parent training — developing a "safe base" concept for the child; ABC programs.
3. Teaching appropriate distance with strangers.
4. Comorbidity treatment (ADHD).
5. Community and school staff coordination — child safety.
6. There is no specific pharmacotherapy; it is for comorbid symptoms.

8.1 Treatment methods

1. Attachment and Biobehavioral Catch-up (ABC) — Dozier M — Also used for RAD; in DSED strengthening caregiver-child synchronization.
2. Safety training — Teaching child appropriate distance from strangers; reinforcement of caregiver-checking behavior.
3. DAI — In differentiation between RAD and DSED.

8.2 Differences between sources

- AACAP 2016 — same guidance as for RAD.
- DSED, unlike RAD, can persist even in a stable environment — long-term follow-up.

9. Prognosis

- With stable care and intervention, improvement occurs; however, DSED sometimes persists long-term.
- Impulsive behaviors and risks of abuse can persist into adulthood.

10. Myths and misconceptions

Myth 1: “An open, friendly child is a positive trait, not a disorder”

Evidence: appropriate social distance is sign of developmental stage; in DSED absence of boundaries increases child's abuse risk.

Myth 2: “DSED resolves quickly in stable care”

Evidence: In contrast to RAD, DSED sometimes persists even in a stable environment (Bucharest study); long-term intervention is required.

Myth 3: “DSED is a ‘mild form’ of autism”

Evidence: distinct disorder; etiology psychosocial deprivation; clinical manifestations opposite (social withdrawal in autism, disinhibited approach in DSED).

Myth 4: “You can ‘fix’ a child with punishment”

Evidence: Punishment exacerbates the disorder and has a traumatic effect; a positive attachment-based approach is recommended.

Myth 5: “Williams syndrome and DSED are the same”

Evidence: Williams syndrome genetic (7q11.23 deletion), specific physical and cognitive profile; “hypersociability” is present but it is a distinct neurobiological and etiological disorder.

11. Sources

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6B60–6B6Z DISSOCIATIVE DISORDERS

Dissociative disorders

ICD-11 **6B64**	**6B64 DISSOCIATIVE IDENTITY DISORDER (DID)** **Dissociative identity disorder**
ICD-10 F44.81	Dissociative identity disorder
DSM-5-TR F44.81	Dissociative Identity Disorder

1. Definition and nosology

Dissociative Identity Disorder (DID; ICD-11: 6B64 Dissociative Identity Disorder; DSM-5-TR: F44.81) — a severe dissociative disorder characterized by transitions between two or more distinct personality states ("identities", "alters"). Each identity has a relatively enduring sense of self, memory, behavior, affect, perception, and cognition patterns. Concurrently, persistent dissociative amnesias (for daily events, important personal information, trauma).

DID — associated with severe childhood trauma (especially repeated physical or sexual abuse); the "split personality" image distorted in mass media differs from clinical reality.

2. History

- 1791 — Eberhardt Gmelin first case description.
- Janet P. (1880s) — concept of 'dissociation'.
- DSM-III (1980) — "Multiple Personality Disorder".
- DSM-IV (1994) — Term 'Dissociative Identity Disorder'.
- 1980s–1990s — 'False Memory' controversy; allegations of 'iatrogenic induction' in some diagnostic cases.

- ISSTD Treatment Guidelines (Adult Treatment, 3rd Revision 2011) — standardized the phase-oriented approach.

3. Epidemiology

- Prevalence in clinical populations: 1–3%; in general population ~1.5% (Sar V. ISRN Psychiatry 2011 review).
- Sex: 5–9 times higher in females in clinical populations.
- Onset: symptoms develop in childhood, clinical presentation often at 20–40 years.
- Comorbidity: MDD, PTSD, complex PTSD, BPD, somatic symptom, eating disorders, substance use, very high suicide risk.

4. Aetiology and pathogenesis

- Trauma model — repeated severe childhood trauma (sexual or physical violence, neglect, caregiver violence); 90%+ of patients show trauma history (Putnam F.W. reviews).
- Neurobiological — hippocampal and amygdala volume reduction; default mode and salience network dysregulation (Reinders A.A. et al. reviews).
- Attachment — disorganized attachment style in childhood.
- Iatrogenesis / supporters of the social-cognitive model — some diagnoses may be the result of therapeutic induction (critique by Lilienfeld S.O.); however, most research supports the trauma-based model.

5. Clinical features

- Two or more distinct identity states — some patients document overt transitions, others appear only with “internal voices,” sense of “loss of control.”
- Persistent amnesias — daily events, personal information, traumatic episodes.
- Depersonalization and derealization.
- Identity transitions as a response to stress, triggers.
- Self-harm, suicidal attempts, somatic symptoms.
- Complex dissociative symptoms (dissociative seizures, somatoform).

6. Diagnosis

6.1 Unified diagnostic criteria

A. Two or more distinct personality states — significant discontinuity in sense of self and agency, alterations in behavior, affect, perception, cognition, or sensorimotor function.

B. Recurrent amnesias regarding daily events, important personal information, and/or traumatic events.

C. Significant distress or functional impairment.

D. Not part of a widespread cultural or religious practice.

E. Exclusion of substance or medical condition (e.g., seizure).

6.2 Differences between sources

- DSM-5-TR — same criteria; imaginary companion in children excluded as part of normal development.
- ICD-11 — amnesia is not required: it says “there are typically episodes of amnesia”, whereas DSM-5-TR requires it as criterion B. Trance and possession disorders are separate ICD-11 categories (6B62, 6B63).
- ISSTD Guidelines 2011 — the diagnosis must be confirmed by a structured interview (SCID-D).

6.3 Diagnostic algorithm

1. Clinical interview — trauma history, dissociative symptoms.
2. SCID-D (Structured Clinical Interview for DSM Dissociative Disorders) is the gold standard.
3. Scale — DES (Dissociative Experiences Scale, ≥30 cut-off, ≥45 DID indicator); MID (Multidimensional Inventory of Dissociation).
4. Comorbidity (PTSD, BPD, MDD, substance).
5. Suicide and self-harm risk (high).
6. Medical and neurological – seizures, encephalitis excluded (EEG, MRI).

6.4 Differential diagnosis

Condition	Distinguishing feature
BPD (6D10.x)	Emotional lability and impulsivity dominant; significant clinical overlap, close comorbidity.
PTSD / cPTSD (6B40/6B41)	Traumatic intrusions and avoidance are primary; dissociation can be a component.
Schizophrenia (6A20)	"Voices" differ from schizophrenia — as internal dialogue; no psychotic characteristics.
Bipolar (6A60)	Affective episodes; no dissociation.
Factitious disorder (6D50)	Deceit for medical benefit; gain motivation.
Seizures (epileptic or non-epileptic)	EEG.
"Possession" — cultural	Context and cultural norm.

7. Examination and assessment

- SCID-D-R is the gold-standard structured interview.
- DES, MID scales.
- C-SSRS suicide; self-harm assessment.
- EEG rule out seizure.

8. Treatment

8.1 General principles (ISSTD 2011)

1. Phase-oriented (three-phase) approach:
    - Phase 1 — stabilization and symptom reduction: safety, emotion regulation, coping skills, hospitalization reduction, self-harm management.
    - Phase 2 — processing of traumatic memories: adapted trauma-focused intervention.
    - Phase 3 — integration and rehabilitation: inter-identity communication, functional restoration, relationships.

2. Specific psychotherapeutic approaches — excerpt from Internal Family Systems (Schwartz R.), modified EMDR.
3. Pharmacotherapy — no specific approval for DID; SSRIs, prazosin (for nightmares) for comorbid PTSD, MDD, anxiety symptoms.
4. Hospitalize — in acute self-harm or suicide risk.
5. Multidisciplinary approach — psychiatrist, psychologist, social support.
6. Long-term treatment (years); clinician continuity and therapeutic alliance are critical.

8.2 Treatment methods

1. Phase-Oriented Treatment (ISSTD) — Three-phase approach; sequence and patient stability are fundamental.
2. Structured Clinical Interview for DSM Dissociative Disorders, Revised (SCID-D-R) — 277-item structured interview; gold standard.
3. Dissociative Experiences Scale (DES) — 28 items; screening.
4. Multidimensional Inventory of Dissociation (MID) — 218 items; detailed dissociative profile.
5. EMDR — modified — Trauma processing after stabilization; sequential and adapted to patient's pace.

8.3 Differences between sources

- ISSTD 2011 — phase-oriented gold standard.
- Trauma-based approach and iatrogenesis caution — avoid searching or inducing ‘alters’.

9. Prognosis

- Functional recovery with long-term phase-oriented therapy.
- High suicide and self-harm risk monitored.
- Comorbidity treatment is important.

10. Myths and misconceptions

Myth 1: “DID is ‘split personality’ as in mass media”

Evidence: Film or television portrayals (dramatic transitions, “different people” do not coincide with the more subtle clinical picture in many patients — more “loss of control”, internal voices, amnesias are dominant).

Myth 2: “DID is a false condition induced by the therapist”

Evidence: strong association with childhood trauma (Putnam reviews); neurobiological markers (Reinders fMRI); international research consistent. However, diagnosis should be cautious — avoiding induction techniques for alters is important.

Myth 3: “DID is a rare or fabricated disorder”

Evidence: Clinical prevalence in populations 1–3% (Sar 2011); an underdiagnosed condition.

Myth 4: “Hypnosis or hospitalization is the main treatment”

Evidence: Phase-oriented psychotherapy is first-line; hypnosis can be used as an adjunct (with caution); inpatient admission only in acute self-harm or suicide risk.

Myth 5: “A patient with DID is prone to committing violence”

Evidence: Patients with DID are more likely to be victims of violence; violence towards others is rare. Parallel to myth ‘people with schizophrenia are dangerous.’

Myth 6: “The main goal is to ‘merge all identities into one’”

Evidence: integration typical goal, but complete assimilation not achievable in all patients; “cooperation” and “functional activity” also accepted as clinical outcomes.

11. Sources

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ICD-11 **6B65**	**6B65 PARTIAL DISSOCIATIVE IDENTITY DISORDER** **Partial dissociative identity disorder**
ICD-10 F44.8	Other dissociative [conversion] disorders
DSM-5-TR F44.89	Other Specified Dissociative Disorder

1. Definition and nosology

Partial Dissociative Identity Disorder (ICD-11: 6B65 Partial Dissociative Identity Disorder) — similar to DID (6B64), but differs in that non-dominant identities do not consistently fully occupy the patient's behaviour or conscious function. Instead, non-dominant identities interfere, but do not fully seize “executive control.”

In ICD-11, added for the first time as an official diagnostic category; not separate in DSM-5-TR (in DSM-5-TR can be assessed under Other Specified Dissociative Disorder).

2. History

- ICD-11 (2019) — recognition as part of the DID spectrum, with the partial form as an official category.
- New diagnostic framework for conditions recognized in clinical practice as ‘DDNOS’ or ‘DID-NOS’.

3. Epidemiology

- Prevalence, precise data limited — believed to be more common than DID.
- Develops on trauma basis.
- Comorbidity resembling DID.

4. Aetiology and pathogenesis

Same as DID — severe childhood trauma, disorganized attachment, neurobiological dissociative processes.

5. Clinical features

- Two or more distinct identity states.
- Non-dominant identities intrude — internal voices, thoughts, feelings, actions – but do not fully occupy patient's behavior and consciousness.
- Dominant identity controls daily functioning.
- Less amnesia compared to DID.
- Distress and functional impairment.

6. Diagnosis

6.1 Unified diagnostic criteria (ICD-11)

A. Two or more distinct identity states.

B. Non-dominant identities do not consistently take full control of the consciousness and behaviour of the dominant identity, but are present as an intrusive experience.

C. Significant distress or functional impairment.

D. Exclusion of substance or medical condition.

6.2 Differences between sources

- ICD-11 — differs from DID by ‘not fully capturing.’
- DSM-5-TR — may be assessed within Other Specified Dissociative Disorder (F44.89).

6.3 Diagnostic algorithm

Same as DID (see 6B64 §6.3).

6.4 Differential diagnosis

Condition	Distinguishing feature
DID (6B64)	Non-dominant identities consistently seize dominant control + amnesia.

Condition	Distinguishing feature
PTSD / cPTSD (6B40/6B41)	Traumatic intrusions; no separate identities.
BPD (6D10.x)	Emotional lability dominant.
Schizophrenia (6A20)	Psychotic symptoms.
Possession trance (cultural)	Cultural and religious context.

7. Examination and assessment

Same as DID (SCID-D-R, DES, MID).

8. Treatment

The phase-oriented approach is adapted for DID (ISSTD 2011). For treatment principles, phases, and specific methodologies, see section 6B64 “Treatment” (including “Treatment methods”).

9. Prognosis

Functional recovery with phase-oriented therapy; similar to DID.

10. Myths and misconceptions

Myth 1: “Partial DID is a ‘mild form’ of DID”

Evidence: distinct diagnostic category; functional impairment and trauma burden may be comparable to DID.

Myth 2: “Internal voices are always a psychotic symptom”

Evidence: dissociative voices differ from psychotic symptoms — internal dialogue, ‘own’ feeling, weak response to antipsychotics; SCID-D-R for differentiation.

Myth 3: “Antipsychotic ‘cures’ voices in partial DID”

Evidence: dissociative voices do not respond to antipsychotics; psychotherapy first-line.

Myth 4: “The patient is ‘role-playing’”

Evidence: Different from factitious disorder (6D50) — no motivation for gain; clinical experience is distressing for the patient.

11. Sources

1. WHO. ICD-11. 6B65 Partial dissociative identity disorder. 2024.
2. APA. DSM-5-TR. 2022.
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	6B66 DEPERSONALIZATION-DEREALIZATION DISORDER
ICD-11 6B66	**Depersonalization-derealization disorder**
ICD-10 F48.1	Depersonalization-derealization syndrome
DSM-5-TR F48.1	Depersonalization/Derealization Disorder

1. Definition and nosology

Depersonalization-Derealization Disorder (ICD-11: 6B66; DSM-5-TR: F48.1) — a dissociative disorder characterized by persistent or recurrent depersonalization (feeling detached from oneself, disconnected from body and thoughts) and/or derealization (surroundings appearing unreal, like a ‘foggy film’). Reality testing is preserved (the patient knows the experience is unreal).

2. History

- Krishaber M. (1873) — first description.
- Dugas L. (1898) — term ‘dépersonnalisation’.
- DSM-III (1980) — Separate diagnosis.
- DSM-5 (2013) — depersonalization and derealization as a combined category.

3. Epidemiology

- Lifetime prevalence: 1–2% in clinical form; subclinical experiences at least once in 50% of the population.
- Sex: approximately equal.
- Onset: adolescence and early adulthood (mean 16 years).
- Comorbidity: MDD, panic, GAD, PTSD, BPD.

4. Aetiology and pathogenesis

- Trauma and stress trigger (especially cannabis, hallucinogen use).
- Neurobiological — impaired prefrontal-limbic regulation; sensory processing anomalies (Simeon D. reviews).
- Comorbid anxiety and panic attack frequency high.

5. Clinical features

- Depersonalization: Sense of observing oneself from outside, feeling like a robot, emotional flattening, feeling that “my body is not mine”.
- Derealization: Environment appears “faded”, “like a faded film” or “artificial”; distance perception change.
- Reality testing preserved — patient knows the experience is unreal.
- Significant distress and functional impairment.
- Typical course — continuous or recurring episodes.

6. Diagnosis

6.1 Unified diagnostic criteria

A. Persistent or recurrent experiences of depersonalization and/or derealization.

B. Reality testing preserved.

C. Significant distress or functional impairment.

D. Exclusion of substance (cannabis, hallucinogen) or medical condition (seizure, brain injury).

E. Not better explained by another mental disorder (transient during panic attack; in context of PTSD; schizophrenia).

6.2 Differences between sources

- DSM-5-TR / ICD-11 — same criteria.

6.3 Diagnostic algorithm

1. Clinical interview.
2. CDS (Cambridge Depersonalisation Scale, Sierra M., Berrios G.E.) — 29 items.
3. Medical and neurological examination (excluding seizures and brain injury).
4. Toxicology screening.
5. Comorbidity.

6.4 Differential diagnosis

Condition	Distinguishing feature
Panic attack	Short-term in the context of an attack.
PTSD (6B40)	Against trauma background; intrusive symptoms.
DID (6B64)	Distinct identities.
Seizure (epileptic)	EEG; postictal state.
Schizophrenia	Reality testing impaired.
Substance-induced (cannabis, LSD)	Toxicology.

7. Examination and assessment

- CDS is the gold standard.
- EEG if seizure is to be ruled out.
- Toxicology.

8. Treatment

1. CBT — adapted for depersonalization; cognitive restructuring + exposure. Hunter E.C. et al. Behav Res Ther 2005.
2. SSRI — effective for comorbid depression and anxiety; evidence base for disorder itself limited.
3. Lamotrigine adjunct — effective in some studies.
4. Against trauma background — trauma-focused approach.

5. Abstinence from cannabis and hallucinogen use is critical.

8.1 Treatment methods

1. CBT for depersonalization — Hunter (Hunter E.C.), Sierra (Sierra M.) — Cognitive restructuring, exposure, grounding techniques.
2. Cambridge Depersonalisation Scale (CDS) — 29 items.
3. Grounding techniques — 5-4-3-2-1 (5 visible objects, 4 sounds, etc.); sensory attention.

8.2 Differences between sources

- Sierra M. reviews — CBT and adjunctive SSRIs.
- There is no specific FDA-approved medication.

9. Prognosis

- Chronic course, but significant improvement with CBT.

10. Myths and misconceptions

Myth 1: “Depersonalization is a sign of psychosis”

Evidence: Reality testing preserved; distinct from psychotic disorder; antipsychotic ineffective.

Myth 2: “Depersonalization is transient and does not require treatment”

Evidence: Clinical form features chronic course and functional impairment; intervention recommended.

Myth 3: “Cannabis improves depersonalization”

Evidence: Cannabis is a trigger and perpetuating factor; abstinence critical.

Myth 4: “Benzodiazepines are the main treatment”

Evidence: benzodiazepine provides short-term relief, but risk of dependence and depersonalization enhancement.

Myth 5: “Only antidepressants are effective”

Evidence: CBT first-line; SSRIs for comorbid symptoms.

11. Sources

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	6B60 DISSOCIATIVE NEUROLOGICAL SYMPTOM DISORDER
ICD-11 **6B60**	**Dissociative neurological symptom disorder**
ICD-10 F44.4–F44.7	Dissociative motor and sensory disorders
DSM-5-TR F44.4–F44.7	Functional Neurological Symptom Disorder (Conversion Disorder)

1. Definition and nosology

Dissociative Neurological Symptom Disorder (ICD-11: 6B60; DSM-5-TR: F44.4–F44.7 Functional Neurological Symptom Disorder / Conversion Disorder) — impairment of motor, sensory, or cognitive functions under voluntary control, not fully explained by a neurological or medical illness. Previously termed “conversion disorder” or “hysteria”.

2. History

- Charcot J.M. (19th century) — Studies on ‘hysteria’ at Salpêtrière.
- Freud S. — psychoanalytic “conversion” model.
- DSM-III (1980) — “Conversion Disorder” within somatoform disorders.

- DSM-5 (2013) — “Functional Neurological Symptom Disorder” — emphasis on joint neurological and psychiatric assessment.
- ICD-11 (2019) — transferred to the category of dissociative disorders.

3. Epidemiology

- In primary care and neurological clinics — 5–10% (reviews by Carson A.J. et al.).
- Sex: 2–3 times higher in females.
- Onset: adulthood, but also in children and the elderly.
- Comorbidity: MDD, anxiety, trauma history, somatic illnesses.

4. Aetiology and pathogenesis

- A traumatic or stressful trigger is common, but in a substantial and variable proportion of patients no trigger can be identified (0–77% across studies; Ludwig L. et al. Lancet Psychiatry 2018 meta-analysis) — which is why DSM-5 and ICD-11 no longer require a stressor for diagnosis.
- Neurobiological — impaired sensorimotor and cognitive prediction processing; predictive coding hypothesis (Edwards M.J.).
- “Functional” term — not telling the patient it is “made up”, but a functional disorder of the nervous system.

5. Clinical features

Typical symptoms

- Motor: Weakness, paralysis, involuntary movements (functional tremor, dystonia), gait disturbance.
- Sensory: sensory loss (anesthesia), visual impairment (functional blindness), hearing impairment.
- Functional (psychogenic) non-epileptic seizures (PNES): during seizure-like episodes there is no epileptic activity on EEG.
- Cognitive: “Foggy” concentration, slow processing — “functional cognitive disorder”.

Clinical signs (positive features).

- Patterns differing from typical neurological pathogenesis (e.g., Hoover sign in motor weakness — positive indicator of functional component).
- Distractibility — symptom changes when attention is directed elsewhere.

- Entrainability — tremor adapts to another rhythm.

6. Diagnosis

6.1 Unified diagnostic criteria

A. Impairment of one or more motor, sensory, or cognitive functions.

B. Clinical findings demonstrate incompatibility of the disorder with known neurological or medical illness (positive clinical features).

C. Symptoms are not better explained by another psychiatric or medical condition.

D. Significant distress or functional impairment.

6.2 Differences between sources

- DSM-5-TR — diagnosis based on “positive features” (not an exclusion method); joint assessment by neurologist and psychiatrist.
- ICD-11 — in the category of dissociative disorders.

6.3 Diagnostic algorithm

1. Neurological examination — functional positive features (Hoover, entrainability, distractibility).
2. Typical neurological condition exception — MRI, EEG, EMG, lumbar puncture (according to clinical presentation).
3. In PNES cases — video-EEG monitoring gold standard.
4. Psychiatric evaluation — comorbid trauma, MDD, anxiety.
5. The patient's diagnosis was explained positively — “not fabricated, but a functional disorder of the nervous system”.

6.4 Differential diagnosis

Condition	Distinguishing feature
Real neurological disease	Clinical and laboratory confirmation.
Factitious disorder (6D50)	Conscious lie; motivation for gain.
Malingering	Overt gain (legal, insurance).
Somatic symptom disorder (6C20)	Numerous somatic complaints; focus on the body.

Condition	Distinguishing feature
Hypochondriasis (6B23)	Illness preoccupation dominant.
Epileptic seizure	Video-EEG.

7. Examination and assessment

- Neurological examination (positive features).
- Video-EEG — in PNES cases.
- MRI, laboratory exclusion of neurological disease.
- Trauma and psychiatric assessment.

8. Treatment

1. Positive explanation of the diagnosis to the patient — "Functional neurological disorder is not fabricated"; the structured explanation of diagnosis is critical for functional improvement (Stone J. summaries).
2. Multidisciplinary approach — neurologist + psychiatrist + physiotherapist.
3. Physiotherapy (for motor symptoms) — evidence-based benefit (Nielsen G. et al. J Neurol Neurosurg Psychiatry 2017 RCT).
4. CBT — cognitive restructuring; specific protocols for PNES (LaFrance W.C. JAMA Psychiatry 2014).
5. Against trauma background — trauma-focused approach.
6. Anticonvulsants for PNES are ineffective and harmful — accurate diagnosis of the patient and discontinuation of antiepileptic drugs is critical.
7. SSRIs for comorbid depression/anxiety.

8.1 Treatment methods

1. Positive clinical diagnosis presentation — Stone (Stone J.) — The diagnosis is made by ruling in, not by ruling out; positive clinical signs explained to patient (e.g., Hoover sign demonstrated).
2. Physiotherapy for FND — Specific manual training, movement instruction; Nielsen G. et al. J Neurol Neurosurg Psychiatry 2017.
3. CBT for PNES — LaFrance (LaFrance W.C.) — Manualized 12 sessions; seizure diary, cognitive restructuring; LaFrance W.C. et al. JAMA Psychiatry 2014.
4. Video-EEG — PNES diagnosis gold standard.

8.2 Differences between sources

- FND-specific international consensus (Stone J., Carson A., LaFrance W.C.).
- NICE — multidisciplinary approach for functional symptoms.

9. Prognosis

- Improvement with early accurate diagnosis and intervention.
- Late diagnosis and polypharmacy — chronicity.

10. Myths and misconceptions

Myth 1: "The patient is malingering / 'faking'"

Evidence: Functional disorder is a real clinical condition — functional impairment of the nervous system; positive clinical signs prove it. Distinct from factitious disorder and simulation.

Myth 2: "Anticonvulsants are needed for PNES"

Evidence: Anticonvulsants ineffective in PNES, cause polypharmacy side effects; correct diagnosis and discontinuation of antiepileptic is critical.

Myth 3: "Functional impairment is psychological 'weakness', not physical"

Evidence: Functional disorder is a biological process of the nervous system; requires joint psychiatric and neurological assessment.

Myth 4: "Just 'dumping' the patient on a psychiatrist is enough"

Evidence: Neurologist and psychiatrist must collaborate; 'dismissing' the patient disrupts therapeutic alliance and reduces chance of improvement.

Myth 5: "Functional symptoms are not treatable"

Evidence: Physiotherapy, CBT, multidisciplinary approach show significant effect.

11. Sources

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ICD-11 **6B61**	**6B61 DISSOCIATIVE AMNESIA** **Dissociative amnesia**
ICD-10 F44.0	Dissociative amnesia
DSM-5-TR F44.0	Dissociative Amnesia

1. Definition and nosology

Dissociative Amnesia (ICD-11: 6B61; DSM-5-TR: F44.0) — inability to recall important autobiographical information (usually traumatic or stressful), not explained by ordinary forgetfulness. No medical or neurological cause.

Subtypes: localized (specific period), selective (part of the period), generalized (entire life history), systematized (specific category), continuous.

"Dissociative fugue" — In DSM-5-TR as a qualifier for dissociative amnesia (was a separate diagnosis in DSM-IV); patient travels unexpectedly, assumes a new identity.

2. History

- Janet P. — originator of the concept of dissociation.
- DSM-IV — Dissociative fugue as a separate diagnosis.
- DSM-5 (2013) — fugue integrated as a specifier.
- "False Memory" controversy (1990s) — debate over therapeutically induced "recovered memory".

3. Epidemiology

- Lifetime prevalence: 1–2%; higher after public misfortunes (war, disaster).
- Sex: relatively higher in females.

- Comorbidity: PTSD, cPTSD, DID, BPD, MDD.

4. Aetiology and pathogenesis

- Trauma — response to severe psychological or physical trauma; hypothesized as ‘defense mechanism’.
- Neurobiological — impaired hippocampal and prefrontal cognitive processing.

5. Clinical features

- Inability to recall the traumatic or stressful event.
- Localized (e.g., wartime), selective, generalized.
- Dissociative fugue — unexpected travel, new identity, usually lasts days to weeks.
- Implicit memory (skills) remains; explicit memory is selective.

6. Diagnosis

6.1 Unified diagnostic criteria

A. Failure to recall significant autobiographical information (typically traumatic), not explained by normal forgetfulness.

B. Significant distress or functional impairment.

C. Exclusion of substance, medical/neurological condition (TBI, encephalitis, dementia).

D. Not better explained by DID, PTSD, cPTSD, a somatic symptom disorder, or another mental disorder.

Qualifier: with dissociative fugue.

6.2 Differences between sources

- DSM-5-TR / ICD-11 — same; fugue qualifier.

6.3 Diagnostic algorithm

1. Clinical interview + trauma history.
2. Medical and neurological examination — TBI, dementia, encephalitis, seizure excluded.
3. MRI, EEG in clinical suspicion.

4. SCID-D-R.
5. Toxicology.

6.4 Differential diagnosis

Condition	Distinguishing feature
Traumatic brain injury (TBI)	History, MRI.
Transient global amnesia (TGA)	Acute, less than 24 hours; vascular etiology.
Dementia	Persistent cognitive decline.
Seizure (post-ictal)	EEG.
Substance-induced amnesia (alcohol blackout)	History.
Factitious disorder / malingering	Reward motivation.
DID (6B64)	Dissociative identity disorder.

7. Examination and assessment

- SCID-D-R.
- MRI, EEG.
- Neuropsychological test (memory profile).

8. Treatment

1. Safe environment and supportive psychotherapy.
2. Trauma-focused psychotherapy — phased approach.
3. During a fugue episode — reuniting the patient with family and community.
4. Pharmacotherapy for comorbid symptoms; ‘amobarbital interview’ or hypnosis was historically used — caution due to risk of induced false memory.
5. Long-term psychotherapy — trauma processing.

8.1 Treatment methods

1. Trauma-focused phased psychotherapy — Stabilization → processing → reintegration.
2. Structured Clinical Interview for DSM Dissociative Disorders, Revised (SCID-D-R) — Gold standard in diagnosis.

3. Hypnosis — cautious use — False memory induction risk; training only by experienced clinician.

8.2 Differences between sources

- ISSTD — general guideline for dissociative disorders.
- There is no specific FDA-approved pharmacotherapy.

9. Prognosis

- Most spontaneous remission in acute form;
- Chronic form requires long-term intervention.

10. Myths and misconceptions

Myth 1: "Hypnosis is a reliable tool for 'recovered memory'"

Evidence: hypnosis-induced memories can be contaminated with false memory — induced or distorted; especially cautious in legal contexts (Loftus E.F. research).

Myth 2: "Dissociative amnesia is fabricated or 'drama' from movies"

Evidence: Clinical condition documented; neurobiological foundations investigated.

Myth 3: "The patient 'must be forced to remember'"

Evidence: pressure may create trauma or distress; supportive approach recommended.

Myth 4: "Amobarbital interview is effective"

Evidence: Historically used, but not recommended in modern practice due to the risk of false memory and adverse effects.

Myth 5: "All traumatic memories are 'real'"

Evidence: Some 'recovered memories' may be a result of therapeutic induction; cautious clinical approach and source verification are important.

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6B80–6B8Z FEEDING AND EATING DISORDERS

Feeding or eating disorders

ICD-11 **6B80**	**6B80 ANOREXIA NERVOSA** **Anorexia Nervosa**
ICD-10 F50.0	Anorexia nervosa
DSM-5-TR F50.0x	Anorexia Nervosa

1. Definition and nosology

Anorexia Nervosa (ICD-11: 6B80; DSM-5-TR: F50.0x) — a severe eating disorder characterized by significant low body weight due to restriction of food intake, intense fear of weight gain, and disturbance in body image. Among the highest mortality rates of psychiatric disorders.

2. History

- Morton R. (1689) — first clinical description.
- Lasègue C. (1873) and Gull W. (1874) — ‘anorexia hysterica’ and ‘nervous anorexia’.
- DSM-III (1980) — Official diagnosis.
- DSM-5 (2013) — amenorrhea criterion eliminated (diagnosis possible in males and premenarchal).

3. Epidemiology

- Lifetime prevalence: in females 0.9–4%, in males 0.3% (Smink F.R. reviews).
- Sex: 10:1 in females; but the diagnosis is often missed in male patients.
- Onset: adolescence (14–18 years); second peak in middle age.

- Mortality: lifetime 5–6%; half medical (cachexia, cardiac), half suicide (Arcelus J. et al. Arch Gen Psychiatry 2011 meta-analysis).
- Comorbidity: MDD, OCD, anxiety, BPD.

4. Aetiology and pathogenesis

- Heritability 50–60% (Bulik C.M. reviews).
- Neurobiological — reduced cognitive flexibility, reward circuit dysregulation, atypical response to hunger.
- Social-cultural factors — thinness ideal; social media influence.
- Personality — perfectionism, anxiety, family history of eating disorder.

5. Clinical features

- Food restriction – calories, types of food.
- Body weight significantly low (BMI < 18.5 in adults; children BMI percentile).
- Marked fear of weight gain or persistent behavior aimed at preventing it.
- Body image disturbance — “I feel fat” even when underweight.
- Subtypes:
    - Restrictive type — only food restriction.
    - Binge/purging type — vomiting, laxatives, exercise.
- Medical complications — bradycardia, hypotension, hypothermia, electrolyte imbalance, osteoporosis, amenorrhea, lanugo, cardiac arrhythmia, death.

6. Diagnosis

6.1 Unified diagnostic criteria

A. Significant low weight due to restricted food intake (BMI <18.5 in adults; below age-adjusted norm in children).

B. Intense fear of gaining weight or persistent behavior to avoid weight gain, even when weight is low.

C. Body image disturbance or lack of recognition of the seriousness of current body weight.

6.2 Differences between sources

- DSM-5-TR — severity based on BMI (mild BMI ≥ 17, moderate 16–16.99, severe 15–15.99, extreme < 15).
- ICD-11 — a different severity scale: 6B80.0 significantly low body weight (adults BMI 18.5–14.0; children 5th–0.3rd percentile BMI-for-age), 6B80.1 dangerously low body weight (BMI < 14.0), 6B80.2 in recovery with normal body weight — the diagnosis is retained until full and lasting recovery. ICD-11 has no separate 'atypical anorexia nervosa' category (the term belongs to DSM-5-TR OSFED): a normal-BMI presentation is 6B80.2, or 6B8Y other specified feeding or eating disorders if the requirements are not met.
- NICE NG69 (Eating disorders, 2017, update) — diagnosis and treatment algorithms.

6.3 Diagnostic algorithm

1. Clinical interview.
2. EDE-Q (Eating Disorder Examination Questionnaire) or EDE — gold standard.
3. Medical examination — BMI, vital signs, electrolytes, EKG, bone densitometry.
4. Laboratory: complete blood count, electrolytes (especially K, P, Mg), liver, kidney, glucose, thyroid.
5. Comorbidity (MDD, OCD, anxiety).
6. Refeeding syndrome risk assessment (especially BMI <13 or minimal intake prolonged).

6.4 Differential diagnosis

Condition	Distinguishing feature
Bulimia Nervosa (6B81)	Normal weight; binge eating + compensatory behavior.
ARFID (6B83)	Weight loss, but no disturbance in body image.
Medical condition (cancer, hyperthyroidism, IBS, Crohn's disease)	Clinical confirmation.
MDD (decreased food intake)	No body image disturbance.
Schizophrenia (food delusion)	Psychotic thought basis.

7. Examination and assessment

- EDE-Q, EDE.
- Vital signs, BMI, weight history.
- Laboratory: complete blood count, electrolytes, liver, kidney, thyroid, FSH/LH, prolactin, vitamin D, B12, iron.
- EKG — bradycardia, QTc.
- DEXA — osteoporosis.
- C-SSRS suicide risk.

8. Treatment

8.1 General Principles (NICE NG69 · APA 2023)

1. Medical stabilization is critical: electrolytes, refeeding syndrome prophylaxis (phosphorus, magnesium, thiamine), cardiac monitoring.
2. Indication for hospitalization: BMI < 13, bradycardia < 40, hypokalemia, orthostatic changes, suicide risk, outpatient failure.
3. Psychotherapy first line:
    - FBT — Family-Based Treatment (Maudsley Model, Lock J., Le Grange D.) — gold standard in children and adolescents; 20 sessions in 3 phases.
    - CBT-E — Enhanced Cognitive Behavioral Therapy (Fairburn C.G.) — first-line in adults; 40 sessions over 40 weeks.
    - MANTRA, SSCM — alternatives for adults.
4. Pharmacotherapy — no specific effective drug for AN — antidepressant and antipsychotic for comorbidity; low-dose olanzapine (2.5–10 mg) may help weight gain in some patients (Attia E. et al. Am J Psychiatry 2019), but is not the primary intervention.
5. Nutrition — group or individual nutritionist; prophylaxis of refeeding syndrome — initial energy intake is low, phosphorus monitoring.
6. Duration of treatment is long (1–2 years or more).

8.2 Treatment methods

1. Family-Based Treatment / Maudsley Model (FBT) — Lock J., Le Grange D — 3-phase 20 sessions: (1) weight restoration – family controls food intake; (2) gradually returning control over eating to the adolescent; (3) adolescent-specific topics. Evidence: Lock J. et al. Arch Gen Psychiatry 2010 RCT.

2. Enhanced Cognitive Behavioral Therapy (CBT-E) — Fairburn C.G — Transdiagnostic (AN, BN, BED combined) approach; 40 sessions over 40 weeks; core focus on excessive value regarding body and weight. cbte.co.
3. Maudsley Anorexia Nervosa Treatment for Adults (MANTRA) — Schmidt U — Manualized for adults; focus on social-perfectionistic traits.
4. Specialist Supportive Clinical Management (SSCM) — Based on clinical supervision and psychoeducation.
5. Eating Disorder Examination Questionnaire (EDE-Q) — 28 items; baseline indicators and monitoring.
6. Refeeding Protocol (Maudsley, AED) — Start by risk: 10 kcal/kg/day at high risk, 5 kcal/kg/day at extreme risk (BMI < 14 or negligible intake for more than 15 days); increase gradually. Phosphorus, magnesium, potassium supplements; thiamine; daily electrolyte monitoring. Faster rates only in a specialist unit under close monitoring.

8.3 Differences between sources

- NICE NG69 (2017) — FBT in children and adolescents; CBT-E in adults.
- APA 2023 Practice Guideline for the Treatment of Patients with Eating Disorders, 4th ed.
- AED (Academy for Eating Disorders) — refeeding syndrome medical care standards.
- NICE — antidepressants not routinely recommended for AN.

9. Prognosis

- 50% remission, 30% residual symptoms, 20% chronic course.
- Mortality high (5–6%).
- Early intervention (especially FBT in adolescence) significantly improves prognosis.

10. Myths and misconceptions

Myth 1: “Anorexia is an ‘image’ disorder, not a serious medical condition”

Evidence: Arcelus 2011 meta-analysis — AN lifetime mortality 5–6%; highest among psychiatric disorders.

Myth 2: “The patient can eat voluntarily — just need to demand it”

Evidence: AN is a neurobiological and psychological complex disorder; the ‘volitional eating’ explanation is incorrect and harmful.

Myth 3: “Antidepressant is the main treatment for AN”

Evidence: NICE NG69 — antidepressants not routinely recommended for AN; may be used for comorbid MDD (more effective after weight restoration).

Myth 4: “AN only occurs in adolescent girls”

Evidence: AN also occurs in males (~10%); can develop in middle age; under-detection is common.

Myth 5: “The problem is resolved after weight restoration”

Evidence: Weight restoration is a crucial step, but without cognitive-behavioral intervention, relapse rates are high; psychotherapy is critical.

Myth 6: “Refeeding should be rapid”

Evidence: Rapid refeeding syndrome (hypophosphatemia, cardiac failure) possible; stepped approach and electrolyte monitoring critical.

Myth 7: “The patient is perfectionistic and high-performing — this trait should be preserved”

Evidence: Perfectionism is among the sustaining factors of AN; therapy is directed at this characteristic.

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ICD-11 **6B81**	**6B81 BULIMIA NERVOSA** **Bulimia Nervosa**
ICD-10 F50.2	Bulimia nervosa
DSM-5-TR F50.2	Bulimia Nervosa

1. Definition and nosology

Bulimia Nervosa (ICD-11: 6B81; DSM-5-TR: F50.2) — an eating disorder characterized by recurrent episodes of binge eating (large amount of food in a short period + sense of loss of control) and compensatory behaviors (vomiting, laxatives, exercise, fasting). Unlike anorexia, weight is typically normal or overweight.

2. History

- Russell G.F. (1979) — term ‘bulimia nervosa’ and first clinical description.
- DSM-III (1980) — Official diagnosis.
- DSM-5 (2013) — frequency criterion reduced to once per week (2 times in DSM-IV).

3. Epidemiology

- Lifetime prevalence: in females 1.5–3%, in males 0.5%.

- Onset: adolescence and early adulthood.
- Comorbidity: MDD ~70%, anxiety, substance use, BPD, history of anorexia.
- Suicide risk high.

4. Aetiology and pathogenesis

- Heritability 50–60%.
- Neurobiological — serotonergic dysregulation, dopamine reward system.
- Social-cultural — thinness ideal, dietary history.
- Personality — impulsivity, perfectionism, low self-esteem.

5. Clinical features

- Binge eating episodes — loss of control, secretive.
- Compensatory behaviors:
    - Purging — self-induced vomiting, laxatives, diuretics;
    - Non-purging — fasting, excessive exercise.
- Body image and weight preoccupation.
- Medical complications: hypokalemia (vomiting), Russell's sign (calluses on fingers), dental erosion, parotid hypertrophy, esophagitis, gastrointestinal issues.

6. Diagnosis

6.1 Unified diagnostic criteria

A. Repeated binge eating episodes — large amount of food in short period (≤ 2 hours) + feeling of loss of control.

B. Recurrent inappropriate compensatory behaviors to prevent weight gain.

C. Frequency: at least once weekly — DSM-5-TR requires ≥ 3 months, ICD-11 ≥ 1 month.

D. Self-evaluation and excessive dependence on body shape.

E. Does not occur in the context of an anorexia nervosa episode.

6.2 Differences between sources

- DSM-5-TR — severity based on episode frequency (mild 1–3, moderate 4–7, severe 8–13, extreme ≥14 per week).

- ICD-11 — criteria overlap, but the required duration differs: ICD-11 requires at least 1 month, DSM-5-TR 3 months; ‘atypical bulimia nervosa’ qualifier.
- NICE NG69 — diagnosis and treatment.

6.3 Diagnostic algorithm

1. Clinical interview.
2. EDE-Q, EDE.
3. Medical: electrolytes (K, P, Mg, Cl), liver, kidney, EKG (QTc).
4. Dental examination (erosion).
5. Comorbidity and suicide risk.

6.4 Differential diagnosis

Condition	Distinguishing feature
Anorexia Nervosa, binge-eating/purging type (6B80)	Significantly low weight.
BED (6B82)	No compensatory behavior.
Night Eating Syndrome	Nocturnal food intake predominates.
Kleine-Levin syndrome	Hypersomnia + hyperphagia.

7. Examination and assessment

- EDE-Q.
- Medical: electrolytes, EKG, dental examination.
- C-SSRS.

8. Treatment

1. CBT-E first-line (Fairburn) — 20 sessions over 20 weeks; evidence: Fairburn C.G. et al. Am J Psychiatry 2009;166(3):311–319.
2. Fluoxetine 60 mg/day — FDA approval for BN; reduces binge eating and purging.
3. FBT (Lock J., Le Grange D.) — in adolescents.
4. IPT alternative.
5. Medical: electrolyte correction, refeeding, dental care.
6. Bupropion is contraindicated (seizure risk).

8.1 Treatment methods

1. CBT-E (Fairburn) — 20 sessions; transdiagnostic; disruption of binge-purge cycle.
2. FBT for BN — Adapted for adolescents.
3. Interpersonal Therapy (IPT) — Alternative; slow response, but sustained effect.

8.2 Differences between sources

- NICE NG69 — CBT-E first-line; FBT in adolescents.
- APA 2023.

9. Prognosis

- 50–70% remission with CBT-E; long-term follow-up is essential.

10. Myths and misconceptions

Myth 1: "Bulimia is a 'diet failure'"

Evidence: Clinical impairment; restrictive diet is a typical trigger.

Myth 2: "Vomiting is an effective weight loss method"

Evidence: Vomiting prevents only 30–50% of calorie absorption; serious medical complications.

Myth 3: "Bupropion can be used as an antidepressant in BN"

Evidence: bupropion in BN is contraindicated — seizure risk is high.

Myth 4: "BN is not a problem if weight is normal"

Evidence: Medical complications (electrolytic, cardiac) and psychiatric burden are significant.

Myth 5: "Only antidepressants are sufficient"

Evidence: CBT-E + pharmacotherapy combination superior.

11. Sources

1. WHO. ICD-11. 6B81 Bulimia Nervosa. 2024.
2. APA. DSM-5-TR. 2022.
3. NICE NG69. 2017/2020.

4. APA. Practice Guideline for Eating Disorders, 4th ed. 2023.
5. Fairburn C.G. Cognitive Behavior Therapy and Eating Disorders. Guilford Press; 2008.
6. Russell G. Bulimia nervosa: an ominous variant of anorexia nervosa. Psychol Med 1979;9(3):429–448.
7. Lock J., Le Grange D. Treatment Manual for Anorexia Nervosa: A Family-Based Approach. 2nd ed. New York: Guilford Press, 2013.

ICD-11 **6B82**	**6B82 BINGE EATING DISORDER (BED)** **Binge eating disorder**
ICD-10 F50.8	Other eating disorders
DSM-5-TR F50.81	Binge-Eating Disorder

1. Definition and nosology

Binge-Eating Disorder (BED) (ICD-11: 6B82; DSM-5-TR: F50.81) — recurrent episodes of binge eating (large amount of food in a short period + sense of loss of control), without compensatory behaviors. Typically, weight is increased and associated with obesity.

2. History

- Stunkard A.J. (1959) — description of binge eating in the context of obesity.
- DSM-IV (1994) — In the research category.
- DSM-5 (2013) — separate official diagnosis.

3. Epidemiology

- Lifetime prevalence: 1.9–3.5%; most common among eating disorders.
- Sex: 1.5–2:1 in females; but also significant in males.
- Strong association with obesity.
- Comorbidity: MDD, anxiety, BPD, substance use.

4. Aetiology and pathogenesis

- Heritability 40–50%.
- Neurobiological — reward circuit dysregulation; ‘food addiction’ hypothesis.
- Emotional eating — stress trigger.
- History of restrictive diet.

5. Clinical features

- Binge eating episodes — large amount of food in short time, loss of control.
- Typical features: rapid eating, continuing despite fullness, secretive eating, subsequent shame, guilt.
- No compensatory behavior.
- Weight gain and obesity complications (Type 2 diabetes, cardiovascular, OSA).

6. Diagnosis

6.1 Unified diagnostic criteria

A. Repeated binge eating episodes.

B. Episodes associated with at least ≥ 3 of the following:

1. Rapid eating
2. Eating until physically full;
3. Binge eating without hunger;
4. Secretive eating (due to shame);
5. Feeling disgusted with oneself, depressed, or very guilty afterward.

C. Significant distress.

D. Frequency: 1 time per week, ≥3 months.

E. Does not occur in the context of bulimia or anorexia; no compensatory behavior.

6.2 Differences between sources

- DSM-5-TR — severity based on episode frequency.

- ICD-11 — no severity scale: DSM-5-TR grades by episode count (mild 1–3, moderate 4–7, severe 8–13, extreme ≥14 per week), whereas ICD-11 says only “once a week or more over a period of several months”.

6.3 Diagnostic algorithm

1. Clinical interview.
2. EDE-Q.
3. Medical assessment – metabolic syndrome screening (glucose, HbA1c, lipids, blood pressure).
4. Comorbidity.

6.4 Differential diagnosis

Condition	Distinguishing feature
Bulimia (6B81)	Compensatory behavior.
Obesity (without compulsive eating)	No binge eating episode features.
Kleine-Levin	Hyperphagia + hypersomnia.
Prader-Willi syndrome	Genetic; hyperphagia out of control.

7. Examination and assessment

- EDE-Q.
- BMI, weight, vital signs.
- Metabolic panel.

8. Treatment

1. CBT-E first-line — Wilson G.T. et al.'s meta-analysis indicated a significant effect in BED.
2. Lisdexamfetamine (Vyvanse) 50–70 mg/day — FDA approval for BED; significantly reduces binge-eating frequency (McElroy S.L. JAMA Psychiatry 2015).
3. SSRI — for comorbid MDD; modest effect on binge eating frequency.
4. IPT alternative.
5. Weight management — diet, physical activity; weight loss is not effective without BED intervention.
6. Bariatric surgery requires long-term follow-up (BED preoperative assessment).

8.1 Treatment methods

1. CBT-E for BED — Breaking binge chain, alternative coping.
2. Lisdexamfetamine — FDA approval for BED; start 30 mg, target 50–70 mg/day.
3. IPT — Focus on interpersonal issues.
4. Guided Self-Help CBT (Guided Self-Help) — Internet-based, book-based.

8.2 Differences between sources

- NICE NG69 — CBT or guided self-help first-line.
- APA 2023.
- McElroy lisdexamfetamine RCTs.

9. Prognosis

- 50–60% remission with CBT-E.
- Weight management and metabolic complications require long-term monitoring.

10. Myths and misconceptions

Myth 1: "BED is just obesity or a 'overeating' personality trait"

Evidence: Clinical eating disorder; distinguishing features from binge eating episode (loss of control, shameful eating).

Myth 2: "Restrictive diet cures BED"

Evidence: Restrictive diet triggers BED — weight control without psychological intervention increases binge eating.

Myth 3: "Bariatric surgery cures BED"

Evidence: Surgery reduces abdominal capacity but does not modify the cognitive-behavioral component; in some patients, a new pattern of "grazing" develops.

Myth 4: "Eating is an 'addiction' (food addiction), must be treated like drugs"

Evidence: "Food addiction" is a concept but not accepted as a clinical diagnosis; standard eating disorder interventions are superior.

Myth 5: "BED only requires an antidepressant"

Evidence: lisdexamfetamine FDA approval for BED; CBT first-line.

11. Sources

1. WHO. ICD-11. 6B82 Binge eating disorder. 2024.
2. APA. DSM-5-TR. 2022.
3. NICE NG69. 2017/2020.
4. McElroy S.L. et al. Efficacy and safety of lisdexamfetamine for treatment of adults with moderate to severe binge eating disorder. JAMA Psychiatry 2015;72(3):235–246.
5. Wilson G.T., Wilfley D.E., Agras W.S., Bryson S.W. Psychological treatments of binge eating disorder. Arch Gen Psychiatry 2010;67(1):94–101.

ICD-11 **6B84**	**6B84 PICA** **Pica**
ICD-10 F98.3	Pica of infancy and childhood
DSM-5-TR F98.3 / F50.89	Pica

1. Definition and nosology

Pica (ICD-11: 6B84; DSM-5-TR: F98.3) — persistent (≥ 1 month) ingestion of non-food, non-nutritive substances (clay, hair, paper, soap, metal, stones) — inappropriate for developmental age.

2. History

- "Pica" — from the Latin word for "magpie" (magpie eats everything).
- DSM-III (1980) — As a childhood disorder.
- DSM-5 (2013) — diagnosis possible at all ages.

3. Epidemiology

- Prevalence: children 4–26%, higher in intellectual disability, pregnant women 8–65%, iron deficiency anemia.

- Comorbidity: autism, intellectual disability, iron deficiency, adult malabsorption.

4. Aetiology and pathogenesis

- Nutritional deficiencies (iron, zinc) — may be worsened by pica substance ingestion in some patients.
- Neurodevelopmental disorders (autism, ID).
- Pregnancy — physiological and micronutrient.
- Cultural — consumption of clay in some traditions (geophagy).

5. Clinical features

- Geophagy (clay, soil), trichophagy (hair), pagophagy (ice), amylophagy (starch), plumbophagy (paint — lead risk).
- Complications: intestinal obstruction (trichobezoar), lead poisoning, parasitic infection, dental injury, perforation.

6. Diagnosis

6.1 Unified diagnostic criteria

A. Persistent intake of substances with no nutritional value ≥ 1 month.

B. The behavior is not developmentally appropriate (≥ 2 years of age).

C. Not a culturally or socially accepted practice.

D. If occurring in the context of another mental disorder or medical condition — additional diagnosis required (especially in ID and ASD).

6.2 Differences between sources

- DSM-5-TR requires a duration of at least 1 month; ICD-11 sets no duration criterion — it says “persistent or severe enough to require clinical attention”. The developmental age of about 2 years is the same in both.

6.3 Diagnostic algorithm

1. Interview + observation.
2. Laboratory — complete blood count, ferritin, iron, lead level, zinc.
3. Abdominal imaging (obstruction).
4. Neurodevelopmental assessment.

6.4 Differential diagnosis

Condition	Distinguishing feature
OCD (6B20)	It is based on obsession.
Factitious (6D50)	Reward motivation.
Anorexia binge-eating type (6B80)	Body image.
Cultural food practices	Context.

7. Examination and assessment

- Iron, lead, zinc, parasite tests.
- Abdominal X-ray / USG.

8. Treatment

1. Correction of nutritional deficiency (iron, zinc).
2. Behavioral intervention — habit reversal, environmental modification, alternative stimuli.
3. Intellectual disability comorbid with ASD — functional behavior analysis and ABA-based approach.
4. Medical management of complications (lead poisoning — chelation only with specific indication).
5. During pregnancy — nutritional support.

8.1 Treatment methods

1. Functional Behavior Analysis (FBA) — Identification of behavior trigger and maintainer.
2. Environmental Modification — Restriction of access to non-food substances.
3. Nutritional Correction — Iron, zinc, vitamin supplements.
4. ABA-based intervention — In ID and ASD.

8.2 Differences between sources

- AAP — pica assessment and lead screening in children.

9. Prognosis

- Remission in children through nutritional correction and behavioral intervention.
- May persist on an ID/ASD background; long-term follow-up.

10. Myths and misconceptions

Myth 1: “Pica is just a child's ‘exploratory’ behavior”

Evidence: General developmental part up to 2 years; subsequent continuation is clinical and poses a risk of complications.

Myth 2: “Lead poisoning requires chelation for everyone”

Evidence: chelation only confirmed at high levels (BLL ≥ 45 μg/dL typical); environmental remediation first-line at lower levels.

Myth 3: “Pica in pregnancy is 'the baby's wish', requires no intervention”

Evidence: iron or other deficiency marker; nutritional correction recommended.

Myth 4: “Pica in an ID/ASD patient is ‘inevitable’”

Evidence: Behavioral interventions (ABA, FBA) are effective; the “inevitable” explanation leads to severe complications such as trichobezoar, obstruction.

11. Sources

1. WHO. ICD-11. 6B84 Pica. 2024.
2. APA. DSM-5-TR. 2022.
3. Young S.L. Pica in pregnancy: new ideas about an old condition. Annu Rev Nutr 2010;30:403–422.
4. Hyman S.L. et al. AAP 2020 Clinical Report (ASD). Pediatrics 2020;145(1):e20193447.

ICD-11 6B85	**6B85 RUMINATION-REGURGITATION DISORDER** **Rumination-regurgitation disorder**
ICD-10 F98.21	Rumination disorder of infancy and childhood
DSM-5-TR F98.21	Rumination Disorder

1. Definition and nosology

Rumination-Regurgitation Disorder (ICD-11: 6B85; DSM-5-TR: F98.21) — characterized by voluntary or semi-voluntary regurgitation of swallowed food followed by re-chewing, re-swallowing, or spitting out. Not fully explained by a medical condition (gastroesophageal reflux or other GI disease).

2. History

- Term ‘mericism’ in ancient times.
- DSM-IV — As an infant and childhood disorder.
- DSM-5 (2013) — diagnosis at all ages.

3. Epidemiology

- Prevalence: rare in infants and children; 6–10% in patients with intellectual disability; increasing recognition in adults (especially comorbid with eating disorders).
- Comorbidity: ID, ASD, anxiety, BN, BED.

4. Aetiology and pathogenesis

- Behavioral conditioning — ‘post-prandial belching’ followed by regurgitation as a learned behavior.
- Stress and anxiety.
- In ID/ASD, self-stimulation with food.

5. Clinical features

- Regurgitation minutes to hours after food intake.
- Eating disorder outcomes: weight loss, malnutrition, electrolyte imbalance, dental erosion, halitosis.
- Social consequences: shame, social isolation.

6. Diagnosis

6.1 Unified diagnostic criteria

A. Recurrent regurgitation of swallowed food — DSM-5-TR: ≥ 1 month; ICD-11: several times a week over at least several weeks; rechewing, swallowing, or spitting.

B. Regurgitation is not related to a medical condition (e.g., reflux, pyloric stenosis, duodenal obstruction).

C. Does not occur in the context of anorexia, bulimia, BED, ARFID.

D. If occurring in the context of another mental disorder (especially ID, ASD), significant severity requires separate assessment.

6.2 Differences between sources

- DSM-5-TR / ICD-11 — criteria overlap; the duration and the age threshold differ: DSM-5-TR requires ≥ 1 month, ICD-11 requires several times a week over at least several weeks and a developmental age of ≥ 2 years.

6.3 Diagnostic algorithm

1. Clinical interview.
2. Medical assessment — exclusion of GERD, pyloric pathology, gastroparesis (upper endoscopy, motility study).
3. Manometry — confirmation of functional regurgitation.
4. Weight and nutritional status.
5. Comorbid eating disorder screening.

6.4 Differential diagnosis

Condition	Distinguishing feature
GERD	Typical regurgitation; pH monitoring.
Bulimia (6B81)	Vomiting voluntarily for compensatory purposes.

Condition	Distinguishing feature
Anorexia (6B80)	Fear of weight.
Gastroparesis	Motility study.
Pyloric obstruction	Endoscopy.

7. Examination and assessment

- Endoscopy, manometry, pH-monitoring.
- Complete blood count, electrolytes.

8. Treatment

1. Diaphragmatic breathing — first-line behavioral intervention (Chitkara D.K. et al. Am J Gastroenterol 2006; effect 50–70%).
2. Habit reversal — learning alternative behavior.
3. Biofeedback.
4. Stress management and CBT.
5. In ID/ASD, functional behavior analysis and ABA.
6. Medical: GERD with proton pump inhibitor initial intervention.

8.1 Treatment methods

1. Diaphragmatic Breathing — Slow abdominal breathing for 5–10 minutes after eating — prevents regurgitation.
2. Habit Reversal Training (HRT — Habit Reversal Training) — Developing alternative behaviors.
3. Biofeedback — Abdominal muscle control.
4. FBA — functional behavior analysis (in ID/ASD) — Analysis of behavior trigger and maintainer.

8.2 Differences between sources

- Rome Foundation criteria (functional GI).
- NICE NG69 — general eating disorder principles.

9. Prognosis

- Significant improvement with diaphragmatic breathing and habit reversal.

10. Myths and misconceptions

Myth 1: “Rumination is a form of GERD”

Evidence: distinct disorder; manometry and clinical model differ.

Myth 2: “Only medical surgery (Nissen) solves it”

Evidence: Nissen fundoplication ineffective for functional rumination and contraindicated; behavioral intervention first-line.

Myth 3: “The patient behaves ‘volitionally’, requires discipline”

Evidence: It is a semi-voluntary behavior, based on conditioning; punishment is ineffective.

Myth 4: “Antiemetics are effective”

Evidence: Antiemetics (e.g., ondansetron) ineffective in rumination.

Myth 5: “Cannot be discontinued in a patient with ID/ASD”

Evidence: Behavioral interventions are effective; active intervention is essential to prevent nutritional complications.

11. Sources

1. WHO. ICD-11. 6B85 Rumination-regurgitation disorder. 2024.
2. APA. DSM-5-TR. 2022.
3. Chitkara D.K., Van Tilburg M., Whitehead W.E., Talley N.J. Teaching diaphragmatic breathing for rumination syndrome. Am J Gastroenterol 2006;101(11):2449–2452.
4. Rome IV Diagnostic Criteria for Disorders of Gut-Brain Interaction. Rome Foundation; 2016.

	6B83 AVOIDANT-RESTRICTIVE FOOD INTAKE DISORDER (ARFID)
ICD-11 **6B83**	**Avoidant-restrictive food intake disorder**
ICD-10 F98.2	Feeding disorder of infancy and childhood
DSM-5-TR F50.82	Avoidant/Restrictive Food Intake Disorder

1. Definition and nosology

Avoidant/Restrictive Food Intake Disorder (ARFID; ICD-11: 6B83; DSM-5-TR: F50.82) — significant weight loss, nutritional deficiency, enteral feeding dependence, or psychosocial impairment resulting from avoidance or restriction of food intake. No body image disturbance or fear of weight gain (key difference from anorexia).

2. History

- DSM-IV ‘Feeding Disorder of Infancy or Early Childhood’ — only in children.
- DSM-5 (2013) — ARFID expanded to all ages.

3. Epidemiology

- Prevalence: 0.3–3.2% (~5–14% in eating disorder clinics).
- Sex: approximately equal (unlike anorexia).
- Onset: predominantly in childhood and adolescence.
- Comorbidity: autism spectrum, anxiety, OCD.

4. Aetiology and pathogenesis

- Sensory sensitivity (food texture, smell, appearance) — especially on autism background.
- Fear of choking or vomiting trauma.

- Low interest in food (low appetite, low food interest).

5. Clinical features

Three clinical presentations

- Sensory sensitivity — "picky eating" – limited food variety, texture sensitivity.
- Low interest in food — low hunger, little interest triggered by food.
- Fear (choking, vomiting) — avoidance following a specific triggering event.

Results: significant weight loss or insufficient weight gain (in children), nutritional deficiencies (iron, zinc, vitamins), dependence on enteral feeding, social disruption (school meals, eating with peers).

6. Diagnosis

6.1 Unified diagnostic criteria

A. As a result of avoidance or restriction of food intake, at least one of:

1. Significant weight loss or failure to achieve adequate weight gain;
2. Significant nutritional deficiency;
3. Enteral feeding or oral nutritional supplement dependence;
4. Significant psychosocial impairment.

B. Disorder not explained by existing nutritional deficiency or cultural experience.

C. Does not occur in the context of anorexia or bulimia; no body image disturbance.

D. Not better explained by another medical or mental condition (e.g., allergy, OCD).

6.2 Differences between sources

- DSM-5-TR / ICD-11 — same.
- NICE NG69 — specific intervention rules for ARFID.

6.3 Diagnostic algorithm

1. Clinical interview and dietary history.
2. NIAS (Nine Item ARFID Screen) — screening.
3. Medical evaluation — weight, height, BMI, laboratory nutritional deficiency.

4. Screening for autism and OCD.
5. Exclusion of allergy and GI pathology.

6.4 Differential diagnosis

Condition	Distinguishing feature
Anorexia (6B80)	Body image and weight fear are central.
Autism spectrum (6A02)	Sensory sensitivity component; ARFID can be comorbid with ASD.
OCD (6B20)	Contamination obsessions primary; ritualized behavior.
Specific phobia (6B03)	Specific object; multiple food types in ARFID.
Allergy / Crohn's / celiac disease	Medical confirmation.

7. Examination and assessment

- NIAS, EDE-Q.
- Weight, height, BMI history.
- Laboratory: complete blood count, ferritin, zinc, vitamin D, B12, folate.
- Autism screening (M-CHAT, ADOS).

8. Treatment

1. Multidisciplinary approach: nutritionist + CBT therapist + feeding-occupational therapist.
2. FBT-ARFID — adapted by Lock J.
3. CBT-AR — Cognitive Behavioral Therapy for ARFID (Thomas J.J.) — manualized.
4. Stepped food exposure — gradual introduction of new food types.
5. Sensory integration elements (on autism background).
6. Medical: nutritional correction; severe case enteral feeding.
7. SSRIs for comorbid anxiety; no FDA-approved specific medication for ARFID.

8.1 Treatment methods

1. CBT for ARFID (CBT-AR — CBT for ARFID) — Thomas (Thomas J.J.) — Manualized; protocols tailored to sensory, low interest, fear subtypes.
2. FBT-ARFID — The role of the family in the exposure to food.
3. Stepped Food Exposure — New food approach → touch → taste → swallow steps.
4. Nine-Item ARFID Screen (NIAS — Nine Item ARFID Screen) — 3 subtype screening.

8.2 Differences between sources

- NICE NG69 — graded food exposure.
- Thomas J.J. CBT-AR — a new evidence base.

9. Prognosis

- With a multidisciplinary approach, improvement in most patients.
- Long-term follow-up in the context of autism and OCD.

10. Myths and misconceptions

Myth 1: “ARFID is just ‘picky eating’”

Evidence: Clinical diagnosis requires significant functional impairment or nutritional deficiency.

Myth 2: “The patient can be forced to eat by starvation”

Evidence: Hunger strategy ineffective in ARFID and harmful to family-child relationship; stepwise exposure recommended.

Myth 3: “ARFID is a ‘mild form’ of anorexia”

Evidence: distinct clinical unit; body image disturbance and weight concern absent; intervention differs.

Myth 4: “ARFID on the autism spectrum cannot be treated”

Evidence: sensory integration and behavioral approaches are also effective in the context of autism.

Myth 5: “The patient will outgrow it”

Evidence: It can progress without treatment; early intervention prevents nutritional complications.

11. Sources

1. WHO. ICD-11. 6B83 ARFID. 2024.
2. APA. DSM-5-TR. 2022.
3. NICE NG69. 2017/2020.
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5. Zickgraf H.F., Ellis J.M. Initial validation of the Nine Item Avoidant/Restrictive Food Intake Disorder Screen (NIAS). Appetite 2018;123:32–42.
6. Lock J., Le Grange D. Treatment Manual for Anorexia Nervosa: A Family-Based Approach. 2nd ed. New York: Guilford Press, 2013.

6C00–6C0Z ELIMINATION DISORDERS

Elimination disorders

ICD-11 **6C00**	**6C00 ENURESIS** **Enuresis**
ICD-10 F98.0	Nonorganic enuresis
DSM-5-TR F98.0	Enuresis

1. Definition and nosology

Enuresis (ICD-11: 6C00; DSM-5-TR: F98.0) — involuntary urination persisting when the child's developmental age reaches at least 5 years. Frequency: at least twice a week for ≥ 3 months or significant functional impairment.

Subtypes (ICCS — International Children's Continence Society 2014):

- Monosymptomatic nocturnal enuresis — only nocturnal, no daytime symptoms;
- Non-monosymptomatic enuresis — daytime symptoms (urgency, increased frequency, incontinence) additional;
- Primary — child has never achieved 6 months of sustained dryness;
- Secondary — relapse after a dry period.

2. History

- In the 19th century, familial clustering of enuresis began to be noted in the clinical literature.
- Mowrer O.H. (1938) — invented the pad-and-bell alarm.
- ICCS (1989) — international standardization.
- NICE CG111 (2010) — nocturnal enuresis clinical guideline.

3. Epidemiology

- Prevalence: Age 5: 15–20%, Age 7: 5–10%, Age 10: 3–5%, Adults: 1–2%.
- Sex: male 2:1.
- Spontaneous remission occurs in approximately 15% per year.
- Comorbidity: ADHD ~30%, anxiety, conduct disorders, constipation.

4. Aetiology and pathogenesis

- Heritability 70% (Bayoumi R.A. reviews).
- Monosymptomatic nocturnal
    - Immaturity of nocturnal vasopressin (ADH) secretion (nocturnal polyuria);
    - Elevation of arousal threshold;
    - Bladder capacity below age norm.
- Non-monosymptomatic form — bladder overactivity, constipation association.
- Secondary enuresis — stress, urinary infection (UTI), diabetes, psychiatric trigger.

5. Clinical features

- Nocturnal enuresis.
- Daytime symptoms (non-monosymptomatic): urgency, holding maneuvers (Vincent's curtsy, leg crossing), increased frequency, incontinence.
- Social-emotional: shame, decreased self-esteem, avoidance of social activities (camping, sleepovers).

6. Diagnosis

6.1 Unified diagnostic criteria (DSM-5-TR / ICD-11 / ICCS 2014)

A. Repeated urinary incontinence (in bed or clothes), voluntary or involuntary.

B. Frequency: ≥2 times per week, ≥3 months; or significant distress/functional impairment.

C. Developmental age ≥ 5 years.

D. Not fully explained by substance (diuretic) or medical condition (diabetes, UTI, structural anomaly).

Qualifiers: only nocturnal / only diurnal / nocturnal and diurnal; primary / secondary; monosymptomatic / non-monosymptomatic.

6.2 Differences between sources

- ICCS 2014 — subtype terminology standard.
- NICE CG111 — diagnosis and algorithm.
- AAP — pediatric assessment.

6.3 Diagnostic algorithm

1. Clinical interview — onset, frequency, daytime symptoms, family history.
2. Urinalysis and culture — rule out UTI.
3. Voiding diary 3 days — for daytime symptoms.
4. Constipation assessment (Bristol Stool Chart).
5. Medical: weight, glucose, blood pressure.
6. If structural anomaly suspected — urinary USG; uroflowmetry in non-monosymptomatic case.

6.4 Differential diagnosis

Condition	Distinguishing feature
UTI	Urine culture positive.
Diabetes (Type 1)	Polyuria, polydipsia; glucose high.
Diabetes insipidus	Specific gravity low.
Structural anomaly (ectopic ureter)	Persistent leakage; USG.
Neurogenic bladder	Spinal cord pathology.
Constipation	Clinical and Bristol Stool.
Seizure (ictal enuresis)	EEG.

7. Examination and assessment

- Urine analysis, culture.
- Voiding and constipation diary.
- Urinary ultrasound (atypical presentation).
- Comorbid ADHD, anxiety screening.

8. Treatment

8.1 General principles (ICCS 2020 · NICE CG111)

1. Behavioral intervention:
    - Conservative recommendations — reducing fluid intake before bedtime, adequate water intake during the day, regular urination, resolving constipation.
    - Positive reinforcement (star chart) — for dry nights.
2. First-line specific intervention:
    - Enuresis alarm — pad-and-bell; ≥ 6 years old, if motivated; long-term good outcome; about 50% achieve 14 consecutive dry nights and about 40% remain dry at follow-up (low-quality evidence). Evidence: Caldwell P.H. Cochrane 2020.
    - Desmopressin — synthetic ADH analog; rapid effect, but high relapse upon discontinuation. ≥ 6–7 years.
3. Second-line:
    - Alarm + desmopressin combination;
    - Anticholinergic (oxybutynin) — in the context of non-monosymptomatic overactive bladder.
4. Refractory: imipramine (TCA) — cardiac monitoring; use cautiously, overdose toxicity.
5. Resolving constipation — PEG (polyethylene glycol) first-line; in non-monosymptomatic or comorbid encopresis context.

8.2 Treatment methods

1. Enuresis Alarm — Mowrer — Bed alarm or clothing alarm (humidity sensor); ≥ 4 weeks continuous use; typical use 12–16 weeks. Cochrane meta-analysis — most evidence-based long-term method.
2. Desmopressin — Oral 0.2–0.4 mg or sublingual; 1 hour before bedtime; limiting fluids before bedtime is important (risk of hyponatremia).
3. Voiding and Constipation Diary — 3-day; frequency, volume, bladder capacity, daytime symptoms, defecation frequency.
4. Anticholinergic (Oxybutynin) — Against background of over-active bladder; if daytime symptoms are present.
5. PEG (Constipation correction) — First-line; also used in encopresis.

8.3 Differences between sources

- ICCS 2020 — alarm and desmopressin first-line; “wait and see” until age 5, then active intervention.
- NICE CG111 — same algorithm.
- AAP — pediatric recommendations.

9. Prognosis

- Spontaneous remission occurs in approximately 15% per year; with intervention, remission occurs in most patients.
- Constipation management is important.
- Comorbid ADHD, anxiety — parallel intervention.

10. Myths and misconceptions

Myth 1: “In nocturnal enuresis, electrophoresis to the groin and buttock area stimulates the urinary center”

Why it is widespread: Soviet-era physiotherapy practice; presented as “safe” by family doctors.

Clinical logic: If daytime urine output is normal and the problem did not spontaneously arise after any illness, the problem is not at the kidney/bladder/spinal cord level. Pathogenesis is ADH dysregulation, arousal threshold and bladder capacity — electrophoresis does not affect these.

Evidence: ICCS 2020, NICE CG111 — no international guideline includes electrophoresis. Risk: skin irritation, psychosexual burden.

Real clinical step: Alarm or desmopressin.

Myth 2: “It will just pass on its own, no intervention needed”

Evidence: Spontaneous remission ~15% per year, but untreated persistence creates socio-emotional burden; ICCS 2020 recommends active intervention from age 5+.

Myth 3: “Punishment or insult corrects a child”

Evidence: Punishment is contraindicated; self-esteem reduction, depression risk; positive reinforcement is superior.

Myth 4: “Pre-bedtime water restriction alone is sufficient”

Evidence: Adjunct measure; ineffective alone in most patients; requires specific intervention.

Myth 5: “Imipramine or antidepressants are first-line”

Evidence: imipramine in refractory cases; cardiac side effects and overdose risk; alarm and desmopressin superior.

Myth 6: “Acupuncture and homeopathy cure enuresis”

Evidence: Cochrane reviews — the effect of these complementary interventions has not been proven.

11. Sources

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ICD-11 **6C01**	**6C01 ENCOPRESIS** **Encopresis**
ICD-10 F98.1	Nonorganic encopresis
DSM-5-TR F98.1	Encopresis

1. Definition and nosology

Encopresis (ICD-11: 6C01; DSM-5-TR: F98.1) — voluntary or involuntary defecation into inappropriate places (clothing, floor) persisting when the child's developmental age reaches at least 4 years. Frequency: at least once a month for ≥ 3 months.

Subtypes:

- Retentive (overflow incontinence) — based on constipation; ~80% of cases;
- Non-retentive — no constipation.

2. History

- DSM-III (1980) — Official diagnosis.
- Rome IV — consensus within the functional GI framework.
- NASPGHAN/ESPGHAN functional constipation guideline (Tabbers M.M. et al. 2014).

3. Epidemiology

- 1–4% among ages 4–17; 3–6 times higher in boys.
- ~80% are primarily constipation-related (retentive).
- Comorbidity: enuresis, ADHD, anxiety, autism spectrum.

4. Aetiology and pathogenesis

- Constipation — key mechanism (in retentive encopresis); painful defecation → retention → rectal dilatation → sensory reduction → overflow.
- Trauma, fear, discipline in toilet training process.
- Diet (low fiber, dehydration).
- Behavioral and emotional factors — oppositional, anxiety.
- In rare cases — Hirschsprung disease (rectal aganglionosis), spinal pathology.

5. Clinical features

- Fecal soiling of clothing.
- Retentive — fecal mass on abdominal palpation; tense rectum; large (massive) defecation episodes.
- Social-emotional: shame, social isolation, school avoidance.

6. Diagnosis

6.1 Unified diagnostic criteria

A. Defecation into inappropriate places (clothes, floor), voluntary or involuntary.

B. Frequency: ≥1 times per month, ≥3 months.

C. Developmental age ≥ 4 years.

D. Not fully explained by substance (laxative) or medical condition (Hirschsprung, anatomical anomaly, spinal pathology).

Qualifier: with/without constipation and overflow.

6.2 Differences between sources

- NASPGHAN/ESPGHAN 2014 — functional constipation guideline (Rome IV).
- NICE CG99 (2010) — constipation in children and adolescents.

6.3 Diagnostic algorithm

1. Clinical interview — defecation frequency, pain, toilet training history.
2. Abdominal palpation — fecal mass.
3. Rectal examination — fecal impaction (or other findings, if clinically indicated).
4. Defecation diary (Bristol Stool Chart)
5. Medical: thyroid (hypothyroid constipation), serum calcium, celiac disease screening.
6. Atypical presentation: ultrasound, anorectal manometry, suction biopsy (Hirschsprung).

6.4 Differential diagnosis

Condition	Distinguishing feature
Hirschsprung disease	Congenital constipation; narrow on rectal exam; biopsy aganglionosis.
Hypothyroidism	TSH.
Spinal pathology	Neurological signs; MRI.
Celiac disease	Anti-tTG.
Anal structural anomaly	Physical examination, USG.

7. Examination and assessment

- Abdominal and rectal examination.
- Bristol Stool, defecation diary.
- TSH, calcium, anti-TTG selectively.
- Ultrasound, manometry in atypical cases.

8. Treatment

8.1 General principles (NASPGHAN/ESPGHAN · NICE CG99)

1. Removal of impaction (disimpaction) — PEG (polyethylene glycol) high dose for 3–7 days; oral route preferred; enemas are not recommended — traumatic.
2. Maintenance — PEG 0.4 g/kg/day for months to years; keeps defecation regular and painless.
3. Behavioral intervention:
    - Regular toilet time (15–20 minutes after meals, gastrocolic reflex);
    - Correct foot support (defecation position);
    - Positive reinforcement (no punishment).
4. Dietary regimen: Increase fiber, water intake (moderate effect; does not replace PEG).
5. Psychological support: anxiety reduction, family training.
6. In refractory cases biofeedback, multidisciplinary approach; surgery rare (Malone antegrade colonic enema).

8.2 Treatment methods

1. PEG (Polyethylene Glycol) — Osmotic laxative; disimpaction 1–1.5 g/kg/day × 3 days; maintenance 0.4 g/kg/day; months-years; side effects minimal.
2. Toilet Time and Position — 5–10 minutes toilet time 15–20 minutes after meals; foot support — hip flexion straightens the anorectal angle and reduces straining; on videomanometry the angle during defecation is ~100° when sitting and ~126° when squatting (Sakakibara R. et al. LUTS 2010).
3. Positive Reinforcement (Star Chart) — Positive behavior reward instead of punishment.
4. Biofeedback — In refractory cases; anorectal coordination training.

8.3 Differences between sources

- NASPGHAN/ESPGHAN 2014 — PEG first-line; enema traumatic and second-line.
- NICE CG99 — same.
- Cochrane (Gordon M. et al. 2016) — PEG superior to lactulose.

9. Prognosis

- PEG and behavioral intervention lead to remission in most patients (50–80% within 1 year).
- Long-term maintenance therapy is recommended; early discontinuation creates relapse risk.

10. Myths and misconceptions

Myth 1: “Encopresis results from psychological ‘stubbornness’ or emotional disorder”

Evidence: Approximately 80% of cases are primarily due to constipation; medical/behavioral consultation is the first-line approach, not psychological. The patient's explanation of “intentional” behavior is incorrect.

Myth 2: “A child can be corrected through punishment”

Evidence: NASPGHAN/ESPGHAN — punishment is contraindicated; increases self-blame and distress.

Myth 3: “Enema is a first-line intervention”

Evidence: Oral PEG is superior; enema is traumatic, second-line.

Myth 4: “Dietary change alone is sufficient”

Evidence: Fiber/water mild adjunct effect; impaction correction and maintenance require PEG.

Myth 5: “Encopresis resolves with age, requires no treatment”

Evidence: Chronic progression without treatment, social-emotional burden; active intervention recommended.

Myth 6: “Probiotics cure encopresis”

Evidence: Cochrane — the significant effect of probiotics has not been proven.

11. Sources

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6C20 BODILY DISTRESS

Disorders of bodily distress or bodily experience

ICD-11 **6C20**	**6C20 BODILY DISTRESS DISORDER** **Bodily distress disorder**
ICD-10 F45.0	Somatization disorder
DSM-5-TR F45.1	Somatic Symptom Disorder

1. Definition and nosology

ICD-11: 6C20 Bodily Distress Disorder; DSM-5-TR: F45.1 Somatic Symptom Disorder — a psychiatric disorder characterized by excessive focus, worry, and behavioral interference created by somatic complaints (pain, fatigue, gastrointestinal or cardiopulmonary symptoms), accompanied by functional impairment. Clinical diagnosis is made not on the ‘inexplicability’ of symptoms, but on persistent preoccupation and distress (major change in DSM-5-TR update).

2. History

- Briquet P. (1859) — “hysteria” with multiple somatic symptoms.
- DSM-III (1980) — “Somatoform Disorders” group.
- DSM-5 (2013) — Fundamental change: requirement for “medically unexplained symptoms” removed; whether the somatic symptom is real or not is not a criterion — the patient's reaction is the diagnostic focus.
- ICD-11 (2019) — “Bodily Distress Disorder” — based on the European model on bodily distress (Fink P., Schröder A.).

3. Epidemiology

- In primary care: 5–15%.
- Sex: 2–3 times higher in females.
- Comorbidity: MDD, anxiety, panic, factitious, real medical illnesses.

- High healthcare utilization and costs.

4. Aetiology and pathogenesis

- Biopsychosocial model — biological vulnerability + cognitive-affective emotional components + social reinforcement.
- Trauma and childhood illness history.
- Cognitive — catastrophizing and hypervigilance to bodily sensations.

5. Clinical features

- Persistent somatic symptoms (pain, fatigue, gastrointestinal, cardiopulmonary).
- ≥ 1 of the following:
    - Excessive and persistent thoughts about the severity of symptoms;
    - High anxiety about health;
    - Excessive time and energy devoted to symptoms.
- Duration ≥ 6 months.
- Functional impairment.
- "Doctor shopping", repeated checking, avoidance behavior.

6. Diagnosis

6.1 Unified diagnostic criteria

A. One or more somatic symptoms — causing distress or functional impairment to the patient.

B. ≥ 1 of the following:

1. Excessive thoughts about the severity of symptoms;
2. High anxiety about health;
3. Excessive time/energy on symptoms.

C. Persistence ≥ 6 months.

6.2 Differences between sources

- DSM-5-TR — "with predominant pain" and severity specifiers.
- ICD-11 — 'Bodily Distress Disorder' — close to the European 'bodily distress syndrome' concept (Fink P.); severity (mild, moderate, severe).

- German S3 Guideline (Funktionelle Körperbeschwerden, 2018) — biopsychosocial approach.

6.3 Diagnostic algorithm

1. Clinical interview.
2. PHQ-15 (somatic symptom scale), SSS-8.
3. Medical assessment — exclusion of somatic illness (but avoid excessive testing).
4. Comorbidity (MDD, anxiety, factitious, real illnesses).

6.4 Differential diagnosis

Condition	Distinguishing feature
Hypochondriasis (6B23)	Illness preoccupation dominant; somatic symptoms minimal.
Real medical illness	Clinical and laboratory confirmation; however, somatic symptom disorder may be comorbid.
FND (6B60)	Specific neurological symptoms; “positive signs”.
Factitious (6D50)	Conscious lie; motivation for gain.
Malingering	Overt material/legal gain.
MDD somatic form	Affective symptoms predominate.

7. Examination and assessment

- PHQ-15, SSS-8, Whiteley Index (overlap).
- One-time comprehensive medical examination; then minimal repetition.
- Comorbidity scales.

8. Treatment

1. Biopsychosocial approach and stepped care — primary care physician at the centre, psychiatrist/psychologist adjunct.
2. Single physician model (single physician approach) — all complaints are managed through that physician.

3. CBT — cognitive restructuring, reduction of catastrophic interpretation of bodily sensations, behavioral activation. Evidence: van Dessel N. Cochrane 2014 (CD011142).
4. SSRI — for comorbid MDD/anxiety; duloxetine, amitriptyline when pain is dominant.
5. Structured limited checking — a single comprehensive examination of the patient, then limited 'reassurance'.
6. Mindfulness, physical activity, occupational rehabilitation.

8.1 Treatment methods

1. CBT for Somatic Symptom — Cognitive restructuring + behavioral intervention (avoidance reduction, exposure). van Dessel N. Cochrane 2014 (CD011142).
2. Single Physician Model — Structured visit; limited number of checks; sequential explanation.
3. PHQ-15, SSS-8 — Somatic complaint scales.
4. Stepped Care — CBT-based brief intervention at the primary care level; then specialized service.

8.2 Differences between sources

- German S3 Guideline — integrated care at the primary care level.
- NICE — no separate guideline for somatic symptom disorder; general principles.

9. Prognosis

- CBT and a multidisciplinary approach lead to significant improvement; chronic course.

10. Myths and misconceptions

Myth 1: "The patient is 'faking' symptoms"

Evidence: symptoms are real for the patient; somatic disorder is a clinical condition, not simulation.

Myth 2: "Saying 'nothing is wrong' after a medical exam will reassure the patient"

Evidence: Negative reassurance is short-term; maintains long-term anxiety. Structured explanation and biopsychosocial model are superior.

Myth 3: "Only a prolonged medical investigation will reveal the diagnosis"

Evidence: repeated checking increases somatic focus and costs; one-time comprehensive assessment, then limited.

Myth 4: "Somatic symptom disorder is only a 'mental illness'"

Evidence: biopsychosocial disorder; biological components (sensory sensitivity, gut-brain axis) are real.

Myth 5: "Antidepressant is the 'treatment' for somatic symptom disorder"

Evidence: SSRI for comorbid symptoms; CBT first-line.

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6C40–6C5Z SUBSTANCE-RELATED

Disorders due to substance use or addictive behaviours

ICD-11 **6C40**	**6C40 DISORDERS DUE TO USE OF ALCOHOL** **Disorders due to use of alcohol**
ICD-10 F10	Mental and behavioural disorders due to use of alcohol
DSM-5-TR F10.20	Alcohol Use Disorder, Moderate or Severe

1. Definition and nosology

Disorders due to use of alcohol (ICD-11: 6C40; DSM-5-TR: F10.20 Alcohol Use Disorder, AUD) — a group of disorders characterized by impaired control over alcohol use, continued use despite physical or psychological harm. ICD-11 subtypes: harmful pattern of use (6C40.1), alcohol dependence (6C40.2), intoxication (6C40.3), withdrawal syndrome (6C40.4), alcohol-induced delirium, psychotic, mood, anxiety disorders.

2. History

- Jellinek E.M. (1960) — ‘The Disease Concept of Alcoholism’ — alcoholism as a disease.
- DSM-IV — ‘Abuse’ and ‘Dependence’ as separate categories.
- DSM-5 (2013) — unified ‘Alcohol Use Disorder’ (mild, moderate, severe).
- ICD-11 — episodic harmful use or addiction pattern separated.

3. Epidemiology

- Lifetime prevalence: 14–29% (NIAAA NESARC); annual 5–14%.
- Sex: 2–3 times higher in males, but increasing in females.
- Mortality: 3 million deaths per year worldwide (WHO).
- Comorbidity: MDD, anxiety, PTSD, BPD, other substance use, liver disease, cardiovascular.

4. Aetiology and pathogenesis

- Heritability ~49% (95% CI 43–53) (Verhulst B. et al. Psychol Med 2015).
- Neurobiological — dopamine reward circuit, GABA, glutamate, opioid systems.
- Environment — peers, stress, alcohol availability, cultural norms.
- Genetic markers — ADH1B, ALDH2 (protective in Asian population).

5. Clinical features

5.1 Dependence syndrome (DSM-5-TR criteria, ≥ 2 out of 11)

- More/prolonged use;
- Attempts to reduce are unsuccessful;
- Most time spent obtaining, using, recovering from substance
- Craving;
- Work/school disruption;
- Continues despite social-interpersonal problem.
- Refusal of important activities;
- Use in a dangerous situation;
- Continuation despite physical/psychological problem;
- Tolerance;
- Withdrawal syndrome.

5.2 Intoxication

- Disinhibition, dysarthria, ataxia, nystagmus, cognitive impairment.
- Severe intoxication — coma, respiratory depression, hypoglycemia, death.

5.3 Withdrawal

- Mild: tremor, sweating, tachycardia, anxiety, nausea (6–24 hours after last drink).
- Moderate: autonomic hyperactivity, hallucinations (alcoholic hallucinosis).
- Severe: delirium tremens (DT) — 48–96 hours; altered consciousness, hallucinations, autonomic crisis; mortality ~1–4% among hospitalised patients (Schuckit M.A. N Engl J Med 2014); substantially higher in historical untreated series; seizures.

5.4 Complications

- Wernicke-Korsakoff (B1 deficiency), alcoholic liver disease, cardiomyopathy, polyneuropathy, FAS during pregnancy.

6. Diagnosis

6.1 Unified diagnostic criteria

DSM-5-TR — ≥2 of 11 criteria (mild 2–3, moderate 4–5, severe ≥6); ICD-11 — pattern of use (single episode of harmful use, harmful pattern of use, dependence); hazardous use belongs to QE10, not to 6C4x. ICD-11 duration requirement: harmful pattern of use — at least 12 months if use is episodic, at least 1 month if continuous; dependence — at least 12 months, or at least 3 months if use is continuous (daily or almost daily).

6.2 Differences between sources

- WHO AUDIT (Saunders 1993) — 10-item screening.
- NICE CG115 — diagnosis and treatment of alcohol disorders.
- SAMHSA TIP — international protocols.

6.3 Diagnostic algorithm

1. AUDIT, AUDIT-C screening at primary care level.
2. Clinical interview — TLFB (Timeline Followback) use pattern.
3. Laboratory: GGT, AST/ALT, MCV, CDT (carbohydrate-deficient transferrin), complete blood count.
4. Comorbidity (MDD, anxiety, PTSD, somatic illnesses).
5. Withdrawal risk (CIWA-Ar).

6.4 Differential diagnosis

Condition	Distinguishing feature
Other substance use	Toxicology screening.
MDD with self-medication	Affective symptoms are primary.
Bipolar disorder	Affective episodes.
Hyperthyroidism	TSH.
Seizure disorder	EEG.

7. Examination and assessment

- AUDIT, CIWA-Ar (withdrawal severity).
- GGT, MCV, CDT, transaminases.
- Thiamine level.
- EKG, cardiac USG (cardiomyopathy).
- Comorbidity screening.

8. Treatment

8.1 General principles (NICE CG115 · SAMHSA TIP 49 · APA 2018)

1. Detoxification:
    - Mild (CIWA-Ar <10) — outpatient;
    - Moderate-severe — inpatient (hospitalization);
    - Benzodiazepine (lorazepam, diazepam) — gold standard for withdrawal treatment;
    - Thiamine 100–300 mg IV/IM × 3–5 days (Wernicke prophylaxis — BEFORE glucose).
    - Hydration, electrolyte correction.
2. Pharmacotherapy for relapse prevention:
    - Naltrexone (oral 50 mg or IM Vivitrol 380 mg monthly) — first-line; reduces cravings.
    - Acamprosate (666 mg × 3 times/day) — first line; maintenance of abstinence.
    - Disulfiram — second-line; in motivated patient; aversive reaction.
    - Topiramate, gabapentin — off-label.
3. Psychosocial: Motivational Interviewing (MI), CBT, 12-Step facilitation (AA, NA), contingency management.
4. SAMHSA — co-occurring substance use disorder and psychiatric comorbidity (dual diagnosis).
5. Long-term relapse prophylaxis.

8.2 Treatment methods

1. Motivational Interviewing (MI) — Miller W.R., Rollnick S — Enhancing readiness for behavior change; targets “ambivalence”.
2. CBT for substance use — Trigger recognition, cravings management, coping skills, relapse prevention.

3. Naltrexone, Acamprosate, Disulfiram — Pharmacotherapy against relapse.
4. 12-Step Facilitation (AA, NA) — Project MATCH RCT — comparative effectiveness of CBT and MI.
5. AUDIT, CIWA-Ar — Screening and withdrawal severity.

8.3 Differences between sources

- NICE CG115 (2011).
- APA Practice Guideline for the Pharmacological Treatment of Patients with Alcohol Use Disorder (2018).
- SAMHSA TIP 49 (2009).

9. Prognosis

- With multimodal approach, 40–50% long-term remission.
- Comorbid MDD, BPD, other substance — poor prognosis.

10. Myths and misconceptions

Myth 1: “Alcoholism is a character weakness”

Evidence: Jellinek 1960; WHO — alcohol use disorder is a medical condition; heritability 50–60%.

Myth 2: “Detoxification alone is treatment”

Evidence: Detoxification is the initial step; intervention against relapse is required; detox alone leads to 90%+ relapse.

Myth 3: “Naltrexone makes alcohol ‘unacceptable’”

Evidence: Naltrexone reduces craving and diminishes alcohol's ‘reward’ effect; differs from disulfiram.

Myth 4: “The patient must hit ‘rock bottom’ before treatment begins”

Evidence: Early intervention is more effective; the “rock bottom” concept is not evidence-based.

Myth 5: “Alcohol is the safest substance, requires no treatment”

Evidence: WHO — alcohol causes 2.6 million deaths annually (WHO 2024, 2019 data); DT mortality is ~1–4% among hospitalised patients (Schuckit M.A. N Engl J Med 2014); substantially higher in historical untreated series.

Myth 6: "'A little alcohol is safe' in pregnant women"

Evidence: CDC, AAP — any amount of alcohol during pregnancy creates a risk of FAS; there is no 'safe' dose.

11. Sources

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ICD-11 **6C43**	**6C43 DISORDERS DUE TO USE OF OPIOIDS** **Disorders due to use of opioids**
ICD-10 F11	Mental and behavioural disorders due to use of opioids
DSM-5-TR F11.20	Opioid Use Disorder, Moderate or Severe

1. Definition and nosology

Disorders due to use of opioids (ICD-11: 6C43; DSM-5-TR: F11.20 Opioid Use Disorder, OUD) — impaired control over opioid use (heroin, morphine, oxycodone, fentanyl, etc.) or harmful pattern of use. High mortality (overdose) related to medical-criminal context.

2. History

- Dole V.P., Nyswander M. (1965) — initial studies of methadone maintenance therapy.
- 2010+ US opioid epidemic — starting with prescription opioids, leading to fentanyl overdose crisis.
- WHO, SAMHSA, NICE — opioid agonist maintenance therapy (OAT) is the gold standard.

3. Epidemiology

- Global: 40 mln+ patients (UNODC); US annual 80,000+ overdose deaths.
- Sex: 2 times higher in males.
- Comorbidity: MDD, anxiety, PTSD, BPD, HIV/HCV, cardiac.
- Mortality: in treated OUD patients, 10–20 times higher compared to the population.

4. Aetiology and pathogenesis

- Heritability 50%.
- Neurobiological — effect of the μ-opioid receptor system on the dopaminergic reward circuit.
- Environment — prescription opioid availability, socioeconomic factors.
- Pain syndromes — transition from prescription initiation to dependence.

5. Clinical features

- Dependence syndrome (same criteria as AUD).
- Intoxication — miosis, sedation, respiratory depression, hypotension, coma.
- Overdose — respiratory depression is main cause of death; naloxone antidote.

- Withdrawal — onset 8–24 hours (short-acting), 36–72 hours (long-acting methadone); rhinorrhea, lacrimation, mydriasis, anxiety, tachycardia, abdominal cramps, diarrhea, piloerection.
- Withdrawal is not life-threatening in adults (unlike alcohol/sedatives), but severe in neonates (NAS — Neonatal Abstinence Syndrome).

6. Diagnosis

6.1 Unified diagnostic criteria

DSM-5-TR: same 11-criteria structure as AUD (mild/moderate/severe). Unlike the single DSM-5-TR scale, ICD-11 uses three separate categories: episode of harmful use, harmful pattern of use, and dependence. ICD-11 duration requirement: harmful pattern of use — at least 12 months if use is episodic, at least 1 month if continuous; dependence — at least 12 months, or at least 3 months if use is continuous (daily or almost daily).

6.2 Differences between sources

- SAMHSA TIP 63 — Medications for Opioid Use Disorder.
- NICE CG52 — opioid dependence.
- WHO mhGAP.

6.3 Diagnostic algorithm

1. Clinical interview.
2. Toxicology screening (urine).
3. HIV, HCV, syphilis, pregnancy test.
4. Comorbidity and overdose history.
5. COWS (Clinical Opiate Withdrawal Scale) — withdrawal severity.

6.4 Differential diagnosis

Condition	Distinguishing feature
Chronic pain (without dependence)	Medical indication; pattern of use.
Other substance use (sedatives, alcohol)	Toxicology.
Functional GI/piloerection (gooseflesh) (discontinuation difference)	Medical evaluation.

7. Examination and assessment

- Toxicology screening.
- COWS scale.
- HIV, HCV, HBV, syphilis.
- EKG (before starting methadone — QTc).
- Pregnancy test.

8. Treatment

8.1 OAT — Opioid Agonist Therapy (gold standard)

1. Methadone (in a maintenance clinic, under daily supervision) — full μ-agonist; 60–120 mg/day; long half-life; QTc monitoring.
2. Buprenorphine/naloxone (Suboxone) — partial μ-agonist + naloxone; office-based prescription; low overdose potential; "induction" — started in mild withdrawal condition.
3. OAT significantly reduces mortality (Sordo L. et al. BMJ 2017;357:j1550 meta-analysis) and HIV transmission (MacArthur G.J. et al. BMJ 2012 meta-analysis).
4. OAT is superior to psychosocial intervention in mortality reduction; however, combination with psychosocial intervention (MI, CBT, contingency management, peer support, 12-step — see "Treatment methods") is optimal.

8.2 Antagonist

- Naltrexone (oral 50 mg or IM Vivitrol 380 mg monthly) — patient must be abstinent for 7–10 days before initiation; in motivated patients or in the absence of formal maintenance programs.

8.3 Overdose Management

- Naloxone — IM/IN; widespread; "take-home" programs (community-based) reduce mortality.

8.4 In Pregnancy

- Methadone or buprenorphine — OAT is continued during pregnancy; detox is contraindicated (risk of relapse and pregnancy loss).
- NAS intervention in the newborn — morphine or methadone.

8.5 Treatment methods

1. Methadone Maintenance Therapy (MMT) — Dole-Nyswander model; clinic-based.
2. Buprenorphine/Naloxone — Office-based; "Suboxone induction" protocol; the ceiling effect gives lower overdose potential.
3. Naloxone Take-Home Program — Community-based; antidote training for family.
4. Clinical Opiate Withdrawal Scale (COWS) — 11 items; crucial in induction.
5. Naltrexone IM (Vivitrol) — Monthly; in an abstinent patient.

8.6 Differences between sources

- SAMHSA TIP 63 (2021).
- NICE CG52 (opioid detoxification) and CG51 (psychosocial interventions).
- WHO — OAT is the gold standard, superior to abstinence-oriented approach.

9. Prognosis

- Long-term stabilization and mortality reduction with OAT.
- Comorbidity intervention important.

10. Myths and misconceptions

Myth 1: "OAT (methadone, buprenorphine) replaces one addiction with another"

Evidence: OAT is medical treatment — significantly reduces mortality, HIV, crime, social disruption; "sensitive" anti-OAT stance is not evidence-based. WHO, SAMHSA, NICE — OAT is the gold standard.

Myth 2: "Detoxification is sufficient, OAT is not needed"

Evidence: detox alone 90%+ relapse; overdose mortality highest immediately after detox (loss of tolerance). OAT reduces mortality by 50%+.

Myth 3: "Ultra-Rapid Opioid Detoxification (UROD) is effective"

Evidence: UROD associated with deaths and serious adverse effects; no evidence of efficacy; SAMHSA and NICE do not recommend.

Myth 4: “Detox is needed in a pregnant woman”

Evidence: detox in pregnancy is contraindicated; OAT (methadone or buprenorphine) is maintained.

Myth 5: “Ibogaine or ayahuasca cures opioid addiction”

Evidence: Not evidenced; ibogaine accompanied by cardiac arrhythmia and fatal cases (FDA warning); not in standard practice.

Myth 6: “Naltrexone is first-line for OUD”

Evidence: SAMHSA TIP 63 — methadone and buprenorphine are more effective in reducing mortality; naltrexone in motivated abstinent patients.

Myth 7: “Naloxone take-home programs encourage use”

Evidence: Walley A.Y. et al. BMJ 2013 — community naloxone programs significantly reduce overdose deaths; do not increase use.

11. Sources

1. WHO. ICD-11. 6C43 Disorders due to use of opioids. 2024.
2. APA. DSM-5-TR. 2022.
3. SAMHSA. TIP 63: Medications for Opioid Use Disorder. 2021.
4. NICE CG52; NICE CG51.
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ICD-11 **6C41**	**6C41 DISORDERS DUE TO USE OF CANNABIS** **Disorders due to use of cannabis**
ICD-10 F12	Mental and behavioural disorders due to use of cannabinoids

ICD-11 6C41	**6C41 DISORDERS DUE TO USE OF CANNABIS** **Disorders due to use of cannabis**
DSM-5-TR F12.20	Cannabis Use Disorder, Moderate or Severe

1. Definition and nosology

Disorders due to use of cannabis (ICD-11: 6C41; DSM-5-TR: F12.20) — impaired control over use of THC-containing cannabis and/or harmful use. Synthetic cannabinoids are coded separately in ICD-11 (6C42).

2. History

- DSM-III (1980) — Cannabis Dependence as an official diagnosis.
- DSM-5 (2013) — “cannabis withdrawal” added as a formal category.
- 2010+ — high-potency cannabis and synthetic cannabinoids (“Spice”, “K2”) epidemic.

3. Epidemiology

- Lifetime prevalence: 7–9% cannabis users develop addiction; risk 17% in those who start in adolescence.
- Sex: higher in males.
- Comorbidity: psychosis risk, anxiety, MDD, ADHD, other substance use.

4. Aetiology and pathogenesis

- Heritability 50–60%.
- Endocannabinoid system.
- Social and cultural factors; accessibility.

5. Clinical features

- Dependence syndrome (same criteria as AUD).
- Intoxication — red eyes (conjunctival injection), tachycardia, increased appetite (“munchies”), time perception impairment, paranoid feelings.

- Severe intoxication — cannabinoid hyperemesis syndrome (vomiting syndrome with chronic use), panic, psychosis.
- Withdrawal (DSM-5 new): irritability, anxiety, sleep disturbance, decreased appetite, mood lowering, somatic complaints; onset within 24–72 hours, duration 1–2 weeks.
- Synthetic cannabinoids ('Spice', "K2") — stronger, unpredictable; severe intoxication, acute psychosis, fatalities.

6. Diagnosis

6.1 Unified diagnostic criteria

DSM-5-TR — 11 criteria (AUD structure). Unlike the single DSM-5-TR scale, ICD-11 uses three separate categories: episode of harmful use, harmful pattern of use, and dependence. ICD-11 duration requirement: harmful pattern of use — at least 12 months if use is episodic, at least 1 month if continuous; dependence — at least 12 months, or at least 3 months if use is continuous (daily or almost daily).

6.2 Differences between sources

- NIDA — principles of cannabis use disorder.
- WHO mhGAP — assessment at the primary care level.

6.3 Diagnostic algorithm

1. Clinical interview.
2. CUDIT-R (Cannabis Use Disorders Identification Test-Revised).
3. Toxicology screening (urine THC).
4. Comorbid psychosis, anxiety, MDD.

6.4 Differential diagnosis

Condition	Distinguishing feature
Synthetic cannabinoid disorder (6C42)	Stronger psychotic and autonomic effect.
Psychosis (cannabis-induced or primary)	Depending on residual state after substance withdrawal.
Functional GI (cannabis hyperemesis)	Remission with discontinuation of use.

7. Examination and assessment

- CUDIT-R.
- Toxicology.
- Comorbidity (psychosis, anxiety, MDD).

8. Treatment

1. First-line psychosocial intervention — MI, CBT, contingency management. Evidence: Davis M.L. et al. Eval Health Prof 2015 meta-analysis.
2. No specific FDA-approved pharmacotherapy exists. Off-label: N-acetylcysteine (in adolescents Gray K.M. Am J Psychiatry 2012 RCT positive — only on a contingency management platform; in adults Gray K.M. Drug Alcohol Depend 2017 RCT negative), gabapentin.
3. Comorbid psychosis — antipsychotic; cannabis abstinence critical.
4. Synthetic cannabinoid intoxication — supportive, atypical antipsychotic if necessary.

8.1 Treatment methods

1. CBT and Motivational Interviewing (MI) — Trigger management, motivation, behavior change.
2. Contingency Management — Reward for negative toxicology tests.
3. CUDIT-R — 8-item screening.
4. N-acetylcysteine (NAC) — Early positive adolescent finding did not replicate (Cochrane 2025 RR 1.17; 95% CI 0.73–1.88) — experimental; off-label.

8.2 Differences between sources

- NIDA — CBT and contingency management first-line.
- WHO mhGAP.

9. Prognosis

- Significant improvement with psychosocial intervention.
- Early onset of use — poor prognosis.

10. Myths and misconceptions

Myth 1: “Cannabis is ‘natural’, not addictive”

Evidence: CUD develops in 7–9% of lifetime users; 17% in adolescents.

Myth 2: “Cannabis withdrawal syndrome does not exist”

Evidence: DSM-5 (2013) — withdrawal syndrome as an official category; irritability, sleep, appetite disturbance.

Myth 3: “Medical cannabis reduces psychosis risk”

Evidence: Marconi 2016 — THC raises psychosis risk in a dose-dependent way: about twofold at average use (OR 1.97) and about fourfold in the heaviest users (OR 3.90); preliminary evidence for CBD, not a clinical recommendation.

Myth 4: “Cannabis is not a ‘gateway drug’”

Evidence: “Gateway” concept is controversial, but sequential use pattern is statistically evident; causality has different explanations.

Myth 5: “High-THC cannabis is safer than regular cannabis”

Evidence: THC dose correlates with risk of psychosis, addiction, cognitive impairment; ‘potency creep’ over recent decades.

Myth 6: “Synthetic cannabinoids (‘Spice’) are safe”

Evidence: synthetic cannabinoids are very potent, unpredictable; acute psychosis and death cases have been recorded.

11. Sources

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5. Gray K.M. et al. A randomized placebo-controlled trial of N-acetylcysteine for cannabis use disorder in adults. Drug Alcohol Depend 2017;177:249–257.
6. Marconi A. et al. Schizophr Bull 2016;42(5):1262–1269.

7. Spiga F., Parkhouse T., Tang V.M. et al. Pharmacotherapies for cannabis use disorder. Cochrane Database Syst Rev 2025;9(9):CD008940.

	6C44 DISORDERS DUE TO USE OF SEDATIVES, HYPNOTICS OR ANXIOLYTICS
ICD-11 **6C44**	**Disorders due to use of sedatives, hypnotics or anxiolytics**
ICD-10 F13	Mental and behavioural disorders due to use of sedatives or hypnotics
DSM-5-TR F13.20	Sedative, Hypnotic, or Anxiolytic Use Disorder, Moderate or Severe

1. Definition and nosology

Disorders due to use of sedatives, hypnotics or anxiolytics (ICD-11: 6C44; DSM-5-TR: F13.20 Sedative, Hypnotic, or Anxiolytic Use Disorder) — impaired control over use of benzodiazepines, z-drugs (zolpidem, zopiclone), barbiturates, and other anxiolytics. Often begins with medical prescription in clinical practice; ‘iatrogenic dependence’ is common.

2. History

- 1960s — clinical introduction of benzodiazepines (chlordiazepoxide 1960, diazepam 1963).
- 1980s — recognition of the dependence potential of long-term use.
- NICE and APA — short-term use recommendation (2–4 weeks).

3. Epidemiology

- Long-term benzodiazepine use in the elderly: 10–20%; problem identification is low.
- Sex: higher prescription rates in women.
- Comorbidity: alcohol and opioid use (dangerous combination — overdose risk).

4. Aetiology and pathogenesis

- Iatrogenesis — long-term prescriptions.
- Adaptation to the GABA-A receptor system — tolerance and discontinuation.
- Self-medication pattern in patients with anxiety, sleep disturbance.

5. Clinical features

- Dependence syndrome — loss of control, tolerance, withdrawal.
- Intoxication — sedation, ataxia, dysarthria, disinhibition, anterograde amnesia.
- Overdose — life-threatening respiratory depression with opioids.
- Withdrawal — life-threatening (like alcohol): tremor, anxiety, tachycardia, hypertension; severe cases: seizures and DT-like syndrome.

6. Diagnosis

6.1 Unified diagnostic criteria

DSM-5-TR — 11 criteria (AUD structure). Unlike the single DSM-5-TR scale, ICD-11 uses three separate categories: episode of harmful use, harmful pattern of use, and dependence. ICD-11 duration requirement: harmful pattern of use — at least 12 months if use is episodic, at least 1 month if continuous; dependence — at least 12 months, or at least 3 months if use is continuous (daily or almost daily).

6.2 Differences between sources

- NICE — benzodiazepine prescription 2–4 weeks; long-term use should be reassessed.
- Beers Criteria — avoid benzodiazepines in the elderly.

6.3 Diagnostic algorithm

1. Clinical interview + prescription history.
2. Toxicology screening.
3. Comorbid alcohol and opioid use (especially overdose risk).
4. Clinical examination — withdrawal signs.

6.4 Differential diagnosis

Condition	Distinguishing feature
Real worry disorder	Comorbid; substance use is symptomatic.
Alcohol use	Frequently comorbid.
Opioid use	Comorbid; overdose risk high.

7. Examination and assessment

- Toxicology screening.
- Vital signs (discontinuation)

8. Treatment

1. Gradual discontinuation (taper) is the gold standard: conversion to equivalent dose of long-acting benzodiazepine (diazepam), then reduction by 10–25% per week; total 6–12 weeks (sometimes months). Rapid discontinuation life-threatening.
2. Hospitalisation — severe withdrawal or comorbid substance use.
3. Psychosocial intervention — CBT for anxiety or sleep disorder; this reduces need for pharmacotherapy.
4. Comorbid disorders treatment (anxiety, depression, sleep).
5. Prescription management — at the primary healthcare level.

8.1 Treatment methods

1. Ashton Manual Taper — Conversion to diazepam equivalent, then gradual reduction; family support.
2. CBT for Anxiety and Sleep — Alternative to benzodiazepine use; CBT-I for sleep.
3. Toxicological Monitoring — During maintenance period.

8.2 Differences between sources

- NICE, BNF — taper protocols.
- Ashton Manual — practical clinical guide by Heather Ashton (C.H. Ashton); 1999 edition, revised 2002.

9. Prognosis

- Significant improvement with gradual taper and CBT.
- In older adults, discontinuation is both medically and psychologically difficult.

10. Myths and misconceptions

Myth 1: "Benzodiazepines can be taken safely for life"

Evidence: long-term use — tolerance, dependence, cognitive impairment, fall risk (in elderly). NICE — short-term use.

Myth 2: "Rapid discontinuation is safe"

Evidence: Rapid discontinuation seizure and death risk; gradual taper critical.

Myth 3: "Z-drugs (zolpidem) are not addictive"

Evidence: Z-drugs also cause dependence; AAFP and NICE recommend short-term use.

Myth 4: "Benzodiazepine + opioid is safe"

Evidence: FDA black box warning — this combination significantly increases risk of respiratory depression and death.

Myth 5: "Detoxification is sufficient"

Evidence: After discontinuation, alternative treatment (CBT, anxiety management) is required.

11. Sources

1. WHO. ICD-11. 6C44 Disorders due to sedative, hypnotic, anxiolytic. 2024.
2. APA. DSM-5-TR. 2022.
3. NICE Quick Reference Guide. Benzodiazepines.
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5. FDA Drug Safety Communication. Opioid pain or cough medicines combined with benzodiazepines: serious side effects. 2016.

ICD-11 6C45	**6C45 DISORDERS DUE TO USE OF COCAINE** **Disorders due to use of cocaine**
ICD-10 F14	Mental and behavioural disorders due to use of cocaine
DSM-5-TR F14.20	Stimulant Use Disorder, Cocaine, Moderate or Severe

1. Definition and nosology

Disorders due to use of cocaine (ICD-11: 6C45; DSM-5-TR: F14.20) — impaired control over cocaine use or harmful use. Includes intoxication, withdrawal, harmful pattern of use, dependence, induced psychotic and mood disorders.

2. History

- Coca leaves have been used in the Andes for thousands of years.
- Niemann A. (1860) — cocaine isolation.
- Freud (1884) — "Über Coca" — later criticized the clinical effect.
- 1970s–1980s — 'crack' epidemic in the USA.

3. Epidemiology

- Annual global users 21 mln (UNODC).
- Sex: 2 times higher in males.
- Comorbidity: alcohol, opioids, MDD, BPD, psychosis, cardiovascular mortality.

4. Aetiology and pathogenesis

- Heritability 40–50%.
- Neurobiological — blockade of dopamine reuptake → increased mesolimbic reward; tolerance and down-regulation.
- Social-cultural — peer, accessibility, socio-economic.

5. Clinical features

- Dependence syndrome (DSM-5-TR criteria).
- Intoxication — euphoria, hypervigilance, mydriasis, tachycardia, hypertension, hyperthermia; in severe cases, paranoid delusions, hallucinations, agitation, myocardial infarction, stroke, seizures.
- Withdrawal — "crash" — depressive mood, anhedonia, fatigue, hypersomnia, increased appetite, intense cravings; high suicide risk.

6. Diagnosis

6.1 Unified diagnostic criteria

DSM-5-TR 11 criteria (AUD structure). Unlike the single DSM-5-TR scale, ICD-11 uses three separate categories: episode of harmful use, harmful pattern of use, and dependence. ICD-11 duration requirement: harmful pattern of use — at least 12 months if use is episodic, at least 1 month if continuous; dependence — at least 12 months, or at least 3 months if use is continuous (daily or almost daily).

6.2 Differences between sources

- SAMHSA TIP 33 — stimulant use.

6.3 Diagnostic algorithm

1. Clinical interview + toxicology.
2. EKG, troponin (especially if chest pain).
3. HIV, HCV, syphilis.
4. Comorbidity and suicide risk (high during withdrawal).

6.4 Differential diagnosis

Condition	**Distinguishing feature**
Amphetamine/stimulant (6C46)	Toxicology differential.
Manic episode	Substance history and temporal relationship.
Psychosis (cocaine-induced and primary)	Remission after substance elimination.
Hyperthyroidism, pheochromocytoma	Endocrine tests.

7. Examination and assessment

- Toxicology.
- EKG, troponin, cardiac assessment.
- HIV, HCV.
- C-SSRS.

8. Treatment

1. Psychosocial — first line:
    - Contingency management — strongest evidence base (Petry N.M. reviews).
    - CBT;
    - MI;
    - Community Reinforcement Approach (CRA).
2. Pharmacotherapy — No medication is approved by the FDA for this indication. Off-label studies — disulfiram, modafinil, topiramate — mixed results.
3. Comorbidity treatment.
4. Acute intoxication — benzodiazepine (lorazepam); cardiac monitoring; antipsychotic in psychosis.

8.1 Treatment methods

1. Contingency Management — Petry (Petry N.M.), Higgins (Higgins S.) — Voucher/reward for negative toxicology tests; evidence base strongest for cocaine.
2. CBT and Community Reinforcement Approach (CRA) — Trigger management, alternative rewarding activities.
3. MI — Motivation for behavior change.

8.2 Differences between sources

- SAMHSA TIP 33 — contingency management gold standard.
- NIDA — no specific pharmacotherapy for cocaine.

9. Prognosis

- Significant improvement with contingency management.
- Comorbid substance use and psychiatric disorder poor prognosis.

10. Myths and misconceptions

Myth 1: "Cocaine is a 'recreational' substance, not addictive"

Evidence: Cocaine has high addiction potential; cardiovascular mortality, psychosis, social impairment.

Myth 2: "FDA-approved treatment for cocaine exists"

Evidence: none yet; psychosocial intervention first-line.

Myth 3: "Cocaine 'crash' is safe, requires no treatment"

Evidence: Suicide risk is high during the withdrawal period; active monitoring is required.

Myth 4: "Crack and cocaine powder are different disorders"

Evidence: Same substance (cocaine); route of administration (smoked vs intranasal) alters pharmacokinetics and addiction speed.

Myth 5: "The 'crack baby' concept in pregnancy — lasting harm in children"

Evidence: latest reviews — long-term effects of prenatal cocaine exposure are less than initially thought; socio-economic factors have a stronger impact (Frank D.A. JAMA 2001 review).

11. Sources

1. WHO. ICD-11. 6C45 Disorders due to use of cocaine. 2024.
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3. SAMHSA. TIP 33: Treatment for Stimulant Use Disorders. 2021 update.
4. NIDA. Research Report: Cocaine. 2016.
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7. Higgins S.T., Budney A.J., Bickel W.K. et al. Incentives improve outcome in outpatient behavioral treatment of cocaine dependence. Arch Gen Psychiatry 1994;51(7):568–576.

ICD-11 **6C46**	**6C46 DISORDERS DUE TO USE OF STIMULANTS INCLUDING AMPHETAMINES, METHAMPHETAMINE OR METHCATHINONE** **Disorders due to use of stimulants including amphetamines, methamphetamine or methcathinone**
ICD-10 F15	Mental and behavioural disorders due to use of other stimulants, including caffeine
DSM-5-TR F15.20	Stimulant Use Disorder, Amphetamine-Type Substance, Moderate or Severe

1. Definition and nosology

Disorders due to use of stimulants including amphetamines, methamphetamine or methcathinone (ICD-11: 6C46; DSM-5-TR: F15.20 Stimulant Use Disorder) — disorders developing from use of amphetamine, methamphetamine, methcathinone and other stimulants. Coded separately: cocaine (6C45), synthetic cathinones including mephedrone (6C47), caffeine (6C48), MDMA or related drugs including MDA (6C4C).

2. History

- Amphetamine first synthesized by Edeleano L. (1887); widespread use in World War II.
- 1950s–1960s — pharmacy-prescription epidemic.
- Methamphetamine epidemic since the 1990s.
- “Bath salts” (synthetic cathinones) — epidemic of 2010+. “Krokodil” (desomorphine) is an opioid and is coded separately in ICD-11 (6C43).

3. Epidemiology

- Global users: amphetamine 30 mln+ (UNODC).
- Sex: higher in males.
- Comorbidity: psychosis, MDD, anxiety, cardiovascular, skin lesions (self-excoriation, ‘meth mouth’).

4. Aetiology and pathogenesis

- Heritability 40–50%.
- Neurobiological — release of dopamine and norepinephrine and blockade of their reuptake; severe neurotoxicity (especially methamphetamine — to dopamine neurons).

5. Clinical features

- Intoxication — euphoria, hypervigilance, mydriasis, tachycardia, hypertension, hyperthermia; in severe cases: paranoid psychosis, agitation, myocardial infarction, stroke, seizures, rhabdomyolysis.
- Withdrawal — “crash” — depressive mood, anhedonia, fatigue, hypersomnia, increased appetite, intense cravings; high suicide risk.
- Chronic use — psychosis (paranoid delusions, hallucinations), cognitive deficit, “meth mouth” (tooth decay), skin picking.
- MDMA (6C4C) — hyponatremia, hyperthermia, serotonin syndrome, neurotoxicity.

6. Diagnosis

6.1 Unified diagnostic criteria

DSM-5-TR 11 criteria (AUD structure). Unlike the single DSM-5-TR scale, ICD-11 uses three separate categories: episode of harmful use, harmful pattern of use, and dependence. ICD-11 duration requirement: harmful pattern of use — at least 12 months if use is episodic, at least 1 month if continuous; dependence — at least 12 months, or at least 3 months if use is continuous (daily or almost daily).

6.2 Differences between sources

- SAMHSA TIP 33 — stimulant use.
- NIDA — methamphetamine research reports.

6.3 Diagnostic algorithm

1. Clinical interview.
2. Toxicology.
3. EKG, troponin.
4. Comorbidity (psychosis, MDD, cardiovascular).

6.4 Differential diagnosis

Condition	Distinguishing feature
Cocaine (6C45)	Toxicology.
Manic episode	Substance temporal relationship.
Schizophrenia	No remission after substance elimination.
Hyperthyroidism, pheochromocytoma	Endocrine.

7. Examination and assessment

- Toxicology.
- EKG, troponin, CPK (rhabdomyolysis).
- Dental and skin examination.
- C-SSRS.

8. Treatment

1. Psychosocial — first line:
    - Contingency management;
    - CBT;
    - MI;
    - Matrix Model — manualized comprehensive program for amphetamine dependence.
2. Pharmacotherapy — No specific FDA-approved medication exists; off-label: mirtazapine, naltrexone + bupropion (Trivedi M.H. NEJM 2021 — ADAPT-2 trial combination for methamphetamine — modest effect).
3. Acute intoxication — benzodiazepine for agitation; antipsychotic in psychosis.
4. Comorbid psychosis — parallel treatment.

8.1 Treatment methods

1. Matrix Model — 16-week structured program — CBT + family + 12-step + single physician. Evidence base for amphetamine/methamphetamine.
2. Contingency Management — Reward for negative test.
3. Naltrexone + Bupropion (ADAPT-2) — Primary RCT evidence in methamphetamine addiction; off-label.

8.2 Differences between sources

- SAMHSA TIP 33.
- NIDA.
- Trivedi M.H. et al. NEJM 2021 — initial evidence base for naltrexone + bupropion in methamphetamine dependence.

9. Prognosis

- With multimodal approach, remission 30–50%.
- Comorbid psychosis indicates a poor prognosis.

10. Myths and misconceptions

Myth 1: "Methamphetamine is safer than 'Spice' or other 'designer drugs'"

Evidence: Methamphetamine induces high neurotoxicity, psychosis, and mortality.

Myth 2: "MDMA is safe as a 'pure' substance"

Evidence: MDMA — hyperthermia, hyponatremia, serotonin syndrome; 'pure' substance is typically adulterated (synthetic cathinones, fentanyl).

Myth 3: "Detoxification is sufficient for stimulant dependence"

Evidence: Contingency management and CBT-based; sole detox leads to relapse >90%.

Myth 4: "Antidepressants cure stimulant addiction"

Evidence: single antidepressant ineffective; ADAPT-2 provides preliminary evidence for combination.

Myth 5: "Stimulant users are 'productive' and do not require intervention"

Evidence: Short-term performance boost illusion; long-term cognitive, psychiatric, cardiac complications.

11. Sources

1. WHO. ICD-11. 6C46 Disorders due to use of stimulants. 2024.
2. APA. DSM-5-TR. 2022.
3. SAMHSA. TIP 33. 2021.
4. NIDA. Methamphetamine Research Report. 2019.
5. Trivedi M.H. et al. Bupropion and Naltrexone in Methamphetamine Use Disorder. NEJM 2021;384(2):140–153.
6. Rawson R.A. et al. Matrix Model. SAMHSA.

ICD-11 **6C49**	**6C49 DISORDERS DUE TO USE OF HALLUCINOGENS** **Disorders due to use of hallucinogens**
ICD-10 F16	Mental and behavioural disorders due to use of hallucinogens
DSM-5-TR F16.20	Other Hallucinogen Use Disorder, Moderate or Severe

1. Definition and nosology

Disorders due to use of hallucinogens (ICD-11: 6C49; DSM-5-TR: F16.20 Hallucinogen Use Disorder) — disorders related to use of LSD, psilocybin, mescaline, DMT, ayahuasca, and similar substances. Dissociative drugs — ketamine and phencyclidine (PCP) — are coded separately in ICD-11 (6C4D).

2. History

- Hofmann A. (1943) — accidental discovery of LSD's hallucinogenic effect.
- 1950s–1960s — clinical research (depression, schizophrenia).
- 1970 — Controlled Substances Act in the USA, Schedule I.
- 2020+ — psilocybin and ketamine “psychedelic therapy” research renaissance.

3. Epidemiology

- Dependence is relatively rare (unlike other substances); typical use is episodic.
- Hallucinogen Persisting Perception Disorder (HPPD) — flashback-like experiences.

4. Aetiology and pathogenesis

- Effect on serotonergic (5-HT2A) or NMDA (ketamine, PCP) receptors.
- Environment, peers, experimentation.

5. Clinical features

- Intoxication — visual hallucinations, synesthesia, time distortion, depersonalization, derealization, “bad trip” (panic, paranoia).
- In severe cases — psychosis, fatal accidents.
- “Persisting Perception Disorder” (HPPD) — persistent visual phenomena (flashbacks) after use.
- Ketamine — dissociation, “K-hole”; chronic use — urinary bladder syndrome.
- PCP — aggressive behavior, analgesia, disorganized behavior.

6. Diagnosis

6.1 Unified diagnostic criteria

DSM-5-TR 11 criteria; HPPD separate diagnosis (DSM-5). Unlike the single DSM-5-TR scale, ICD-11 uses three separate categories: episode of harmful use, harmful pattern of use, and dependence. ICD-11 duration requirement: harmful pattern of use — at least 12 months if use is episodic, at least 1 month if continuous; dependence — at least 12 months, or at least 3 months if use is continuous (daily or almost daily).

6.2 Differences between sources

- NIDA, SAMHSA.

6.3 Diagnostic algorithm

1. Clinical interview.
2. Toxicology (LSD not in standard panels).

3. Comorbid psychosis, anxiety, MDD.

6.4 Differential diagnosis

Condition	Distinguishing feature
Schizophrenia	Persistent symptoms.
Depersonalization-derealization disorder (6B66)	Reality testing preserved.
Seizure	EEG.

7. Examination and assessment

- Toxicology.
- Comorbidity scales.

8. Treatment

1. Acute intoxication (“bad trip”) — calm environment, benzodiazepine (lorazepam); antipsychotic (haloperidol) only in severe psychotic state; caution.
2. Dependence rare; psychosocial intervention (MI, CBT).
3. HPPD — antipsychotics usually ineffective; clonazepam helps some patients; long-term monitoring.
4. Psilocybin and ketamine therapeutic research (refractory MDD, PTSD) — used under supervision in research protocols; approved protocols in standard clinical practice are still limited (FDA esketamine Spravato is an exception).

8.1 Treatment methods

1. “Bad trip” management — Calm environment, reassurance, benzodiazepine as needed.
2. Clonazepam for HPPD — Off-label; evidence base small.
3. Psychedelic-assisted therapy (research phase) — Psilocybin + psychotherapy, ketamine + psychotherapy; in clinical trial protocols.

8.2 Differences between sources

- NIDA — psychedelic research renaissance; clinical recommendation still pending.

9. Prognosis

- Most users do not develop dependence.
- Chronic course in patients with severe psychotic or HPPD complications.

10. Myths and misconceptions

Myth 1: "Hallucinogens are not addictive, they are completely safe"

Evidence: Physical dependence rare, but psychological dependence and severe psychotic episodes possible; HPPD is a long-term complication.

Myth 2: "Psilocybin therapy is approved for all depression"

Evidence: still under research phase; not fully approved into standard clinical practice. FDA 'Breakthrough Therapy' status, but broad recommendation premature.

Myth 3: "Ayahuasca is safe for religious ritual"

Evidence: serotonin syndrome (interaction with SSRI), psychotic decompensation cases have been reported.

Myth 4: "Ketamine is a 'club drug', no medical use"

Evidence: Ketamine used as anesthetic for decades; intranasal esketamine (Spravato) FDA approved in 2019 for severe refractory depression.

Myth 5: "LSD 'flashback' is a myth"

Evidence: HPPD DSM-5 official diagnosis; some patients have persistent visual disturbances.

11. Sources

1. WHO. ICD-11. 6C49 Disorders due to use of hallucinogens. 2024.
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ICD-11 6C4A	**6C4A DISORDERS DUE TO USE OF NICOTINE** **Disorders due to use of nicotine**
ICD-10 F17	Mental and behavioural disorders due to use of tobacco
DSM-5-TR F17.200	Tobacco Use Disorder, Moderate or Severe

1. Definition and nosology

Disorders due to use of nicotine (ICD-11: 6C4A; DSM-5-TR: F17.200 Tobacco Use Disorder) — impaired control over use of cigarettes, tobacco products, e-cigarettes, or other nicotine substances. Leading preventable cause of mortality worldwide.

2. History

- 1964 — Surgeon General Report (USA) — recognition of tobacco as a cause of disease.
- DSM-IV — Nicotine Dependence; DSM-5 — Tobacco Use Disorder.
- 2003 — WHO FCTC (Framework Convention on Tobacco Control).
- 2010+ — e-cigarette epidemic, “EVALI” (e-cigarette/vaping product use-associated lung injury).

3. Epidemiology

- Global: 1.3 bln users; annual 8 mln+ deaths (WHO).
- Sex: higher in males (but decreasing); increase observed in females in some regions.
- Comorbidity: MDD, schizophrenia (~60–80% of patients smoke), anxiety, other substance use.
- Tobacco — cancer, COPD, cardiovascular, stroke, perinatal complications.

4. Aetiology and pathogenesis

- Heritability 50%.
- Neurobiological — nicotine α4β2 nicotinic acetylcholine receptors; increased dopamine reward.

- Social-cultural, peer, marketing.

5. Clinical features

- Dependence syndrome.
- Withdrawal — irritability, anxiety, concentration difficulty, appetite, sleep disturbance, depressive mood; onset within 24 hours, peak 2–3 days, duration 2–4 weeks.
- E-cigarette — nicotine dependence + EVALI risk.

6. Diagnosis

6.1 Unified diagnostic criteria

DSM-5-TR 11 criteria (AUD structure). Unlike the single DSM-5-TR scale, ICD-11 uses three separate categories: episode of harmful use, harmful pattern of use, and dependence. ICD-11 duration requirement: harmful pattern of use — at least 12 months if use is episodic, at least 1 month if continuous; dependence — at least 12 months, or at least 3 months if use is continuous (daily or almost daily).

6.2 Differences between sources

- USPSTF — screening all adult patients for tobacco use; 5A approach (Ask, Advise, Assess, Assist, Arrange) for smokers.
- NICE NG209 (Tobacco: preventing uptake, promoting quitting and treating dependence, 2021).

6.3 Diagnostic algorithm

1. Clinical interview (5A).
2. Fagerström Test for Nicotine Dependence (FTND).
3. Expired CO monitoring (motivation).
4. Comorbid cardiac and respiratory assessment.

6.4 Differential diagnosis

Differentiation among numerous tobacco products; electronic cigarette nicotine addiction can be presented in multiple ways.

7. Examination and assessment

- FTND.
- Expired-air CO monitor.

- Spirometry (COPD screening).

8. Treatment

8.1 General Principles (USPSTF · NICE NG209 · APA 2010)

1. 5A model (Ask, Advise, Assess, Assist, Arrange).
2. First-line pharmacotherapy:
    - NRT — Nicotine Replacement Therapy (patches, gum, lozenge, inhaler, spray) — combination (long-acting + short-acting) is superior to monotherapy.
    - Varenicline (Chantix) — α4β2 partial agonist; most effective single agent (Cahill K. Cochrane 2013 meta-analysis).
    - Bupropion SR (Zyban) — dopaminergic and noradrenergic effect.
3. Behavioral intervention — CBT, MI, telephone ‘quitline’ service; intensity increases effect.
4. Combination — pharmacotherapy + behavioral therapy is superior.
5. E-cigarettes as a smoking cessation aid are controversial — effective in some studies (Hajek P. NEJM 2019), but long-term safety and gateway risk are being investigated; NRT is preferred.
6. Comorbidity (MDD, schizophrenia) — tobacco cessation does not worsen psychiatric condition, but improves it (Taylor G. BMJ 2014 meta-analysis).

8.2 Treatment methods

1. 5A Model (Ask, Advise, Assess, Assist, Arrange) — Ask (inquire), Advise (recommend), Assess (evaluate), Assist (support), Arrange (organize follow-up).
2. Nicotine Replacement Therapy Combination (NRT) — Long-acting patch + short-acting (cheek, lozenge) ‘rescue.’
3. Varenicline — 1 mg × 2/day; 12 weeks standard.
4. Bupropion SR — 150 mg × 2/day; seizure risk.
5. CBT and Motivational Interviewing (MI) — Trigger management, motivation enhancement.
6. Telephone Support Line (Quitline) — Community-based support; evidence base extensive.

8.3 Differences between sources

- USPSTF 2021 — recommends intervention for all cigarette smokers.

- NICE NG209 — active intervention also in comorbid psychiatric disorders.
- Cahill K. et al. Cochrane 2013 — varenicline is most effective.

9. Prognosis

- With multimodal intervention, 6-month abstinence is 25–35%.
- Smoking cessation significantly reduces mortality and illness risk.

10. Myths and misconceptions

Myth 1: "Tobacco is not addictive and can be quit voluntarily"

Evidence: Nicotine has high addiction potential; voluntary cessation failure rate is high (~5% after three months); pharmacotherapy increases effect 2–3 times.

Myth 2: "Weight gain after quitting smoking is harmful to health"

Evidence: Average 4–5 kg weight gain, but risk of continued tobacco use significantly higher; weight can be managed.

Myth 3: "Varenicline increases suicide risk"

Evidence: initial FDA warning later reevaluated with EAGLES trial (Anthenelli R.M. Lancet 2016) — psychiatric adverse event rate not significantly higher than placebo; FDA boxed warning removed in 2016.

Myth 4: "A psychiatric patient cannot quit tobacco and should not"

Evidence: Taylor BMJ 2014 — smoking cessation improves psychiatric symptoms; cessation effective in schizophrenia, MDD patients.

Myth 5: "E-cigarettes are safer than tobacco"

Evidence: Current evidence indicates that electronic cigarettes are less harmful than combusted tobacco, but are not "safe"; EVALI, cardiac, respiratory risks; gateway to nicotine addiction in adolescents.

Myth 6: "Once 'cut and return' prohibition"

Evidence: Most cigarette smokers make 6–11 attempts to quit; each relapse temporary and ongoing attempt plan recommended.

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ICD-11 **6C50**	**6C50 GAMBLING DISORDER** **Gambling disorder**
ICD-10 F63.0	Pathological gambling
DSM-5-TR F63.0	Gambling Disorder

1. Definition and nosology

Gambling disorder (ICD-11: 6C50; ICD-10: F63.0; DSM-5-TR: F63.0) is a persistent or recurrent pattern of gambling behaviour characterised by impaired control, increasing priority given to gambling over other interests, and continuation despite negative consequences. It causes significant functional impairment. Gaming is a separate ICD-11 category — 6C51 Gaming disorder.

2. History

- DSM-IV (1994) — “Pathological Gambling” within the impulse control disorders.
- DSM-5 (2013) — “Gambling Disorder” was moved to the substance use and addiction category (similar neurobiology).

- ICD-11 (2019) — the behavioural addictions block was expanded: alongside gambling (6C50), gaming disorder (6C51) was added as a separate category.

3. Epidemiology

- Gambling Disorder lifetime prevalence: 0.2–5.3% (wide range).
- Sex: gambling is more prevalent in men; the sex difference narrows in online gambling.
- Comorbidity: MDD, anxiety, other substance use, ADHD, BPD, suicide risk.

4. Aetiology and pathogenesis

- Heritability 50–60%.
- Neurobiological — dysregulation of the reward circuit (dopamine), similar to substance use disorders.
- Personality — impulsivity, sensation-seeking.

5. Clinical features

- Persistent/recurrent gambling behaviour.
- Loss of control.
- Occupational, family, social, and academic impairment.
- “Chasing losses” (in gambling) — attempts to win back losses.
- Lying, financial disruption, concealment.
- Comorbid MDD and high suicide risk.

6. Diagnosis

6.1 Unified diagnostic criteria

Gambling (DSM-5-TR): ≥ 4 of 9 symptoms within a 12-month period:

1. Gambling with increasing amounts — tolerance;
2. Irritability or restlessness when cutting down gambling — withdrawal;
3. Unsuccessful attempts to cut down;
4. Preoccupation with gambling;
5. Gambling when distressed;
6. “Chasing” after losses;
7. Lying;
8. Loss of relationships/job;

9. Reliance on financial bailouts.

6.2 Differences between sources

- ICD-11 (6C50) — subtypes: 6C50.0 predominantly offline, 6C50.1 predominantly online, 6C50.Z unspecified. Diagnosis normally requires ≥ 12 months; the duration may be shortened if all diagnostic requirements are met and symptoms are severe.
- DSM-5-TR — "Gambling Disorder" (F63.0) is the only behavioural addiction in the main text; DSM-5 (2013) moved it from impulse-control disorders to substance-related and addictive disorders. Unlike 6C51, the 6C50 requirements admit significant distress alongside functional impairment.

6.3 Diagnostic algorithm

1. Clinical interview.
2. SOGS (South Oaks Gambling Screen) or PGSI (Problem Gambling Severity Index).
3. Comorbid substance use, MDD, suicide.
4. Assessment of the financial situation.

6.4 Differential diagnosis

Condition	Distinguishing feature
Manic episode (gambling disinhibition)	Affective symptoms.
OCD (compulsive checking)	Obsessions are primary.
BPD (impulsivity)	A persistent pattern across all domains.
Social gambling (non-problematic)	No functional impairment.

7. Examination and assessment

- Comorbidity scales.
- C-SSRS for suicide.

8. Treatment

1. CBT — first line; trigger management, behavioural activation, cognitive restructuring ("near miss" and "gambler's fallacy"). Cowlishaw S. Cochrane 2012 meta-analysis.

2. Naltrexone, nalmefene — the evidence base is evolving (reviews by Grant J.E.).
3. SSRIs — for comorbid MDD/anxiety.
4. Self-help (Gamblers Anonymous, Gam-Anon) — adjunct.
5. Financial counselling — critical in gambling.

8.1 Treatment methods

1. CBT for gambling — Correction of cognitive distortions, exposure (reducing responses to triggers), problem-solving.
2. Naltrexone off-label — Reduction of cravings; 50–150 mg/day.
3. Self-help groups (GA) — 12-step model.

9. Prognosis

- Remission with CBT 30–50%.
- Comorbid MDD predicts a poor prognosis.

10. Myths and misconceptions

Myth 1: "Gambling addiction is a 'character' disorder"

Evidence: DSM-5, ICD-11 — it is a medical/behavioural disorder; neurobiological and genetic predisposition.

Myth 2: "The patient will get out of debt with a 'big win'"

Evidence: "chasing losses" is a diagnostic feature of gambling disorder; the pattern of losses is statistically inevitable.

Myth 3: "Financial support alone is sufficient"

Evidence: reliance on bailouts is a diagnostic feature; psychosocial intervention is required.

Myth 4: "The patient will 'grow out of it'"

Evidence: active intervention is required; risk of chronicity.

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ICD-11 **6C51**	**6C51 GAMING DISORDER** **Gaming disorder**
ICD-10 —	No ICD-10 equivalent
DSM-5-TR —	Internet Gaming Disorder (Section III, condition for further study)

1. Definition and nosology

Gaming disorder (ICD-11 code 6C51) is defined by the World Health Organization as a pattern of persistent or recurrent gaming behaviour — digital gaming or video-gaming, either online or offline — manifested by three core features:

- impaired control over gaming (onset, frequency, intensity, duration, termination, context);
- increasing priority given to gaming to the extent that gaming takes precedence over other life interests and daily activities;
- continuation or escalation of gaming despite the occurrence of negative consequences.

The behaviour pattern must be of sufficient severity to result in significant impairment in personal, family, social, educational, occupational or other important areas of functioning. Note: ICD-11 does not contain a distress criterion for 6C51. Impairment is the obligatory element, and adding “marked distress” as an alternative route into the diagnosis would remove the very feature that gives the category its threshold. The pattern is normally evident over a period of at least 12 months, although the required duration may be shortened if all diagnostic requirements are met and symptoms are severe.

Nosological position

6C51 belongs to the ICD-11 grouping Disorders due to addictive behaviours (6C50–6C5Z), alongside 6C50 Gambling disorder. These are the only two behavioural

addictions with dedicated ICD-11 categories; the residual codes 6C5Y (other specified) and 6C5Z (unspecified) exist for other candidate conditions. ICD-11 was adopted unanimously by the 72nd World Health Assembly on 25 May 2019 and entered into force on 1 January 2022.

Subtypes and exclusions

- 6C51.0 — Gaming disorder, predominantly online.
- 6C51.1 — Gaming disorder, predominantly offline.
- 6C51.Z — Gaming disorder, unspecified.

The exclusions listed under 6C51 are Hazardous gaming (QE22), Bipolar type I disorder (6A60) and Bipolar type II disorder (6A61). Excessive gaming occurring exclusively within a manic or hypomanic episode is therefore not coded as 6C51.

The sub-threshold category QE22

QE22 Hazardous gaming, in the ICD-11 chapter on factors influencing health status, denotes a pattern of gaming, online or offline, that appreciably increases the risk of harmful physical or mental health consequences to the individual or to others around the individual, but that does not meet the diagnostic requirements for gaming disorder. It exists precisely so that a pattern warranting professional attention and advice need not be forced into a disorder diagnosis.

The American Psychiatric Association has not recognised gaming as a formal disorder: internet gaming disorder (IGD) remains in DSM-5-TR Section III, “Conditions for Further Study”, and the two criterion sets are described and compared in section 6 of this chapter.

2. History

Pre-history: internet addiction (1990s)

The conceptual precursor was “internet addiction”, proposed as a clinical entity by Young, who adapted DSM-IV pathological gambling criteria to internet use (Young, 1998). Through the 1990s and 2000s gaming was studied mainly inside this broader internet-addiction framework — a conflation later identified as a major source of measurement error, and one that still contaminates systematic reviews of gaming disorder today (Kuss et al., 2017; Colder Carras et al., 2020).

2013 — DSM-5

The APA placed internet gaming disorder in Section III of DSM-5 as a condition requiring further study, explicitly declining to make it an official diagnosis. This status was carried over unchanged into DSM-5-TR in 2022.

2014–2017 — WHO consultations

WHO convened a series of expert meetings on the public-health implications of excessive use of the internet, computers, smartphones and similar devices, and on behavioural addictions. The working-group output that shaped the ICD-11 text argued that gaming disorder displays the core features required of an addictive disorder and that a diagnostic category was needed for people already presenting for treatment (Saunders et al., 2017).

2016–2018 — the scientific controversy

Before ICD-11 was finalised, 26 scholars published an open debate paper opposing inclusion, warning of low research quality, uncritical borrowing from substance-use and gambling criteria, moral panic, and harm to healthy gamers (Aarseth et al., 2017). A partly overlapping group followed with an argument that the burden of evidence for moving from research construct to formal disorder had not been met (van Rooij et al., 2018). Clinicians and public-health researchers replied in the same journal defending inclusion (Griffiths et al., 2017; Rumpf et al., 2018). The substance of this dispute is set out in section 10.

2018–2019 — formal inclusion

The ICD-11 draft containing 6C51 was released in June 2018; the classification was adopted by the 72nd World Health Assembly on 25 May 2019, with entry into force on 1 January 2022. In 2024 WHO published the Clinical Descriptions and Diagnostic Requirements (CDDR) for ICD-11 mental, behavioural and neurodevelopmental disorders, the operational guidance clinicians now use alongside the MMS entry.

Policy context (non-clinical sources)

The following are government and press announcements, not clinical evidence, and no high-quality evaluation of their clinical effect was identified.

- South Korea. The shutdown law (a night-time gaming curfew for minors) was passed on 19 May 2011 and came into force on 20 November 2011. The government announced its abolition on 25 August 2021; the repeal was legislated later that year and the curfew ended from 1 January 2022.

- China. From the National Press and Publication Administration notice of 30 August 2021, users under 18 may play online games only between 20:00 and 21:00 on Fridays, weekends and official holidays — approximately three hours in an ordinary week.
- England and Wales. The NHS National Centre for Gaming Disorders opened in London in October 2019.

3. Epidemiology

Pooled prevalence

Two large meta-analyses converge on a global figure of roughly 3 per cent, which falls substantially once methodological quality is taken into account.

Source	Base	Estimate
Stevens et al., 2021	Worldwide pooled	3.05% (95% CI 2.38–3.91)
Stevens et al., 2021	Studies meeting stringent sampling criteria	1.96% (95% CI 0.19–17.12)
Kim et al., 2022	61 studies, 227,665 participants, 29 countries	3.3% (95% CI 2.6–4.0)
Kim et al., 2022	28 representative-sample studies	2.4% (95% CI 1.7–3.2)
Kim et al., 2022	After trim-and-fill adjustment for publication bias	1.4% (95% CI 0.9–1.9)
Przybylski et al., 2017	Four surveys, N = 18,932, DSM-5 criteria	0.3–1.0%

The Stevens confidence interval of 0.19–17.12 per cent must be printed with the point estimate. Quoting 1.96 per cent alone, in a paragraph where every other figure carries an interval, creates a false impression of precision — the stringent-sampling subset is small and the estimate is correspondingly unstable.

The lowest estimates come from Przybylski, Weinstein and Murayama, who applied DSM-5 criteria across four large international cohorts (samples described in secondary sources as drawn from the United States, the United Kingdom, Canada and Germany). More than two of every three people who played games reported no

IGD symptoms at all, and the authors found the links between IGD and physical, social and mental health outcomes to be mixed.

Sex distribution

Male preponderance is the most consistent epidemiological finding: 8.5 per cent in males versus 3.5 per cent in females (Kim et al., 2022). Reviews report, citing a meta-analysis by Su and colleagues, that men are more likely than women to show disordered gaming while women are more likely to show problematic social-media use (Musetti et al., 2025).

Age

Prevalence is highest in adolescents and young adults; in Kim et al. (2022) mean sample age was negatively associated with reported prevalence — as were sample size and study quality. In the Singapore panel study of Gentile et al. (2011), which followed 3,034 children in grades 3, 4, 7 and 8 over two years, between 7.6 and 9.9 per cent would be classified as pathological gamers at any single point in time (summarised in the abstract as about 9 per cent).

Two paediatric syntheses are frequently quoted together and should not be: Gentile et al. (2017) report a range of roughly 1 to 9 per cent depending on age, country and sample characteristics, whereas Paulus et al. (2018) report about 2 per cent in representative samples of children and adolescents and a mean prevalence of 5.5 per cent when all samples, including clinical ones, are pooled.

Sources of heterogeneity

Reported prevalence varies by an order of magnitude across studies, and the dominant reason is methodological rather than geographical. In Stevens et al. (2021) the choice of screening tool accounted for 77 per cent of the variance, and adolescent samples, lower cut-off scores and smaller sample size were the significant predictors of higher prevalence. Kim et al. (2022) list region among the moderators influencing heterogeneity; Stevens et al. examined study region as a candidate moderator but did not report it among the significant predictors. A regional difference cannot therefore be asserted on the strength of both meta-analyses, and region remains confounded with instrument, sample type and study quality.

Regional data gap

No peer-reviewed prevalence study of gaming disorder conducted in Azerbaijan was identified in this search, and no ICD-11-based prevalence estimate for the South Caucasus was located. Any local figure must be presented as extrapolation, never as measured data.

4. Aetiology and pathogenesis

No single causal mechanism has been established. The evidence base is dominated by cross-sectional studies; the strongest available data come from longitudinal cohorts and from meta-analyses of risk factors.

Genetics

Direct heritability estimates for ICD-11 gaming disorder do not exist. The closest evidence is a Netherlands Twin Register study of 5,247 monozygotic and dizygotic adolescent twins, in which 48 per cent of the individual differences in compulsive internet use were explained by genetic factors, with identical estimates in boys and girls and the remaining 52 per cent attributable to non-shared environment (Vink et al., 2016).

This figure must not be transferred to gaming disorder. The instrument used, the Compulsive Internet Use Scale, is a 14-item measure of compulsive internet use as a single general construct — loss of control, preoccupation, withdrawal, coping or mood modification, and conflict — and contains no gaming-specific items. Time spent gaming was measured separately in that study and was not the phenotype for which the 48 per cent estimate was obtained.

Theoretical model

The most widely used integrative framework is the I-PACE model (Interaction of Person-Affect-Cognition-Execution), which holds that addictive behaviours emerge from interactions between predisposing person variables, affective and cognitive responses to internal and external cues, and executive and inhibitory control, with reinforcement over time shifting behaviour from gratification-driven to compensation-driven and habitual (Brand et al., 2016; Brand et al., 2019). The model is theoretically influential but has not been validated as a diagnostic or predictive tool, and no clinical decision should rest on it.

Neurobiology

A whole-brain meta-analysis of 40 functional imaging studies, using seed-based d mapping, found that neural alterations in internet gaming disorder are task-dependent (Zheng et al., 2019):

Paradigm	Hyperactivation	Hypoactivation
Cue reactivity	Bilateral precuneus, bilateral cingulate	Insula; **no differences in the striatum**

Paradigm	Hyperactivation	Hypoactivation
Executive control	Right superior temporal gyrus, bilateral precuneus, bilateral cingulate, insula	Left inferior frontal gyrus
Risky decision-making	Left striatum, right inferior frontal gyrus, insula	Left superior frontal gyrus, left inferior frontal gyrus, right precentral gyrus

Two points of accuracy matter here. First, the pooled null result in cue-reactivity paradigms concerns the striatum as a whole; a whole-brain analysis of this kind does not license a claim about the ventral striatum specifically. Second, the authors' own conclusion is that the study confirms the critical role of reward and executive-control circuitry in IGD, but not under all conditions — it is not a refutation of reward-system involvement. What the clinician should take from this is narrower and firmer: the frontal findings are not uniformly "hypofrontal" (the right inferior frontal gyrus was hyperactive while the left was hypoactive during risky decision-making), the findings are cross-sectional, and no imaging finding and no biomarker has diagnostic value.

Risk and protective factors — cross-sectional synthesis

The largest synthesis to date pooled 1,586 effects from 253 studies and 210,557 participants published before the COVID-19 pandemic. Empirically robust protective factors were few — self-esteem, intelligence, life satisfaction and education — and all had markedly smaller effect sizes than the risk factors (Ropovik et al., 2023).

Risk and protective factors — longitudinal synthesis

A meta-analysis of 39 longitudinal studies with 37,042 participants gives the most clinically usable structure (Zhuang et al., 2023).

Domain	Risk factors	Protective factors
Intrapersonal	Gaming time r = 0.33; loneliness r = 0.29; social-media disorder r = 0.24 (95% CI 0.03–0.43); aggression r = 0.23; anxiety r = 0.21	Self-control r = −0.27; conscientiousness r = −0.25; agreeableness r = −0.20; self-esteem r = −0.19

Domain	Risk factors	Protective factors
Interpersonal	Deviant peer affiliation r = 0.20; abuse by family r = 0.15	Peer relationship r = −0.23; parent–child relationship r = −0.15; social support r = −0.14
Environmental	—	School engagement r = −0.18

Intrapersonal factors were substantially more predictive than interpersonal or environmental factors. The wide confidence interval on social-media disorder should be noted before that association is used clinically.

Direction of causality

The Singapore two-year panel study is one of the few datasets that can speak to temporal ordering. Greater amounts of gaming, lower social competence and greater impulsivity appeared to act as risk factors for becoming a pathological gamer, whereas depression, anxiety, social phobia and lower school performance appeared to act as outcomes of pathological gaming rather than as its causes (Gentile et al., 2011). The authors' own hedging verb — these variables “seemed to act” in these roles — should be preserved.

Attention-deficit/hyperactivity disorder

A systematic review and meta-analysis found a moderate association between ADHD symptom severity and gaming disorder: r = 0.296 for combined ADHD symptoms, r = 0.306 for inattention and r = 0.266 for hyperactivity (Koncz et al., 2023). This is the single most consistent comorbid association in the field and the highest-yield screening target in practice.

Game design and monetisation

In a survey of 7,422 gamers, spending on loot boxes was associated with problem-gambling severity ($\eta^2 = 0.054$), an association an order of magnitude stronger than for other in-game purchases with real money ($\eta^2 = 0.004$), which suggests specificity for the gambling-like element (Zendle and Cairns, 2018). The design is cross-sectional and the authors state plainly that it cannot distinguish loot boxes acting as a gateway to problem gambling from loot boxes appealing more to people who already gamble problematically.

Competing psychological account

The compensatory internet use model conceptualises excessive gaming as a coping response to unmet psychosocial needs and adverse life situations rather than as a primary addiction (Kardefelt-Winther, 2014). This account competes directly with the addiction model, remains unresolved, and has a practical consequence: in a patient whose gaming escalated in step with a depressive episode or a social crisis, treating the gaming in isolation is unlikely to work.

5. Clinical features

Core presentation

The clinical picture is defined by the three ICD-11 features, not by hours played.

ICD-11 feature	How it presents clinically
Impaired control	Repeated failure to stop at an intended time; sessions longer than planned; unsuccessful attempts to cut down; “one more match” extending into the night
Increasing priority	Gaming displaces sleep, meals, hygiene, study, work, family obligations and previously valued hobbies
Continuation despite negative consequences	Persistence after academic failure, job loss, family conflict, financial loss or documented health harm

Associated features

Preoccupation with gaming between sessions, irritability, restlessness or dysphoria when gaming is prevented, deception about the extent of gaming, and use of gaming to relieve negative mood are described in the DSM-5-TR criterion set. ICD-11 deliberately does not require tolerance or withdrawal, and the status of withdrawal-like phenomena in gaming remains contested. In practice these features support the diagnosis but cannot carry it.

Functional consequences

In clinical presentations the leading complaints are typically brought by family: falling school performance, school refusal or absenteeism, day–night reversal, withdrawal from offline peers, and conflict escalating around attempts to limit

access. The impairment requirement is what separates the disorder from intensive hobby use (Billieux et al., 2019).

Sleep

Night-time gaming, delayed sleep onset and shortened sleep are commonly reported. The American Academy of Sleep Medicine consensus recommendation is 8 to 10 hours of sleep per 24 hours for adolescents aged 13 to 18 years (Paruthi et al., 2016). Evidence linking gaming to sleep disruption is observational, but sleep timing is one of the few functional metrics that can be documented objectively and tracked over treatment.

Physical health

Reported physical correlates include musculoskeletal pain in the back, neck and shoulders, repetitive-strain injuries of the hand and thumb, eye strain with headache and dizziness, and sedentary-lifestyle sequelae. A narrative review notes that four in every ten e-sports athletes report pain, and that gaming for more than five hours a day increased musculoskeletal symptoms among adolescents aged 13 to 18 years (Satish Kumar et al., 2023). The same review states that very few studies exist, that the evidence is mixed, that assessment tools are not validated and that no longitudinal work in adolescents has been done. This is a weak evidence area and must be presented as such.

Comorbidity

ADHD shows the most consistent association ($r \approx 0.30$; Koncz et al., 2023). Depression and anxiety are frequently reported, but the synthesis literature underpinning that impression does not survive appraisal: of seven systematic reviews of gaming disorder with depression or anxiety, covering 196 primary studies in total, evidence of selective outcome reporting was found in all seven, only one assessed risk of bias, and exposure definitions were inconsistent — some included studies used internet-addiction rather than gaming-disorder measures. None was classified as reliable, and the authors concluded that systematic reviews related to gaming disorder do not meet methodological standards (Colder Carras et al., 2020). Comorbidity should therefore be described as commonly observed but poorly quantified, and a comorbid mood or anxiety disorder must be assessed and treated on its own merits rather than assumed to be part of the gaming disorder.

Online versus offline subtype

The predominantly online form (6C51.0) is the more frequently described presentation and is associated with multiplayer, persistent-world and competitive titles. The predominantly offline form (6C51.1) is recognised but far less studied.

What is not part of the clinical picture

Long playing hours in a person whose functioning, sleep, relationships and obligations are intact do not constitute gaming disorder. A person-centred study reported in a recent overview identified a “passionate gamer” subgroup who played more than four hours a day and showed low levels of psychological maladjustment (Musetti et al., 2025). The distinction between high involvement and pathological involvement is essential to the validity of the diagnosis (Billieux et al., 2019).

6. Diagnosis

6.1 Unified diagnostic criteria

All of the following must be present:

- impaired control over gaming — onset, frequency, intensity, duration, termination and context;
- increasing priority given to gaming, such that gaming takes precedence over other life interests and daily activities;
- continuation or escalation of gaming despite the occurrence of negative consequences;
- and the pattern results in significant impairment in personal, family, social, educational, occupational or other important areas of functioning.

Duration: the pattern of gaming behaviour and other features are normally evident over a period of at least 12 months, although the required duration may be shortened if all diagnostic requirements are met and symptoms are severe. Specify the subtype: 6C51.0 predominantly online, 6C51.1 predominantly offline, 6C51.Z unspecified.

6.2 How reliable is this diagnosis in practice?

One field study has tested the ICD-11 guidelines directly. In Shanghai and Hebei, 200 gamers aged 15 to 18 years (mean 16.5, 85.6 per cent male) were assessed by 21 rater pairs formed from seven psychiatrists. Inter-rater agreement was $\kappa = 0.545$ (95% CI 0.490–0.600) for the ICD-11 gaming disorder guidelines — moderate, by the authors' own Landis and Koch classification — and $\kappa = 0.622$ (95% CI 0.553–0.691) for DSM-5 IGD. Agreement between the two systems applied to the same patients was $\kappa = 0.847$ (95% CI 0.814–0.880). Clinicians endorsed the guidelines'

clinical applicability as quite or extremely good in 86.0 per cent of 400 rating sets (Ma et al., 2021).

The head-to-head comparison of these two kappas is not like-for-like and must not be reported as a clean demonstration that ICD-11 is the less reliable system. The ICD-11 value comes from a three-category judgement (gaming disorder / hazardous gaming / neither), the DSM-5 value from a two-category judgement (IGD yes / no). A three-category kappa is structurally penalised by disagreements over the intermediate hazardous-gaming category, which has no DSM-5 counterpart. Three further limitations belong with the figures: all 200 gamers were screened from a single vocational and technical education centre; there were only seven raters; and the concurrent joint-rater design used in WHO field studies tends to generate higher kappa values than an independent re-interview would, so 0.545 is more likely an optimistic than a pessimistic estimate of everyday practice. There is no replication.

The same study also disposes of a common assumption. Applied to the same 400 rating sets, the ICD-11 criteria yielded slightly *more* cases (86) than the DSM-5 criteria (78), and agreement between the systems was almost perfect. Differences in reported prevalence between studies therefore cannot be attributed simply to which manual an instrument follows; the instrument itself, its cut-off and the sample matter far more (see section 3).

6.3 Why the 12-month requirement is not arbitrary

Gaming-disorder caseness is markedly unstable over time. A meta-analysis of 50 longitudinal studies found categorical stability of approximately 43 to 45 per cent at one year and 34 to 38 per cent at two years (Sun et al., 2025), and a one-year longitudinal study found substantial movement across the diagnostic threshold in both directions (Hong et al., 2023). A large share of people who cross the threshold at a single assessment will not be above it a year later without any treatment at all. The 12-month requirement is what prevents a transient episode — an examination period, a lost job, a bereavement, a summer holiday with an unlimited connection — from being converted into a psychiatric diagnosis. The permitted shortening applies only when every diagnostic requirement is already met and symptoms are severe; it is not a licence to diagnose early on impression.

6.4 Boundary with normality

Intensive gaming undertaken for skill development, competition or recreation, in the absence of impaired control and of functional impairment, is not a disorder. There is no time threshold in the criteria; hours played are not diagnostic. WHO states in its public guidance that gaming disorder affects only a small proportion of people who

engage in digital or video gaming. This is the principal safeguard against false-positive diagnosis (Billieux et al., 2019).

6.5 Diagnostic algorithm

- Step 1. Establish that a gaming pattern is present and characterise it: titles, platform, online or offline, session structure, monetisation exposure including loot boxes.
- Step 2. Assess each of the three core components separately, with concrete behavioural examples rather than self-labelling.
- Step 3. Document functional impairment objectively — school or work records, sleep, family report, loss of previous activities. In the absence of impairment, consider QE22 Hazardous gaming instead.
- Step 4. Establish the 12-month time frame, or document severity sufficient to shorten it.
- Step 5. Exclude a manic or hypomanic episode (6A60, 6A61) as the sole context of the behaviour.
- Step 6. Screen systematically for comorbidity: ADHD first, then depressive disorders, anxiety and social anxiety, substance use and sleep disorders.
- Step 7. Use a validated instrument (GDT, ACSID-11, IGDT-10, IGDS9-SF) as an adjunct only. No instrument is diagnostic on its own.

6.6 ICD-11 and DSM-5-TR compared

	ICD-11 6C51 Gaming disorder	**DSM-5-TR Internet gaming disorder**
Status	Formal diagnosis	Section III, Conditions for Further Study
Structure	Three obligatory components, all required	Nine proposed criteria, five or more required
Impairment	Obligatory, separate requirement	Contained in the diagnostic stem (clinically significant impairment or distress), not a separate additional criterion
Tolerance and withdrawal	Not included	Included among the nine criteria

	ICD-11 6C51 Gaming disorder	**DSM-5-TR Internet gaming disorder**
Time frame	Normally at least 12 months; may be shortened if severe	Within a 12-month period
Scope	Online or offline; subtypes coded	Framed around online games
Sub-threshold code	QE22 Hazardous gaming	None

The greater emphasis ICD-11 places on impairment is generally read as giving it the higher diagnostic threshold (Musetti et al., 2025).

6.7 Diagnostic stability over one year

In a one-year longitudinal study of 279 participants — 120 patients with problematic gaming and 159 gamers from the general population — 34.7 per cent of participants had a change in DSM-5 IGD diagnosis, while the number of ICD-11 gaming disorder cases increased to 60.4 per cent (Hong et al., 2023). These two figures are different quantities and must not be set against each other as though they were the same: 34.7 per cent is a rate of diagnostic change, whereas 60.4 per cent is the level to which the gaming disorder caseload rose. The authors state explicitly that there was no significant difference between the amounts of IGD and GD diagnostic change over the year, and their interpretive conclusion is the cautious one — that a gaming disorder diagnosis *might* be more prone to change than an IGD diagnosis. ADHD carried the highest odds ratio for both; the point estimate reported for gaming disorder is of the order of 10^6 with a correspondingly vast confidence interval and is not interpretable as an effect size, almost certainly reflecting quasi-complete separation in the model.

The clinical consequence is direct: a single cross-sectional assessment establishes a diagnosis at that moment, not a prognosis, and re-assessment is part of management rather than an optional extra.

6.8 Differential diagnosis

Code	Condition	Distinguishing point
QE22	Hazardous gaming	Gaming that appreciably increases risk of harm but does not meet the requirements for 6C51 — typically because impaired control or significant impairment is absent. Use this code when the pattern warrants advice but not a disorder diagnosis.
6C50	Gambling disorder	Central behaviour is wagering money or valuables on outcomes of uncertain result. The boundary is blurred by gambling-like in-game monetisation; assess both behaviours, and code both where criteria are met.
6A60 / 6A61	Bipolar type I / type II disorder	Formal ICD-11 exclusions. Excessive gaming occurring exclusively within a manic or hypomanic episode is a symptom of the mood episode.
6A05	Attention deficit hyperactivity disorder	Strongest and most consistent comorbidity. Distinguish primary ADHD with disorganised, poorly regulated gaming from an independent gaming disorder; the two frequently coexist and both should be coded when criteria are met.

Code	Condition	Distinguishing point
6A70 / 6A71 / 6A72	Depressive disorders	Gaming used predominantly to escape or relieve depressed mood may be a secondary phenomenon. Assess the temporal sequence and whether gaming-related impairment persists once the mood disorder is treated.
6B00 / 6B04	Generalised anxiety disorder / Social anxiety disorder	In social anxiety the avoidance is driven by fear of negative evaluation; in gaming disorder the driver is impaired control over and prioritisation of gaming itself.
6A02	Autism spectrum disorder	Intense, circumscribed interest may reflect the restricted-interest domain rather than an addictive process. A long-standing, non-escalating special interest that does not produce impairment is not 6C51.
6C40–6C4G	Disorders due to substance use	Assess whether the gaming pattern persists independently of substance use or intoxication.

Code	Condition	Distinguishing point
6A20	Schizophrenia	Prolonged solitary immersion may occur in the context of negative symptoms, social withdrawal or delusional preoccupation with game content.
6B20	Obsessive-compulsive disorder	Ritualised in-game behaviour driven by intrusive thoughts and anxiety reduction is ego-dystonic from the outset, unlike the gratification-then-habit pattern of gaming disorder.
6C5Y / 6C5Z	Other specified / unspecified disorders due to addictive behaviours	For addictive-behaviour patterns centred on other online activities — social networks, buying-shopping, pornography use — that meet neither the gaming nor the gambling definition.
7A00–7A0Z	Insomnia disorders	Determine whether the sleep disturbance is a consequence of the gaming pattern or an independent sleep–wake disorder.

One differential question has no settled answer: whether gaming that arises and remits with a mood or attention disorder is a primary condition at all. It remains an acknowledged research priority (Musetti et al., 2025), and in an individual patient it is answered pragmatically — by treating the comorbid disorder properly and observing whether the gaming-related impairment resolves with it.

7. Examination and assessment

General principle

There is no biological test and no imaging finding that can diagnose gaming disorder, and no diagnostic biomarker has been established. Diagnosis rests on clinical interview; psychometric instruments are screening and severity tools only, and none should be used to make or exclude a diagnosis on its own.

Instruments

Instrument	Framework	Structure and scoring	Validation and cut-off
Gaming Disorder Test (GDT) Pontes et al., 2021	ICD-11	4 items, 5-point Likert, total 4–20; higher = more severe	Developed in 236 Chinese and 324 British gamers recruited online. Internal consistency supported by a 2024 reliability-generalisation meta-analysis of 17 studies across 14 languages (pooled α = 0.86, 95% CI 0.83–0.89). No single universally validated diagnostic cut-off.

Instrument	Framework	Structure and scoring	Validation and cut-off
ACSID-11 Müller et al., 2022	ICD-11	11 items applied identically to gaming, buying-shopping, pornography use, social-networks use and online gambling	Developed in 985 active internet users (German). Thai (2023) and Chinese adolescent (2024) validations cover gaming; the English-language validation published to date covers Tinder and online pornography use, not gaming. Dimensional; no diagnostic cut-off.
IGDT-10 Király et al., 2017	DSM-5	10 items operationalising the nine criteria; score range 0–9	Developed in 4,887 online gamers (14–64 years, mean 22.2, 92.5% male). Cut-off ≥ 5 indicates probable IGD. Scalar measurement invariance across seven language-based samples (7,193 participants) shown in a separate 2019 study.

Instrument	Framework	Structure and scoring	Validation and cut-off
IGDS9-SF Pontes and Griffiths, 2015	DSM-5	9 items, 5-point scale; criterion-based interpretation (endorsement of at least five of nine at the highest frequency)	Developed in 1,397 English-speaking gamers (85.1% male, mean age 27). Systematic review of 21 studies across 15 language versions confirmed a unidimensional structure (Poon et al., 2021). No definitively validated single total-score threshold.
AICA-S	Clinical (internet and computer game addiction)	Severity scale	Primary outcome measure in the STICA multicentre randomised trial (d = 1.19 at treatment termination).

Critical caveat on instruments

A face-validity evaluation of 29 gaming-disorder screening instruments, comprising an item bank of 417 items rated independently by three professional raters, identified systematic problems of scope, wording and overpathologising — items that capture normal intensive engagement (King et al., 2020). This is not a theoretical concern: in the largest prevalence meta-analysis the choice of screening tool accounted for 77 per cent of the between-study variance (Stevens et al., 2021). An instrument score above a cut-off is a reason to interview carefully, never a diagnosis.

Recommended assessment package

- Structured clinical interview against the three ICD-11 components plus documented functional impairment.
- Collateral history from family or school — essential in minors.

- One ICD-11-aligned instrument (GDT or ACSID-11) for severity tracking over time.
- Systematic comorbidity screening: ADHD first (r ≈ 0.30), then depressive and anxiety disorders, substance use and social anxiety.
- Sleep assessment, with the AASM adolescent recommendation of 8–10 hours as the reference point.
- Assessment of gambling-like monetisation exposure (loot boxes, in-game purchases) and of any co-occurring gambling behaviour.
- Baseline functional metrics for follow-up: school or work attendance and performance, sleep timing, offline social contacts, physical activity.

8. Treatment

There is no approved pharmacotherapy for gaming disorder anywhere. No medication holds FDA approval for this indication, and every agent that has been studied was used off-label. No Cochrane systematic review dedicated to gaming disorder was identified; NICE has published a guideline on gambling-related harms (NG248, 28 January 2025) but none covering gaming disorder; and no WFSBP, CANMAT, NIMH, SAMHSA or VA-DoD clinical guidance specific to gaming disorder was identified. For a formally classified disorder this is a striking gap, and it means the clinician is working from primary trials rather than from guidelines.

8.1 Pharmacotherapy: what has actually been studied

The entire pharmacological literature was summarised in a systematic review of 12 clinical trials reporting on 724 participants. Its limitations define what can and cannot be concluded from it:

- 98.6 per cent of participants were male (684 of the 694 for whom sex was reported);
- 100 per cent of the trials were conducted in South Korea;
- 8 of the 12 trials were open-label, and only four studies overall were rated at low risk of bias.

Across all trials, IGD symptom reductions from pre- to post-treatment ranged from 15.4 to 51.4 per cent — a range driven entirely by the bupropion studies (Sá et al., 2023).

Agent	Share of participants	Reported symptom reduction
Bupropion (incl. sustained release), 6 articles	26.9% (n = 195)	15.4–51.4%
SSRIs — fluoxetine, escitalopram, paroxetine, 6 articles	23.3% (n = 169)	17.6–24.0%
Methylphenidate, 2 articles	14.6% (n = 106)	23.7–25.7%
Atomoxetine, 1 article	5.5% (n = 40)	18.3%

Comorbidity is central to reading these figures. Seven of the 12 trials focused primarily on IGD, accounting for 461 participants (63.6 per cent of the total); five of those seven also enrolled participants with psychiatric comorbidity, most commonly ADHD and major depressive disorder, accounting for 298 participants — about 41 per cent of the whole pooled sample. In other words, a substantial share of the observed improvement may belong to the comorbid disorder rather than to the gaming behaviour.

The two most informative individual trials:

- Han and Renshaw (2012). Twelve-week prospective, randomised, double-blind trial (eight weeks active treatment, four weeks follow-up) in 50 men with major depressive disorder and excessive online game play. Bupropion plus internet-use education reduced Young Internet Addiction Scale scores and mean gaming time more than placebo plus education. Clinically relevant detail: during the four-week follow-up the reduction in online game play persisted while depressive symptoms recurred.
- Nam et al. (2017). Twelve-week double-blind comparison of bupropion and escitalopram in patients with major depressive disorder and excessive internet game play. Both groups improved on depressive and IGD symptoms, but with 30 patients in total (15 per arm) the trial cannot support any comparative efficacy claim.

Clinical rule. Pharmacotherapy in gaming disorder is currently directed at comorbidity — ADHD, depression — and not at the gaming behaviour itself. It should be prescribed on the indication of the comorbid disorder, with the gaming outcome monitored rather than promised.

8.2 Psychotherapy: cognitive-behavioural therapy is the best-supported option

Three syntheses and one multicentre trial define the position.

- Stevens et al. (2019). Meta-analysis of 12 independent CBT studies. The pre–post effect on IGD symptoms at post-test was g = 0.92 (95% CI 0.50–1.34); depression g = 0.80 (0.21–1.38); anxiety g = 0.55 (0.17–0.93). These are pre–post treatment effects, not controlled between-group comparisons, and the distinction matters because this is the field's headline number. The finding that governs practice is the next one: treatment gains at follow-up were non-significant across all four outcomes — IGD symptoms included, not only depression and anxiety. Gains from CBT are demonstrated in the short term and are not demonstrated to last.
- Wölfling et al. (2019) — the STICA trial. The highest-quality randomised evidence: a multicentre RCT at four outpatient clinics in Germany and Austria, 143 men randomised to manualised short-term CBT (n = 72) or wait-list control (n = 71). Remission occurred in 69.4 per cent of the treatment group versus 23.9 per cent of controls (odds ratio 10.10, 95% CI 3.69–27.65, adjusted for baseline severity, comorbidity, centre and age). Effect sizes at treatment termination: d = 1.19 (AICA-S), d = 0.88 (weekday time online), d = 0.64 (psychosocial functioning), d = 0.67 (depression). The dose was 15 weekly group sessions plus up to eight fortnightly individual sessions — up to about 23 sessions over roughly four months, which is what a service planning to reproduce this result must resource. Limitations: male-only sample, and inclusion was based on internet addiction as the primary diagnosis rather than strictly on ICD-11 gaming disorder.
- Ock et al. (2025). Meta-analysis of 18 RCTs of non-pharmacological interventions (1,950 participants): pooled g = −0.82 (95% CI −1.23 to −0.52; I^2 = 90.36%). Psychotherapy, mainly CBT, showed the largest effect (10 studies, 1,036 participants; g = −1.34); behavioural interventions g = −0.38; prevention-focused work g = −0.33 versus treatment g = −1.13; adults improved more than adolescents (g = −1.14 versus −0.45). Publication bias was statistically confirmed (Begg p = 0.043; Egger p = 0.002). A further warning sign: the four studies rated at low risk of bias produced the largest effect of all (g = −4.08), an implausible estimate that points to an unstable evidence base rather than to a large true effect.
- Danielsen et al. (2024). The most conservative synthesis — 38 studies, 76 effect sizes, 9,524 participants — yielded a moderate summary estimate, with the authors stating that confidence in the finding is compromised by small-study effects, possible publication bias, a limited study pool and a lack of standardisation.

8.3 Brief abstinence

A voluntary 84-hour abstinence protocol in 24 adult gamers, 9 of whom screened positive for IGD, reduced gaming hours, maladaptive gaming cognitions and IGD symptoms, with total compliance and no attrition; at 28 days, 75 per cent of the IGD group showed clinically significant improvement in IGD symptoms and 63 per cent showed reliable improvement in maladaptive cognitions (King et al., 2017). This is a small pilot. Abstinence and controlled-use goals have never been compared in an adequately powered trial, and the treatment goal should be individualised rather than dictated.

8.4 Family and developmental interventions

The PIPATIC programme — a manualised six-module individualised psychotherapy for adolescents aged 12 to 18, comprising psychoeducation and motivation, addiction treatment as usual adapted to IGD, intrapersonal, interpersonal, family and new-lifestyle modules, delivered over six months in 22 sessions of about 45 minutes — was compared with standard CBT in two public mental health centres. Both groups showed significant reduction of IGD symptoms (Torres-Rodríguez et al., 2018). The decisive limitation is the sample: 31 adolescents in total across both arms.

Family, parental-mediation, school-engagement and lifestyle measures are widely recommended and are consistent with the longitudinal protective-factor data — peer-relationship quality $r = -0.23$, school engagement $r = -0.18$, parent–child relationship $r = -0.15$, social support $r = -0.14$ (Zhuang et al., 2023) — but they have not been tested as stand-alone interventions in randomised trials. National regulatory measures such as curfews and weekly time caps for minors exist in several countries; no high-quality evaluation of their effect on clinical outcomes was identified, and the regulation of loot boxes and microtransactions remains an open policy question (Musetti et al., 2025).

8.5 Summary of evidence levels

Intervention	Level	Basis and main limitation
CBT, individual and group (STICA dose: 15 group + up to 8 individual sessions)	MODERATE	Multiple RCTs plus one multicentre RCT; high heterogeneity and confirmed publication bias; follow-up gains non-significant

Intervention	Level	Basis and main limitation
Brief structured abstinence	LOW	Single pilot, 24 participants
Family-integrated adolescent programmes	LOW	Small controlled study, 31 adolescents in total
Pharmacotherapy for comorbid ADHD or depression	LOW	12 trials, 724 participants, 98.6% male, all South Korean, 8 of 12 open-label
Pharmacotherapy targeting gaming disorder itself	INSUFFICIENT	No approved agent; no adequately powered placebo-controlled trial in a non-comorbid ICD-11-diagnosed sample
Parental, school and environmental measures	INDIRECT	Supported by longitudinal risk-factor data, not by trials
National regulatory restrictions	UNEVALUATED	No high-quality evaluation of clinical outcomes identified

9. Prognosis

Prognostic statements about gaming disorder must be confined to a one- to two-year horizon. Nothing is known beyond it: no follow-up study extending five to ten years was identified, there are no data on the adult outcomes of adolescent-onset gaming disorder, and there are no mortality data.

Natural course

Gaming disorder is not a uniformly chronic condition. A meta-analysis of 50 longitudinal studies found categorical stability of approximately 43 to 45 per cent at one-year follow-up and 34 to 38 per cent at two years — between one third and one half of those who met the threshold at baseline still met it at follow-up, and the remainder did not. Dimensional symptom severity behaved differently: baseline-to-follow-up correlations were positive and statistically significant, indicating moderate to high continuity of severity even where categorical status changed. Stability of this

order is similar to that of personality disorder and gambling disorder, and lower than that of schizophrenia or bipolar disorder (Sun et al., 2025). Two qualifiers belong with these figures: the pooled samples were predominantly adolescent, and these are data on untreated natural course.

The one-year longitudinal study of 279 participants points in the same direction: 34.7 per cent had a change in DSM-5 IGD diagnosis over the year, while the number of ICD-11 gaming disorder cases increased to 60.4 per cent, with no significant difference between the amounts of diagnostic change in the two systems (Hong et al., 2023).

Predictors

Prospective risk factors for developing pathological gaming are greater amounts of gaming, lower social competence and greater impulsivity (Gentile et al., 2011). Meta-analytic longitudinal predictors of higher symptom trajectories are gaming time ($r = 0.33$), loneliness ($r = 0.29$), co-occurring social-media disorder ($r = 0.24$), aggression ($r = 0.23$) and anxiety ($r = 0.21$); protective predictors are self-control ($r = −0.27$), conscientiousness ($r = −0.25$), peer-relationship quality ($r = −0.23$), self-esteem ($r = −0.19$) and school engagement ($r = −0.18$) (Zhuang et al., 2023). ADHD carried the highest odds ratio for diagnostic change in the one-year study (Hong et al., 2023).

Predictors of relapse after successful treatment have not been established. No validated relapse predictor exists, which means that follow-up after remission cannot be targeted at a high-risk subgroup and must be offered on a routine basis.

Outcomes attributable to the disorder

In the two-year Singapore panel, depression, anxiety, social phobia and lower school performance behaved as outcomes of pathological gaming rather than as its causes. Youths who stopped being pathological gamers ended up with lower levels of depression, anxiety and social phobia than those who remained pathological gamers (Gentile et al., 2011). This is a between-group comparison, not a documented within-person improvement, and school performance is not among the variables reported as differing between those who stopped and those who did not. It remains the strongest available evidence that the condition carries prospective harm rather than merely co-occurring with distress.

Treated prognosis

In the STICA multicentre trial, remission occurred in 69.4 per cent of treated men versus 23.9 per cent of wait-list controls (Wölfling et al., 2019). Meta-analysis of

non-pharmacological interventions found adults improving substantially more than adolescents (g = −1.14 versus −0.45), an age gradient that should temper prognostic expectations in younger patients (Ock et al., 2025). Against this, the CBT meta-analysis found treatment gains at follow-up to be non-significant across all four outcomes (Stevens et al., 2019): short-term response is well documented, durability is not.

Who is missing from the prognostic evidence

Women and girls are effectively unrepresented in the treatment literature. The one multicentre randomised trial enrolled men only, and 98.6 per cent of participants in the entire pharmacological literature were male. No statement about treatment response or prognosis in female patients can be made from the published evidence, and this should be said to female patients and their families rather than concealed behind figures derived from male samples.

10. Myths and misconceptions

Everyday misconceptions

Myth 1: “Anyone who plays a lot is addicted”

Refuted. ICD-11 contains no time threshold; the diagnosis requires impaired control, displacement of other life interests, continuation despite harm *and* functional impairment. WHO states that gaming disorder affects only a small proportion of people who engage in digital or video gaming. A person-centred study reported in a recent overview identified a subgroup of “passionate gamers” playing more than four hours a day with low levels of psychological maladjustment (Musetti et al., 2025; Billieux et al., 2019).

Myth 2: “Gaming disorder affects a large share of gamers”

Refuted at the high end. The best-quality estimates are 2.4 per cent in representative samples and 1.4 per cent after adjustment for publication bias (Kim et al., 2022); 1.96 per cent (95% CI 0.19–17.12) in the stringent-sampling subset (Stevens et al., 2021); and 0.3 to 1.0 per cent using DSM-5 criteria in four large international samples (Przybylski et al., 2017).

Myth 3: “A fixed number of hours per day defines the disorder”

Refuted. ICD-11 defines the condition by control, priority, persistence despite harm and impairment. Time is a prospective risk factor (r = 0.33; Zhuang et al., 2023) but it is not a criterion, and it is neither necessary nor sufficient.

Myth 4: “Video games cause violence”

Refuted as a causal claim. The American Psychological Association's February 2020 revision of its resolution states that there is insufficient scientific evidence to support a causal link between violent video games and violent behaviour, and warns explicitly against attributing violence, including mass shootings, to video game use. A reanalysis of the data underlying the APA's 2015 task-force report found negligible relationships with aggressive and prosocial behaviour, small relationships with aggressive affect and cognitions, and stronger relationships with desensitization; effect sizes appeared elevated by non-best practices and researcher-expectancy effects, particularly in experimental studies (Ferguson et al., 2020). The desensitization finding runs against the general argument and is reported here for that reason. Violence and gaming disorder are separate questions and must not be merged.

Myth 5: “Gaming disorder is nothing but a symptom of depression or ADHD”

Partly true and genuinely contested. Comorbidity is high and ADHD shows the most consistent association ($r \approx 0.30$; Koncz et al., 2023), and critics argue that disordered gaming is largely secondary to mood and attention problems (van Rooij et al., 2018). Against this, the two-year Singapore panel found depression, anxiety and social phobia behaving as outcomes of pathological gaming rather than as its causes (Gentile et al., 2011). Both readings must be presented; the question is not settled.

Myth 6: “Total abstinence from all games is the only correct treatment goal”

Not established. Brief voluntary abstinence has pilot-level support only (King et al., 2017), and no adequately powered trial has compared abstinence with controlled-use goals. The treatment goal should be individualised.

Myth 7: “There is a medication that cures gaming disorder”

Refuted. No agent has regulatory approval for this indication; the entire pharmacological literature comprises 12 trials, 724 participants, 98.6 per cent male, all conducted in South Korea, with 8 of the 12 open-label (Sá et al., 2023). Medications are used off-label for comorbid ADHD or depression.

Myth 8: “Loot boxes and in-game purchases are harmless entertainment”

Not supported. In 7,422 gamers, loot-box spending was associated with problem-gambling severity ($\eta^2 = 0.054$), an effect an order of magnitude larger than for other in-game purchases (Zendle and Cairns, 2018). The evidence is cross-sectional and

does not establish causation in either direction, but the association is robust enough to warrant asking about it in every assessment.

Myth 9: "Gaming disorder is an Asian problem"

Overstated. Prevalence estimates from East and South-East Asian samples tend to be higher, but in the largest meta-analysis the screening tool accounted for 77 per cent of the between-study variance and region was not among the reported significant predictors (Stevens et al., 2021); Kim et al. (2022) list region as one moderator among several. Region is confounded with instrument, sample type and study quality, and cases are reported worldwide.

Myth 10: "The literature on gaming and depression is solid"

Refuted. An appraisal of seven systematic reviews of gaming disorder with depression or anxiety, covering 196 primary studies, found selective outcome reporting in all seven, risk-of-bias assessment in only one, and inconsistent exposure definitions; none was classified as reliable (Colder Carras et al., 2020).

The scientific controversy over the category itself

6C51 is the most contested diagnosis in the ICD-11 addictive-behaviours grouping. The dispute is academic, not commercial, and both of the popular explanations for it — industry lobbying on one side, pure moral panic on the other — are wrong.

The case against inclusion. In 2017, 26 scholars published an open debate paper opposing the WHO proposal (Aarseth et al., 2017). Their arguments were that the research base was of low quality; that the operationalisation borrowed uncritically from substance-use and gambling criteria; that the diagnosis risked pathologising normal, healthy, intensive play; and that formalisation could fuel moral panic, stigmatise a large recreational population and lead to unnecessary treatment of children. A partly overlapping group then argued that the burden of evidence required to move from a research construct to a formal disorder is very high, precisely because diagnoses can be misused, and that WHO should postpone formalisation (van Rooij et al., 2018).

The case for inclusion. The WHO expert-consultation output argued that gaming disorder displays the core features of an addictive disorder and that a category was needed for people already presenting for treatment (Saunders et al., 2017). Rumpf and colleagues replied from a clinical and public-health standpoint: treatment services for these patients already existed in many countries; without a category patients could not be reliably identified, services could not be planned or funded, and research could not be standardised; and the risk of over-diagnosis is mitigated by the

strict ICD-11 impairment requirement (Rumpf et al., 2018). Griffiths and colleagues argued that problematic gaming demonstrably exists and constitutes disordered gaming (Griffiths et al., 2017).

WHO's position. WHO maintained the category, framed inclusion as following the independent development of treatment programmes for identical clinical presentations in many parts of the world, and emphasised in its public guidance that gaming disorder affects only a small proportion of people who game — an explicit attempt to defuse the over-pathologising objection.

The industry position. Contrary to a common assumption, the video-game industry opposed the diagnosis. The Entertainment Software Association publicly rejected the addiction framing and urged WHO to reverse direction, and similar concerns were voiced by the European Games Developer Federation (reported in Musetti et al., 2025). The diagnosis cannot be attributed to industry lobbying, and the moral-panic objection came from within academia rather than from the industry.

What remains genuinely unresolved. Three things, each of which changes what a clinician should say to a patient. Inter-rater reliability of the ICD-11 guidelines has been tested once, in one country, in one school-based sample, and was moderate ($\kappa = 0.545$). Categorical caseness is unstable over a single year in both diagnostic systems. And the synthesis literature on the commonest comorbidities does not meet accepted methodological standards. None of this makes the diagnosis unusable; all of it makes a confident prognosis unjustified.

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6C70–6C7Z IMPULSE CONTROL

Impulse control disorders

ICD-11 **6C70**	**6C70 PYROMANIA** **Pyromania**
ICD-10 F63.1	Pathological fire-setting [pyromania]
DSM-5-TR F63.1	Pyromania

1. Definition and nosology

Pyromania (ICD-11: 6C70; DSM-5-TR: F63.1) — deliberate, purposeful fire-setting behavior; tension or emotional arousal before the act, relief or pleasure after the act. Without financial gain, revenge, or ideological motive; a rare impulse control disorder.

2. History

- In the 1830s, Marc — ‘monomanie incendiaire’.
- DSM-III (1980) — Within impulse control disorders.
- ICD-11 (2019) — within the impulse control disorders category.

3. Epidemiology

- Rare — only ~3–10% of arson incidents meet criteria for pyromania (most arson has financial, revenge, or other motivation).
- Sex: higher in males.
- Onset: adolescence.
- Comorbidity: conduct disorder, ASPD, ID, ADHD, other substance use.

4. Aetiology and pathogenesis

- Multiple factors — neurobiological impulse control, cognitive deficit, family dysfunction.
- Comorbid disorders are typical.

5. Clinical features

- Repeated deliberate fire-setting.
- Pre-act tension/emotional arousal.
- Admiration for the fire, its attributes (flame, tools).
- Post-act pleasure, relief.
- No financial gain, revenge, ideological goal, or psychotic compulsion (delusion).

6. Diagnosis

6.1 Unified diagnostic criteria

A. Deliberately setting fires on more than one occasion.

B. Pre-act tension or emotional arousal.

C. Fascination with, curiosity about, or attraction to fire and its context.

D. The act, its observation, or its outcome provides pleasure.

E. The fire is not motivated by financial, revenge, ideological, socio-political, or psychotic motives.

F. Not better explained by conduct disorder, a manic episode, or ASPD.

6.2 Differences between sources

- DSM-5-TR / ICD-11 — same.
- Rare in clinical practice; most firesetting is motivation-based.

6.3 Diagnostic algorithm

1. Clinical interview + family/witness information.
2. Motivational history — compatibility with pyromania criteria.
3. Comorbidity (ASPD, conduct disorder, manic, psychosis, ID, ADHD).
4. Legal context — forensic psychiatry consultation.

6.4 Differential diagnosis

Condition	Distinguishing feature
Conduct Disorder (6C91)	Multiple instances of antisocial behavior.

Condition	Distinguishing feature
ASPD (Antisocial personality)	Persistent antisocial pattern.
Manic episode	Related to the affective episode.
Psychotic motivation	Delusion (e.g., “voices”); schizophrenia, delusional disorder.
Revenge / financial / ideological	Not a clinical diagnosis — forensic topic.

7. Examination and assessment

- Psychiatric interview.
- Intellectual assessment.
- Impulsivity scales (Barratt Impulsiveness Scale).
- Structured risk assessment in forensic context (HCR-20, VRAG).

8. Treatment

1. Evidence base is limited (rare disorder).
2. CBT — impulse control, trigger management, exposure to alternative behavior.
3. Treatment of comorbid disorders (therapeutic limit in ASPD, behavioral intervention in ID, stimulant in ADHD).
4. SSRI — off-label for impulse control.
5. Legal-psychiatric coordination (hospitalization, court order).

8.1 Treatment methods

1. CBT for impulse control — Trigger identification, behavior replacement.
2. Comorbidity treatment — Behavioral intervention, specific for ID, ADHD.
3. Forensic risk assessment — HCR-20, VRAG.

9. Prognosis

- Evidence base is limited; comorbid disorder and psychosocial support determine prognosis.

10. Myths and misconceptions

Myth 1: “All fire-setters are pyromaniacs”

Evidence: Most arson motivation-based (financial, revenge) — pyromania not; ~3–10% meet clinical diagnosis.

Myth 2: “Pyromania is only a childhood phenomenon”

Evidence: onset typically in adolescence, but can persist into adulthood.

Myth 3: “There is specific effective pharmacotherapy”

Evidence: No FDA approval; SSRIs off-label, evidence base limited.

Myth 4: “Legal punishment alone is sufficient”

Evidence: Clinical intervention and comorbidity treatment reduce relapse risk.

11. Sources

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ICD-11 **6C71**	**6C71 KLEPTOMANIA** **Kleptomania**
ICD-10 F63.2	Pathological stealing [kleptomania]
DSM-5-TR F63.2	Kleptomania

1. Definition and nosology

Kleptomania (ICD-11: 6C71; DSM-5-TR: F63.2) — a recurrent impulse to steal objects that are not needed for personal use or monetary value; tension before the act, relief or pleasure after the act.

2. History

- Esquirol (1838) — “kleptomanie”.
- DSM-III — Impulse control disorders.
- ICD-11 — impulse control disorders.

3. Epidemiology

- Only ~5% of shoplifting incidents meet criteria for kleptomania (most theft is motivation-based).
- Sex: higher in females (in clinical populations).
- Comorbidity: mood disorders 45–100% across series, MDD up to ~65%; anxiety, OCD, eating disorders, substance use.

4. Aetiology and pathogenesis

- Hypothesis of serotonin and opioid system dysregulation (Grant J.E.).
- Comorbid affective and impulsivity disorders.

5. Clinical features

- Failure to resist stealing act — items unnecessary for personal or financial value.
- Pre-act increase in tension.
- Post-act relief, pleasure.
- Stealing is not motivated by anger, revenge, or psychotic motivation.
- Objects are typically discarded, given away, or hoarded; sometimes surreptitiously returned.

6. Diagnosis

6.1 Unified diagnostic criteria

A. Repeated failure to resist the impulse to steal items that are not needed for personal use or monetary value.

B. Pre-act tension.

C. Post-act relief, pleasure.

D. Stealing is not motivated by anger, revenge, delusion, or hallucination.

E. Not better explained by conduct disorder, a manic episode, or ASPD.

6.2 Differences between sources

DSM-5-TR / ICD-11 — same criteria.

6.3 Diagnostic algorithm

1. Clinical interview.
2. K-SAS (Kleptomania Symptom Assessment Scale) — Grant J.E.
3. Comorbid MDD, anxiety, OCD, eating disorders.

6.4 Differential diagnosis

Condition	Distinguishing feature
Simple theft	Motivation — gain, need.
ASPD	Persistent antisocial pattern.
Manic episode	Affective context.
Conduct Disorder (6C91)	Multiple instances of antisocial behavior.
Dementia	Cognitive decline.

7. Examination and assessment

- K-SAS.
- Comorbidity scales.

8. Treatment

1. Naltrexone — Grant J.E. et al. Biol Psychiatry 2009 RCT — efficacy in kleptomania (50–150 mg/day).
2. CBT — cognitive restructuring, behavioral activation, exposure and ritual prevention.
3. SSRI — for comorbid MDD.
4. Multidisciplinary approach — legal context.

8.1 Treatment methods

1. Naltrexone — Opioid antagonist; reduces kleptomania cravings.
2. CBT for impulse control — Trigger management.
3. Kleptomania Symptom Assessment Scale (K-SAS) — Severity scale.

8.2 Differences between sources

- Grant J.E. — naltrexone evidence base.

9. Prognosis

- Significant improvement with naltrexone and CBT.
- Comorbid MDD predicts a poor prognosis.

10. Myths and misconceptions

Myth 1: “All repeated stealing is kleptomania”

Evidence: Most theft is motivation-based; kleptomania rare.

Myth 2: “The patient can stop ‘voluntarily’”

Evidence: impulse control disorder — failure to resist is clinical diagnostic feature.

Myth 3: “Legal punishment alone is sufficient”

Evidence: Clinical intervention and comorbidity treatment reduce relapse risk.

Myth 4: “There is no specific treatment”

Evidence: Naltrexone evidence base (Grant 2009 RCT); CBT is effective.

11. Sources

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2. APA. DSM-5-TR. 2022.
3. Grant J.E. et al. A double-blind, placebo-controlled study of the opiate antagonist naltrexone in the treatment of kleptomania. Biol Psychiatry 2009;65(7):600–606.

	6C72 COMPULSIVE SEXUAL BEHAVIOUR DISORDER
ICD-11 6C72	**Compulsive sexual behaviour disorder**
ICD-10 F52.7	Excessive sexual drive

	6C72 COMPULSIVE SEXUAL BEHAVIOUR DISORDER
ICD-11 **6C72**	**Compulsive sexual behaviour disorder**
DSM-5-TR —	No DSM-5-TR equivalent

1. Definition and nosology

Compulsive Sexual Behavior Disorder (ICD-11: 6C72) — a persistent pattern of failure to control intense, repetitive sexual impulses and behaviors, causing significant distress or functional impairment. Included in the impulse control disorders category in ICD-11.

DSM-5-TR has no separate diagnosis ("Hypersexual Disorder" was not included in DSM-5). The term "Sex addiction" is controversial in the scientific community.

2. History

- Krafft-Ebing (1886) — "hyperaesthesia sexualis".
- 1980s–1990s — concept of 'sexual addiction' (Carnes P.) became popular, but remained controversial within the scientific community.
- DSM-5 (2013) — "Hypersexual Disorder" not included.
- ICD-11 (2019) — Compulsive Sexual Behavior Disorder (impulse control, not addiction) added as an official category.

3. Epidemiology

- Prevalence: 1–6% (broad range, depending on assessment tools).
- Sex: higher in males (5:1 in clinical populations).
- Comorbidity: MDD, anxiety, BPD, substance use, OCD, ADHD.

4. Aetiology and pathogenesis

- Multiple factors — impulsivity, compulsivity, cognitive-emotional dysregulation, trauma.
- Neurobiological similarities with substance use disorders and OCD.

- Social-cultural — accessibility of internet pornography.

5. Clinical features

- Excessive time spent on sexual fantasies, urges, or behaviors.
- Failure of attempts to reduce behavior despite negative consequences.
- Use of sexual behavior to reduce distress or achieve emotional acceptance.
- Significant functional impairment due to impaired control of sexual behavior (relationships, work, health, legal).
- Typical forms — compulsive masturbation, multiple sexual partners, patronizing commercial sex services, problematic pornography use.

6. Diagnosis

6.1 Unified diagnostic criteria (ICD-11)

A. Pattern of persistent failure to control intense sexual impulses and behaviors.

B. ≥ 1 of the following:

1. Repeated sexual activity leads to impairment in other areas of the individual's life (health, relationships, responsibility);
2. The patient has made repeated and significant efforts to regain control — unsuccessfully.
3. The behavior continues despite negative consequences;
4. Behavior persists even when it does not produce significant pleasure.

C. Duration ≥ 6 months.

D. Significant distress or functional impairment.

E. Distress arising entirely from moral or religious judgement and disapproval does not satisfy the criterion; functional impairment, or distress independent of that judgement, is required.

6.2 Differences between sources

- ICD-11: impulse control disorders; “sex addiction” term not used.
- DSM-5-TR: no distinct diagnosis; in clinical practice falls under ‘Other Specified Disruptive, Impulse-Control, and Conduct Disorder.’
- Diagnosis should not be based solely on spiritual-religious distress.

6.3 Diagnostic algorithm

1. Clinical interview.
2. Scale — CSBI (Compulsive Sexual Behavior Inventory).
3. Comorbidity (MDD, anxiety, BPD, substance, OCD).
4. Screening for sexually transmitted infections (STIs).
5. Legal and relationship consequences.

6.4 Differential diagnosis

Condition	Distinguishing feature
Manic episode (hypersexuality)	Affective context.
BPD (impulsivity)	A persistent pattern across all domains.
OCD (sexual obsessions)	Ego-dystonic obsession; ritual.
Paraphilic disorder (6D3x)	Specific atypical sexual object.
High sexual interest (normal variant)	No functional impairment.
Moral-religious disapproval alone	Not a basis for clinical diagnosis.

7. Examination and assessment

- CSBI.
- Comorbidity scales.
- STI screening.

8. Treatment

1. Psychotherapy first line — CBT, ACT, mindfulness-based approach. Evidence base is developing.
2. Naltrexone off-label — opioid antagonist; modest effect in some studies.
3. SSRIs — for comorbid MDD/anxiety.
4. Group support (S-Anon, SAA) — adjunct.
5. Relationship/couple therapy — in comorbid context.
6. There is no specific FDA-approved pharmacotherapy.

8.1 Treatment methods

1. CBT and Acceptance and Commitment Therapy (ACT) — Trigger management, value-based behavior.

2. Naltrexone off-label — Craving reduction.
3. Compulsive Sexual Behavior Inventory (CSBI) — Severity scale.
4. Self-help (SAA, SLAA) — 12-step-based; evidence base limited.

8.2 Differences between sources

- WHO — ICD-11 official category.
- APA — DSM-5-TR does not have a separate diagnosis; controversial area.

9. Prognosis

- Significant improvement with CBT and ACT.
- Comorbidity treatment important.

10. Myths and misconceptions

Myth 1: “Sex addiction is a diagnosis like substance addiction”

Evidence: ICD-11 — impulse control disorder; ‘addiction’ concept controversial, not adopted in DSM-5.

Myth 2: “High sexual interest is a sign of sex addiction”

Evidence: Clinical diagnosis requires functional impairment and failure of control; mere high sexual activity not a disease.

Myth 3: “Deviation from moral-religious norms is a basis for diagnosis”

Evidence: ICD-11 — religious/spiritual cause not sufficient; functional impairment or marked distress is required.

Myth 4: “Pornography use is always a disorder”

Evidence: Normal use and compulsive use are different; loss of control and functional burden are diagnostic.

Myth 5: “Chemical castration (testosterone suppression) is standard treatment”

Evidence: This approach is only indicated for severe paraphilic disorders (e.g., pedophilic activity risk) according to WFSBP guideline; not recommended for general compulsive sexual behavior.

11. Sources

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ICD-11 **6C73**	**6C73 INTERMITTENT EXPLOSIVE DISORDER (IED)** **Intermittent explosive disorder**
ICD-10 F63.8	Other habit and impulse disorders
DSM-5-TR F63.81	Intermittent Explosive Disorder

1. Definition and nosology

Intermittent Explosive Disorder (IED; ICD-11: 6C73 Intermittent Explosive Disorder; DSM-5-TR: F63.81) — recurrent, uncontrolled verbal or physical aggressive outbursts; disproportionately intense relative to triggering provocation. Remorse or shame after the act.

2. History

- DSM-III (1980) — Official diagnosis of IED.
- DSM-5 (2013) — criteria refined.
- ICD-11 (2019) — under impulse control disorders.

3. Epidemiology

- Lifetime prevalence: 4–7% (Kessler R.C. et al. Arch Gen Psychiatry 2006).
- Sex: higher in males.
- Onset: mid-adolescence.
- Comorbidity: MDD ~70%, anxiety, substance use, BPD, ASPD.

4. Aetiology and pathogenesis

- Heritability 40%.
- Serotonin dysregulation — SSRI effect.
- Trauma and abuse history.
- Disruption of cortex-amygdala regulation.

5. Clinical features

- Recurrent aggression outbursts — verbal (screaming, threat) or physical (object damage, harm to others).
- Explosion out of control — patient unsuccessful in restraining.
- Trigger minimal or minor.
- Post-act remorse, shame, guilt.
- Relationship, work, legal consequences.

6. Diagnosis

6.1 Unified diagnostic criteria

A. One of the following:

1. Verbal or physical aggression outbursts, without destructive harm, ≥2 times per week, ≥3 months
2. Three or more disruptive/damaging outbursts within a 12-month period.

B. Aggression disproportionate to the trigger.

C. Impulsive (not planned) and not for a specific purpose.

D. Significant distress or functional impairment.

E. Chronological age ≥ 6 years.

F. Not better explained by another mental disorder (manic, ASPD, BPD, conduct disorder); rule out substance effects.

6.2 Differences between sources

- DSM-5-TR — criterion A1 (mild) and A2 (destructive) separately.
- ICD-11 — neither frequency nor age: DSM-5-TR requires the A1/A2 split, twice weekly for three months or three destructive outbursts in 12 months, and age ≥6; ICD-11 sets none of these.

6.3 Diagnostic algorithm

1. Clinical interview + family information.
2. Scale — OAS (Overt Aggression Scale), BPAQ (Buss-Perry Aggression Questionnaire).
3. Comorbidity and exclusion of medical causes (frontal lobe injury, epilepsy, hypoglycemia).
4. Toxicology.

6.4 Differential diagnosis

Condition	Distinguishing feature
ASPD (Antisocial)	Planned aggression, no regret.
BPD	Emotional lability and relationships central.
Manic episode	In the context of an affective episode.
Conduct Disorder (6C91)	Multiple instances of antisocial behavior.
ODD (6C90)	Persistent oppositional component with aggression, but outbursts are not primary.
Frontal lobe pathology	Neurological.
Substance-induced aggression	Toxicology.

7. Examination and assessment

- OAS, BPAQ.
- Medical and neurological.
- Toxicology.
- Comorbidity scales.

8. Treatment

1. Fluoxetine (SSRI) — Coccaro E.F. et al. J Clin Psychiatry 2009 RCT — most evidence-based pharmacotherapy in IED; 20–60 mg/day.
2. CBT — anger management, cognitive restructuring, behavioral activation, relaxation. McCloskey M.S. et al. J Consult Clin Psychol 2008.

3. Mood stabilizers — valproate, lithium (off-label in refractory cases).
4. Comorbid disorder treatment.
5. Legal coordination when needed.

8.1 Treatment methods

1. Fluoxetine — SSRI; evidence base from Coccaro RCT in IED.
2. CBT for Anger Management — Trigger identification, cognitive restructuring, time-out, relaxation, problem-solving.
3. Overt Aggression Scale (OAS) — Aggression Scale.

8.2 Differences between sources

- Coccaro J Clin Psychiatry 2009 — fluoxetine evidence-based.
- McCloskey 2008 — CBT efficacy.

9. Prognosis

- Significant improvement with fluoxetine and CBT.
- Comorbid ASPD, substance use poor prognosis.

10. Myths and misconceptions

Myth 1: “IED is just ordinary ‘temper tantrum’”

Evidence: Clinical diagnosis based on frequency and functional impairment.

Myth 2: “The patient can stop ‘voluntarily’”

Evidence: impulse control disorder — failure of control is clinical feature; evidence-based treatment (fluoxetine, CBT) required.

Myth 3: “Benzodiazepine calms aggression”

Evidence: benzodiazepine paradoxical disinhibition can occur; contraindicated or used cautiously in IED.

Myth 4: “Only legal punishment is effective”

Evidence: legal punishment alone ineffective; clinical treatment required.

Myth 5: “Antipsychotic is first-line”

Evidence: SSRI first-line; antipsychotic in refractory case.

11. Sources

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6C90–6C9Z DISRUPTIVE BEHAVIOUR

Disruptive behaviour or dissocial disorders

ICD-11 **6C91**	**6C91 CONDUCT-DISSOCIAL DISORDER** **Conduct-dissocial disorder**
ICD-10 F91	Conduct disorders
DSM-5-TR F91.1	Conduct Disorder, Childhood-Onset Type

1. Definition and nosology

Conduct-dissocial disorder (ICD-11: 6C91; DSM-5-TR: F91.1 Conduct Disorder) — a persistent and repetitive pattern of violating the rights of others or age-appropriate social norms. Typically begins in childhood/adolescence; may transition to ASPD in adulthood.

2. History

- DSM-III (1980) — Official diagnosis.
- DSM-5 (2013) — “limited prosocial emotions” specifier (childhood marker of psychopathic traits).
- NICE CG158 (2017) — child and adolescent antisocial behavior.

3. Epidemiology

- Prevalence: 12-month prevalence 2–10%, median 4%; higher in boys than in girls (DSM-5-TR).
- Sex: 3 times higher in males.
- Onset — childhood (before age 10, poor prognosis) or adolescence (age 10+, good prognosis).
- Comorbidity: ADHD 50%, ODD, MDD, anxiety, substance use, learning disorders.

4. Aetiology and pathogenesis

- Heritability 40–60%.
- Neurobiological — amygdala, prefrontal dysregulation, low sympathetic arousal.
- Environment — childhood trauma, harsh parenting, family dysfunction, peers.
- Prenatal — tobacco, alcohol.

5. Clinical features

DSM-5-TR 15 symptoms in 4 categories (≥ 3 symptoms in 12 months)

- Aggression — towards people/animals: threat, assault, use of weapons, physical violence, sexual violence.
- Object injury: Intentional arson, damage of other objects.
- Deception/theft: Breaking and entering, lying, theft.
- Serious rule violation: Arriving late, running away from home, truancy.

“Limited prosocial emotions” qualifier (psychopathic traits)

- Lack of empathy or remorse;
- Lack of concern about performance;
- Callous lack of emotion;
- Shallow affect.

6. Diagnosis

6.1 Unified diagnostic criteria

DSM-5-TR criteria (≥ 3 symptoms from 15 in 12 months; at least 1 symptom in the last 6 months). Severity: mild, moderate, severe.

Onset: childhood type (≥1 symptom by age 10), adolescent type.

6.2 Differences between sources

- DSM-5-TR — ‘limited prosocial emotions’ qualifier.
- ICD-11 — ‘Conduct-Dissocial Disorder’; according to child, adolescent, adult patterns.
- NICE CG158 — diagnosis and intervention in children and adolescents.

6.3 Diagnostic algorithm

1. Clinical interview + parent, teacher.
2. Structured interview (K-SADS).
3. CBCL (Child Behavior Checklist), TRF (Teacher Report Form).
4. The Inventory of Callous-Unemotional Traits — for psychopathic traits.
5. Comorbidity (ADHD, MDD, substance, learning).

6.4 Differential diagnosis

Condition	Distinguishing feature
ODD (6C90)	No serious legal violation.
ADHD (6A05)	Impulsivity and attention; common comorbidity.
IED (6C73)	Explosive type; absence of persistent antisocial pattern.
Manic episode	Affective context.
RAD/DSED (6B44/6B45)	Attachment disturbance since childhood.
Substance-induced	Toxicology.

7. Examination and assessment

- CBCL, TRF, ICU.
- Comorbidity screening.
- Intellectual and academic assessment.

8. Treatment

8.1 General Principles (NICE CG158 · AACAP)

1. Parent training programs first-line — for ages 3–11:
    - Incredible Years (Webster-Stratton);
    - PCIT (Parent-Child Interaction Therapy);
    - Triple P.
2. For adolescents:
    - MST — Multisystemic Therapy (Henggeler S.W.) — encompassing family, peer, school, community;

   - FFT — Functional Family Therapy;
   - MTFC — Multidimensional Treatment Foster Care.
3. Comorbid ADHD — stimulant (also reduces aggression).
4. Antipsychotic (risperidone) — off-label in severe aggression, with caution; no FDA approval (specific for childhood aggression).
5. "Scared Straight" and similar programs are contraindicated — Cochrane (Petrosino A. 2013) — these programs increase antisocial behaviour.
6. Boot camp type "tough love" programs — no evidence, harmful.

8.2 Treatment methods

1. "Incredible Years" program — Webster-Stratton — group program for parents of children aged 3–8 years; 14–22 sessions.
2. Parent-Child Interaction Therapy (PCIT) — 4–7 years; CDI + PDI.
3. Multisystemic Therapy (MST) — Home-based multisystem intervention for adolescents; evidence base strong for severe behavioral disturbances.
4. Functional Family Therapy (FFT) — Family-based, 8-30 sessions.
5. Multidimensional Treatment Foster Care (MTFC) — Structured fostering program; for severe case.

8.3 Differences between sources

- NICE CG158 (2017).
- AACAP Practice Parameter (Steiner H. 2007).
- Cochrane (Petrosino 2013) — Scared Straight is counterproductive.

9. Prognosis

- With childhood-type and psychopathic traits — risk of ASPD in adulthood ~40–50%.
- Adolescent-type good prognosis.
- Early parent training and multisystem intervention significantly improve outcomes.

10. Myths and misconceptions

Myth 1: "Harsh punishment/discipline is the most effective intervention"

Evidence: Harsh punishment strengthens antisocial behavior; positive parenting training is evidenced.

Myth 2: "'Scared Straight' programs 'correct' youth"

Evidence: Petrosino A. Cochrane 2013 — Scared Straight increases antisocial behaviour (OR 1.68; 95% CI 1.20–2.36); contraindicated; sometimes presented as "effective" in media, but meta-analysis negative.

Myth 3: "Conduct disorder inevitably turns into ASPD"

Evidence: Approximately 50% of cases with adolescent-onset type remit; childhood-onset type and psychopathic traits indicate a poor prognosis.

Myth 4: "Stimulants increase aggression"

Evidence: In comorbid ADHD, stimulants REDUCE aggression; use with monitoring of behaviour and adverse effects.

Myth 5: "Boot camp or military environment cures severe CD"

Evidence: No evidence; some studies show counter-effect efficacy.

Myth 6: "Antipsychotic standard first-line"

Evidence: parent training and psychosocial approaches first-line; risperidone off-label in severe refractory cases.

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ICD-11 6C90	**6C90 OPPOSITIONAL DEFIANT DISORDER (ODD)** **Oppositional defiant disorder**
ICD-10 F91.3	Oppositional defiant disorder
DSM-5-TR F91.3	Oppositional Defiant Disorder

1. Definition and nosology

Oppositional defiant disorder (ODD; ICD-11: 6C90 Oppositional Defiant Disorder; DSM-5-TR: F91.3) — pattern of angry/irritable mood, argumentative/defiant behavior, or vindictive behavior. Unlike Conduct Disorder (CD), there is no interference with the fundamental rights of others.

2. History

- DSM-III (1980) — Official diagnosis.
- DSM-5 (2013) — emotional component (angry/irritable) separately emphasized.

3. Epidemiology

- Prevalence: 1–11% (mean 3.3%) in preschool and school-aged children (DSM-5-TR).
- Sex: higher in prepubertal boys; postpubertal difference decreases.
- Comorbidity: ADHD ~40–60%, MDD, anxiety, transition to CD in 30% (severe childhood type).

4. Aetiology and pathogenesis

- Heritability 50%.
- Environment — harsh parenting, family dysfunction.
- Neurobiological — affective regulation, frontal-amygdala circuitry.
- Temperamental traits (irritability).

5. Clinical features

8 symptoms in 3 categories (≥ 4 symptoms for 6 months)

- Angry/Irritable mood. Easily irritated, discontented.
- Quarrelsome/oppositional behavior: Arguing with adults; non-compliance with rules; deliberately annoying others; blaming others.
- Vengeful behavior: at least twice in the last 6 months.

6. Diagnosis

6.1 Unified diagnostic criteria

DSM-5-TR — ≥4 of 8 symptoms for 6 months; in at least one context (home, school, peers); in at least one other context with different individuals.

Age qualifier: symptoms on most days for 6 months until age 5; age ≥5 — ≥1 per week for 6 months.

6.2 Differences between sources

- DSM-5-TR — severity (mild, moderate, severe) based on number of contexts.
- NICE CG158 — guidance for child antisocial behavior.

6.3 Diagnostic algorithm

1. Clinical interview + parent, teacher.
2. CBCL, TRF, ECBI (Eyberg Child Behavior Inventory).
3. Comorbid ADHD screening.
4. Difference between Disruptive Mood Dysregulation Disorder (DMDD) and depressive disorder.

6.4 Differential diagnosis

Condition	Distinguishing feature
CD (6C91)	Serious legal violation.
ADHD	Attention/impulsivity; frequent comorbidity.

Condition	Distinguishing feature
DMDD (DSM-5)	Persistent irritability + severe anger outbursts, ≥ 12 months (with no symptom-free period of ≥ 3 consecutive months); if criteria for ODD are also met, only DMDD is diagnosed.
Anxiety disorder	Anxiety component dominant.
Intellectual disability	Behavioral disturbance in the context of cognitive deficit.
Age-appropriate 'child resistance'	No functional impairment.

7. Examination and assessment

- ECBI, CBCL.
- Comorbid ADHD, anxiety, MDD.

8. Treatment

1. Parent training first-line:
    - PCIT — 2–7 years;
    - Incredible Years — ages 3–8;
    - Parent Management Training (Barkley) — age 6+;
    - Triple P.
2. Comorbid ADHD — stimulant.
3. SSRI — comorbid anxiety/depression.
4. School-based intervention.
5. Suspected child maltreatment or domestic violence — child protection and social service coordination.

8.1 Treatment methods

1. Parent-Child Interaction Therapy (PCIT) — Real-time coaching by therapist.
2. "Incredible Years" program — Group format.
3. Triple P — Positive Parenting Program — 5-tier system.
4. Parent Management Training — Barkley (Barkley) — Training based on behavioral principles.
5. Eyberg Child Behavior Inventory (ECBI) — Parent-rated behavior scale.

8.2 Differences between sources

- NICE CG158.
- AACAP — parent training.

9. Prognosis

- Most patients improve with early parent training.
- Comorbid ADHD, severe variant — risk of CD/ASPD.

10. Myths and misconceptions

Myth 1: “ODD is just ordinary ‘stubborn child’ behavior”

Evidence: Clinical diagnosis based on functional impairment and frequency; distinct from normal life variant.

Myth 2: “Strict discipline will fix ODD”

Evidence: Severe punishment exacerbates symptoms; positive parenting training is evidence-based.

Myth 3: “Antipsychotic is first-line”

Evidence: parent training first-line; antipsychotics rarely in refractory cases.

Myth 4: “ODD always turns into CD”

Evidence: 30% transition to CD; risk higher with childhood type and comorbidity; majority do not convert to CD.

Myth 5: “Only the child should be treated, not the family”

Evidence: parent training is a central component; child psychotherapy alone is less effective.

11. Sources

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6D10–6D1Z PERSONALITY DISORDERS

Personality disorders and related traits

ICD-11 **6D10**	**6D10 PERSONALITY DISORDER** **Personality disorder**
ICD-10 F60.9	Personality disorder, unspecified
DSM-5-TR F60.9	Unspecified Personality Disorder

1. Definition and nosology

Personality Disorder (ICD-11: 6D10; DSM-5-TR: F60.9; 10 categories) — persistent and pervasive pattern in personal and social functioning; thoughts, feelings, and behaviors regarding self and others. ICD-11 has fundamentally moved to a model of a single category + severity degrees + trait domains; classical 10 distinct personality disorders (paranoid, schizoid, borderline, antisocial, etc.) have been abolished as categorical diagnoses.

2. History

- DSM-III (1980) — 11 categorical personality disorders (Cluster A, B, C).
- DSM-IV (1994), DSM-5-TR (2022) — Same categorical model retained; however, DSM-5 Section III has an alternative ‘dimensional’ model in the research category.
- ICD-11 (2019) — radical change: Categorical 10 disorders abolished, unified Personality Disorder (mild, moderate, severe + Personality Difficulty) + assessment of 5 trait domains.

3. Epidemiology

- Lifetime prevalence: ~9% (Lenzenweger M.F. et al. Biol Psychiatry 2007); BPD ~1.4%, ASPD ~1%, schizoid 3%.

- Sex: BPD higher in females in clinical populations; ASPD higher in males.
- Onset: adolescence and early adulthood.
- Mortality: in BPD, suicide 8–10% lifetime; in ASPD, violent death.
- Comorbidity: MDD, anxiety, substance use, eating disorders.

4. Aetiology and pathogenesis

- Heritability 40–60%.
- Early trauma, childhood abuse (especially BPD).
- Disorganized attachment.
- Neurobiological — frontal-limbic regulation, serotonergic dysregulation.

5. Clinical features

5.1 ICD-11 5 characteristic domains

- Negative affectivity — anxiety, depression, variability.
- Detachment — social and emotional distance.
- Dissociality — lack of empathy, egocentricity.
- Disinhibition — impulsivity, irresponsibility.
- Anankastia — perfectionism, control, rigid rules.

“Borderline model” — a separate qualifier in ICD-11 (corresponding to BPD, instead of 10 categorical separate diagnoses).

5.2 DSM-5-TR 10 categorical disorders (brief)

- Cluster A (odd/eccentric): Paranoid, Schizoid, Schizotypal.
- Cluster B (dramatic/erratic): Antisocial (ASPD), Borderline (BPD), Histrionic, Narcissistic.
- Cluster C (anxious/fearful): Avoidant, Dependent, Obsessive-Compulsive (OCPD).

6. Diagnosis

6.1 Unified diagnostic criteria (ICD-11)

A. Persistent and pervasive personality dysfunction — in self (identity, self-worth, self-direction) and/or interpersonal (relationships, empathy, intimacy, responsibility) functioning.

B. Duration — long-term (years).

C. Not consistent with developmental age, cultural norm, or family situation.

D. Severity: Personality Difficulty (subclinical), Mild PD, Moderate PD, Severe PD.

E. Feature domain qualifiers (above).

6.2 Differences between sources

- ICD-11: Radical change to categorical system; requires training in clinical practice.
- DSM-5-TR: 10 categorical diagnoses retained; Section III alternative model.
- NICE CG78 (BPD), CG77 (ASPD): Separate clinical guidelines for BPD and ASPD.

6.3 Diagnostic algorithm

1. Clinical interview + long history.
2. SCID-5-PD (DSM categorical) or ICD-11 PD assessment.
3. Personality scales — PID-5 (DSM-5 dimensional), MMPI-2-RF.
4. Comorbidity (MDD, substance, trauma).
5. Suicide and violence risk.

6.4 Differential diagnosis

Condition	**Distinguishing feature**
Change in the context of an affective episode	Episodic course; personality stable.
Substance-induced	Substance history.
PTSD / complex PTSD	In trauma context; clinical overlap with BPD.
Bipolar	Affective episodes.
Medical condition (frontal lobe, dementia)	Neurological.

7. Examination and assessment

- SCID-5-PD, PID-5.
- MMPI-2-RF, MCMI-IV.
- Comorbidity and suicide.

8. Treatment

8.1 General Principles (NICE CG78 (BPD), CG77 (ASPD); APA 2024 (BPD, 2nd ed.))

1. Psychotherapy first line — gold standard in personality disorders.
2. Evidence-based approaches for BPD (5):
    - DBT (Dialectical Behavior Therapy, Linehan M.) — emotional regulation, distress tolerance, social skills, mindfulness; standard 12 months;
    - MBT (Mentalization-Based Treatment, Bateman A., Fonagy P.)
    - Schema Therapy (Young J.)
    - TFP (Transference-Focused Psychotherapy, Kernberg O.)
    - STEPPS (Systems Training for Emotional Predictability and Problem Solving).
3. For ASPD — therapeutic limitation; CBT in structured, motivated patient.
4. Pharmacotherapy — no specific approval for personality disorder itself; for comorbid symptoms (impulsivity, depression, anxiety, psychosis-like) — SSRIs, mood stabilizers, low-dose atypical antipsychotics.
5. Crisis management — DBT skills, short-term inpatient stay.
6. Long-term treatment (years).

8.2 Treatment methods

1. Dialectical Behavior Therapy (DBT) — Linehan (Linehan M.) — 4 modules (mindfulness, distress tolerance, emotional regulation, social skills); individual + group + telephone coaching + therapist consultation. 12-month standard. Evidence: Linehan M.M. JAMA Psychiatry 2015.
2. Mentalization-Based Therapy (MBT) — Bateman A., Fonagy P — Mentalization — recognition of one's own and others' mental states; based on psychodynamic roots. Bateman A. Am J Psychiatry 2009.
3. Schema Therapy — Young J. — Restructuring early maladaptive schemas; long-term psychotherapy.
4. Transference-Focused Psychotherapy (TFP — Transference-Focused Psychotherapy) — Kernberg (Kernberg O.) — Processing of primitive defense mechanisms in the context of transference.
5. STEPPS — 20-week group; participation of the patient's social system in BPD.

6. Structured Clinical Interview for Personality Disorders (SCID-5-PD) — DSM-5 categorical diagnosis.

8.3 Differences between sources

- NICE CG78 (BPD, 2009) — DBT, MBT, and similar structured approaches; pharmacotherapy not routinely recommended for BPD itself.
- NICE CG77 (ASPD, 2009).
- APA Practice Guideline for the Treatment of Patients With Borderline Personality Disorder, 2nd ed. (2024) — supersedes the 2001 edition.

9. Prognosis

- BPD — 50%+ remission at 10-year follow-up (Zanarini M.C. reviews).
- ASPD — worst prognosis; but aggression decreases with age.
- Treatment significantly improves the prognosis.

10. Myths and misconceptions

Myth 1: “Personality disorders are untreatable”

Evidence: DBT, MBT, Schema, TFP — evidence-based approaches; most patients improve significantly; pessimistic prognostic stance is outdated.

Myth 2: “BPD patients are manipulative and require strict boundaries”

Evidence: Term “manipulative” is stigmatizing and incorrect; BPD behaviors stem from emotional dysregulation; DBT recommends validation + structure balance.

Myth 3: “Pharmacotherapy cures BPD”

Evidence: NICE CG78 — pharmacotherapy not routinely recommended for BPD itself; psychotherapy first-line.

Myth 4: “ASPD cannot be treated”

Evidence: in severe ASPD intervention limited, but in motivated patients structured treatment may yield improvement; nihilistic stance contraindicated.

Myth 5: “Personality disorder stabilizes with age and does not require treatment”

Evidence: long-term follow-up — BPD remission rate 50%+ over 10 years; treatment accelerates this process.

Myth 6: “Hypnotherapy, esoteric interventions cure personality disorders”

Evidence: No evidence; evidenced structured psychotherapy is recommended.

11. Sources

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ICD-11 6D11	**6D11 PROMINENT PERSONALITY TRAITS OR PATTERNS** **Prominent personality traits or patterns**
ICD-10 —	No ICD-10 equivalent
DSM-5-TR F60.3	Borderline Personality Disorder (matches only the 6D11.5 borderline pattern qualifier)

1. Definition and nosology

Prominent personality traits or patterns (ICD-11: 6D11 Prominent personality traits or patterns) is not a stand-alone diagnosis but a set of qualifiers: they record which trait domains a personality disorder (6D10) or personality difficulty is expressed through. Severity is coded in 6D10 and the domains here; 6D11 is never assigned on its own.

Personality difficulty is not a mental disorder in ICD-11 — it sits in the chapter "Factors influencing health status" under the code QE50.7. Personality traits cause functional difficulty but fall short of the 6D10 criteria.

2. History

- ICD-11 (2019) — the 10 categorical personality disorders were dropped in favour of a severity rating (6D10) plus five trait domains (6D11.0–6D11.4).
- The borderline pattern (6D11.5) was added separately as the successor to ICD-10 borderline personality disorder (F60.3); in DSM-5-TR the only parallel is the Section III dimensional model.

3. Epidemiology

Precise epidemiological data are limited; subclinical personality traits are widespread in the population.

4. Aetiology and pathogenesis

Same as 6D10 — genetic and environmental interaction.

5. Clinical features

- 6D11.0 Negative affectivity — a broad range of negative emotions: anxiousness, irritability, low self-esteem, emotional lability.
- 6D11.1 Detachment — social and emotional distance: avoidance, restricted affect, refusal of intimacy.
- 6D11.2 Dissociality — disregard for the rights and feelings of others: self-centredness, lack of empathy, exploitativeness.
- 6D11.3 Disinhibition — impulsivity, distractibility, irresponsibility, a pull towards immediate reward.
- 6D11.4 Anankastia — perfectionism, rigid rules, emotional and behavioural control.
- 6D11.5 Borderline pattern — instability of relationships, self-image and affect, impulsivity, fear of abandonment, self-harm and suicidal behaviour.

6. Diagnosis

6.1 Unified diagnostic criteria

A. The diagnosis is made under 6D10 (personality disorder) or QE50.7 (personality difficulty); 6D11 is added only as a qualifier.

B. All domains present to a clinically significant degree are recorded — there may be several.

C. The borderline pattern (6D11.5) may be recorded in addition to any combination of domains.

6.2 Differences between sources

ICD-11 — category somewhat similar to the Z-code for ‘clinical attention to personality problems.’

6.3 Diagnostic algorithm

1. Clinical interview.
2. 5-domain assessment.
3. Exclusion of full PD diagnosis.

6.4 Differential diagnosis

Condition	Distinguishing feature
Personality Disorder (6D10)	Significant functional impairment.
Adjustment disorder (6B43)	Response to stress factor.
Normal personality variant	No functional difficulty.

7. Examination and assessment

Personality inventories (PID-5).

8. Treatment

1. Psychotherapy — short-to-medium term; problem-focused CBT or supportive.
2. Stress management and coping skills.
3. Specific intervention for comorbid symptoms.

8.1 Treatment methods

1. DSM-5 — Personality Inventory for DSM-5 (PID-5) — Dimensional personality scale.

9. Prognosis

Improvement with treatment; risk of progression to stable or full PD.

10. Myths and misconceptions

Myth 1: “Personality Difficulty is not a real diagnosis”

Evidence: ICD-11 is the official code; it provides clinical attention to subclinical personality problems.

Myth 2: “Personality difficulty inevitably turns into PD”

Evidence: Some patients experience deterioration, but most stabilize or improve.

Myth 3: “Should only be treated with medication”

Evidence: Psychotherapy is first-line; pharmacotherapy for comorbid symptoms.

Myth 4: “This category is only for labeling ‘anxious’ patients”

Evidence: ICD-11 — clinical response to functional difficulty; provides clinical support.

11. Sources

1. WHO. ICD-11. 6D11 Prominent personality traits or patterns. 2024.
2. WHO. ICD-11. QE50.7 Personality difficulty. 2024.
3. Tyrer P. et al. Lancet 2015;385(9969):717–726.

6D30–6D3Z PARAPHILIC DISORDERS

Paraphilic disorders

ICD-11 **6D30**	**6D30 EXHIBITIONISTIC DISORDER** **Exhibitionistic disorder**
ICD-10 F65.2	Exhibitionism
DSM-5-TR F65.2	Exhibitionistic Disorder

1. Definition and nosology

Exhibitionistic disorder (ICD-11: 6D30; DSM-5-TR: F65.2) — persistent or intense sexual arousal from exposing genitals to non-consenting persons; the patient experiences significant distress or functional impairment in response to these fantasies, urges, or behaviors, and/or acts on them with actual non-consenting individuals.

2. History

- Lasègue C. (1877) — term ‘exhibitionnistes’.
- DSM-III (1980) — Official diagnosis.
- DSM-5 (2013) — distinction between paraphilia (atypical sexual interest) and paraphilic disorder (clinical distress or harm to others).
- WFSBP 2020 (Thibaut F. et al.) — international recommendation for paraphilic disorders.

3. Epidemiology

- Prevalence: males 2–4%; rare in females.
- Onset: adolescence and early adulthood.
- Comorbidity: other paraphilia (50–80% of paraphilic disorder patients), ASPD, substance use, MDD.

4. Aetiology and pathogenesis

- Numerous factors — biological, behavioral conditioning, psychosocial.
- Comorbid impulsivity and hypersexuality.

5. Clinical features

- Repeated and intense sexual arousal for exposing genitals to nonconsenting persons.
- The patient acts on fantasy, impulse, or behavior or experiences distress.
- Typical context — street, public transportation.
- Legal and social consequences (sexual crime).

6. Diagnosis

6.1 Unified diagnostic criteria

A. ≥6 months repeated and intense sexual arousal from genital exposure to non-consenting individuals.

B. The patient has acted on fantasies/impulses or experiences significant distress/functional impairment.

6.2 Differences between sources

- DSM-5-TR requires a 6-month duration; ICD-11 sets no duration criterion — it speaks of a “sustained, focused and intense pattern of arousal”.
- WFSBP 2020 — step-by-step therapy for paraphilic disorders.

6.3 Diagnostic algorithm

1. Clinical interview.
2. Comorbidity (ASPD, substance, other paraphilic).
3. Risk assessment (forensic context).

6.4 Differential diagnosis

Condition	Distinguishing feature
Hypersexuality	Partners with consent.
Manic episode	Affective context.

Condition	Distinguishing feature
Dementia	Disinhibition due to frontal lobe pathology.
Substance-induced	Toxicology.

7. Examination and assessment

- Clinical interview.
- Risk scales (Static-99R for sexual recidivism).

8. Treatment

8.1 General Principles (WFSBP 2020 Stepwise)

1. CBT and cognitive-behavioral intervention — first-line; prevention of relapse, management of cognitive distortions.
2. SSRIs — high dose (fluoxetine 40–80 mg); reduces impulsivity and sexual drive.
3. In refractory cases / severe cases — antiandrogen (CPA cyproterone, MPA medroxyprogesterone).
4. In most severe case — GnRH agonist (leuprolide, triptorelin) — chemical castration; continuous monitoring.
5. Legal-medical coordination.

8.2 Treatment methods

1. CBT (relapse prevention) — Trigger management, cognitive distortion.
2. SSRI High Dose — First pharmacotherapy.
3. Antiandrogen (CPA, MPA) — In refractory cases.
4. GnRH Agonist — In most severe case.

8.3 Differences between sources

- WFSBP 2020 — step-by-step algorithm.

9. Prognosis

Reduction of relapse with evidence-based treatment; comorbid ASPD and substance abuse poor prognosis.

10. Myths and misconceptions

Myth 1: “Exhibitionism is harmless”

Evidence: Psychological trauma for victim; sexual crime; risk of progression to more severe paraphilias.

Myth 2: “Chemical castration is first-line”

Evidence: WFSBP 2020 step-by-step — CBT and SSRI first; GnRH in most severe cases

Myth 3: “Treatment is ineffective”

Evidence: CBT and pharmacotherapy significantly reduce recidivism.

Myth 4: “Only legal punishment is required”

Evidence: Clinical intervention reduces relapse risk.

11. Sources

1. WHO. ICD-11. 6D30 Exhibitionistic disorder. 2024.
2. APA. DSM-5-TR. 2022.
3. Thibaut F., Cosyns P., Fedoroff J.P. et al.; WFSBP Task Force on Paraphilias. The World Federation of Societies of Biological Psychiatry (WFSBP) 2020 guidelines for the pharmacological treatment of paraphilic disorders. World J Biol Psychiatry 2020;21(6):412–490.

ICD-11 **6D31**	**6D31 VOYEURISTIC DISORDER** **Voyeuristic disorder**
ICD-10 F65.3	Voyeurism
DSM-5-TR F65.3	Voyeuristic Disorder

1. Definition and nosology

Voyeuristic disorder (ICD-11: 6D31; DSM-5-TR: F65.3) — persistent and intense sexual arousal from secretly observing undressing, nakedness, sexual activity of

non-consenting persons; patient experiences distress from these fantasies or acts upon them with non-consenting individuals.

2. History

Voyeurism has been described in the clinical literature in parallel with exhibitionistic disorder (6D30).

3. Epidemiology

- Prevalence: the population prevalence of the disorder is unknown; based on voyeuristic acts in non-clinical samples, DSM-5-TR gives a highest possible lifetime prevalence of ~12% in males and ~4% in females (Långström N., Seto M.C. Arch Sex Behav 2006, n=2450). Most people with voyeuristic behaviour do not meet criteria for the disorder.
- Comorbid other paraphilic disorders.
- Internet voyeurism (hidden camera, “upskirt”) is developing.

4. Aetiology and pathogenesis

General etiology for paraphilic disorders.

5. Clinical features

- Observing unsuspecting individuals naked or during sexual activity without their consent.
- Typical context — window, dressing rooms, public places.
- “Upskirting” — the modern form.
- Legal consequences.

6. Diagnosis

6.1 Unified diagnostic criteria

A. ≥6 months repeated sexual arousal from observing non-consenting individuals.

B. The patient has acted on fantasy or experiences distress.

C. Age ≥18 (DSM-5-TR; normal curiosity in adolescents is an exception).

6.2 Differences between sources

- DSM-5-TR requires a 6-month duration; ICD-11 sets no duration criterion — it speaks of a “sustained, focused and intense pattern of arousal”.

- WFSBP 2020.

6.3 Diagnostic algorithm

1. Clinical interview.
2. Comorbidity.
3. Risk assessment.

6.4 Differential diagnosis

Condition	Distinguishing feature
Adolescent curiosity	Part of normal development.
Consensual viewing (pornography)	Consensual context.
Manic episode	Affective context.

7. Examination and assessment

- Clinical interview.
- Risk scales.

8. Treatment

1. WFSBP 2020 step-by-step algorithm (same as 6D30):
    - CBT for relapse prevention;
    - SSRI high dose;
    - Antiandrogen;
    - GnRH in the most severe case.
2. Legal-medical coordination.

8.1 Treatment methods

Same as 6D30.

9. Prognosis

Reduction of relapse with evidence-based treatment.

10. Myths and misconceptions

Myth 1: “Voyeurism is harmless; the victim remains unaware”

Evidence: Trauma if victim evident; consent violation sexual crime.

Myth 2: “Pornography use is voyeurism”

Evidence: Consensual production and consumption — voyeurism absent; clinical diagnosis focuses on those without consent.

Myth 3: “Internet voyeurism is only new; it has no clinical significance”

Evidence: “Upskirting”, hidden camera — sexual crime; requires clinical and legal intervention.

Myth 4: “Treatment is ineffective”

Evidence: CBT and pharmacotherapy are effective.

11. Sources

1. WHO. ICD-11. 6D31 Voyeuristic disorder. 2024.
2. APA. DSM-5-TR. 2022.
3. Thibaut F., Cosyns P., Fedoroff J.P. et al.; WFSBP Task Force on Paraphilias. The World Federation of Societies of Biological Psychiatry (WFSBP) 2020 guidelines for the pharmacological treatment of paraphilic disorders. World J Biol Psychiatry 2020;21(6):412–490.
4. Långström N., Seto M.C. Exhibitionistic and voyeuristic behavior in a Swedish national population survey. Arch Sex Behav 2006;35(4):427–435.

ICD-11 **6D32**	**6D32 PEDOPHILIC DISORDER** **Pedophilic disorder**
ICD-10 F65.4	Paedophilia
DSM-5-TR F65.4	Pedophilic Disorder

1. Definition and nosology

Paedophilic Disorder (ICD-11: 6D32; DSM-5-TR: F65.4) — persistent and intense sexual arousal toward prepubescent children (typically <13 years) for ≥6 months; the patient has acted on fantasies involving children or experiences significant distress/functional impairment. Patient is at least 16 years and victim at least 5 years younger.

Important: “Pedophilic interest” (paraphilia) and “Pedophilic Disorder” (clinical diagnosis) are distinct — the diagnosis requires functional impairment and/or harm to the child.

2. History

- Krafft-Ebing (1886).
- DSM-III (1980).
- WFSBP 2020 — international guideline for paraphilic disorders.

3. Epidemiology

- Prevalence — precise data are hard to obtain (stigmatised category); DSM-5-TR gives 3–5% as the highest possible figure in the male population; true pedophilic preference ~1%.
- Sex: almost always in males.
- Comorbidity: other paraphilic, ASPD, MDD, anxiety, substance use.

4. Aetiology and pathogenesis

- Biological — neurodevelopmental anomalies (Cantor J.M. reviews — IQ, neuroanatomical differences).
- History of childhood trauma or sexual abuse (but most individuals with pedophilic disorder were not victims themselves; the “abused-abuser” concept is semi-scholarly).
- Comorbid impulsivity and hypersexuality.

5. Clinical features

- Persistent and intense sexual arousal toward prepubertal children.
- The patient has acted on fantasy (sexual abuse of a child) or experiences distress/functional impairment.

- Subtypes (DSM-5-TR): exclusive type (attracted only to children), nonexclusive type (attracted to both children and adults); male/female/both.
- Legal consequences (crime).

6. Diagnosis

6.1 Unified diagnostic criteria

A. ≥6 months repeated and intense sexual arousal towards prepubertal children (typically ≤13).

B. The patient has acted on fantasy or experiences significant distress/functional impairment.

C. Patient ≥ 16 years old and at least 5 years older than the child.

6.2 Differences between sources

- DSM-5-TR requires a 6-month duration and an age rule (patient ≥16 years and at least 5 years older than the child); ICD-11 has neither a duration nor an age criterion — it only excludes behaviour between peers close in age.
- WFSBP 2020.
- Structured risk assessment in forensic psychiatry context.

6.3 Diagnostic algorithm

1. Clinical interview (difficult to confess; patient may conceal).
2. Penile plethysmography — investigation and forensic context.
3. Forensic risk assessment (Static-99R, STABLE-2007).
4. Comorbidity.

6.4 Differential diagnosis

Condition	**Distinguishing feature**
Substance-induced disinhibition	Toxicology.
Dementia (frontal)	Cognitive decline, neurological.
Intellectual disability	In the context of cognitive deficit.
Hypersexuality in adults	No paedophilic focus.
“Hebephilia” / “ephebophilia”	Focus on puberty and post-puberty; DSM-5-TR no separate category.

7. Examination and assessment

- Forensic clinical interview.
- Static-99R, STABLE-2007.
- Penile plethysmography (investigation).
- Comorbidity.

8. Treatment

8.1 General Principles (WFSBP 2020 Stepwise)

1. CBT and prevention of relapse — first-line; trigger management, management of cognitive distortions, empathy development.
2. SSRI high dose — first pharmacotherapy — reduces impulsivity and sexual urge.
3. Antiandrogen (CPA, MPA) — in moderate-to-severe cases; libido and sexual activity decrease.
4. GnRH agonist (leuprolide, triptorelin) — chemical castration; in most severe or high-risk cases; continuous monitoring (osteoporosis, metabolic).
5. Forensic intervention and community monitoring parallel.
6. Multidisciplinary approach — psychiatrist, therapist, social service, legal.

8.2 Treatment methods

1. CBT and Relapse Prevention — Trigger and context management; cognitive distortion; empathy training.
2. SSRI High Dose — First pharmacotherapy.
3. Antiandrogen (CPA, MPA) — Libido reduction.
4. GnRH Agonist — Chemical castration; ongoing monitoring.
5. Static-99R, STABLE-2007 — Forensic risk assessment.

8.3 Differences between sources

- WFSBP 2020 — Thibaut et al. — step-by-step pharmacotherapy.
- STABLE-2007 – dynamic risk factors.

9. Prognosis

- Behavior management improves with evidence-based treatment.
- Comorbid ASPD and substance use — poor prognosis.
- Continuous monitoring critical.

10. Myths and misconceptions

Myth 1: “Pedophilic interest always means child abuse”

Evidence: “Having pedophilic fantasies” and “committing child abuse” are different; some patients never abuse and seek clinical support (e.g., “Dunkelfeld” program in Germany).

Myth 2: “All child sexual abusers are pedophilic”

Evidence: Most child sex offenders are not pedophilic — the offence is situational, with other motivations. Pedophilic diagnosis requires persistent sexual arousal for ≥ 6 months.

Myth 3: “Chemical castration should be mandatory for all patients”

Evidence: WFSBP 2020 step-by-step approach — GnRH in most severe cases; CBT and SSRI first. Ethical and legal caution.

Myth 4: “Once a pedophile, always a pedophile”

Evidence: Sexual interest may be stable, but behavioral control can significantly improve with CBT and pharmacotherapy.

Myth 5: “Treatment is ineffective”

Evidence: WFSBP 2020 and meta-analyses — multimodal intervention reduces recidivism

Myth 6: “The patient is a ‘victim’ of childhood trauma”

Evidence: “Abused-abuser” concept not fully supported; most patients were not abused in childhood; victimhood is not the basis for perpetration.

11. Sources

1. WHO. ICD-11. 6D32 Pedophilic disorder. 2024.
2. APA. DSM-5-TR. 2022.
3. Thibaut F., Cosyns P., Fedoroff J.P. et al.; WFSBP Task Force on Paraphilias. The World Federation of Societies of Biological Psychiatry (WFSBP) 2020 guidelines for the pharmacological treatment of paraphilic disorders. World J Biol Psychiatry 2020;21(6):412–490.
4. Cantor J.M. et al. Cerebral white matter deficiencies in pedophilic men. J Psychiatr Res 2008;42(3):167–183.

5. Beier K.M. et al. Encouraging self-identified pedophiles and hebephiles to seek professional help (Prevention Project Dunkelfeld). Child Abuse Negl 2009;33(8):545–549.

ICD-11 **6D33**	**6D33 COERCIVE SEXUAL SADISM DISORDER** **Coercive sexual sadism disorder**
ICD-10 F65.5	Sadomasochism
DSM-5-TR F65.52	Sexual Sadism Disorder

1. Definition and nosology

Coercive sexual sadism disorder (ICD-11: 6D33; DSM-5-TR: F65.52 Sexual Sadism Disorder) — persistent and intense sexual arousal from inflicting physical or psychological suffering on a non-consenting person. The patient has acted on these fantasies or experiences significant distress/functional impairment.

Important difference: Consensual BDSM (Bondage/Discipline, Dominance/Submission, Sadism/Masochism) practice — not a clinical disorder; ICD-11 emphasizes this distinction.

2. History

- Krafft-Ebing (1886) — term sadism from Marquis de Sade.
- DSM-5 (2013) — difference between paraphilia and paraphilic disorder.
- ICD-11 (2019) — “Coercive” emphasizes the importance of consent.
- WFSBP 2020.

3. Epidemiology

- Rare in clinical populations; 5–15% in forensic contexts of sexual offense samples.
- Almost always in males.
- High comorbidity ASPD, other paraphilic disorders.

4. Aetiology and pathogenesis

- Biological and psychosocial factors; childhood trauma, antisocial traits.
- Hypersexuality and impulsivity are frequently comorbid.

5. Clinical features

- Sexual arousal from inflicting suffering on a non-consenting victim.
- Typical context — forensic offence (rape-sadistic).
- In severe cases — sadistic murders, serious physical injury.
- Forensic risk high.

6. Diagnosis

6.1 Unified diagnostic criteria

A. ≥6 months persistent and intense sexual arousal from physical or psychological suffering of a non-consenting individual.

B. The patient has acted on fantasy or experiences significant distress.

6.2 Differences between sources

- DSM-5-TR requires a 6-month duration; ICD-11 sets no duration criterion — it speaks of a “sustained, focused and intense pattern of arousal”.
- Consensual BDSM is not a clinical disorder.

6.3 Diagnostic algorithm

1. Forensic clinical interview.
2. Risk assessment (Static-99R, SVR-20).
3. Comorbid disorders (ASPD, psychopathy — PCL-R).

6.4 Differential diagnosis

Condition	Distinguishing feature
Consensual BDSM	Consensual context — not a disorder.
ASPD	Violence without a sexual component.
Psychopathy	PCL-R; clinical overlap.
Substance-induced	Toxicology.

7. Examination and assessment

- Forensic clinical interview.
- PCL-R (Psychopathy Checklist), Static-99R, SVR-20.

8. Treatment

1. WFSBP 2020 step-by-step algorithm:
    - CBT and prevention of relapse;
    - SSRI high dose;
    - Antiandrogen;
    - GnRH agonist in the most severe case.
2. Forensic psychiatric supervision; in hospital or correctional setting.
3. In cases with comorbid psychopathy, standard psychotherapy is less effective.

8.1 Treatment methods

1. CBT and Relapse Prevention — Limited effectiveness; limited in psychopathy context.
2. SSRI + Antiandrogen + GnRH — WFSBP 2020 stepwise.
3. PCL-R, Static-99R, SVR-20 — Forensic risk assessment.

8.2 Differences between sources

- WFSBP 2020 — in severe cases combination (CBT + antiandrogen + GnRH).

9. Prognosis

In severe cases, high risk of relapse; multimodal intervention and continuous monitoring are essential.

10. Myths and misconceptions

Myth 1: “BDSM = sadistic disorder”

Evidence: Consensual BDSM is not a clinical disorder; ICD-11 distinguishes precisely with term ‘coercive’. APA and other professional associations do not consider consensual BDSM pathology.

Myth 2: “Sadistic disorder cannot be treated”

Evidence: WFSBP 2020 — multimodal intervention reduces relapse risk; effect is less in comorbid psychopathy.

Myth 3: “Only legal punishment is required”

Evidence: Clinical intervention recommended jointly with legal oversight.

Myth 4: “All sexual murderers have sadistic disorder”

Evidence: Most sex crimes are ASPD or otherwise motivated; sadistic clinical diagnosis requires a concrete fantasy pattern.

11. Sources

1. WHO. ICD-11. 6D33 Coercive sexual sadism disorder. 2024.
2. APA. DSM-5-TR. 2022.
3. Thibaut F., Cosyns P., Fedoroff J.P. et al.; WFSBP Task Force on Paraphilias. The World Federation of Societies of Biological Psychiatry (WFSBP) 2020 guidelines for the pharmacological treatment of paraphilic disorders. World J Biol Psychiatry 2020;21(6):412–490.
4. Hare R.D. Psychopathy Checklist-Revised (PCL-R). 2nd ed. 2003.

ICD-11 **6D34**	**6D34 FROTTEURISTIC DISORDER** **Frotteuristic disorder**
ICD-10 F65.8	Other disorders of sexual preference
DSM-5-TR F65.81	Frotteuristic Disorder

1. Definition and nosology

Frotteuristic disorder (ICD-11: 6D34; DSM-5-TR: F65.81) — persistent and intense sexual arousal from touching or rubbing one's genitals against a non-consenting person. The patient has acted on the fantasies or experiences significant distress/functional impairment. Typical context — crowded public transport and crowds.

2. History

- “Frotteur” — from the French word for “to rub”.
- DSM-IV (1994) — ‘Other Specified Paraphilia’; DSM-5 (2013) — separate category.

3. Epidemiology

- Prevalence: frotteuristic acts may occur in as many as 30% of adult males; 10–14% of adult males seen as outpatients for paraphilic disorders and hypersexuality meet the diagnostic criteria (DSM-5-TR).
- Comorbid other paraphilic disorders.

4. Aetiology and pathogenesis

General etiology for paraphilia; comorbid hypersexuality and impulsivity.

5. Clinical features

- Touching an unwilling person in crowded public places (bus, metro, crowd)
- Patient escapes — victim cannot complain immediately.
- Legal consequences (sexual harassment, sexual crime).

6. Diagnosis

6.1 Unified diagnostic criteria

A. ≥6 months persistent and intense sexual arousal from touching or rubbing against a non-consenting individual.

B. The patient has acted on fantasy or experiences significant distress.

6.2 Differences between sources

DSM-5-TR / ICD-11 — same; WFSBP 2020.

6.3 Diagnostic algorithm

1. Clinical interview.
2. Risk assessment.
3. Comorbidity.

6.4 Differential diagnosis

Condition	Distinguishing feature
Accidental touching (in dense crowd)	Consent issues and fantasy not present.
ASPD	Broad antisocial model.
Manic episode	Affective context.

7. Examination and assessment

- Forensic clinical interview.
- Risk scales.

8. Treatment

1. WFSBP 2020 step-by-step:
    - CBT and prevention of relapse;
    - SSRI high dose;
    - Antiandrogen / GnRH in severe cases.
2. Legal-medical coordination.

8.1 Treatment methods

1. CBT and SSRIs + WFSBP 2020 protocol — same as 6D30/6D31/6D32 tiered.

9. Prognosis

Reduction of relapse with WFSBP intervention.

10. Myths and misconceptions

Myth 1: "Frotteurism is harmless"

Evidence: Trauma for victim; sexual crime and legal consequences.

Myth 2: "Just an impulse — does not require clinical intervention"

Evidence: repeated pattern requires clinical diagnosis.

Myth 3: "Only legal punishment"

Evidence: Clinical intervention reduces relapse.

11. Sources

1. WHO. ICD-11. 6D34 Frotteuristic disorder. 2024.
2. APA. DSM-5-TR. 2022.
3. Thibaut F., Cosyns P., Fedoroff J.P. et al.; WFSBP Task Force on Paraphilias. The World Federation of Societies of Biological Psychiatry (WFSBP) 2020 guidelines for the pharmacological treatment of paraphilic disorders. World J Biol Psychiatry 2020;21(6):412–490.

6D50–6D5Z FACTITIOUS DISORDERS

Factitious disorders

ICD-11 **6D50**	**6D50 FACTITIOUS DISORDER IMPOSED ON SELF** **Factitious disorder imposed on self**
ICD-10 F68.1	Intentional production or feigning of symptoms or disabilities, either physical or psychological [factitious disorder]
DSM-5-TR F68.10	Factitious Disorder Imposed on Self

1. Definition and nosology

Factitious disorder imposed on self (ICD-11: 6D50; DSM-5-TR: F68.10) — intentional production of physical or psychological symptoms or injury in oneself, presentation of these symptoms to others; no obvious external incentive (distinguishing it from malingering). The patient's primary motivation is to assume the sick role or be perceived as a patient.

2. History

- Asher R. (1951) — term “Munchausen syndrome” (from Baron Munchausen's fantastic stories).
- DSM-III (1980) — Formalization of Factitious Disorder.
- DSM-5 (2013) — “Imposed on Self” and “Imposed on Another” subtypes.
- Eisendrath S.J. reviews — diagnosis and clinical approach.

3. Epidemiology

- Rare in clinical populations (1–2% of hospital admissions); under-detection in clinical approach.
- Sex: relatively higher in females (in the context of medical profession).
- Comorbidity: BPD, depressive disorders, history of trauma.

4. Aetiology and pathogenesis

- Psychological — deep emotional need in medical role; childhood trauma, neglect.
- Medical system proximity (being a healthcare professional).
- BPD is frequently comorbid.

5. Clinical features

- Multiple medical consultations and examinations; numerous clinic visits ("hospital shopping").
- Multiple surgical procedures.
- Intentional creation of symptom or injury (bleeding, creating infection, poisoning with medication, fabricating somatic complaint).
- Clinical presentation "impossible" or atypical; laboratory results contradict the patient's account.
- The patient may lie during the interview; the medical history is unreliable.
- No overt material gain (insurance, legal, work).

6. Diagnosis

6.1 Unified diagnostic criteria

A. Deliberate induction or feigning of physical or psychological symptoms or injury.

B. The patient presents themselves as ill or injured.

C. Deceptive behavior with no apparent material gain.

D. The behavior is not better explained by another mental disorder (delusional disorder, other psychotic).

6.2 Differences between sources

- DSM-5-TR / ICD-11 — same.
- Diagnosis is difficult — requires evidence (insurance documents, information from other clinics, video).

6.3 Diagnostic algorithm

1. Clinical suspicion — atypical clinical presentations, multiple clinics, "impossible" symptom.
2. Obtain medical history from other clinics (with patient consent).

3. Insurance and pharmacy records.
4. Observation — method of creating laboratory samples.
5. Comorbid BPD, MDD, trauma history.
6. Legal and ethical coordination.

6.4 Differential diagnosis

Condition	Distinguishing feature
Real medical illness	Clinical confirmation; however, factitious disorder may be comorbid (complex cases).
Somatic symptom disorder (6C20)	Conscious lie absent; patient feels symptoms as real.
Conversion (6B60)	Not conscious; functional disturbance of the nervous system.
Malingering	Overt material/legal gain (insurance, legal).
Delusional disorder — somatic	Insight absent.

7. Examination and assessment

- Medical history (from other clinics).
- Insurance records.
- Psychiatric interview for comorbid disorders.

8. Treatment

1. Non-confrontational approach (Eisendrath) — the diagnosis is explained to the patient within a framework of care rather than punishment; “your distress is real, we can help in another way”.
2. Single physician model — all complaints are managed through that physician.
3. Limitation of invasive procedures and unnecessary examinations.
4. Psychotherapy — supportive, CBT; long-term follow-up.
5. Comorbid BPD, MDD, trauma treatment.
6. Confrontation abruptly ‘burns’ the patient; better in a gentle and caring context.

8.1 Treatment methods

1. Supportive Psychotherapy — Attention to patient's emotional needs.
2. Multidisciplinary approach — Medical and psychiatric combined.

8.2 Differences between sources

- Eisendrath S.J. — non-confrontational approach and therapeutic alliance.
- APA Textbook of Consultation-Liaison Psychiatry (Levenson).

9. Prognosis

- Chronic course; “hospital shopping” continues.
- Long-term therapeutic alliance and treatment of comorbidities improve prognosis.

10. Myths and misconceptions

Myth 1: “The patient is a ‘liar’, legal punishment is required”

Evidence: Factitious disorder is a psychiatric disorder — response to emotional need; a non-confrontational approach and psychotherapy are recommended.

Myth 2: “Directly saying ‘you are lying’ will ‘correct’ the patient”

Evidence: a harsh confrontation causes the patient to “flee” and transfer to another clinic; a gentle approach builds a therapeutic alliance.

Myth 3: “Factitious disorder = somatic symptom disorder”

Evidence: Factitious deliberate lying; somatic symptom — patient's genuine feeling.

Myth 4: “Invasive procedures that have been started must be carried through”

Evidence: Unnecessary invasive procedures are harmful and should be limited after a diagnosis is made.

Myth 5: “Factitious disorder cannot be treated”

Evidence: Evidence base is limited, but improvement is possible with long-term therapeutic alliance and multidisciplinary care.

11. Sources

1. WHO. ICD-11. 6D50 Factitious disorder imposed on self. 2024.

2. APA. DSM-5-TR. 2022.
3. Eisendrath S.J. Factitious physical disorders: treatment without confrontation. Psychosomatics 1989;30(4):383–387.
4. Levenson J.L. (ed.). APA Publishing Textbook of Psychosomatic Medicine and Consultation-Liaison Psychiatry. 3rd ed. 2018.
5. Asher R. Munchausen's syndrome. Lancet 1951;1(6650):339–341.

	6D51 FACTITIOUS DISORDER IMPOSED ON ANOTHER
ICD-11 **6D51**	**Factitious disorder imposed on another**
ICD-10 F68.1	Intentional production or feigning of symptoms or disabilities, either physical or psychological [factitious disorder]
DSM-5-TR F68.A	Factitious Disorder Imposed on Another

1. Definition and nosology

Factitious disorder imposed on another (ICD-11: 6D51; DSM-5-TR: F68.10) — the patient creates or feigns physical or psychological symptoms in another person (typically their own child, elderly or disabled dependent); they bring that person to the hospital as a victim. Originally termed ‘Munchausen by proxy’ (Meadow R. 1977).

The diagnosis is made in the perpetrator, not in the victim. This disorder is at the same time child abuse — child protection and legal intervention are mandatory.

2. History

- Meadow R. (1977) — ‘Munchausen syndrome by proxy’ Lancet.
- DSM-IV (1994) — In the research category.
- DSM-5 (2013) — official diagnosis.
- APSAC, AAP — child protection protocols.

3. Epidemiology

- Rare — annual incidence ~2/100,000 children (Sheridan M.S. Child Abuse Negl 2003 review); under-detection.
- Typical perpetrator — biological mother (>90%); victim — child (typically <5 years).
- Victims' mortality 6–9% (Sheridan); morbidity high.

4. Aetiology and pathogenesis

- Psychological needs of the patient — attention in the medical role, relationships, ‘hero’ image.
- Occupational burnout.
- History of trauma or neglect in the patient.

5. Clinical features

- Child's symptoms occur only with mother; improve when the child is put to bed and the mother is absent.
- Multiple clinics, atypical symptoms, “impossible” medical history.
- Mother — a medical professional or has extensive medical knowledge.
- The mother appears overly polite and an “ideal caregiver.”
- Typical induction methods: drug poisoning, mechanical obstruction of food or airway, blood collection, infection creation.
- Victim child with unexplained recurrent seizures, apnea, hypoglycemia, infection, skin injury.

6. Diagnosis

6.1 Unified diagnostic criteria

A. Deliberate induction of physical or psychological symptoms or harm in another person (victim).

B. The patient presents the victim to others as ill or injured.

C. Deceptive behavior with no apparent material gain.

D. The behavior is not better explained by another mental disorder.

The diagnosis is made in the perpetrator, not in the victim.

6.2 Differences between sources

- DSM-5-TR / ICD-11 — same.
- APSAC, AAP — child protection protocols; this clinical condition also category of child abuse.

6.3 Diagnostic algorithm

1. Clinical suspicion — atypical symptom pattern, related to mother.
2. Pediatrician, psychiatrist, social worker multidisciplinary team.
3. Obtaining medical documentation from other clinics.
4. Separation test — monitoring symptoms when the patient is separated from the caregiver.
5. Video monitoring (under child protection and legal intervention)
6. Toxicological and biochemical tests (evidence of creation).
7. Referral to child protection and legal coordination is mandatory.

6.4 Differential diagnosis

Condition	Distinguishing feature
Real medical illness	Medical proof.
Overanxious parent	The deliberate induction does not exist; it expresses medical suspicions but does not create them.
Real child abuse (other form)	Physical abuse, sexual abuse, neglect — different motivation.
Factitious disorder imposed on self (6D50)	Victim is self.

7. Examination and assessment

- Pediatric clinical assessment.
- Medical history gathering.
- Social service and child protection referral.
- Legal coordination.

8. Treatment

1. Victim's safety is the first priority — child separation (from mother or caregiver).
2. Child protection referral — legal protocols.
3. The patient's (perpetrator's) psychiatric treatment:
    - Psychotherapy (long-term);
    - Comorbid disorders (BPD, MDD, trauma);
    - Medical occupational context — occupational prohibitions may be considered.
4. Family intervention and sibling protection assessment.
5. Victim child's psychotherapy – trauma intervention.

8.1 Treatment methods

1. Child Protection Referral — Absolutely when suspicion exists; APSAC, AAP protocols.
2. Multidisciplinary Group — Pediatrician + psychiatrist + social worker + forensic specialist.
3. Separation Test — The mother is separated from the victim; symptoms are observed.
4. Video Monitoring — Under legal and ethical supervision.

8.2 Differences between sources

- APSAC — child protection protocols.
- AAP — pediatrician role.
- Coordination of the legal system is mandatory.

9. Prognosis

- For the victim – high mortality (6–9%) and morbidity without intervention.
- For the patient — improvement with psychotherapy; long-term follow-up.

10. Myths and misconceptions

Myth 1: “The mother is truly caring, the diagnosis is wrong”

Evidence: Clinical features (mother-attributed symptoms, atypical medical history) require substantial proof; clinical suspicion is sufficient for referral to child protection.

Myth 2: “This condition is not a psychiatric problem; it is just child abuse”

Evidence: both approaches important — child protection + perpetrator psychiatric treatment.

Myth 3: “Treatment of the perpetrator delays cessation of care for the victim”

Evidence: Victim safety first priority; psychiatric treatment separately continued.

Myth 4: “It only occurs in the mother”

Evidence: In most cases mother, but other caregivers — fathers, carers, healthcare professionals — can also be the perpetrator.

Myth 5: “Confrontation ‘treats’ the patient”

Evidence: Confrontation may necessitate transfer to another clinical unit; the patient is being followed up; child protection and forensic coordination are mandatory.

11. Sources

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6D70–6D8Z NEUROCOGNITIVE (DEMENTIA)

Neurocognitive disorders

ICD-11 **6D70**	**6D70 DELIRIUM** **Delirium**
ICD-10 F05	Delirium, not induced by alcohol and other psychoactive substances
DSM-5-TR F05	Delirium

1. Definition and nosology

Delirium (ICD-11: 6D70; DSM-5-TR: F05) — a neuropsychiatric syndrome characterized by an acute, fluctuating disturbance of attention and consciousness, cognitive deficit (memory, orientation, language, visuospatial) and perceptual disturbances (illusions, hallucinations). Develops in response to a typical medical cause (infection, medication, metabolic, neurological).

This is a medical emergency — the mortality rate for hospitalized elderly patients is 25–33%.

2. History

- Hippocrates “phrenitis” — classic description of delirium.
- Lipowski Z.J. (1980-90s) — modern conceptualization.
- CAM (Confusion Assessment Method, Inouye S.K. 1990) — gold standard screening.

3. Epidemiology

- Prevalence in hospitalized elderly 15–30%; in ICU 50–80%; post-surgery 15–50%.
- Comorbidity: dementia (delirium superimposed on dementia), infection, polypharmacy.
- Mortality and morbidity high; risk of long-term cognitive decline.

4. Aetiology and pathogenesis

- Multiple factors:
  - Infections — sepsis, pneumonia, UTI;
  - Medication — anticholinergics, opioids, benzodiazepines, steroids;
  - Substance withdrawal (alcohol, benzodiazepine);
  - Metabolic — electrolyte, glucose, ammonia, uremia;
  - Hypoxia, hypoperfusion;
  - Neurological — seizure, stroke, brain tumor, encephalitis;
  - Pain, constipation, urinary retention;
  - Environment — sensory deprivation, sleep disruption (especially in ICU).

5. Clinical features

Three motor subtypes

- Hyperactive — agitation, hallucination — diagnosis easy;
- Hypoactive — sedation, withdrawal — frequently underdiagnosed, resembles depression.
- Mixed — fluctuation.

Clinical signs

- Acute onset (hours-days) and fluctuation.
- Attention impairment.
- Cognitive deficits — memory, orientation (time, place, person).
- Perceptual disturbances – visual hallucinations typical.
- Sleep-wake cycle disruption.
- Sundowning — worsening in evening hours.

6. Diagnosis

6.1 Unified diagnostic criteria

A. Attention impairment (orientation, maintenance, shifting) and consciousness impairment.

B. Disorder develops over short period (hours–days); differs from baseline indicators; fluctuates within day.

C. Additional cognitive impairment (memory, orientation, language, visuospatial, perception).

D. Not fully explained by dementia or another neurocognitive disorder

E. Evidence — history, examination, laboratory indicate that disturbance is directly due to medical condition, substance intoxication/withdrawal, or multiple etiologies.

6.2 Differences between sources

- DSM-5-TR / ICD-11 — same.
- NICE CG103 — Delirium: prevention, diagnosis, management (2010, 2019 update).

6.3 Diagnostic algorithm

1. CAM (Confusion Assessment Method) — gold standard screening.
2. CAM-ICU — for the intensive care unit.
3. Medical cause investigation: complete blood count, electrolytes (Na, K, Ca, Mg, P), glucose, liver, kidney, TSH, B12, troponin, EKG, urine analysis and culture, blood culture, chest X-ray, brain MRI/CT, EEG (seizure suspicion), lumbar puncture (encephalitis/meningitis).
4. Medication list review — anticholinergics, opioids, benzodiazepines (Beers Criteria).
5. Toxicology.
6. Comorbid dementia assessment (initial indicators).

6.4 Differential diagnosis

Condition	Distinguishing feature
Dementia (6D8x)	Gradual onset; attention relatively preserved.
Depression (psychomotor retardation as hypoactivity)	Affective symptoms are dominant; attention is relatively preserved.
Psychotic episode	Attention is preserved; onset is different.
Non-convulsive status epilepticus	EEG.
Wernicke encephalopathy	Thiamine test; classic triad (ophthalmoplegia, ataxia, confusion).

7. Examination and assessment

- CAM, CAM-ICU.
- MMSE or MoCA (initial indicators of cognitive function).
- From the above medical panel.
- EEG — rule out non-convulsive status.

8. Treatment

8.1 General principles (NICE CG103)

1. Etiological treatment is the first priority — correction of infection, metabolic disturbance, medication, and the hospitalisation context.
2. Environmental modification — sensory (glasses, hearing aid), daylight, nighttime darkness, quiet environment, presence of loved ones.
3. Bedside care — hydration, nutrition, urination and defecation, movement.
4. Antipsychotic (haloperidol 0.25–2 mg, quetiapine, risperidone) — only for severe agitation or when the patient poses a danger to self/others, short-term, low dose. NICE CG103 — antipsychotics not routinely recommended; evidence base is limited (Burry L. Cochrane 2018).
5. Benzodiazepine — only in alcohol or benzodiazepine withdrawal delirium; otherwise is contraindicated (exacerbates delirium).
6. Prevention — ABCDEF bundle in ICU (Awakening, Breathing, Coordination, Delirium monitoring, Early mobility, Family).

8.2 Treatment methods

1. Confusion Assessment Method (CAM — Confusion Assessment Method) — Inouye (Inouye S.K.) — 4 core features: acute onset/fluctuation + inattention + disorganized thinking or altered consciousness.
2. CAM-ICU — Adapted for intensive care; in patients on ventilator.
3. Hospital Elder Life Program (HELP — Hospital Elder Life Program) — Inouye (Inouye S.K.) — Multicomponent delirium prevention program; evidence base strong.

8.3 Differences between sources

- NICE CG103 — etiological and environmental approach first-line; antipsychotics limited.
- SCCM PADIS Guidelines (for ICU, 2018).

- Burry L. Cochrane 2018 — antipsychotics do not reduce delirium duration or mortality.

9. Prognosis

- Hospital mortality 25–33%.
- In survivors — functional decline, long-term cognitive impairment, increased dementia risk.
- Early etiological intervention improves prognosis.

10. Myths and misconceptions

Myth 1: "Delirium is 'normal for an elderly person'"

Evidence: Delirium is a clinical emergency with high mortality; it requires active intervention.

Myth 2: "Antipsychotics are a treatment for delirium"

Evidence: NICE CG103, Burry Cochrane 2018 — antipsychotics do not reduce delirium duration or mortality; only for severe agitation.

Myth 3: "Benzodiazepine is safe for an elderly agitated patient"

Evidence: benzodiazepines exacerbate delirium; only in the case of alcohol/BZ withdrawal.

Myth 4: "Hypoactive delirium is 'good' and requires no intervention"

Evidence: hypoactive delirium linked to worse mortality; problem of under-detection.

Myth 5: "Sleeping is best for the patient with delirium"

Evidence: Daytime activity, presence of close ones, sensory stimulation are recommended; HELP program is evidence-based.

11. Sources

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ICD-11 **6D71**	**6D71 MILD NEUROCOGNITIVE DISORDER (MCI)** **Mild neurocognitive disorder**
ICD-10 F06.7	Mild cognitive disorder
DSM-5-TR G31.84	Mild Neurocognitive Disorder

1. Definition and nosology

Mild neurocognitive disorder (ICD-11: 6D71; DSM-5-TR: G31.84) — mild decline in one or more cognitive domains (memory, complex attention, executive function, language, visuospatial, social cognition); Daily activity is maintained (though compensation may be needed). Typical name — “Mild Cognitive Impairment” (MCI).

2. History

- Petersen R.C. (1999) — clinical conceptualization of MCI.
- DSM-5 (2013) — Major (dementia) and Mild Neurocognitive Disorders categories.
- AAN (2018) — Practice Guideline for MCI (Petersen).

3. Epidemiology

- Prevalence 10–20% in those aged 65+.
- Annual 10–15% of MCI transition to Alzheimer dementia.
- Risk factors: age, ApoE4, cardiovascular.

4. Aetiology and pathogenesis

- Alzheimer's prodromal (amnestic MCI);
- Vascular
- Lewy bodies prodromal;
- Frontotemporal;
- Medical (depression, thyroid, B12, medication effects).

5. Clinical features

- The patient or a close person complains of cognitive decline.
- Mild deficit on standardized tests (typically -1 to -2 SD).
- Daily functioning is preserved (minimal compensation in complex activities).
- Subtypes: amnestic (memory dominant — high Alzheimer's risk), non-amnestic (executive or other domain).

6. Diagnosis

6.1 Unified diagnostic criteria

A. Mild decline in one or more cognitive domains (noted by patient, informant, or clinician, and standardized neuropsychological testing).

B. Daily functioning is preserved.

C. Not better explained by delirium

D. Not better explained by another mental disorder (MDD).

6.2 Differences between sources

- DSM-5-TR — separation of Major and Mild.
- AAN 2018 — etiological investigation (Alzheimer, vascular) and biomarkers in developmental stage.

6.3 Diagnostic algorithm

1. Clinical interview + informant.
2. MoCA (Montreal Cognitive Assessment), MMSE.
3. Neuropsychological testing.
4. Laboratory — B12, folate, TSH, liver, kidney, glucose, HIV/syphilis.
5. Brain MRI — vascular, atrophy, other pathology.

6. Biomarkers (if clinical suspicion) — CSF amyloid-β/tau, PET imaging (Alzheimer's prodrome).
7. Depression screening (GDS, PHQ-9).

6.4 Differential diagnosis

Condition	Distinguishing feature
Dementia (Major NCD)	Daily functioning is significantly impaired.
Normal age-related cognitive change	Performance normal during testing.
Depression (pseudodementia)	Affective symptoms are dominant; improves with antidepressants.
Delirium (6D70)	Acute onset, fluctuating attention.
B12 or thyroid deficiency	Laboratory; improvement with correction.

7. Examination and assessment

- MoCA (≤ 25 pathological) — the validated screen for MCI; MMSE has no established MCI cut-off and is insensitive to mild impairment.
- Neuropsychological battery (memory, executive, language, visuospatial).
- Laboratory panel.
- MRI (routine), biomarkers (selective).

8. Treatment

1. Etiological treatment — modifiable factors (cardiovascular risk, B12, thyroid, depression, medication).
2. Lifestyle — physical activity (aerobic), Mediterranean diet, cognitive stimulation, social connections (FINGER trial Lancet 2015 – multimodal approach effective).
3. Pharmacotherapy — no specific approval for MCI. Donepezil and other AChEIs — limited evidence base; not routinely recommended.
4. Lecanemab, Donanemab — new anti-amyloid antibodies; FDA approval (lecanemab 2023, donanemab 2024) for Alzheimer prodrome and mild dementia; clinical effect modest, infusion reactions and ARIA (Amyloid-Related Imaging Abnormalities) risk.
5. Comorbid depression, anxiety treatment (SSRIs).

6. Monitoring risk of transition to dementia.

8.1 Treatment methods

1. Montreal Cognitive Assessment (MoCA) — Nasreddine Z.S — 30 items; MCI screening (cut-off 26).
2. FINGER Multimodal Intervention (FINGER — Finnish Geriatric Intervention Study) — Diet + exercise + cognitive training + vascular risk management.
3. Lecanemab, Donanemab — Anti-amyloid antibodies; FDA approval; ARIA monitoring (MRI).

8.2 Differences between sources

- AAN 2018 — modifiable factors and lifestyle.
- FINGER trial (Ngandu Lancet 2015) — multimodal intervention.
- Lecanemab — CLARITY-AD (van Dyck NEJM 2023); donanemab — TRAILBLAZER-ALZ 2 (Sims JAMA 2023).

9. Prognosis

- Stable 30%, transition to dementia 10–15%/year, reversion to normal cognition 15–20%.
- Amnestic MCI as Alzheimer's prodrome — high conversion rate.

10. Myths and misconceptions

Myth 1: “MCI inevitably turns into dementia”

Evidence: annual rate 10–15%; some remain stable or improve.

Myth 2: “Vitamin B and ginkgo cure MCI”

Evidence: B vitamins only in deficiency; ginkgo Cochrane (Birks 2009) — weak evidence of effect.

Myth 3: “AChEI is the standard treatment for MCI”

Evidence: AAN 2018 — evidence base insufficient; not routinely recommended.

Myth 4: “Lecanemab cures MCI/Alzheimer's”

Evidence: Reduces clinical decline rate by ~25%; ARIA and infusion reactions; not “cure.”

Myth 5: “Cognitive decline is inevitable with age”

Evidence: Cardiovascular risk, physical activity, social connections are modifiable factors.

11. Sources

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ICD-11 **6D72**	**6D72 AMNESTIC DISORDER** **Amnestic disorder**
ICD-10 F04	Organic amnesic syndrome, not induced by alcohol and other psychoactive substances
DSM-5-TR F02.8x	Major Neurocognitive Disorder Due to Another Medical Condition (amnestic presentation)

1. Definition and nosology

Amnestic Disorder (ICD-11: 6D72; DSM-5-TR: F02.8x and similar) — significant impairment in learning and recalling new information; other cognitive domains relatively preserved. Due to a medical condition (Wernicke-Korsakoff, brain injury, encephalitis, anoxia), substance effect, or other causes.

2. History

- Korsakoff S. (1887) — description of alcohol-related amnesia.
- Scoville W.B., Milner B. (1957) — H.M. case (amnesia after bilateral medial temporal resection).

3. Epidemiology

- Korsakoff syndrome — in 12–25% of chronic heavy alcohol users.
- Amnesia after traumatic brain injury — in 30–50% of TBI patients.
- Transient global amnesia (TGA) — prevalence in age 65+ 5–25/100,000/year.

4. Aetiology and pathogenesis

- Wernicke-Korsakoff — thiamine (B1) deficiency (alcohol, malnutrition).
- Traumatic brain injury (TBI).
- Hypoxia/anoxia — cardiac arrest, carbon monoxide.
- Herpes simplex encephalitis.
- Autoimmune encephalitis (anti-NMDA, anti-LGI1).
- Transient global amnesia (TGA) — suspected vascular etiology.
- Substance (alcohol blackout, benzodiazepine).
- Stroke (especially bilateral thalamic or hippocampal).

5. Clinical features

- Anterograde amnesia — impairment in learning new information (core feature).
- Retrograde amnesia — memory for the period before the injury (“Ribot's law” — recent memories are lost earlier than remote ones).
- Confabulation — in Korsakoff syndrome — patient fills memory gaps with fabrications.
- Other cognitive domains are relatively preserved (intellect, language, visuospatial).

6. Diagnosis

6.1 Unified diagnostic criteria

A. Impairment in learning new information or impairment in recall of previously learned information.

B. Memory impairment causes significant functional impairment.

C. Does not occur exclusively in the context of delirium or another NCD.

D. Evidence — history, examination, laboratory indicate that memory impairment is due to specific cause (medical condition, substance).

6.2 Differences between sources

- DSM-5-TR — integrated into Major and Mild NCD structure; amnestic subtype.
- ICD-11 — separate category.

6.3 Diagnostic algorithm

1. Clinical interview + informant.
2. Neuropsychological test — memory (WMS-IV, RAVLT, Wechsler Logical Memory).
3. Brain MRI — bilateral hippocampal, medial temporal, thalamic pathology.
4. Vitamin B1 level or empirical thiamine (suspected Korsakoff syndrome).
5. Toxicology.
6. Autoimmune encephalitis panel (in atypical cases).
7. EEG.

6.4 Differential diagnosis

Condition	Distinguishing feature
Alzheimer's dementia	Multiple cognitive domains, gradual onset.
Delirium (6D70)	Fluctuation of attention.
Dissociative amnesia (6B61)	In trauma context; selective past memory; no medical cause.
Factitious disorder / malingering	Atypical pattern, secondary gain.

Condition	Distinguishing feature
TGA	Acute, less than 24 hours; complete recovery.

7. Examination and assessment

- WMS-IV (Wechsler Memory Scale), RAVLT.
- Brain MRI.
- Thiamine, B12.
- Toxicology.

8. Treatment

1. Etiological — thiamine (Wernicke-Korsakoff — 500 mg IV × 3 times/day × 3 days, then 250 mg IM/day × 3–5 days; long-term oral thiamine in alcohol withdrawal).
2. Autoimmune encephalitis — immunotherapy (steroids, IVIG, plasmapheresis, rituximab).
3. Herpes encephalitis — acyclovir.
4. Compensatory strategies (external memory aids, daily structuring).
5. Treatment of substance use (alcohol).
6. Rehabilitation — cognitive rehabilitation (Wilson B.A.).

8.1 Treatment methods

1. Wernicke-Korsakoff Prophylaxis and Treatment — High-dose IV/IM thiamine; administration BEFORE glucose is critical (glucose exacerbates Wernicke's in thiamine deficiency).
2. Cognitive Rehabilitation — Wilson (Wilson B.A.) — External memory aids (journal, phone reminders), strategy teaching.
3. Immunotherapy (Autoimmune Encephalitis) — Graus 2016 protocol.

8.2 Differences between sources

- Wernicke-Korsakoff — Royal College of Physicians thiamine protocol.
- Graus F. et al. Lancet Neurol 2016 — autoimmune encephalitis.

9. Prognosis

- TGA — full recovery.

- Korsakoff — residual deficit typical; acute intervention with thiamine for Wernicke critical.
- TBI — partial recovery with rehabilitation.

10. Myths and misconceptions

Myth 1: “Amnesia can be ‘recovered’ with hypnosis”

Evidence: hypnosis generates false memory; ineffective in organic amnesia.

Myth 2: “Oral thiamine is sufficient in Wernicke's”

Evidence: Oral absorption insufficient; IV/IM high dose required.

Myth 3: “Glucose ‘cures’ Wernicke's”

Evidence: Glucose given in thiamine deficiency precipitates or worsens Wernicke's encephalopathy; administer thiamine first.

Myth 4: “All amnesia is temporary”

Evidence: Korsakoff, TBI-based amnesia typically chronic; rehabilitation yields improvement, but full recovery rare.

Myth 5: “Confabulation is lying”

Evidence: Confabulation in Korsakoff syndrome is a neurobiological phenomenon, not a conscious lie.

11. Sources

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ICD-11 **6D80**	**6D80 DEMENTIA DUE TO ALZHEIMER DISEASE** **Dementia due to Alzheimer disease**
ICD-10 F00	Dementia in Alzheimer disease
DSM-5-TR G30.9 + F02.8x	Major Neurocognitive Disorder Due to Alzheimer's Disease

1. Definition and nosology

Dementia due to Alzheimer's disease (ICD-11: 6D80; DSM-5-TR: G30.9 + F02.8x Major Neurocognitive Disorder due to Alzheimer's Disease) — gradual and persistent cognitive decline, initially episodic memory, then multiple cognitive domains and significant impairment of daily functioning. Most common cause of dementia (60–70%).

2. History

- Alzheimer A. (1906) — first clinical and pathohistological description.
- Amyloid hypothesis (Hardy J., Higgins G.A. Science 1992).
- NICE NG97 (2018) — dementia clinical guideline.
- FDA — aducanumab (2021, controversial), lecanemab (2023), donanemab (2024) — anti-amyloid antibodies.

3. Epidemiology

- Global 55 mln+ dementia patients (WHO 2023); 60–70% Alzheimer's.
- Prevalence by age: 65 years 5%, 80+ years 25%+.
- Sex: higher in females (related to life expectancy).
- Comorbidity: cardiovascular, depression, anxiety, BPSD (behavioral and psychological symptoms of dementia).

4. Aetiology and pathogenesis

- Familial Alzheimer's (1%): APP, PSEN1, PSEN2 mutations (early onset, dominant).

- Sporadic (99%) – multiple genes (ApoE4 strongest risk; LOAD GWAS loci).
- Modifiable factors (Lancet Commission 2024) — alongside age, ApoE4: low education, hearing loss, hypertension, obesity, smoking, depression, physical inactivity, social isolation, diabetes, air pollution, alcohol, TBI, vision loss, high LDL.
- Pathogenesis — amyloid-β plaques + tau neurofibrillary tangles + neuroinflammation.

5. Clinical features

- Typical onset — episodic memory decline (difficulty retaining new information).
- Stages:
    - Early — memory, anomia, orientation difficulty;
    - Moderate — visual-spatial, executive function, daily activities require support;
    - Late — speech severely limited, loss of recognition of relatives, restricted mobility, feeding and swallowing difficulties.
- BPSD — agitation, aggression, delusions (particularly theft), hallucinations, depression, anxiety, sleep disturbances, sundowning.
- Atypical forms: posterior cortical atrophy (visuospatial dominant), logopenic primary progressive aphasia (language dominant).

6. Diagnosis

6.1 Unified diagnostic criteria

A. Major NCD criteria (≥ 1 cognitive domain significant decline + impairment in daily activities).

B. Clinical characteristics consistent with Alzheimer's (gradual, continuous decline, memory predominant).

C. Probable AD — genetic mutation and/or 3 elements (memory + 1+ cognitive domain, sustained decline, no other cause).

D. Possible AD — criteria not fully met, but atypical clinical presentation or comorbidity.

6.2 Differences between sources

- NIA-AA Research Framework (Jack C.R. 2018) — biomarker-based diagnosis (ATN — amyloid, tau, neurodegeneration).
- NICE NG97 — based on clinical diagnosis; biomarkers selective.
- AAN — diagnostic tools.

6.3 Diagnostic algorithm

1. Clinical interview + informant.
2. MoCA, MMSE, ACE-III.
3. Neuropsychological battery.
4. Laboratory — B12, folate, TSH, liver, kidney, glucose, HIV/syphilis (medical cause exclusion).
5. Brain MRI (atrophy, vascular changes, other pathology).
6. Biomarkers (selective, if clinical suspicion):
    - CSF amyloid-β42 (low), tau (high);
    - Amyloid PET, tau PET;
    - Blood biomarkers (p-tau217, GFAP) — are under development.
7. Comorbid depression screening (GDS).

6.4 Differential diagnosis

Condition	Distinguishing feature
Vascular dementia (6D81)	Stepwise descent, focal neurological signs, vascular MRI.
Lewy bodies (6D82)	Visual hallucinations, parkinsonism, REM sleep behavior disorder
Frontotemporal (6D83)	Personality/conduct or language dominant; earlier onset.
Delirium (6D70)	Acute, fluctuating attention.
Depressive pseudodementia	Affective, improvement with antidepressants.
Normal pressure hydrocephalus (NPH)	Classic triad (gait disturbance, urinary incontinence, cognitive decline).

Condition	Distinguishing feature
Medical (B12, thyroid, neurosyphilis, HIV)	Laboratory.

7. Examination and assessment

- MoCA, MMSE, ACE-III, neuropsychological.
- Laboratory panel.
- MRI, biomarkers.
- BPSD scales (NPI — Neuropsychiatric Inventory).

8. Treatment

8.1 General Principles (NICE NG97 · AAN · APA)

1. AChEI (donepezil, galantamine, rivastigmine) — for mild-to-moderate Alzheimer; modestly slows cognitive decline. Donepezil 5–10 mg (later 23 mg), rivastigmine patch.
2. Memantine (NMDA antagonist) — for moderate-to-severe Alzheimer's; combination with AChEI.
3. Anti-amyloid antibodies (lecanemab, donanemab) — fDA approval for early symptomatic AD (lecanemab 2023, donanemab 2024); reduces clinical decline rate by ~25%; ARIA (Amyloid-Related Imaging Abnormalities – oedema, cerebral microhaemorrhage) risk high (especially in ApoE4 homozygotes); MRI monitoring crucial; infusion. Clinical benefit is debated.
4. Management of BPSD:
    - First-line — non-pharmacological (environment, social support, physical activity, music therapy);
    - In refractory cases — atypical antipsychotic (risperidone, quetiapine) at low dose, short-term; FDA black box — antipsychotics increase mortality in elderly dementia patients (stroke, cardiovascular).
    - SSRI — comorbid depression/anxiety;
    - Trazodone — for sleep disturbance;
    - Benzodiazepine — is contraindicated (falls, cognitive decline).
5. Carer support and psychoeducation – a critical component.
6. Lifestyle — physical activity, cognitive stimulation, social connections.
7. Advance directives — care and decision-making planning.

8.2 Treatment methods

1. Acetylcholinesterase Inhibitors (AChEI) — Donepezil, Galantamine, Rivastigmine — Acetylcholinesterase inhibitor; synaptic acetylcholine increase. NICE NG97.
2. Memantine — NMDA antagonist; reduces excitotoxicity.
3. Lecanemab, Donanemab — Anti-amyloid monoclonal antibodies; for early AD; ARIA monitoring.
4. Non-Pharmacological Interventions for BPSD — Validation therapy, music therapy, reminiscence therapy, ABC analysis.
5. FINGER Multimodal Intervention (FINGER — Finnish Geriatric Intervention Study) — For prophylaxis.
6. Carer Support — Psychoeducation, respite, legal-financial planning.

8.3 Differences between sources

- NICE NG97 (2018) — AChEI for mild-moderate; memantine for moderate-severe; combination selective.
- Livingston G. et al. Lancet Commission 2024 — 14 modifiable risk factors.
- FDA Lecanemab 2023, Donanemab 2024 — for early AD; ARIA monitoring.
- APA — guidelines for antipsychotic use in the elderly.

9. Prognosis

- After diagnosis, median life expectancy is 8–10 years.
- Rapid course, family history, altered by ApoE4.
- Comorbid cardiovascular mortality.

10. Myths and misconceptions

Myth 1: “Alzheimer's is normal aging”

Evidence: Alzheimer's is a disease; normal aging can occur without cognitive decline; up to 45% of dementia cases are attributable to modifiable factors (Lancet Commission 2024).

Myth 2: “Dementia is inevitable, prevention is impossible”

Evidence: Lancet Commission 2024 — management of modifiable factors can delay up to 45% of potential dementia cases.

Myth 3: “Lecanemab cures Alzheimer's”

Evidence: Reduces clinical decline rate by ~25% (moderate effect); ARIA and infusion risks significant; not “cure.”

Myth 4: “Antipsychotics are safe for elderly dementia patients”

Evidence: FDA black box — antipsychotics increase mortality in elderly dementia patients (stroke, cardiovascular); only for severe agitation/psychosis, short-term.

Myth 5: “Ginkgo biloba is a prophylaxis for dementia”

Evidence: Cochrane (Birks J. 2009) — ginkgo shows no evidence of efficacy for dementia.

Myth 6: “Coconut oil cures Alzheimer's”

Evidence: No clinical evidence; promotion in popular media, not a clinical recommendation.

Myth 7: “The patient should not be told the diagnosis of dementia”

Evidence: NICE — patient has the right to diagnosis; early decision-making, advance documents important.

11. Sources

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ICD-11 **6D81**	**6D81 DEMENTIA DUE TO CEREBROVASCULAR DISEASE** **Dementia due to cerebrovascular disease**
ICD-10 F01	Vascular dementia
DSM-5-TR F01.5x	Major Vascular Neurocognitive Disorder

1. Definition and nosology

Dementia due to cerebrovascular disease (Vascular Dementia; ICD-11: 6D81; DSM-5-TR: F01.5x and similar) — cognitive decline and impairment of daily functioning developing as a result of cerebrovascular disease (stroke, small vessel disease, hypoperfusion). Second most common cause of dementia (~15–20%).

2. History

- Binswanger O. (1894) — classic description of subcortical vascular dementia.
- NINDS-AIREN (1993) — international diagnostic criteria.
- VASCOG (2014) — Updated consensus (Vascular Cognitive Disorders).

3. Epidemiology

- 15–20% of dementia cases; often mixed with Alzheimer's.
- Prevalence increases with age.
- Risk factors: hypertension, diabetes, hyperlipidemia, atrial fibrillation, smoking, history of stroke.

4. Aetiology and pathogenesis

- Multi-infarct dementia.
- Small vessel disease (subcortical leukoaraiosis, lacunar infarcts — Binswanger).

- Strategic infarct (thalamic, hippocampal).
- Hypoperfusion (systemic hypotension, cardiac arrest).
- CADASIL (autosomal dominant subcortical dementia, NOTCH3).

5. Clinical features

- “Stepwise” decline (as opposed to the gradual decline of Alzheimer's).
- Executive function, processing speed and attention are predominantly affected — memory is relatively less impaired (difference from Alzheimer).
- Focal neurological signs — hemiparesis, dysarthria, extrapyramidal, visual field defect.
- Mood disorders (vascular depression), apathy, abulia frequent.
- Postural disturbances (gait, balance).

6. Diagnosis

6.1 Unified diagnostic criteria (NINDS-AIREN, VASCOG)

A. Meeting major NCD criteria.

B. Clinical features consistent with vascular etiology: either (1) stepwise decline + history of stroke or (2) executive function and processing speed predominant.

C. Brain imaging evidence of vascular pathology — infarcts, leukoaraiosis, microhemorrhages.

D. Temporal relationship between cognitive decline and vascular event (Probable VaD).

6.2 Differences between sources

- VASCOG 2014 (Sachdev P. et al. Alzheimer Dis Assoc Disord) — introduced the Vascular Cognitive Impairment umbrella term.
- NICE NG97 — management of vascular risk factors.

6.3 Diagnostic algorithm

1. Clinical interview + informant.
2. MoCA (attention to executive function subscales).
3. Neurological examination — focal signs.
4. Brain MRI (FLAIR — leukoaraiosis, DWI — acute infarcts, T2/SWI — microhemorrhages).

5. Cardiovascular evaluation — blood pressure, EKG (atrial fibrillation), Holter, carotid USG, echo.
6. Laboratory — HbA1c, lipid, coagulation, hyperhomocysteinemia.

6.4 Differential diagnosis

Condition	Distinguishing feature
Alzheimer (6D80)	Gradual, memory dominant.
Mixed Dementia (Alzheimer's + VaD)	In many cases; clinically co-occurrent.
Lewy bodies (6D82)	Visual hallucinations, parkinsonism.
Frontotemporal (6D83)	Personality/behavior dominant.
Normal pressure hydrocephalus	Triad (gait, urinary incontinence, cognitive impairment).
Depressive pseudodementia	Affective, improvement with antidepressants.

7. Examination and assessment

- MoCA, neuropsychological testing.
- MRI, cardiovascular.
- Laboratory panel.

8. Treatment

1. Vascular risk factor management — first-line
    - Hypertension (target SBP <140, sometimes <130);
    - Diabetes HbA1c management;
    - Statin (lipid);
    - Antiplatelet (aspirin) or anticoagulant (atrial fibrillation — DOAC);
    - Smoking cessation;
    - Physical activity, diet.
2. AChEI and memantine — evidence base limited for VaD, but may be effective in mixed dementia; NICE NG97 — considered in mixed cases.
3. BPSD management — non-pharmacological first; antipsychotics limited (same FDA black box for Alzheimer's).
4. Depression (vascular depression) — SSRIs.

5. Rehabilitation after stroke — physical, occupational, speech therapy.

8.1 Treatment methods

1. Vascular Risk Management — Hypertension, diabetes, lipids, atrial fibrillation, smoking.
2. AChEI/Memantine in Mixed Dementia — Donepezil, rivastigmine off-label in pure VaD; may be considered in mixed cases.
3. Stroke Rehabilitation — Multidisciplinary.

8.2 Differences between sources

- NICE NG97 — vascular risk management.
- AHA/ASA stroke prophylaxis.

9. Prognosis

- Risk factor management reduces the rate of decline.
- Stroke history and comorbidity determine the prognosis.

10. Myths and misconceptions

Myth 1: “VaD is completely different from Alzheimer's”

Evidence: In most dementia cases mixed pathology (Alzheimer + VaD); clinical differentiation sometimes difficult.

Myth 2: “There is no specific treatment for VaD”

Evidence: vascular risk factor management — first-line and potent intervention; prevention particularly strong.

Myth 3: “Aspirin for dementia prophylaxis”

Evidence: ASPREE trial (McNeil J.J. NEJM 2018) — aspirin in primary prevention showed no evidence of preventing dementia; only in specific stroke prophylaxis indications.

Myth 4: “Vitamins and homocysteine are prophylaxis for vascular dementia”

Evidence: cognitive prophylaxis with homocysteine-lowering vitamins evidence weak.

Myth 5: “Stepwise decline is mandatory for diagnosing VaD”

Evidence: Gradual decline typical in small vessel VaD; stepwise only in multi-infarct form.

11. Sources

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ICD-11 **6D82**	**6D82 DEMENTIA DUE TO LEWY BODY DISEASE (DLB)** **Dementia due to Lewy body disease**
ICD-10 F02.8	Dementia in other specified diseases classified elsewhere
DSM-5-TR G31.83 + F02.8x	Major or Mild Neurocognitive Disorder With Lewy Bodies

1. Definition and nosology

Dementia due to Lewy body disease (DLB; ICD-11: 6D82; DSM-5-TR: G31.83 + F02.8x) — dementia developing from α-synuclein pathology; classic triad — cognitive fluctuations, visual hallucinations, parkinsonism. 5–15% of dementias.

Parkinson's disease dementia (PDD) — same pathology, but dementia develops at least 1 year after parkinsonism; DLB and PDD clinical spectrum.

2. History

- Lewy F. (1912) — Description of Lewy bodies in Parkinson's disease.
- McKeith I. (1996, 2017 update) — DLB consensus criteria.

3. Epidemiology

- 5–15% of dementia cases; relatively higher in males.
- Prevalence increases with age ≥65.
- Comorbidity: depression, delusions, REM sleep behavior disorder.

4. Aetiology and pathogenesis

- α-Synuclein aggregation — Lewy bodies in cortical and substantia nigra regions.
- SNCA, LRRK2, and GBA genes confer risk.

5. Clinical features

Four core clinical features (McKeith 2017)

- Fluctuating cognition — pronounced variation in attention and alertness (hours-days).
- Recurrent visual hallucinations — typically well formed and detailed (human, animal figures); the patient may be relatively tolerant of them.
- REM sleep behaviour disorder (RBD) — may precede cognitive decline by years.
- Parkinsonism — one or more spontaneous cardinal features: bradykinesia, rest tremor or rigidity (tremor typically milder than in Parkinson's disease).

Indicative biomarkers

- Reduced dopamine transporter uptake in the basal ganglia on SPECT/PET (123I-FP-CIT).
- Abnormal (low uptake) 123I-MIBG myocardial scintigraphy.
- Polysomnographic confirmation of REM sleep without atonia.

Supportive clinical features

- Severe antipsychotic sensitivity — neuroleptic malignant syndrome-like reaction (not part of the diagnostic algorithm).
- Postural instability, repeated falls, syncope, severe autonomic dysfunction, hypersomnia, hyposmia, systematised delusions, apathy, anxiety, depression.

6. Diagnosis

6.1 Unified diagnostic criteria (McKeith 2017)

Probable DLB: 2+ core clinical features (with or without indicative biomarkers), or 1 core feature + 1 or more indicative biomarkers. It must not be diagnosed on biomarkers alone.

Possible DLB: only 1 core clinical feature with no indicative biomarker; or 1 or more indicative biomarkers with no core clinical feature.

6.2 Differences between sources

- McKeith I. et al. Neurology 2017 — diagnostic consensus.
- DAT-SPECT or 123I-MIBG cardiac scintigraphy — indicative biomarkers (not supportive).

6.3 Diagnostic algorithm

1. Clinical interview + informant (especially RBD).
2. MoCA, MMSE, neuropsychological (visuospatial and executive function dominant).
3. Neurological examination.
4. Brain MRI (hippocampal atrophy less than in Alzheimer's).
5. DAT-SPECT (when indicated).
6. Polysomnography — RBD confirmation.

6.4 Differential diagnosis

Condition	Distinguishing feature
Alzheimer (6D80)	Memory dominant, visual hallucination minimal.
Parkinson's disease dementia (PDD)	Dementia occurring ≥ 1 year after parkinsonism (1-year rule)
Vascular (6D81)	Stroke history, MRI.
Delirium (6D70)	Acute onset; fluctuation is similar.
Schizophrenia late onset	Dementia absent.

7. Examination and assessment

- MoCA, neuropsychological testing.
- Polysomnography (RBD).
- DAT-SPECT.
- Brain MRI.

8. Treatment

1. AChEI (rivastigmine, donepezil) — In DLB particularly effective (cortical acetylcholine reduction stronger than in Alzheimer); rivastigmine McKeith I. Lancet 2000 RCT.
2. Memantine — for moderate-severe cases.
3. Parkinsonism — use levodopa with caution; it can intensify psychotic symptoms.
4. For visual hallucinations: non-pharmacological measures first. In refractory cases quetiapine or clozapine at low dose; typical antipsychotics (haloperidol) and risperidone are contraindicated — severe neuroleptic sensitivity reaction.
5. RBD — melatonin (3–12 mg at night) or clonazepam (low dose).
6. For BPSD, non-pharmacological measures come first.
7. FDA antipsychotic black box in elderly dementia patients.

8.1 Treatment methods

1. Rivastigmine AChEI — In DLB, specifically effective; patch and oral.
2. Quetiapine/Clozapine (refractory psychosis) — Low dose; clozapine is the safest but requires monitoring.
3. Melatonin for RBD — 3–12 mg at night.
4. DAT-SPECT — Indicative diagnostic biomarker.

8.2 Differences between sources

- McKeith I. Lancet 2000 — rivastigmine in DLB.
- NICE NG97 — rivastigmine or donepezil for DLB.
- Antipsychotic sensitivity — McKeith I. reviews.

9. Prognosis

- Mean life expectancy from diagnosis 5–8 years; sometimes shorter than Alzheimer's.

- Antipsychotic reaction is a critical complication.

10. Myths and misconceptions

Myth 1: “DLB is only a subtype of Alzheimer's”

Evidence: distinct pathological and clinical unit; α-synuclein vs amyloid-β/tau; antipsychotic sensitivity is critical difference.

Myth 2: “Haloperidol is safe in DLB”

Evidence: Haloperidol and risperidone are contraindicated — neuroleptic malignant syndrome-like reaction, mortality risk high.

Myth 3: “Visual hallucinations always require treatment”

Evidence: Some patients are tolerant to hallucinations; different approach if no distress.

Myth 4: “Levodopa fully cures parkinsonism in DLB”

Evidence: The levodopa response is smaller than in Parkinson's disease; it can exacerbate psychotic symptoms; balance is required.

Myth 5: “RBD is an insignificant symptom”

Evidence: RBD is a prodrome of α-synucleinopathy (DLB, Parkinson's, MSA) — 80%+ of patients go on to develop one of these conditions within 10–15 years.

11. Sources

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ICD-11 **6D83**	**6D83** **FRONTOTEMPORAL DEMENTIA (FTD)** **Frontotemporal dementia**
ICD-10 F02.0	Dementia in Pick disease
DSM-5-TR G31.09 + F02.8x	Major or Mild Neurocognitive Disorder Due to Frontotemporal Lobar Degeneration

1. Definition and nosology

Frontotemporal Dementia (FTD; ICD-11: 6D83; DSM-5-TR: G31.09 + F02.8x) — dementia developing from atrophy of the frontal and/or temporal lobes; typical onset between ages 45–65 (most common among early-onset dementias). Three clinical variants: bvFTD (behavioral variant) and two forms of PPA (Primary Progressive Aphasia — semantic and non-fluent).

2. History

- Pick A. (1892) — “Pick's disease” — description of frontotemporal atrophy.
- Neary D. et al. (1998) — FTD clinical consensus criteria.
- Rascovsky K. et al. (2011) — updated criteria for bvFTD.

3. Epidemiology

- 5–10% of dementia cases before age 65; Alzheimer's dominates after age 65.
- Sex: equal.
- Family history >40%.
- Comorbidity: ALS (Amyotrophic Lateral Sclerosis) — FTD-ALS spectrum (C9orf72 mutation).

4. Aetiology and pathogenesis

- Genetic (40%) — C9orf72 (most common), MAPT, GRN mutations.
- Pathology — tau (Pick body, MAPT), TDP-43 (TDP-43A, B, C, D, GRN, C9orf72), FUS proteins.

5. Clinical features

bvFTD (Behavioral Variant, ~60% of FTD)

- Early personality change — apathy, disinhibition, socially inappropriate behavior, loss of empathy.
- Stereotypical behavior (ritualistic).
- Hyperoral behavior (food changes, “sweet tooth”).
- Executive function significantly impaired; memory and visuo-spatial relatively preserved at onset.

PPA — Primary Progressive Aphasia

- Non-fluent variant (nfvPPA) — grammatical difficulty, slow speech, apraxia of speech.
- Semantic variant (svPPA) — loss of word meaning, agnosia; fluent, but empty speech.
- Logopenic — associated with Alzheimer's, distinct.

6. Diagnosis

6.1 Unified diagnostic criteria (Rascovsky 2011 bvFTD; Gorno-Tempini 2011 PPA)

bvFTD (Probable): ≥ 3 of 6 behavioral symptoms (disinhibition, apathy, loss of empathy, stereotypy, hyperorality, executive dysfunction) + functional impairment + neuroimaging support.

PPA: language is primary deficit; subvariant based on clinical and neuroimaging.

6.2 Differences between sources

- Rascovsky K. et al. Brain 2011 — Behavioral Variant Frontotemporal Dementia.
- Gorno-Tempini M.L. et al. Neurology 2011 — PPA.

6.3 Diagnostic algorithm

1. Clinical interview + informant (behavioral change).
2. Neuropsychological battery (executive function, language).
3. Brain MRI — frontal and/or temporal atrophy.
4. FDG-PET — hypometabolism.
5. Genetic testing (family history).

6. ALS screening (clinical or electromyography).

6.4 Differential diagnosis

Condition	Distinguishing feature
Alzheimer (6D80)	Memory dominant, late onset.
Late-onset schizophrenia	No atrophy.
Mania, depression	Affective episodes.
Bipolar disorder	Affective, cyclic.
Personality disorder	Persistent, long-standing; change in FTD is new.
Brain tumour	MRI.

7. Examination and assessment

- FAB (Frontal Assessment Battery), neuropsychological.
- MRI, FDG-PET.
- Genetic test.
- ALS screening.

8. Treatment

1. There is no disease-modifying medication — treatment targeting the pathological process is not yet approved (clinical trials ongoing).
2. AChEI — ineffective or harmful in FTD (may worsen behaviour); unlike in Alzheimer's disease.
3. SSRI — for behavioral symptoms (disinhibition, compulsivity, depression) in bvFTD; some evidence.
4. Trazodone — for agitation; studies by Lebert F. et al.
5. Atypical antipsychotic — only in severe behavioral disturbance, short-term, FDA black box rule.
6. Carer support and psychoeducation critical (behavioral symptoms are difficult for the family).
7. Nutritional support (control of hyperoral behaviour).
8. Legal-financial planning.

8.1 Treatment methods

1. SSRI for bvFTD — Citalopram, sertraline — behavioral symptoms.
2. Trazodone — Agitation, sleep.
3. Non-Pharmacological — Environmental modification, structure, caregiver training.
4. Speech-Language Therapy (SLT) — For PPA; compensatory strategies.
5. Genetic Consultation — For family members.

8.2 Differences between sources

- NICE NG97 — does not recommend AChEI for FTD.
- Lebert F. et al. Dement Geriatr Cogn Disord 2004 — trazodone.

9. Prognosis

- After diagnosis, median life expectancy is 6–11 years; FTD-ALS is shorter (3 years).
- Rapid decline with some mutations.

10. Myths and misconceptions

Myth 1: “FTD is a subtype of Alzheimer's”

Evidence: distinct pathological and clinical unit; tau, TDP-43, FUS pathologies; AChEI ineffective.

Myth 2: “AChEIs can be used in all dementias”

Evidence: AChEIs ineffective or harmful in FTD; NICE NG97 does not recommend.

Myth 3: “Behavioral change is only a psychiatric problem”

Evidence: Early onset behavioral change (45–65 years) should raise suspicion for FTD; neuroimaging is important.

Myth 4: “FTD is not genetic without a family history”

Evidence: 40% of cases have a family history; sporadic cases are also possible; C9orf72 expansion may be occult.

Myth 5: “The behavior of a patient with FTD is voluntary”

Evidence: behavioral alterations neurobiological — frontal lobe dysfunction; volitional control impaired.

11. Sources

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4. Gorno-Tempini M.L. et al. Classification of primary progressive aphasia and its variants. Neurology 2011;76(11):1006–1014.
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6E20–6E2Z PERINATAL DISORDERS

Mental or behavioural disorders associated with pregnancy, childbirth or the puerperium

ICD-11 **6E20**	**6E20 MENTAL OR BEHAVIOURAL DISORDERS ASSOCIATED WITH PREGNANCY, CHILDBIRTH OR THE PUERPERIUM, WITHOUT PSYCHOTIC SYMPTOMS** **Mental or behavioural disorders associated with pregnancy, childbirth or the puerperium, without psychotic symptoms**
ICD-10 F53.0	Mild mental and behavioural disorders associated with the puerperium, not elsewhere classified
DSM-5-TR —	Peripartum onset specifier (no separate DSM-5-TR category)

1. Definition and nosology

Postnatal Depression (ICD-11: 6E20; DSM-5-TR: F32.x / F33.x Major Depressive Disorder with peripartum onset specifier) — a major depressive episode developing after childbirth (typically first 6 months; some guidelines up to 1 year). Affects 10–15% of women.

2. History

- Marcé L.V. (1858) — first clinical description of postpartum psychosis and depression.

- EPDS — Cox J.L. et al. (1987) — Edinburgh Postnatal Depression Scale.
- NICE CG192 (2014, 2018) — antenatal and postnatal mental health.
- FDA — brexanolone (2019), zuranolone (2023) — first specifically approved drugs for postpartum depression.

3. Epidemiology

- Prevalence: 10–15% (high-income countries), 20–30% (low- and middle-income countries).
- "Baby blues" — milder, short-term (1–2 weeks) — occurs in 50–80% of women; does not require clinical intervention.
- Comorbidity: anxiety, OCD, panic.
- Risk factors: previous depression, family history, depression during pregnancy, lack of partner support, socio-economic stress.

4. Aetiology and pathogenesis

- Hormonal — sharp drop in estrogen and progesterone after childbirth; allopregnanolone dysregulation.
- Sleep disturbance with newborn care.
- Psychosocial stress, relationships, social support.
- Genetic and prior history of affective disorder.

5. Clinical features

- Major depressive episode symptoms (sleep disturbance, appetite loss, anhedonia, worthlessness, suicidal thoughts).
- Specific characteristics: affective distancing towards the infant, difficulties in infant bonding, belief in inability to fulfill the maternal role, intrusive thoughts regarding harming the infant (ego-dystonic — similar to OCD; distinct from postpartum psychosis).
- Suicide risk and, in rare cases, infanticide (higher in postnatal psychosis).

6. Diagnosis

6.1 Unified diagnostic criteria

Major depressive episode criteria (DSM-5-TR 5+ symptoms for 2 weeks; ICD-11) + peripartum onset specifier (DSM-5-TR — during pregnancy or within 4 weeks postpartum; ICD-11 — broader, up to 6 months-1 year).

6.2 Differences between sources

- NICE CG192 (2014, 2018) — diagnosis and treatment.
- ACOG Committee Opinion 757 (2018) — perinatal depression screening.
- USPSTF — screening in pregnant and postpartum women.

6.3 Diagnostic algorithm

1. EPDS (Edinburgh Postnatal Depression Scale, 10 items) — gold standard screening; cut-off ≥ 10 — possible depression (sensitive screening threshold), ≥ 13 — probable major depression.
2. Clinical interview (assessment of the need for hospitalisation and of suicide risk).
3. Postnatal psychosis differential — urgent hospitalization if psychotic symptoms present.
4. Medical evaluation — thyroid (postpartum thyroiditis), anemia.
5. Comorbid anxiety, OCD screening.

6.4 Differential diagnosis

Condition	Distinguishing feature
“Baby blues”	≤ 2 weeks, mild, does not require clinical intervention.
Postnatal psychosis (6E21)	Psychotic symptoms; urgent hospitalization.
Bipolar depressive episode	History of manic/hypomanic episode.
Postpartum thyroiditis	TSH.
Anemia	Hemoglobin.
Postnatal OCD	Ego-dystonic intrusions of harm to infant; comorbid postnatal depression.

7. Examination and assessment

- EPDS, PHQ-9.
- C-SSRS — suicide and risk of harm to infant.
- TSH, hemoglobin.

8. Treatment

8.1 General principles (NICE CG192 · ACOG · APA)

1. Mild-moderate: Psychotherapy is first-line — CBT, IPT (particularly adapted to the perinatal context).
2. Moderate-severe: psychotherapy + SSRI. In breastfeeding context:
    - Sertraline (least excreted into milk) and paroxetine — first-line;
    - Fluoxetine — long half-life, use with caution;
    - Citalopram, escitalopram — appropriate.
3. Refractory severe PPD:
    - Brexanolone (Zulresso) — IV 60-hour infusion; FDA 2019 approval; monitored in clinic under REMS program; rapid effect.
    - Zuranolone (Zurzuvae) — oral 14 days; FDA 2023 approval; more convenient.
4. ECT — psychotic depression, severe suicide risk, impaired food intake, also safe during pregnancy.
5. Family support, peer support (e.g., PND helpline).
6. Social service and mother-infant bonding support.
7. Sleep hygiene and involvement of family members in night care.

8.2 Treatment methods

1. Perinatal Interpersonal Therapy (IPT) — Stuart (Stuart S.), Robertson (Robertson M.) — Major role transitions, partner relationships, social support system.
2. CBT Perinatal Adapted — Cognitive disturbances, behavioral activation.
3. Brexanolone (Zulresso) — IV 60-hour; REMS program; clinic monitoring.
4. Edinburgh Postnatal Depression Scale (EPDS) — 10-item self-assessment.
5. ECT — Safe in pregnancy and lactation; in severe refractory cases.

8.3 Differences between sources

- NICE CG192 — sertraline and paroxetine first-line in breastfeeding.
- Louik C. et al. NEJM 2007 · Huybrechts K.F. et al. NEJM 2014 — SSRI in pregnancy cardiac malformations (paroxetine cautiously).
- Meltzer-Brody S. et al. Lancet 2018 — brexanolone RCT.

9. Prognosis

- 70-80% remission with adequate treatment.
- Risk of recurrence in subsequent pregnancies (~25%).
- Without treatment, chronicity and comorbidity.

10. Myths and misconceptions

Myth 1: “PPD is just ordinary ‘baby blues’, transient”

Evidence: baby blues ≤ 2 weeks, mild; PPD clinical depressive episode, requires intervention; can be prolonged.

Myth 2: “Antidepressants are dangerous during breastfeeding”

Evidence: sertraline and paroxetine are acceptable during breastfeeding; milk transfer minimal; clinical counseling.

Myth 3: “Mother is weak, she should ‘fix herself’”

Evidence: PPD is a clinical condition; risk of suicide and infanticide exists; intervention is required.

Myth 4: “Brexanolone cures PPD”

Evidence: Rapid effect, but cost and need for clinic monitoring significant; standard approach SSRI + psychotherapy.

Myth 5: “PPD does not affect the infant”

Evidence: PPD affects mother-infant bonding and impacts the child's cognitive and emotional development; early intervention is crucial.

Myth 6: “Intrusive thoughts (harming a baby) are always a sign of psychosis”

Evidence: In postpartum OCD, intrusive thoughts are ego-dystonic — the patient suffers from them and avoids them; in postpartum psychosis they are ego-syntonic and can lead to behavior — different intervention.

11. Sources

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ICD-11 **6E21**	**6E21 MENTAL OR BEHAVIOURAL DISORDERS ASSOCIATED WITH PREGNANCY, CHILDBIRTH OR THE PUERPERIUM, WITH PSYCHOTIC SYMPTOMS** **Mental or behavioural disorders associated with pregnancy, childbirth or the puerperium, with psychotic symptoms**
ICD-10 F53.1	Severe mental and behavioural disorders associated with the puerperium, not elsewhere classified

ICD-11 **6E21**	**6E21 MENTAL OR BEHAVIOURAL DISORDERS ASSOCIATED WITH PREGNANCY, CHILDBIRTH OR THE PUERPERIUM, WITH PSYCHOTIC SYMPTOMS** **Mental or behavioural disorders associated with pregnancy, childbirth or the puerperium, with psychotic symptoms**
DSM-5-TR F23	Brief Psychotic Disorder, With Peripartum Onset

1. Definition and nosology

Mental or behavioural disorders associated with pregnancy, childbirth or the puerperium, with psychotic symptoms (commonly termed Postpartum Psychosis, PPP; ICD-11: 6E21; DSM-5-TR: Brief Psychotic Disorder or Bipolar I with peripartum onset specifier) — an acute-onset psychotic episode after childbirth (typically within the first 2 weeks); hallucinations, delusions, cognitive disorganization, mood lability. This is a medical emergency — infanticide (~4%) and maternal suicide risk high.

2. History

- Marcé L.V. (1858) — classic description of postpartum psychosis.
- Bergink V. et al. (2015) — 4-step treatment algorithm.
- NICE CG192 — peripartum psychiatric conditions.

3. Epidemiology

- Incidence: 1–2/1000 births.

- 25–50% in women with a history of bipolar disorder; 30% with a history of previous PPP.
- Comorbidity: mostly bipolar in nature.
- Mortality: infanticide 4%, suicide up to 5% (untreated).

4. Aetiology and pathogenesis

- Hormonal — sharp postpartum drop in estrogen and progesterone.
- Sleep deprivation.
- Bipolar disorder predisposition (most PPP — manifestation of a bipolar episode).
- Family history.

5. Clinical features

- Rapid onset (24–72 hours, typically within first 2 weeks).
- Polymorphic psychotic symptoms — delusions (particularly regarding baby — baby being “evil” or “demanded”; mother “must escape”), hallucinations.
- Mood lability — manic or depressive components.
- Cognitive disorganization, confusion.
- Sleep disturbance severe.
- Behavior — bizarre, extreme, impaired infant care.
- High risk: Suicidal mother, harm to infant (infanticide).

6. Diagnosis

6.1 Unified diagnostic criteria

Clinical diagnosis: acute-onset psychotic episode postpartum; coded under bipolar manic or acute psychotic category + peripartum onset specifier.

6.2 Differences between sources

- NICE CG192 — emergency medical condition.
- Royal College of Psychiatrists perinatal psychiatric service standards.

6.3 Diagnostic algorithm

1. Clinical interview — rapid onset, nature of symptoms.
2. Medical assessment — postpartum medical conditions (thyroiditis, infection, encephalopathy, eclampsia, B12 deficiency) to be ruled out.

3. Laboratory — TSH, complete blood count, electrolytes, liver, kidney, ammonia, glucose, infection markers.
4. Brain MRI and EEG — if atypical presentation or neurological signs (autoimmune encephalitis — anti-NMDA-R especially!).
5. Toxicology.
6. Assessment of suicide and infanticide risk (C-SSRS, infant safety).

6.4 Differential diagnosis

Condition	Distinguishing feature
Postnatal depression (6E20)	Psychotic symptom absent.
Postnatal OCD	Ego-dystonic intrusions; insight preserved.
Delirium (6D70)	Fluctuation of attention; medical cause (infection).
Anti-NMDA-R encephalitis	Atypical psychosis + motor symptoms + seizures; CSF antibodies; classic presentation in young woman!
Eclampsia/HELLP	Hypertension, proteinuria, thrombocytopenia.
Sheehan syndrome	Hypopituitarism (post-hemorrhage).

7. Examination and assessment

- Clinical psychiatric and neurological.
- The above medical panel.
- Anti-NMDA-R antibodies (CSF) — in atypical cases.
- C-SSRS, infant safety.

8. Treatment

8.1 General principles (NICE CG192 · Bergink 2015)

1. Urgent hospitalisation is mandatory — for mother and baby; if possible, Mother-Baby Unit (joint inpatient service for mother and baby).
2. Bergink 4-step algorithm (Am J Psychiatry 2015):
    - Step 1: Benzodiazepine (lorazepam) acute stabilization;

    - Step 2: atypical antipsychotic (olanzapine, quetiapine, risperidone);
    - Step 3: Addition of lithium;
    - Step 4: ECT in refractory or catatonic features.
3. Bipolar primarily — lithium first line. Valproate is contraindicated in women of childbearing potential and permissible only where the conditions of the Pregnancy Prevention Programme are met (EMA/MHRA 2018); in bipolar disorder it is absolutely contraindicated during pregnancy.
4. Breastfeeding decision — lithium is a relative contraindication in breastfeeding (passes into breast milk); olanzapine and quetiapine relatively safe; discuss this with the patient and the family.
5. Risk of recurrence in subsequent pregnancies is high (~30–40%) — prophylactic lithium is considered in the peri-postpartum period.
6. ECT — safe in pregnancy and postpartum period; rapid effect.
7. Multidisciplinary coordination — psychiatrist, obstetrician-gynecologist, pediatrician, social worker.

8.2 Treatment methods

1. Bergink 4-Step Algorithm (Bergink) — Benzodiazepine → antipsychotic → lithium → ECT.
2. Mother-Baby Unit (MBU) — Shared hospitalization of mother and infant; preserving bonding and safety.
3. ECT Postpartum — Safe in pregnancy and lactation; rapid effect; first-line for catatonic features.
4. Prophylactic Lithium in Subsequent Pregnancies — Bergink V. et al. Am J Psychiatry 2012 — relapse prophylaxis during the peripartum period.

8.3 Differences between sources

- NICE CG192 — Mother-Baby Unit recommendation.
- Bergink V. et al. Am J Psychiatry 2015 — evidence base for the 4-step algorithm.

9. Prognosis

- Full recovery in most patients with adequate treatment.
- Risk of recurrence in subsequent pregnancies is high (~30–40%).
- Long-term management of bipolar disorder is crucial.

10. Myths and misconceptions

Myth 1: "PPP is a strong expression of 'maternal feelings'"

Evidence: Clinical psychotic state; risk of infanticide and suicide high; emergency intervention.

Myth 2: "Monitoring at home is sufficient"

Evidence: NICE CG192 — hospital admission mandatory; MBU preferred.

Myth 3: "Antipsychotics and lithium are contraindicated during breastfeeding"

Evidence: Olanzapine and quetiapine relatively safe in breastfeeding; lithium passes into milk, but may be recommended with infant monitoring; individual balance.

Myth 4: "It will not recur in a subsequent pregnancy"

Evidence: Previous PPP history shows relapse of ~30–40% (up to 44% without prophylaxis); a prophylactic plan is crucial.

Myth 5: "Anti-NMDA-R encephalitis is rare, no need to search for it"

Evidence: In a young woman atypical psychosis + movement symptoms + seizures is the classic presentation of Anti-NMDA-R encephalitis; antibody test saves lives.

11. Sources

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2. APA. DSM-5-TR. 2022.
3. NICE CG192. 2014/2018.
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6E40 PSYCHOSOMATIC FACTORS

Psychological or behavioural factors affecting disorders classified elsewhere

ICD-11 **6E40**	**6E40 PSYCHOLOGICAL OR BEHAVIOURAL FACTORS AFFECTING DISORDERS OR DISEASES CLASSIFIED ELSEWHERE** **Psychological or behavioural factors affecting disorders or diseases classified elsewhere**
ICD-10 F54	Psychological and behavioural factors associated with disorders or diseases classified elsewhere
DSM-5-TR F54	Psychological Factors Affecting Other Medical Conditions

1. Definition and nosology

ICD-11: 6E40 Psychological or behavioural factors affecting disorders or diseases classified elsewhere; DSM-5-TR: F54 Psychological Factors Affecting Other Medical Conditions — psychological factors interfering with somatic disease course (depression, anxiety, medication adherence, stress, beliefs about illness).

Distinct from factitious disorder (6D50) and somatic symptom disorder (6C20) — here there is a confirmed medical illness, but psychological factors influence the course.

2. History

- Alexander F. (1950) — conceptualization of ‘psychosomatic medicine’.
- Engel G.L. (1977) — ‘biopsychosocial model’.
- Levenson J.L. (2018) — APA Textbook of Psychosomatic Medicine.

3. Epidemiology

- Universal — psychological factors influence all chronic somatic diseases.
- Depression in cardiovascular disease — 20–40%; diabetes — 25%; cancer — 15–25%.
- Wide use of consultation-liaison psychiatry service.

4. Aetiology and pathogenesis

Biopsychosocial interaction — biological predisposition + psychological factors + social context.

5. Clinical features

- Comorbid depression/anxiety affects illness (e.g., increased mortality after myocardial infarction).
- Medication non-adherence (compliance/adherence).
- False beliefs about illness.
- Stress worsens illness course.
- Behavioral factors (smoking, alcohol, diet, physical inactivity).

6. Diagnosis

6.1 Unified diagnostic criteria

A. Confirmed medical illness present.

B. Psychological or behavioral factors interfere with illness course:

1. Reduces treatment response;
2. Increases health risk;
3. The pathophysiology is worsened;
4. Medical intervention alters the necessity.

C. Not better explained by factitious disorder, somatic symptom disorder, or another mental disorder.

6.2 Differences between sources

- DSM-5-TR / ICD-11 — same.
- Consultation-liaison psychiatry model.

6.3 Diagnostic algorithm

1. Evaluation of medical illness.
2. Identification of psychological factors — depression (PHQ-9), anxiety (GAD-7), medication adherence (Morisky).
3. Behavioral factors — smoking, alcohol, diet, physical activity.
4. Screening for comorbid psychiatric disorder.

6.4 Differential diagnosis

Condition	Distinguishing feature
Somatic symptom disorder (6C20)	Somatic complaint dominant.
Factitious (6D50)	Conscious lie.
MDD comorbid with medical illness	Affective symptoms meet independent criteria — separate diagnosis.

7. Examination and assessment

- PHQ-9, GAD-7, Morisky.
- Illness-specific scales (DDS — Diabetes Distress Scale, Beck-cardiac).

8. Treatment

1. An integrated multidisciplinary approach — somatic specialist + psychiatrist + psychologist.
2. Illness-specific psychosocial intervention:
    - Cardiac CBT (post-MI depression);
    - Diabetes self-management education (DSME);
    - Oncology distress monitoring;
    - Chronic pain CBT and ACT.
3. Antidepressant — disease-specific side effect profile:
    - Cardiac — sertraline, escitalopram;
    - Oncology — mirtazapine (sleep, appetite benefit);
    - Chronic pain — duloxetine, amitriptyline.
4. Motivational interviewing — behavior change (smoking, physical activity).

8.1 Treatment methods

1. Consultation-Liaison Psychiatry — integrated service in medical-surgical departments.
2. Cardiac CBT — Post-MI depression management.
3. Diabetes Self-Management Education (DSME) — Diabetes self-management training.
4. CBT and ACT for chronic pain — Williams Cochrane 2020.

8.2 Differences between sources

- NICE CG91 — CBT for depression in cardiac patients.
- Levenson J.L. APA Textbook of Psychosomatic Medicine 2018.

9. Prognosis

Integrative approach improves both somatic and psychological outcomes.

10. Myths and misconceptions

Myth 1: “The illness is entirely psychological in origin”

Evidence: “Biopsychosocial” model — psychological factors influence but do not alone cause somatic illness.

Myth 2: “The psychiatrist is ‘running away’ from the illness”

Evidence: integrated approach — somatic and psychiatric intervention parallel.

Myth 3: “Alternative medicine (yoga, mindfulness) cures somatic illness”

Evidence: Complementary interventions may be adjunctive; does not replace standard somatic treatment (oncologic, cardiac).

Myth 4: “Telling the patient ‘you got sick because of stress’ is therapeutic”

Evidence: This approach undermines accusatory and therapeutic relationship.

11. Sources

1. WHO. ICD-11. 6E40 Psychological or behavioural factors affecting disorders or diseases classified elsewhere. 2024.
2. APA. DSM-5-TR. 2022.
3. Levenson J.L. (ed.). APA Publishing Textbook of Psychosomatic Medicine and Consultation-Liaison Psychiatry. 3rd ed. 2018.

4. NICE NG185. Acute coronary syndromes. 2020.
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6E60–6E6Z SECONDARY PSYCHIATRY

Secondary mental or behavioural syndromes

ICD-11 **6E61**	**6E61 SECONDARY PSYCHOTIC SYNDROME** **Secondary psychotic syndrome**
ICD-10 F06.2	Organic delusional [schizophrenia-like] disorder
DSM-5-TR F06.2	Psychotic Disorder Due to Another Medical Condition, With Delusions

1. Definition and nosology

Secondary Psychotic Syndrome (ICD-11: 6E61; DSM-5-TR: F06.2/F06.0 Psychotic Disorder Due to Another Medical Condition) — psychotic episode developing in the context of another medical condition (autoimmune, infectious, endocrine, metabolic, neurological) or a substance. First-line intervention — treatment directed at the primary etiology.

2. History

- The term “Symptomatic psychosis” was formulated in the 20th century.
- Dalmau J. (2007) — discovery of anti-NMDA-R encephalitis; autoimmune psychosis revolution.
- Graus F. et al. (2016) — autoimmune encephalitis clinical guideline.

3. Epidemiology

- 3–8% of all first-episode psychosis cases have secondary etiologies.
- Anti-NMDA-R encephalitis — rare but reversible psychosis in young women; paraneoplastic with teratoma.
- Hashimoto encephalopathy, lupus encephalopathy.

4. Aetiology and pathogenesis

- Autoimmune: Anti-NMDA-R, anti-LGI1, anti-CASPR2 encephalitis; Hashimoto (TPO antibodies); lupus (NPSLE).
- Infectious: HIV, syphilis, herpes encephalitis, COVID-19.
- Endocrine: hyper/hypothyroidism, cortisol disorders (Cushing, Addison), parathyroid disorders (hypercalcemia).
- Metabolic: uremia, hepatic encephalopathy, hyponatremia, hypo/hyperglycemia, B12 deficiency, Wilson's disease.
- Neurological: brain tumor, stroke, epilepsy (postictal-), dementia.
- Substance-induced: Amfetamine, cocaine, cannabis, hallucinogen, alcohol withdrawal, steroids.
- Paraneoplastic: Ovarian teratoma (anti-NMDA-R), small cell lung carcinoma.

5. Clinical features

- Psychotic symptoms (delusion, hallucination, disorganized behavior).
- Atypical features (characteristic of autoimmune encephalitis):
    - Rapid onset
    - Motor symptoms (catatonic, dystonias, orofacial dyskinesia);
    - Autonomic disturbance (hypertension, tachycardia, hyperthermia);
    - Seizures;
    - Impairment of consciousness.
- Compared to classic psychosis (schizophrenia) — atypical presentation raises suspicion.

6. Diagnosis

6.1 Unified diagnostic criteria

A. Psychotic symptoms.

B. Evidence of medical condition (autoimmune, infectious, endocrine, metabolic, neurological) or substance effect.

C. Psychosis does not occur in the context of delirium (but may overlap with delirium).

D. Not better explained by another primary psychotic disorder.

6.2 Differences between sources

- Graus F. et al. Lancet Neurol 2016 — autoimmune encephalitis clinical approach.
- ICD-11 — substance-induced psychosis does NOT belong here: it lives in the substance chapter (6C40.6 alcohol, 6C41.6 cannabis and so on). 6E61 covers only health conditions outside the mental-disorders chapter and is assigned alongside the diagnosis of the underlying disease.

6.3 Diagnostic algorithm

1. Clinical suspicion — new-onset psychosis, especially with atypical features or in young women.
2. Medical assessment:
    - Complete blood count, liver/renal, electrolytes (Na, Ca), glucose, ammonia;
    - TSH, TPO antibodies (Hashimoto);
    - Cortisol level (Cushing/Addison);
    - HIV, syphilis;
    - ANA, lupus panel;
    - B12, folate, ceruloplasmin (Wilson);
    - Toxicology screening.
3. Brain MRI.
4. EEG — non-convulsive status, encephalitis.
5. Lumbar puncture — pleocytosis, antibody panel (anti-NMDA-R, anti-LGI1, anti-CASPR2, paraneoplastic).
6. Tumor screening (when paraneoplastic antibody positive — ovarian USG, pelvic MRI, chest CT).

6.4 Differential diagnosis

Condition	Distinguishing feature
Schizophrenia (6A20)	Medical cause absent; typical clinical course.
Acute and transient psychotic (6A23)	Secondary causes excluded.
Delirium (6D70)	Fluctuation of attention; psychosis in the context of delirium.

Condition	Distinguishing feature
Substance-induced psychosis	Substance confirmation and temporal relationship.
Dementia with psychotic features	Cognitive decline.

7. Examination and assessment

- The above laboratory panel.
- MRI, EEG.
- CSF antibody panel.
- Tumor screening.

8. Treatment

1. Etiological treatment is first-line:
    - Autoimmune encephalitis — IV methylprednisolone 1 g × 5 days → IVIG and/or plasmapheresis; second-line rituximab, cyclophosphamide;
    - Paraneoplastic — tumor treatment (ovarian teratoma surgery);
    - Endocrine — thyroid, cortisol, calcium correction;
    - Infectious — antibiotic, antiviral;
    - Metabolic — correction of the disorder;
    - Medication-induced — discontinuation of the suspected agent.
2. Symptomatic psychiatric intervention — until etiological treatment yields results:
    - Atypical antipsychotic (quetiapine, olanzapine) — with caution;
    - In case of autoimmune encephalitis — typical antipsychotic (haloperidol) contraindicated — Risk of NMS and disease exacerbation;
    - Benzodiazepine (lorazepam) — acute agitation.
3. ECT — alternative for refractory catatonic or autoimmune encephalitis.

8.1 Treatment methods

1. Immunotherapy (Autoimmune Encephalitis) — IV steroids → IVIG → plasmapheresis → rituximab/cyclophosphamide.
2. Treatment of Tumor (Paraneoplastic) — Surgical or oncological treatment.

3. Atypical Antipsychotic, Symptomatic — Until etiological treatment produces results; quetiapine, olanzapine superior.
4. ECT — in refractory cases — Effective in catatonic or immunotherapy-resistant autoimmune encephalitis.

8.2 Differences between sources

- Graus F. et al. Lancet Neurol 2016 — autoimmune encephalitis protocols.
- APA Textbook of Consultation-Liaison Psychiatry (Levenson).

9. Prognosis

- Most cases respond to etiological treatment; reversible condition.
- Anti-NMDA-R encephalitis — 75% good recovery with early immunotherapy.

10. Myths and misconceptions

Myth 1: “First-episode psychosis is always schizophrenia”

Evidence: 3–8% secondary etiologies; atypical features require attention; medical evaluation is mandatory in first-episode psychosis.

Myth 2: “Typical antipsychotic (haloperidol) is safe for every psychotic episode”

Evidence: If autoimmune encephalitis is suspected, haloperidol and similar first-generation antipsychotics are prohibited — risk of NMS, disease exacerbation.

Myth 3: “Anti-NMDA-R encephalitis is rare, not worth looking for”

Evidence: In a young woman atypical psychosis + movement symptoms + seizures is the classic presentation of Anti-NMDA-R; antibody test saves lives; reversible condition.

Myth 4: “If no medical cause is found, a psychiatric diagnosis is made — no further search is needed”

Evidence: Atypical clinical presentation requires expanded investigation (CSF, MRI, paraneoplastic panel).

Myth 5: “Immunotherapy is not suitable for psychiatric conditions”

Evidence: If evidence of autoimmune encephalitis, immunotherapy first-line; standard antipsychotic ineffective.

11. Sources

1. WHO. ICD-11. 6E61 Secondary psychotic syndrome. 2024.
2. APA. DSM-5-TR. 2022.
3. Graus F. et al. A clinical approach to diagnosis of autoimmune encephalitis. Lancet Neurol 2016;15(4):391–404.
4. Dalmau J. et al. Anti-NMDA-receptor encephalitis: case series and analysis of the effects of antibodies. Lancet Neurol 2008;7(12):1091–1098.
5. Levenson J.L. (ed.). APA Publishing Textbook of Psychosomatic Medicine and Consultation-Liaison Psychiatry. 3rd ed. 2018.

ICD-11 **6E62**	**6E62 SECONDARY MOOD SYNDROME** **Secondary mood syndrome**
ICD-10 F06.3	Organic mood [affective] disorders
DSM-5-TR F06.3x	Depressive Disorder Due to Another Medical Condition

1. Definition and nosology

Secondary mood syndrome (ICD-11: 6E62; DSM-5-TR: F06.3x Mood Disorder Due to Another Medical Condition) — depressive, manic, or mixed mood episodes in the context of another medical condition, substance, or medication effect.

2. History

The term “Organic Affective Disorder” belongs to the 20th century; current clinical practice has moved toward the biopsychosocial model.

3. Epidemiology

- Depressive symptoms are prevalent in chronic medical conditions; some meet criteria for clinical depressive episode.
- Steroid-induced mood disorders — high-dose prednisone 5–18% mania/hypomania, depression.
- Interferon-induced depression — 20–40% in HCV treatment.
- Post-stroke depression — 30%.

4. Aetiology and pathogenesis

- Endocrine — hypothyroid (depression), hyperthyroid (mania-like), Cushing, Addison.
- Neurological — stroke (left frontal — depression, right — mania), Parkinson's, MS, brain tumor, dementia, TBI.
- Substance/medication — steroids, interferon, dopamine agonists, isotretinoin, β-blocker, anticonvulsant.
- Metabolic — uremia, B12, folate.
- Infectious — HIV, syphilis, COVID.
- Autoimmune — lupus, Hashimoto.
- Oncology — pancreatic cancer (classic depressive presentation), paraneoplastic.

5. Clinical features

- Symptoms of depressive, manic, mixed, and hypomanic episodes.
- Medical condition or substance-related temporal association.
- Features not characteristic of classic affective disorder (late onset, atypical presentation).

6. Diagnosis

6.1 Unified diagnostic criteria

A. Persistent mood disturbance (depressive, manic, or mixed).

B. Medical condition or substance effect evidence.

C. Not better explained by a primary affective disorder.

D. Does not occur in the context of delirium

6.2 Differences between sources

- ICD-11 — a substance-induced mood disorder does NOT belong here: it lives in the substance chapter. 6E62 covers only health conditions outside the mental-disorders chapter and is assigned alongside the diagnosis of the underlying disease.
- Consultation-liaison protocols.

6.3 Diagnostic algorithm

1. Clinical interview + medical history.
2. Medical and laboratory assessment (TSH, B12, cortisol, renal, hepatic).
3. Medication list review.
4. Brain imaging (stroke, MS, tumor).
5. Toxicology.
6. Comorbid disorders.

6.4 Differential diagnosis

Condition	Distinguishing feature
MDD first (6A70/6A71)	There is no medical cause.
Bipolar first	History of affective episodes without a medical cause.
Adjustment disorder (6B43)	Response to stress factor.
Delirium (6D70)	Fluctuation of attention.

7. Examination and assessment

- PHQ-9, MADRS, YMRS.
- Laboratory: TSH, B12, cortisol, ANA, kidney, liver.
- MRI in atypical cases.
- C-SSRS.

8. Treatment

1. Etiological treatment — correction of medical condition or medication.
2. Symptomatic psychiatric treatment:
    - Depressive — SSRIs (disease-specific selection; cardiac — sertraline, escitalopram; oncology — mirtazapine);
    - Manic — antipsychotic (quetiapine, olanzapine); in steroid-induced mania, taper steroids or use antipsychotic;
    - Mixed or refractory — ECT.
3. Multidisciplinary coordination.

8.1 Treatment methods

1. SSRI Illness-Specific — Choice appropriate to cardiac, oncological, neurological context.
2. ECT — In refractory and severe cases.

9. Prognosis

Most remissions with etiological intervention.

10. Myths and misconceptions

Myth 1: “Depression is ‘normal’ for a patient with a medical illness”

Evidence: Clinical depression worsens medical condition course; requires active intervention.

Myth 2: “Steroid-induced mania resolves on its own”

Evidence: dose reduction or antipsychotic addition required.

Myth 3: “Antidepressant is first-line in hypothyroidism”

Evidence: thyroid correction first; antidepressants for residual symptoms.

Myth 4: “Interferon depression is rare”

Evidence: HCV interferon therapy 20–40% — active monitoring required.

11. Sources

1. WHO. ICD-11. 6E62 Secondary mood syndrome. 2024.
2. APA. DSM-5-TR. 2022.
3. Levenson J.L. (ed.). APA Publishing Textbook of Psychosomatic Medicine and Consultation-Liaison Psychiatry. 3rd ed. 2018.
4. Robinson R.G., Jorge R.E. Post-stroke depression: a review. Am J Psychiatry 2016;173(3):221–231.

ICD-11 **6E63**	**6E63 SECONDARY ANXIETY SYNDROME** **Secondary anxiety syndrome**
ICD-10 F06.4	Organic anxiety disorder
DSM-5-TR F06.4	Anxiety Disorder Due to Another Medical Condition

1. Definition and nosology

Secondary Anxiety Syndrome (ICD-11: 6E63; DSM-5-TR: F06.4 Anxiety Disorder Due to Another Medical Condition) — anxiety, panic or obsessive-like symptoms within context of another medical condition or substance effect.

2. History

Development of consultation-liaison psychiatry; formalization from DSM-III.

3. Epidemiology

- Frequency of anxiety is high in chronic medical conditions (cardiac, COPD, endocrine).
- In hyperthyroidism, 60–70% have anxiety symptoms.
- Caffeine, stimulant-induced anxiety.

4. Aetiology and pathogenesis

- Endocrine — hyperthyroidism, pheochromocytoma, hypoglycemia.
- Cardiac — arrhythmia, myocardial infarction, mitral valve prolapse.
- Respiratory — COPD, asthma, pulmonary embolism.
- Neurological — TBI, Parkinson's, MS.
- Substance/medication — caffeine, stimulant, corticosteroid, β-blocker (paradoxical), benzodiazepine/alcohol withdrawal.
- Vestibular disorders.

5. Clinical features

- GAD, panic attack, OCD-like symptoms in the context of a medical condition.
- Atypical features (late onset, focal symptoms).

6. Diagnosis

Medical evidence + distress symptoms + temporal congruence.

6.1 Differences between sources

DSM-5-TR / ICD-11 — same.

6.2 Diagnostic algorithm

1. Clinical interview.
2. Medical evaluation — TSH, glucose, cardiac, vestibular.
3. Medication and caffeine history.
4. Toxicology.

6.3 Differential diagnosis

- Primary anxiety disorders (6B0x);
- Delirium;
- Adjustment disorder.

7. Examination and assessment

- GAD-7, HAM-A.
- TSH, glucose, EKG.
- Toxicology.

8. Treatment

1. Etiological treatment.
2. Symptomatic — SSRIs for residual anxiety; benzodiazepines short-term for acute condition.
3. Caffeine restriction.

8.1 Treatment methods

1. Etiological Intervention — Correction of medical condition.

2. SSRI For Residual Anxiety — For residual symptoms after correction of medical condition.

9. Prognosis

Significant improvement with etiological intervention.

10. Myths and misconceptions

Myth 1: “Anxiety in a patient with a medical illness is ‘normal’”

Evidence: Clinical distress worsens the course of the medical condition; intervention required.

Myth 2: “Anxiolytics are first-line in hyperthyroidism”

Evidence: Primary is thyroid correction; β-blocker (propranolol) is a symptomatic adjunct.

Myth 3: “Caffeine is safe”

Evidence: High doses can cause panic, anxiety, and tachycardia; limitation is recommended.

11. Sources

1. WHO. ICD-11. 6E63 Secondary anxiety syndrome. 2024.
2. APA. DSM-5-TR. 2022.
3. Levenson J.L. (ed.). APA Publishing Textbook of Psychosomatic Medicine and Consultation-Liaison Psychiatry. 3rd ed. 2018.

7A00–7A8Z SLEEP DISORDERS

Sleep-wake disorders

ICD-11 **7A00**	**7A00 CHRONIC INSOMNIA** **Chronic insomnia**
ICD-10 F51.0	Nonorganic insomnia
DSM-5-TR F51.01	Insomnia Disorder

1. Definition and nosology

Chronic insomnia (ICD-11: 7A00; DSM-5-TR: F51.01 Insomnia Disorder) — difficulty initiating sleep, maintaining sleep, and/or early morning awakening; daytime functional impairment. Duration ≥ 3 months, frequency ≥ 3 times per week.

2. History

- DSM-IV — ‘Primary Insomnia’; DSM-5 (2013) — ‘Insomnia Disorder’ — primary/secondary distinction abolished.
- AASM (American Academy of Sleep Medicine) — Clinical Practice Guidelines (2021 CBT-I, 2017 pharmacological).
- European Insomnia Guideline (Riemann D. 2023).
- NICE TA922 (2023).

3. Epidemiology

- Lifetime prevalence: chronic insomnia 10–15%; insomnia symptoms 30–35%.
- Sex: 1.5 times higher in females.
- Increases with age.
- Comorbidity: MDD ~50%, anxiety, pain, COPD, circadian disorders.

4. Aetiology and pathogenesis

- Spielman 3P Model — Predisposing, Precipitating, Perpetuating.
- Hyperarousal hypothesis — physiological and cognitive stimulation.
- Behavioral factors — irregular schedule, caffeine, alcohol, screen use, non-sleep activities in the bedroom.

5. Clinical features

- Sleep difficulty (falling asleep), maintenance, early awakening.
- Daytime symptoms: fatigue, irritability, concentration difficulties, occupational/academic impairment, somatic (headache, GI).
- Impact on performance, risk of accident.

6. Diagnosis

6.1 Unified diagnostic criteria

A. Sleep difficulty (onset, maintenance, or early awakening).

B. Despite adequate sleep opportunities.

C. Daytime functional impairment.

D. Duration ≥ 3 months, ≥ 3 times per week.

E. Not fully explained by another sleep disorder, substance, or medical condition.

6.2 Differences between sources

- AASM — CBT-I first-line.
- NICE TA922 (2023) — daridorexant only if CBT-I has been tried and did not work, or is unavailable; that is, CBT-I comes first.

6.3 Diagnostic algorithm

1. Clinical interview + sleep diary for 1–2 weeks.
2. ISI (Insomnia Severity Index), PSQI.
3. Differential — obstructive sleep apnea (STOP-BANG), restless legs, circadian.
4. Comorbidity (MDD, anxiety).
5. Polysomnography only in atypical cases or suspicion of another sleep disorder.

6.4 Differential diagnosis

Condition	Distinguishing feature
Obstructive sleep apnoea (7A41)	Snoring, daytime sleepiness; polysomnography.
Circadian rhythm disorders (7A60)	Shift in the sleep schedule.
Restless Legs Syndrome	Akathisia-like leg discomfort.
MDD with insomnia	Affective symptoms predominate.
Hyperthyroidism	TSH.

7. Examination and assessment

- ISI, PSQI.
- Sleep diary.
- STOP-BANG (OSA), Epworth Sleepiness Scale.
- TSH, ferritin.

8. Treatment

8.1 General principles (AASM 2021 · European Insomnia Guideline 2023 · NICE TA922 2023)

1. CBT-I (cognitive behavioural therapy for insomnia) — first line; the gold standard in chronic insomnia; superior long-term efficacy to pharmacotherapy (Mitchell M.D. BMC Fam Pract 2012 meta-analysis). 5 main components:
    - Stimulus control (Bootzin) — bed only for sleep;
    - Sleep restriction (Spielman) — restricting time in bed to equal sleep time;
    - Cognitive therapy — catastrophic thoughts about sleep;
    - Sleep hygiene;
    - Relaxation training.
2. Digital CBT-I (Sleepio, SHUTi) — NICE HTG624 (2022): a cost-saving option in primary care for people who would otherwise be offered sleep hygiene or sleeping pills; direct comparison with face-to-face CBT-I is not established.

3. Pharmacotherapy — short-term (2–4 weeks), when there is no response to CBT-I or it is unavailable:
    - Z-drugs (zolpidem, zopiclone, eszopiclone);
    - Benzodiazepine (temazepam) — tolerance, dependence risk;
    - Newer — suvorexant, lemborexant (orexin antagonists) — less dependence;
    - Doxepin at low dose (3–6 mg);
    - Melatonin — for circadian component or in elderly; ramelteon (melatonin agonist).
4. In older adults — Beers Criteria; avoid benzodiazepines and antihistamines (diphenhydramine).
5. Comorbid depression — sedative antidepressant (mirtazapine, trazodone) — long-term use side effects should be considered.

8.2 Treatment methods

1. CBT for insomnia (CBT-I — Cognitive Behavioral Therapy for Insomnia), 5 components — Stimulus control, sleep restriction, cognitive therapy, sleep hygiene, relaxation.
2. Digital CBT-I (Digital) — Sleepio, SHUTi — internet-based.
3. Orexin Antagonists — Suvorexant, lemborexant — lower addiction potential.
4. Insomnia Severity Index (ISI — Insomnia Severity Index) — 7-item screening.
5. Sleep Diary — 1–2 weeks baseline indicators.

8.3 Differences between sources

- AASM Clinical Practice Guideline for Behavioral and Psychological Treatments (Edinger J.D. 2021).
- European Insomnia Guideline (Riemann D. J Sleep Res 2023).
- NICE TA922 (2023) — CBT-I must be tried before moving to medication.

9. Prognosis

- Response to CBT-I 70–80%, remission ~40%; the effect is sustained long-term.
- Without treatment, chronic course and comorbidity (MDD, anxiety).

10. Myths and misconceptions

Myth 1: “Sleep medications are the main treatment for insomnia”

Evidence: AASM 2021 — CBT-I first-line; pharmacotherapy short-term adjunct; long-term use tolerance, dependence.

Myth 2: “Alcohol helps with sleep”

Evidence: Alcohol reduces REM, causes midnight awakenings, leads to chronic insomnia.

Myth 3: “Antihistamine (diphenhydramine) is safe in the elderly”

Evidence: Beers Criteria — antihistamines in the elderly: anticholinergic side effects (falls, cognitive decline); avoid.

Myth 4: “Valerian, kava are safe alternatives for insomnia”

Evidence: valerian — Leach M.J., Page A.T. Sleep Med Rev 2015;24:1–12 — evidence for effect insufficient; kava hepatotoxicity.

Myth 5: “8 hours of sleep is necessary for everyone”

Evidence: Individual need 6–9 hours; important is daytime function.

Myth 6: “Waking up during the night is harmful”

Evidence: Brief awakenings are normal; stimulus control — rise if you can't sleep for 20 min.

11. Sources

1. WHO. ICD-11. 7A00 Insomnia disorders. 2024.
2. APA. DSM-5-TR. 2022.
3. Edinger J.D. et al. Behavioral and psychological treatments for chronic insomnia disorder in adults: an AASM clinical practice guideline. J Clin Sleep Med 2021;17(2):255–262.
4. Riemann D. et al. The European Insomnia Guideline 2023. J Sleep Res 2023;32(6):e14035.
5. Mitchell M.D. et al. Comparative effectiveness of cognitive behavioral therapy for insomnia: a systematic review. BMC Fam Pract 2012;13:40.
6. Buysse D.J. et al. Efficacy of brief behavioral treatment for chronic insomnia in older adults. Arch Intern Med 2011;171(10):887–895.

7. National Institute for Health and Care Excellence. Sleepio to treat insomnia and insomnia symptoms. HealthTech guidance HTG624. London: NICE; 2022.
8. National Institute for Health and Care Excellence. Daridorexant for treating long-term insomnia. Technology appraisal guidance TA922. London: NICE; 18 October 2023.

ICD-11 **7A41**	**7A41 OBSTRUCTIVE SLEEP APNOEA (OSA)** **Obstructive sleep apnoea**
ICD-10 G47.3	Sleep apnoea
DSM-5-TR G47.33	Obstructive Sleep Apnea Hypopnea

1. Definition and nosology

Obstructive Sleep Apnea (OSA; ICD-11: 7A41; DSM-5-TR: G47.33) — recurrent complete (apnea) or partial (hypopnea) obstruction of the upper airway during sleep; daytime sleepiness, cognitive impairment, increased cardiovascular mortality.

2. History

- Gastaut H. (1965) — classic description of OSA.
- Sullivan C.E. (1981) — invented CPAP.
- AASM Practice Parameters (update).

3. Epidemiology

- In adults: approximately 13% in men, approximately 6% in women (moderate-to-severe OSA).
- Increases with age and BMI.
- Comorbidity: hypertension (50%), cardiovascular disease, stroke, atrial fibrillation, diabetes.

4. Aetiology and pathogenesis

- Anatomical — adipose tissue in the upper airway, small mandible, enlarged tonsils/adenoids.
- Genetic and family history.
- Posture — worsens in supine position.
- Alcohol, sedative — worsens.

5. Clinical features

- Waking up with loud snoring, observed apnea, or choking sensation.
- Daytime sleepiness (dangerous while driving), fatigue.
- Morning headache, dry mouth.
- Concentration, memory problems.
- Sexual dysfunction, depressive symptoms.

6. Diagnosis

6.1 Unified diagnostic criteria

Polysomnography (PSG) — AHI (Apnea-Hypopnea Index) ≥ 5/hour + symptoms; AHI ≥ 15 irrespective of symptoms.

6.2 Differences between sources

- AASM Clinical Practice Guideline for Diagnostic Testing (2017).
- Home Sleep Apnea Test (HSAT) — appropriate for moderate-severe OSA.

6.3 Diagnostic algorithm

1. STOP-BANG screening (Snoring, Tiredness, Observed apnea, BP, BMI, Age, Neck, Gender).
2. Epworth Sleepiness Scale.
3. Polysomnography or HSAT.
4. Cardiovascular evaluation.

6.4 Differential diagnosis

Condition	Distinguishing feature
Insomnia (7A00)	Sleeping and retention problems; no snoring.

Condition	Distinguishing feature
Central sleep apnoea	Respiratory effort absent.
Narcolepsy (7A20)	Cataplexy, MSLT.
RLS	Restless legs.

7. Examination and assessment

- STOP-BANG, Epworth.
- Polysomnography or Home Sleep Apnea Testing.
- BMI, neck circumference.

8. Treatment

1. CPAP — Continuous Positive Airway Pressure is first-line. For moderate-severe OSA; AHI significantly reduced; effect on cardiovascular mortality has not been confirmed in randomized trials (SAVE, NEJM 2016).
2. BiPAP — in refractory cases or CPAP non-response.
3. Weight loss — mild OSA; bariatric surgery in severe obesity.
4. Mandibular advancement device (MAD) — mild-moderate OSA, CPAP non-responsive.
5. Postural therapy — supine-avoidant.
6. Surgery (UPPP, hypoglossal nerve stimulation — Inspire) — in selected cases.
7. Withdrawal from alcohol and sedatives.

8.1 Treatment methods

1. CPAP — First-line; adherence main problem.
2. Mandibular Advancement Device (MAD) — Mild-moderate OSA; when CPAP fails.
3. Hypoglossal Nerve Stimulation (Inspire) — New surgical option; in selected cases.
4. STOP-BANG, Epworth, PSG — Screening and diagnosis.

8.2 Differences between sources

- AASM CPAP standard, MAD selective.
- NICE NG202 (OSA-COPD-overlap).

9. Prognosis

- CPAP reliably reduces symptoms and AHI and improves quality of life; its effect on cardiovascular mortality is unproven (observational studies — Marin 2005 — linked it to reduced mortality, but randomized trials — SAVE, NEJM 2016 — did not confirm this, likely due to poor adherence in trials).
- Without treatment — stroke, atrial fibrillation, diabetes, increased mortality.

10. Myths and misconceptions

Myth 1: "OSA only occurs in obese people"

Evidence: Possible in normal-weight patients as well (anatomical factors).

Myth 2: "Snoring is the main sign of OSA, nothing more is needed"

Evidence: snoring alone not proof; AHI measurement required; untreated OSA high mortality and morbidity.

Myth 3: "CPAP only reduces symptoms, does not affect mortality"

Evidence: The truth is more nuanced. Observational data (Marin, Lancet 2005) linked CPAP to reduced mortality, but subsequent randomized trials (SAVE, NEJM 2016) did not confirm reduced cardiovascular mortality — likely due to poor adherence in trials (mean ~3.3 h/night vs. the recommended ~4 h). CPAP reliably reduces symptoms and AHI and improves quality of life; its mortality effect remains unproven.

Myth 4: "Central sleep apnea and OSA are treated the same"

Evidence: Different intervention (ASV — Adaptive Servo-Ventilation for central apnea); differentiation is critical.

11. Sources

1. WHO. ICD-11. 7A41 Obstructive sleep apnoea. 2024.
2. APA. DSM-5-TR. 2022.
3. Kapur V.K. et al. AASM Clinical Practice Guideline for Diagnostic Testing for Adult OSA. J Clin Sleep Med 2017;13(3):479–504.
4. Marin J.M. et al. Long-term cardiovascular outcomes in men with OSA-hypopnoea with or without treatment with CPAP. Lancet 2005;365(9464):1046–1053.

	7A60–7A65 CIRCADIAN RHYTHM SLEEP-WAKE DISORDERS
ICD-11 **7A60–7A65**	**Circadian rhythm sleep-wake disorders**
ICD-10 F51.2	Nonorganic disorder of the sleep-wake schedule
DSM-5-TR G47.21	Circadian Rhythm Sleep-Wake Disorders, Delayed Sleep Phase Type

1. Definition and nosology

Circadian Rhythm Sleep-Wake Disorders (ICD-11: 7A60–7A65; DSM-5-TR: G47.21) — mismatch between the internal circadian rhythm and the external social and light schedule; resulting in insomnia and/or sleepiness.

Subtypes: Delayed Sleep-Wake Phase Disorder (DSWPD), Advanced Sleep-Wake Phase Disorder, Irregular Sleep-Wake Rhythm, Non-24-hour Sleep-Wake (in blind individuals), Shift Work Disorder, Jet Lag Disorder.

2. History

- In the 1960s, Aschoff J. — foundations of circadian biology.
- AASM (2015) — clinical practice guideline for circadian rhythm disorders.

3. Epidemiology

- DSWPD — 7–16% in adolescents and young adults.
- Shift Work Disorder — 10–30% of shift workers.
- Comorbidity: MDD, anxiety, bipolar, cardiovascular.

4. Aetiology and pathogenesis

- Endogenous circadian rhythm disorder (clock gene mutations rare — Familial Advanced Sleep Phase).
- Environment — light exposure, social schedule.
- In adolescents, DSWPD — pubertal hormonal changes.

5. Clinical features

- DSWPD — late sleep and wake times; mismatch with school/work; “night owl” pattern.
- Advanced — early sleep and wake times; in elderly.
- Irregular — sleep episodes scattered throughout the day; dementia, ID.
- Non-24-hour — in blind people; the circadian rhythm does not entrain to the external light signal.
- Shift Work — shift work; insomnia + daytime sleepiness.
- Jet Lag — transient, rapid time zone change.

6. Diagnosis

6.1 Unified diagnostic criteria

A. Mismatch between sleep-wake schedule and internal circadian rhythm.

B. Insomnia, drowsiness, or both.

C. Duration ≥ 3 months (excluding jet lag and shift work).

D. Functional impairment.

6.2 Differences between sources

- AASM 2015 Clinical Practice Guideline.

6.3 Diagnostic algorithm

1. Clinical interview + sleep diary for 1–2 weeks.
2. Actigraphy 2 weeks.
3. Dim Light Melatonin Onset (DLMO) — in research context.
4. Exclusion of comorbid sleep disorders.

6.4 Differential diagnosis

Insomnia, OSA, depressive disorder, narcolepsy.

7. Examination and assessment

- Sleep diary, actigraphy.
- Munich Chronotype Questionnaire.

8. Treatment

1. Bright light therapy — In DSWPD morning; in Advanced evening.
2. Exogenous melatonin — physiological low dose (0.3–5 mg); in DSWPD evening (time critical); in jet lag.
3. Chronotherapy — gradual shift of sleep timing.
4. Sleep hygiene and behavioral intervention.
5. Shift work — strategic resting, dark room, caffeine timing.
6. Non-24-hour in blind patients — tasimelteon (Hetlioz, FDA approved).

8.1 Treatment methods

1. Bright Light Therapy — 10,000 lux × 30–60 minutes; timing adjusted to the circadian shift.
2. Melatonin (Circadian) — Low dose; timing is critical.
3. Actigraphy — 2 weeks.
4. Tasimelteon — FDA approval for Non-24-hour.

9. Prognosis

With intervention, most patients improve; chronic form requires ongoing strategy.

10. Myths and misconceptions

Myth 1: “The adolescent is ‘lazy’ and can get up early”

Evidence: DSWPD is the neurobiological basis of circadian delay during puberty.

Myth 2: “Melatonin is a sleep medication”

Evidence: melatonin regulates circadian phase, hypnotic effect is minimal; timing selection is critical.

Myth 3: “There is no treatment for shift work insomnia”

Evidence: Strategic rest, light therapy, melatonin, caffeine timing improves.

Myth 4: “Jet lag resolves on its own”

Evidence: 1 day approximately for 1 hour of shift; acceleration with light + melatonin.

11. Sources

1. WHO. ICD-11. 7A60–7A65 Circadian rhythm sleep-wake disorders. 2024.
2. APA. DSM-5-TR. 2022.
3. Auger R.R. et al. AASM Clinical Practice Guideline for the Treatment of Intrinsic Circadian Rhythm Sleep-Wake Disorders. J Clin Sleep Med 2015;11(10):1199–1236.

ICD-11 **7A21–7A26**	**7A21–7A26** **HYPERSOMNIA** **Hypersomnolence disorders**
ICD-10 F51.1	Nonorganic hypersomnia
DSM-5-TR F51.11	Hypersomnolence Disorder

1. Definition and nosology

Hypersomnia (ICD-11: 7A21–7A26; DSM-5-TR: F51.11 Hypersomnolence Disorder) — excessive daytime sleepiness or sleep episodes (after ≥ 7 hours of nighttime sleep); functional impairment. In the context of idiopathic hypersomnia, narcolepsy, or other sleep disorders.

2. History

In the 1970s–1980s, differentiation from narcolepsy developed; AASM clinical guidelines.

3. Epidemiology

- Idiopathic hypersomnia: ~50/100,000.
- Kleine-Levin syndrome — rare (~1/1 million); recurrent episodes of hypersomnia + appetite/sexual disturbance.
- Comorbidity: depressive symptoms, OSA, narcolepsy.

4. Aetiology and pathogenesis

- Unknown (idiopathic) — neurobiological dysregulation hypothesis.
- Dysregulation of the GABA-ergic system (idiopathic).

- Secondary — Kleine-Levin (autoimmune suspicion), post-TBI, MS, depressive disorder.

5. Clinical features

- Excessive Daytime Sleepiness (EDS).
- Long night sleep (≥ 9 hours) is insufficient.
- "Sleep drunkenness" — confusion, disorientation upon awakening.
- Daytime sleep episodes are prolonged and non-restorative (in contrast to narcolepsy, where they are brief and restorative).
- Kleine-Levin — 1–2 week episodes of hypersomnia + hyperphagia + disinhibited behavior.

6. Diagnosis

6.1 Unified diagnostic criteria

A. Even after sufficient sleep (≥7 hours), excessive sleepiness or prolonged sleep.

B. Frequency and duration — ≥3 times per week, ≥3 months.

C. Significant distress or functional impairment.

D. Not fully explained by another sleep disorder, substance, or medical condition.

6.2 Differences between sources

- AASM Practice Parameters — diagnostic PSG and MSLT (Multiple Sleep Latency Test).
- Idiopathic hypersomnia — mean sleep latency on MSLT ≤ 8 min; SOREMP ≤ 1 (different from narcolepsy).

6.3 Diagnostic algorithm

1. Clinical interview and sleep diary.
2. Epworth Sleepiness Scale.
3. PSG + MSLT (excluding narcolepsy).
4. Medical assessment — thyroid, anemia, depressive, OSA.

6.4 Differential diagnosis

Condition	Distinguishing feature
Narcolepsy (7A20)	Cataplexy, SOREMP ≥ 2.

Condition	Distinguishing feature
OSA (7A41)	Snoring, AHI ↑.
Depressive disorder	Affective symptoms.
Insomnia (7A00)	Insomnia.
Kleine-Levin syndrome	Episodic 1–2 week hypersomnia + appetite.

7. Examination and assessment

- Epworth, PSG + MSLT.
- Medical laboratory tests.

8. Treatment

1. Modafinil, armodafinil — first-line.
2. Stimulants (methylphenidate, amphetamine) — refractory.
3. Low-sodium oxybate (Xywav, FDA 2021) — the only agent approved for idiopathic hypersomnia.
4. Pitolisant (FDA 2019) — approved for narcolepsy; solriamfetol (FDA 2019) — approved for narcolepsy and obstructive sleep apnoea; both off-label in idiopathic hypersomnia.
5. Sleep hygiene and behavioral intervention (planned daytime naps).
6. Kleine-Levin — supportive; asymptomatic between episodes.

8.1 Treatment methods

1. Modafinil/Armodafinil — Wakefulness-promoting; first-line; headache, anxiety side effects.
2. Multiple Sleep Latency Test (MSLT) — 5 sleep attempts with 2-hour intervals; sleep latency measurement.
3. Pitolisant and Solriamfetol — New medications; in refractory cases.

8.2 Differences between sources

- AASM Treatment Guidelines (Maski K. et al. 2021).

9. Prognosis

Chronic; functional improvement with pharmacotherapy.

10. Myths and misconceptions

Myth 1: “Daytime sleepiness is ‘laziness’”

Evidence: Clinical condition; increased risk of mortality and accidents.

Myth 2: “Caffeine is sufficient for hypersomnia”

Evidence: Caffeine adjunct; modafinil or stimulant required.

Myth 3: “Hypersomnia is only a symptom of depression”

Evidence: idiopathic hypersomnia is a distinct disorder; MSLT differentiates.

11. Sources

1. WHO. ICD-11. 7A21–7A26 Hypersomnolence disorders. 2024.
2. APA. DSM-5-TR. 2022.
3. Maski K. et al. Treatment of central disorders of hypersomnolence: AASM Clinical Practice Guideline. J Clin Sleep Med 2021;17(9):1881–1893.

ICD-11 **7A20**	**7A20 NARCOLEPSY** **Narcolepsy**
ICD-10 G47.4	Narcolepsy and cataplexy
DSM-5-TR G47.4xx	Narcolepsy

1. Definition and nosology

Narcolepsy (ICD-11: 7A20; DSM-5-TR: G47.4xx) — irresistible daytime sleep episodes; two types: Type 1 (with cataplexy, orexin-A/hypocretin deficiency) and Type 2 (without cataplexy).

2. History

- Gélineau J.B. (1880) — the term narcolepsy.
- Mignot E. et al. (2000) — discovery of the orexin/hypocretin system in narcolepsy etiology.

3. Epidemiology

- Prevalence: 25–50 per 100,000.
- Onset: 10–25 years (bimodal).
- Comorbidity: OSA, depressive symptoms, OCD.

4. Aetiology and pathogenesis

- Type 1 — autoimmune damage to orexin-A neurons (CSF orexin <110 pg/ml); HLA-DQB1*06:02 strong association.
- H1N1 vaccine (Pandemrix, 2009) — increased incidence of narcolepsy type 1 in Scandinavian countries (Miller E. BMJ 2013).
- Type 2 — etiology not fully clear.

5. Clinical features

Classic tetrad

- Excessive daytime sleepiness (EDS) — irresistible sleep attacks.
- Cataplexy (Type 1) — sudden partial or complete loss of muscle tone triggered by emotion (laughter, anger); consciousness preserved.
- Sleep paralysis — inability to move at sleep onset or upon awakening.
- Hypnagogic/hypnopompic hallucinations — visual/auditory hallucinations at sleep transition.

Complete tetrad rare; EDS main feature.

6. Diagnosis

6.1 Unified diagnostic criteria

A. EDS ≥ 3 months.

B. MSLT — mean sleep latency ≤8 min + ≥2 SOREMPs (Sleep Onset REM Period); or CSF orexin-A <110 pg/ml.

C. Type 1 — cataplexy + orexin deficiency; Type 2 — no cataplexy, normal orexin.

6.2 Differences between sources

- ICSD-3 (International Classification of Sleep Disorders) — AASM.
- HLA typing and CSF orexin — in atypical cases.

6.3 Diagnostic algorithm

1. Clinical interview.
2. PSG (night) + MSLT (day) — gold standard.
3. CSF orexin — selective.
4. HLA-DQB1*06:02 — supportive.

6.4 Differential diagnosis

Condition	Distinguishing feature
Idiopathic hypersomnia (7A21)	SOREMP < 2.
OSA (7A41)	Elevated AHI.
Seizure	Loss of consciousness; EEG.
Syncope	Cardiovascular cause.
Functional/psychogenic	Typical clinical pattern and mismatch with PSG/MSLT results.

7. Examination and assessment

- Epworth.
- PSG + MSLT.
- CSF orexin, HLA-DQB1*06:02.

8. Treatment

1. EDS: modafinil, armodafinil, solriamfetol, pitolisant; stimulants (methylphenidate, amphetamine) in refractory cases.
2. Cataplexy: sodium oxybate (Xyrem) — gold standard; new — calcium/magnesium/potassium/sodium oxybate (Xywav, FDA 2020, low sodium); SSRI/SNRI (venlafaxine, fluoxetine); pitolisant.
3. Behavior — planned daytime naps (15–20 min)
4. Driving restriction if EDS present.

8.1 Treatment methods

1. Sodium Oxybate (Xyrem/Xywav) — GHB; two doses at night; effect on cataplexy and EDS.
2. Modafinil/Armodafinil — First-line for EDS.
3. Pitolisant — H3 receptor antagonist; for EDS and cataplexy.

4. SSRI/SNRI for cataplexy — Venlafaxine, fluoxetine off-label.
5. MSLT — ≥2 SOREMP diagnostic.

8.2 Differences between sources

- AASM Treatment Guidelines (Maski 2021).
- FDA approvals — pitolisant 2019, solriamfetol 2019, Xywav 2020.

9. Prognosis

- Chronic; functional improvement with pharmacotherapy.
- Comorbidity (MDD, OSA) intervention essential.

10. Myths and misconceptions

Myth 1: “Narcolepsy is just EDS alone”

Evidence: Tetrad (EDS + cataplexy + paralysis + hallucinations); SOREMPs differentiating in MSLT.

Myth 2: “Cataplexy is a seizure”

Evidence: consciousness preserved; EEG no epileptic activity; emotional trigger.

Myth 3: “H1N1 vaccine causes narcolepsy — vaccines should be avoided”

Evidence: Risk indicated specifically with Pandemrix (2009 Scandinavia); other vaccines are safe; avoiding vaccination overall is not recommended.

Myth 4: “The patient can drive”

Evidence: Driving must be restricted in a narcoleptic patient with EDS.

11. Sources

1. WHO. ICD-11. 7A20 Narcolepsy. 2024.
2. APA. DSM-5-TR. 2022.
3. Maski K. et al. AASM CPG. J Clin Sleep Med 2021;17(9):1881–1893.
4. Miller E. et al. Risk of narcolepsy in children and young people receiving AS03 adjuvanted pandemic A/H1N1 2009 influenza vaccine. BMJ 2013;346:f794.
5. Mignot E. et al. The role of cerebrospinal fluid hypocretin measurement in the diagnosis of narcolepsy. Arch Neurol 2002;59(10):1553–1562.

HA00–HA0Z SEXUAL HEALTH

Conditions related to sexual health

ICD-11 **HA01.1**	**HA01.1 MALE ERECTILE DYSFUNCTION (ED)** **Male erectile dysfunction**
ICD-10 F52.2	Failure of genital response
DSM-5-TR F52.21	Erectile Disorder

1. Definition and nosology

Erectile Dysfunction (ED; ICD-11: HA01.1 Erectile Dysfunction; DSM-5-TR: F52.21) — persistent difficulty in obtaining or maintaining an erection sufficient for sexual intercourse. Duration ≥ 6 months, in most attempts.

2. History

- 1998 — FDA approval of sildenafil (Viagra) — revolution in ED treatment.
- AUA (American Urological Association) Guideline (Burnett 2018, 2024 update).
- EAU (European Association of Urology) Guidelines.

3. Epidemiology

- 5–10% at age 40, 50%+ at age 70.
- Risk factors: diabetes, cardiovascular disease (ED as cardiovascular early marker), hypertension, hyperlipidemia, smoking, obesity, depression, antidepressants.

4. Aetiology and pathogenesis

- Organic (~70%) — vascular, neurogenic, endocrine (hypogonadism), medication-related.

- Psychogenic (~10%) — performance anxiety, relationship problems, depression.
- Mixed (~20%).
- Princeton III Consensus (Nehra 2012) — ED an early cardiovascular marker, cardiac risk assessment important.

5. Clinical features

- Difficulty obtaining or maintaining erection.
- Presence or absence of morning or nighttime erections (for psychogenic vs organic differentiation)
- Relationship and emotional burden.

6. Diagnosis

6.1 Unified diagnostic criteria (AUA 2018 · EAU)

A. Persistent difficulty obtaining and/or maintaining erection.

B. Duration ≥ 6 months, in most attempts.

C. Distress or relationship impairment.

6.2 Differences between sources

- AUA 2018/2024 and EAU — stepwise approach.

6.3 Diagnostic algorithm

1. Clinical interview (sexual and medical history, relationships).
2. IIEF (International Index of Erectile Function) / SHIM.
3. Laboratory — morning testosterone, glucose, HbA1c, lipid, prolactin, thyroid.
4. Cardiovascular evaluation.
5. Penile ultrasound (vascular suspicion) — selective.

6.4 Differential diagnosis

Condition	Distinguishing feature
Hypogonadism	Low testosterone + symptoms.
Premature ejaculation (HA03)	Ejaculation time.

Condition	Distinguishing feature
HSDD	Decreased desire dominant.
Peyronie's disease	Penile curvature and pain.

7. Examination and assessment

- IIEF.
- Medical and laboratory (above).
- Cardiac evaluation (EKG, stress test if needed).

8. Treatment

8.1 General principles (AUA 2018/2024 · EAU stepwise)

1. 1st line:
    - Lifestyle — smoking cessation, weight, physical activity, cardiovascular management;
    - PDE5 inhibitors — sildenafil (25–100 mg), tadalafil (5–20 mg as needed or 2.5–5 mg daily), vardenafil, avanafil; absolutely contraindicated with nitrates; caution with α-blockers.
2. 1st line (in hypogonadism): Testosterone replacement therapy (TRT) — only for confirmed hypogonadism (≥ 2 morning measurements low + symptoms); monitoring PSA and hematocrit.
3. Second-line:
    - Intracavernosal alprostadil (PGE1);
    - Transurethral alprostadil (MUSE);
    - Vacuum erectile device.
4. Third-line: penile prosthesis (semirigid or inflatable); high satisfaction.
5. Psychotherapy — in performance anxiety; couple therapy.
6. Antidepressant-induced ED — bupropion, mirtazapine, vortioxetine alternative; sildenafil adjunct (Nurnberg JAMA 2003).

8.2 Treatment methods

1. PDE5 Inhibitors — Phosphodiesterase 5 inhibitor; absolutely contraindicated with nitrates.
2. Intracavernosal Alprostadil — PGE1; if PDE5 fails.
3. Vacuum Device — Non-invasive; can be used with anticoagulant.

4. Penile Prosthesis — Refractory ED; high satisfaction.
5. International Index of Erectile Function (IIEF) — Severity scale.

8.3 Differences between sources

- Burnett A.L. et al. AUA Guideline. J Urol 2018, 2024 update.
- Salonia A. et al. EAU Guidelines 2023.

9. Prognosis

PDE5 with 70% efficacy; refractory cases 2nd–3rd line; cardiovascular risk management reduces mortality.

10. Myths and misconceptions

Myth 1: “ED is a normal part of aging, requires no treatment”

Evidence: ED is an early cardiovascular marker; active intervention is essential.

Myth 2: “Over-the-counter ‘natural’ potency enhancers are safe”

Evidence: FDA warnings — many “natural” products contain undeclared sildenafil/tadalafil — interaction with nitrates, risk of death.

Myth 3: “Testosterone replacement therapy is needed for every ED patient”

Evidence: AUA — only in confirmed hypogonadism; routine TRT not recommended.

Myth 4: “Shock wave therapy (Li-ESWT) is standard treatment”

Evidence: AUA 2024 — under investigation; not standard first-line.

Myth 5: “Yohimbine is an effective natural alternative”

Evidence: Ernst E., Pittler M.H. J Urol 1998;159(2):433–436 — small effect, side effects of anxiety and hypertension; PDE5 superior.

11. Sources

1. WHO. ICD-11. HA01.1 Male erectile dysfunction. 2024.
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3. Burnett A.L. et al. Erectile Dysfunction: AUA Guideline. J Urol 2018;200(3):633–641.
4. Salonia A. et al. EAU Guidelines on Sexual and Reproductive Health 2023.

5. Nehra A. et al. The Princeton III Consensus recommendations for the management of erectile dysfunction and cardiovascular disease. Mayo Clin Proc 2012;87(8):766–778.
6. Yuan J. et al. Comparative effectiveness and safety of oral phosphodiesterase type 5 inhibitors for erectile dysfunction: a systematic review and network meta-analysis. Eur Urol 2013;63(5):902–912.

ICD-11 **HA03.0**	**HA03.0 MALE EARLY EJACULATION (PE)** **Male early ejaculation**
ICD-10 F52.4	Premature ejaculation
DSM-5-TR F52.4	Premature (Early) Ejaculation

1. Definition and nosology

Male early ejaculation (PE; ICD-11: HA03.0; DSM-5-TR: F52.4 Premature Ejaculation) — ejaculation occurring involuntarily within a short time, often before or within ~1 minute of penetration; loss of control and distress.

Subtypes: Lifelong (primary) — present from onset; Acquired (secondary) — developed later; Variable and Subjective — ISSM classification.

2. History

- Semans J.H. (1956) — ‘stop-start’ technique.
- Masters & Johnson (1970) — “squeeze” behavioral technique.
- ISSM (2014) — Althof S.E. international diagnostic and treatment guideline.
- 2009 — dapoxetine first approved drug for PE in the EU.

3. Epidemiology

- Prevalence in males 20–30% (self-reported); with strict ISSM criteria 4–5%.
- Age-related variability low; occurs in all age groups.
- Comorbidity: ED, anxiety, depression, relationship strain.

4. Aetiology and pathogenesis

- Neurobiological — serotonergic (5-HT2C, 5-HT1A balance); genetic component (lifelong).
- Psychological — performance anxiety, early sexual experience, relationship problems.
- Acquired — ED, prostatitis, hyperthyroidism, substance use.

5. Clinical features

- Intravaginal ejaculation latency time (IELT) short (lifelong ~1 min; acquired ≤ 3 min).
- Loss of control.
- Distress, avoidance, relationship tension.

6. Diagnosis

6.1 Unified diagnostic criteria (ISSM 2014 · DSM-5-TR)

A. Ejaculation always or almost always before or within ~1 minute of penetration (lifelong) or a clinically meaningful decrease in latency time to ≤ 3 minutes (acquired).

B. Inability to delay in all or almost all penetrations.

C. Negative personal consequences — distress, avoidance.

6.2 Differences between sources

- ISSM (Althof S.E. et al. J Sex Med 2014).
- AUA and EAU guidelines.

6.3 Diagnostic algorithm

1. Clinical interview (sexual history, IELT, control, distress).
2. PEDT (Premature Ejaculation Diagnostic Tool) — 5-item.
3. Comorbid ED assessment (IIEF).
4. Endocrine and urologic (acquired form).

6.4 Differential diagnosis

Condition	Distinguishing feature
ED (HA01)	Difficulty maintaining erection.

Condition	Distinguishing feature
Variable PE	Not a clinical entity — normal variation.
Subjective PE	IELT normal, but perception disturbance.
Prostatitis	Pelvic pain; acquired PE.

7. Examination and assessment

- PEDT, IIEF.
- IELT (chronometry — when recommended).
- Endocrine and urologic (suspected acquired).

8. Treatment

8.1 General principles (ISSM 2014 · AUA · EAU)

1. Behavioral methods (first-line or combination):
    - Stop-start (Semans);
    - Squeeze technique (Masters & Johnson);
    - Pelvic floor exercises (Pastore A. RCT 2014).
2. Dapoxetine — short-acting SSRI; 30–60 mg 1–3 hours before intercourse; approved for PE in EU and many countries. Not approved for PE by US FDA.
3. SSRI off-label daily — paroxetine (10–40 mg) highest efficacy (Waldinger meta-analysis); sertraline, fluoxetine, citalopram alternatives; possible sexual side effects.
4. Topical anesthetics - lidocaine/prilocaine (EMLA, TEMPE/Fortacin spray) before penetration (cream 20–30 minutes, spray ~5 minutes); protection against partner transmission (condom or wipe).
5. Tramadol — in selected cases (addiction risk must be considered).
6. PDE5 inhibitor — only with comorbid ED.
7. Couple therapy and CBT — for performance anxiety.

8.2 Treatment methods

1. Dapoxetine — Short-acting SSRI; when needed; EU approval.
2. Topical Anaesthetic — Lidocaine/prilocaine (EMLA cream 20–30 min; Fortacin spray ~5 min prior).

3. SSRI Daily (off-label) — Paroxetine highest efficacy.
4. Start-Stop and Squeeze techniques — Behavioral methods; singly or in combination.
5. Pelvic Floor Exercises — Pastore RCT — IELT increase.

8.3 Differences between sources

- Althof S.E. et al. ISSM Guideline. J Sex Med 2014.
- Waldinger M.D. et al. SSRI meta-analysis.

9. Prognosis

Dapoxetine and SSRIs increase IELT by 2–4 times; behavioral combination supports long-term effect.

10. Myths and misconceptions

Myth 1: “PE is only a psychological problem”

Evidence: Lifelong PE is associated with serotonergic neurobiology; the effects of dapoxetine and SSRIs confirm this.

Myth 2: “Dapoxetine is FDA-approved in the USA”

Evidence: Dapoxetine is not approved by the US FDA for PE; it is approved in the EU and many countries; in the US, PE is treated off-label with daily SSRIs.

Myth 3: “Topical anesthetic is safe and has no effect on the partner”

Evidence: Condom or switch to wiping important for partner protection; otherwise partner may experience reduced sensation.

Myth 4: “Over-the-counter ‘PE enhancers’ are safe”

Evidence: FDA warnings — many “natural” products contain undeclared SSRIs or anesthetic components.

Myth 5: “Behavioral therapy is only an old method”

Evidence: ISSM recommends start-stop, squeeze, and pelvic floor exercises; effectiveness increases in combination with pharmacotherapy.

11. Sources

1. WHO. ICD-11. HA03.0 Male early ejaculation. 2024.

2. APA. DSM-5-TR. 2022.
3. Althof S.E. et al. An update of the International Society of Sexual Medicine's guidelines for the diagnosis and treatment of premature ejaculation. J Sex Med 2014;11(6):1392–1422.
4. Waldinger M.D. et al. Relevance of methodological design for the interpretation of efficacy of drug treatment of PE: a systematic review and meta-analysis. Int J Impot Res 2004;16(4):369–381.
5. Pastore A.L. et al. Pelvic floor muscle rehabilitation for patients with lifelong premature ejaculation: a novel therapeutic approach. Ther Adv Urol 2014;6(3):83–88.

ICD-11 **HA00**	**HA00 HYPOACTIVE SEXUAL DESIRE DYSFUNCTION (HSDD)** **Hypoactive sexual desire dysfunction**
ICD-10 F52.0	Lack or loss of sexual desire
DSM-5-TR F52.0 / F52.22	Male Hypoactive Sexual Desire Disorder / Female Sexual Interest-Arousal Disorder

1. Definition and nosology

Hypoactive sexual desire dysfunction (HSDD; ICD-11: HA00; DSM-5-TR in females Female Sexual Interest/Arousal Disorder F52.22, in males Male Hypoactive Sexual Desire Disorder F52.0) — persistent decrease or absence of sexual fantasies and desire for sexual activity; with distress.

Duration ≥ 6 months; situational or generalized; lifelong or acquired.

2. History

- Kaplan H.S. (1977) — inclusion of the desire phase in the sexual response model.
- Basson R. (2000) — circular desire-arousal model in women.
- 2015 — flibanserin FDA approval; 2019 — bremelanotide FDA approval.
- ISSWSH (International Society for the Study of Women's Sexual Health) Process of Care 2018 (Clayton A.H.).

3. Epidemiology

- Women: 8–10% distress (HSDD); men: 14–17%.
- Age-related increase; perimenopause is a risk period.
- Comorbidity: depression, antidepressant use (SSRIs), relationship problems.

4. Aetiology and pathogenesis

- Biological — hormonal (testosterone, estrogen), medication-related (SSRIs, OCs, antihypertensives), chronic diseases.
- Psychological — depression, anxiety, body image, trauma history.
- Relationship — conflict, partner sexual dysfunction.
- Social-cultural — taboo, lack of knowledge.

5. Clinical features

- Reduction or absence of spontaneous sexual fantasy and thought.
- Not initiating and not responding to partner's initiative.
- Distress (critical criterion — mere low desire is insufficient).

6. Diagnosis

6.1 Unified diagnostic criteria (DSM-5-TR · ISSWSH)

A. Persistent reduction or absence of sexual fantasies and desire.

B. Duration ≥ 6 months.

C. Clinically significant distress.

D. Not better explained by another mental disorder, medication, or medical condition.

6.2 Differences between sources

- ISSWSH Process of Care (Clayton A.H. et al. Mayo Clin Proc 2018).
- DSM-5-TR — desire and arousal combined in women (FSIAD).

6.3 Diagnostic algorithm

1. Clinical interview (sexual and relationship history).
2. DSDS (Decreased Sexual Desire Screener) — 5-item in women.

3. Hormonal assessment (morning testosterone in men; when indicated in women).
4. Medication and comorbidity analysis.
5. Depression, anxiety screening.

6.4 Differential diagnosis

Condition	Distinguishing feature
Hypogonadism	Low testosterone + symptoms.
Depressive disorder	Anhedonia, mood disturbance.
SSRI-induced	Medication-related timing.
Anorgasmia (HA02)	Orgasm disorder is dominant.
Relationship problem	Situational.

7. Examination and assessment

- DSDS, FSFI, IIEF.
- Hormonal panel.
- Depression/anxiety scales.

8. Treatment

8.1 General principles (ISSWSH 2018)

1. Biological-psychological-social approach — multimodal according to etiology.
2. Psychotherapy and couple therapy — CBT, mindfulness, sensate focus (Masters & Johnson).
3. Medication change — In SSRI-induced cases, bupropion, mirtazapine, vortioxetine alternatives.
4. Pharmacotherapy in Women (Premenopausal Hypoactive Sexual Desire Disorder):
    - Flibanserin (100 mg at night) — FDA 2015 approval; with alcohol, a risk of severe hypotension and syncope (the FDA relaxed the boxed warning in 2019: wait at least 2 hours after 1–2 standard drinks, skip that evening's dose after 3 or more);
    - Bremelanotide (1.75 mg s/c as needed) — FDA 2019 approval; melanocortin agonist.

5. In postmenopausal women: Transdermal testosterone in low doses (off-label, international consensus Davis S.R. 2019) – careful monitoring.
6. In males: hypogonadism confirmed → TRT (≥2 low morning measurements + symptoms).
7. Relationship and communication work is critical.

8.2 Treatment methods

1. Flibanserin — Serotonin 1A agonist / 2A antagonist; nightly dose; in premenopausal women.
2. Bremelanotide — Melanocortin agonist; s/c as needed.
3. Testosterone In women (off-label) — Postmenopausal; low dose; monitoring.
4. CBT and Mindfulness — Brotto L.A. studies — effective in female sexual dysfunction.
5. Sensate Focus — Couple therapy technique.

8.3 Differences between sources

- Clayton A.H. et al. ISSWSH Process of Care. Mayo Clin Proc 2018.
- Davis S.R. et al. Global Consensus Position Statement on Testosterone Therapy for Women. J Clin Endocrinol Metab 2019.

9. Prognosis

With multimodal approach, most patients improve; chronic form requires ongoing strategy.

10. Myths and misconceptions

Myth 1: “Low desire is always a disorder”

Evidence: distress critical criterion; low desire without distress normal variation — not pathological.

Myth 2: “Testosterone is a libido enhancer for all women”

Evidence: Davis 2019 consensus — low-dose off-label only for postmenopausal HSDD; general timing of administration is not an official recommendation.

Myth 3: “Flibanserin is ‘female Viagra’”

Evidence: The mechanisms are entirely different (serotonergic, central nervous system); daily use; the effect is modest (comparison with sildenafil is incorrect).

Myth 4: “Desire dysfunction is only due to psychological causes”

Evidence: SSRI, OC, antihypertensives, and hormonal changes have a strong effect; biological research is important.

Myth 5: “Over-the-counter ‘aphrodisiacs’ are effective”

Evidence: Cochrane and systematic reviews — small or no effect; FDA warnings about hidden components.

11. Sources

1. WHO. ICD-11. HA00 Hypoactive sexual desire dysfunction. 2024.
2. APA. DSM-5-TR. 2022.
3. Clayton A.H. et al. The International Society for the Study of Women's Sexual Health Process of Care for Management of HSDD in Women. Mayo Clin Proc 2018;93(4):467–487.
4. Davis S.R. et al. Global Consensus Position Statement on the Use of Testosterone Therapy for Women. J Clin Endocrinol Metab 2019;104(10):4660–4666.
5. Basson R. The female sexual response: a different model. J Sex Marital Ther 2000;26(1):51–65.
6. Brotto L.A. Better Sex Through Mindfulness: How Women Can Cultivate Desire. Vancouver: Greystone Books, 2018.

ICD-11 **HA02.0**	**HA02.0 ANORGASMIA** **Anorgasmia**
ICD-10 F52.3	Orgasmic dysfunction
DSM-5-TR F52.31 / F52.32	Female Orgasmic Disorder / Delayed Ejaculation

1. Definition and nosology

Anorgasmia (ICD-11: HA02.0; DSM-5-TR Female Orgasmic Disorder F52.31 in women; Delayed Ejaculation F52.32 in men) — persistent difficulty, delay, or absence of reaching orgasm after sufficient sexual arousal; or reduced intensity of orgasm.

Duration ≥ 6 months; situational or generalized; lifelong or acquired.

2. History

- Masters & Johnson (1970) — classic description of orgasmic phase disorder.
- ISSWSH and ISSM guidelines.

3. Epidemiology

- Women experience clinically significant anorgasmia in 5–10% of cases; men experience delayed ejaculation in 1–4%.
- 30–70% sexual side effects with antidepressant (SSRI) use.

4. Aetiology and pathogenesis

- Medication-related (SSRI, antipsychotic, opioid).
- Neurological (pelvic surgery, MS, diabetic neuropathy).
- Endocrine (hypothyroidism, hyperprolactinemia).
- Psychological — anxiety, trauma, knowledge deficiency.
- Age and hormonal changes.

5. Clinical features

- Difficulty or absence of achieving orgasm.
- Decreased intensity of orgasm.
- Distress, avoidance.

6. Diagnosis

6.1 Unified diagnostic criteria (DSM-5-TR)

A. Delay, absence, or reduced intensity of orgasm following sufficient sexual arousal — in all or almost all instances (75–100%).

B. Duration ≥ 6 months.

C. Distress.

6.2 Differences between sources

- ISSM Delayed Ejaculation Guideline.
- ISSWSH — orgasm dysfunction in women.

6.3 Diagnostic algorithm

1. Clinical interview (sexual history, orgasm with masturbation, orgasm with partner).
2. FSFI (in women) / IIEF (in men).
3. Medication and comorbidity analysis.
4. Endocrine (testosterone, prolactin, thyroid).

6.4 Differential diagnosis

Condition	Distinguishing feature
HSDD (HA00)	Decreased desire dominant.
Retrograde ejaculation	Orgasm present, ejaculate into bladder.
SSRI-induced	Medication-related timing.
ED (HA01)	Erectile disorder.

7. Examination and assessment

- FSFI, IIEF.
- Hormonal panel.
- Neurological and urologic (acquired suspicion).

8. Treatment

1. Etiological approach — management of medication, endocrine, neurological causes.
2. SSRI-induced anorgasmia:
    - Medication switch — bupropion, mirtazapine, vortioxetine alternative;
    - Dose reduction; drug holiday (with caution in SSRIs);
    - Bupropion adjunct;
    - Sildenafil adjunct in women for SSRI-induced dysfunction — Nurnberg JAMA 2008 RCT.
3. Psychotherapy and behavior — orgasm-oriented therapy (Heiman & LoPiccolo); CBT, mindfulness (Brotto L.A.).
4. Directed masturbation training in women — directed masturbation; vibrator.
5. Delayed Ejaculation in males — psychotherapy, modification of masturbation pattern; no specific FDA-approved pharmacotherapy.

6. Hormonal therapy only in confirmed deficiency.

8.1 Treatment methods

1. Bupropion — As adjunct or switch in SSRI-induced anorgasmia.
2. Directed Masturbation — Heiman & LoPiccolo program; effective for primary anorgasmia in women.
3. Mindfulness-CBT — Brotto research; in female sexual dysfunction.

8.2 Differences between sources

- Nurnberg H.G. et al. JAMA 2008 — sildenafil in SSRI-induced female sexual dysfunction.
- Brotto L.A. mindfulness studies.

9. Prognosis

For women, directed masturbation yields 60–90% efficacy for primary anorgasmia; in SSRI-induced cases, pharmacological strategies provide improvement.

10. Myths and misconceptions

Myth 1: "Women must achieve orgasm through penetration"

Evidence: Most women (~70%) require clitoral stimulation; orgasm with only penetration is normal variation, not anatomical norm.

Myth 2: "Anorgasmia is a sign of 'frigidity' in women"

Evidence: Anorgasmia often related to knowledge, technique, anxiety, or medication; not a character trait.

Myth 3: "SSRI-induced anorgasmia is transient, requires no treatment"

Evidence: Chronic; pharmacological interventions or adjunctive strategies are effective; ongoing distress is clinically significant.

Myth 4: "Over-the-counter 'orgasm enhancers' are effective"

Evidence: Evidence weak; FDA warnings hidden component.

Myth 5: "Vibrator use deprives the patient of orgasm with a partner"

Evidence: On the contrary, it is effective in training for primary anorgasmia; can be integrated into intimate relationship with partner.

11. Sources

1. WHO. ICD-11. HA02.0 Anorgasmia. 2024.
2. APA. DSM-5-TR. 2022.
3. Nurnberg H.G. et al. Sildenafil treatment of women with antidepressant-associated sexual dysfunction. JAMA 2008;300(4):395–404.
4. Brotto L.A. et al. Psychological and Interpersonal Dimensions of Sexual Function and Dysfunction. J Sex Med 2016;13(4):538–571.
5. Heiman J.R., LoPiccolo J. Becoming Orgasmic. Prentice Hall, 1988.

ICD-11 **HA20**	**HA20 SEXUAL PAIN-PENETRATION DISORDER (VAGINISMUS)** **Sexual pain-penetration disorder**
ICD-10 F52.5	Nonorganic vaginismus
DSM-5-TR F52.6	Genito-Pelvic Pain/Penetration Disorder

1. Definition and nosology

Sexual pain-penetration disorder (vaginismus; ICD-11: HA20; DSM-5-TR: Genito-Pelvic Pain/Penetration Disorder F52.6) — during penetration attempts: (a) difficulty, (b) genital or pelvic pain, (c) fear of pain, (d) tension of pelvic floor muscles. Classic vaginismus and dyspareunia are combined in this entity.

Duration ≥ 6 months; lifelong or acquired.

2. History

- Sims J.M. (1861) — description of vaginismus.
- Masters & Johnson (1970) — pelvic floor tension.
- DSM-5 (2013) — vaginismus and dyspareunia combined into GPPPD.
- ISSWSH and ACOG guidelines.

3. Epidemiology

- Prevalence in women is 5–17% (in clinical and non-clinical samples).
- Lifelong vaginismus — in young women, at first penetration attempt.
- Acquired — trauma, infection, atrophic vaginitis, post-surgery.

4. Aetiology and pathogenesis

- Biological — atrophic vaginitis (postmenopausal), vulvodynia, endometriosis, infection, surgical wound.
- Psychological — sexual trauma, anxiety, abstinence education, fear of pain.
- Relationship and partner factors.

5. Clinical features

- Reflexive tension of pelvic floor muscles during penetration.
- Pain (burning, stabbing).
- Penetration avoidance.
- Gynecological examination also difficult or impossible.

6. Diagnosis

6.1 Unified diagnostic criteria (DSM-5-TR)

A. At least one of the following:

- Difficulty in penetration;
- Pain in vulvovaginal or pelvic region during or attempted penetration.
- Fear of pain associated with anticipation of penetration.
- In attempted penetration, tension of the pelvic floor muscles.

B. Duration ≥ 6 months.

C. Distress.

6.2 Differences between sources

- ISSWSH and ACOG.

6.3 Diagnostic algorithm

1. Clinical interview (sexual history, pain characteristics, trauma).
2. Careful gynecological examination — with caution, with patient consent.

3. Vulvodynia screening (Q-type test).
4. Infection, atrophic vaginitis, exclusion of endometriosis.
5. Pelvic floor assessment.

6.4 Differential diagnosis

Condition	Distinguishing feature
Vulvodynia	Localized vulvar pain, Q-type positive.
Atrophic vaginitis	Postmenopausal, estrogen deficiency.
Endometriosis	Deep dyspareunia, dysmenorrhoea.
Infection (candida, vaginosis)	Exudation, microbiological.

7. Examination and assessment

- Gynecological examination.
- Pelvic floor assessment.
- FSFI; pain scales.

8. Treatment

1. Multidisciplinary approach — pelvic floor physiotherapy + CBT + education.
2. Pelvic Floor Physiotherapy — biofeedback, manual therapy, dilator therapy (gradual size increase).
3. CBT and exposure therapy — fear of pain and avoidance; ter Kuile M.M. RCT.
4. Sensate focus and couple therapy.
5. Topical anesthetic (lidocaine) — in provoked vulvodynia (Goldstein A.T.).
6. Local estrogen for atrophic vaginitis.
7. Botulinum toxin A — in refractory vaginismus; evidence base is growing.
8. Antidepressant/anticonvulsant (amitriptyline, gabapentin) — for neuropathic component.

8.1 Treatment methods

1. Dilator Therapy — Gradual increase in size; combined with relaxation.
2. Pelvic Floor Physiotherapy — Biofeedback, manual therapy.
3. CBT/Exposure — Fear of pain and avoidance.

4. Topical Estrogen — Atrophic vaginitis.
5. Botox (off-label) — For refractory vaginismus; selective.

8.2 Differences between sources

- ter Kuile M.M. et al. CBT vaginismus.
- Goldstein A.T. et al. ISSWSH vulvodynia consensus.

9. Prognosis

With a multidisciplinary approach, prognosis is good (70–90%); in refractory cases, botox or intensive psychological intervention.

10. Myths and misconceptions

Myth 1: "Vaginismus is an emotional rejection of the partner"

Evidence: Reflex muscle tension — outside conscious control; accusing harmful.

Myth 2: "Vaginismus cannot be treated"

Evidence: CBT + dilator + physiotherapy combination 70–90% efficacy; ter Kuile RCTs.

Myth 3: "Treatment should be done to increase penetration frequency"

Evidence: target is reducing patient distress, restoring sense of control and choice; not a performance indicator.

Myth 4: "Local anesthetic is effective for all sexual pain"

Evidence: Effective in provoked vulvodynia; use limited in generalized vaginismus — main problem is muscle tension.

Myth 5: "Forcing 'compulsory exercise' helps with penetration attempts"

Evidence: urgent, painful repetition increases avoidance and fear; gradual, controlled exposure effective.

11. Sources

1. WHO. ICD-11. HA20 Genito-pelvic pain/penetration disorder. 2024.
2. APA. DSM-5-TR. 2022.
3. Ter Kuile M.M. et al. Therapist-aided exposure for women with lifelong vaginismus: a RCT. J Consult Clin Psychol 2013;81(6):1127–1136.

4. Goldstein A.T. et al. Vulvodynia: Assessment and Treatment. J Sex Med 2016;13(4):572–590.
5. Bergeron S. et al. Genital pain in women and men: It can hurt more than your sex life. Sex Med Rev 2020.

ICD-11 **HA40**	**HA40 AETIOLOGICAL CONSIDERATIONS IN SEXUAL DYSFUNCTIONS AND SEXUAL PAIN DISORDERS** **Aetiological considerations in sexual dysfunctions and sexual pain disorders**
ICD-10 F52.8	Other sexual dysfunction, not caused by organic disorder or disease
DSM-5-TR —	—

1. Definition and nosology

Aetiological considerations in sexual dysfunctions and sexual pain disorders (ICD-11: HA40) — a set of qualifiers identifying the aetiology of sexual dysfunctions (which may involve desire, arousal, orgasm, or pain) arising in the context of another physical illness, medication, substance, or mental disorder. Cases that do not fit specific HA01–HA20 categories or are comorbid.

Subtypes (ICD-11): a medical condition, injury, or the effects of surgery or radiation treatment (HA40.0); psychological or behavioural factors, including mental disorders (HA40.1); use of a psychoactive substance or medication (HA40.2); lack of knowledge or experience (HA40.3); relationship factors (HA40.4); cultural factors (HA40.5); other specified (HA40.Y).

2. History

- DSM-5 (2013) — formalization of “Substance/Medication-Induced Sexual Dysfunction”.

- 2000s — SSRI-induced sexual dysfunction was widely described (Montejo A.L. studies).
- PSSD (Post-SSRI Sexual Dysfunction) — Bahrick A. (2008), EMA 2019 confirmation.

3. Epidemiology

- Sexual dysfunction with SSRI use 30–70%.
- ED in diabetic men 35–75%.
- High prevalence in cardiovascular diseases.
- Antipsychotic (especially risperidone, paliperidone) — due to hyperprolactinemia.

4. Aetiology and pathogenesis

- Medications: SSRI, SNRI, antipsychotics (D2 blockade, hyperprolactinemia), antihypertensive (beta-blocker, thiazide), opioids, 5-alpha reductase inhibitors (finasteride — suspected Post-Finasteride Syndrome), anticonvulsants.
- Substances: alcohol, opioids, chronic cannabis use, stimulants.
- Medical diseases: diabetes, cardiovascular, neurological (MS, Parkinson's disease, spinal trauma), endocrine (hypogonadism, hyperprolactinemia, thyroid), pelvic surgery and radiotherapy, chronic renal failure.
- Mental disorders: depression, anxiety, PTSD, schizophrenia.

5. Clinical features

- Decreased desire, excitement/erection dysfunction, anorgasmia, pain — etiology-dependent.
- Temporal relationship — onset with medication/substance.
- PSSD — persistent sexual dysfunction after SSRI discontinuation, genital numbness.

6. Diagnosis

6.1 Unified diagnostic criteria (DSM-5-TR)

A. Clinically significant sexual dysfunction symptomatology.

B. Evidence from history, physical examination, or laboratory results: symptoms are a direct consequence of medication, substance, or medical illness.

C. Distress.

6.2 Differences between sources

- Montejo A.L. et al. — SSRI-induced dysfunction.
- EMA (2019) — included PSSD in SSRI label.

6.3 Diagnostic algorithm

1. Full medication and substance history.
2. Medical and endocrine assessment (testosterone, prolactin, glucose, HbA1c, lipids, thyroid).
3. Sexual function scales (FSFI, IIEF, ASEX).
4. Mental comorbidity screening.
5. Pelvic examination (in women).

6.4 Differential diagnosis

Condition	Distinguishing feature
Primary HSDD/ED/Anorgasmia	There is no medical or pharmacological evidence.
Depressive disorder	Anhedonia, mood symptoms dominant.
PSSD	Sexual dysfunction persisting after SSRI discontinuation.

7. Examination and assessment

- Hormonal panel, glucose, lipids.
- FSFI, IIEF, ASEX (Arizona Sexual Experience Scale).
- Pelvic or urological examination when indicated.

8. Treatment

1. Etiological approach — first-line:
    - Medication-related — dose reduction, medication switch (bupropion, mirtazapine, vortioxetine, aripiprazole — in antidepressant or antipsychotic strategies);
    - Medical condition management (diabetes, hypertension, hypogonadism).

2. Adjunct pharmacotherapy — sildenafil (PDE5) in ED and SSRI-induced female dysfunction (Nurnberg JAMA 2008); bupropion adjunct.
3. In hyperprolactinemia — antipsychotic switch (aripiprazole, quetiapine) or dose modification; dopamine agonist only with endocrine assistance.
4. Psychotherapy and couple therapy — in psychogenic component.
5. Patient education and informed consent — prior to initiating antidepressants and antipsychotics, it is important to provide information about sexual side effects.
6. PSSD — no specific validated treatment exists; clinical support and monitoring; research ongoing.

8.1 Treatment methods

1. Medication Switch — SSRI → bupropion/mirtazapine/vortioxetine; risperidone → aripiprazole.
2. Sildenafil Adjunct — SSRI-induced dysfunction (male and female); Nurnberg JAMA RCT.
3. Bupropion Adjunct — In desire and orgasm disorders.
4. Antipsychotic Switch (aripiprazole) — In case of hyperprolactinemia.
5. Patient Education — Informed consent; discussing long-term risk.

8.2 Differences between sources

- Montejo A.L. et al. — antidepressant sexual side effect algorithms.
- EMA 2019 — PSSD risk information.
- NICE and APA guidelines — management of antidepressant sexual side effects.

9. Prognosis

Most cases improve with etiological management and medication switch; PSSD may rarely persist.

10. Myths and misconceptions

Myth 1: “Antidepressant sexual side effects are ‘temporary’ and require no intervention”

Evidence: Montejo studies — side effects are persistent (30–70%); require active management.

Myth 2: “PSSD is not a clinical entity”

Evidence: EMA 2019 — PSSD risk included in SSRI label; clinically recognized entity.

Myth 3: “Antipsychotic sexual dysfunction is the same for everyone”

Evidence: risperidone and paliperidone high hyperprolactinemia risk; aripiprazole and quetiapine less; switching is effective.

Myth 4: “Sexual dysfunction due to a medical illness is only a consequence of the illness, no intervention exists”

Evidence: In diabetic ED, PDE5 inhibitors are effective; sexual function improves with cardiovascular rehabilitation; active intervention is important.

Myth 5: “It is not necessary to discuss sexual side effects before starting an antidepressant”

Evidence: informed consent is ethical and clinical requirement; later emerging side effects lead to patient distrust and reduced treatment adherence.

11. Sources

1. WHO. ICD-11. HA40 Aetiological considerations in sexual dysfunctions and sexual pain disorders. 2024.
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5. Nurnberg H.G. et al. Sildenafil treatment of women with antidepressant-associated sexual dysfunction. JAMA 2008;300(4):395–404.
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APPENDICES

APPENDIX A

MONITORING AND EXAMINATION STANDARDS

This appendix presents standard protocols for safety monitoring in the use of psychotropic medications and for monitoring the medical condition of psychiatric patients.

A.1. Metabolic monitoring during antipsychotic therapy

Atypical antipsychotics – especially olanzapine, quetiapine, clozapine – Metabolic syndrome increases the risk. (weight gain, dyslipidemia, insulin resistance, diabetes).

Monitoring chart (APA/ADA 2004, ADA/APA/AACE/NAASO consensus):

Parameter	**Onset**	**4 weeks**	**8 weeks**	**12 weeks**	**Quarterly**	**Annual**
Personal/ Family history	✓					✓
Weight, waist circumference	✓	✓	✓	✓	✓	✓
Blood pressure	✓			✓		✓
Fasting glucose	✓			✓		✓
Fasting lipid profile	✓			✓		✓ (every 2 years)

If abnormality is detected:

- Weight gain ≥7%: consider medication switch (aripiprazole, ziprasidone — metabolically neutral);
- HbA1c ≥6.5% or fasting glucose ≥7: Diabetes Consultation

- LDL ≥3.4 mmol/L: Diet, statin.

Specific monitoring for clozapine (due to agranulocytosis):

Duration	Frequency
First 6 months	Weekly WBC and absolute neutrophil count (ANC)
7–12 months	Every 2 weeks
12+ months	Monthly

ANC indicators:

- ≥2.0 × 10^9/L Normal – continue treatment;
- 1.5–2.0: weekly monitoring;
- 1.0–1.5: Monitor every 2 days, hematology consultation;
- <1.0: medication discontinued, hematologist intervention.

EKG monitoring with antipsychotics:

- QT interval prolongation risk: specifically ziprasidone, haloperidol IV, ondansetron combination;
- Baseline EKG in patients with risk factors (heart disease, electrolyte disturbance, >65 years);
- QTc >500 ms or >60 ms increase from baseline — Medication change;
- Medications increasing risk: chlorpromazine, haloperidol (high), thioridazine.

Hyperprolactinemia:

- Risperidone, amisulpride, paliperidone — high risk;
- Symptoms: amenorrhea, galactorrhea, sexual dysfunction, osteoporosis.
- Basal prolactin; with recurrent symptom.

A.2. Lithium monitoring

Narrow therapeutic window — level 0.6–1.2 mEq/L, >1.5 toxic.

Parameter	Onset	Every 6 months	Annual
Lithium level	Weekly for the 1st month, then every 6 months	✓	
Creatinine, eGFR	✓	✓	
TSH	✓	✓	
Electrolytes	✓	✓	
Calcium	✓		✓
EKG	✓ (50+ years old)		
Pregnancy test	Female of childbearing age		

Signs of lithium intoxication:

- Tremor (more severe);
- Tachycardia;
- Diarrhea, vomiting;
- Slow, confused speech;
- Ataxia;
- Convulsion – in severe conditions.

Check level in state change: dehydration, NSAID addition, ACE inhibitor addition, kidney function change.

A.3. Anticonvulsants (valproate, carbamazepine, lamotrigine)

Valproate:

Parameter	Onset	Every 6 months
CBC (thrombocytopenia)	✓	✓
Liver function (AST, ALT)	✓	✓
Valproate level		With symptom or suspicion

Parameter	Onset	Every 6 months
Monitoring signs of pancreatitis		Every visit
Pregnancy test	**ABSOLUTELY DEFINITE in elderly woman**	**Every quarter**

Critical: valproate postpartum-aged women Due to neural tube defect and cause of cognitive impairment only with effective contraception, after other options have been evaluated first.

Carbamazepine:

- CBC, liver – baseline, 1 month, quarterly;
- HLA-B*1502 test In patients of Asian origin (Stevens-Johnson risk);
- Level drop due to auto-induction in the first 4 weeks – check level.

Lamotrigine:

- Slow titration — Stevens-Johnson syndrome risk;
- Starting 25 mg per week, to target dose within 4–8 weeks;
- Along with valproate: starting dose 12.5 mg — valproate slows lamotrigine metabolism;
- If a skin reaction occurs – immediately discontinue.

A.4. Monitoring of SSRIs and SNRIs

- Hyponatremia risk — especially in elderly, in those using diuretics;
- Baseline Na, after 1 month, then with symptom;
- Increased suicidality especially with onset under 25 years — close monitoring during first 1-2 months;
- QTc prolongation — escitalopram, citalopram (at high doses);
- Serotonin syndrome (MAOI or triptan, etc., in combination) – symptom education.

A.5. Benzodiazepine use

- Maximum duration: 2–4 weeks;

- Long-term use inevitably leads to addiction;
- Reduction schedule (tapering): 25% every 2 weeks, sometimes slower;
- Re-experiencing symptoms. (rebound anxiety, insomnia, tremor) is normal and temporary.

A.6. Psychotherapy monitoring

Quality indicators:

- Session duration, frequency;
- Patient symptom scales (PHQ-9, GAD-7, etc.) every 4–6 weeks;
- Functional indicators;
- Assessment of therapeutic alliance (Working Alliance Inventory).
- Achieving external goals;
- Signs of relapse.

Supervision and peer consultation — in complex cases, and whenever the clinician asks for it.

APPENDIX B

SCREENING SCALES FOR NEURODEVELOPMENTAL DISORDERS

Usage guidelines: These scales are not diagnostic — they are screening tools. A positive result is an indication for specialized clinical assessment, not a diagnosis. A definitive diagnosis is established through clinical interview, comprehensive assessment, and (when necessary) standardized diagnostic tools (ADOS-2, ADI-R, WISC-V, etc.).

Licence status and disclaimer. The screening scales reproduced in full in this appendix (M-CHAT-R, AQ-10, ASRS v1.1, SNAP-IV) are instruments that their developers have made freely available for research and clinical use. This volume is a clinical guide, not a compendium of diagnostic tests. Licensed instruments referenced elsewhere in the text (WISC-V, ADOS-2, Vineland-3, and others; see Appendix B.5) are not reproduced here. Obtaining and administering them requires a valid licence from the rights holder (Pearson, WPS, MHS, or equivalent) and appropriate professional qualification.

B.1. M-CHAT-R: Modified Checklist for Autism in Toddlers, Revised (16–30 months)

Original authors: Diana L. Robins, Deborah Fein, Marianne Barton, 2009.

Original reference: Robins DL, Fein D, Barton M. M-CHAT-R/F. Self-published, 2009; Pediatrics, 2014; 133: 37–45.

Instruction

This questionnaire consists of 20 questions about your child's daily behavior. To be completed by parent or primary caregiver. Answer each question YES or NO — based on how your child typically behaves. If you've seen the behavior once or twice, but the child generally does not do it, answer NO.

Questions

№	Question	YES	No
1	If you look at something on the other side of the room, does your child also look there? (For example, at a flying plane, a large animal, or a toy)		
2	Have you ever thought your child might be deaf?		
3	Does your child play pretend games? (For example, drinking from empty cup, talking on phone, feeding doll)		
4	Does your child like climbing objects? (Furniture, stairs, tree)		
5	Does your child make unusual movements with palms, fingers or other parts of body in front of eyes?		

№	Question	YES	No
6	Does your child point to something they want? (For example, a toy far away in buffet)		
7	Does your child point to attract your attention to something interesting? (For example, airplane in sky or big car)		
8	Does your child show interest in other children? (For example, looks at children in playground, tries to join them)		
9	Does your child share with you by showing or placing something before you (not for help, but for sharing)?		
10	Does your child respond when their name is called?		
11	When you smile to your child, do they smile back to you?		

№	Question	YES	No
12	Does your child get bothered by everyday sounds? (For example, vacuum cleaner, loud music)		
13	Does your child walk?		
14	Does your child look into your eyes when you are with them, dressing them or playing?		
15	Does your child imitate what you do? (For example, when you make activity on your face, they do same)		
16	Does your child look at you when they hear something to see what happened?		
17	Does your child try to get your attention? (e.g., looking at you, for praise, saying 'look!')		
18	Does your child respond to understood words? (e.g., 'give me the ball' or 'take the book')		

№	Question	YES	No
19	If something new or strange happens, does your child look at your face to see your reaction?		
20	Does your child enjoy active games? (For example, bouncing in air, hopping on knees)		

Scoring

Risk score = number of YES responses, exception: for items 2, 5, and 12 answer NO — risk indicator (YES answer on these three items considered risk).

That is:

- 1, 3, 4, 6, 7, 8, 9, 10, 11, 13, 14, 15, 16, 17, 18, 19, 20-ci suallarda NO response considered a risk indicator.
- 2, 5, 12-ci suallarda YES answer considered a risk indicator.

Interpretation of results

Score	Risk level	Recommendation
0–2	Low risk	Additional assessment is not required (if no other risk factors); rescreening recommended up to 24 months
3–7	Moderate risk	**M-CHAT-R/F (Follow-up) required** — structured follow-up interview; conducted by medical staff

Score	Risk level	Recommendation
8–20	High risk	**Urgent ASD diagnostic assessment and referral to an early intervention program is required**

Note: M-CHAT-R – used in various clinics in Azerbaijani, but official validation in the Azerbaijani population is limited. The above translation is for clinical practice; original official translation should be used in scientific research (for official translations: https://mchatscreen.com).

B.2. AQ-10: Short Autism Spectrum Quotient (16 years+)

Original authors: Carrie Allison, Bonnie Auyeung, Simon Baron-Cohen, 2012.

Original reference: Allison C, Auyeung B, Baron-Cohen S. Toward brief "red flags" for autism screening: the short Autism Spectrum Quotient (AQ-10). *J Am Acad Child Adolesc Psychiatry*, 2012; 51: 202–212.

Instruction

What do you think about the following 10 statements? For each statement Strongly agree, Somewhat agree, Somewhat disagree, Strongly disagree select one of the variants.

Questions

№	Statement
1	I often notice small sounds that others do not notice
2	I usually pay more attention to the overall picture rather than small details
3	I have difficulty following a conversation among several people in a social situation
4	I often focus so much on one topic or activity that I lose track of other things

№	Statement
5	I easily understand what someone is thinking or feeling when they say something
6	I am good at reading people's intentions
7	I have difficulty understanding what characters are thinking when reading a story
8	I like to collect detailed information about activities I have done before (e.g., car models, birds, trains)
9	I have difficulty understanding what a person is thinking or feeling when I look at them
10	I have difficulty knowing how to behave in social situations

Scoring

Question	"Absolutely agree / Partially agree" – 1 point	"Absolutely not agree / Partially not agree" – 1 point
1	✓	
2		✓
3	✓	
4	✓	
5		✓
6		✓
7	✓	
8	✓	
9	✓	
10	✓	

Total score = out of 10

Interpretation of results

Score	Recommendation
0–5	ASD diagnostic assessment probability is low
≥6	**Detailed ASD clinical assessment is recommended**

Note: AQ-10 is a short version of the classic AQ-50. It is officially recommended by NICE CG142 (UK) for adult ASD screening in primary care. Sensitivity ~77%, specificity 36–41%. AQ-10 is particularly reliable in "self screening" and should be used together with clinician's initial interview.

B.3. ASRS v1.1: Adult ADHD Self-Report Scale

Original authors: WHO, Ronald C. Kessler and colleagues, Harvard Medical School, 2005.

Original reference: Kessler RC, Adler L, Ames M, et al. The World Health Organization Adult ADHD Self-Report Scale (ASRS). *Psychological Medicine*, 2005; 35: 245–256.

Instruction

This scale is for screening HDHD symptoms in adults. The following 6 questions – within the last 6 months Answer based on how you felt. For each question Never, Rarely, Sometimes, Often, Very often select one of the variants.

Part A – Primary screening (6 questions)

№	Question	Never	Rarely	Sometimes	Often	Very often
1	How often do you have difficulty finishing the final details after a difficult part of a project is completed?					
2	How often do you have difficulty prioritizing tasks when organizing a demanding job?					
3	How often do you have trouble remembering appointments or obligations?					

№	Question	Never	Rarely	Sometimes	Often	Very often
4	When you have a task that requires a lot of thinking, how often do you hesitate to start or procrastinate?					
5	How often do you move or fidget your hands and feet when you have to sit for a long time?					
6	How often do you feel 'motor-driven' hyperactive or compelled to do something?					

Calculation (for Part A)

Responses in the bold cells of the table — a positive sign.

Question	Responses counted as positive signs
1	**Sometimes, Often, Very often**
2	**Sometimes, Often, Very often**
3	**Sometimes, Often, Very often**
4	**Often, Very often**
5	**Often, Very often**
6	**Often, Very often**

Total positive symptom count = _____ / 6

Interpretation of results

Number of positive signs	Comment
0–3	Symptoms not typical for adult ADHD
≥4	**Symptoms highly consistent with adult ADHD. Medical evaluation recommended.**

Part B – Extended questionnaire (12 additional questions)

When Part A is positive, Part B questions (12 additional questions) are used in conjunction with clinical assessment. The full version of Part B can be obtained from the developers' official website: https://www.hcp.med.harvard.edu/ncs/asrs.php

Note: ASRS v1.1 – Developed by WHO, the most widely used adult ADHD screening scale in the world. By Kessler et al. worldwide ASRS-5 (APA, 2013 — according to DSM-5 criteria, 2017) Processed — a shorter 5-question version. Both versions are suitable for screening; ASRS v1.1 remains widely used.

B.4. SNAP-IV: Swanson, Nolan, and Pelham Rating Scale (child ADHD)

Original authors: James M. Swanson, 1983.

Original reference: Swanson JM. School-based assessments and interventions for ADD students. Irvine, CA: KC Publishing, 1992.

Instruction

SNAP-IV – the assessment of ADHD symptoms in children based on DSM-IV/APA, 1994 criteria, DSM-IV/APA, 2013 – DSM-5 criteria. To be completed by parent or teacher. How is each symptom observed in your child over the past 1 month: None (0), Mild (1), Moderate (2), Severe (3).

Questions (SNAP-IV short version – 26 questions)

Inattention subscale (Items 1–9):

№	Statement	0	1	2	3
1	Often makes careless mistakes in schoolwork or other activities				
2	Often has difficulty sustaining attention in tasks or play				
3	Often appears not to listen when spoken to directly				
4	Often fails to follow instructions and to finish schoolwork or daily chores				

№	Statement	0	1	2	3
5	Often has difficulty organizing tasks and activities				
6	Often avoids, dislikes, or reluctantly performs tasks requiring sustained mental effort				
7	Often loses necessary items for tasks or activities (school materials, pencils, books)				
8	Often easily distracted by extraneous stimuli				
9	Often forgetful in daily activities				

Hyperactivity-Impulsivity Subscale (Items 10–18):

№	Statement	0	1	2	3
10	Often fidgets with hands or feet or squirms in seat				
11	Often leaves seat in room when remaining seated is required				
12	Often runs about or climbs excessively in inappropriate situations				
13	Often has difficulty playing quietly				
14	Often behaves as ‘motor-driven’ or ‘hyperactive’				
15	Often talks excessively				

№	Statement	0	1	2	3
16	Often blurts out answers before questions are completed				
17	Often unable to wait for his/her turn				
18	Often interrupts others' activities (conversation, game)				

Oppositional-defiant behavior subscale (Items 19–26):

№	Statement	0	1	2	3
19	Often loses temper				
20	Often argues with adults				
21	Often defies adult requests or rules				
22	Often deliberately annoys others				

№	Statement	0	1	2	3
23	Often blames others for his/her mistakes or misbehavior				
24	Often easily irritated by others				
25	Often irritable and quick-tempered				
26	Often behaves vindictively or spitefully				

Scoring

Mean of each subscale:

- Inattention subscale (1–9): sum ÷ 9 = ____
- Hyperactivity-Impulsivity Subscale (10–18): sum ÷ 9 = _____
- Oppositional-defiant subscale (19–26): sum ÷ 8 = _____

Interpretation of results (moderate levels)

Subscale	Parent: positive boundary	Teacher: positive boundary
Inattention	≥1.78	≥2.56
Hyperactivity-impulsivity	≥1.44	≥1.78
Oppositional-defiant	≥1.88	≥2.00

These cutoffs are based on data from Bussing et al., 2008 and are for the US population. No specific validation has been conducted in the Azerbaijani population – results should be interpreted in a clinical context.

Diagnostic interpretation

Presentation type of ADHD according to DSM criteria:

- Primarily inattentive: Only inattentiveness subscale positive;
- Primarily hyperactive-impulsive: Only hyperactivity subscale positive;
- Combined: Both subscales positive.

A positive result on the oppositional-defiant subscale indicates the likelihood of comorbid oppositional-defiant disorder (ODD, ICD-11 6C90).

Note: SNAP-IV is freely available for research and clinical use. The full 90-item version, the short 26-item version, and teacher versions can be downloaded from the official website of Prof. Swanson: http://www.shared-care.ca/files/Scoring_for_SNAP_IV_Guide_26-item.pdf

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B.5. Key Instruments for Intellectual and Adaptive Assessment (Data Table)

The following tools are licensed psychological instruments and cannot be fully provided within the text. Clinical psychologist or specialized expert required for their acquisition and use.

B.5.1. Intellectual Assessment

Instrument	Age group	Publisher	Typical time
WPPSI-IV (Wechsler Preschool and Primary Scale of Intelligence)	2 years 6 months – 7 years 7 months	Pearson	60–90 min
WISC-V (Wechsler Intelligence Scale for Children)	6–16 years	Pearson	60–90 min
WAIS-IV/V (Wechsler Adult Intelligence Scale)	16–90 years	Pearson	60–90 min

Instrument	Age group	Publisher	Typical time
Stanford-Binet 5	2 years+	Riverside Insights	45–75 min
Leiter-3 (non-verbal)	3–75+ years	Stoelting	25–40 min
Raven's Progressive Matrices	5 years+	Pearson	20–40 min

B.5.2. Adaptive behavior

Instrument	Age range	Publisher	Format
Vineland-3 (Vineland Adaptive Behavior Scales)	Birth to 90+ years	Pearson	Semi-structured parent/caregiver interview
ABAS-3 (Adaptive Behavior Assessment System)	Birth to 89 years	WPS	Parent/teacher rating form

B.5.3. Autism diagnostic tools

Instrument	Age group	Publisher	Typical time
ADOS-2 (Autism Diagnostic Observation Schedule)	12 months – adult	WPS	40–60 min (5 modules)
ADI-R (Autism Diagnostic Interview – Revised)	2 years – adult	WPS	90–150 min
CARS-2 (Childhood Autism Rating Scale)	2 years+	WPS	15–30 min

Instrument	Age group	Publisher	Typical time
SRS-2 (Social Responsiveness Scale)	2.5 years+	WPS	15–20 min

B.5.4. Detailed assessment of ADHD

Instrument	Age group	Publisher
Conners-3 (Conners Rating Scales)	6–18 years	MHS
Conners CBRS	6–18 years	MHS
CAARS (Conners Adult ADHD Rating Scales)	18+ years	MHS
BRIEF-2 (Behavior Rating Inventory of Executive Function)	5–18 years	PAR
CPT-3 (Conners Continuous Performance Test)	Age 8+	MHS

Author's note

The screening scales presented in Appendix B (M-CHAT-R, AQ-10, ASRS v1.1, SNAP-IV) are ready for immediate use in clinical practice. The authors and their affiliated institutions — the World Health Organization, Harvard Medical School, Professor Simon Baron-Cohen and colleagues, Diana Robins, Deborah Fein, Marianne Barton, and James Swanson — are gratefully acknowledged for their substantial contributions to modern psychiatry.

Licensed diagnostic instruments (ADOS-2, WISC-V, Vineland-3, and others) cannot be reproduced in full within this volume. Their administration requires specialised training and an officially purchased licence. Clinicians should obtain these instruments directly through the authorised publishers (Pearson, WPS, MHS, or equivalent distributors in their jurisdiction).

Note on translation and local validation. The Azerbaijani-language translations provided in this volume represent a working clinical version. They have not yet

undergone formal psychometric validation for the Azerbaijani population. For use in scientific publications or validation research, clinicians should consult the official publishers to obtain authorised translations. The development of population-specific normative data for Azerbaijan remains an important priority for future work.

APPENDIX C

PSYCHIATRIC EMERGENCY SITUATIONS

In a psychiatric emergency, time is the scarcest resource. Decisions are measured in minutes and must almost always be made on incomplete information. This appendix is written for precisely those moments: it offers concise, sequenced action lists rather than extended discussion. These lists do not replace clinical judgement; they serve as a safeguard against the step most easily overlooked under pressure.

C.1. Active suicide risk

Assessing suicide risk is a conversation, not a questionnaire. Ask directly: the question does not plant the idea — more often it gives the patient the first chance to say it out loud. What you are listening for above all is how far the intention has taken concrete shape.

Primary assessment:

- Are there suicidal thoughts? Plans? Means?
- How concrete the plan is: when the patient names a time, a place and a means, the risk rises sharply; a general “sometimes I want to die” is a different order of thing;
- Previous attempts — the strongest predictor;
- Substance intoxication — inhibition decreased;
- Protective factors — family, religious beliefs, future plans.

High risk signs:

- A concrete plan and easy access to the means;
- Recent situational change (loss, severe illness);
- Loneliness and chronic pain;
- Psychosis or severe depression;
- The early phase of recovery from depression — energy returns while mood is still low;
- Adult male, disability, alcohol.

Management:

Low-moderate risk:

- Safety plan — written;
- Reducing access to means (gun removal, medication in small quantities)
- Close outpatient follow-up (a repeat visit within 24–48 hours);
- The crisis line number;
- Family involvement (with consent).

High risk:

- Hospitalization (voluntary or involuntary);
- Close observation — one-to-one if the risk is very high;
- Removal of dangerous means (sharp objects, rope, access to windows);
- Controlled medication intake.

Involuntary hospitalization within the Azerbaijani framework:

- "On psychiatric assistance" Law of the Republic of Azerbaijan;
- For involuntary hospitalization decision by two psychiatrists and legal procedure;
- 72-hour emergency observation followed by a court order.

C.2. Delirium tremens and alcohol withdrawal

Symptoms:

- Tremor — hands, then general;
- Anxiety, agitation;
- Hallucinations — visual and tactile ("insects under the skin");
- Disorientation;
- Autonomic hyperactivity — tachycardia, pressure, sweating, heating;
- Seizures (~5-10%).

Treatment (in ICU or IMU setting):

- Benzodiazepine — diazepam 10–20 mg IV/IM every 15–20 min until sedation is achieved; then maintenance regimen;
- Alternative: lorazepam (in those with hepatic dysfunction);

- Thiamine 100-300 mg IV — Prevention of Wernicke's encephalopathy. Before glucose administration.
- Fluids and electrolytes Recovery;
- Magnesium sulfate, potassium – often low;
- Mortality: untreated 15-20%, with adequate treatment <1%.

CIWA-Ar scale — for monitoring.

C.3. Serotonin syndrome

Cause SSRI + MAOI, SSRI + triptan, SSRI + tramadol, SSRI + linezolid, SSRI + methylene blue, etc.

Symptoms:

- Mental change — agitation, confusion;
- Autonomic — tachycardia, hypertension, hyperthermia, sweating;
- Neuromuscular — tremor, hyperreflexia, clonus (lower extremity – specific sign).

Treatment:

- Immediate discontinuation of causative medication
- Support — fluids, electrolytes, cooling;
- In severe cases cyproheptadine 12 mg initial, 2 mg every hour maximum 32 mg/day;
- Invasive intervention (intubation) in severe hyperthermia.

C.4. Antipsychotic Malignant Syndrome (NMS)

Cause Antipsychotics (especially high potency, haloperidol), acute initiation or dose increase.

Symptoms (4 main):

1. Muscle stiffness (“lead pipe”);
2. Hyperthermia (38–42°C);
3. Autonomic lability (high blood pressure, tachycardia, sweating);
4. Mental state change (confusion → stupor → coma).

Laboratory: CPK acutely elevated. (>1000), myoglobinuria, leukocytosis, ALT/AST elevated.

Treatment (ICU):

- Immediate discontinuation of antipsychotics
- Cooling.
- Fluids (risk of rhabdomyolysis and acute kidney failure);
- Dantrolene 1-2.5 mg/kg IV – reduces muscle rigidity;
- Bromocriptine 2.5-5 mg every 8 hours – dopamine agonist;
- ECT in non-responsive cases.

Deathfulness: 5-20% with adequate treatment.

C.5. Acute dystonia

Cause Antipsychotic initiation, especially potent D2 antagonists.

Symptoms:

- Acute muscle spasms – neck (torticollis), tongue, eyes (oculogyric crisis), certain esophageal spasm;
- Life-threatening – laryngeal spasm.

Treatment:

- Diphenhydramine 50 mg IM/IV – relief within 15 minutes;
- Benztropine 1-2 mg IM/IV;
- Reducing or changing the dose of antipsychotics
- Prophylactic anticholinergic in high-risk patients.

C.6. Acute mania and psychomotor agitation

Management:

- Oral preferred: haloperidol 2-5 mg + lorazepam 1-2 mg;

- IM regimen: haloperidol 5 mg + lorazepam 2 mg (50/50 syringe) or olanzapine IM 10 mg;

- Safe environment — quiet, free from stimuli;

- Verbal de-escalation first step;
- Medication:
- Involuntary hospitalization By instruction;
- Physical restraint — only as a last resort, when life is at risk, and only briefly.

C.7. Acute psychosis

Similar protocol as above. Differential: organic causes, substance intoxication, postpartum psychosis, NMS.

C.8. Catatonia

Symptoms:

- Stupor, mutism;
- Catalepsy (‘waxy flexibility’);
- Negativism;
- Mannerisms, stereotypies;
- Echolalia, echopraxia.

First-line treatment:

- Lorazepam 1-2 mg IV/IM test — Significant response within 1-2 hours → diagnosis and treatment of catatonia;
- If response is obtained — increase to a dose of 4–24 mg/day;
- No answer – ECT Second-line, highly effective;
- Avoiding antipsychotics During acute phase – NMS administration may be provocative.

Search for cause organic (encephalitis, epilepsy), mood disorders, schizophrenia, autism, certain substances.

APPENDIX D

MILITARY-MEDICAL EXPERTISE (IN AZERBAIJAN)

This appendix provides practical guidance on psychiatric aspects for assessing fitness for military service in the Republic of Azerbaijan.

D.1. Legal basis

- “On military service call-up” Law of the Republic of Azerbaijan;
- Regulation on military-medical expertise;
- Ministry of Defense Normative acts;
- Table of diseases — divided into halves: mild/severe degrees, groups A–D.

D.2. Psychiatric assessment structure

Main expert examination:

- Complaints and history (patient and close relatives);
- Previous medical records;
- Clinical interview and mental status examination;
- Experimental-Psychological Examination (EPM) – by psychologist;
- Laboratory and instrumental examinations when necessary;
- Differential, diagnosis, and assessment of disease severity;
- Conclusion.

Typical psychological tests (EPM):

- MMPI-2 (Minnesota Multiphasic Personality Inventory-2) – personality profile;
- Raven Progressive Matrices — intellect;
- Luria tests — memory, attention;
- Wechsler IQ scale (WAIS);
- Rorschach test — a projective method (its contemporary reliability is contested);

- Beck, Hamilton scales — mood.

D.3. Psychiatric disorders and military fitness assessment

Classification by group (adaptation – based on local normative acts):

Group A – fully fit for service

No psychiatric history or minor transient issues.

Group B – useful with limitations

- Mild personality traits (below impairment threshold);
- Transient adjustment disorders;
- Mild sleep disturbances;
- Neurotic disorders in remission state.

Group V — limited during service

- Moderate psychiatric disturbances in partial remission state;
- Specific personality disorders;
- Past PTSD, compensated;
- Moderate neurocognitive dysfunction.

Group Q – Unfit for Service

- Schizophrenia spectrum;
- Bipolar disorder;
- Severe depression;
- Active PTSD/complex PTSD (Karabakh war veterans – separate treatment program);
- Substance use disorders;
- Severe personality disorders;
- Mental retardation;
- Severe forms of autism spectrum;
- Disorders with active suicide risk;
- Paraphilic disorders (severe, active).

D.4. Postpartum trauma and veteran assessment

In veterans of the Karabakh war:

- Complex PTSD high prevalence — trauma-focused intervention;
- Alcohol and substance use Comorbid;
- Depression, suicide risk;
- Family dysfunction;
- Moral injury — ethical conflict during war (killing, witnessing);
- Burning and re-traumatization risk factors.

Specific rehabilitation programs:

- Evidence-based trauma therapies (PE, CPT, EMDR);
- Group therapy for veterans;
- Family therapy;
- Medication (SSRI/SNRI);
- Social rehabilitation (work, education).

APPENDIX E

RELATED NEUROLOGICAL CONDITIONS

Brief description of neurological diseases important for clinical practice in areas intersecting psychiatry and neurology.

E.1. Epilepsy (G40) and psychiatric manifestations

Epileptic syndromes and psychiatry:

- Interictal personality changes (Geschwind syndrome – hyper religiosity, hypergraphia, “viscosity”);

- Inter-ictal psychosis – long-standing TLE;

- Post-ictal psychosis – psychotic episodes within 24-72 hours after a seizure;

- Ictal anxiety and dissociative experiences;

- Temporal lobe epilepsy (TLE) — psychiatric comorbidity most common:
- Absence epilepsy — attention problems, differential with ADHD;
- Juvenile myoclonic epilepsy — high comorbid anxiety, impulsivity.

Psychogenic (non-epileptic) seizures (PNES) – dissociative neurological symptom disorder (6B64.4):

- Video-EEG — gold standard differential;
- PNES frequently comorbid with epilepsy, 10-30%;
- Treatment: CBT-PNES, slow tapering of anticonvulsants.

Psychiatric side effects of antiepileptics:

- Levetiracetam: Aggression, irritability, depression;
- Phenobarbital, primidone: depression, cognitive slowing;
- Topiramate: cognitive dysfunction, depression;
- Lamotrigine: Gene stabilizes mood.

E.2. Headache and psychiatric comorbidity

- Migraine — depression risk 2-4× higher; anxiety disorders;

- Cluster headache — depression, suicide risk;
- Chronic daily headache — analgesic overuse (medication overuse headache – MOH) – mood disorder comorbid frequently;
- Recurrent tension headache — associated with intense stress, anxiety.

Management:

- SSRI/SNRI improves many types of headache;
- TCA (amitriptiline) historical standard for migraine prophylaxis
- Topiramate, valproate prophylactic;
- CBT is effective for chronic headache;
- Avoidance of opioids and excessive analgesic use.

E.3. Cerebrovascular diseases (stroke)

- Post-stroke depression: 30-40% first year;
- Post-stroke mania: Rare, in right frontal or thalamic lesions;
- Pseudobulbar affect (pathological laughing/crying): dextromethorphan-quinidine combination confirmatory treatment is;
- Vascular cognitive disorder/dementia: classified under 6D81;
- Rehabilitation: Multidisciplinary, psychiatric support plays a central role.

E.4. Parkinson's disease

- Depression: 30-50%;
- Anxiety: 30-40%;
- Dopamine agonist-induced impulse control disorder: 13-36%;
- Psychosis in Parkinson's disease: Mainly visual hallucinations; levodopa-induced;
- Dementia in Parkinson's disease: 6D85;
- REM sleep behavior disorder Early marker for Parkinson's disease.

Treatment characteristics:

- Use of antipsychotics in Parkinson's disease: quetiapine, clozapine (D2 minimal blockade);
- Haloperidol, risperidone — worsens motor symptoms;
- SSRIs depression;

- Pimavanserin — Confirmed for Parkinson psychosis in the USA.

E.5. Multiple sclerosis (MS)

- Depression: 30-50%;
- Anxiety: 30%;
- Bipolar-like symptoms: 5-10%;
- Pseudobulbar affect;
- Cognitive dysfunction — in most illnesses;
- Lethargy and fatigue — closely associated with depression, differential diagnosis difficult.

Interferon-beta therapy Depression worsens – active monitoring.

E.6. Traumatic brain injury (TBI)

- Post-concussion syndrome: headache, dizziness, memory issues, mood changes;
- Chronic traumatic encephalopathy (CTE): repeated injuries, sports (boxing, American football);
- PTSD + TBZ — very frequently comorbid;
- Personality change (6E68) – in frontal lesions.

E.7. Dementia (detailed in Section 17)

- 6D80 Alzheimer's;
- 6D81 Vascular
- 6D82 Lewy body;
- 6D83 Frontotemporal;
- 6D84 Substance-related;
- 6D85 With Parkinson's disease;
- 6D86 Behavioral/psychological disturbances in dementia (BPSD)

E.8. Seizures, movement disorders, and psychiatry

- Tourette syndrome (8A05.00) — In ICD-11, under neurological disorders section, but highly comorbid with ADHD, OCD.

- Wilson's disease — genetic disorder of copper metabolism; psychiatric presentation (depression, psychosis) frequent; investigation – ceruloplasmin, 24-hour urinary copper;
- Huntington's disease — mood disorders, OCD, psychosis, motor chorea.

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4. ISTSS (International Society for Traumatic Stress Studies). *PTSD Prevention and Treatment Guidelines.* 3rd ed. 2019.
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2. *World Psychiatry* (WPA)
3. *Lancet Psychiatry*
4. *JAMA Psychiatry*
5. *British Journal of Psychiatry*
6. *Psychological Medicine*
7. *Journal of Clinical Psychiatry*
8. *European Psychiatry*

Online resources

1. ICD-11: https://icd.who.int
2. MGH Center for Women's Mental Health: https://womensmentalhealth.org
3. Cochrane Library: psychiatric systematic reviews
4. UpToDate Psychiatry — a clinical reference resource
5. Psychiatry Online (APA Publishing) — textbooks, guidelines
6. Columbia Suicide Severity Rating Scale: https://cssrs.columbia.edu

Baku, 2026

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FINAL RECOMMENDATIONS

A practical synthesis for clinicians.

Key clinical lessons drawn from all chapters of this volume:

Diagnosis is always the second step, not the first

Before diagnosing a mental disorder, always:

- Exclude medical causes — particularly new onset after age 40–50, acute presentation, atypical profile, somatic symptoms, hormonal period changes;
- Check substance use — covert substance component in 30% of each case;
- Ask trauma history with open questions — complex PTSD and personality disorders are often the result of trauma, but trauma history does not emerge without screening.

ICD-11 'primary vs secondary' principle — the main framework for clinicians.

Psychotherapy – first choice in most cases

Central message of the ICD-11 era: Psychotherapy effectiveness is confirmed, at least equal to pharmacotherapy, superior in many disorders:

- Mild-moderate depression — CBT, IPT equivalent to SSRIs;
- In anxiety disorders — CBT first-line;
- In OCD — ERP is essential; SSRIs adjunctive;
- In PTSD — TF-CBT, EMDR first-line;
- Personality Disorders — DBT, MBT, TFP – more effective than pharmacotherapy;
- In eating disorders — CBT-E, FBT;
- In childhood behavioral disorders — parent education programs.

Pharmacotherapy is in many cases an adjunct — it helps the patient to gain and to keep the effect of psychotherapy.

Principle of minimum effective dose

- Start low, increase slowly;
- Reaching the lowest effective dose and staying on it;

- Avoid excessive polypharmacy — especially in elderly, in comorbidity, in personality disorders;
- Discontinuation or dose reduction also takes knowledge — SSRIs must not be stopped abruptly; benzodiazepines should be gradually tapered.

Working with culture and context

In the Azerbaijani context:

- Somatic presentation is widespread — depression and anxiety call for active screening in primary care;
- Stigma and privacy — a visit to the psychiatrist is concealed; working in primary care or internal medicine base is effective;
- Family system is strong — family involvement in treatment is a major resource;
- Religious and cultural beliefs — should be taken into account, but not turned into pathology;
- Karabakh War veterans — PTSD, cPTSD, high need for trauma-focused intervention.

Screening should be standard in primary care

The main screening tools should be routinely applied in primary care:

- PHQ-9 — depression (at every medical visit);
- GAD-7 — anxiety;
- AUDIT — alcohol use;
- EPDS — perinatal women;
- C-SSRS — suicide risk.

Investments should be made in tools translated and validated into Azerbaijani.

Assessment of suicide risk

Active inquiry at every psychiatric interview:

- Have you had thoughts about death or ending your life recently?
- Concrete plan, means?
- Previous attempts?

Risk factors:

- Previous attempts (strongest predictor);
- History of suicide in the family;
- How near the plan is and how specific;
- Access to means;
- Substance use, depression, BPD, psychosis;
- Acute stressor, loss.

Intervention:

- Safety plan;
- Reducing access to means;
- Close monitoring;
- Indications for hospitalization;
- Assessment structured with a scale such as the C-SSRS.

Comorbidity is the rule, not the exception

In mental disorders comorbidity is the norm:

- Depression + anxiety — 60% comorbid;
- PTSD + depression — 50%+;
- Personality disorder + depression/anxiety/substance — 80%+;
- Autism + ADHD — 50%;
- Eating disorder + depression + trauma — typical triad.

The treatment plan must cover every comorbidity — treating the main disorder alone is often not enough.

Age and developmental stage must be considered.

- Mental disorder in child — parent and context are crucial; medications less, psychotherapy and family orientation more;
- Adolescent — identity development, early intervention critical;
- Adult — work, family, social function;
- Older adults — comorbid medical illnesses, polypharmacy, cognitive impairments; dementia vs depression special attention.

Multidisciplinary team – the foundation of effective clinical work

In complex cases:

- Psychiatrist;
- Psychologist / psychotherapist;
- Social worker;
- Medical physician (comorbidities);
- Family members (with consent);
- People at school or at work (when the patient is a child).

Developing this structure in Azerbaijan – a strategic priority.

Ethical and legal framework

Every clinician should know:

- Confidentiality and its limits (danger to others, child protection);
- Informed consent;
- Legal framework for involuntary hospitalization;
- Report of child/elder abuse;
- Forensic assessment basics;
- Importance of documentation

Attention to your own health

Psychiatric work carries a high emotional load; this is the nature of the profession. Burnout, secondary trauma, empathetic fatigue are real risks for the clinician.

- Supervision and collegial consultation;
- Individual psychotherapy;
- Healthy lifestyle;
- Knowing your limits – you cannot save all patients.

Continuing education and development

ICD-11 will remain stable for decades, but:

- Treatment standards are updated every 3–5 years;

- New medications, new psychotherapeutic protocols emerge;
- Regular monitoring of the guidelines of organizations such as AACAP, APA, NICE, WFSBP, ISTSS;
- Peer-reviewed journals: Lancet Psychiatry, JAMA Psychiatry, American Journal of Psychiatry, World Psychiatry.

Purpose of the book: To contribute to the quality of psychiatric care in Azerbaijan, to patients and families, and to professional development. If the reader benefits from this book, the goal will be considered achieved.

I wish you healthy treatment, professional advancement, and to be helpful to your patients.

Kenan Rahimov

Baku, 2026